

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 18.03.2013

Einzelfallausdrucke

Kriterien: Paracetamol, Deutschland ab 1993, SMQ Hepatic disorders

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93018945

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19331218 Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	19920315				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mevinacor lovastatin	Oral	020 Mg milligram(s)/Day	1989-19920315	Pure hypercholesterolaemia
2 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	19920312-19920314	Debility, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other psoriasis			k.A.	concomitant disease

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94002083

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 15 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug abuse	19940124				recovered/resolved
2 Hepatic enzymes increased	19940124				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	5.5 G gram(s)/	-	
paracetamol			-	
2 theophylline	Oral	2.2 G gram(s)/	-	
theophylline			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94011991

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	19940820				Unknown
2 Feeling bad	19940820				Unknown
3 Leucopenia	19940820				Unknown
4 Appetite lost	19940820				Unknown
5 Hyperpyrexia	19940820				Unknown
6 Thrombocytopenia	19940820				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Rectal	/	-	
paracetamol			-	
2 timonil 300 retard	Oral	/	-	
carbamazepine			-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94009028

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19560805	Age: 37 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19940629	deathcause: Other sequelae of chronic liver disease
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting	19940626				Unknown
2 Abdominal pain lower	19940626				Unknown
3 Elevated liver enzyme levels	19940626				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol 500 stada paracetamol	Oral	5 G gram(s)/	199406-199406 -	Throat pain

Drugs (concomitant)

2 ass-ratiopharm acetylsalicylic acid	Oral	10 G gram(s)/	- -	Headache
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nondependent abuse of alcohol			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease
Acute alcoholic hepatitis			k.A.	concomitant disease
Alcoholic psychosis, other			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900164

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Glutamate dehydrogenase increased					Unknown
2 Gamma-GT increased					Unknown
3 SGPT increased					Unknown
4 SGOT increased					Unknown
5 Prothrombin time increased					Unknown
6 Hepatic failure					Unknown
7 Hepatitis toxic					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 haloperidol haloperidol	Unknown	/	-	-
2 paracetamol-ratiopharm paracetamol	Oral	/	-	-

Drugs (concomitant)

3 neurocil levomepromazine maleate	Unknown	/	-	-
4 taxilan perazine	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95004426

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19910329	Age: 4 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Reye's syndrome	199503				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 metifex 100mg ethacridine lactate	Oral	/	-	
2 panoral cefaclor	Oral	/	-	
3 paracetamol-ratiopharm 250 paracetamol	Rectal	/	-	
4 parkemed mefenamic acid	Rectal	/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005793

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19780429 Age: 17 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	19950806				Unknown
2 Nephritis interstitial	19950806				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005907

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19731005 Age: 22 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain abdominal	19951011				Unknown
2 Vomiting	19951011				Unknown
3 Nausea	19951011				Unknown
4 Hepatitis	19951011				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ben-u-ron	Unknown	/	-	
paracetamol			-	
2 pepdul	Unknown	/	-	
famotidine			-	
3 sempera	Oral	/	-	
itraconazole			-	
4 zantic	Unknown	/	-	
ranitidine hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96015811

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19831005	Age: 12 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Reye's syndrome	19960302				Unknown
2 Renal failure acute	19960302				Unknown
3 Cerebral oedema	19960302				Unknown
4 Pneumonia	19960302				Unknown
5 Hepatic failure	19960302				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ethrane	Unknown	/	-	
enflurane			-	
2 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96018835

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19620110	Age: 34 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Urinary tract infection	19960711				Unknown
2 Oedema pulmonary	19960711				Unknown
3 Toxic epidermal necrolysis	19960711				Unknown
4 Corneal perforation	19960711				Unknown
5 Gamma globulin decreased	19960711				Unknown
6 Shock lung	19960711				Unknown
7 Pancreatitis acute	19960711				Unknown
8 Hepatitis cholestatic	19960711				Unknown
9 Pancytopenia	19960711				Unknown
10 Stevens Johnson syndrome	19960711				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buscopan	Oral	/	-	
hyoscine butylbromide			-	
2 diazepam	Oral	/	-	
diazepam			-	
3 doxycycline	Oral	/	-	
doxycycline			-	
4 korodin	Oral	/	-	
camphor; convallaria tincture; crataegus extract; menthol; valerian tincture			-	
5 kyttä-balsam f	Topical	/	-	
methyl nicotinate; symphytum officinale			-	
6 muskel trancopal	Oral	/	-	
chlormezanone			-	
7 paracetamol	Oral	/	-	
paracetamol			-	
8 yxin	Ophthalmic	/	-	
tetryzoline hydrochloride			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001891

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19901129	Age: 6 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	1996				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol-ratiopharm 500 zaepfchen paracetamol	Rectal	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001240

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19651201 Age: 31 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	19970215				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97005366

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19690429 Age: 27 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	19970324				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol-ratiopharm 500 tabletten paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97008791

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: 7 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dizziness	19971029				Unknown
2 Griping abdominal	19971029				Unknown
3 Nausea	19971029				Unknown
4 Hepatic damage	19971102				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 elzogram	Unknown	/	-	
cefazolin sodium			-	
2 paracetamol	Unknown	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97500067

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Male	Birth: 19401022	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxic effect	19971005				recovering/resolving
2 Elevated liver enzyme levels	19971005				recovering/resolving
3 Somnolence	19971005				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diflucan 200 fluconazole	Oral	400 Mg milligram(s)/Day	19971001-19971005	-
2 fortum ceftazidime	Intravenous (not otherwis	004 G gram(s)/Day	19971001-19971005	Other cholecystitis
3 gernebcin tobramycin sulfate	Intravenous (not otherwis	160 Mg milligram(s)/Day	19971001-19971005	Pneumonia, organism unspecified
4 paracetamol paracetamol	Rectal	001 G gram(s)/Day	19971002-19971002	Other general symptoms

Drugs (concomitant)

5 berlosin metamizole	Intravenous (not otherwis	001 Df dosage form/Day	19971002-19971002	Other general symptoms
6 digitoxin 0,07 digitoxin	Oral	070 µg microgram(s)/Day	1996-19971005	Congestive heart failure
7 mst 60 mundipharma morphine sulfate	Oral	180 Mg milligram(s)/Day	19971002-19971003	Other general symptoms

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Heart failure, unspecified			k.A.	concomitant disease
Acute cholecystitis			k.A.	concomitant disease
Neoplasm of uncertain behaviour of other lymphatic and ha			k.A.	concomitant disease
Anaemia, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000595

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19231117 Age: 73 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Phosphatase alkaline increased	19971106				Unknown
2 Elevated liver enzyme levels	19971106				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98001469

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 chlormadinone chlormadinone	Oral	/	-	
2 furanthril tabletten 40 furosemide	Oral	/	-	
3 paracetamol paracetamol	Oral	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98001796

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19480120 Age: 50 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	19980121				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98002692

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19580331 Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	19980219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol stada paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98005277

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	199803				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 benuron paracetamol	Oral	/	- -	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98004910

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 17 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itching					Unknown
2 Rash					Unknown
3 Respiratory distress					Unknown
4 Raised liver enzymes					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol-ratiopharm 500 tabletten	Oral	/	-	
paracetamol			-	

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007258

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 14 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting					Unknown
2 Nausea					Unknown
3 Elevated liver enzyme levels					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-98008110

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19950414	Age: 19 Month	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Angioedema	19961213				Unknown
2 Ascites	19961213				Unknown
3 Mouth irritation	19961213				Unknown
4 Respiratory system disorder	19961213				Unknown
5 Stevens Johnson syndrome	19961213				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwis /	-
2	aldactone spironolactone	Intravenous (not otherwis /	-
3	ambisome amphotericin b	Intravenous (not otherwis /	-
4	arterenol norepinephrine hydrochloride	Intravenous (not otherwis /	-
5	bactrim sulfamethoxazole; trimethoprim	Intravenous (not otherwis /	-
6	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-
7	fentanyl fentanyl	Intravenous (not otherwis /	-
8	fluconazole fluconazole	Oral /	-
9	fortum ceftazidime	Intravenous (not otherwis /	-
10	hydromedin etacrynic acid	Intravenous (not otherwis /	-
11	lasix furosemide	Intravenous (not otherwis /	-
12	luminal phenobarbital	Intravenous (not otherwis /	-
13	methotrexat-dinatrium methotrexate sodium	Intravenous (not otherwis /	-
14	morphine hydrochloride morphine hydrochloride	Intravenous (not otherwis /	-
15	mucosolvan ambroxol hydrochloride	Intravenous (not otherwis /	-
16	novalgin metamizole	Intravenous (not otherwis /	-
17	pancuronium pancuronium	Intravenous (not otherwis /	-
18	paracetamol paracetamol	Oral /	-
19	phenhydan phenytoin	Unknown /	-
20	prostavasin alprostadiol	Intravenous (not otherwis /	-
21	racementhine-hydrochlorid epinephrine hydrochloride	Respiratory (inhalation) /	-
22	sobelin clindamycin hydrochloride	Intravenous (not otherwis /	-
23	soluvit vitamins nos	Intravenous (not otherwis /	-
24	TOBRAMYCIN tobramycin	intraperitoneal /	-
25	triglobe saft sulfadiazine; trimethoprim	Oral /	-
26	Urbason methylprednisolone	Intravenous (not otherwis /	-
27	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis /	-
28	vp-16 etoposide	Intravenous (not otherwis /	-

29	vp-16 etoposide	Intravenous (not otherwis /	-
30	zantic ranitidine hydrochloride	Intravenous (not otherwis /	-
31	zienam cilastatin sodium; imipenem	Intravenous (not otherwis /	-
32	zovirax aciclovir	Intravenous (not otherwis /	-

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008326

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19081010 Age: 88 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19961230				Unknown
2 Diarrhea	19970101				Unknown
3 SGOT increased	19961226				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	aldactone 100 spironolactone	Oral	/	-
2	ben-u-ron paracetamol	Oral	/	-
3	bvk roche for injection biotin; cyanocobalamin; nicotinamide; panthenol; pyridoxine hydrochloride; riboflavin sodium phosphate; thiamine hydrochloride	Intravenous (not otherwis	/	-
4	cefuroxime cefuroxime	Intravenous (not otherwis	/	-
5	cytotec 200 misoprostol	Oral	/	-
6	digimerck digitoxin	Intravenous (not otherwis	/	-
7	heparin heparin	Unknown	/	-
8	humatin paromomycin sulfate	Oral	/	-
9	lactulose lactulose	Oral	/	-
10	lasix furosemide	Oral	/	-
11	lopirin cor captopril	Oral	/	-
12	multibionta ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis	/	-
13	osyrol 100 - lasix furosemide; spironolactone	Oral	/	-
14	refobacin gentamicin sulfate	Intravenous (not otherwis	/	-
15	tarivid ofloxacin	Intravenous (not otherwis	/	-
16	vitintra ergocalciferol; phytomenadione; retinol; retinol palmitate	Intravenous (not otherwis	/	-
17	zienam cilastatin sodium; imipenem	Intravenous (not otherwis	/	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000717

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19601030 Age: 37 Year weight (kg): 66 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cough	19981019				Unknown
2 Dysaesthesia	19981031				Unknown
3 Red eye	19981031				Unknown
4 SGOT increased	19981031				Unknown
5 SGPT increased	19981031				Unknown
6 Gamma GT increased	19981031				Unknown
7 LDH increased serum	19981031				Unknown
8 Stevens Johnson syndrome	19981028				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	003 Df dosage form/Day	19981005-19981025	Headache
paracetamol			-	
2 viramune	Oral	400 Mg milligram(s)/Day	-19981101	HIV infection NOS
nevirapine			-	

Drugs (concomitant)

3 videx tabletten-150	Oral	003 Df dosage form/Day	19981005-19981102	HIV infection NOS
didanosine			-	
4 zerit 40mg-kapseln	Oral	002 Df dosage form/Day	19981005-19981102	HIV infection NOS
stavudine			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lymphadenitis, unspecified, except mesenteric			k.A.	concomitant disease
Staphylococcus infection in conditions classified elsewhere			k.A.	concomitant disease
Unspecified septicemia			k.A.	concomitant disease
Cellulitis and abscess of unspecified sites			k.A.	concomitant disease
Pyrexia of unknown origin			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000802

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19510305 Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	19981213				Unknown
2 Epidermal necrolysis	19981212				Unknown
3 Stevens Johnson syndrome	19981212				Unknown
4 SGPT increased	19981213				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allobeta 300	Oral	/	-	
allopurinol			-	
2 diclofenbeta uno	Oral	/	-	
diclofenac sodium			-	
3 herz ass-ratiopharm	Oral	/	-	
acetylsalicylic acid			-	
4 meditonsin h	Oral	/	-	
homeopatics nos			-	
5 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000892

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19420410 Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased	19980827				Unknown
2 AST increased	19980827				Unknown
3 Stevens Johnson syndrome	19980826				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	-
2 paracetamol paracetamol	Oral	/	-	-
3 tegretal 400 retard carbamazepine	Unknown	/	-	-
4 thym-uvocal thymus extract	Oral	/	-	-
5 transpulmin baby eucalyptus oil; pine oil	Respiratory (inhalation)	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000894

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19390317	Age: 58 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970410				recovered/resolved
2 Leukocytosis	19970430				Unknown
3 SGPT increased	19970424				Unknown
4 Gamma GT increased	19970424				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	atropine sulfate atropine sulfate	Intravenous (not otherwis /	-	
2	ben-u-ron paracetamol	Unknown /	-	Pyrexia of unknown origin
3	dipidolor piritramide	Intravenous (not otherwis /	-	Other general symptoms
4	disoprivan propofol	Oral /	-	
5	dormicum 7,5 midazolam maleate	Oral /	-	Unspecified psychophysiological malfunction
6	elzogram 2,0 cefazolin sodium	Intravenous (not otherwis /	-	Unspecified infectious and parasitic diseases
7	fluimucil acetylcysteine	Intravenous (not otherwis /	-	
8	fluimucil granulat acetylcysteine	Oral /	-	Acute upper respiratory infections of unspecified site
9	fortecortin dexamethasone	Oral /	-	Cerebral oedema
10	gastrozepin pirenzepine dihydrochloride	Intravenous (not otherwis /	-	Unspecified functional disorder of stomach
11	haes-steril 10% hetastarch; sodium chloride	Intravenous (not otherwis /	-	Volume depletion
12	heparin heparin	Intravenous (not otherwis /	-	Thrombosis
13	liquemin n heparin sodium	Subcutaneous /	-	Thrombosis
14	mucosolvan ambroxol hydrochloride	Oral /	-	
15	mucosolvan saft ambroxol hydrochloride	Oral /	-	Acute upper respiratory infections of unspecified site
16	nimbex cisatracurium besilate	Intravenous (not otherwis /	-	Unspecified disorder of muscle, ligament, and fascia
17	phenhydant phenytoin	Oral /	-	Other musculoskeletal symptoms referable to limbs
18	rohypnol 1 flunitrazepam	Oral /	-	
19	ulcogant-suspension sucralfate	Oral /	-	Unspecified functional disorder of stomach
20	ultima remifentanil hydrochloride	Intravenous (not otherwis /	-	Other specified complications of procedures, not elsewhere
21	zantic ranitidine hydrochloride	Oral /	-	Unspecified functional disorder of stomach

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000897

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19820802 Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19971210				Unknown
2 LDH increased	19971210				Unknown
3 Lymphocytosis	19971210				Unknown
4 Leukocytosis	19971210				Unknown
5 Stevens Johnson syndrome	19971124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxypen saft	Oral	/	-	
amoxicillin trihydrate			-	
2 ben-u-ron	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99901337

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19420317 Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	1998				Unknown
2 Halitosis	1998				Unknown
3 Delirium	1998				Unknown
4 Coma hepatic	1998		4 Week		Unknown
5 Renal failure NOS					Unknown
6 Sleepiness	1998				Unknown
7 Headache	1998				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol paracetamol	Unknown	/	-	
2 tegretal retard	Oral	1.6 G gram(s)/Day	199806-	Neuralgia, neuritis, and radiculitis, unspecified
3 tegretal 400 retard	Oral	002 Df dosage form/Day	1995-199806	Neuralgia, neuritis, and radiculitis, unspecified

Drugs (concomitant)

4 Acetylsalizylsaeure acetylsalicylic acid	Unknown	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			k.A.	concomitant disease
Alcohol dependence syndrome			k.A.	concomitant disease
Neuralgia, neuritis, and radiculitis, unspecified			k.A.	concomitant disease
Neuralgia, neuritis, and radiculitis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001245

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19940615 Age: 4 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19981225				Unknown
2 Fever	19981224				Unknown
3 Lymphocytes raised	19981227				Unknown
4 LDH increased	19981227				Unknown
5 SGOT increased	19981227				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 capval-saft noscaphine hydrochloride	Oral	/	-	-
2 paracetamol paracetamol	Unknown	/	-	-
3 pen-lich ts phenoxymethylpenicillin potassium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001815

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19840116 Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time increased	19990524				Unknown
2 PTT prolonged	19990524				Unknown
3 Gamma-glutamyltransferase incr.	19990524				Unknown
4 SGOT increased	19990524				Unknown
5 Leucopenia	19990524				Unknown
6 Hepatic damage	19990524				Unknown
7 Amylase serum increased	19990524				Unknown
8 Coagulation disorder	19990524				Unknown
9 Elevated liver enzyme levels	19990524				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 metamizol natrium	Oral	/	-	
metamizole			-	
2 paracetamol	Rectal	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001898

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19680101	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19941111	deathcause: Hepatic coma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19941101				Unknown
2 Exanthema	19941101				Unknown
3 Blistering	19941101				Unknown
4 Coma hepatic	19941101				Unknown
5 Sepsis	19941101				Unknown
6 Stomatitis	19941101				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil tropfen promethazine hydrochloride	Oral	/	19941025-19941026	Unspecified functional disorder of stomach
2 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19941023-19941029	Unspecified infectious and parasitic diseases
3 ben-u-ron paracetamol	Rectal	/	19941021-19941023	Pyrexia of unknown origin
4 bifiteral lactulose	Oral	/	19941023-19941031	Unspecified chronic liver disease without mention of
5 dopamin dopamine hydrochloride	Intravenous (not otherwis	/	19941027-	Renal failure, unspecified
6 furorese furosemide	Oral	/	19941028-19941029	Oedema
7 humatin paromomycin sulfate	Oral	/	19941023-19941031	Encephalopathy, unspecified
8 konakion phytomenadione	Unknown	/	-	Deficiency of vitamin K
9 lasix furosemide	Intravenous (not otherwis	/	19941024-19941031	Oedema
10 novalgin injektionsloesung metamizole	Intravenous (not otherwis	/	19941030-19941030	Pyrexia of unknown origin
11 quantalan colestyramine	Oral	/	19941031-	Cirrhosis of liver without mention of alcohol
12 rocephin ceftriaxone sodium	Intravenous (not otherwis	/	19941030-	Unspecified infectious and parasitic diseases
13 voltaren diclofenac sodium	Rectal	/	19941101-19941101	Other general symptoms

Drugs (concomitant)

14 l-polamidon methadone hydrochloride	Unknown	/	-	
15 maalox aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Unknown	/	19941005-19941006	Unspecified functional disorder of stomach

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug dependence of morphine type			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001902

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19921203 Age: 1 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	19940511				Unknown
2 Leukocytosis	19940511				Unknown
3 Gamma GT increased	19940511				Unknown
4 SGPT increased	19940511				Unknown
5 SGOT increased	19940511				Unknown
6 Stevens Johnson syndrome	19940511				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Oral	/	-	
amoxicillin			-	
2 paracetamol	Rectal	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001912

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19561020 Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocytosis	19940729				Unknown
2 Leucopenia	19940729				Unknown
3 SGOT increased	19940729				Unknown
4 SGPT increased	19940729				Unknown
5 Gamma GT increased	19940729				Unknown
6 Stevens Johnson syndrome	19940727				Unknown
7 LDH increased	19940729				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Unknown	/	-	-
2 diclofenac-ratiopharm sf diclofenac sodium	Intramuscular	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001913

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19741018 Age: 20 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19950107				Unknown
2 Genital lesion NOS	19950107				Unknown
3 Stomatitis	19950107				Unknown
4 Exanthema	19950107				Unknown
5 Gamma GT increased	19950107				Unknown
6 SGPT increased	19950107				Unknown
7 SGOT increased	19950107				Unknown
8 Leucopenia	19950107				Unknown
9 Erythema multiforme severe	19950107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Unknown	/	-	-
2 ben-u-ron paracetamol	Oral	/	-	-
3 fluimucil 200 brausetabletten acetylcysteine	Oral	/	-	-
4 nystatin "lederle" nystatin	Oral	/	-	-
5 tarivid 200 ofloxacin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001927

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19470730 Age: 50 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased					Unknown
2 Gamma GT increased					Unknown
3 LDH increased					Unknown
4 Fever	19970920				Unknown
5 Leukocytosis					Unknown
6 Hepatic failure	19970920				Unknown
7 SGOT increased					Unknown
8 Stevens Johnson syndrome	19970920				Unknown
9 Sore throat	19970919				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine	Oral	/	-	
acetylcysteine			-	
2 orelox	Oral	/	-	
cefprozime proxetil			-	
3 paracetamol	Oral	/	-	
paracetamol			-	
4 rulid	Oral	/	-	
roxithromycin			-	
5 silomat tropfen gegen reizhusten	Oral	/	-	
clobutinol hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001937

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19101012 Age: 83 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19941017				Unknown
2 Toxic epidermal necrolysis	19941010				Unknown
3 SGOT increased	19941016				Unknown
4 SGPT increased	19941017				Unknown
5 LDH increased	19941016				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 agarol emulsion paraffin, liquid; phenolphthalein	Oral	/	-	-
2 dalmadorm flurazepam hydrochloride	Oral	/	-	-
3 paracetamol paracetamol	Oral	/	-	-
4 pepdul mite famotidine	Oral	/	-	-
5 pepsin-wein ethanol; pepsin	Unknown	/	-	-
6 prednisolon-21-hydrogensuccinat natriumsalz prednisolone sodium succinate	Intravenous (not otherwis	/	-	-
7 sostril filmltabletten ranitidine hydrochloride	Oral	/	-	-
8 vergentan alozapride hydrochloride	Oral	/	-	-
9 vomex a injektionsloesung i.v. dimenhydrinate	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001943

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19401015 Age: 53 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lymphopenia	19941011				Unknown
2 Leukocytosis	19941015				Unknown
3 Stevens Johnson syndrome	19941015				Unknown
4 Exanthema	19941015				Unknown
5 Genital lesion NOS	19941015				Unknown
6 SGOT increased	19941024				Unknown
7 SGPT increased	19941024				Unknown
8 Stomatitis	19941015				Unknown
9 Gamma GT increased	19941024				Unknown
10 Fever	19941015				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	
paracetamol			-	
2 phenhydantoin	Oral	/	-	
phenytoin			-	
3 tarivid	Oral	/	-	
ofloxacin			-	
4 zentropil	Oral	/	-	
phenytoin			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00500129

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19270304	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatocellular damage	19990921				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 isoptin 120 mg verapamil hydrochloride	Oral	240 Mg milligram(s)/Day	-19990930 -	Essential hypertension, unspecified
2 paracetamol paracetamol	Oral	500 Mg milligram(s)/	-19990930 -	Abdominal pain
3 renacor enalapril maleate; hydrochlorothiazide	Oral	001 Df dosage form/Day	-19990930 -	Essential hypertension, unspecified
4 spalt acetylsalicylic acid	Oral	000 Mg milligram(s)/Day	1990-199909 -	Headache

Drugs (concomitant)

5 digitoxin awd 0,07 digitoxin	Oral	070 µg microgram(s)/Day	- -	Coronary atherosclerosis
6 loftan 8 mg salbutamol sulfate	Oral	016 Mg milligram(s)/Day	- -	Obstructive chronic bronchitis
7 prednisolon 5mg prednisolone	Oral	005 Mg milligram(s)/Day	1989- -	Obstructive chronic bronchitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001263

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes No

Patient

Initials: Sex: Female Birth: 19300130 Age: 70 Year weight (kg): 47 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Haemorrhage NOS	20000404		10 Day		recovered/resolved
2 Bullous eruption	20000404		10 Day		recovered/resolved
3 Creatine kinase increased	20000404		10 Day		recovered/resolved
4 Coagulation factors decreased	20000404		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 firin norfloxacin	Oral	/	-	
2 marcumar phenprocoumon	Unknown	/	-	
3 norfloxacin norfloxacin	Oral	002 Df dosage form/Day	20000331-20000403	Urinary tract infection, site not specified
4 paracetamol paracetamol	Oral	002 G gram(s)/Day	-	
5 tarivid ofloxacin	Oral	002 Df dosage form/Day	20000403-20000405	Urinary tract infection, site not specified

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00005281

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19760103 Age: 24 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased	200004				Unknown
2 Glutamate dehydrogenase increased	200004				Unknown
3 Coagulation abnormal	200004				Unknown
4 Hepatotoxic effect	20000404				Unknown
5 Suicide attempt	20000404				Unknown
6 SGOT increased	200004				Unknown
7 SGPT increased	200004				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol-ratiopharm 500 tabletten	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001480

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19410904 Age: 57 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990707				Unknown
2 Gamma GT increased	19990707				Unknown
3 SGPT increased	19990707				Unknown
4 Leukocytosis	19990707				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	10146828	pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral		/	-	
paracetamol				-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001505

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19840801 Age: 15 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	19991022				Unknown
2 SGPT increased	19991022				Unknown
3 SGOT increased	19991022				Unknown
4 Lymphocytes raised	19991022				Unknown
5 Leukocytosis	19991022				Unknown
6 Erythema multiforme severe	19991022				Unknown
7 Infection viral	19991022				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fluimucil 200 granulat acetylcysteine	Oral	/	-	-
2 lindoxyl loesung ambroxol hydrochloride	Oral	/	-	-
3 paracetamol 500 paracetamol	Oral	/	-	-
4 rhinotussal saft carbinoxamine; dextromethorphan; phenylpropanolamine	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001506

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19630623 Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19991028				Unknown
2 Gamma GT increased	19991019				Unknown
3 SGPT increased	19991016				Unknown
4 Lymphocytosis	19991016				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Oral	/	-	
amoxicillin			-	
2 cotrim	Oral	/	-	
sulfamethoxazole; trimethoprim			-	
3 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200085

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19390511 Age: 60 Year weight (kg): 95 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20000228		26 Day		recovered/resolved
2 Elevated liver enzyme levels	20000228		26 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	20000217-20000229	Cephalgia
2 polyglobin aminoacetic acid; immunoglobulin human normal; maltose	Intravenous (not otherwis	035 G gram(s)/Day	20000222-20000226	Unspecified inflammatory and toxic neuropathies
3 ranitidin basics 150mg ranitidine	Oral	150 Mg milligram(s)/Day	20000225-20000229	Other specified gastritis

Drugs (concomitant)

4 bisobloc bisoprolol fumarate	Oral	2.5 Mg milligram(s)/Day	-	Hypertension arterial
5 thyronajod henning levothyroxine sodium; potassium iodide	Oral	50 µg microgram(s)/Week	-	Goitre, unspecified
6 Urbason methylprednisolone	Oral	80 Mg milligram(s)/Day	20000225-20000229	Unspecified inflammatory and toxic neuropathies

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment
Thyroiditis, unspecified				concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00002139

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 63 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 marcumar phenprocoumon	Unknown	/	-	
2 paracetamol paracetamol	Oral	/	-	
3 vitamin c ascorbic acid	Unknown	/	-	

Primary source

Qualification: Pharmacist observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200149

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19460521 Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia					recovered/resolved
2 Hypertension					recovered/resolved
3 SGPT increased					recovered/resolved
4 SGOT increased					recovered/resolved
5 Phosphatase alkaline increased					recovered/resolved
6 Gamma-GT increased					recovered/resolved
7 Neutropenia					recovered/resolved
8 Eosinophilia					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprobay ciprofloxacin hydrochloride	Oral	250 Mg milligram(s)/Day	20000322-20000405	Fracture of upper end or unspecified part of radius and
2 optocillin mezlocillin sodium; oxacillin sodium	Intravenous (not otherwis	006 G gram(s)/Day	20000320-20000320	Fracture of upper end or unspecified part of radius and
3 paracetamol 500 paracetamol	Oral	1.5 G gram(s)/Day	20000321-20000405	Fracture of upper end or unspecified part of radius and
4 paracetamol 500 paracetamol	Oral	003 G gram(s)/Day	-	Fracture of upper end or unspecified part of radius and

Drugs (concomitant)				
5	antra mups omeprazole magnesium	Oral	/Day	20000321-20000405 -
6	carbostesin bupivacaine hydrochloride		/	- -
7	dalmadorm flurazepam hydrochloride	Oral	/	- -
8	haes-steril 6% hetastarch; sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/		20000322-20000327 -
9	hydergin dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Oral	45 Gtt drop(s)/Day	20000322-20000405 -
10	jonosteril calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/		20000320-20000320 -
11	karil calcitonin, salmon	Nasal	/Day	20000322-20000405 -
12	mono-embolex certoparin sodium	Subcutaneous	0.3 ml millilitre(s)/Day	20000319-20000402 -
13	novalgin metamizole	Oral	20 Gtt drop(s)/	- -
14	tramal long tramadol hydrochloride	Oral	/	20000321-20000321 -
15	tranxilium clorazepate dipotassium	Oral	20 Mg milligram(s)/	20000419-20000420 -
16	vioxx rofecoxib	Oral	025 Mg milligram(s)/Day	20000321-20000405 -

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000192

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19400326	Age: 60 Year	weight (kg): 77	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 CRP positive	20000925				recovered/resolved
2 Buccal mucosa ulceration	20000925				recovered/resolved
3 Gamma-glutamyltransferase incr.	20000925				recovered/resolved
4 Stevens Johnson syndrome	20000925				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bepanthen roche tabletten	Oral	/	-	Throat pain
panthenol			-	
2 dolo-dobendan	Oral	/	-	Throat pain
benzocaine; cetylpyridinium chloride			-	
3 maaloxan forte	Oral	/	-	Unspecified functional disorder of stomach
aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide			-	
4 paracetamol	Oral	/	-	Other general symptoms
paracetamol			-	
5 phenhydan	Oral	/	-	Epilepsy, unspecified
phenytoin			-	
6 remestan mite	Oral	/	-	Sleep disturbances
temazepam			-	
7 tramadol	Oral	/	-	Other general symptoms
tramadol			-	
8 zyrtec	Oral	/	-	Unspecified pruritic disorder
cetirizine hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Colitis, enteritis, and gastroenteritis of presumed infectious			k.A.	concomitant disease
Malignant neoplasm of brain, unspecified			k.A.	concomitant disease
Hepatitis non-A non-B			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.radiatio

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000197

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: 19470102	Age: 53 Year	weight (kg): 78	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Other specified septicaemias
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGOT increased	20000124				Unknown
2 Stevens Johnson syndrome	20000116		16 Day		Unknown
3 SGPT increased	20000122				Unknown
4 Lymphocytes raised	20000120				Unknown
5 Gamma GT increased	20000124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	Unspecified otitis media
2 ass 100 acetylsalicylic acid	Oral	/	-	Myeloid leukaemia, acute
3 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	-	Unspecified otitis media
4 gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	Unspecified otitis media
5 paracetamol paracetamol	Oral	/	-	Unspecified otitis media

Primary source

Qualification:	Physician	observestudytype:
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Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01000891

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: 20000529 Age: 7 Month weight (kg): 6 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	20010121		2 Day		recovered/resolved
2 Vomiting	20010121		2 Day		recovered/resolved
3 Hyperventilation	20010121		2 Day		recovered/resolved
4 Diarrhea	20010121		2 Day		recovered/resolved
5 Drug level increased	20010121		2 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	20010121-20010122	Pyrexia of unknown origin
2 paracetamol paracetamol	Rectal	001 G gram(s)/Day	-20010122	Pyrexia of unknown origin

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Down's syndrome			k.A.	concomitant disease
Unspecified defect of septal closure			k.A.	concomitant disease
Psychomotor retardation			k.A.	concomitant disease
Other symptoms concerning nutrition, metabolism, and dev			k.A.	concomitant disease
Microcephalus			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01002241

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19560624	Age: 44 Year	weight (kg): 125	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxic effect	200101				not recovered/not resolved
2 Icterus	200101				not recovered/not resolved
3 Ascites	200101				not recovered/not resolved
4 Elevated liver enzyme levels	200101				not recovered/not resolved
5 Bilirubinaemia	200101				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	-	Influenza with other respiratory manifestations
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			k.A.	concomitant disease
Obesity			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01002033

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 28 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic function abnormal	20001228		20 Day		recovered/resolved
2 Aggravation of existing disorder	20001228		20 Day		recovered/resolved
3 Raised liver enzymes	20001228		20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	2 G gram(s)/	20001220-	-
2 stangyl trimipramine maleate	Oral	075 Mg milligram(s)/Day	20001213-20001219	-
3 stangyl trimipramine maleate	Oral	100 Mg milligram(s)/Day	20001208-20001213	-
4 zyprexa olanzapine	Oral	005 Mg milligram(s)/Day	20001208-20001219	-

Drugs (concomitant)

5 seroxat paroxetine hydrochloride	Oral	020 Mg milligram(s)/Day	20001228-20010108	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other personality disorders			k.A.	concomitant disease
Nondependent abuse of alcohol			k.A.	concomitant disease
Obesity			k.A.	concomitant disease
Other, mixed, or unspecified drug abuse			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01003215

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19730101	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 CRP positive	20010305				recovered/resolved
2 LDH increased urine	20010305				recovered/resolved
3 Stevens Johnson syndrome	20010305				recovered/resolved
4 Conjunctivitis	20010305				recovered/resolved
5 SGOT increased	20010305				recovered/resolved
6 SGPT increased	20010305				recovered/resolved
7 Toxic epidermal necrolysis	20010305				recovered/resolved
8 Fever	20010305				recovered/resolved
9 Mucosal bulla	20010305				recovered/resolved
10 Rash haemorrhagic	20010305				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	-20010308	Cephalgia
paracetamol			-	
2 tegretal	Oral	200 Mg milligram(s)/Day	20010223-20010308	Epilepsy, unspecified
carbamazepine			-	

Drugs (concomitant)

3 novalgin	Unknown	/	-	Pyrexia of unknown origin
metamizole			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Erythema multiforme			k.A.	concomitant disease
Viral hepatitis B without mention of hepatic coma			k.A.	concomitant disease
Adjustment reaction, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01003457

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 48 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20001205		34 Day		recovering/resolving
2 Hepatic function abnormal	20001205		34 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dafalgan paracetamol	Oral	004 Df dosage form/Day	20001202-	
2 prazine promazine hydrochloride	Oral	150 Mg milligram(s)/Day	20001202-20001206	
3 spasmo-cibalgin allobarbital; aminophenazone; drofenine hydrochloride	Rectal	002 Df dosage form/Day	20001201-20001206	
4 stilnox zolpidem	Oral	020 Mg milligram(s)/Day	20000501-	
5 zyprexa olanzapine	Oral	005 Mg milligram(s)/Day	20001123-20010111	Unspecified psychosis

Drugs (concomitant)

6 efexor venlafaxine hydrochloride	Oral	375 Mg milligram(s)/Day	20001103-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Anorexia nervosa			k.A.	concomitant disease
Unspecified psychophysiological malfunction			k.A.	concomitant disease
Myalgia and myositis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01005555

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 14 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased serum					recovered/resolved
2 Elevated liver enzyme levels					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol 500 paracetamol	Oral	28 Df dosage form/	20010424-20010424	Unspecified neurotic disorder
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01005591

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 12 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus					Unknown
2 Elevated liver enzyme levels					Unknown
3 Prothrombin time increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ethrane enflurane	Unknown	/	-	-
2 midazolam midazolam	Unknown	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-
4 propofol propofol	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01006310

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: 19400804	Age:	weight (kg): 52	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased					recovered/resolved
2 General malaise					recovered/resolved
3 SGPT increased					recovered/resolved
4 Glutamate dehydrogenase increased					recovered/resolved
5 SGOT increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol 500 stada	Oral	002 Df dosage form/Day	-	Pyrexia of unknown origin
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified disorder of liver			k.A.	concomitant disease
Other and unspecified disorders of bone and cartilage			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease
Allergy, unspecified, not elsewhere classified			k.A.	concomitant disease

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01007842

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: 19600123 Age: 41 Year weight (kg): 60 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oedema legs					Unknown
2 Hepatosis	200109				recovered/resolved with sequel
3 Thrombopenia	200109				recovered/resolved with sequel
4 Elevated liver enzyme levels					Unknown
5 Leucopenia	200109				recovered/resolved with sequel
6 Anaemia normochromic	200109				recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol tabletten 500 mg von ct paracetamol	Oral	002 Df dosage form/Day	2000-2001 -	Headache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol dependence syndrome			k.A.	concomitant disease
Obsessive-compulsive disorders			k.A.	concomitant disease
Underweight			k.A.	concomitant disease
Abnormal loss of weight			k.A.	concomitant disease
Other specified cardiac dysrhythmias			k.A.	concomitant disease

Primary source

Qualification: Consumer or other non health professional observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004367

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 61 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaemia haemolytic	200203				not recovered/not resolved
2 Hepatic cirrhosis	200203				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fluctin fluoxetine hydrochloride	Oral	020 Mg milligram(s)/Day	2000-200109	Obsessive-compulsive disorders
2 paracetamol paracetamol	Unknown	/	-2002	-

Drugs (concomitant)

3 aspirin acetylsalicylic acid	Unknown	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Jaundice, unspecified, not of newborn			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: transaminasenerhoeung
Other, mixed, or unspecified drug abuse			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-02005294

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: Age: 12 Year weight (kg): BMI_int: Age fetus:

MedDRA LLT	Begin	End	duration	TtO	outcome
1 General malaise					recovered/resolved
2 Bilirubinaemia					recovered/resolved
3 Dizziness					recovered/resolved

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	6 G gram(s)/	-	
paracetamol			-	
2 rewodina 50	Oral	4 Df dosage form/	-	
diclofenac sodium			-	

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-02005223

Patient

Initials: Sex: Male Birth: 19621108 Age: 39 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestasis	20020322				recovered/resolved
2 Icterus	20020322				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 codeinum phosphoricum berlin-chemie codeine phosphate	Oral	001 Df dosage form/Day	20020318-20020322	-
2 paracetamol-ratiopharm 500 tabletten paracetamol	Oral	1.5 G gram(s)/Day	20020318-20020322	Pneumonia, organism unspecified
3 penicillin v "ratiopharm" phenoxymethylpenicillin potassium	Oral	4.5 Mg milligram(s)/Day	20020318-20020322	Pneumonia, organism unspecified
4 theophyllin retard-ratiopharm theophylline	Oral	500 Mg milligram(s)/Day	20020318-20020322	Pneumonia, organism unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bronchitis, not specified as acute or chronic			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02006107

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19870108	Age: 15 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels	20020415				not recovered/not resolved
2 Icterus	20020415				not recovered/not resolved
3 Nausea	20020413				Unknown
4 Vomiting	20020413				Unknown
5 Pain abdominal	20020413				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 loratadine loratadine	Oral	001 Df dosage form/Day	20020403-20020415	Allergic rhinitis due to pollen
2 paracetamol paracetamol	Oral	003 Df dosage form/Day	20020414-	Cephalgia

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02007236

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19440410	Age:	weight (kg): 92	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20020617	deathcause: Other sequelae of chronic liver disease
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxic effect					Unknown
2 Hepatic failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	20 G gram(s)/	-	Unspecified disorder of the
paracetamol			-	teeth and supporting structures

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nondependent abuse of alcohol			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02008127

Serious Yes	Death Yes	Lifethr. Yes	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19771223	Age: 24 Year	weight (kg): 34	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20020623	deathcause: Cerebral oedema
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Coma hepatic	20020619				Unknown
2 Hepatic failure	20020619				Unknown
3 Cerebral oedema	20020619				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 500 acetylsalicylic acid	Unknown	001 Df dosage form/Day	20020616-	
2 augmentan forte-trockensaft amoxicillin trihydrate; clavulanate potassium	Oral	006 ml millilitre(s)/Day	20020617-20020620	Acute tonsillitis
3 clindamycin clindamycin	Oral	1.2 G gram(s)/Day	20020613-20020616	Urinary tract infection, site not specified
4 ergenyl 300 valproate sodium	Unknown	005 Df dosage form/Day	-	
5 lamictal lamotrigine	Oral	100 Mg milligram(s)/Day	20020606-20020620	Epilepsy, unspecified
6 novalgin metamizole	Oral	020 Gtt drop(s)/Day	20020615-20020616	
7 paracetamol paracetamol	Oral	/	20020614-20020617	Acute tonsillitis
8 resimatil primidone	Unknown	1.5 Df dosage form/Day	-	

Drugs (concomitant)

9 effortil plus dihydroergotamine mesilate; etilefrine hydrochloride	Unknown	002 Df dosage form/Day	-	
10 mcp metoclopramide hydrochloride	Oral	020 Gtt drop(s)/Day	20020618-	
11 vomex a suppositorien dimenhydrinate	Rectal	001 Df dosage form/Day	20020618-20020619	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified mental retardation			k.A.	concomitant disease
Grand mal			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-02010754

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Male Birth: Age: weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	20020918				not recovered/not resolved
2 Hepatitis	20020918				not recovered/not resolved
3 Bilirubin increased	20020918				not recovered/not resolved
4 Rhinitis	200209				not recovered/not resolved
5 Pharyngitis	200209				not recovered/not resolved
6 Icterus	20020918				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 lamisil	Oral	/	200207-200208	
terbinafine hydrochloride			-	
2 paracetamol	Unknown	/	200209-	Nasopharyngitis
paracetamol			-	

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: ggt erhoeht

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02012014

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: Age: 36 Year weight (kg): 74 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oedema peripheral	20020315				recovered/resolved
2 Hepatomegaly	20020315				recovered/resolved
3 Weight increase	20020315				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 neupogen	Subcutaneous	700 µg microgram(s)/Day	20020314-20020317	
2 neupogen	Subcutaneous	400 µg microgram(s)/	200203-	
3 paracetamol	Unknown	500 Mg milligram(s)/	20020314-20020318	Bone pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pyelonephritis or pyonephrosis, not specified as acute or ch			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03000152

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: 19540913	Age: 48 Year	weight (kg): 87	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20020918		10 Day		recovered/resolved
2 Hyperbilirubinaemia	20020918		10 Day		recovered/resolved
3 Hepatitis	20020918		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	001 G gram(s)/Day	20020916-20020918	Influenza with other respiratory manifestations
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Onychomycosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03001872

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose intentional	2001				recovered/resolved
2 Hepatitis fulminant	200301				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ludiomil maprotiline hydrochloride	Oral	/	-2001	Depression
2 ludiomil maprotiline hydrochloride	Oral	60 Df dosage form/	2001-2001	Depression
3 paracetamol paracetamol	Unknown	/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-03014566

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Initials:	Sex: Male	Birth:	Age: 8 Year	weight (kg):	BMI int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Myopathy	200302				recovered/resolved
2 Hepatic function abnormal	200302				recovered/resolved
3 Creatine kinase increased	200302		10 Day		recovered/resolved
4 Muscle pain	200302		10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	/	200302-	
paracetamol			-	

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: rast pos.

Qualification: Physician
Literature: observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015058

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth:	Age: 23 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased					Unknown
2 Prothrombin time increased					Unknown
3 Elevated liver enzyme levels					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	4.0 G gram(s)/	-	Malignant neoplasm of colon,
paracetamol			-	unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral hepatitis B without mention of hepatic coma			k.A.	concomitant disease
Hepatitis non-A non-B			k.A.	concomitant disease
Malignant neoplasm of rectum			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: blasenektrophie
unbekannt			k.A.	concomitant disease: penisrekonstruktion 93

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03017084

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	200301				Unknown
2 Hepatitis	200301		6 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	500 Mg milligram(s)/Day	200301-200301 -	Headache
2 trancopal dolo flupirtine maleate	Oral	200 Mg milligram(s)/Day	200301-200301 -	Unspecified musculoskeletal disorders and symptoms

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute nasopharyngitis (common cold)			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: hepatitis b impfung 2000
Appendicectomy			k.A.	concomitant disease
Pyelonephritis or pyonephrosis, not specified as acute or ch			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03018901

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Female	Birth: 19790321	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-glutamyltransferase incr.	20031123				Unknown
2 Exanthema	20031123				not recovered/not resolved
3 Erythema exudativum	20031123				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Unknown	001 G gram(s)/Day	20031014-20031123	-
2 rebif interferon beta-1a	Subcutaneous	/	20031014-20031124	Multiple sclerosis

Drugs (concomitant)

3 pantoprazole sodium pantoprazole sodium	Unknown	040 Mg milligram(s)/Day	-	-
4 prednisolone prednisolone	Unknown	1 G gram(s)/	200309-20031123	Multiple sclerosis
5 tetrazepam tetrazepam	Unknown	050 Mg milligram(s)/Day	20031007-20031123	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-03019249

Patient

Reactions

Drugs (suspected or interacting)**Drugs (concomitant)**

Primary source

81 / 640

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-03019706

Serious Yes	Death	Lifethr. No	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19330130	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Klebsiella pneumoniae infection	20020928		13 Day		Unknown
2 Fever	20020928		13 Day		Unknown
3 Urinary infection	20020928		13 Day		Unknown
4 Ascites	20020928				Unknown
5 Gastritis	20020928				Unknown
6 Feeling bad	200209				Unknown
7 Vomiting	200209				Unknown
8 Nausea	200209				Unknown
9 Appetite lost	200209				Unknown
10 Sepsis	200209				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Kodein codeine	Unknown	/	-20020927	
2 mcp metoclopramide hydrochloride	Intravenous (not otherwis 1 Df dosage form/		-20020525	
3 paracetamol paracetamol	Unknown	500 Mg milligram(s)/Day	-20020829	
4 proleukin aldesleukin	Subcutaneous	018 Miu iu(1,000,000s)/D	-20020830	Secondary malignant neoplasm of kidney
5 roferon-a	Unknown	009 Miu iu(1,000,000s)/D	20020805-20020927	Secondary malignant neoplasm of kidney
6 5-fu	Unknown	1.9 G gram(s)/Day	20020805-20020923	Secondary malignant neoplasm of kidney

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n. nephektomie
Essential hypertension, unspecified			k.A.	concomitant disease
Diabetes mellitus without mention of complication			k.A.	concomitant disease
Atrophic gastritis			k.A.	concomitant disease
Secondary malignant neoplasm of lung			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000996

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: 19850605	Age:	weight (kg): 64	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever					Unknown
2 Raised liver enzymes					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 doxycyclin 100 mg doxycycline hyclate	Oral	003 Df dosage form/Day	20031008-20031010	Unspecified infectious and parasitic diseases
2 paracetamol paracetamol	Oral	001 G gram(s)/Day	20031005-20031010	Unspecified infectious and parasitic diseases

Drugs (concomitant)

3 valette dienogest; ethinylestradiol	Oral	001 Df dosage form/Day	20010101-	Contraception
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Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003543

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19411214	Age: 62 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ESR increased	20040305			22 Day	Unknown
2 Mucosal erosion	20040229			17 Day	not recovered/not resolved
3 Vesicle	20040228			16 Day	not recovered/not resolved
4 Skin reaction	20040221			9 Day	not recovered/not resolved
5 Skin disorder	20040225			13 Day	not recovered/not resolved
6 Rash pruritic	200402				not recovered/not resolved
7 Pain of skin	20040221			9 Day	not recovered/not resolved
8 Marginal keratitis					not recovered/not resolved
9 Lip swelling	20040229			17 Day	not recovered/not resolved
10 Heartburn					not recovered/not resolved
11 Flu-like symptoms	20040213			1 Day	not recovered/not resolved
12 Fever	20040220			8 Day	not recovered/not resolved
13 Eye redness	20040302			19 Day	not recovered/not resolved
14 Conjunctivitis					not recovered/not resolved
15 Chills	20040220			8 Day	not recovered/not resolved
16 Transaminases increased	20040301			18 Day	not recovered/not resolved
17 Stevens Johnson syndrome	20040221	20040312	21 Day	11 Day	not recovered/not resolved
18 CRP increased	20040301			18 Day	not recovered/not resolved
19 Allergic exanthema	20040215	20040218	4 Day	3 Day	not recovered/not resolved
20 Pruritus	20040221			9 Day	not recovered/not resolved
21 Creatinine increased	20040301			18 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil dimetindene maleate	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20040216-20040216	Itching 1440 Minute
2 PARACETAMOL paracetamol	Oral	/ Tablet	20040212-20040212	Fever 1440 Minute
3 PARACETAMOL paracetamol	Oral	/ Tablet	20040216-20040218	Fever 3 Day
4 prednisolon prednisolone	Oral	/	20040216-20040227	Allergic exanthema 12 Day
5 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	250 Mg milligram(s)/Day Solution for injection	20040216-20040216	Exanthema 1440 Minute
6 VOLTAREN RESINAT diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20040213-20040213	Fever 1440 Minute

Drugs (concomitant)

7 concor plus bisoprolol fumarate; hydrochlorothiazide	Unknown	/	-	Hypertension
8 KETEK telithromycin	Oral	1 Df dosage form/Day	20040224-20040224	Purulent bronchitis 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Upper respiratory tract infection			Unknown	
Hypertension arterial			Unknown	concomitant disease
Dermatitis			Unknown	due to drugs and medicaments taken internally

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.02.2008 DE-002147023-PHNU2004DE01207

Sender diagnosis: not reported

Narrative

Initial report received on 12 Mar 2004. Physician reported about patient who took one Voltaren (diclofenac) dose, about 10-12 days ago, for flu symptoms. Shortly thereafter, the patient showed an "extreme" skin reaction, and antibiotic treatment with Ketek (telithromycin) was started. The patient was hospitalized on 1 Mar 2004, and high dose prednisolone over 3 days (starting with 250mg/d, then tapering) was given. Due to affection of the conjunctiva, a dermatological consultation was requested, and Ketek (telithromycin) was stopped. Histology confirmed the diagnosis of Stevens Johnson syndrome. The patient remained on topical treatment, and the condition greatly improved during the following days. The patient's discharge was foreseen for 12 Mar 2004. Both Voltaren and Ketek were named as suspected drugs.

Follow up received on 24 Mar 2004 (written documentation from internal specialist): The patient took Voltaren on 22 Feb 2004, for rheumatic pains due to upper respiratory tract infection. Stevens Johnson syndrome started on ("about") 23 Feb 2004 and lasted for "more than 14 days". According to reporter, pruritic rash started after Voltaren use, and the picture of Stevens Johnson syndrome occurred one day after starting Ketek therapy. On an attached lab sheet, the following "lab data relevant for the AE" were reported: increased creatinine, increased transaminases, CRP increase and ESR increase. The condition did not abate after stopping therapy (no rechallenge since). Outcome was indicated as "not yet recovered". The reporting physician rated causality for Voltaren as probable (Ketek was not explicitly named as suspect drug).

Follow up received on 4 Feb 2008 from RegiSCAR project (internal case number 9150136). The Voltaren drug was specified as Voltaren Resinat (diclofenac, colestyramin); and according to this information (in contrast to previous data), Voltaren Resinat was used on 13 Feb 2004, for fever. No previous Voltaren drug use was recorded in this follow up. Fenistil (dimetinden maleate) was used on 16 Feb 2004, due to pruritus. On 13 Feb 2004, flu like symptoms occurred; and allergic exanthema was present from 15 until 18 Feb 2004. Chills and fever occurred on 20 Feb, and skin pains, pruritus, generalized macules and spots were visible on 21 Feb 2004. Purulent bronchitis was seen on 23 Feb, and skin changes further spread on 25 Feb 2004. The patient later also presented heartburn, marginal keratitis, eye redness, lip swellings and mucosal erosions. The first vesicle was seen on 28 Feb 2004. The patient was hospitalised on 1 Mar 2004. Outcome was reported as "not yet recovered". By the expert from the reporting center, the following drugs were mentioned as suspected (i.e. with a causality of at least possible): Voltaren Resinat, Fenistil inj., paracetamol, Solu-Decortin (prednisolone-21-hydrogensuccinate), and prednisolone; Ketek was not regarded as suspected. As other contributing or causal factor, flu infection was mentioned.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003768

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	Yes				

Patient

Initials:	Sex: Female	Birth: 19390322	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20040307	20040429	54 Day		recovering/resolving
2 Multiple organ failure					Unknown
3 Acute respiratory distress syndrome					Unknown
4 Hypotension	20040304	20040304	5 Minute		recovered/resolved
5 Haemothorax					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan paracetamol	Intravenous (not otherwis	004 G gram(s)/Day	20040304-20040307 4 Day	
2 suprane desflurane	Respiratory (inhalation)	/	20040304-20040304	Anaesthesia
			-	

Drugs (concomitant)

3	acetylcysteine acetylcysteine	Intravenous (not otherwis	900 Mg milligram(s)/	-
4	briserin clopamide; dihydroergocryptine mesilate; reserpine	Oral	/	-
5	cefuroxime cefuroxime	Unknown	80 Mg milligram(s)/	-
6	diazepam diazepam	Unknown	/	20040304-20040304
7	esomeprazole esomeprazole	Unknown	20 Mg milligram(s)/	-
8	fentanyl fentanyl	Unknown	/	20040304-20040304
9	furosemide furosemide	Intravenous (not otherwis	/	-
10	intramin g acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate	Unknown	1 l litre(s)/	-
11	inzolen-hk cobalt hydrogenaspartate; cobalt(ii)-dl-hydrogenaspartat 5 h(2)o; copper hydrogenaspartate; dexpanthenol; kaliumhydrogenaspartat; kupfer(ii)-dl-hydrogenaspartat 0.5 h(2)o; magnesium hydrogen aspartate; magnesiumhydrogenaspartat-dihydrat; mangan(ii)	Unknown	/	-
12	Kaliumchlorid potassium chloride	Intravenous (not otherwis	/	-
13	metronidazole metronidazole	Unknown	1.5 G gram(s)/	-
14	Natriumchlorid sodium chloride	Intravenous (not otherwis	20 ml millilitre(s)/	-
15	nfx aminophenazone; caffeine; codeine phosphate; phenacetin	Intravenous (not otherwis	500 ml millilitre(s)/	-
16	oliclinomel amino acids nos; glucose; soya oil	Intravenous (not otherwis	1.5 l litre(s)/	-
17	pancuronium pancuronium	Unknown	/	20040304-20040304
18	phytomenadione phytomenadione	Intravenous (not otherwis	/	-
19	ranitidine ranitidine	Intravenous (not otherwis	/	-
20	ringer-laktat loesung calcium chloride dihydrate; potassium chloride; sodium chloride; sodium lactate	Intravenous (not otherwis	500 ml millilitre(s)/	-
21	ropivacaine ropivacaine	Unknown	100 ml millilitre(s)/	-
22	thiopental thiopental	Unknown	/	20040304-20040304
23	valsartan valsartan	Oral	80 Mg milligram(s)/	-
24	xylitol xylitol	Intravenous (not otherwis	500 ml millilitre(s)/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatitis A			Unknown	
Toxoplasmosis			Unknown	
Hypertension arterial			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04900268

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19650408	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20040415				Unknown
2 Coma	20040415				Unknown
3 Pyrexia NOS	20040415				recovered/resolved
4 SGPT increased	200404				Unknown
5 Prothrombin time prolonged	200404				Unknown
6 Creatine phosphokinase increased	200404				Unknown
7 Sepsis	20040415				Unknown
8 Pneumonia	20040415				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cipralex escitalopram oxalate	Oral	015 Mg milligram(s)/Day	-	
2 folverlan folic acid	Oral	005 Mg milligram(s)/Day	-	
3 lamictal 100 lamotrigine	Oral	300 Mg milligram(s)/Day	-	Epilepsy, unspecified
4 paracetamol paracetamol	Rectal	001 G gram(s)/Day	-	
5 trileptal oxcarbazepine	Oral	1.8 G gram(s)/Day	-	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04006540

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19640907	Age: 39 Year	weight (kg): 81	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	20040314				recovering/resolving
2 Fever	20040314				recovered/resolved
3 Vomiting aggravated	20040314				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprofloxacin "ratiopharm" ciprofloxacin hydrochloride	Oral	500 Mg milligram(s)/Day	20040305-20040312	Colitis, enteritis, and gastroenteritis of presumed
2 paracetamol 500 paracetamol	Oral	55 Df dosage form/	20040305-20040324	Colitis, enteritis, and gastroenteritis of presumed

Drugs (concomitant)

3 mcp metoclopramide hydrochloride	Oral	060 Gtt drop(s)/Day	200403-20040325	Nausea and vomiting
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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04007054

Serious Yes Death Lifethr. Hospital. Yes Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19590130 Age: 44 Year weight (kg): 58 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Disease aggravation	20040115				recovered/resolved
2 Acute abdomen	20040115				recovered/resolved
3 Constipation	20040115				recovered/resolved
4 Tenesmus	20040115				recovered/resolved
5 Elevated liver enzyme levels	20040115		18 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol 500 paracetamol	Oral	13 G gram(s)/	20040112-20040115	Abdominal pain

Drugs (concomitant)

2 ass 100 acetylsalicylic acid	Oral	300 Mg milligram(s)/Day	1997-	Coronary atherosclerosis
3 corvaton retard 8mg tabletten molsidomine	Oral	008 Mg milligram(s)/Day	1997-20040115	Coronary atherosclerosis
4 dilatrend 25mg carvedilol	Oral	050 Mg milligram(s)/Day	1999-20040115	Hypertension arterial
5 isoket 40 isosorbide dinitrate	Oral	002 Df dosage form/Day	1997-20040115	Coronary atherosclerosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: alkohol-konsum
unbekannt			k.A.	concomitant disease: nikotin-konsum
unbekannt			k.A.	concomitant disease: z.n. koronar-stent
unbekannt			k.A.	concomitant disease: z.n. myokard-infarkt
unbekannt			k.A.	concomitant disease: z.n. koronar-ptca
Hypertension arterial			k.A.	concomitant disease
Pancreatitis chronic			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n. hoden-karzinom
Other and unspecified hyperlipidaemia			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04007646

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth:	Age: 44 Year	weight (kg): 75	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes					recovered/resolved
2 Coagulation disorder					recovered/resolved
3 Suicide attempt					Unknown
4 Delirium					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 alkohol	Oral	/	-	Unspecified neurotic disorder
ethanol			-	
2 lonarid	Oral	30 Df dosage form/	-	Unspecified neurotic disorder
codeine phosphate; paracetamol			-	
3 paracetamol-ratiopharm	Oral	10 G gram(s)/	-	Unspecified neurotic disorder
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04009308

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19540921 Age: weight (kg): 70 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased					Unknown
2 SGPT increased					Unknown
3 SGOT increased					Unknown
4 Pain abdominal					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	1 Df dosage form/	20040229- -	Influenza with other respiratory manifestations

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n.cholezystektomie 2004

Primary source

Qualification: Physician observestudytype:
 Literature:

case: DE-BFARM-04011002

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04900809

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests abnormal NOS					Unknown
2 Cholestasis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis /		-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04900849

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan paracetamol	Intravenous (not otherwis /		-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05001097

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19261208 Age: 77 Year weight (kg): 63 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	20041021		1 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	20041015-20041021	Other general symptoms

Drugs (concomitant)

2 decortin prednisone	Oral	010 Mg milligram(s)/Day	-	Rheumatoid arthritis
3 enahexal enalapril maleate	Oral	005 Mg milligram(s)/Day	-	Hypertonia
4 nexium mups 20 mg esomeprazole	Oral	020 Mg milligram(s)/Day	20041014-	Other specified gastritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Postinflammatory pulmonary fibrosis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05001098

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19510302 Age: 53 Year weight (kg): 87 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	20041026				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	1.5 G gram(s)/Day	20041014-20041025	-

Drugs (concomitant)

2 decortin prednisone	Oral	030 Mg milligram(s)/Day	20041015-	Psoriatic arthropathy
3 tramal long tramadol hydrochloride	Oral	300 Mg milligram(s)/Day	-	Other general symptoms
4 xanef 5 mg enalapril maleate	Oral	005 Mg milligram(s)/Day	-	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Occlusion and stenosis of basilar artery			k.A.	concomitant disease
Diabetes with unspecified complications			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05004470

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19400624	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Somnolence	20050302				Unknown
2 Drug level increased	20050302				Unknown
3 Delirium	20050302				Unknown
4 Asterixis	20050302				Unknown
5 Consciousness abnormal	20050302				Unknown
6 Intoxication	20050302				Unknown
7 Overdose	20050302				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	10 G gram(s)/Day	-20050302	Pain
2 QUILONUM RETARD lithium carbonate	Oral	2 Df dosage form/Day	-	Mania

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05005508

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			2 Day		recovered/resolved
2 Hepatic dysfunction NOS			2 Day		recovered/resolved
3 Overdose			2 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	5 G gram(s)/Day	-	Medication error
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug dependence			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Clin Toxicol (Phila); 2005, 43(1):31-37	
	Author: von Mach, et al	
	Title: Experiences of a poison center network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		2.4 bis 5,0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05005517

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 23 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic dysfunction NOS					recovered/resolved
2 Renal insufficiency			20 Day		recovered/resolved
3 Overdose			20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	45 G gram(s)/Day	- -	Medication error

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dehydration			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Clin Toxicol (Phila); 2005, 43(1):31-37 Author: von Mach, et al Title: Experiences of a poison with renal insufficiency in acetamiophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		2.4 bis 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05005523

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: Age: 35 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic dysfunction NOS					recovered/resolved
2 Renal insufficiency			2 Day		recovered/resolved
3 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 metamizole	Oral	25 G gram(s)/Day	-	
metamizole sodium			-	
2 paracetamol	Oral	5 G gram(s)/Day	-	
paracetamol			-	

Primary source

Qualification: Physician observestudytype:
 Literature: Journal: Clin Toxicol (Phila); 2005, 43(1):31-37
 Author: von Mach, et al
 Title: Experiences of a poison with renal insufficiency in acetamiophen overdose

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		2.4 bis 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05005557

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth:	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage					fatal
2 Renal insufficiency					fatal
3 Multiorgan failure					fatal
4 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	5000 Mg milligram(s)/	- -	Medication error

Primary source

Qualification:	Physician	observestudytype:
Literature:	J: Clin.Toxicol; 2005, 43(1):31-37 Author: Mach Ma, et al Title:Experiences of a poison center network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006717

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage					fatal
2 Multiorgan failure					fatal
3 Renal insufficiency					fatal
4 Overdose					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	/	-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:	Von Mach u.a. Experiences of a Poison Center Network J Toxicol Clin Toxicol 2005;43 (1):31-37	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006719

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 22 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage					Unknown
2 Overdose			24 Day		recovered/resolved
3 Renal insufficiency			24 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	9 G gram(s)/Day	- -	Medication error

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcoholic liver disease			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	J Toxicol Clin Toxicol; 2005; 43(1):31-37 Author: von March, Hermanns-Clausen, et al Title: Experiences of a Poison Center Network with Renal Insufficiency in Acetaminophen Overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006726

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose			23 Day		recovered/resolved
2 Hepatic damage			23 Day		recovered/resolved
3 Renal insufficiency					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen	Oral	/	-	Medication error
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse			Unknown	
Hepatitis C			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Clin.Toxicol (Phila); 2005, 43(1):31-37	
	Author: von Mach, et al	
	Title: Experiences of a poison center network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006732

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage					Unknown
2 Renal insufficiency			19 Day		recovered/resolved
3 Overdose			19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	10 G gram(s)/Day	- -	Medication error

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006733

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					recovered/resolved
2 Renal insufficiency			22 Day		recovered/resolved
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen	Oral	25 G gram(s)/Day	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37	
	Author: von Mach, et al	
	Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006736

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency					Unknown
2 Overdose					Unknown
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	5 G gram(s)/Day	-	Medication error
2 acetylsalicylic acid acetylsalicylic acid	Oral	500 Mg milligram(s)/Day	-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		über 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006761

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			8 Day		recovered/resolved
2 Hepatic damage					Unknown
3 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	8 G gram(s)/Day	-	
2 diclofenac diclofenac		1 G gram(s)/Day	-	Medication error
3 ibuprofen ibuprofen	Unknown	3.6 G gram(s)/Day	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		ueber 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006764

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose			30 Day		Unknown
2 Renal insufficiency			30 Day		recovered/resolved
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	15 G gram(s)/Day	-	Medication error
2 penicillin-v phenoxymethylpenicillin		20000000 Iu international	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		ueber 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05006152

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: 19280825	Age: 76 Year	weight (kg): 53	BMI_int: 21.23	Age fetus:
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Patientdeath

deathdate: 20050405	deathcause: Hepatic failure
deathdate: 20050405	deathcause: Cerebrovascular accident
deathdate: 20050405	deathcause: Multi-organ failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	200504				fatal
2 Multi-organ failure	20050404			3 Day	fatal
3 Anticoagulant effect decreased	20050404			3 Day	fatal
4 Elevated liver enzyme levels	20050404			3 Day	Unknown
5 Cerebrovascular accident	200504				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 carbostesin bupivacaine hydrochloride	Intravenous bolus	15 Mg milligram(s)/Day	20050404-20050404	Anaesthesia
2 etomidate etomidate	Intravenous bolus	10 Mg milligram(s)/Day	20050404-20050404	Anaesthesia
3 PERFALGAN 10 mg/ml Infusionsloesung paracetamol	Intravenous (not otherwis	2 G gram(s)/Day	20050402-20050404	Postoperative pain
4 sevoflurane sevoflurane	Respiratory (inhalation)	/	20050404-20050404	Anaesthesia

Drugs (concomitant)

5 acetylcysteine acetylcysteine	Unknown	/	-	-
6 amitriptyline amitriptyline	Unknown	/	-	-
7 dipidolor piritramide	Subcutaneous	15 Mg milligram(s)/Day	20050402-20050403	Pain management
8 furosemide furosemide	Unknown	/	-	-
9 prednisolone prednisolone	Unknown	/	-	-
10 promethazine promethazine hydrochloride	Unknown	/	-	-
11 verapamil verapamil	Unknown	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Joint prosthesis insertion	20050404		No	
Laryngectomy			Yes	
COPD			Yes	
Cardiac failure			Yes	
Hypertonus			Yes	
Larynx cancer			No	
Femoral neck fracture			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

case: DE-BFARM-05006844

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19881213 Age: 16 Year weight (kg): 65 BMI int: 23.03 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Coma NOS	20050406				recovered/resolved
2 Transaminases increased	20050406	20050415	10 Day		recovered/resolved
3 Myoclonus	20050406	20050515	40 Day		recovered/resolved
4 Aspiration	20050406				recovered/resolved
5 Stupor	20050406	20050415	10 Day		recovered/resolved
6 Somnolence	20050406				recovered/resolved
7 Agitation	20050405				recovered/resolved
8 Disorientation	20050406				recovered/resolved
9 Tachycardia NOS	20050406				recovered/resolved
10 Intentional overdose	20050405				recovered/resolved
11 Pyrexia	20050406				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 leponex clozapine	Oral	3 G gram(s)/Day	20050405-20050405	Intentional overdose
2 paracetamol paracetamol	Oral	/	20050405-	Intentional overdose
3 vomex a dimenhydrinate	Oral	/	20050405-	Intentional overdose

Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006916

Serious Yes	Death Yes	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency				2 Week	fatal
2 Multiorgan failure				2 Week	fatal
3 Hepatotoxic effect					fatal
4 Overdose NOS					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		5000 Mg milligram(s)/	-	Medication error
paracetamol			-	

Drugs (concomitant)

2 digoxin	/	-	
digoxin		-	
3 phenytoin	/	-	
phenytoin		-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	J: Clin. Toxicol (Philla); 20t05, 43(1):31-37 Author: Mach MA, et al Title: Experiences of a poison center network with renal insufficiency in acetaminophen overdose	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006917

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Male	Birth:	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency				1 Day	fatal
2 Multiorgan failure				1 Day	fatal
3 Hepatotoxic effect				1 Day	recovered/resolved with sequel
4 Overdose					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Drugs (concomitant)

2 chlorphenamine	/	-	
chlorphenamine		-	
3 coffeinum	/	-	
caffeine		-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	clin.toxicol (phila) 2005;43:31-7	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05010412

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19660701	Age: 38 Year	weight (kg): 94	BMI_int: 32.15	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20050623			2 Day	not recovered/not resolved
2 Prothrombin time prolonged	20050623			2 Day	not recovered/not resolved
3 LDH increased	20050623			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL-RATIOPHARM 500 paracetamol	Oral	6 Df dosage form/Day	20050622-20050623 2 Day	Headache NOS

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05011824

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: 19400630 Age: 64 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme increased	20050419			5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan paracetamol	Intravenous (not otherwis	1000 Mg milligram(s)/3 D	20050415-20050419	Pain
			5 Day	

Drugs (concomitant)

2 amlodipine amlodipine	Unknown	/	-	-
3 beta-sitosterin sitosterol	Unknown	/	-	-
4 levothyroxine sodium levothyroxine sodium	Unknown	/	-	-
5 oxygesic oxycodone hydrochloride	Unknown	/	20050414-20050421	8 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension NOS			Yes	
Surgery			Unknown	struma surgery
Colon cancer			Yes	
Thyroid operation NOS			Yes	
Colon operation NOS			Yes	

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05013780

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19540824	Age: 50 Year	weight (kg): 00070	BMI_int: 24.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Diarrhea					not recovered/not resolved
2 Urine discoloration					not recovered/not resolved
3 Tiredness					not recovered/not resolved
4 Elevated liver enzymes					not recovered/not resolved
5 Overdose					Unknown
6 Gastrointestinal discomfort					not recovered/not resolved
7 Faeces pale					not recovered/not resolved
8 Lack of motivation					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CEFAZOLIN cefazolin	Intravenous (not otherwis	00000002 G gram(s)/	20041021-	Total hip replacement
2 PERFALGAN IV paracetamol	Intravenous (not otherwis	/	20041021-20041022	Total hip replacement
3 SEVORANE sevoflurane		/	20041021-	Total hip replacement
4 voltaren resinat diclofenac resinate		280 Mg milligram(s)/Day	20041021-20041111	Total hip replacement

Drugs (concomitant)

5 acc acetylcysteine	/	20041021-	
6 dipidolor piritramide	/	20041021-	Total hip replacement
7 DISOPRIVAN propofol	/	20041021-	Total hip replacement
8 dolantin pethidine hydrochloride	/	20041021-	Total hip replacement
9 MONO-EMBOLEX heparin	/	20041021-20041210	Total hip replacement
10 nimbex cisatracurium besilate	/	20041021-	Total hip replacement
11 remifentanil hydrochloride remifentanil hydrochloride	/	20041021-	Total hip replacement
12 tranxilium clorazepate dipotassium	/	20041021-	Total hip replacement

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol use		200410	No	prior to Oct-2004.
Hepatitis A virus IgG antibody present	1982		Yes	
Penicillin allergy	1982		Yes	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
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Literature:

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit weitere Erläuterung
C-reactive protein	20051019	2.0	NA
C-reactive protein	2004	0.7	NA
Cholesterol	20051019	238: High	MG/DL
Gamma-glutamyltransferase	20051019	200: High	U/L
Glutamic-pyruvate transaminase	20051019	42.7: High	U/L
International normalised ratio	20041022	0.96	NA
International normalised ratio	20041028	0.88	NA
International normalised ratio	20041028	0.88	NA
Liver function tests		: High	NA
Mean corpuscular haemoglobin	20051019	32.6: High	PG
Mean corpuscular volume	20051019	95.4: High	FL
Serum glutamic-oxaloacetic transferase	20051019	11.5: Normal	U/L
Hepatitis viral test NOS		: Negative	NA
Autoantibody NOS		: Negative	NA
Quick's test	20041108	>130: High	%
Quick's test	20041104	>130: High	%
Quick's test	20041022	116: Normal	%
Quick's test	20041028	>130: High	%
LDL/HDL ratio	20051019	3.4	NA

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 09.02.2006 13100169

Sender diagnosis: not reported

Narrative

A male consumer reported that he experienced increased liver values after receiving an overdose of acetaminophen. The event had persisted for about 10 months on the date of this report. The event outcome was not reported. This report was considered medically important.

Supplemental information received on 27-Jan-2006 reported that a 50-year-old male consumer developed increased hepatic enzymes, chromaturia, pale feces, abdominal discomfort, diarrhea, fatigue, and apathy, after 2 days of therapy with a possible overdose of acetaminophen solution for infusion (dosage not provided) following a total hip replacement. Additional suspect therapy included cephalosporin, sevoflurane, and diclofenac + cholestyramine. Concomitant therapy included heparin, potassium clorazepate, propofol, remifentanyl, cisatracurium, meperidine, pirinitramide, and acetylcysteine. Medical history was significant for positive Hepatitis A antibody, penicillin hypersensitivity, and alcohol use prior to the month of surgery. At the time of reporting, the events had not yet resolved.

Of note, the form that this information was reported on was completely filled out by the consumer and signed by his physician.

As of 07 February 2006, this was the 1st report that contained the MedDRA PTs Chromaturia, Faeces pale, Abdominal discomfort, Fatigue, and/or Apathy, and the 3rd report of Diarrhoea, entered into the CARES safety database coincident with the use of acetaminophen solution for infusion.

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Under an overdose increase of hepatic enzymes and abdominal discomfort are expected according the current SPC of PERFALGAN. The events 6-8 are non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05013714

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19700829	Age:	weight (kg): 50	BMI_int: 19.53	Age fetus:
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Patientdeath

deathdate: 20050904	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rhabdomyolysis					Unknown
2 Cholestatic hepatitis					Unknown
3 Acute liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 doxycyclin doxycycline		/	- 4 Day	Otitis media NOS
2 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	- -	Otitis media NOS
3 paracetamol paracetamol	Ophthalmic	/	- -	Otitis media NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus NOS			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05015182

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: 19700829	Age: 34 Year	weight (kg): 50	BMI_int: 19.53	Age fetus:
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Patientdeath

deathdate:	deathcause: Hepatic coma
deathdate:	deathcause: Cerebral infarction

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20050822	20050904	14 Day		fatal
2 Hepatitis	20050822	20050904	14 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-saar 600mg clindamycin hydrochloride	Oral	/ Film-coated tablet	20050711- 5 Day	Otitis
2 dolormin ibuprofen lysinate	Unknown	2 Df dosage form/Day	-	
3 doxycycline doxycycline	Unknown	/	- 14 Day	
4 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	-	
5 paracetamol paracetamol	Oral	/ Film-coated tablet	-	Otitis

Drugs (concomitant)

6 insuman comb 15 insulin human; isophane insulin	Unknown	/	-	
7 insuman comb 25 insulin human; isophane insulin	Unknown	/	-	
8 novaminsulfon metamizole	Oral	/	20050708- -	Otitis
9 otobacid calcium pantothenate; chloramphenicol; dexamethasone; propyphenazone; tetracaine hydrochloride	Unknown	/	20050621- -	Otitis
10 panotile fludrocortisone acetate; lidocaine hydrochloride; neomycin sulfate; polymyxin b sulfate	Unknown	/	20050714-200507 -	Otitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Type I diabetes mellitus			Yes	
Allergic exanthema	20050715		Unknown	...auf Clindamycin

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

case: DE-BFARM-05016179

Patient

Initials: Sex: Female Birth: 19400710 Age: weight (kg): 81 BMI int: 31.25 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Failure hepatorenal					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan		4 G gram(s)/Day	-	Postoperative care
paracetamol			3 Day	

Drugs (concomitant)

2	nexium	/	-
	esomeprazole		-
3	norvasc	/	-
	amlodipine besilate		-
4	piritramide	/	-
	piritramide		-
5	vesdil	/	-
	ramipril		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol NOS normal			Yes	
Hepatitis (non-A non-B)			Yes	
Abdominal operation			Yes	upper
Neoplasm NOS			Yes	"first"
Duodenal cancer			Yes	operation

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05016255

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19540824	Age: 49 Year	weight (kg): 70	BMI_int: 24.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Diarrhea NOS	2004				Unknown
2 Liver function test abnormal	2004				not recovered/not resolved
3 Transaminases increased	200411				not recovered/not resolved
4 Hepatitis toxic	2004				not recovered/not resolved
5 Chromaturia	2004				Unknown
6 Urine output decreased	200410				Unknown
7 Dysgeusia	2004				Unknown
8 Gastrointestinal disorder NOS	200410				not recovered/not resolved
9 Ascites	200410				not recovered/not resolved
10 International normalised ratio increased	20041022		48 Day		Unknown
11 Prothrombin time shortened	20041022		48 Day		Unknown
12 Hypersomnia	200410				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mono-embolex certoparin sodium		/	-20041210	
2 perfalgan paracetamol	Parenteral	2 Df dosage form/	20041022-	
3 Voltaren Resinat diclofenac resinate		2 Df dosage form/Day	20041021-20041111 22 Day	Hip surgery

Drugs (concomitant)

4 acc long acetylcysteine		/	-20041022	
5 cephalosporin c cephalosporin c	Parenteral	2 G gram(s)/	20041021-20041021	
6 dipidolor piritramide		/	-	
7 nimbex cisatracurium besilate		/	-	
8 pethidine pethidine		/	-	
9 remifentanil remifentanil		/	-	
10 sevorane sevoflurane	Respiratory (inhalation)	/	20041021-20041021	
11 tranxilium clorazepate dipotassium		/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Blood immunoglobulin G increased			Yes	
Drug hypersensitivity			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05016207

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN 10 mg/ml Infusionsloesung		6 G gram(s)/Day	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Liver transplant			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05018677

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19330827	Age: 71 Year	weight (kg): 68	BMI_int: 23.53	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20050221			5 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN 10 mg/ml Infusionsloesung paracetamol	Intravenous (not otherwis /		20050217-20050302 14 Day	Sepsis

Drugs (concomitant)					
2	acc	Intravenous (not otherwis 300 Mg milligram(s)/	20050216-20050222		
	acetylcysteine		7 Day		
3	actrapid	/	20050216-20050305		
	insulin human		18 Day		
4	antithrombin iii	Intravenous (not otherwis 1 Kiu iu (1000s)/	20050217-20050220		
	antithrombin iii		4 Day		
5	antra	Intravenous (not otherwis 40 Mg milligram(s)/Day	20050216-20050218		
	omeprazole		3 Day		
6	aspirin	Oral 4*500 Mg milligram(s)/We	1995-20050216	Rheumatism	
	acetylsalicylic acid		-		
7	ass 100	Oral 100 Mg milligram(s)/Day	20050218-20050306		
	acetylsalicylic acid		17 Day		
8	beloc i.v.	Intravenous (not otherwis /	20050218-20050223		
	metoprolol tartrate		6 Day		
9	beloc-zok mite	Oral /	20050218-20050222		
	metoprolol succinate		5 Day		
10	clethane	0.4 ml millilitre(s)/Day	20050216-20050306		
	enoxaparin		19 Day		
11	delix 2,5	Oral 1 Df dosage form/Day	20050222-20050222		
	ramipril		-		
12	diazepam	5 Mg milligram(s)/Day	20050216-20050216		
	diazepam		-		
13	digitoxin	/	20050216-20050217		
	digitoxin		2 Day		
14	digitoxin 0,07	/	20050221-20050225		
	digitoxin		5 Day		
15	dobutamine	Intravenous (not otherwis /	20050216-20050216		
	dobutamine		-		
16	dormicum	/	20050216-20050223		
	midazolam maleate		8 Day		
17	erythromycin	Intravenous (not otherwis 2 G gram(s)/Day	20050217-20050223		
	erythromycin		7 Day		
18	etomidate	15 Mg milligram(s)/Day	20050216-20050216		
	etomidate		-		
19	falicard	10 Mg milligram(s)/Day	20050216-20050216		
	verapamil hydrochloride		-		
20	hydrocortisone	100 Mg milligram(s)/Day	20050216-20050224		
	hydrocortisone		9 Day		
21	irenat	20 Gtt drop(s)/Day	20050216-20050216		
	sodium perchlorate		-		
22	jonosteril	/	20050216-20050222		
	calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride		7 Day		
23	kalium	Intravenous (not otherwis /	20050220-20050306		
	potassium		15 Day		
24	lasix	Intravenous (not otherwis /	20050219-		
	furosemide		-		
25	movicol	Oral 1 Df dosage form/Day	20050218-20050218		
	macrogol; potassium chloride; sodium bicarbonate; sodium chloride		-		
26	nexium 40	40 Mg milligram(s)/Day	20050219-20050306		
	esomeprazole		16 Day		
27	noradrenaline	/	20050216-20050220		
	norepinephrine		5 Day		
28	pipril 4 g	Intravenous (not otherwis 12 G gram(s)/Day	20050216-20050223		
	piperacillin sodium				

Drugs (concomitant)

29	prostagutt populus extract; sabal extract; urtica extract	Oral	2 Df dosage form/Day	8 Day 200112-20050216	Prostate adenoma
30	rocephin 2g ceftriaxone sodium	Intravenous (not otherwis	2 G gram(s)/Day	- 20050216-20050223	
31	sufenta sufentanil citrate		/	8 Day 20050217-20050223	
				7 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pneumonia			Unknown	
Atrial fibrillation paroxysmal			Unknown	
Herpes infection			Unknown	
Pharyngotonsillitis			Unknown	
Drug eruption NOS			Unknown	
Myopathy			Unknown	
Renal failure acute	200502		Unknown	
Smoker	1968	1979	No	
Pleural effusion	200502		Unknown	
Prostatic hyperplasia	2001		Unknown	
Stomatitis			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06003068

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
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1 [Liver function test abnormal](#)**Drugs (suspected or interacting)**

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN 10 mg/ml Infusionsloesung	Intravenous (not otherwis /		-	
paracetamol	Intravenous infusion		-	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)[Bristol-Myers Squibb Company, 26.01.2006 DE-Bristol-Myers Squibb Company-13249305](#)

Sender diagnosis: not reported

Narrative

A physician reported, via a BMS sales representative and BMS-Germany (local #2006011606), that "some older patients" (specific patient demographics not provided) developed a massive increase in liver values while receiving intravenous acetaminophen. The event was considered medically significant. The patients had undergone joint surgery. Additional information will be requested.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen intravenous

Event number; label*; causality**

#1; expected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: The information currently available is not sufficient for a final scientific assessment. The occurrence of an increase of liver transaminases is listed in the SPC for PERFALGAN.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06004277

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN	Intravenous (not otherwis	500 Mg milligram(s)/	-	
paracetamol	Intravenous infusion		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Underweight			Yes	less than 50 kg

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 03.02.2006 DE-Bristol-Myers Squibb Company-13257977

Sender diagnosis: not reported

Narrative

A physician reported, via a BMS sales representative and BMS Germany (2006012305), that a female patient experienced drastically increased liver enzymes after receiving an acetaminophen 500 mg infusion. The physician noted that the patient was underweight (less than 50 kg). Laboratory values were not reported. The event outcome was not reported. The reporter's causality assessment was not reported. This report was considered medically important.

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen iv
 Event number; label*; causality**
 #1; expected; PSB

* according to current Summary of Product Characteristics
 ** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

No further comments.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06004954

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure					Unknown
2 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 parfalgan	Intravenous (not otherwis /		-	
paracetamol	Intravenous infusion		-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 08.02.2006 DE-Bristol-Myers Squibb Company-13262100

Sender diagnosis: not reported

Narrative

A physician reported, via a BMS sales representative and BMS Germany (2006012609), that a child (age and gender not reported) was hospitalized due to severe hepatic failure after receiving an overdose of intravenous acetaminophen. The event outcome was not reported.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen iv
 Event number; label*; causality**
 #1; expected; PSB
 #2; not applicable; not applicable

* according to current Summary of Product Characteristics
 ** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

No further comments.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06006554

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials: Sex: Male Birth: 19540824 Age: 50 Year weight (kg): 00070 BMI_int: 24.22 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic drug-induced					not recovered/not resolved
2 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin w/clavulanic acid		/	-	
amoxicillin; clavulanate potassium			-	
2 CEFAZOLIN	Intravenous (not otherwis	00000002 G gram(s)/	20041021-	Total hip replacement
cefazolin			-	
3 HEPARINE		/	-	
heparin sodium			-	
4 perfalgan IV		00000001 G gram(s)/	20041021-20041022	Total hip replacement
paracetamol		124	2 Day	
5 SEVORANE		/	20041021-	Total hip replacement
sevoflurane			-	
6 voltaren	Oral	00000075 Mg milligram(s)	20041021-20041022	
diclofenac sodium			2 Day	
7 Voltaren Resinat		280 Mg milligram(s)/Day	20041021-20041111	Total hip replacement
diclofenac resinate			22 Day	

Drugs (concomitant)

8 acc		/	20041021-	Total hip replacement
acetylcysteine			-	
9 dipidolor	Intravenous (not otherwis	00000011 Mg milligram(s)	20041021-	Total hip replacement
piritramide			-	
10 DISOPRIVAN		/	20041021-	Total hip replacement
propofol			-	
11 dolantin		00000020 Mg milligram(s)	20041021-	Total hip replacement
pethidine hydrochloride			-	
12 MONO-EMBOLEX		/	20041021-20041210	Total hip replacement
heparin			51 Day	
13 nimbex		/	20041021-	Total hip replacement
cisatracurium besilate			-	
14 remifentanil hydrochloride		/	20041021-	Total hip replacement
remifentanil hydrochloride			-	
15 tranxilium		/	20041021-	Total hip replacement
clorazepate dipotassium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chromosomal mutation			Unknown	Heterocygous mutation C282Y and H63 D without HFE-manifestation
Infection NOS	1981		Unknown	
Transaminases increased	1981		Unknown	after receiving mebendazol.
Alcohol use		200410	Unknown	prior to Oct-2004.
Penicillin allergy	1982		Unknown	
Hepatitis A virus IgG antibody present	1982		Unknown	
Hip dysplasia			Unknown	bilateral
Unspecified disorder of liver	1981		Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	
Quick's test	20041022	116: Normal	%	
Quick's test	20041028	>130: High	%	
Quick's test	20041104	>130: High	%	
Quick's test	20041108	>130: High	%	
International normalised ratio	20041022	0.96	NA	
International normalised ratio	20041028	0.88	NA	
International normalised ratio	20041028	0.88	NA	
Serum glutamic-oxaloacetic transferase	20051019	11.5: Normal	U/L	
Glutamic-pyruvate transaminase	20051019	42.7: High	U/L	
Gamma-glutamyltransferase	20051019	200: High	U/L	
Cholesterol	20051019	238: High	MG/DL	
LDL/HDL ratio	20051019	3.4	NA	
Mean corpuscular volume	20051019	95.4: High	FL	
Mean corpuscular haemoglobin	20051019	32.6: High	PG	
C-reactive protein	2004	0.7	NA	
Hepatitis viral test NOS		: Negative	NA	
Autoantibody NOS		: Negative	NA	
C-reactive protein	20051019	2.0	NA	
Glutamic-oxaloacetic transferase	20041130	61: High	U/L	
Glutamic-pyruvate transaminase	20041130	206: High	U/L	
Gamma GT	20041130	705: High	U/L	
Gamma GT	200507	158: High	U/L	
Glutamic-oxaloacetic transferase	200507	20: Normal	U/L	
Glutamic-pyruvate transaminase	200507	36: Normal	U/L	
Alkaline phosphatase	20041130	188: High	U/L	
Alkaline phosphatase	20050421	103: Normal	U/L	
Ultrasound abdomen		: Normal	NA	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 20.02.2006 DE-Bristol-Myers Squibb Company-13100169

Sender diagnosis: not reported

Narrative

A male consumer reported that he experienced increased liver values after receiving an overdose of acetaminophen. The event had persisted for about 10 months on the date of this report. The event outcome was not reported. This report was considered medically important.

Supplemental information received on 27-Jan-2006 reported that a 50-year-old male consumer developed increased hepatic enzymes, chromaturia, pale feces, abdominal discomfort, diarrhea, fatigue, and apathy, after 2 days of therapy with a possible overdose of acetaminophen solution for infusion (dosage not provided) following a total hip replacement. Additional suspect therapy included cephalosporin, sevoflurane, and diclofenac + cholestyramine. Concomitant therapy included heparin, potassium clorazepate, propofol, remifentanyl, cisatracurium, meperidine, pirinitramide, and acetylcysteine. Medical history was significant for positive Hepatitis A antibody, penicillin hypersensitivity, and alcohol use prior to the month of surgery. At the time of reporting, the events had not yet resolved.

Of note, the form that this information was reported on was completely filled out by the consumer and signed by his physician.

As of 07 February 2006, this was the 1st report that contained the MedDRA PTs Chromaturia, Faeces pale, Abdominal discomfort, Fatigue, and/or Apathy, and the 3rd report of Diarrhoea, entered into the CARES safety database coincident with the use of acetaminophen solution for infusion.

Follow-up information was received from the physician on 07-Feb-2006 and 10-Feb-2006. The patient received 1 gram of acetaminophen on 21-Oct-2004 at 0930 hours. The patient had been diagnosed with acute drug toxic hepatitis after the administration of non-steroidal anti-rheumatics, heparine and amoxicillin + clavulanic acid. Therefore, heparine and amoxicillin + clavulanic acid were added as suspect drugs. The acute drug toxic hepatitis was characterized by dark urine, nearly colorless, pale stool, severe gastrointestinal discomfort with diarrhea, tiredness, lack of drive, and pathological liver values and pathological INR and Quick values. Following the administration of the acetaminophen IV, the patient had a marked increased in gamma-GT of up to 705 U/L (nl: <55 U/L). His GOT increased to 61 (nl: <35), GPT increased to 206 (nl: <45U/L), and alkaline phosphatase increased to 188 (nl: 56-119 U/L). Other tests included an ultrasound of the abdomen that was without pathological findings. The patient used no permanent medication and he had no alcohol consumption. On an unspecified date after 21-Jul-2005 the gamma-gt value was improved to 158, the GOT and GPT values were normal. The events were considered probably related to the acetaminophen therapy.

Medical history also included liver disease with increased transaminases, unclear infection after a stay in a foreign country, increased in transaminases after receiving mebendazole, heterocygous mutation C282Y and H63 D without HFE manifestation, and bilateral hip dysplasia.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06007001

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminase value increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan iv	Intravenous (not otherwis	2 G gram(s)/Day	-	Postoperative pain
paracetamol	Intravenous infusion		00003 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Surgery			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Transaminase NOS		: High		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 22.02.2006 DE-Bristol-Myers Squibb Company-13281811

Sender diagnosis: not reported

Narrative

A physician, via BMS Germany (2006021006), reported that a female patient experienced a distinct increase in liver transaminases while receiving acetaminophen IV for post-operative pain. The patient received 1 g two times daily of acetaminophen for three days. The laboratory values were not reported. The event outcome was not reported. The physician assessed the causality as related to acetaminophen IV.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophene
 Event number; label*; causality**
 #1; expected; PSB

* according to current Summary of Product Characteristics
 ** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

No further comments.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06012361

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19560512	Age: 49 Year	weight (kg): 111	BMI_int: 37.52	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	20060306			29 Day	recovered/resolved
2 Brown urine	20060304			27 Day	recovered/resolved
3 Bilirubin total increased	20060306			29 Day	recovered/resolved
4 Transaminases increased	20060306			29 Day	recovered/resolved
5 Intestinal hypomotility	20060311			34 Day	recovered/resolved
6 Pancreas lipomatosis	20060311			34 Day	recovered/resolved
7 Acute liver damage	200603				recovered/resolved
8 Acute renal failure	200603				recovered/resolved
9 Alkaline phosphatase increased	20060306			29 Day	recovered/resolved
10 Bundle branch block left					Unknown
11 Creatinine increased	20060306			29 Day	recovering/resolving
12 CRP increased	20060306			29 Day	recovering/resolving
13 Direct bilirubin increased	20060306			29 Day	recovered/resolved
14 Effusion pleural	20060311			34 Day	recovered/resolved
15 Hypertension arterial					Unknown
16 Icterus	20060304			27 Day	recovered/resolved
17 Influenza	20060228	20060305	6 Day	23 Day	recovered/resolved
18 Leukocytosis	200603				recovering/resolving
19 Lipase increased	200603				recovering/resolving
20 Lung disorder	200603				recovered/resolved
21 Pancreatitis	200603				recovered/resolved
22 Red throat	200603				recovered/resolved
23 Steatosis hepatic					not recovered/not resolved
24 Tongue white					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Cranoc 40mg Hartkapseln fluvastatin sodium	Oral	80 Mg milligram(s)/Week	20060206-20060306 29 Day	Hypercholesterolemia
2 paracetamol paracetamol	Oral	1500 Mg milligram(s)/Day	200603-200603 4 Day	Influenza like illness

Drugs (concomitant)

3 CONCOR bisoprolol fumarate	5 Mg milligram(s)/Day	-
4 Disalunil hydrochlorothiazide	1 Df dosage form/Day	-
5 moxonidine moxonidine	1 Df dosage form/Day	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza	20060228	200603	No	
Adipositas			Yes	
Hypertension arterial			Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 09.05.2007 DE-002147023-PHNU2006DE01573

Sender diagnosis: not reported

Narrative

Initial report received on 04 Apr 2006 via Astellas Germany (marketing fluvastatin as Cranoc / Cranoc 80 Retard), report from German Health Authority, originating source hospital internal specialist (Astellas case number DE02006-0064). The patient started Cranoc 80 Retard therapy on 06 Feb 2006, for hypercholesterolemia. During the week from 28 Feb 2006, until 05 Mar 2006, patient suffered from influenzal infection and was therefore treated with paracetamol 500 t.i.d.; the influenzal infection was not mentioned as adverse event but under the "current condition / anamnesis" section. On 04 Apr 2006, patient showed beerbrown urine and silent icterus. Laboratory tests (apparently from 06 Mar 2006) revealed increased liver function tests (but no upper limits of normal and sometimes no units were given): total bilirubin 42.0, direct bilirubin 30.0, ALAT 9.9 "micromol/l" (?), ASAT 2.46, gamma GT 17.52, AP 3.95. Cranoc was discontinued on 06 Mar 2006. On the following day (07 Mar 2006), liver values slightly improved, as follows: total bilirubin 35.5, direct bilirubin 23.6, ALAT 8.6, ASAT 2.1, gamma GT 15.61. Sonography showed steatosis hepatic but cholecystolithiasis was excluded. An ERCP was done on 9 Mar 2006; despite normal findings patient developed pancreatitis after the ERCP. Outcome was reported as "not yet recovered". The originating source (physician) ticked Cranoc 80 Retard as causally related.

Follow up report received on 05 May 2006. The following additional symptoms were mentioned: liver parenchyma disorder, acute renal failure, hypertension, white tongue, red throat, pulmonary breath noises, lipase increase max. 138 (no units provided), CRP increase 315, leukocytes 28, creatinine 160, ECG left bundle blockade, pancreatic lipomatosis, abdominal free fluid and intestinal atony. Patient's condition improved after antibiotic therapy with Rocephin (ceftriaxon). Signs of inflammation disappeared. Lab parameters (especially liver values) returned to normal ranges. Patient had been hospitalized due to jaundice. The other events occurred or were found during hospitalization. No seriousness criteria were provided for these events. Patient recovered and was discharged from hospital. The relationship to Cranoc and paracetamol was assessed as suspected for jaundice.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06012154

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19511014	Age: 54 Year	weight (kg): 00081	BMI_int: 28.03	Age fetus:
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Patientdeath

deathdate:	deathcause: Hepatic failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sepsis					fatal
2 Coagulation disorder					fatal
3 Hepatic failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan IV paracetamol paracetamol	Intravenous (not otherwis	00000003 G gram(s)/001 124	20051130-20051214 15 Day	Fever
2 PERFALGAN 10 mg/ml Infusionsloesung	Intravenous (not otherwis	00000003 G gram(s)/001 124	20051130-20051214 15 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcoholic			Yes	
Liver damage			Yes	
Sepsis			Yes	
Peritonitis			Unknown	
Duodenal ulcer perforated			Unknown	
Fever			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test	20051128	53	%	
Quick's test	20051216	21	%	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 27.03.2006 DE-Bristol-Myers Squibb Company-13326004

Sender diagnosis: not reported

Narrative

A physician reported that 2 patients with sepsis and pre-existing liver damage developed liver failure during therapy with acetaminophen solution for infusion (dosage not specified). This file pertains to patient 1 of 2, an alcoholic male who developed hepatic failure, a coagulation disorder, and expired.

Cross reference file 13326020 (patient 2 of 2).

As of 24 March 2006, this was the 15th report that contained the MedDRA PT Hepatic failure (the 3rd with a fatal outcome), entered into the CARES

safety database coincident with the use of acetaminophen solution for infusion.

Event number; label*; causality**

#1; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: The information on this case is insufficient for a medical assessment. Hepatic failure, which might become fatal, is listed in the current SPC of PERFALGAN under overdose. Dose administered in this patient was not specified.

Reporter's comment:

Bristol-Myers Squibb Company, 16.08.2006 DE-Bristol-Myers Squibb Company-13326004

Sender diagnosis: not reported

Narrative

A physician reported that 2 patients with sepsis and pre-existing liver damage developed liver failure during therapy with acetaminophen solution for infusion (dosage not specified). This file pertains to patient 1 of 2, an alcoholic male who developed hepatic failure, a coagulation disorder, and expired.

Cross reference file 13326020 (patient 2 of 2).

As of 24 March 2006, this was the 15th report that contained the MedDRA PT Hepatic failure (the 3rd with a fatal outcome), entered into the CARES safety database coincident with the use of acetaminophen solution for infusion.

Follow-up information was received on 10-Aug-2006 from the reporting physician. The physician reported that a 54-year-old female patient, 170 cm and 81 kg, experienced coagulation disorder and hepatic failure from 28-Nov-2005 to 16-Dec-2005 while under therapy with acetaminophen for the treatment of pain and fever. The patient had undergone an operation due to a perforated ulcer duodeni and peritonitis on 28-Nov-2005. From 30-Nov-2005 until 14-Dec-2005 the patient had administered acetaminophen 3 g daily due to pain and fever. After the operation she experienced sepsis and multiorgan failure - liver failure, which was possibly accelerated by the use of acetaminophen. The patient died on 16-Dec-2005 due to hepato-renal failure. No autopsy was performed. The reporting physician considered the event as possibly related to acetaminophen.

Lab values: The quick value decreased from 53% on 28-Nov-2005 to 21% on 16-Dec-2005.

Comments: It is mentioned in the current SPC under the section precautions, that PERFALGAN should be handled with care in patients with liver damage. This patient had preexisting liver damage (hepatocellular damage) which might have become worse under PERFALGAN resulting in liver failure. Coagulopathy is most likely related to liver failure. The patient was under intensive care, had undergone surgery for perforated ulcer duodeni and was suffering from sepsis at the time when PERFALGAN was started.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06014333

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19410430	Age: 00064 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 General physical health deterioration		20051231	5 Day	2 Day	fatal
2 Hepatic failure		20051231		2 Day	fatal
3 Multiple organ failure		20051231		2 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN 10 mg/ml Infusionsloesung	Intravenous (not otherwis	00000004 G gram(s)/001	20051228-20051231	Pain
paracetamol		Intravenous infusion	00004 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Colon cancer			Unknown	
Liver metastases			Unknown	
Liver damage			Unknown	
Sepsis			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		0.54	NA	
Bilirubin		9.3	NA	
Quick's test		91	%	
Quick's test		31	%	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 04.04.2006 DE-Bristol-Myers Squibb Company-13326020

Sender diagnosis: not reported

Narrative

A physician, via BMS Germany (2006032202), reported on two patients. This report represents patient 2 of 2.

A male patient in the intensive care unit of the hospital with pre-existing liver damage and suffering from sepsis, experienced hepatic failure while receiving acetaminophen IV for an unreported indication. The event outcome was not reported. This event was considered medically important.

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Generic acetaminophen
 Event number; label*; causality**
 #1; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: It is mentioned in the current SPC under the section precautions, that PERFALGAN should be handled with care in patients with liver damage. This patient had preexisting liver damage, which may have led into liver failure.

Reporter's comment:

Bristol-Myers Squibb Company, 16.08.2006 DE-Bristol-Myers Squibb Company-13326020

Sender diagnosis: not reported

Narrative

A physician, via BMS Germany (2006032202), reported on two patients. This report represents patient 2 of 2.

A male patient in the intensive care unit of the hospital with pre-existing liver damage and suffering from sepsis, experienced hepatic failure while receiving acetaminophen IV for an unreported indication. The event outcome was not reported. This event was considered medically important.

Follow-up information was received on 10 Aug 2006. The physician reported that a 64-year-old male with a medical history of hepatic metastases of colon carcinoma and chemotherapy experienced hepatic failure from 27 Dec 2005 to 31 Dec 2005 while under therapy with acetaminophen 4 grams daily from 28 Dec 2005 until 31 Dec 2005 due to pain. For upper gastro-intestinal hemorrhage a subtotal gastric resection was performed (exact date not specified). Two days after extubation the patient experienced per-acute deterioration of his general condition with multiorgan failure (anuria, liver insufficiency and respiratory insufficiency). Subsequently the patient died (date of death not provided). The reporting physician considered the event as possibly related to acetaminophen.

Lab values: the quick value decreased from 91% to 31% and the bilirubin increased from 0.54 to 9.3.

*** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: It is mentioned in the current SPC under the section precautions, that PERFALGAN should be handled with care in patients with liver damage. This patient had preexisting liver damage (hepatocellular damage, metastases to liver) which might have become worse resulting in liver failure under PERFALGAN. Furthermore he had undergone gastric surgery, was on intensive care and suffering from sepsis which altogether may have contributed to the events.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06015793

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19381102	Age: 67 Year	weight (kg): 00090	BMI_int: 29.39	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20060123			2 Day	recovered/resolved
2 Death unexplained	20060208			18 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	00000001 G gram(s)/	20060122-20060123	Pain
paracetamol	Intravenous infusion		2 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic obstructive pulmonary disease			Unknown	
Hypertension			Unknown	
Diabetes mellitus			Unknown	
Pneumoconiosis			Unknown	
Assisted ventilation			Unknown	
Packed red blood cell transfusion	20060120	20060120	Unknown	
Silicosis			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hepatitis viral test NOS		: Negative	NA	
Glutamic-oxaloacetic transferase	20060122	31	NA	
Glutamic-oxaloacetic transferase	20060123	4875	NA	
Glutamic-oxaloacetic transferase	20060124	1371	NA	
Glutamic-oxaloacetic transferase	20060127	98	NA	
Glutamic-pyruvate transaminase	20060123	4937	NA	
Glutamic-pyruvate transaminase	20060124	3826	NA	
Glutamic-pyruvate transaminase	20060127	905	NA	
Gamma-glutamyltransferase	20060123	363	NA	
Gamma-glutamyltransferase	20060124	415	NA	
Gamma-glutamyltransferase	20060127	438	NA	
Bilirubin	20060122	2,2	NA	
Bilirubin	20060123	2,11	NA	
Bilirubin	20060124	1,73	NA	

Bilirubin	20060127	1,28	NA
Lipase	20060122	121	NA
Lipase	20060123	520	NA
Lipase	20060124	449	NA
Lipase	20060127	1,091	NA
Quick's test	20060122	90	NA
Quick's test	20060123	54	NA
Quick's test	20060124	53	NA
Quick's test	20060127	74	NA
C-reactive protein	20060122	18,3	NA
C-reactive protein	20060123	26	NA
C-reactive protein	20060124	13,9	NA
C-reactive protein	20060127	4,1	NA
Leukocyte count NOS	20060122	8	NA
Leukocyte count NOS	20060123	7,2	NA
Leukocyte count NOS	20060124	6,5	NA
Leukocyte count NOS	20060127	6,4	NA

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 11.04.2006 DE-Bristol-Myers Squibb Company-13333166

Sender diagnosis: not reported

Narrative

A physician reported, via BMS Germany (2006033001), that a 67-year-old male patient experienced the beginning of liver failure on 23-Jan-2006 after receiving acetaminophen 3 x 1g intravenously (via short term infusion) for the treatment of pain. Acetaminophen was administered on 22-Jan and 23-Jan-2006. On 22-Jan-2006, the patient's lab values were as follows: GOT - 31; Bili - 2.2; Lipase - 121; Quick - 90; CRP 18.3; and Leuko - 8. After administration of acetaminophen IV on 23-Jan-2006, the patient's lab values were GOT - 4875; GPT - 4937; GGT - 363; Bili - 2.11; Lipase - 520; Quick - 54; CRP - 26; and Leuko - 7.2. On 27-Jan-2006, the patient's lab values were as follows: GOT - 98; GPT - 905; GGT - 438; Bili - 1.28; Lipase - 1091; Quick - 74; CRP - 4.1; and Leuko - 6.4. Hepatitis serology was negative and the patient recovered.

Prior to this event, the patient had received a transfusion of red blood cells on 20-Jan-2006. The patient's status was post long term ventilation.

Additional medical history included chronic obstructive pulmonary disease (COPD), hypertension, Diabetes Mellitus, pneumokoniosis, and ventilation.

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Increase of liver enzymes are expected under the treatment with PERFALGAN. The extreme outcome may be due to the patients poor condition prior to the administration of PERFALGAN:

Reporter's comment:

Bristol-Myers Squibb Company, 19.04.2006 DE-Bristol-Myers Squibb Company-13333166

Sender diagnosis: not reported

Narrative

A physician reported, via BMS Germany (2006033001), that a 67-year-old male patient experienced the beginning of liver failure on 23-Jan-2006 after receiving acetaminophen 3 x 1g intravenously (via short term infusion) for the treatment of pain. Acetaminophen was administered on 22-Jan and 23

Jan-2006. On 22-Jan-2006, the patient's lab values were as follows: GOT - 31; Bili - 2.2; Lipase - 121; Quick - 90; CRP 18.3; and Leuko - 8. After administration of acetaminophen IV on 23-Jan-2006, the patient's lab values were GOT - 4875; GPT - 4937; GGT - 363; Bili - 2.11; Lipase - 520; Quick - 54; CRP - 26; and Leuko - 7.2. On 27-Jan-2006, the patient's lab values were as follows: GOT - 98; GPT - 905; GGT - 438; Bili - 1.28; Lipase - 1091; Quick - 74; CRP - 4.1; and Leuko - 6.4. Hepatitis serology was negative and the patient recovered.

Prior to this event, the patient had received a transfusion of red blood cells on 20-Jan-2006. The patient's status was post long term ventilation.

Additional medical history included chronic obstructive pulmonary disease (COPD), hypertension, Diabetes Mellitus, pneumokoniosis, and ventilation.

Follow-up information was received by the reporting physician on 11-Apr-2006. The physician provided information concerning the patient's medical history: On 21-Dec-2005 the patient had been admitted to the hospital due to exacerbation of burnt-out COPD and was treated until his death on 08-Feb-2006. The patient had been retired and had received a pension due to silicosis (quarry worker) since his 36th year of life. He had artificial ventilation at home with a respirator for years. In addition, the physician reported that the patient received red packed blood cells due to low hemoglobin in order to increase the oxygen capacity in the context of an attempt to wean him from the respirator on 20-Jan-2006. The patient had been artificially ventilated from 02-Jan-2006 to 08-Feb-2006. The patient had not been treated with acetaminophen in the past. The cause of death was not reported.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06027895

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19630310	Age: 42 Year	weight (kg): 115	BMI_int: 42.24	Age fetus:
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Patientdeath

deathdate: 20060530	deathcause: Unknown cause of death
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Embolism	20060530			29 Day	fatal
2 Liver failure	20060530			4 Day	Unknown
3 Drug abuse	2006				Unknown
4 Sudden death	20060530			4 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ENBREL etanercept	Intramuscular	/ Injection	20060526- -	Psoriasis
2 PARACETAMOL paracetamol		/ -	- -	

Drugs (concomitant)

3 Kontrazeptivum nnb docusate sodium; lactic acid; nonoxinol	/	- -
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Leg fracture			No	lower leg
Medical diet			Yes	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Wyeth-Pharma GmbH, 13.06.2006 DEWYE671731MAY06

Sender diagnosis: not reported

Narrative

Information was received from a healthcare professional regarding a 43-year-old female patient who received Enbrel (etanercept injection) therapy and possibly experienced sudden death or embolism leading to death.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

Indication for Enbrel was psoriasis. Therapy began in May-2006. Dose regimen was not provided.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

After three weeks of Enbrel therapy the patient suddenly died on 30-May-2006. Cause of death is still unclear but reporting physician assumes embolism (embolism) or sudden death (sudden death) but this was not confirmed at time of report. There is no information regarding patient's medical history and concomitant medication. Further information was requested.

The cause of death was reported as unknown cause of death.

Basierend auf der aktuellen Information ist mit dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. R. J?stingmeier

Reporter's comment:

Wyeth-Pharma GmbH, 25.07.2006 DEWYE671731MAY06

Sender diagnosis: not reported

Narrative

This case was considered medically important.

Additional information regarding details of events, product, medical history and assessment was received.

Information was received from a healthcare professional regarding a 43-year-old female patient who received Enbrel (etanercept injection) therapy and experienced sudden death due to pain killer abuse (paracetamol) and toxic liver failure leading to death.

MEDICAL HISTORY:

The patient's concurrent illness includes medical diet with a past history of lower limb fracture (lower leg). Past therapies included Heparin.

PRODUCT DETAILS:

Indication for Enbrel was psoriasis. Therapy began on 26-May-2006. Dose regimen was 50 mg (intramuscular).

Additional suspect medication included Paracetamol.

CONCOMITANT THERAPY:

Concomitant therapy included Unspecified Contraceptive (unspec).

EVENT DETAILS:

After three days of Enbrel therapy the patient suddenly died (sudden death) on 30-May-2006. The reporting physician cited the patient's husband, who believed that the patient was suspected of having abused paracetamol (drug abuser) in 2006, and experienced toxic liver failure (hepatic failure) that may have lead to her death. However, the cause of death is still unclear. Drug abuser and toxic liver failure were considered medically important and the outcome is unknown.

The physician considered sudden death not related to Enbrel.

Dr. R. J?stingmeier

Drug Safety Manager

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06064311

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19700211	Age: 35 Year	weight (kg): 75	BMI_int: 21.68	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Deranged liver function tests	20050901	20051028	58 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	3000 Mg milligram(s)/Day	20051005-20051010 6 Day	Bronchial infection
2 paracetamol paracetamol	Oral	500 Mg milligram(s)/	20050926-20051011 16 Day	Fever
3 roxithromycin roxithromycin	Oral	300 Mg milligram(s)/Day	20051011-20051018 8 Day	Bronchial pneumonia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatic damage	20050901		Yes	bei Antibiotikatherapie
Alcohol use			Unknown	vor ca. 4 Wochen größere Feier + etwas mehr Alkoholkonsum

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ASAT	20051023	Anstieg/increase	12.69 µmol	
Gamma GT	20051028	Anstieg/increase	215 µmol/s	
ALAT_W	20051028	Anstieg/increase	805 µmol/s	
ASAT	20051028	Anstieg/increase	825 µmol/s	
HDL	20051026	Abfall/decrease	0.57 mmol/	
Triglycerides	20051026	Anstieg/increase	21 mmol/l	
Gamma GT	20051026	Anstieg/increase	228 µmol/s	
AP_W	20051026	Anstieg/increase	257 µmol/s	
ALAT_W	20051026	Anstieg/increase	12.06 µmol	
ASAT	20051026	Anstieg/increase	10.34 µmol	
LDH	20051024	Anstieg/increase	15.13 µmol	
CK MB	20051024	Anstieg/increase	642 µmol/s	
CK	20051024	Anstieg/increase	865 µmol/s	
Gamma GT	20051024	Anstieg/increase	228 µmol/s	
AP_W	20051024	Anstieg/increase	257 µmol/s	
ALAT_W	20051024	Anstieg/increase	12.06 µmol	
ASAT	20051024	Anstieg/increase	15.79 µmol	
Calcium	20051023	Abfall/decrease	1.94 mmol/	

Sodium	20051023	Abfall/decrease	134 mmol/l
INR	20051023	Abfall/decrease	1.01
Gamma GT	20051023	Anstieg/increase	24 μmol/s.l.
ALAT_W	20051023	Anstieg/increase	10.42 μmol

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06081681

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19630728 Age: 42 Year weight (kg): 73 BMI_int: 25.26 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20051001	20051207	68 Day		recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxypen amoxicillin trihydrate	Oral	3 G gram(s)/Day	20051101-20051107 7 Day	Respiratory infection
2 bronchipret primula rhizome; thyme extract	Oral	3* Gtt drop(s)/Day	-20051107	Throat pain
3 cerazette desogestrel	Oral	0.075 Mg milligram(s)/Day	20031001-20051107 2 Year	Hypermenorrhea
4 doxycyclin doxycycline	Oral	200 Mg milligram(s)/Day	20051028-20051107 11 Day	Respiratory tract infection
5 laif hypericum extract	Oral	900 Mg milligram(s)/	20030401-	Depression
6 paracetamol paracetamol	Oral	1500 Mg milligram(s)/Day	20051004-20051107 35 Day	Throat pain
7 syneudon amitriptyline hydrochloride	Oral	50 Mg milligram(s)/Day	20050110-20051107 9 Month	Depression
8 voltaren resinat diclofenac resinate	Oral	150 Mg milligram(s)/Day	20030923-20051107 2 Year	

Drugs (concomitant)

9 ferro sanol ferroglycine sulfate complex	Oral	100 Mg milligram(s)/Day	20021203-20051107 2 Year	Iron deficiency
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Uterine myoma	20051101		Yes	
Liver transplant	20051207		Yes	
Adenoma adrenal			Yes	V.a.
Throat tightness	20050101		Yes	Kloss im Hals
Genital infection female	20050101		Yes	rezidivierende Pilzinfektion evtl. bei wiederholter Antibiotikatherapie
Hernia hiatal	20051001		Yes	
Haemangioma	20051001		Yes	Leber
Apoplexy	1999		No	
Allergy			Yes	Nickel
Contraception NOS			Yes	
Alcohol use			Yes	gelegentlich
Smoker			No	

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin direct		erhöht/increased	NA	
Haematocrit		erniedrigt/decreased	NA	
Haemoglobin		erniedrigt/decreased	NA	
Red blood cell count		erniedrigt/decreased	NA	
Albumin		erniedrigt/decreased	NA	
Quick's test		Abfall/decreased	NA	
Lactate dehydrogenase		erhöht/increased	NA	
Alkaline phosphatase		erhöht/increased	NA	
Gamma GT		erhöht/increased	NA	
Glutamate dehydrogenase		erhöht/increased	NA	
Alanine aminotransferase		erhöht/increased	NA	
Aspartate aminotransferase		erhöht/increased	NA	
Bilirubin total		erhöht/increased	NA	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06032411

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function test abnormal					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan IV	Intravenous (not otherwis /		-	
paracetamol	Intravenous infusion		-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 20.07.2006 DE-Bristol-Myers Squibb Company-13439633

Sender diagnosis: not reported

Narrative

A physicina reported, via BMS Germany (local no. 2006 07 11 01), that several patients were noted to have a considerable increase of liver values while under perisurgical therapy with intravenous acetaminophen. The event was considered medically significant.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

acetaminophene

Event number; label*; causality**

#1; expected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

No further comments.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06032853

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth:	Age: 55 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxicity					recovered/resolved
2 Drug interaction					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL		/	-	
paracetamol			-	
2 TEGRETAL		/	-	
carbamazepine			-	

Drugs (concomitant)

3 LAMOTRIGINE	/	-
lamotrigine		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Disability			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 25.07.2006 DE-002147023-PHNU2006DE01866

Sender diagnosis: not reported

Narrative

Report received from a pharmacy on 4 May 2006: A patient of about 55 years has been treated for a longer period of time with Tegretal (carbamazepine) and Lamictal (lamotrigine). During a hospital stay a short time ago (reason for hospitalization unknown) the patient additionally was treated with paracetamol (trade name not reported). It was unknown if the patient had taken paracetamol as self-medication or if it had been prescribed by the hospital and both dosage and treatment duration of paracetamol were unknown as well. After the treatment with paracetamol, which took place last week, the patient developed a massive hepatotoxic reaction "almost up to liver failure". Under symptomatic treatment the patient's condition meanwhile has improved considerably. The reporter wanted to know if the hepatotoxic reaction might have been caused by a drug-drug interaction of Tegretal and paracetamol. No more information available at the time of reporting.

Follow-up report received from a pharmacy on 20 Jul 2006: The patient had a history of disability. It was unknown if the patient had taken paracetamol by himself or if he had received it in the hospital. The patient has recovered completely. Further information was not available as the patient's relatives had not contacted the pharmacy again and as the treating physician and hospital were unknown.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06039679

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					Yes

Patient

Initials:	Sex: Female	Birth:	Age: 37 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic encephalopathy					Unknown
2 Fulminant hepatic failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute hepatitis B			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:		observestudytype:
Literature:	Tillmann H, Hadem J, Leifeld L, Zachou K, Canbay A, Eisenbach C, et al. Safety and efficacy of lamivudine in patients with severe acute or fulminant hepatitis B, a multicenter experience. J Viral Hepat 2006;13:4:256-263.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 28.08.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-04415GD

Sender diagnosis: not reported

Narrative

A 37-year-old female patient (referred to as patient 4, table 2) with severe acute hepatitis B who had ingested more than 5 g of paracetamol is reported. On admission the subject had already hepatic encephalopathy and thus was presenting with fulminant hepatic failure. She was treated with lamivudine but had to undergo a orthotopic liver transplantation because of a worse outcome. Ingestion of paracetamol was outlined as potential cofactor of patient's condition.

This case is from a multicenter study in which patients with severe acute or fulminant hepatitis B were treated with lamivudine in an attempt to prevent HBV reinfection after potential liver transplantation.

Fulminant hepatitis was present if the deterioration of liver function is accompanied by the development of hepatic encephalopathy.

An inclusion criterion for patients with severe acute hepatitis B was a sudden onset of liver disease (within less than 4 weeks).

The authors state: '(the) patient requiring liver transplantation had ingested more than 5 g paracetamol (acetaminophen), which might have contributed to the poor outcome.'

An additional individual case description is also available:

2006-DE-04413GD

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06039680

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth:	Age: 30 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic encephalopathy					Unknown
2 Fulminant hepatic failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute hepatitis B			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	Tillmann H, Hadem J, Leifeld L, Zachou K, Canbay A, Eisenbach C, et al. Safety and efficacy of lamivudine in patients with severe acute or fulminant hepatitis B, a multicenter experience. J Viral Hepat 2006;13:4:256-263.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)Boehringer Ingelheim GmbH, Germany, 28.08.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-04413GD

Sender diagnosis: not reported

Narrative

A 30-year-old male patient (referred to as patient 3, table 2) with severe acute hepatitis B who had ingested more than 5 g of paracetamol is reported. On admission the subject had already hepatic encephalopathy and thus was presenting with fulminant hepatic failure. He was treated with lamivudine and survived without liver transplantation. Ingestion of paracetamol was outlined as potential cofactor of patient's condition.

This case is from a multicenter study in which patients with severe acute or fulminant hepatitis B were treated with lamivudine in an attempt to prevent HBV reinfection after potential liver transplantation.

Fulminant hepatitis was present if the deterioration of liver function is accompanied by the development of hepatic encephalopathy.

An inclusion criterion for patients with severe acute hepatitis B was a sudden onset of liver disease (within less than 4 weeks).

An additional individual case description is also available:

2006-DE-04415GD

JUNKERN

Case assessment created by JU on 25 Aug 2006

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06043619

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19560316	Age: 50 Year	weight (kg): 61	BMI_int: 33.97	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme abnormal	20060828			5 Day	recovered/resolved
2 Function liver abnormal	20060828	20060901	5 Day	5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	4 G gram(s)/Day	20060824-20060828	Postoperative pain
paracetamol			5 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intestinal operation			Unknown	
Trisomy 21			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20060828	98.12: High	UKAT/L	
Alkaline phosphatase	20060828	5.13: High	UKAT/L	
Aspartate aminotransferase	20060828	55.11: High	UKAT/L	
Fibrin D dimer	20060828	1950: High	UG/L	
Gamma GT	20060828	4.58: High	UKAT/L	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 18.01.2007 DE-Bristol-Myers Squibb Company-13520754

Sender diagnosis: not reported

Narrative

This case was received from the Drug Commission of German Medical Association (AkdA #143290) via German Health Authority (BfArM #06043619).

BMS Germany Case Number 2006 09 22 02

A physician reported that a 50-year-old female (weight 61 kg, height 134 cm) with a medical history of trisomy G21, developed liver function disorder while receiving acetaminophen 4g daily due to pain after an intestinal operation. Laboratory data included: aspartate aminotransferase (AST/ASAT): 55.11 ukat/l (ref. 0.17-0.60); alanine aminotransferase (ALAT): 98.12 ukat/l (ref. 0.17-0.60), Gamma-GT: 4.58 ukat/L (ref. 0.1-0.7); alkaline phosphatase (AP): 5.13 ukat/l (ref. 0.58-1.74); D-dimers: 1950 ug/l (ref. 63.8-249.4). Acetaminophen was discontinued and the patient was kept under observation in the intensive care unit. No therapy was given for the event. At the time of reporting, the event resolved. The reporting physician considered the event related to acetaminophen. The event was considered medically significant.

Supplemental information was received on 10-Jan-2007 from the physician: The physician confirmed that the acetaminophen was infused intravenously as a short infusion.

Change of current scientific knowledge: no

Comments: Increases of AST and ALT are listed in the current SPC for PERFALGAN, but increases of gamma-GT, AP and D-dimere are not described.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06051428

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19690408	Age: 44 Year	weight (kg): 74	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					Unknown
2 Drug abuse					Unknown
3 Tiredness					Unknown
4 Abnormal liver function tests					Unknown
5 Muscle pain					Unknown
6 Epigastric pain					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 LENDORMIN	Oral	/	-	
brotizolam		Tablet	-	
2 PARACETAMOL	Oral	500 Mg milligram(s)/	-	
paracetamol		Tablet	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 07.11.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-05804DE

Sender diagnosis: not reported

Narrative

Initial information has been received from the poisoning information centre at the university hospital Mainz.

A 44-years-old male patient took 1x40 tablets Lendormin (brotizolam) and 1x 60 tablets Paracetamol in suicidal intention and was hospitalized.

As symptom 'tired' was reported. Intensity of the event was reported to be mild.

Treatment is not reported.

Outcome was reported as unknown.

No further information is expected. Case closed

The events in this case report are not noted adverse drug reactions for the product in question. The level of detail in the report does not permit a coherent assessment of the case. Based on the information provided, no change to the company core safety information or other measures are recommended at this time.

DUETTM

Case assessment created by DU on 06 Nov 2006

Reporter's comment:

Sender diagnosis: not reported

Narrative

Initial information has been received from the poisoning information centre at the university hospital Mainz.

A 44-years-old male patient took 1x40 tablets Lendormin (brotizolam) and 1x 60 tablets Paracetamol in suicidal intention and was hospitalized.

As symptom 'tired' was reported. Intensity of the event was reported to be mild.

Treatment is not reported.

Outcome was reported as unknown.

No further information is expected. Case closed

27Nov06

Further information has been received from the centre at the university hospital Mainz concerning personal specification, the events and therapy.

The events muscle pain, epigastric pain and increased liver values were added.

Intensity of the events was reported to be mild.

The patient was hospitalized into an intensive care unit and then transferred to a department for internal medicine. Treatment with Silibinin, induced diarrhoea and prolonged ACC scheme was performed.

Outcome of the events was reported as unknown.

Laboratory values: Quick 44, gammaGT 155

Diagnostic measures: monitoring, blood level was controlled (values not reported).

Further information is not expected. - Case closed

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06054329

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bilirubin increased					Unknown
2 Liver failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 GLIVEC imatinib	Oral	400 Mg milligram(s)/Day	-	-
2 PARACETAMOL paracetamol	Unknown	/	-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 20.11.2006 DE-002147023-PHNU2006DE03648

Sender diagnosis: not reported

Narrative

This poorly documented preliminary physician report was received via sales representative on 06 Nov 2006. This patient has been treated with Glivec (imatinib) 400 mg daily and with paracetamol for an unknown duration. She experienced liver failure and an increase of bilirubin to 26 (unit not provided) on an unknown date. Outcome and causality were not assessed in this initial report. Further information was requested.

Sender's comment:

Serious spontaneous report, [medically significant], already listed in the Basic Prescribing Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058367

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Female	Birth:	Age: 35 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Suicide attempt					Unknown
3 Drug abuse					Unknown
4 Renal insufficiency					recovered/resolved
5 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 METAMIZOLE		/	-	
metamizole			-	
2 PARACETAMOL	Oral	/	-	
paracetamol			-	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06518GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 35;

Gender: female;

Group: A;

Ingested amount of acetaminophen (mg): 5000;

Possibly influencing factor: metamizole (25000 mg; overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 1;

Duration of renal insufficiency: 2 days (end of hospital stay).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity."

16 additional cases are presented in this paper:

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06518GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 μ mol/l]). Patients were classified into 4 groups (A: creatinine 2.4 - 5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 35;

Gender: female;

Group: A;

Ingested amount of acetaminophen (mg): 5000;

Possibly influencing factor: metamizole (25000 mg; overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 1;

Duration of renal insufficiency: 2 days (end of hospital stay).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity."

16 additional cases are presented in this paper:

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

Boehringer Ingelheim GmbH, Germany, 13.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06518GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 μ mol/l]). Patients were classified into 4 groups (A: creatinine 2.4 - 5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 35;

Gender: female;

Group: A;

Ingested amount of acetaminophen (mg): 5000;

Possibly influencing factor: metamizole (25000 mg; overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 1;

Duration of renal insufficiency: 2 days (end of hospital stay).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity."

16 additional cases are presented in this paper:

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058438

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Renal insufficiency					recovered/resolved
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06530GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 28;

Gender: male;

Group: A;

Ingested amount of acetaminophen (mg): 5000;

Possibly influencing factor: renal insufficiency, chronic excessive dosing (overdose) of acetaminophen;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 1;

Duration of renal insufficiency: 2 days (end of hospital stay).

It was outlined in general that renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

The authors state: "The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose ... could be the chronic excessive dosing (overdose) of acetaminophen as presently observed in 4 patients. An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058443

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							Yes

Patient

Initials:	Sex: Male	Birth:	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiple organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					fatal
2 Overdose					fatal
3 Drug abuse					fatal
4 Multiple organ failure					fatal
5 Hepatic damage					fatal
6 Renal insufficiency					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 caffeine		/	-	
caffeine			-	
2 chlorphenamine		/	-	
chlorphenamine			-	
3 digitoxin		/	-	
digitoxin			-	
4 PARACETAMOL	Oral	/	-	
paracetamol			-	
5 phenytoin		/	-	
phenytoin			-	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06560GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported. The patient died due to multiple organ failure.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 mcml/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 71;

Gender: male;

Group: D;

Ingested amount of acetaminophen (mg): ~5000;

Possibly influencing factor: advanced age;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): low;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 1;

Duration of renal insufficiency: 16 days (day when patient died).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above).

The authors state: "... the second peak of the age distribution in the present investigation at 70 to 75 yrs was of particular interest. 2 patients in this group showed multiple organ failure with fatal outcome. The fatal outcome was probably not only caused by acetaminophen as both patients concomitantly ingested additional drugs (phenytoin and digoxin/cafeine and chlorphenamine)."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058445

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 22 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Suicide attempt					Unknown
3 Drug abuse					Unknown
4 Renal insufficiency			24 Day		recovered/resolved
5 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse chronic			Unknown	
Alcoholic hepatitis			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06553GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 22;

Gender: female;

Group: C;

Ingested amount of acetaminophen (mg): 9000;

Possibly influencing factor: alcoholic hepatitis;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 3;

Duration of renal insufficiency: 24 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "In 2 patients the renal impairment occurred in the context of a pre-existing liver disease due to chronic alcohol abuse and to hepatitis C virus infection. Liver disease may therefore be an additional influencing factor, particularly as several case reports on acetaminophen-related renal insufficiency in alcoholics as well as in patients with hepatitis C have been published."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058471

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			19 Day		recovered/resolved
2 Overdose					Unknown
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:		observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06551GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 39;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 10000;

Possibly influencing factor: chronic excessive dosing (overdose) of acetaminophen;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 3;

Time of onset of renal insufficiency (after overdose): day 3;

Duration of renal insufficiency: 19 days.

It was outlined in general that renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "An additional condition which might increase the risk for renal insufficiency could be the chronic excessive dosing (overdose) of acetaminophen as presently observed in 4 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058474

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			23 Day		recovered/resolved
2 Overdose					Unknown
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatitis C			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06552GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 mcml/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 19;

Gender: female;

Group: C;

Ingested amount of acetaminophen (mg): 20000 - 25000;

Possibly influencing factor: hepatitis C, chronic excessive dosing (overdose) of acetaminophen;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 3;

Duration of renal insufficiency: 23 days.

It was outlined in general that renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "An additional condition which might increase the risk for renal insufficiency could be the chronic excessive dosing (overdose) of acetaminophen as presently observed in 4 patients. ... In 2 patients the renal impairment occurred in the context of a pre-existing liver disease due to chronic alcohol abuse and to hepatitis C virus infection. Liver disease may therefore be an additional influencing factor, particularly as several case reports on acetaminophen-related renal insufficiency in alcoholics as well as in patients with hepatitis C have been published."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058477

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							Yes

Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiple organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					fatal
2 Overdose					fatal
3 Drug abuse					fatal
4 Renal insufficiency					fatal
5 Hepatic damage					fatal
6 Multiple organ failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 caffeine		/	-	
caffeine			-	
2 chlorphenamine		/	-	
chlorphenamine			-	
3 digitoxin		/	-	
digitoxin			-	
4 PARACETAMOL	Oral	/	-	
paracetamol			-	
5 phenytoin		/	-	
phenytoin			-	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06556GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported. The patient died due to multiple organ failure.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 mcml/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 75;

Gender: male;

Group: D;

Ingested amount of acetaminophen (mg): ~5000 for 1 - 2 weeks;

Possibly influencing factor: advanced age;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 7 - 14;

Time of onset of renal insufficiency (after overdose): day 7 - 14;

Duration of renal insufficiency: 3 days (day when patient died).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above).

The authors state: "... the second peak of the age distribution in the present investigation at 70 to 75 yrs was of particular interest. 2 patients in this group showed multiple organ failure with fatal outcome. The fatal outcome was probably not only caused by acetaminophen as both patients concomitantly ingested additional drugs (phenytoin and digoxin/cafeine and chlorphenamine)."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058478

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Renal insufficiency					recovered/resolved
3 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid		/	-	
acetylsalicylic acid			-	
2 PARACETAMOL	Oral	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06548GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 75;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 5000;

Possibly influencing factor: renal insufficiency, chronic excessive dosing (overdose) of acetaminophen, 500 mg acetylsalicylic acid (therapeutic dose), advanced age;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: -.

It was outlined in general that renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional condition which might increase the risk for renal insufficiency could be the chronic excessive dosing (overdose) of acetaminophen as presently observed in 4 patients. An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058479

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					Unknown
2 Overdose					Unknown
3 Drug abuse					Unknown
4 Hepatic damage					Unknown
5 Renal insufficiency			22 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Primary source

Qualification:	observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06550GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 mcml/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 18;

Gender: female;

Group: B;

Ingested amount of acetaminophen (mg): 25000;

Possibly influencing factor: -;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 3;

Duration of renal insufficiency: 22 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058480

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 23 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Suicide attempt					Unknown
3 Drug abuse					Unknown
4 Renal insufficiency			20 Day		recovered/resolved
5 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL paracetamol	Oral	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dehydration			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:		observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06527GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 23;

Gender: male;

Group: A;

Ingested amount of acetaminophen (mg): 45000 (this amount was accompanied only by a moderate renal insufficiency);

Possibly influencing factor: dehydration;

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): probable;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 4;

Duration of renal insufficiency: 20 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "With 4 patients presenting with dehydration due to recurrent vomiting this prerenal problem may also represent an important comorbidity."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058484

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Suicide attempt					Unknown
3 Renal insufficiency			8 Day		recovered/resolved
4 Drug abuse					Unknown
5 Hepatic damage			8 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac sodium		/	-	
2 IBUPROFEN ibuprofen		/	-	
3 PARACETAMOL paracetamol	Oral	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:		observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

Boehringer Ingelheim GmbH, Germany, 13.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D:

creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058485

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth:	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose					Unknown
2 Suicide attempt					Unknown
3 Drug abuse					Unknown
4 Renal insufficiency					recovered/resolved
5 Hepatic damage					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	
2 PENICILLIN V		/	-	
phenoxymethylpenicillin			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dehydration			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:		observestudytype:
Literature:	von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06544GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 19;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 15000;

Possibly influencing factor: dehydration, 20 x 10E6 units of penicillin V (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 5;

Time of onset of renal insufficiency (after overdose): day 5;

Duration of renal insufficiency: 30 days (end of hospital stay).

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs (or beta-lactam antibiotics), which are well known for their acute and chronic nephrotoxicity. With 4 patients presenting with dehydration due to recurrent vomiting this prerenal problem may also represent an important comorbidity."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06545GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

case: DE-BFARM-06062587

Patient

Initials: Sex: Male Birth: 20050714 Age: 0 Day weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20050714				Unknown
2 Ventricular septal defect	20050714				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atovaquone atovaquone	Transplacental	/	-	Malaria prophylaxis
2 Kodein codeine	Transplacental	/	-	Migraine
3 paracetamol paracetamol	Transplacental	/	-	Migraine
4 PROGUANIL proguanil hydrochloride	Transplacental	/	-	Malaria prophylaxis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07001097

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 00077 Year	weight (kg): 00077	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Postoperative renal failure	20070108				Unknown
2 Liver enzyme abnormal					recovered/resolved
3 Accidental overdose	20070108				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan iV paracetamol	Intravenous (not otherwis	00007000 Mg milligram(s)	20070108- Intravenous infusion -	Femur fracture
2 perfalgan iV paracetamol	Intravenous (not otherwis	00007000 Mg milligram(s)	20070108- Intravenous infusion -	Postoperative care

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Femur fracture			Unknown	
Fracture treatment			Unknown	femur

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20070111	73: Normal	UI	
Alkaline phosphatase	20070106	37: Low	UI	
Alkaline phosphatase	20070107	59: Normal	UI	
Alkaline phosphatase	20070108	48: Normal	UI	
Alkaline phosphatase	20070109	56: Normal	UI	
Alkaline phosphatase	20070110	79: Normal	UI	
Alkaline phosphatase	20070123	95: Normal	UI	
Alkaline phosphatase	20070112	56: Normal	UI	
Alkaline phosphatase	20070113	52: Normal	UI	
Alkaline phosphatase	20070115	66: Normal	UI	
Alkaline phosphatase	20070116	66: Normal	UI	
Alkaline phosphatase	20070121	65: Normal	UI	
Alkaline phosphatase	20070122	89: Normal	UI	
Creatinine	20070123	0.96: Normal	MG/DL	
Creatinine		2.1	NA	

Creatinine	20070111	1.68: High	MG/DL
Creatinine	20070120	1.17: Normal	MG/DL
Creatinine	20070116	1.35: High	MG/DL
Creatinine	20070122	1.08: Normal	MG/DL
Creatinine	20070115	0.98: Normal	MG/DL
Creatinine	20070113	2.09: High	MG/DL
Creatinine	20070112	2.08: High	MG/DL
Creatinine	20070121	1.03: Normal	MG/DL
Creatinine	20070109	1.90: High	MG/DL
Creatinine	20070108	2.10: High	MG/DL
Creatinine	20070107	1.99: High	MG/DL
Creatinine	20070106	1.20: High	MG/DL
Creatinine	20070110	1.46: High	MG/DL
Glutamic-oxaloacetic transferase	20070121	26: Normal	UI
Glutamic-oxaloacetic transferase	20070115	123: High	UI
Glutamic-oxaloacetic transferase	20070116	25: Normal	UI
Glutamic-oxaloacetic transferase	20070113	47: Normal	UI
Glutamic-oxaloacetic transferase	20070122	52: High	UI
Glutamic-oxaloacetic transferase	20070123	41: Normal	UI
Glutamic-oxaloacetic transferase	20070106	16: Normal	UI
Glutamic-oxaloacetic transferase	20070110	49: Normal	UI
Glutamic-oxaloacetic transferase	20070112	28: Normal	UI
Glutamic-oxaloacetic transferase	20070111	34: Normal	UI
Glutamic-oxaloacetic transferase	20070109	35: Normal	UI
Glutamic-oxaloacetic transferase	20070108	42: Normal	UI
Glutamic-oxaloacetic transferase	20070107	53: High	UI
Glutamic-pyruvate transaminase	20070122	64: High	UI
Glutamic-pyruvate transaminase	20070107	30: Normal	UI
Glutamic-pyruvate transaminase	20070111	37: Normal	UI
Glutamic-pyruvate transaminase	20070106	12: Normal	UI
Glutamic-pyruvate transaminase	20070108	29: Normal	UI
Glutamic-pyruvate transaminase	20070109	39: Normal	UI
Glutamic-pyruvate transaminase	20070110	57: High	UI
Glutamic-pyruvate transaminase	20070123	59: High	UI
Glutamic-pyruvate transaminase	20070112	28: Normal	UI
Glutamic-pyruvate transaminase	20070113	35: Normal	UI
Glutamic-pyruvate transaminase	20070115	125: High	UI
Glutamic-pyruvate transaminase	20070116	73: High	UI
Glutamic-pyruvate transaminase	20070121	47: Normal	UI

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 16.01.2007 DE-Bristol-Myers Squibb Company-13638226

Sender diagnosis: not reported

Narrative

A physician, via the Poison Control Center Mainz (PCCM) and BMS Germany (2007 01 08 03), reported that a 77-year-old male patient (weight 77 kg)

developed post-operative subacute renal failure 24 hours after administration of an accidental overdose of intravenous acetaminophen following surgery of a femur fracture. The physician had inadvertently administered 7 ampoules of acetaminophen within 24 hours (last administration approximately 16 hours prior to the call to PCCM on 08-Jan-2007 at 3:34 p.m.). The patient's creatinine level was 2.1. Therapy with acetylcysteine (ACC) if necessary and inpatient treatment were recommended. The PCCM also recommended the physician to clarify if the patient had a medical history of a pre-existing hepatic damage and to perform liver value monitoring. The patient did not show any symptoms. The patient's current condition was most likely related to the surgery. At the time of reporting, the outcome was unknown.

Sender's comment:

This report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Acetaminophen i.v.

Event number; label*; causality**

#1; not applicable; not applicable

#2; unexpected; NOL

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: The renal failure might be related to the surgery the patient had undergone, however, it is unknown whether this elderly patient had pre-existing conditions that could have also led to renal failure.

Reporter's comment:

Bristol-Myers Squibb Company, 23.02.2007 DE-Bristol-Myers Squibb Company-13638226

Sender diagnosis: not reported

Narrative

A physician, via the Poison Control Center Mainz (PCCM) and BMS Germany (2007 01 08 03), reported that a 77-year-old male patient (weight 77 kg) developed post-operative subacute renal failure 24 hours after administration of an accidental overdose of intravenous acetaminophen following surgery of a femur fracture. The physician had inadvertently administered 7 ampoules of acetaminophen within 24 hours (last administration approximately 16 hours prior to the call to PCCM on 08-Jan-2007 at 3:34 p.m.). The patient's creatinine level was 2.1. Therapy with acetylcysteine (ACC) if necessary and inpatient treatment were recommended. The PCCM also recommended the physician to clarify if the patient had a medical history of a pre-existing hepatic damage and to perform liver value monitoring. The patient did not show any symptoms. The patient's current condition was most likely related to the surgery. At the time of reporting, the outcome was unknown.

Supplemental information received from a physician via BMS Germany and Poison Control Center Mainz, on 16-Feb-2007, reported the patient initials as ER and date of birth as 16-Mar-1929. In addition to the previous reported events the patient experienced liver value changes, the outcome for all events is recorded as resolved. The patient had been hospitalized on 04-Jan-2007, probably for the femur fracture surgery. The physician reported that liver value monitoring and therapy with acetylcysteine (ACC) had been performed. The follow up information also stated that the physician had clarified if the patient had a medical history of a pre-existing hepatic damage, however no further information was provided. At the time of reporting, the events had completely resolved.

Abnormal laboratory values included but are not limited to the following, see laboratory values enclosed for full listing.

Creatine (normal values 0.7-1.2mg/dl) of 07-Jan- 16-Jan-2007: 1.99mg/dl, 2.10mg/dl, 1.90mg/dl, 1.46mg/dl, 1.68mg/dl, 2.08mg/dl, 2.09mg/dl, 1.35mg/dl. Creatine levels returned to normal from 20-Jan-2007 to 23-Jan-2007 with values ranging from 0.96 - 1.17 mg/dl.

Glutamic pyruvic transaminase(GPT)(normal value <50 UI): 10-Jan-2007 57 UI, 15-Jan-2007 125 UI, 16-Jan-2007 73 UI and 22-Jan-2007 64 UI.

Glutamate-oxaloacetate transaminase (GOT)(normal value <50 UI): 07-Jan-2007 53 UI, 15-Jan-2007 123 UI, and 22-Jan-2007 52 UI.

Alkaline Phosphatase (normal value 40-129 UI): 06-Jan-2007 37 UI.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07008716

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes	20070315				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL paracetamol		/	200703-200703	Postoperative pain
2 parecoxib parecoxib		/	20070315-200703	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Surgery	200703		No	Not specified

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests	200703	AT ABOUT 800U/L		

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 27.03.2007 DE-PFIZER INC-2007020857

Sender diagnosis: not reported

Narrative

This pharmacist reports of a female patient, age not mentioned, was treated with Dynastat (parecoxib) and Paracetamol for postoperative pain. Immediately after the first application of Dynastat the patient's liver values, not specified, increased at about 800U/l (baseline and normal range were not provided. Treatment with Dynastat and Paracetamol was discontinued in response to the adverse event.

Company Clinical Evaluation:

The information available in this report is limited, and does not allow a medically meaningful assessment of the case. In particular the following relevant information is not available: which liver values are elevated, treatment, outcome, and patient medical history/concomitant medications. Follow up information has been requested.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07011435

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19571202	Age: 49 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting	20070320	20070330	11 Day	7 Day	recovered/resolved
2 Icterus	20070320	20070330	11 Day	7 Day	recovered/resolved
3 Elevated liver enzymes	20070320			7 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Avalox 400 mg Filmtabletten moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day Tablet	20070314-20070317 4 Day	Bronchitis
2 paracetamol paracetamol	Oral	/ Tablet	- -	Bone pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Abscess			k.A.	abscess pubic eminence
Gastritis erosive			k.A.	erosive antrum gastritis
Fatty liver			k.A.	fatty liver hepatitis
Hypertension arterial			k.A.	arterial hypertension
Urinary tract infection			k.A.	urinary tract infection

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20070326	445	U/L	
Alkaline phosphatase	20070322	569	U/L	
Alkaline phosphatase	20070320	470	U/L	
Bilirubin direct	20070320	56	MCMOL/L	
Bilirubin total	20070322	75	MCMOL/L	
Bilirubin total	20070320	142	MCMOL/L	
Bilirubin total	20070326	35	MCMOL/L	
C-reactive protein	20070320	11.7	MG/DL	
C-reactive protein	20070322	5.4	MG/DL	
C-reactive protein	20070326	2	MG/DL	
Gamma-glutamyltransferase	20070320	533	U/L	
Gamma-glutamyltransferase	20070326	666	U/L	
Gamma-glutamyltransferase	20070322	651	U/L	
Hematocrit	20070326	0.38	L/L	
Hematocrit	20070320	0.37	L/L	

Hematocrit	20070322	0.33	L/L
Hemoglobin	20070326	121	G/L
Hemoglobin	20070322	111	G/L
Hemoglobin	20070320	130	G/L
Platelet count	20070326	532	GIGA/L
Platelet count	20070322	248	GIGA/L
Platelet count	20070320	93	GIGA/L
SGOT	20070418	23.5	U/L
SGOT	20070320	516	U/L
SGOT	20070322	292	U/L
SGOT	20070326	119	U/L
SGPT	20070418	24.2	U/L
SGPT	20070320	437	U/L
SGPT	20070326	257	U/L
SGPT	20070322	367	U/L
White blood cells	20070326	5.91	/1
White blood cells	20070322	4.35	/1
White blood cells	20070320	3.88	/1
Lipase	20070320	99.8	U/L

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 24.04.2007 DE-BAYER-200710477BVD

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This spontaneous report was received on 17-Apr-2007 from a health professional in Germany via the German Health Professional Board, referring to a 50 year-old female.

Medical and drug history:

It is unknown whether any concomitant drugs have been given.

Suspect drug(s), timing and conditions surrounding the onset of the reaction(s):

Patient took Avalox (moxifloxacin hydrochloride) for bronchitis. Administration dates not reported, continuing at time of event at a total daily dose of 400 MG unknown.

The progression of the event(s) and its (their) outcome in the patient:

On an unknown date, patient was hospitalized due to ICTERUS. Patient had not recovered at time of report.

Bayer Vital GmbH, 10.05.2007 DE-BAYER-200710477BVD

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This spontaneous report was received on 17-Apr-2007 from a health professional in Germany via the German Health Professional Board, referring to a 50 year-old female.

Medical and drug history:

It is unknown whether any concomitant drugs have been given.

Suspect drug(s), timing and conditions surrounding the onset of the reaction(s):

Patient took Avalox (moxifloxacin hydrochloride) for bronchitis. Administration dates not reported, continuing at time of event at a total daily dose of 400 MG unknown.

The progression of the event(s) and its (their) outcome in the patient:

On an unknown date, patient was hospitalized due to ICTERUS. Patient had not recovered at time of report.

FU-information received on 30-APR-2007:

The physician reports that the female patient presented with an acute attack of an otherwise chronic bronchitis and was treated with Avalox (moxifloxacin) at a daily dose of 400 mg from 14-MAR-2007 to 17-MAR-2007 when the patient withdrew the drug. On 20-MAR-2007 the patient observed ICTERUS and experienced VOMITING. A FU visit at her family doctor on the same day led to a hospital re-admission where a "history of fatty liver with steato hepatitis (caused by drugs)" was diagnosed. Alcohol abuse was denied, GGT was normal. The patient's condition started to improve four days after withdrawal. On 30-MAR-2007, she had recovered.

Bayer Vital GmbH, 23.05.2007 DE-BAYER-200710477BVD

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This spontaneous report was received on 17-Apr-2007 from a health professional in Germany via the German Health Professional Board, referring to a 50 year-old female.

Medical and drug history:

It is unknown whether any concomitant drugs have been given.

Suspect drug(s), timing and conditions surrounding the onset of the reaction(s):

Patient took Avalox (moxifloxacin hydrochloride) for bronchitis. Administration dates not reported, continuing at time of event at a total daily dose of 400 MG unknown.

The progression of the event(s) and its (their) outcome in the patient:

On an unknown date, patient was hospitalized due to ICTERUS. Patient had not recovered at time of report.

FU-information received on 30-APR-2007:

The physician reports that the female patient presented with an acute attack of an otherwise chronic bronchitis and was treated with Avalox (moxifloxacin) at a daily dose of 400 mg from 14-MAR-2007 to 17-MAR-2007 when the patient withdrew the drug. On 20-MAR-2007 the patient observed ICTERUS and experienced VOMITING. A FU visit at her family doctor on the same day led to a hospital re-admission where a "history of fatty liver with steato hepatitis (caused by drugs)" was diagnosed. Alcohol abuse was denied, GGT was normal. The patient's condition started to improve four days after withdrawal. On 30-MAR-2007, she had recovered.

Hospital report provided on 18-MAY-2007:

Patient was admitted on 20-MAR2007 suffering from severe skin and scleral icterus, probably caused by fatty liver hepatitis.

Laboratory values showed increased cholesterin and triglycerides and increased liver enzymes.

Patient reported that quite recently she had taken paracetamol twice/ three times daily due to bone pain and arthralgia. Alcohol abuse was denied

Sonography: liver steatosis without hepatomegaly.

Abdominal CT: No pathological findings of pancreas, gall bladder and bile ducts.

No signs of tumor, lymph node enlargement nor organ metastasis.

Gastroscopy 22-MAR-2007: erosive antrum gastritis.

Further findings: abscess pubic eminence, bronchitis, urinary tract infection.

Therapy: After infusion therapy and diet condition of icterus and liver values improved. Gastritis was treated with proton pump inhibitors, bronchitis with ceftriaxone over five days, acetylcysteine and ambroxol.

Incision of abscess was performed.

Patient was discharged from hospital on 30-MAR-2007 well-being.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-07011832

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 19 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain					Unknown
2 Acidosis					Unknown
3 Renal failure					Unknown
4 Hepatic insufficiency					Unknown
5 Drug poisoning					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid	Unknown	/	-	
acetylsalicylic acid			-	
2 COCAINE	Unknown	/	-	
cocaine			-	
3 METOCLOPRAMIDE	Unknown	/	-	
metoclopramide			-	
4 OTHER ANALGESICS	Unknown	/	-	
			-	
5 PARACETAMOL	Unknown	/	-	
paracetamol			-	
6 THC-COOH		/	-	
tetrahydrocannabinol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	von Mach M,Weber C,Meyer M,Weilemann L,Maurer H,Peters F. Comparison of urinary on-site immunoassay screening and gas chromatography-mass spectrometry results of 111 patients with suspected poisoning presenting at an emergency department. Ther Drug Monit 2007 Feb;29:1:27-39.		

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 23.04.2007 DE-GlaxoSmithKline-B0467309A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of drug poisoning in a 19-year-old female subject who received aspirin (brandname/tradename unknown) for an unknown drug indication. The subject was enrolled in a retrospective study of patients with suspected drug poisoning from January to December 2003 in the emergency department of a tertiary urban hospital.

Co-suspect medication included paracetamol (brandname/tradename unknown), cocaine, metoclopramide hydrochloride and other analgesics .

On an unknown date, the subject presented to the emergency department with suspected intoxication/poisoning. The suspect drugs detected by gas chromatography-mass spectrometry included cocaine, paracetamol, metoclopramide and aspirin. The suspect drugs detected by triage 8 on site drug screening device included cocaine and cannabinoids. Suspect anamnestic drugs (poisoning or medication) included paracetamol and other analgesics. The patient had symptoms including hepatic insufficiency, renal failure, abdominal pain and acidosis (pH 6.9). The poisoning severity score (PSS) was 3. The patient was hospitalised for 29 day. This case was assessed as medically serious by GSK. At the time of reporting, the outcome of the event was unknown. The author considered the event were related to the suspect drugs.

Boehringer Ingelheim GmbH, Germany, 24.08.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-04906GD

Sender diagnosis: not reported

Narrative

A retrospective study was performed aiming to systematically compare the results obtained with the Triage 8 panel, an on-site testing device used in many emergency departments, with those of a well-established gas chromatography - mass spectrometry (GC-MS) screening method. Urine samples from patients presenting to an emergency department with suspected poisoning within a 1 year period (January to December 2003) were used for this purpose.

Patient data:

Patient No. (Table 2): 40;

Sex: female;

Age (years): 19;

Triage 8 positives: cocaine, cannabinoids;

GC-MS results: cocaine/benzoyllecognine, THC-COOH, paracetamol, metoclopramide, salicylic acid;

Anamnestic drugs, poisoning or medication: paracetamol, other analgesics;

Symptoms: hepatic insufficiency, renal failure, abdominal pain, acidosis (pH 6.9);

Poisoning Severity Score: 3;

Drugs used in treatment: -;

Hospital stay (days): 29.

34 additional cases are presented in this paper:

2007-DE-04896GD

2007-DE-04901GD

2007-DE-04902GD

2007-DE-04903GD

2007-DE-04905GD

2007-DE-04907GD

2007-DE-04908GD

2007-DE-04909GD

2007-DE-04910GD

2007-DE-04911GD

2007-DE-04913GD

2007-DE-04914GD

2007-DE-04916GD

2007-DE-04917GD

2007-DE-04918GD

2007-DE-04919GD

2007-DE-04920GD

2007-DE-04923GD

2007-DE-04924GD

2007-DE-04925GD

2007-DE-04926GD

2007-DE-04927GD

2007-DE-04928GD

2007-DE-04929GD

2007-DE-04931GD

2007-DE-04932GD

2007-DE-04933GD

2007-DE-04935GD

2007-DE-04937GD

2007-DE-04938GD

2007-DE-04941GD

2007-DE-04943GD

2007-DE-04944GD

2007-DE-04946GD

Boehringer Ingelheim GmbH, Germany, 27.08.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-04906GD

Sender diagnosis: not reported

Narrative

A retrospective study was performed aiming to systematically compare the results obtained with the Triage 8 panel, an on-site testing device used in many emergency departments, with those of a well-established gas chromatography - mass spectrometry (GC-MS) screening method. Urine samples from patients presenting to an emergency department with suspected poisoning within a 1 year period (January to December 2003) were used for this purpose.

Patient data:

Patient No. (Table 2): 40;

Sex: female;

Age (years): 19;

Triage 8 positives: cocaine, cannabinoids;

GC-MS results: cocaine/benzoylecognine, THC-COOH, paracetamol, metoclopramide, salicylic acid;

Anamnestic drugs, poisoning or medication: paracetamol, other analgesics;

Symptoms: hepatic insufficiency, renal failure, abdominal pain, acidosis (pH 6.9);

Poisoning Severity Score: 3;

Drugs used in treatment: -;

Hospital stay (days): 29.

34 additional cases are presented in this paper:

2007-DE-04896GD

2007-DE-04901GD

2007-DE-04902GD

2007-DE-04903GD

2007-DE-04905GD

2007-DE-04907GD

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2007-DE-04909GD

2007-DE-04910GD

2007-DE-04911GD

2007-DE-04913GD

2007-DE-04914GD

2007-DE-04916GD

2007-DE-04917GD

2007-DE-04918GD

2007-DE-04919GD

2007-DE-04920GD

2007-DE-04923GD

2007-DE-04924GD

2007-DE-04925GD

2007-DE-04926GD

2007-DE-04927GD

2007-DE-04928GD

2007-DE-04929GD

2007-DE-04931GD

2007-DE-04932GD

2007-DE-04933GD

2007-DE-04935GD

2007-DE-04937GD

2007-DE-04938GD

2007-DE-04941GD

2007-DE-04943GD

2007-DE-04944GD

2007-DE-04946GD

Boehringer Ingelheim GmbH, Germany, 27.08.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-04906GD

Sender diagnosis: not reported

Narrative

A retrospective study was performed aiming to systematically compare the results obtained with the Triage 8 panel, an on-site testing device used in many emergency departments, with those of a well-established gas chromatography - mass spectrometry (GC-MS) screening method. Urine samples from patients presenting to an emergency department with suspected poisoning within a 1 year period (January to December 2003) were used for this purpose.

Patient data:

Patient No. (Table 2): 40;

Sex: female;

Age (years): 19;

Triage 8 positives: cocaine, cannabinoids;

GC-MS results: cocaine/benzoyllecognine, THC-COOH, paracetamol, metoclopramide, salicylic acid;

Anamnestic drugs, poisoning or medication: paracetamol, other analgesics;

Symptoms: hepatic insufficiency, renal failure, abdominal pain, acidosis (pH 6.9);

Poisoning Severity Score: 3;

Drugs used in treatment: -;

Hospital stay (days): 29.

34 additional cases are presented in this paper:

2007-DE-04896GD

2007-DE-04901GD

2007-DE-04902GD

2007-DE-04903GD

2007-DE-04905GD

2007-DE-04907GD

2007-DE-04908GD

2007-DE-04909GD

2007-DE-04910GD

2007-DE-04911GD

2007-DE-04913GD

2007-DE-04914GD

2007-DE-04916GD

2007-DE-04917GD

2007-DE-04918GD

2007-DE-04919GD

2007-DE-04920GD

2007-DE-04923GD

2007-DE-04924GD

2007-DE-04925GD

2007-DE-04926GD

2007-DE-04927GD

2007-DE-04928GD

2007-DE-04929GD

2007-DE-04931GD

2007-DE-04932GD

2007-DE-04933GD

2007-DE-04935GD

2007-DE-04937GD

2007-DE-04938GD

2007-DE-04941GD

2007-DE-04943GD

2007-DE-04944GD

2007-DE-04946GD

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07034252

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis fulminant					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07034283

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis fulminant					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07034934

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis fulminant					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07035085

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis fulminant					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07035107

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis fulminant					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07045811

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 00043 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctiva colouring yellow	20070609			1 Day	not recovered/not resolved
2 Transaminases increased	20070609			1 Day	not recovered/not resolved
3 Conjunctiva colouring yellow					recovering/resolving
4 Liver failure					recovering/resolving
5 Thrombopenia	20070609			1 Day	not recovered/not resolved
6 Liver necrosis	20070609			1 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	00000001 G gram(s)/	20070609-	Fever
paracetamol		Intravenous infusion	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fainting			Unknown	
Blood electrolytes decreased			Unknown	
Fever			Unknown	
Circulatory collapse			Unknown	with stupor due to heat and physical overstraining long run of 7.5 Km and was hospitalized

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20070620	25.55: High	UMOL/L	
Alanine aminotransferase	20070613	214.40: High	UMOL/L	
Alkaline phosphatase	20070620	2.33: High	UMOL/L	
Aspartate aminotransferase	20070609	1.02: High	UMOL/L	
Aspartate aminotransferase	20070620	1.84: High	UMOL/L	
Aspartate aminotransferase	20070613	89.10: High	UMOL/L	
Bilirubin total	20070614	148.9: High	UMOL/L	
Bilirubin total	20070613	133.2: High	UMOL/L	
Bilirubin total	20070620	67.9: High	UMOL/L	
Body temperature		40: High	DEG C	
Creatine kinase	20070613	8.34: High	UMOL/L	
Creatine kinase	20070616	5.60: High	UMOL/L	
Creatine phosphokinase MB	20070613	0.58: High	UMOL/L	

Creatinine	20070620	107: High	UMOL/L
Creatinine	20070609	153: High	UMOL/L
Creatinine	20070609	161: High	UMOL/L
Fibrin split products	20070614	0.52: High	UG/ML
Fibrin split products	20070613	0.76: High	UG/ML
Gamma-glutamyltransferase	20070620	3.75: High	UMOL/L
Lactate	20070609	4.15: High	NMOL/L
Lactate dehydrogenase	20070613	15.71: High	UMOL/L
Lactate dehydrogenase	20070615	7.26: High	UMOL/L
Protein urine	20070609	2.60: High	G/L
Protein urine	20070611	4.54: High	G/L
Prothrombin ratio	20070609	1.18: High	NA
Prothrombin ratio	20070613	2.93: High	NA
Serum glucose	20070609	10.27: High	MMOL/L
Thrombocyte count		nicht vorhanden	NA/absent
Transaminase NOS		89: High	UMOL/L
Glutamate dehydrogenase	20070613	1145: High	NMOL/L

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 26.06.2007 DE-Bristol-Myers Squibb Company-13820154

Sender diagnosis: not reported

Narrative

A hospital physician reported via a sales representative that a 43-year-old healthy, non-alcoholic male patient experienced life-threatening yellow colour of his eyes, and a massive elevation of transaminases after receiving 1 g of acetaminophen IV. Prior to receiving the acetaminophen, the patient had fainted and a blood sample showed an absolute electrolyte loss. Additionally, the he had a fever of 38.5°C; hence he was given the acetaminophen by a physician (not the reporting physician). The next day the patient visited the reporting hospital physician with yellow colour of the eyes. A massive elevation of the transaminases was diagnosed and the patient was on the verge of a liver transplantation. The reporting physician considered the situation severely critical. The patient was currently recovering.

BMS Medical Evaluation Comment: A 43-year-old non-alcoholic male patient experienced yellow colour of the eyes along with massive elevation of the transaminases after receiving 1 g of acetaminophen for fever. Reportedly, the patient had participated in a marathon and his blood sample had showed an absolute electrolyte loss. A therapeutic dose of acetaminophen in a non-alcoholic patient is not known to cause liver damage. In this case, role of dehydration as a major contributing factor for drug-event association can not be denied.

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: Both events should be seen in the context of a massive dehydration of the patient and associated absolute electrolyte loss which might be a consequence of the marathon. The event "transaminases increased" is expected according to the current SPC of PERFALGAN but a lifethreatening increase is not specified.

Reporter's comment:

Bristol-Myers Squibb Company, 02.07.2007 DE-Bristol-Myers Squibb Company-13820154

Sender diagnosis: not reported

Narrative

A hospital physician reported via a sales representative that a 43-year-old healthy, non-alcoholic male patient experienced life-threatening yellow

colour of his eyes, and a massive elevation of transaminases after receiving 1 g of acetaminophen IV. Prior to receiving the acetaminophen, the patient had fainted and a blood sample showed an absolute electrolyte loss. Additionally, the he had a fever of 38.5°C; hence he was given the acetaminophen by a physician (not the reporting physician). The next day the patient visited the reporting hospital physician with yellow colour of the eyes. A massive elevation of the transaminases was diagnosed and the patient was on the verge of a liver transplantation. The reporting physician considered the situation severely critical. The patient was currently recovering.

BMS Medical Evaluation Comment: A 43-year-old non-alcoholic male patient experienced yellow colour of the eyes along with massive elevation of the transaminases after receiving 1 g of acetaminophen for fever. Reportedly, the patient had participated in a marathon and his blood sample had showed an absolute electrolyte loss. A therapeutic dose of acetaminophen in a non-alcoholic patient is not known to cause liver damage. In this case, role of dehydration as a major contributing factor for drug-event association can not be denied.

Supplemental information received from the reporting physician on 27-Jun-2007 indicated that the patient had developed thrombopenia and liver necrosis after administration of intravenous acetaminophen (1 g) on 09-Jun-2007 for febrile temperatures. At the time of reporting, the events had not yet resolved. The reporting physician considered the events life-threatening and probably related to acetaminophen.

Laboratory results of 09-Jun-2007 included the following: Prothrombin ratio 1.18 (Normal Range (NR) = 0.90-1.10), urine protein 2.60 g/l (NR = 0.00-0.07), serum glucose (10.27 mmol/L (NR = 3.88-6.38), creatinine 161 umol/l (NR = 15-102), ASAT 1.02 (NR = 0.24-0.44), and lactate 4.15 nmol/l (NR = 0.2-80). Laboratory results of 11-Jun-2007 included a urine protein of 4.54 g/l (NR = 0.07-0.07). Laboratory results of 13-Jun-2007 included the following: Prothrombin ratio 2.93 (NR = 0.90-1.10), fibrin split products 0.76 ug/ml (NR = 0.00-0.20), ASAT 89.10 umol/l (NR = 0.24-0.44), ALAT 214.40 umol/l (NR = 0.10-0.75), creatine kinase 8.34 umol/l (NR = 0.00-2.85), CK-MB 0.58 umol/l (NR = 0.00-0.42), LDH 15.71 umol/l (NR = 0.00-0.42), total bilirubin 133.2 umol/l (NR = 2.0-19.0), and glutamate dehydrogenase 1145 nmol/l (NR = 0-120). Laboratory results of 14-Jun-2007 included a fibrin split products of 0.52 ug/ml (NR = 0.00-0.20) and a total bilirubin of 148.9 umol/l (NR = 2.0-19.0). 16-Jun-2007 laboratory results included a creatine kinase of 5.60 umol/l (NR = 0.00-2.85). Laboratory results of 20-Jun-2007 included the following: ASAT 1.84 UMOL/L (NR = 0.24-0.44), ALAT 25.55 umol/l (NR = 0.10-0.75), ALP 2.33 umol/l (NR = 0.65-2.20), total bilirubin 67.9 (NR = 2.0-19.0), and GGT 3.75 umol/l (NR = 0.00-1.00).

Medical history included circulatory collapse due to heat and physical overstraining (marathon).

Reporter's comment:

Bristol-Myers Squibb Company, 17.01.2008 DE-Bristol-Myers Squibb Company-13820154

Sender diagnosis: not reported

Narrative

A hospital physician reported via a sales representative that a 43-year-old healthy, non-alcoholic male patient experienced life-threatening yellow colour of his eyes, and a massive elevation of transaminases after receiving 1 g of acetaminophen IV. Prior to receiving the acetaminophen, the patient had fainted and a blood sample showed an absolute electrolyte loss. Additionally, the he had a fever of 38.5°C; hence he was given the acetaminophen by a physician (not the reporting physician). The next day the patient visited the reporting hospital physician with yellow colour of the eyes. A massive elevation of the transaminases was diagnosed and the patient was on the verge of a liver transplantation. The reporting physician considered the situation severely critical. The patient was currently recovering.

BMS Medical Evaluation Comment: A 43-year-old non-alcoholic male patient experienced yellow colour of the eyes along with massive elevation of the transaminases after receiving 1 g of acetaminophen for fever. Reportedly, the patient had participated in a marathon and his blood sample had showed an absolute electrolyte loss. A therapeutic dose of acetaminophen in a non-alcoholic patient is not known to cause liver damage. In this case, role of dehydration as a major contributing factor for drug-event association can not be denied.

Supplemental information received from the reporting physician on 27-Jun-2007 indicated that the patient had developed thrombopenia and liver necrosis after administration of intravenous acetaminophen (1 g) on 09-Jun-2007 for febrile temperatures. At the time of reporting, the events had not yet resolved. The reporting physician considered the events life-threatening and probably related to acetaminophen.

Laboratory results of 09-Jun-2007 included the following: Prothrombin ratio 1.18 (Normal Range (NR) = 0.90-1.10), urine protein 2.60 g/l (NR = 0.00-0.07), serum glucose (10.27 mmol/L (NR = 3.88-6.38), creatinine 161 umol/l (NR = 15-102), ASAT 1.02 (NR = 0.24-0.44), and lactate 4.15 nmol/l (NR = 0.2-80). Laboratory results of 11-Jun-2007 included a urine protein of 4.54 g/l (NR = 0.07-0.07). Laboratory results of 13-Jun-2007 included the following: Prothrombin ratio 2.93 (NR = 0.90-1.10), fibrin split products 0.76 ug/ml (NR = 0.00-0.20), ASAT 89.10 umol/l (NR = 0.24-0.44), ALAT 214.40 umol/l (NR = 0.10-0.75), creatine kinase 8.34 umol/l (NR = 0.00-2.85), CK-MB 0.58 umol/l (NR = 0.00-0.42), LDH 15.71 umol/l (NR = 0.00-0.42), total bilirubin 133.2 umol/l (NR = 2.0-19.0), and glutamate dehydrogenase 1145 nmol/l (NR = 0-120). Laboratory results of 14-Jun-2007 included a fibrin split products of 0.52 ug/ml (NR = 0.00-0.20) and a total bilirubin of 148.9 umol/l (NR = 2.0-19.0). 16-Jun-2007 laboratory results included a creatine kinase of 5.60 umol/l (NR = 0.00-2.85). Laboratory results of 20-Jun-2007 included the following: ASAT 1.84 UMOL/L (NR = 0.24-0.44), ALAT 25.55 umol/l (NR = 0.10-0.75), ALP 2.33 umol/l (NR = 0.65-2.20), total bilirubin 67.9 (NR = 2.0-19.0), and GGT 3.75 umol/l (NR = 0.00-1.00).

Medical history included circulatory collapse due to heat and physical overstraining (marathon).

Supplemental information received on 14-Jan-2008 stated that after participation in a long distance run of 7.5 km (not marathon as previously reported) the patient was hospitalized due to collapse with persistent stupor and fever for which he was given acetaminophen i.v. Two days later, he developed life threatening transient liver failure. The patient was administered with a single dose of intravenous acetaminophen for fever with 40 degrees Celsius. Two days later thrombocytes decreased and transaminases increased up to 89 umol/L. During hospitalization an infusion therapy was performed. Transient liver failure was diagnosed and was treated with ITS (intensive care therapy), replacement of coagulation factors, application of ornithin-aspartate. The event rapidly improved no liver transplantation was performed. The patient was discharged with minimally increased liver values and he was well with a mild general weakness. Reporter stated that the liver damage could have possibly been due to ischemia or thermic and perhaps additionally damaged due to acetaminophen therapy.

BMS Medical Evaluation comment:

This non-alcoholic male patient experienced Liver failure, yellow colour of the eyes along with massive elevation of the transaminases after receiving 1 g of acetaminophen for fever. Reportedly, the patient had participated in a long distance run and his blood sample had showed an electrolyte loss. A therapeutic dose of acetaminophen in a non-alcoholic patient is not known to cause liver damage. In this case, role of dehydration as a major contributing factor for drug-event association can not be denied.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07056840

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth: 19641209	Age: 42 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug abuse					recovered/resolved
2 GGT increased					recovered/resolved
3 Intentional overdose					recovered/resolved
4 Hepatic enzymes increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL HEXAL (NGX)	Oral	/	-	Tooth pain
paracetamol			-	

Drugs (concomitant)

2 CARBAMAZEPINE	Oral	/	200612-
carbamazepine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bipolar affective disorder			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 28.06.2007 DE-SZ09-GXKR2007DE05214

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 18 Jun 2007 from a physician:

This patient with a medical history of bipolar disorder developed hepatic enzymes increased with GGT increased because of drug abuse by taking intentional overdoses of Paracetamol 500 HEXAL bei Fieber und Schmerzen (paracetamol) for tooth pain. The patient took paracetamol at a dosage of 3000 - 6000 mg daily for an unknown time period. Exact therapy dates were not stated. Concomitantly the patient received carbamazepine. When the physician detected a serious increase of hepatic enzymes with GGT increased to 524 U/l, all medications were stopped. After normalisation of hepatic enzymes the administration of paracetamol and carbamazepine was started again. The reporter assessed the causality of the adverse events and administration of paracetamol as not correlated.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07065343

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting					
2 Abdominal pain					
3 Intentional overdose				18 Hour	recovered/resolved
4 Malaise					
5 Liver function test abnormal					

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol	Unknown	/	-	Intentional overdose
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Suicide attempt			k.A.	suicide tentative

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Martens F Wohin? Indikation zur Intensivtherapie nach Intoxikation Der Notarzt 2007 23 53-55	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 23.07.2007 DE-BAYER-200714105GDS

Sender diagnosis: not reported

Narrative

This literature was received on 18-Jul-2007. The report was published by a health professional in Germany.

Case report:

This 24-year-old female patient was admitted to ER due to intentional overdose of paracetamol 18 hours ago. She was presented with abdominal pain, vomiting, malaise and moderately increased liver enzymes. She was treated with N-acetylcysteine.

A psychiatric survey revealed a history of suicidality that had subsided.

After ingestion of paracetamol a liver failure could not be excluded. No critical situation of her vital functions was observed. The interventional therapy with N-acetylcysteine was continued over three days.

Lab values were monitored without any signs of liver failure, clinically no signs of an encephalopathy were observed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07065854

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							Yes

Patient

Initials:	Sex: Female	Birth:	Age: 59 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Drug intoxication
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug intoxication					fatal
2 Hepatic encephalopathy					fatal
3 Acute liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Eisenbach C,Sieg O,Stremmel W,Encke J,Merle U. Diagnostic criteria for acute liver failure due to Wilson disease. World J Gastroenterol 2007 Mar 21;13:11:1711-1714.	

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 25.07.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-04310GD

Sender diagnosis: not reported

Narrative

A 59-year-old female patient with acute liver failure and grade 4 hepatic encephalopathy is reported. Etiology of acute liver failure was acetaminophen toxicity. The patient died from acetaminophen intoxication.

The baseline characteristics of the patient at initial presentation were as follows:

Alanine aminotransferase (normal range below 34 for females): 2301 U/l;

Alkaline phosphatase (normal range below 126 for females): 59 U/l;

Bilirubin: 4.0 mg/dl;

INR: 1.8;

Creatinine: 1.25 mg/dl;

Urinary copper: 0.80 mcmol/24 h.

It was outlined in general that all patients of the study had some degree of hepatic encephalopathy on admission.

This case is from a study with the purpose to outline differences in clinical and biochemical findings between fulminant Wilson disease (FWD) and acute liver failure (ALF) of other etiologies that might identify cases of FWD.

ALF was defined as new onset liver disease characterized by coagulopathy (international normalized ration (INR) ≥ 1.5) and any grade of hepatic encephalopathy (HE) in absence of any pre-diagnosed liver disease.

The clinical and biochemical findings of 15 patients meeting diagnostic criteria for ALF were analyzed. 7 Patients had presented with ALF due to

Wilson disease (WD) and 8 with ALF of other etiologies. Patients with ALF of other causes were analyzed as controls.

Boehringer Ingelheim GmbH, Germany, 26.07.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-04310GD

Sender diagnosis: not reported

Narrative

A 59-year-old female patient with acute liver failure and grade 4 hepatic encephalopathy is reported. Etiology of acute liver failure was acetaminophen toxicity. The patient died from acetaminophen intoxication.

The baseline characteristics of the patient at initial presentation were as follows:

Alanine aminotransferase (normal range below 34 for females): 2301 U/l;

Alkaline phosphatase (normal range below 126 for females): 59 U/l;

Bilirubin: 4.0 mg/dl;

INR: 1.8;

Creatinine: 1.25 mg/dl;

Urinary copper: 0.80 mcmol/24 h.

It was outlined in general that all patients of the study had some degree of hepatic encephalopathy on admission.

This case is from a study with the purpose to outline differences in clinical and biochemical findings between fulminant Wilson disease (FWD) and acute liver failure (ALF) of other etiologies that might identify cases of FWD.

ALF was defined as new onset liver disease characterized by coagulopathy (international normalized ration (INR) ≥ 1.5) and any grade of hepatic encephalopathy (HE) in absence of any pre-diagnosed liver disease.

The clinical and biochemical findings of 15 patients meeting diagnostic criteria for ALF were analyzed. 7 Patients had presented with ALF due to Wilson disease (WD) and 8 with ALF of other etiologies. Patients with ALF of other causes were analyzed as controls.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07066720

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 00044 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hemoglobin low					Unknown
2 Respiratory failure					Unknown
3 Respiratory acidosis					Unknown
4 Hyperkalemia					recovered/resolved
5 Leukocytosis					Unknown
6 Segmented neutrophil count					Unknown
7 Abnormal liver function tests					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen		/	-	
paracetamol			-	
2 amitriptyline		/	-	
amitriptyline			-	
3 meropenem		/	-	
meropenem			-	
4 methadone		/	-	
methadone			-	
5 metoprolol tartrate		/	-	
metoprolol tartrate			-	
6 omeprazole		/	-	
omeprazole			-	
7 ondansetron		/	-	
ondansetron			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Respiratory acidosis			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Beliaev AM, Smith W. Msetoprolol-induced hyerkalaemia in chronic respiratory acidosis. New Zealand Medical Journal.	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Potassium		6.9	MMOL/L	
Hemoglobin		90	G/L	
White blood cells		20.06	X10 9/L	
Neutrophils		17.85	X10 9/L	
Liver function test		: Abnormal		
pH body fluid NOS		7.277		

Narratives (nur bei elektronischem Eingang)Bristol-Myers Squibb Company, 26.07.2007 NZ-Bristol-Myers Squibb Company-13854260

Sender diagnosis: not reported

Narrative

This case was derived from scientific literature and describes a 44-year-old man that developed medically significant: hyperkalemia, respiratory acidosis, type 2 respiratory failure, leukocytosis, and segmented neutrophil count while on metoprolol, acetaminophen, methadone, meropenem, amitriptyline, ondansetron, and omeprazole. The patient also experienced low hemoglobin and abnormal liver function tests. The patient was admitted with acute bacterial endocarditis. At this time, his medications comprised of: oral methadone, intravenous meropenem, acetaminophen, amitriptyline, ondansetron and omeprazole. Persistent sinus tachycardia prompted an increase in the dosage of controlled-release (CR) metoprolol (therapeutic indication not stated) from 47.5 mg/day to 95 mg/day. Laboratory testing 3 days later revealed an increased blood potassium level of 6.9 mmol/L, a low haemoglobin level (90 g/L), leucocytosis (white blood cell count 20.06 times $10^9/L$, segment neutrophils 17.85 times $10^9/L$) and mildly abnormal liver function tests. Arterial blood gas analysis revealed a potassium level of 6.4 mmol/L and 'acute on chronic' respiratory acidosis (pH 7.277) with type 2 respiratory failure. The man was treated with nebulized salbutamol (albuterol) and calcium polystyrene sulfonate; however, his hyperkalaemia persisted for 3 consecutive days. After the combination of respiratory acidosis and an increased metoprolol dosage was suspected as a possible cause of his symptoms, metoprolol, salbutamol and calcium polystyrene sulfonate were stopped and his methadone dosage was significantly reduced. Subsequently, his hyperkalaemia resolved.

Medical history: who had a history of IV morphine abuse, chronic respiratory acidosis.

Author Comment: We conclude that "a combination of a moderate dose of metoprolol CR with chronic respiratory acidosis may be the possible cause of (the) patient's hyperkalaemia".

Literature citation: Beliaev AM, Smith W. Metoprolol-induced hyperkalaemia in chronic respiratory acidosis. New Zealand Medical Journal.

BMS Medical Evaluation Comment: A 44-year-old man developed medically significant hyperkalemia, after taking metoprolol for unknown period of time for unknown indication. Limited information provided makes causality inassessable.

Comments: The article does not specify whether the patient effectively received PERFALGAN (acetaminophen i.v.). The patient had a history of chronic respiratory acidosis, therefore respiratory failure was most likely connected to the underlying condition. The reporting physician assumes that chronic respiratory acidosis and metoprolol administration were causative for the development of hyperkalaemia. However, limited information and the fact that the article lists a multitude of suspect drugs make a final medical assessment concerning a single drug impossible.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07071198

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19630730	Age: 00043 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intravascular hemolysis	20070726				not recovered/not resolved
2 Anaemia macrocytic					not recovered/not resolved
3 Icterus	20070726				not recovered/not resolved
4 Fever					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Oral	00000004 G gram(s)/	20070722-	Influenza
2 reyataz atazanavir	Oral	00000400 Mg milligram(s)	200404-20070801 00003 Year	HIV infection
3 truvada emtricitabine; tenofovir disoproxil fumarate		/	- 00003 Year	HIV infection

Drugs (concomitant)

4 NORVIR ritonavir	Oral	00000100 Mg milligram(s)	200404-20070801 -	HIV infection
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
HIV infection			Unknown	
Alcohol abuse			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood bilirubin indirect		15.6	MG/DL	
Lactate dehydrogenase		507	U/L	
Haptoglobin		>7.1	MG/DL	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 08.08.2007 DE-Bristol-Myers Squibb Company-13867080

Sender diagnosis: not reported

Narrative

A physician reported via medical information that a male patient with a medical history of HIV had been immunologically and virologically stable while under therapy with atazanavir and emtricitabine + tenofovir for about 3 years. One week after taking paracetamol for influenza symptoms, the patient experienced jaundice and indirect bilirubinemia of 15.6 mg/dL (previously 6.1 mg/dL). His lactate dehydrogenase (LDH) was 507 U/L and haptoglobin < 7.1 mg/dL indicating intravascular haemolysis. The event was considered medically serious.

BMS medical evaluation comments: This patient, who had been on atazanavir and tenofovir + emtricitabine for three years developed intravascular hemolysis after treatment with paracetamol for influenza symptoms. Although self limiting hemolysis is known to occur after influenza, limited

information on underlying condition prevents a meaningful causality assessment.

In accordance with § 13 (2-4) GCP-Verordnung and the European Commission Directive 2001/20/EC (Clinical Trial Directive) we are obliged to send you the following SUSAR for IMP atazanavir. You receive this report in the name of Bristol-Myers Squibb International Corporation.

SUSAR:

- event#1

Additionally, this report is submitted to you in the name of the marketing authorisation holder Bristol-Myers Squibb GmbH & Co. KGaA.

Generic: acetaminophen

Event number; label*; causality**

#1; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: As there was no clear information provided concerning the route of administration of acetaminophen an intravenous application cannot definitely be excluded. However, the patient most likely took an oral formulation for treating influenza symptoms. Although self limiting hemolysis is known to occur after influenza, limited information on underlying condition prevents a meaningful causality assessment.

Reporter's comment:

Bristol-Myers Squibb Company, 16.08.2007 DE-Bristol-Myers Squibb Company-13867080

Sender diagnosis: not reported

Narrative

A physician reported via medical information that a male patient with a medical history of HIV had been immunologically and virologically stable while under therapy with atazanavir and emtricitabine + tenofovir for about 3 years. One week after taking paracetamol for influenza symptoms, the patient experienced jaundice and indirect bilirubinemia of 15.6 mg/dL (previously 6.1 mg/dL). His lactate dehydrogenase (LDH) was 507 U/L and haptoglobin < 7.1 mg/dL indicating intravascular haemolysis. The event was considered medically serious.

BMS medical evaluation comments: This patient, who had been on atazanavir and tenofovir + emtricitabine for three years developed intravascular hemolysis after treatment with paracetamol for influenza symptoms. Although self limiting hemolysis is known to occur after influenza, limited information on underlying condition prevents a meaningful causality assessment.

Supplemental information was received on 10-Aug-2007 from the physician who indicated the 43-year-old male patient experienced medically serious macrocytic hyperchromic anaemia, icterus, and intravascular haemolysis during therapy with atazanavir. Therapy with atazanavir 400 mg and ritonavir 100 mg was initiated in Apr-2004. The macrocytic hyperchromic anaemia began prior to 21-Jan-2005 which was not resolved. Laboratory results on 23-May-2007 showed total bilirubin 6.73 +, direct bilirubin 0.61+, indirect bilirubin 6.12 +, folic acid 4.5; leukocytes 6.8, neutrophils 56.6, lymphocytes 32.5, hemoglobin 11.5-, hematocrit 35.6-, lactate dehydrogenase (LDH) 372+, cholesterol 137, creatinine 1.16, uric acid 6.8, and glucose 89. On 22-Jul-2007, the patient was treated for fever with oral paracetamol/acetaminophen 4 g. On 26-Jul-2007, he experienced icterus and intravascular haemolysis. Therapy with atazanavir and ritonavir was discontinued on 01-Aug-2007. Indirect bilirubin was measured at 15.7 mg/dL, lactate dehydrogenase (LDH) was 507 U/L and haptoglobin < 7 mg/dL (normal range 30-200). The bilirubin value was regressive, nevertheless the antiretroviral therapy (ART) was switched. On 30-Jul-2007 revealed the following: total bilirubin 8.38+; bilirubin direct 0.67; indirect bilirubin 7.71+; iron 77; folic acid 6.9; leukocytes 8.0 +; neutrocytes 57.6, lymphocytes 30.8, hemoglobin 11.8, and hematocrit 36.7. At the time of reporting, the events had not yet resolved. The patient was not hospitalized. The reporting physician considered the events to be possibly related to atazanavir and paracetamol.

Medical history included HIV infection and alcohol abuse.

Bristol-Myers Squibb Company, 21.11.2007 DE-Bristol-Myers Squibb Company-13867080

Sender diagnosis: not reported

Narrative

A physician reported via medical information that a male patient with a medical history of HIV had been immunologically and virologically stable while under therapy with atazanavir and emtricitabine + tenofovir for about 3 years. One week after taking paracetamol for influenza symptoms, the patient experienced jaundice and indirect bilirubinemia of 15.6 mg/dL (previously 6.1 mg/dL). His lactate dehydrogenase (LDH) was 507 U/L and haptoglobin < 7.1 mg/dL indicating intravascular haemolysis. The event was considered medically serious.

BMS medical evaluation comments: This patient, who had been on atazanavir and tenofovir + emtricitabine for three years developed intravascular hemolysis after treatment with paracetamol for influenza symptoms. Although self limiting hemolysis is known to occur after influenza, limited information on underlying condition prevents a meaningful causality assessment.

Supplemental information was received on 10-Aug-2007 from the physician who indicated the 43-year-old male patient experienced medically serious macrocytic hyperchromic anaemia, icterus, and intravascular haemolysis during therapy with atazanavir. Therapy with atazanavir 400 mg and ritonavir 100 mg was initiated in Apr-2004. The macrocytic hyperchromic anaemia began prior to 21-Jan-2005 which was not resolved. Laboratory results on 23-May-2007 showed total bilirubin 6.73 +, direct bilirubin 0.61+, indirect bilirubin 6.12 +, folic acid 4.5; leukocytes 6.8, neutrophils 56.6, lymphocytes 32.5, hemoglobin 11.5-, hematocrit 35.6-, lactate dehydrogenase (LDH) 372+, cholesterol 137, creatinine 1.16, uric acid 6.8, and glucose 89. On 22-Jul-2007, the patient was treated for fever with oral paracetamol/acetaminophen 4 g. On 26-Jul-2007, he experienced icterus and intravascular haemolysis. Therapy with atazanavir and ritonavir was discontinued on 01-Aug-2007. Indirect bilirubin was measured at 15.7 mg/dL, lactate dehydrogenase (LDH) was 507 U/L and haptoglobin < 7 mg/dL (normal range 30-200). The bilirubin value was regressive, nevertheless the antiretroviral therapy (ART) was switched. On 30-Jul-2007 revealed the following: total bilirubin 8.38+; bilirubin direct 0.67; indirect bilirubin 7.71+; iron 77; folic acid 6.9; leukocytes 8.0 +; neutrophils 57.6, lymphocytes 30.8, hemoglobin 11.8, and hematocrit 36.7. At the time of reporting, the events had not yet resolved. The patient was not hospitalized. The reporting physician considered the events to be possibly related to atazanavir and paracetamol.

Medical history included HIV infection and alcohol abuse.

Supplemental information received on 14-Nov-2007: The physician stated that the patient did not have any concomitant diseases such as prosthetic cardiac valves, G-6-PD deficiency, thrombotic thrombocytopenia purpura, disseminated intravascular coagulation or paroxysmal nocturnal hemoglobinuria that could have explained the intravascular hemolysis. The patient also did not have previous episodes of anemia, hemolysis or jaundice. HIV was first diagnosed in 1995. Paracetamol had previously been administered and was well tolerated.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07071756

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth:	Age: 38 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice	2005				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	Influenza
paracetamol			-	
2 reyataz		/	-	
atazanavir sulfate			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 09.08.2007 DE-Bristol-Myers Squibb Company-13869664

Sender diagnosis: not reported

Narrative

A physician reported, via BMS-Germany (#2007080305), that a patient experienced jaundice in 2005 while under therapy with atazanavir and paracetamol. The paracetamol was used to treat the symptoms of influenza.

This case was considered medically serious.

BMS medical evaluation comments: This patient developed jaundice while under treatment with atazanavir for HIV and paracetamol for influenza symptoms. Although jaundice could be attributed to atazanavir, it is unclear if the jaundice developed after treatment with paracetamol. Limited information prevents a more meaningful causality assessment.

Reporter's comment:

Bristol-Myers Squibb Company, 16.08.2007 DE-Bristol-Myers Squibb Company-13869664

Sender diagnosis: not reported

Narrative

A physician reported, via BMS-Germany (#2007080305), that a patient experienced jaundice in 2005 while under therapy with atazanavir and paracetamol. The paracetamol was used to treat the symptoms of influenza.

This case was considered medically serious.

BMS medical evaluation comments: This patient developed jaundice while under treatment with atazanavir for HIV and paracetamol for influenza symptoms. Although jaundice could be attributed to atazanavir, it is unclear if the jaundice developed after treatment with paracetamol. Limited information prevents a more meaningful causality assessment.

Supplemental information received from the physician via BMS-Germany on 09-Aug-2007 reported that the 38 year-old male patient experienced positive cells while taking atazanavir. He began taking 300 milligrams of atazanavir sulfate for the treatment of his HIV on 20-Sep-2004. On 01-Jun-2005 the patient developed anemia, with an increase in LDH along with a decrease in haptoglobin. On 26-Jul-2005 he experienced icterus and macro-hematuria. The medications atazanavir, ritonavir and tenofovir disoproxil fumarate with emtricitabine were stopped on 26-Jul-2005. The icterus and macro-hematuria resolved after about 14 days, and the anemia and, LDH increased and haptoglobin decrease resolved after 120 days. After the first occurrence of icterus and hematuria ART (antiviral therapy with atazanavir, ritonavir and tenofovir (ART) was paused and symptoms and lab values declined. They re-exposed the patient to the drug from 05-Dec-2005 until 13-Dec-2005 and the patient was a positive challenge for icterus, LDH increase, hemoglobin decrease and haptoglobin decrease during that interval. Additionally on 12-Dec-2007, he had hyperbilirubinemia which lasted for about 7 days. It was also reported that the patient did not receive paracetamol as was previously stated.

Relevant medical history included HIV, alcohol abuse, smoker, toxic nutritional hepatopathy and reactive depression.

Laboratory values were listed as follows: hemoglobin from Jan-2005

15.4 grams per deciliter (g/dL), Jul-2005 9.7 g/dL; Nov-2005 14.0 g/dL, Dec-2005 12.9 g/dL. LDH values: Nov-2004 210 units per liter (U/L), Jun-2005 525 U/L; Nov-2005 215 U/L, Dec-2005 427 U/L. Haptoglobin values: Sep-2005 <6.69 mg/dL.

Also received was a translated report from a physician who had seen the patient on 27-Sep-2005, which reported the following information: For several months the patient suffered from anemic symptoms with hemoglobin decrease to 9.7 g/dL, LDH increase, increased bilirubin even during the atazanavir pause and increased reticulocytes. During therapy pause, CD4 positive cells decreased from 450 to 266/ul within one month. His physical exam findings were as follows: physical exam normal, no icterus, no anemia, no significantly enlarged lymph nodes palpable. Sonography: remarkable splenomegaly with 17 cm longitudinal diameter. No abdominal lymph nodes. Findings were normal for liver and 2 kidneys, biliary tracts inconspicuous. Medications were as follows: therapy with lamivudine, didanosine and efavirenz. The most recent laboratory results were as follows: laboratory examination showed considerable regression of his hemolysis situation. The hemoglobin value had almost normalized with 12.6 g/dL, the reticulocytes and haptoglobin values were within the normal range. Only the LDH remained considerably elevated at 358 U/L, however was also regressive. The physician suspected a hypersplenism with intensified metabolism of erythrocytes due to the Coombs' negativity and the large spleen. A cold agglutinin examination was not performed due to the spontaneous improvement of the symptoms. The medications of lamivudine, didanosine and efavirenz were held (paused). The physician also felt that with this size of a spleen and with the rapid decrease of the D4 positive cells antiretroviral therapy should probably be re-initiated. The patient also had a significantly low vitamin B 12 value; however this was probably not related to the events. The patient's MCV value of about 110 fL was not adequate to be diagnosed as perniciousa. He felt that therapy with 1 ampule vitamin B12 could be tentatively initiated. He also felt that the events diagnosed as hemolysis with Zieve's syndrome, possibly within the scope of an episode of alcohol abuse. The event of jaundice captured in version 1 is deleted since jaundice is a symptom of Zieve Syndrome, a possible diagnosis reported by reporting physician in version 2.

In physician's opinion, event of Zieve Syndrome was within scope of an episode of alcohol abuse.

BMS Medical Evaluation Comment: A 38 year old male having history of alcohol abuse, experienced Zieve syndrome, approximately after 8 months of atazanavir therapy for HIV infection. Alcohol abuse is a known risk factor for Zieve syndrome. In this case, due to drug-event temporal relationship, positive de-challenge and positive re-challenge, the role of drug can not be excluded.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07084454

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19930115	Age: 14 Year	weight (kg): 45	BMI_int: 17.58	Age fetus:
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Patientdeath

deathdate: 20070830	deathcause: Lactic acidosis
deathdate: 20070830	deathcause: Multi-organ failure
deathdate: 20070830	deathcause: Cardiac failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Seizures	20070830			2 Day	not recovered/not resolved
2 Lactic acidosis	20070830	20070830		2 Day	fatal
3 Disseminated intravascular coagulation	20070830	20070830		2 Day	fatal
4 Respiratory failure					Unknown
5 Cardiac failure					Unknown
6 Hepatic failure					Unknown
7 Renal failure					Unknown
8 Intentional overdose	20070829	20070829	1 Day	1 Day	fatal
9 Multi-organ failure	20070829	20070830	2 Day	1 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Norfenefrine	Oral	/	20070829-20070829	Drug use for unknown indication
norfenefrine		Tablet	-	indication
2 Paracetamol	Oral	/	20070829-20070829	Drug use for unknown indication
paracetamol			1 Day	indication
3 THIOCTACID HR	Oral	60 G gram(s)/Day	20070829-20070829	Drug use for unknown indication
thioctic acid		Tablet	1 Day	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Suicide attempt			Unknown	suicide attempt

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Drug level		30900	µg/l	
Drug level		966	µg/l	

Narratives (nur bei elektronischem Eingang)

MEDA, 07.09.2007 DE-MEDA-V-DE-2007-1330

Sender diagnosis: not reported

Narrative

A phone call was received from a physician on 30-Aug-2007, his written report was received on 05-09-2007.

On 29-AUG-2007 a 14-year-old female adolescent (1.60 meters, 45 kg) suicidally ingested an unknown amount of THIOCTACID 600HR (a potential maximum of 60 g) and NORFENEFINE of unknown dosage. Additionally, she ingested a small amount of PARACETAMOL. An emergency treatment (ventilation, catecholamines, etc.) was performed at an ICU. The clinical course included seizures, lactic acidosis, respiratory, cardiac, hepatic and renal failure, DIC (dissiminated intravascular coagulation) and death on 30-AUG-2007. It was determined by autopsy that the patient died because of lactic acidosis and multi-organ failure including cardiac failure. Relevant laboratory data (Thioctacid 600 HR level) were not yet available at the time of the report.

The patient's medical history included a prior suicide attempt. Otherwise she was healthy.

The physician assessed the events as probably related to THIOCTACID.

Reporter's comment:

The physician assessed the events as probably related to THIOCTACID.

Bayer Vital GmbH, 13.09.2007 DE-BAYER-200715260GDS

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This spontaneous report was received on 30-Aug-2007 from a health professional by phone and on 05-Sep-2007 in written form via MEDA Pharma in Germany, referring to a 14 year-old female.

Medical and drug history:

Patient's relevant medical history includes a previous suicide attempt.

It is unknown whether any concomitant drugs have been given.

Suspect drugs, timing and conditions surrounding the onset of the reactions:

The patient took

- THIOCTACID HR (thioctic acid) for an unknown indication at a potential maximum dose of 60 G orally on 29-Aug-2007;
- Paracetamol for an unknown indication at a small amount on 29-Aug-2007;
- Norfenefrine (norfenefrine) for an unknown indication at an unknown amount on 29-Aug-2007.

The progression of the events and their outcome in the patient:

On 29-Aug-2007, the patient experienced INTENTIONAL OVERDOSE, which was considered serious as finally leading to death.

On 30-Aug-2007, the patient died from MULTIORGAN FAILURE (symptoms: respiratory failure, cardiac failure, hepatic failure, renal failure).

The events were treated with catecholamines.

Relevant details of fatal outcome:

The patient died on 30-Aug-2007. Autopsy was performed.

The clinical course included seizures, lactic acidosis, and dissiminated intravascular coagulation (DIC).

According to the reporter, the patient died due to lactic acidosis and multi-organ failure including cardiac failure.

The original reporter's clinical assessment:

For the drug THIOCTACID HR the relationship was considered as probable.

No assessment was given for Paracetamol and Norfenefrine.

Further information is expected.

MEDA, 20.05.2008 DE-MEDA-V-DE-2007-1330

Sender diagnosis: not reported

Narrative

A phone call was received from a physician on 30-Aug-2007, his written report was received on 05-09-2007.

On 29-AUG-2007 a 14-year-old female adolescent (1.60 meters, 45 kg) suicidally ingested an unknown amount of THIOCTACID 600HR (a potential maximum of 60 g) and NORFENEFINE of unknown dosage. Additionally, she ingested a small amount of PARACETAMOL. An emergency treatment (ventilation, catecholamines, etc.) was performed at an ICU. The clinical course included seizures, lactic acidosis, respiratory, cardiac, hepatic and renal failure, DIC (dissiminated intravascular coagulation) and death on 30-AUG-2007. It was determined by autopsy that the patient died because of lactic acidosis and multi-organ failure including cardiac failure. Relevant laboratory data (Thioctacid 600 HR level) were not yet available at the time of the report.

The patient's medical history included a prior suicide attempt. Otherwise she was healthy.

The physician assessed the events as probably related to THIOCTACID.

Follow-up information was received from the physician on 13-May-2008:

The physician provided information on thioctacid plasma levels: After release by the public prosecutor's office two blood samples of the patient were examined for thioctacid plasma levels. The tests revealed significantly increased thioctacid plasma levels: 30900 µg/l and 966 µg/l respectively. There is no information on the time the samples were taken.

The physician assessed the events as probably related to THIOCTACID.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07097128

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No	YES	

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cardiopulmonary arrest					fatal
2 Pericardial effusion					Unknown
3 Left atrial dilatation					Unknown
4 Pleural effusion					Unknown
5 Ascites					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Intra-uterine	/	-	Drug exposure in utero
2 BUSCOPAN hyoscine butylbromide	Intra-uterine	/	-	Drug exposure in utero
3 DIPIDOLOR piritramide	Intra-uterine	/ INJECTION	-	Drug exposure in utero
4 PARACETAMOL paracetamol	Intra-uterine	/	-	Drug exposure in utero

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany, 10.10.2007 DE-JNJFOC-20070905813

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician concerns a newborn of unknown age and sex from Germany: A.

The patient's weight and medical history were unknown.

One week before the patients birth (after the 34th gestational week), the patient was exposed to piritramide (injection, intrauterine) in utero.

Non-company suspect drugs included: acetylsalicylic acid, hyoscine butylbromide, and paracetamol.

Directly after the patients birth (on 35th gestational week), the patient looked healthy, and the amniotic fluid was clear, but a short time later, the patient experienced a decrease in heart frequency and apnoea.

The patient died despite ventilation and resuscitation. The autopsy showed pleural effusion, pericardial effusion, ascites, severe dilation of the left atrium and no postpartal deployment of the lung. The cardiotocogram right before the birth was without pathological findings. The last dose of piritramide was given 6 hours before the birth.

Concomitant medications were not reported.

Treatment with piritramide, acetylsalicylic acid, hyoscine butylbromide and paracetamol was withdrawn.

This report was serious (death).

This parent/child case is linked to 20070905788.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07098132

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19881110	Age: 18 Year	weight (kg): 64	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased					Unknown
2 Creatine kinase increased					Unknown
3 Suicide attempt					Unknown
4 Tachycardia					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 APC acetylsalicylic acid; caffeine; paracetamol	Oral	5 Df dosage form/	20071009-20071009	Drug use for unknown indication
2 DOLOMO acetylsalicylic acid; caffeine; codeine phosphate; paracetamol	Oral	2 Df dosage form/	20071009-20071009	Drug use for unknown indication
3 EQUASYM methylphenidate hydrochloride	Oral	100 Mg milligram(s)/	20071009-20071009	Drug use for unknown indication
4 ERGENYL valproate sodium	Oral	300 Mg milligram(s)/	20071009-20071009	Drug use for unknown indication
5 PARACETAMOL paracetamol	Oral	2000 Mg milligram(s)/	20071009-20071009	Drug use for unknown indication
6 RISPERDAL risperidone	Oral	5 Mg milligram(s)/ TABLET	20071009-20071009	Drug use for unknown indication

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatine kinase		increased		
Aspartate aminotransferase		increased		
Alanine aminotransferase		increased		

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany, 18.10.2007 DE-JNJFOC-20071002134

Sender diagnosis: not reported

Narrative

This spontaneous report received from an anonymous physician via Toxicologic Center Mainz concerns an 18-year-old male patient from Germany: A. The patient's medical history was unknown, and the patient's weight was 64 kilograms.

On 9-OCT-2007, the patient attempted suicide by taking risperidone (tablets, oral) 5 mg a total and non-company suspect drugs: thomapyrin, codiphen, methylphenidate hydrochloride, valproate sodium and paracetamol. It was not reported whether the patient was prescribed risperidone routinely. On 10-OCT-2007 (two hours after the intake of the drugs) the patient experienced tachycardia (120-<130), increased liver enzymes (values unspecified) and increased creatine kinase (values unspecified). The aspartate aminotransferase and alanine aminotrasferase were reported elevated (values unspecified). Concomitant medications were not reported.

Treatment with risperidone, thomapyrin, codiphen, methylphenidate hydrochloride, valproate sodium and paracetamol was withdrawn on 09-OCT-2007.

The patient outcome was unknown for suicide attempt, tachycardia, increased liver enzymes and increased creatine kinase.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07098927

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19881110	Age: 18 Year	weight (kg): 64	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20071009			1 Day	Unknown
2 Creatine kinase increased	20071009			1 Day	Unknown
3 Intentional overdose	20071009	20071009	1 Day	1 Day	Unknown
4 Suicide attempt	20071009	20071009	1 Day	1 Day	Unknown
5 Tachycardia	20071009			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 APC acetylsalicylic acid; caffeine; paracetamol	Oral	5 Df dosage form/	20071009-20071009	1 Day
2 DOLOMO acetylsalicylic acid; caffeine; codeine phosphate; paracetamol	Oral	2 Df dosage form/	20071009-20071009	1 Day
3 EQUASYM methylphenidate hydrochloride		/	-	-
4 EQUASYM methylphenidate hydrochloride	Oral	100 Mg milligram(s)/	20071009-20071009	1 Day
5 ERGENYL valproate sodium	Oral	300 Mg milligram(s)/	20071009-20071009	1 Day
6 PARACETAMOL paracetamol	Oral	2000 Mg milligram(s)/	20071009-20071009	1 Day
7 RISPERDAL risperidone	Oral	5 Mg milligram(s)/	20071009-20071009	1 Day

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Heart rate		120-130	/min	

Narratives (nur bei elektronischem Eingang)

UCB GmbH, 23.10.2007 DE-UCBSA-8027477

Sender diagnosis: not reported

Narrative

A clinician reported via the drug intoxication center in Mainz that an 18-year-old male patient who was on Equasym (methylphenidate) medication made a suicide attempt on 09-Oct-2007. He took Equasym 100 mg PO, APC (aspirin/acetaminophen/caffeine) 5 tablets PO, Dolomo (aspirin/acetaminophen/caffeine) 2 tablets PO, Ergenyl (valproic acid) 300 mg 1 tablet PO, paracetamol 500 mg 4 tablets PO (2000 mg) and Risperdal (risperdon) 1 mg 5 tablets PO (5 mg). The patient experienced tachycardia (120-130/min) and increased GOT, GPT and CK. At the time of reporting, outcome was unknown.

INTENTIONAL OVERDOSE OF EQUASYM (100 MG), APC (5 TABLETS), DOLOMO (2 TABLETS), ERGENYL (300 MG), PARACETAMOL (2000 MG) AND RISPERDAL (5 MG) IN THE FRAME OF A SUICIDE ATTEMPT. THE PATIENT EXPERIENCED TACHYCARDIA AND INCREASED CK, GOT AND GPT LEVELS. UNKNOWN FURTHER EVOLUTION.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07101143

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19911128	Age: 15 Year	weight (kg): 68	BMI_int: 22.72	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GOT increased	20071019			8 Day	recovered/resolved
2 GPT increased	20071019			8 Day	recovered/resolved
3 Abdominal pain	20071019				Unknown
4 Shivering	20071018				Unknown
5 Hepatotoxicity	20071019				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Microgynon 30 (21) ethinylestradiol; levonorgestrel	Oral	1 Df dosage form/24 Hour Coated tablet	20071019-20071031 12 Day	Oral contraceptive
2 PARACETAMOL paracetamol	Oral	500 Mg milligram(s)/	20071011-20071017 6 Day	Abdominal discomfort

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Abdominal discomfort	20071011	20071017	No	abdominal complaints
House dust mite allergy			Unknown	house dust mite allergy
Liver biopsy			Unknown	Liver biopsy revealed a drug induced toxic liver damage.

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Amylase, serum	20071019	56,6	U/L	
Amylase, serum	20071022	50.4	U/L	
Amylase, serum	20071029	56.5	U/L	
Bilirubin direct	20071022	0.86	mg/dl	
Bilirubin direct	20071021	0.98	mg/dl	
Bilirubin direct	20071026	0.74	mg/dl	
Bilirubin direct	20071019	0.96	mg/dl	
Bilirubin direct	20071029	0.67	mg/dl	
Bilirubin total	20071022	1.76	mg/dl	
Bilirubin total	20071026	1.59	mg/dl	
Bilirubin total	20071021	2.26	mg/dl	
Bilirubin total	20071019	1.61	mg/dl	
Bilirubin total	20071029	1.57	mg/dl	

Bilirubin total	20071031	0.99	mg/dl
GGT	20071022	24.25	U/L
GGT	20071031	24.02	U/L
GGT	20071021	21.62	U/L
GGT	20071019	21.44	U/L
GGT	20071026	22.40	U/L
GGT	20071029	23.58	U/L
SGOT	20071031	196	U/L
SGOT	20071019	778	U/L
SGOT	20071022	746	U/L
SGOT	20071021	908	U/L
SGOT	20071026	532	U/L
SGOT	20071029	305	U/L
SGPT	20071021	2074.65	U/L
SGPT	20071022	1898.18	U/L
SGPT	20071026	1436.20	U/L
SGPT	20071019	1917.49	U/L
SGPT	20071029	961.74	U/L
SGPT	20071031	689.06	U/L

Narratives (nur bei elektronischem Eingang)

SCHERING, 29.10.2007 DE-SHR-DE-2007-040112

Sender diagnosis: not reported

Narrative

A physician - OB/Gyn reported on 24 Oct 2007, the occurrence of increased transaminases in a 16 year-old female who was prescribed ethinylestradiol, levonorgestrel (Microgynon 30 (21), coated tablet).

The patient's past drug included Cerazette. Concurrent medical conditions and concomitant medications were not reported.

On an unspecified date, the patient received ethinylestradiol, levonorgestrel (Microgynon 30 (21)) at 1 tab(s), 21d/28d via oral route of administration for oral contraception.

On an unspecified date, the patient presented with increased transaminases. A laboratory test confirmed this finding. The patient was hospitalized. It was unknown if/when the patient was discharged from the hospital.

The outcome was not reported.

Dechallenge/Rrechallenge was not reported for Microgynon 30 (21).

Follow-up has been requested.

The physician - ob/gyn considered the relationship of increased transaminases to treatment with Microgynon 30 (21) as not classified.

SCHERING, 14.11.2007 DE-SHR-DE-2007-040112

Sender diagnosis: not reported

Narrative

A physician - OB/Gyn reported on 24 Oct 2007, the occurrence of increased transaminases in a 16 year-old female who was prescribed ethinylestradiol, levonorgestrel (Microgynon 30 (21), coated tablet).

The patient's past drug included Cerazette. Concurrent medical conditions and concomitant medications were not reported.

On an unspecified date, the patient received ethinylestradiol, levonorgestrel (Microgynon 30 (21)) at 1 tab(s), 21d/28d via oral route of administration for oral contraception.

On an unspecified date, the patient presented with increased transaminases. A laboratory test confirmed this finding. The patient was hospitalized. It was unknown if/when the patient was discharged from the hospital.

The outcome was not reported.

Dechallenge/Rchallenge was not reported for Microgynon 30 (21).

Follow-up has been requested.

Suppl. (05-NOV-2007): Gynecologist's report

The patient's historical condition included abdominal complaints. The patient's past drug included desogestrel (Cerazette). Patient had been on treatment with Cerazette (desogestrel) for one year and switched to Microgynon. Concurrent medical conditions included BMI of 22.7 and house dust mite allergy. Concomitant medications were not reported. It was not reported if Microgynon 30 (21) was used previously and tolerated. It was not reported if Paracetamol was used previously and tolerated.

On 19 Oct 2007 the patient started the intake of ethinylestradiol, levonorgestrel (Microgynon 30 (21)). From 11 Oct 2007 to 17-OCT-2007, few days prior to the first intake of Microgynon, the patient took paracetamol (Paracetamol) at 3x500 mg for abdominal complaints.

On an unspecified date the patient was hospitalized (no precise admission date or discharge date provided). The laboratory findings showed significantly increased transaminases: SGOT (778 U/L) on 19 Oct 2007, SGOT (908 U/L) on 21 Oct 2007, SGOT (746 U/L) on 22 Oct 2007, SGOT (532 U/L) on 26 Oct 2007, SGOT (305 U/L) on 29 Oct 2007, SGOT (196 U/L) on 31 Oct 2007, SGPT (1917.49 U/L) on 19 Oct 2007, SGPT (2074.65 U/L) on 21 Oct 2007, SGPT (1898.18 U/L) on 22 Oct 2007, SGPT (1436.20 U/L) on 26 Oct 2007, SGPT (961.74 U/L) on 29 Oct 2007, SGPT (689.06 U/L) on 31 Oct 2007, GGT (21.44 U/L) on 19 Oct 2007, GGT (21.62 U/L) on 21 Oct 2007, GGT (24.25 U/L) on 22 Oct 2007, GGT (22.40 U/L) on 26 Oct 2007, GGT (23.58 U/L) on 29 Oct 2007, GGT (24.02 U/L) on 31 Oct 2007, Amylase, serum (56.6 U/L) on 19 Oct 2007, Amylase, serum (50.4 U/L) on 22 Oct 2007, Amylase, serum (56.5 U/L) on 29 Oct 2007, Bilirubin total (1.61 mg/dl) on 19 Oct 2007, Bilirubin total (2.26 mg/dl) on 21 Oct 2007, Bilirubin total (1.76 mg/dl) on 22 Oct 2007, Bilirubin total (1.59 mg/dl) on 26 Oct 2007, Bilirubin total (1.57 mg/dl) on 29 Oct 2007, Bilirubin total (0.99 mg/dl) on 31 Oct 2007, Bilirubin direct (0.96 mg/dl) on 19 Oct 2007, Bilirubin direct (0.98 mg/dl) on 21 Oct 2007, Bilirubin direct (0.86 mg/dl) on 22 Oct 2007, Bilirubin direct (0.74 mg/dl) on 26 Oct 2007, Bilirubin direct (0.67 mg/dl) on 29 Oct 2007, HBs-antigen: negative and nonnucleosis (test): negative

A liver biopsy was performed and revealed a drug induced toxic liver damage. On 31 Oct 2007, during hospital stay, the treatment with Microgynon was discontinued. After discontinuation patient's complaints subsided.

The patient recovered.

Microgynon 30 (21) was withdrawn on 31 Oct 2007. Dechallenge for Microgynon 30 (21) was positive. No rechallenge information was reported for Microgynon 30 (21).

Further information has been requested.

Bayer Vital GmbH, 28.02.2008 DE-BAYER-DE-2007-040112

Sender diagnosis: not reported

Narrative

A physician - OB/Gyn reported on 24 Oct 2007, the occurrence of increased transaminases in a 16 year-old female who was prescribed ethinylestradiol, levonorgestrel (Microgynon 30 (21), coated tablet).

The patient's past drug included Cerazette. Concurrent medical conditions and concomitant medications were not reported.

On an unspecified date, the patient received ethinylestradiol, levonorgestrel (Microgynon 30 (21)) at 1 tab(s), 21d/28d via oral route of administration for oral contraception.

On an unspecified date, the patient presented with increased transaminases. A laboratory test confirmed this finding. The patient was hospitalized. It was unknown if/when the patient was discharged from the hospital.

The outcome was not reported.

Dechallenge/Rchallenge was not reported for Microgynon 30 (21).

Follow-up has been requested.

Suppl. (05-NOV-2007): Gynecologist's report

The patient's historical condition included abdominal complaints. The patient's past drug included desogestrel (Cerazette). Patient had been on treatment with Cerazette (desogestrel) for one year and switched to Microgynon. Concurrent medical conditions included BMI of 22.7 and house dust mite allergy. Concomitant medications were not reported. It was not reported if Microgynon 30 (21) was used previously and tolerated. It was not reported if Paracetamol was used previously and tolerated.

On 19 Oct 2007 the patient started the intake of ethinylestradiol, levonorgestrel (Microgynon 30 (21)). From 11 Oct 2007 to 17-OCT-2007, few days prior to the first intake of Microgynon, the patient took paracetamol (Paracetamol) at 3x500 mg for abdominal complaints.

On an unspecified date the patient was hospitalized (no precise admission date or discharge date provided). The laboratory findings showed significantly increased transaminases: SGOT (778 U/L) on 19 Oct 2007, SGOT (908 U/L) on 21 Oct 2007, SGOT (746 U/L) on 22 Oct 2007, SGOT (532 U/L) on 26 Oct 2007, SGOT (305 U/L) on 29 Oct 2007, SGOT (196 U/L) on 31 Oct 2007, SGPT (1917.49 U/L) on 19 Oct 2007, SGPT (2074.65 U/L) on 21 Oct 2007, SGPT (1898.18 U/L) on 22 Oct 2007, SGPT (1436.20 U/L) on 26 Oct 2007, SGPT (961.74 U/L) on 29 Oct 2007, SGPT (689.06 U/L) on 31 Oct 2007, GGT (21.44 U/L) on 19 Oct 2007, GGT (21.62 U/L) on 21 Oct 2007, GGT (24.25 U/L) on 22 Oct 2007, GGT (22.40 U/L) on 26 Oct 2007, GGT (23.58 U/L) on 29 Oct 2007, GGT (24.02 U/L) on 31 Oct 2007, Amylase, serum (56,6 U/L) on 19 Oct 2007, Amylase, serum (50.4 U/L) on 22 Oct 2007, Amylase, serum (56.5 U/L) on 29 Oct 2007, Bilirubin total (1.61 mg/dl) on 19 Oct 2007, Bilirubin total (2.26 mg/dl) on 21 Oct 2007, Bilirubin total (1.76 mg/dl) on 22 Oct 2007, Bilirubin total (1.59 mg/dl) on 26 Oct 2007, Bilirubin total (1.57 mg/dl) on 29 Oct 2007, Bilirubin total (0.99 mg/dl) on 31 Oct 2007, Bilirubin direct (0.96 mg/dl) on 19 Oct 2007, Bilirubin direct (0.98 mg/dl) on 21 Oct 2007, Bilirubin direct (0.86 mg/dl) on 22 Oct 2007, Bilirubin direct (0.74 mg/dl) on 26 Oct 2007, Bilirubin direct (0.67 mg/dl) on 29 Oct 2007, HBs-antigen: negative and nonnucleosis (test): negative

A liver biopsy was performed and revealed a drug induced toxic liver damage. On 31 Oct 2007, during hospital stay, the treatment with Microgynon was discontinued. After discontinuation patient's complaints subsided.

The patient recovered.

Microgynon 30 (21) was withdrawn on 31 Oct 2007. Dechallenge for Microgynon 30 (21) was positive. No rechallenge information was reported for Microgynon 30 (21).

Further information has been requested.

[Reporters comment] The physician - ob/gyn considered the relationship of increased transaminases to treatment with Microgynon 30 (21) as not classified. Suppl. (05 Nov 2007): Despite Microgynon is reported as suspected drug, the reporting clinician considers the event associated with Paracetamol intake.

[Addendum 30-NOV-2007: This case was converted from the Schering AG Argus database into the Bayer Schering Pharma AG Clintrace. The medical content of the case was not changed.]

Suppl. (30-Jan-2008): Despite repeated requests no follow-up information could be obtained.

Case closed.

Suppl. (20-Feb-2008): Hospital physician's report

On 19-Oct-2007 the patient applied to internal department due to persistent dull abdominal pain since 1.5 weeks which were severe in the evening. The day before patient had suffered from shivering. According to anamnesis, the patient had been treated with Cerazette for one year. Three to four weeks ago she changed to Microgynon. Moreover the patient had taken a cumulative dose of Paracetamol of 1500 mg during precedent week. Alcohol or drugs were not consumed. Sonographies of abdomen on 19-Oct-2007 and 22-Oct-2007 were without pathological findings.

Serology and serum assay see other relevant tests section.

Result of liver puncture: A distinct cholestasis with disperse bile pigment sedimentation, mild fatty degeneration of hepatocytes (10 %) and mild eosinophilia. Evidence of cholestasis proven by iron stain, no significant siderosis nor fibrosis.

Assessment: Liver tissue with significant unspecific inflammation of mesenchyme, moderate cholestasis, fatty liver degeneration of 10 % of hepatocytes.

During hospitalisation the transaminases decreased. The physician considered a histological result as compatible to drug induced toxic liver damage. The paracetamol treatment was assessed as most probable cause. It was recommended to continue the treatment with Microgynon which had been discontinued after admission. On 31-Oct-2007 the patient could be discharged from the hospital in good general condition.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07103068

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hyponatraemia					Unknown
2 Lack of drug effect					Unknown
3 Hypocalcaemia					Unknown
4 Hypoalbuminaemia					recovered/resolved
5 Hyperbilirubinaemia					recovered/resolved
6 Fever					Unknown
7 Fatigue					recovered/resolved
8 Exanthem					recovered/resolved
9 CRP increased					Unknown
10 Coagulopathy					recovered/resolved
11 Arthralgia					recovered/resolved
12 Anaemia hypochromic					Unknown
13 Leukocytosis					
14 Transaminases increased					Unknown
15 Acute liver failure					recovered/resolved
16 Still's disease					recovered/resolved
17 Proteinuria					Unknown
18 Night sweat					Unknown
19 Myalgia					Unknown
20 Lymphadenopathy					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	1800 Mg milligram(s)/Day	-	Fever
ibuprofen			-	
2 Paracetamol	Unknown	4000 Mg milligram(s)/Day	-	Fever
paracetamol			-	

Drugs (concomitant)

3 Ceftriaxon	Unknown	/	-	Drug use for unknown indication
ceftriaxone sodium			-	
4 Clarithromycin	Unknown	/	-	Drug use for unknown indication
clarithromycin			-	
5 methylprednisolone	Intravenous (not otherwis	250 Mg milligram(s)/ Solution for infusion	- 3 Day	Still's disease
methylprednisolone				
6 methylprednisolone	Intravenous (not otherwis	250 Mg milligram(s)/ Solution for infusion	- -	Still's disease
methylprednisolone				
7 Piperacillin/Tazobactam	Unknown	/	- -	Drug use for unknown indication
piperacillin; tazobactam				
8 prednisolone	Oral	400 Mg milligram(s)/Day Tablet	- -	Still's disease
prednisolone				
9 prednisolone	Oral	12 Mg milligram(s)/Day Tablet	- -	Still's disease
prednisolone				

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Still's disease			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Linde B, Oelzner P, Katenkamp K, Hein G, Wolf G. [Fulminate liver failure in a 39-year-old female patient with leukocytosis, unclear Fever, and arthralgic pain.] Med Klin (Munich) 2007; 102: 846-51	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ALT increased		1.03	umol/l.s	
AST increased		1.06	umol/l.s	
Bilirubin		313	umol/L	
C-reactive protein increased		185	mg/l	
ESR		93	mm/h	
Ferritin high		1740	Âµg/l	
Gamma-glutamyltransferase increased		2.73	Âµmol/l.s	
INR		2.05	NA	
LDH increased		12.48	umol/l.s	
Leukocytosis		23.6	Gpt/l	
Quick's test		29	%	
Sonogram		mild hepatosplenomegaly		
Liver biopsy		subacute hepatitis with necrosis + an cholangitis		
Albumin		16	G/L	
Alkaline phosphatase		8.57	MCMOL/L	
Bilirubin direct		228.5	MCMOL/L	
Blood cholinesterase		41	MCMOL/L	
C-reactive protein		28.3	MG/L	
Calcium		1.9	MMOL/L	
Fibrinogen		1.0	G/L	
Lactate dehydrogenase		15.54	MCMOL/L	
Serum ferritin		15405	MCG/L	
SGOT		15.93	MCMOL/L	
SGPT		26.21	MCMOL/L	
Sodium		134	MMOL/L	
Ammonia		133	MCMOL/L	
Protein total		34	G/L	

Narratives (nur bei elektronischem Eingang)

Krewel Meuselbach GmbH, 31.10.2007 DE-KREWEL-000414

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of

corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

could have also played a role in the development of the liver failure. A causality with the concomitant drugs cannot be excluded. 4 g paracetamol is a subtoxic daily dose that could be toxic in special patient groups e.g. diseases like Still's disease. Liver failure is listed in cases of high doses for Captin our paracetamol containing medicinal products. The patient recovered after treatment. Further measures are not necessary.

Reporter's comment:

AbZ Pharma GmbH, 02.11.2007 DE-ABZ-2007A106056

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with Paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of Paracetamol. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

to diverse antibiotic treatment. But the opinion of the authors is endorsed that the most probable cause for hepatic failure in this case was the underlying Still disease. Actions regarding the wording of the RSI is not necessary.

Reporter's comment:

ratiopharm GmbH, 02.11.2007 DE-RATIOPH-2007R105989

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with Paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of Paracetamol. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

to diverse antibiotic treatment. But the opinion of the authors is endorsed that the most probable cause for hepatic failure in this case was the underlying Still disease. Actions regarding the wording of the RSI is not necessary.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 05.11.2007 DE-PFLEGER-000042

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol and ibuprofen was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

CAUSALITY ASSESSMENT for ibuprofen: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: unlikely, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

Molimin Arzneimittel GmbH, 06.11.2007 DE-MOLIMIN-000011

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol and ibuprofen was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

Still's disease must be considered as part of the differential diagnosis of acute liver failure.

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co., 08.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

As Azur and Azur compositum contain 350 mg, 450 mg and 600 mg of paracetamol, respectively, it is not possible to take a single dose of 1000 mg without parting the medicinal product. With regard to the reduced liver function caused by the underlying Still's disease and with a view to concomitant drugs one cannot clearly appoint a causal relationship of paracetamol nor either rule out. However, paracetamol is known to cause liver failure following high dose/long term administration. A corresponding hint is given in the SPCs of Azur and Azur compositum. Furthermore, it is mentioned not to use Azur and Azur compositum in cases of hepatocellular insufficiency. Therefore, there is no change of the safety profile, no measures to be taken.<dbtext>To_Char_NARRATIVE_2</dbtext>

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co. 09.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal anti-inflammatories, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

As Azur and Azur compositum contain 350 mg, 450 mg and 600 mg of paracetamol, respectively, it is not possible to take a single dose of 1000 mg without parting the medicinal product. With regard to the reduced liver function caused by the underlying Still's disease and with a view to concomitant drugs one cannot clearly appoint a causal relationship of paracetamol nor either rule out. However, paracetamol is known to cause liver failure following high dose/long term administration. A corresponding hint is given in the SPCs of Azur and Azur compositum. Furthermore, it is mentioned not to use Azur and Azur compositum in cases of hepatocellular insufficiency. Therefore, there is no change of the safety profile, no measures to be taken.

Follow-up report created on 09-Nov-2007 following an error message of the EMEA because of a wrong entry in the field "date received by manufacturer". In fact, no new information was received today, this update is due only because of technical reporting errors.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co. 13.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal anti-inflammatories, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

As Azur and Azur compositum contain 350 mg, 450 mg and 600 mg of paracetamol, respectively, it is not possible to take a single dose of 1000 mg without parting the medicinal product. With regard to the reduced liver function caused by the underlying Still's disease and with a view to concomitant drugs one cannot clearly appoint a causal relationship of paracetamol nor either rule out. However, paracetamol is known to cause liver failure following high dose/long term administration. A corresponding hint is given in the SPCs of Azur and Azur compositum. Furthermore, it is mentioned not to use Azur and Azur compositum in cases of hepatocellular insufficiency. Therefore, there is no change of the safety profile, no measures to be taken.

Follow-up report created on 09-Nov-2007 following an error message of the EMEA because of a wrong entry in the field "date received by manufacturer". In fact, no new information was received today, this update is due only because of technical reporting errors.

Follow-up report created on 12-Nov-2007 following a 2nd error message of the EMEA despite correction of the field "date received by manufacturer". No new information was received until to date. Thus, this update is again due only because of a technical reporting error.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.11.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06834GD

Sender diagnosis: not reported

Narrative

A female patient with Still's disease is reported, in whom symptomatic antiphlogistic treatment with ibuprofen and paracetamol was administered in addition to antibiotic therapy. The patient experienced fulminate liver failure.

In the opinion of the authors, the most likely cause of the liver failure was hepatotoxic adverse drug reactions (especially due to paracetamol) in combination with a pre-existing liver damage associated with the concomitant Still's disease (Still's disease was judged to be the main factor). An involvement of the antibiotic therapy in the development of the liver failure was also considered by the authors.

The 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degC, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and oral penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 mcg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 mcmol/l.s, aspartate aminotransferase 1.06 mcmol/l.s, gamma-glutamyltransferase 2.73 mcmol/l.s, and lactate dehydrogenase 12.48 mcmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued (intravenous ceftriaxone was started, later switched to piperacillin/tazobactam and finally to clarithromycin) without any improvement. In addition, acetaminophen (up to 4 x 1,000 mg/day) and ibuprofen (up to 3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital and treated with acetylcysteine for suspected paracetamol toxicity. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. 16 Months after the incident, the patient was without symptoms under prednisone 3 mg/day; the liver function was normal.

The authors state: 'A mild to moderate increase in liver enzymes is often found as part of this disease (Still's disease). Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen.'

This is a summary of an article written in German. An English translation has been ordered.

Boehringer Ingelheim GmbH, Germany, 06.12.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06834GD

Sender diagnosis: not reported

Narrative

A female patient with Still's disease is reported, in whom symptomatic antiphlogistic treatment with ibuprofen and paracetamol was administered in addition to antibiotic therapy. The patient experienced fulminate liver failure.

In the opinion of the authors, the most likely cause of the liver failure was hepatotoxic adverse drug reactions (especially due to paracetamol) in combination with a pre-existing liver damage associated with the concomitant Still's disease (Still's disease was judged to be the main factor). An

involvement of the antibiotic therapy in the development of the liver failure was also considered by the authors.

The 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degC, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and oral penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 mcg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 mcmol/l.s, aspartate aminotransferase 1.06 mcmol/l.s, gamma-glutamyltransferase 2.73 mcmol/l.s, and lactate dehydrogenase 12.48 mcmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued (intravenous ceftriaxon was started, later switched to piperacillin/tazobactam and finally to clarithromycin) without any improvement. In addition, acetaminophen (up to 4 x 1,000 mg/day) and ibuprofen (up to 3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital and treated with acetylcysteine for suspected paracetamol toxicity. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. 16 Months after the incident, the patient was without symptoms under prednisone 3 mg/day; the liver function was normal.

The authors state: 'A mild to moderate increase in liver enzymes is often found as part of this disease (Still's disease). Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen.'

This is a summary of an article written in German. An English translation has been ordered.

The English translation of the case report:

A 39-year-old woman initially presented to a hospital elsewhere with an extensive, reddish, non-pruritic maculopapular rash on both thighs which subsequently spread to the trunk and the other extremities. Over the previous week, she had had difficulty swallowing and had experienced a feeling of tightness in the throat. She reported symmetric arthralgias and myalgias and had also had spiking fevers of up to 38 degC and night sweats, as well as symptoms of marked fatigue, loss of vitality and reduced performance. The patient had no known history of chronic illness, was a non-smoker and had not been abroad in the last few months. As the fever was of unknown origin and there was no definitive source of infection, she was treated empirically with non-steroidal anti-inflammatory drugs (NSAIDs), antihistamines and oral penicillin on an outpatient basis.

Laboratory studies carried out at the hospital of presentation revealed a leukocytosis of $23.6 \times 10^9/l$ (normal $3.8 - 9.8 \times 10^9/l$) with predominant neutrophilia, in addition to a C-reactive protein (CRP) of 185 mg/l (normal < 5 mg/l), an erythrocyte sedimentation rate (ESR) of 52/93 mm/h (normal $6 - 10$ mm/h) and a ferritin level of 1740 $\mu g/l$ (normal $10 - 130$ mcg/l). Levels of transaminases were also increased (alanine aminotransferase (ALAT) 1.03 mcmol/l (normal < 0.74 mcmol/l), aspartate aminotransferase (ASAT) 1.06 mcmol/l (normal < 0.58 mcmol/l)), as were levels of gamma-glutamyltransferase (gamma-GT) (2.73 mcmol/l; normal < 0.92 mcmol/l) and lactate dehydrogenase (LDH) (12.48 mcmol/l; normal < 4.13 mcmol/l).

Ultrasound findings were unremarkable except for a mild hepatosplenomegaly. No pathognomonic features were identified on radiological examination of the chest, wrists or ankles or on echocardiography. Extensive serological testing, which comprised assays for hepatitis A, B and C, Epstein-Barr virus (EBV), cytomegalus virus (CMV), syphilis and human immunodeficiency virus (HIV), gave negative results, as did repeated blood cultures. On suspicion that the rash was infection-related, intravenous antibiotic treatment was started, initially with ceftriaxone, then with piperacillin plus tazobactam and finally, as fever spikes of up to 40 degC continued to occur, clarithromycin. Symptomatic treatment with paracetamol 4 x 1000 mg/day and ibuprofen 3 x 600 mg/day was also started. However, marked jaundice ensued (direct bilirubin 313 mcmol/l (normal < 3 mcmol/l), g-GT 16.28 mcmol/l), there was a massive increase in the levels of liver enzymes (ALAT 30.45 mcmol/l, ASAT 22.10 mcmol/l, LDH 20.64 mcmol/l) and a coagulopathy developed (Quick value 29%/International Normalized Ratio (INR) 2.05); the spiking fevers also persisted. Still's disease was suspected and a single dose of 250 mg methylprednisolone was given. After this, the patient was transferred to the authors hospital for management of her acute liver failure, which now dominated the clinical picture.

On admission, the patient was of normal body habitus but in reduced general condition, with marked scleral and cutaneous jaundice and a generalised maculopapular rash. Her left cervical lymph nodes were palpably enlarged but there were no other abnormal findings in the head and neck area. Fine bubbling rales were heard over the bases of both lungs on auscultation. Heart sounds were pure and rhythmical and the pulse rate was 72/min. All peripheral pulses were palpable. The liver was palpable about 3 - 4 cm below the costal arch. The spleen was impalpable. The spine and the extremities were normal on clinical examination. There were no neurological abnormalities.

Initially, the patient had low levels of calcium (1.9 mmol/l; normal 2.2 - 2.65 mmol/l), total protein (34 g/l; normal 69 - 86 g/l) and albumin (16 g/l; normal 35 - 53 g/l). She was mildly hyponatraemic (134 mmol/l; normal 135 - 145 mmol/l) but levels of all other electrolytes were within the normal range. Levels of creatinine and urea were also normal. After the dose of methylprednisolone administered earlier, CRP was only slightly raised at 28.3 mg/l. The ferritin level was extremely high (15,405 mcg/l; normal 10 - 291 mcg/l) and transaminase levels were also significantly elevated (ASAT 15.93 mcmol/l, ALAT 26.21 mcmol/l). Elevations were also observed in the levels of glutamate dehydrogenase (1666 nmol/l; normal < 80 nmol/l), LDH (15.54 mcmol/l) and alkaline phosphatase (8.57 mcmol/l; normal < 2.0 mcmol/l). There was also marked hyperbilirubinaemia (direct bilirubin 228.5 mcmol/l). The level of cholinesterase was 41 mcmol/l (normal 65 - 180 mcmol/l), reflecting impaired hepatic synthesis. Levels of ammonia were raised (133 mcmol/l; normal 11.5 - 38.0 mcmol/l). Microcytic hypochromic anaemia was present, as was leukocytosis ($13.9 \times 10^9/l$). The platelet count was normal. The Quick value was 40% (INR 1.8) and fibrinogen was reduced (1.0 g/l; normal 1.8 - 3.5 g/l). Tests for classical and perinuclear

antineutrophil cytoplasmic antibodies (c-ANCA and p-ANCA), antinuclear antibodies (ANA) and anti-double stranded DNA antibodies were negative. Proteinuria was present (766 mg/l; normal < 150 mg/l) but there were no leukocytes or erythrocytes in the urinary sediment. On the assumption that the patient was suffering from toxic liver damage, a paracetamol assay was carried out. This gave negative results, although admittedly the test was performed several days after the last administration of the drug.

To determine the cause of the liver failure, a percutaneous liver biopsy was carried out after coagulation products had been administered to normalise coagulation parameters and platelet counts.

Still's disease was presumptively diagnosed and the patient was treated with intravenous methylprednisolone 250 mg/day for 3 days followed by oral prednisolone 100 mg/day. For the first time, her fever resolved and she reported a subjective improvement in well-being. However, liver failure persisted and hepatic synthesis continued to be markedly impaired, so evaluation for high-urgency liver transplantation was begun. Conservative treatment with acetylcysteine was started on the suspicion that there might have been additional liver damage due to paracetamol. When 300 mg/kg had been infused over a period of 20 h, however, the lab results showing that paracetamol had not been detected in the blood became available and treatment with acetylcysteine was discontinued. In the absence of specific therapeutic options, it was decided to administer ornithine aspartate 15 g/day by intravenous infusion and ursodeoxycholic acid 750 mg/day orally. High-dose corticosteroid therapy was maintained and the raised levels of liver enzymes fell significantly, thus avoiding the need for a liver transplant.

The patient's liver function eventually returned to normal. As regards her Still's disease, follow-up tests showed complete resolution of inflammatory activity despite a reduction in the prednisolone dosage. Immunosuppressant therapy therefore did not need to be increased and, 14 months after the initial event, the patient is completely symptom-free on a regime of low-dose prednisolone (3 mg/day) and her liver function is normal.

The authors state: "In summary, the cause of the liver failure in our patient is difficult to identify retrospectively. As she had only taken paracetamol at therapeutic dosages, her liver failure is probably most likely to have been due to a toxic effect of the drug superimposed on liver damage arising as a result of the underlying Still's disease. A further rare cause of liver failure, idiosyncratic antibiotic toxicity, must also be considered in view of the various antibiotics she received. However, the fact that her liver function rapidly improved on corticosteroid therapy suggests to us that her Still's disease was a principal factor in the acute liver damage she sustained."

Bayer Vital GmbH, 15.01.2008 DE-BAYER-200718258GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 27-Nov-2007.

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degrees C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 microg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 micromol/l.s, aspartate aminotransferase 1.06 micromol/l.s, gamma-glutamyltransferase 2.73 micromol/l.s, and lactate dehydrogenase 12.48 micromol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 x 1,000 mg/day) and ibuprofen (3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

Authors comment: The etiology of Still's disease is unknown and the disease is characterized by fever episodes, a typical macular-papular exanthema, lymphadenopathy, hepatosplenomegaly, and arthralgias. A mild to moderate increase in liver enzymes is often found as part of this disease. Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen. Still's disease must be considered as part of the differential diagnosis of acute liver failure, because an early diagnosis and consequent therapy with prednisone may prevent the need for liver transplantation.

Bayer Vital GmbH, 18.01.2008 DE-BAYER-200718258GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 27-Nov-2007.

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degrees C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 microg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 micromol/l.s, aspartate aminotransferase 1.06 micromol/l.s, gamma-glutamyltransferase 2.73 micromol/l.s, and lactate

dehydrogenase 12.48 micromol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 x 1,000 mg/day) and ibuprofen (3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

Authors comment: The etiology of Still's disease is unknown and the disease is characterized by fever episodes, a typical macular-papular exanthema, lymphadenopathy, hepatosplenomegaly, and arthralgias. A mild to moderate increase in liver enzymes is often found as part of this disease. Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen. Still's disease must be considered as part of the differential diagnosis of acute liver failure, because an early diagnosis and consequent therapy with prednisone may prevent the need for liver transplantation.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07105658

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes Yes Yes

Patient

Initials: Sex: Female Birth: 19911128 Age: 15 Year weight (kg): 68 BMI_int: 22.2 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20071018			8 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL-RATIOPHARM 500 paracetamol	Oral	/ Tablet	20071011-20071018 8 Day	Abdominal pain

Drugs (concomitant)

2 Microgynon (Dragees) ethinylestradiol; levonorgestrel	Oral	/ Film-coated tablet	20070918-20071021 34 Day	Contraception
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
House dust allergy			Yes	

Primary source

Qualification: Pharmacist observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20071031	689.06	U/l	
GPT	20071019	1917.49	U/l	
GPT	20071021	2074.65	U/l	
GPT	20071026	1436.20	U/l	
GPT	20071022	1898.18	U/l	
Bilirubin	20071019	1.61	mg/dl	
Bilirubin	20071021	2.26	mg/dl	
Bilirubin	20071029	1.57	mg/dl	
Bilirubin	20071026	1.59	mg/dl	
Bilirubin	20071031	0.99	mg/dl	
Bilirubin	20071022	1.76	mg/dl	
GPT	20071029	961.74	U/l	

Narratives (nur bei elektronischem Eingang)

ratiofarm GmbH, 08.11.2007 DE-RATIOPH-2007R105957

Sender diagnosis: not reported

Narrative

Reported terms: Hepatic enzymes increased(Hepatic enzyme increased)

Case reference number 2007R105957 is a spontaneous case report sent by a pharmacist which refers to a female aged 16 years. Case was confirmed by a health professional.

The patient's past medical history included no medical records. There is no information about risk factors. Concomitant diseases include House dust allergy (House dust allergy). The following drugs are known to have been taken by the patient prior to the event: Cerazette(indication: Contraception, started: ??-??-2006).

The patient started taking Paracetamol-ratiopharm 500 (PARACETAMOL), indication Abdominal pain on 11-Oct-2007. The patient has been exposed to the following concomitant drug(s): Microgynon (Dragees) (LEVONORGESTREL AND ESTROGEN), indication Contraception. The patient developed Hepatic enzymes increased (Hepatic enzyme increased)(start date 18-Oct-2007). The patient was hospitalized.

The following action was taken: for Paracetamol-ratiopharm 500 (PARACETAMOL) - Drug withdrawn

The outcome for Hepatic enzymes increased (Hepatic enzyme increased) is Recovering / Resolving.

Rechallenge information for Paracetamol-ratiopharm 500 is not applicable. Dechallenge information for Paracetamol-ratiopharm 500 is not applicable.

The reporter's causality for the case is Certain. The reporter's assessment for drug Paracetamol-ratiopharm 500 related with Hepatic enzymes increased is as follows: Certain. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07108635

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Condition aggravated					Unknown
2 Fulminant hepatic failure					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL paracetamol	Oral	4000 Mg milligram(s)/Day -	-	Drug use for unknown indication

Drugs (concomitant)

2 ANTI HISTAMINES	Unknown	/	-	Drug use for unknown indication
3 ibuprofen ibuprofen	Oral	1800 Mg milligram(s)/Day -	-	Drug use for unknown indication
4 penicillin penicillin nos	Oral	/	-	Drug use for unknown indication

Primary source

Qualification: Other Health Professional

observestudytype:

Literature: Linde B, Oelzner P, Katenkamp K, Hein G, Wolf G. Fulminant liver failure in a 39-year-old female patient with leukocytosis, unclear fever and arthralgic pain. Med Klin. 2007;102:846-51.

Narratives (nur bei elektronischem Eingang)

[SANOFI-SYNTHELABO, 12.11.2007 DE-SANOFI-SYNTHELABO-A01200712250](#)

Sender diagnosis: not reported

Narrative

Reaction as reported: fulminant liver failure
 LLT: FULMINANT HEPATIC FAILURE / 10017469
 PT: ACUTE HEPATIC FAILURE / 10000804

Reaction as reported: condition aggravated
 LLT: CONDITION AGGRAVATED / 10010264
 PT: CONDITION AGGRAVATED / 10010264

Narrative:
 Literature abstract received on 30-Oct-2007 (Lit PCM 06/07).

A 39-year-old woman was admitted because of a non-itching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degrees C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/L), an increase in C-reactive protein (185 mg/L), and a ferritin level of 1,740 mcg/L were found. Liver enzymes were increased (alanine aminotransferase 1.03 mcmol/L.s, aspartate aminotransferase 1.06mcmol/L.s, gamma-glutamyltransferase 2.73 mcmol/L.s, lactate dehydrogenase 12.48 mcmol/L.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 x 1,000 mg/day) and ibuprofen (3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and

improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

Conclusion: The etiology of Still's disease is unknown and the disease is characterized by fever episodes, a typical macular-papular exanthema, lymphadenopathy, hepatosplenomegaly, and arthralgias. A mild to moderate increase in liver enzymes is often found as part of this disease. Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen. Still's disease must be considered as part of the differential diagnosis of acute liver failure, because an early diagnosis and consequent therapy with prednisone may prevent the need for liver transplantation.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07108262

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatosplenomegaly					Unknown
2 Liver failure					recovered/resolved
3 Hepatitis					Unknown
4 Necrosis					Unknown
5 Cholangitis					Unknown
6 Icterus					recovered/resolved
7 Coagulopathy					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ansaid	Unknown	/	-	
flurbiprofen			-	
2 CEFTRIAZONE (NGX)	Intravenous (not otherwis	/	-	Fever
ceftriaxone			-	
3 CLARITHROMYCIN (NGX)	Intravenous (not otherwis	/	-	Fever
clarithromycin			-	
4 IBUPROFEN (NCH)	Unknown	1800 Mg milligram(s)/Day	-	
ibuprofen			-	
5 PARACETAMOL (NGX)	Unknown	4000 Mg milligram(s)/Day	-	
paracetamol			-	
6 PHENOXYMETHYLPENICILLIN (NGX)	Oral	/	-	Pyrexia of unknown origin
phenoxymethylpenicillin			-	
7 PIPERACILLIN+TAZOBACTAM (NGX)	Intravenous (not otherwis	/	-	
piperacillin; tazobactam			-	

Drugs (concomitant)

8 ANTI-HISTAMINES	Unknown	/	-
			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lymphadenopathy			Unknown	associated with Morbus Still
Dysphagia			Unknown	for 2 weeks, associated with Morbus Still
Arthralgia			Unknown	symmetrical, associated with Morbus Still
Myalgia			Unknown	symmetrical, associated with Morbus Still
Fever			Unknown	up to 38.5°C, associated with Morbus Still
Night sweats			Unknown	associated with Morbus Still
Still's disease			Unknown	Morbus Still with elevated inflammatory parameters
Papular rash circumocular-blepharal			Unknown	on both thighs with spreading to trunk, associated with Morbus Still

Primary source

Qualification:	observestudytype:
Literature:	Med Klin (Munich); Linde B, Oelzner P, Katenkamp K, Hein G, Wolf G; 102; 2007; 846-51; Fulminate liver failure in a 39-year-old female patient with leukocytosis, unclear fever, and arthralgic pain

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 13.11.2007 DE-SZ09-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics. adverse event.

Reporter's comment:NOVARTIS, 13.11.2007 DE-NOVNCHHQ-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics. adverse event.

Reporter's comment:NOVARTIS, 19.11.2007 DE-NOVNCHHQ-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics.

Significant FU received on 09 Nov 2007. Full text translation received. No new information provided. The SPC for piperacillin-tazobactam was available and the listedness was provided.

Serious literature report (life-threatening) assessed as unlisted according to the Core Data Sheet Information due to greater specificity/severity. The following terms (hepatitis, liver enzymes elevated) are already included in the Core Data Sheet Information.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 20.11.2007 DE-SZ09-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics.

Significant FU received on 09 Nov 2007. Full text translation received. No new information provided. The SPC for piperacillin-tazobactam was available and the listedness was provided.

Serious literature report (life-threatening) assessed as unlisted according to the Core Data Sheet Information due to greater specificity/severity. The following terms (hepatitis, liver enzymes elevated) are already included in the Core Data Sheet Information.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07108240

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19870919	Age: 20 Year	weight (kg): 40	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug abuse					Unknown
2 Suicide attempt					Unknown
3 Overdose					Unknown
4 Liver failure					Unknown
5 Vomiting					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Buscopan	Oral	/	-	
hyoscine butylbromide		Tablet	-	
2 PARACETAMOL 500	Oral	500 Mg milligram(s)/	-	
paracetamol		Tablet	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 13.11.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06651DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 20 -years -old female patient took 1 x 9 tablets Buscopan (scopolaminbutylbromide), 1 x 10 tablets Paracetamol 500 in suicidal intention and hospitalisation into an ICU was recommended.

Suicide attempt, drug abuse, overdose, permanent vomiting and incipient liver failure were reported.

Symptoms were moderate.

Outcome unknown. Medical history, concomitant diagnosis and medication is not reported.

Boehringer Ingelheim GmbH, Germany, 14.11.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06651DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 20 -years -old female patient took 1 x 9 tablets Buscopan (scopolaminbutylbromide), 1 x 10 tablets Paracetamol 500 in suicidal intention and hospitalisation into an ICU was recommended.

Suicide attempt, drug abuse, overdose, permanent vomiting and incipient liver failure were reported.

Symptoms were moderate.

Outcome unknown. Medical history, concomitant diagnosis and medication is not reported.

The level of detail in the report does not permit a coherent assessment of the case. Based on the information provided, no change to the company core safety information or other measures are recommended at this time.

DUEMPELM

Case assessment created by DP on 13 Nov 2007

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08019333

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 8 Year	weight (kg): 27	BMI_int: 14.18	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Tumour lysis syndrome					recovered/resolved
2 Serum lactate dehydrogenase abnormal			72 Hour	12 Hour	recovered/resolved
3 Hepatomegaly					Unknown
4 Tumor lysis syndrome					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 alfentanil		/	-	Anesthesia
alfentanil			-	
2 allopurinol		/	-	Postoperative renal failure
allopurinol			-	
3 desflurane		/	-	Anesthesia
desflurane			-	
4 dexamethasone	Intravenous bolus	4 Mg milligram(s)/Day Injection	-	Prophylaxis against postoperative nausea and
dexamethasone			-	
5 furosemide		/	-	Postoperative renal failure
furosemide			-	
6 paracetamol	Intravenous bolus	500 Mg milligram(s)/Day	-	Postoperative pain relief
paracetamol			-	
7 propofol		/	-	Anesthesia
propofol			-	
8 remifentanil		/	-	Anesthesia
remifentanil			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute lymphoblastic leukemia			Unknown	
Nausea prophylaxis			Unknown	prophylaxis against postoperative nausea and vomiting
Central venous catheterisation			Unknown	
Hepatomegaly			Yes	Liver 2cm and spleen 5cm below costal margin.
Hepatosplenomegaly			Unknown	
Precursor B-lymphoblastic leukaemia acute			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Osthaus WA, Linderkamp C, Bünte C, Jütner B, Sümpelmann R. Tumor lysis associated with dexamethasone use in a child with leukemia. Paediatr Anaesth. 2008 Mar,18(3);268-70.	
Qualification:	Physician	observestudytype:
Literature:	Osthaus WA, Linderkamp C, Bunte C, Juttner B, Sumpelmann R. Tumor lysis associated with dexamethasone use in a child with leukemia. Pediatr-Anesth 2008; 18(3) 268-270	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Serum phosphate		2.22	mmol/l	

	DE-BFARM-08019333	
Leukocyte count NOS	58.8	x 10e9/l
Leukocyte count NOS	10.4	x 10e9/l
Creatinine	Increased 2-fold from baseline over 2 days	
Lactate dehydrogenase	(day of operation) 3220	
Lactate dehydrogenase	(1st postoperative day, peak) 3486	
Phosphorus	(Serum) increased from 1.93 to 2.22	
Serum potassium	Remained unchanged	
Serum uric acid	Remained unchanged	
Leukocyte count NOS	(Day after dexamethasone) dropped 58.8 to 10.4	
Blood phosphorus	day 1: 2.22	mmol/l
Serum lactate dehydrogenase	day 1: 3220; day 2: 3486	
Leukocyte count NOS	day 1: 10.400	per 10exp9

Narratives (nur bei elektronischem Eingang)

BETA, 22.02.2008 DE-BETA-7336/08L

Sender diagnosis: not reported

Narrative

An 8-year-old male patient with pre B-cell acute lymphoblastic leukaemia developed tumour lysis after treatment with a single dose of dexamethasone. He presented for implantation of a central venous port catheter for receiving chemotherapy. After institution of routine monitoring, anaesthesia was induced intravenously with propofol and alfentanil. Anaesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anaesthesia, 4 mg dexamethasone intravenously immediately after induction of anaesthesia was administered. For prophylaxis of postoperative pain, the patient received 500 mg paracetamol intravenously 15 min after induction. A cerebrospinal fluid and bone marrow tap was performed and a port catheter into the right internal jugular vein was implanted. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l) was noted, peaking on the first postoperative day (at 3486 U/l). On the third day, LDH returned to preoperative levels. Serum phosphate increased slightly from 1.93 to 2.22 mmol/l on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. The leukocyte count dropped from 58.8 x 10e9/l to 10.4 x 10e9/l the day after administration of dexamethasone as a sign of tumour cell lysis. To protect the kidney from failure, fluid intake was increased and corrective treatment was given. Taking account of the intraoperative dose of dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the treatment protocol. On postoperative day 9 he was discharged from the hospital. The authors associate the event to dexamethasone treatment and suggest to use other antiemetic drugs such as antiserotonergic agents in cancer patients.

NOTE: According to the reporting authors "in rare cases TLS can occur spontaneously or through surgical intervention but usually follows cytotoxic therapy".

ratiopharm GmbH, 25.02.2008 DE-RATIOPH-2008R021640

Sender diagnosis: not reported

Narrative

An 8-year-old male patient with pre B-cell acute lymphoblastic leukaemia developed tumour lysis after treatment with a single dose of Dexamethasone. He presented for implantation of a central venous port catheter for receiving chemotherapy. After institution of routine monitoring, anaesthesia was induced intravenously with propofol and alfentanil. Anaesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anaesthesia, 4 mg Dexamethasone was administered intravenously immediately after induction of anaesthesia. For prophylaxis of postoperative pain, the patient received 500 mg paracetamol intravenously 15 min after induction. A cerebrospinal fluid and bone marrow tap was performed and a port catheter was implanted into the right internal jugular vein. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l) was noted, peaking on the first postoperative day (at 3486 U/l). On the third day, LDH returned to preoperative levels. Serum phosphate increased slightly from 1.93 to 2.22 mmol/l on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. The leukocyte count dropped from 58.8 x 10e9/l to 10.4 x 10e9/l the day after administration of Dexamethasone as a sign of tumour cell lysis. To protect the kidney from failure, fluid intake was increased and corrective treatment was given. Taking account of the intraoperative dose of Dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the treatment protocol. On postoperative day 9 he was discharged from the hospital. The authors associate the event to Dexamethasone treatment. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

TLS after administration of Dexamethasone. In all cases, it remains unclear whether TLS occurred spontaneously, due to concomitantly administered drugs, or due to Dexamethasone therapy. The topic "tumour lysis syndrome associated with Dexamethasone" is already under close surveillance by the MAH. So far, no actions concerning the wording of the Dexamethasone RSI are regarded as necessary. Close surveillance will be continued.

Reporter's comment:

HAMELN, 25.02.2008 DE-HAMELN-20080222-01

Sender diagnosis: not reported

Narrative

An 8-year-old male patient with pre B-cell acute lymphoblastic leukaemia developed tumour lysis after treatment with a single dose of dexamethasone. He presented for implantation of a central venous port catheter for receiving chemotherapy. After institution of routine monitoring, anaesthesia was induced intravenously with propofol and alfentanil. Anaesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anaesthesia, 4 mg dexamethasone intravenously immediately after induction of anaesthesia was administered. For prophylaxis of postoperative pain, the patient received 500 mg paracetamol intravenously 15 min after induction. A cerebrospinal fluid and bone marrow tap was performed and a port catheter into the right internal jugular vein was implanted. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l) was noted, peaking on the first postoperative day (at 3486 U/l). On the third day, LDH returned to preoperative levels. Serum phosphate increased slightly from 1.93 to 2.22 mmol/l on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. The leukocyte count dropped from $58.8 \times 10^9/l$ to $10.4 \times 10^9/l$ the day after administration of dexamethasone as a sign of tumour cell lysis. To protect the kidney from failure, fluid intake was increased and corrective treatment was given. Taking account of the intraoperative dose of dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the treatment protocol. On postoperative day 9 he was discharged from the hospital.

CAUSALITY ASSESSMENT FOR dexamethasone: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event: no, Other Risk Factors: unknown, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: According to the reporting authors "in rare cases TLS can occur spontaneously or through surgical intervention but usually follows cytotoxic therapy". REACTION ABATE AFTER STOPPING DRUG: Not applicable.

NOVARTIS, 04.03.2008 2008DE01935

Sender diagnosis: not reported

Narrative

Literature case report received on 22 Feb 2008. A young patient with pre B-cell acute lymphocytic leukaemia developed tumour lysis after treatment with a single dose of dexamethasone. The patient was also suffering from hepatosplenomegaly (liver 2cm and spleen 5cm below the costal margin). He presented for implantation of a central venous port catheter for receiving chemotherapy. After institution of routine monitoring, anaesthesia was induced intravenously with propofol and alfentanil. Anaesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anaesthesia, 4 mg dexamethasone intravenously immediately after induction of anaesthesia was administered. For prophylaxis of postoperative pain, the patient received 500 mg paracetamol intravenously 15 min after induction. A cerebrospinal fluid and bone marrow tap was performed and a port catheter into the right internal jugular vein was implanted. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l) was noted, peaking on the first postoperative day (at 3486 U/l). On the third day, LDH returned to preoperative levels. Serum phosphate increased slightly from 1.93 to 2.22 mmol/l on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. The leukocyte count dropped from $58.8 \times 10^9/l$ to $10.4 \times 10^9/l$ the day after administration of dexamethasone as a sign of tumour cell lysis. The patient did not receive any other medications before or during anaesthesia, which could provoke tumour lysis. So as to protect the kidney from failure, fluid intake was increased and corrective treatment with allopurinol and furosemide was given. Taking account of the intraoperative dose of dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the treatment protocol. Additional chemotherapy with vincristine and daunorubicin was started on postoperative day 8. Therapy was well tolerated and the patient was discharged from the hospital the following day. The authors associated the event with dexamethasone treatment and suggested to use other antiemetic drugs such as antiserotonergic agents in cancer patients.

Outcome: recovered.

SANOFI-SYNTHELABO, 05.03.2008 DE-SANOFI-SYNTHELABO-A01200802302

Sender diagnosis: not reported

Narrative

Reaction as reported: tumor lysis syndrome
LLT: TUMOR LYSIS SYNDROME / 10045152
PT: TUMOUR LYSIS SYNDROME / 10045170

Narrative:

Initial report: This serious case from Germany was reported in the medical literature abstract (Lit DEXA 01/08).

An 8-year-old male patient with pre B-cell acute lymphoblastic leukemia developed tumor lysis after treatment with a single dose of dexamethasone. He presented for implantation of a central venous port catheter for receiving chemotherapy. After institution of routine monitoring, anaesthesia was induced intravenously with propofol and alfentanil. Anaesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anaesthesia, 4 mg dexamethasone intravenously immediately after induction of anaesthesia was administered. For prophylaxis of postoperative pain, the patient received 500 mg paracetamol intravenously 15 min after induction. A cerebrospinal fluid and bone marrow tap was performed and a port catheter into the right internal jugular vein was implanted. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l) was noted, peaking on the first postoperative day (at 3486 U/l). On the third day, LDH returned to preoperative levels. Serum phosphate increased slightly from 1.93 to 2.22 mmol/l on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. The leukocyte count dropped from $58.8 \times 10^9/l$ to $10.4 \times 10^9/l$ the day after administration of dexamethasone as a sign of tumour cell lysis. To protect the kidney from failure, fluid intake was increased and corrective treatment was given. Taking account of the intraoperative dose of dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the treatment protocol. On postoperative day 9 he was discharged from the hospital. The authors associate the event to dexamethasone treatment and suggest to use other antiemetic drugs such as antiserotonergic agents in cancer patients.

Additional information received on 28-Feb-08:

A copy of the literature article was received.

Merck KGaA, 08.04.2008 DE-Merck KGaA-6041905

Sender diagnosis: not reported

Narrative

Case was received from the literature (Adis reactions database) on 31 Mar 2008.

*

Tumor lysis associated with dexamethasone use in a child with leukemia.

Osthaus-W-A; Linderkamp-C; Bunte-C; Jüttner-B; Sumpelmann-R.

Pediatric-Anesthesia, 01 Mar 2008; Vol. 18:pages 268-270

*

An 8-year-old child with pre-B cell acute lymphoblastic leukaemia developed tumor lysis syndrome following administration of dexamethasone for postoperative nausea and vomiting.

The boy, who also displayed hepatomegaly, underwent a 2 hour procedure to implant a central venous port catheter prior to receiving treatment with antineoplastics. Anaesthesia was induced using alfentanil and propofol; it was then maintained with desflurane and remifentanyl. He was administered a single dose of IV dexamethasone 4 mg immediately after anaesthetic induction, and paracetamol (acetaminophen) approximately 15 minutes later. After the procedure he was transferred to the post-anaesthetic care unit. That evening (approximately 12 hours after the procedure), he was noted to have a four-fold increase in lactate dehydrogenase (LDH; 3220 U/L). In addition, his serum phosphorus had increased slightly from 1.93 to 2.22 mmol/L. The next day his leucocyte count dropped from 58.8 to 10.4 times $10^9/l$, and on this day and the next, his serum creatinine level doubled from baseline. During this time he did not experience any clinical symptoms.

The boy's serum phosphorus returned to normal 2 days after the procedure. After peaking on the first postoperative day at 3486 his LDH level returned to pre-operative levels 2 days later. He was administered a reduced dose of prednisone. Vincristine and daunorubicin were initiated on the eighth postoperative day, and were well tolerated. He was discharged home on the ninth day.

Author Comment: (T)he dexamethasone dose given to prevent (postoperative nausea and vomiting) seems to be too high for the patient presented here, because tumor lysis was provoked.

Reporter's comment:

Generics [UK] Ltd, 10.04.2008 DE-MYLANLABS-2008S1005229

Sender diagnosis: not reported

Narrative

Osthaus WA, Linderkamp C, Bunte C, Juttner B, Sumpelmann R. Tumor lysis associated with dexamethasone use in a child with leukemia. *Pediatr-Anesth* 2008; 18(3) 268-270.

An 8-year-old child with pre-B cell acute lymphoblastic leukaemia developed tumour lysis syndrome following administration of dexamethasone for postoperative nausea and vomiting. The boy, who also displayed hepatomegaly, underwent a 2 hour procedure to implant a central venous port catheter prior to receiving treatment with antineoplastics. Anaesthesia was induced using alfentanil and propofol; it was then maintained with desflurane and remifentanyl. He was administered a single dose of IV dexamethasone 4mg immediately after anaesthetic induction, and paracetamol [acetaminophen] approximately 15 minutes later. After the procedure he was transferred to the post-anaesthetic care unit. That evening

(approximately 12 hours after the procedure), he was noted to have a four-fold increase in lactate dehydrogenase (LDH; 3220 U/L). In addition, his serum phosphorus had increased slightly from 1.93 to 2.22 mmol/L. The next day his leucocyte count dropped from 58.8 to 10.4 x 10⁹ /L, and on this day and the next, his serum creatinine level doubled from baseline. During this time he did not experience any clinical symptoms. The boy's serum phosphorus returned to normal 2 days after the procedure. After peaking on the first postoperative day at 3486 U/L, his LDH level returned to pre-operative levels 2 days later. He was administered a reduced dose of prednisone. Vincristine and daunorubicin were initiated on the eighth postoperative day, and were well tolerated. He was discharged home on the ninth day.

Merck KGaA, 15.04.2008 DE-Merck KGaA-6041905

Sender diagnosis: not reported

Narrative

Case was received from the literature (Adis reactions database) on 31 Mar 2008.

*

Tumor lysis associated with dexamethasone use in a child with leukemia.

Osthaus-W-A; Linderkamp-C; Bünte-C; Jüttner-B; Sümpelmann-R.

Pediatric-Anesthesia, 01 Mar 2008; Vol. 18:pages 268-270

*

An 8-year-old child with pre-B cell acute lymphoblastic leukaemia developed tumor lysis syndrome following administration of dexamethasone for postoperative nausea and vomiting.

The boy, who also displayed hepatomegaly, underwent a 2 hour procedure to implant a central venous port catheter prior to receiving treatment with antineoplastics. Anaesthesia was induced using alfentanil and propofol; it was then maintained with desflurane and remifentanyl. He was administered a single dose of IV dexamethasone 4 mg immediately after anaesthetic induction, and paracetamol (acetaminophen) approximately 15 minutes later. After the procedure he was transferred to the post-anaesthetic care unit. That evening (approximately 12 hours after the procedure), he was noted to have a four-fold increase in lactate dehydrogenase (LDH; 3220 U/L). In addition, his serum phosphorus had increased slightly from 1.93 to 2.22 mmol/L. The next day his leucocyte count dropped from 58.8 to 10.4 times 10⁹/L, and on this day and the next, his serum creatinine level doubled from baseline. During this time he did not experience any clinical symptoms.

The boy's serum phosphorus returned to normal 2 days after the procedure. After peaking on the first postoperative day at 3486 his LDH level returned to pre-operative levels 2 days later. He was administered a reduced dose of prednisone. Vincristine and daunorubicin were initiated on the eighth postoperative day, and were well tolerated. He was discharged home on the ninth day.

Author Comment: (T)he dexamethasone dose given to prevent (postoperative nausea and vomiting) seems to be too high for the patient presented here, because tumor lysis was provoked.

Follow-up from full text: The child was suffering from hepatosplenomegaly before the induction of anaesthesia and administration of dexamethasone. Therefore hepatomegaly is not an ADR.

Merck KGaA, 30.04.2008 DE-Merck KGaA-6041905

Sender diagnosis: not reported

Narrative

Case was received from the literature (Adis reactions database) on 31 Mar 2008.

*

Tumor lysis associated with dexamethasone use in a child with leukemia.

Osthaus-W-A; Linderkamp-C; Bünte-C; Jüttner-B; Sümpelmann-R.

Pediatric-Anesthesia, 01 Mar 2008; Vol. 18:pages 268-270

*

Follow-up was received from the literature on 23-Apr-2008:

Abstract was replaced by fulltext in the event description.

*

Summary

Postoperative nausea and vomiting (PONV) occurs in every third patient undergoing general anesthesia without PONV prophylaxis. Antiemetic prophylaxis with dexamethasone is commonly used in patients at moderate risk. We present a case in which PONV prophylaxis with a single dose of dexamethasone led to tumor lysis in a patient with acute leukemia. In case of a cancer patient at moderate risk for PONV, the anaesthesiologist should contact the oncologist first, or use other antiemetic drugs such as antiserotonergic agents for PONV prophylaxis.

Introduction

Postoperative nausea and vomiting (PONV) occurs in every third patient undergoing general anesthesia without PONV prophylaxis. Patients often suffer more from PONV than from postoperative pain. Therefore, prevention of PONV improves satisfaction among patients likely to experience

PONV. A number of different antiemetic drugs [e. g. metoclopramide, droperidol, odansetron, dexamethasone, propofol, ginger root] or interventions / techniques (e. g. total intravenous anesthesia, omitting nitrous oxide or antagonism of neuromuscular blockade, acupuncture, transcutaneous electroneurostimulation (TENS)) for preventing or treating PONV have been investigated. Dexamethasone is a well known drug with antiemetic properties which is routinely used to prevent and treat PONV. The cytotoxic properties of steroids through the induction of growth arrest and apoptosis in treated lymphocytes are often not recognized. We present a case in which PONV prophylaxis with a single dose of dexamethasone led to tumor lysis in a child with pre B-cell acute lymphoblastic leukaemia (pre B-ALL).

Case report

An 8-year-old boy (weight 27 kg, height 138 cm) with newly diagnosed pre B-ALL, with 58.8×10^9 sup(9) 1 sup(-1) leukocytes (72 % leukemic cells) presented for implantation of a central venous port catheter for receiving chemotherapy. He was also suffering from hepatosplenomegaly (liver 2 cm and spleen 5 cm below the costal margin). After institution of routine monitoring, anesthesia was induced intravenously with propofol and alfentanil. The patient was intubated orotracheally with a cuffed tube and ventilated with 30% oxygen in air. The ventilatory demand was adjusted to reach an endtidal carbon dioxide of 4.6 kPa (35 mmHg). Anesthesia was maintained with a combination of 6-7% desflurane and a continuous infusion of remifentanyl. Because of nausea and vomiting following previous anesthesia, we administered 4 mg of dexamethasone intravenously immediately after induction of anesthesia. For prophylaxis of postoperative pain, the patient received 500 mg of paracetamol intravenously 15 min after induction. The patient did not receive any other medications before or during anesthesia, which could provoke tumor lysis. First, the pediatric oncologists performed a cerebrospinal fluid and bone marrow tap, and then the pediatric surgeons implanted a port catheter into the right internal jugular vein. The entire procedure took 2 h. Shortly thereafter, the boy was extubated and transferred to the PACU. He stayed there for further 40 min until consciousness was completely recovered; the PACU stay was uneventful and the patient did not complain of pain or nausea. The same evening in routinely performed measurements a fourfold increase in lactate dehydrogenase (LDH) (3220 U/l sup(-1)) was noted, peaking on the first postoperative day (at 3486 U/l sup(-1)). On the third day, LDH returned to preoperative levels. Serum phosphorus increased slightly from 1.93 to 2.22 mmol/l sup(-1) on the same evening, and returned to normal on the second postoperative day. For the following two days creatinine values increased twofold from baseline. Serum levels of potassium and uric acid remained unchanged. The leukocyte count dropped from 58.8 to 10.400×10^9 sup(9) 1 sup(-1) the day after admission of dexamethasone as a sign of tumor cell lysis. During this period the patient did not show any clinical symptoms such as cardiac arrhythmias or seizures. To protect the kidney from failure, fluid intake was increased to 3000 ml/m sup(-2), allopurinol was given and furosemide was administered, starting the day before surgery. Taking account of the intraoperative dose of dexamethasone, the patient received a reduced initial dose of prednisone as recommended in the ALL BFM 2000 protocol (7 mg once instead of 10 mg twice). Additional chemotherapy (vincristine and daunorubicin) was started on postoperative day 8. It was well tolerated, and the patient was discharged from hospital the following day.

Discussion

Dexamethasone is a commonly used steroid with antiemetic properties, and it is becoming increasingly popular since a growing number of published studies have demonstrated its utility for PONV prophylaxis. Dexamethasone prevents PONV if given close to the beginning of surgery, probably by reducing surgery-induced inflammation. It is approved for PONV prophylaxis even in children at the age of 2 years; the recommended dosage is 0.15 - 0.5 mg/kg sup(-1) intravenously. Many known side effects are associated with long-term use, and are based on increased hormone effects. Especially when administered by the intravenous route, dexamethasone can rarely trigger anaphylaxis, more frequently in patients with known allergic diathesis. No studies have identified complications associated with the administration of a single dose of dexamethasone for PONV prophylaxis.

Dexamethasone is sometimes used for prophylaxis of nausea and vomiting after emetogenic chemotherapy in cancer patients. A recent study has revealed moderate to severe side effects such as insomnia, indigestion/epigastric discomfort, agitation, increased appetite, but a tumor lysis syndrome (TLS) has not been observed in any other of the 60 patients enrolled in the study. This depends on the type of cancer because TLS occurs particularly in patients with lymphoproliferative diseases during initial therapy with potent myelosuppressive drugs.

Tumor lysis syndrome is a known complication in pediatric oncology, especially in children with leukemia and lymphomas suffering from huge tumor mass. It is characterized by metabolic derangements occurring in the setting of rapid tumor destruction caused by the shift of intracellular contents into the extracellular compartment with resulting increase of LDH, hyperuricemia, hyperkalemia, hyperphosphatemia, acidosis and acute renal failure sometimes resulting in the need for dialysis. In rare cases a TLS can occur spontaneously or through a surgical intervention but usually it follows cytotoxic therapy. The only cytotoxic agents used by anesthesiologists are glucocorticoids. Referring to the definition of Cairo and Bishop a laboratory TLS exists when two or more of the serum values (uric acid, potassium, phosphate and calcium) changed more than 25% from baseline. Additionally in our clinic we use LDH as indicator for tumor cell lysis. These characteristics usually becomes manifest 24-48 h after chemotherapy, but there are cases which report a TLS within hours after administration of dexamethasone.

Therapeutic interventions depend on the main symptoms with a focus on renal function and control of hyperkalemia. In life-threatening situations, such as hyperkalemia-induced arrhythmias, emergency hemodialysis should be initiated immediately.

Patients suffering from lymphoproliferative diseases are treated prophylactically with allopurinol or rasburicase and hydration before starting chemotherapy in order to avoid renal failure. In our case this procedure might have prevented a laboratory TLS. Another mechanism to avoid tumor lysis is to begin with a small dose of glucocorticoids. Therefore, the dexamethasone dose given to prevent PONV seems to be too high for the patient presented here, because tumor lysis was provoked.

Another important reason not to use dexamethasone in pediatric oncology for PONV prophylaxis is treatment according to standardized protocols. In some protocols, dexamethasone is contraindicated because only one study drug (e.g. rituximab in patients with lymphoma) is to be evaluated. In other studies, a fixed dose of prednisone has to be given to evaluate the prednisone response in order to subdivide patients into risk groups. In other settings, the combination of low cost and apparent safety makes dexamethasone still an attractive first-line agent for the prophylaxis of PONV. In conclusion, anesthesiologists should exercise particular caution in cancer patients. Frequent measurements of serum electrolyte levels could help to recognize complications early and avoid possible life-threatening situations. Whenever a cancer patient is to receive PONV prophylaxis, other antiemetic drugs such as antiserotonergic agents should be considered instead of dexamethasone.

Follow-up from full text: The child was suffering from hepatosplenomegaly before the induction of anaesthesia and administration of dexamethasone. Therefore hepatomegaly is not an ADR.

Galenpharma GmbH, 02.05.2008 DE-HRB2237KIP-000009

Sender diagnosis: not reported

Narrative

An 8-year-old boy with newly diagnosed pre B-ALL had a surgery for implantation of a central venous port catheter for receiving chemotherapy. To protect the kidney from failure, fluid intake was increased and allopurinol such as furosemide was administered, starting the day before surgery. He was anesthetized routinely (propofol, alfentanil, desflurane, remifentanil, paracetamol) and 4 mg dexamethasone were administered intravenously to prevent postoperative nausea and vomiting (PONV).

While the surgery (duration: 2 h) and the post anesthesia care unit (duration: 40 min) the patient was uneventful and did not complain of pain or nausea.

The same evening in routinely performed measurements a fourfold increase of lactat dehydrogenase (LDH) was noted, peaking on the first postoperative day. On the third day LDH returned to preoperative levels; the leukocyte count dropped from 58.8 to 10.400 x 10 exp9 per liter the day after admission of dexamethasone, which was suggested as a sign of tumor cell lysis. During this period the patient did not show any clinical symptoms (such as cardiac arrhythmias or seizures).

In the following days the patient got his chemotherapy (ALL BFM 2000 protocol; additional chemotherapy (vincristine and daunorubicine) was started on postoperative day 8.).

It was well tolerated and the patient was discharged from the hospital on postoperative day 9.<dbtext>To_Char_NARRATIVE_2</dbtext>

Galenpharma GmbH, 09.05.2008 DE-HRB2237KIP-000009

Sender diagnosis: not reported

Narrative

An 8-year-old boy with newly diagnosed pre B-ALL had a surgery for implantation of a central venous port catheter for receiving chemotherapy. To protect the kidney from failure, fluid intake was increased and allopurinol such as furosemide was administered, starting the day before surgery. He was anesthetized routinely (propofol, alfentanil, desflurane, remifentanil, paracetamol) and 4 mg dexamethasone were administered intravenously to prevent postoperative nausea and vomiting (PONV).

While the surgery (duration: 2 h) and the post anesthesia care unit (duration: 40 min) the patient was uneventful and did not complain of pain or nausea.

The same evening in routinely performed measurements a fourfold increase of lactat dehydrogenase (LDH) was noted, peaking on the first postoperative day. On the third day LDH returned to preoperative levels; the leukocyte count dropped from 58.8 to 10.400 x 10 exp9 per liter the day after admission of dexamethasone, which was suggested as a sign of tumor cell lysis. During this period the patient did not show any clinical symptoms (such as cardiac arrhythmias or seizures).

In the following days the patient got his chemotherapy (ALL BFM 2000 protocol; additional chemotherapy (vincristine and daunorubicine) was started on postoperative day 8.).

It was well tolerated and the patient was discharged from the hospital on postoperative day 9.<dbtext>To_Char_NARRATIVE_2</dbtext>

Die aufgetretene UAW wird somit als ~~et~~not listed~~et~~ und als medizinischer Zustand von groÄYer Bedeutung als ~~et~~schwerwiegend~~et~~ eingestuft.

Beurteilung im Hinblick auf den wissenschaftlichen Erkenntnisstand (ggf. MaßÄYnahmen):

Die zytolytische, insbesondere lymphozytolytische pharmakodynamische Wirkung von Corticoiden ist in der Medizin allgemein bekannt und wird sogar therapeutisch genutzt, z.B. bei der Behandlung akuter lymphoblastischer LeukÄYmien. Der durch Corticoide induzierte Zellzerfall (Apoptose) kann dabei so massiv werden, dass ein Tumorlysesyndrom mit Fieber und ÄYbelkeit auftritt und eine lÄYckenlose ÄYberwachung sowie symptomatische Behandlung des Patienten (Elektrolyt- und FlÄYssigkeitssubstitution) erforderlich wird, d.h. es handelt sich um einen potentiell lebensbedrohenden Zustand.

Fazit:

Aufgrund der in der Medizin allgemein bekannten ZusammenhÄYnge sind keine weiteren MaßÄYnahmen vorgesehen, insbesondere kann ein entsprechender Hinweis in der Fach- und Gebrauchsinformation fÄYr diese spezielle Anwendung von Corticoid-Arzneimitteln, die aufgrund der Voraussetzung einer bekannten LeukÄYmieerkrankung onkologischen Fachzentren vorbehalten ist,

Reporter's comment:

Hormosan Pharma GmbH, 19.05.2008 DE-HORMOSAN PHARMA GMBH-951L

Sender diagnosis: not reported

Narrative

A case of an 8-year-old male patient with tumour lysis syndrome was reported. The patient had a history of pre B-ALL with 72% leukemic cells and

hepatosplenomegaly. He presented for implantation of a central venous port catheter for chemotherapy. The patient was intubated orotracheally and ventilated with 30% oxygen. He received propofol, alfentanil, desflurane (6-7%), paracetamol (500 mg) and remifentanil. Because of nausea and vomiting following previous anaesthesia, the patient also received 4 mg of dexamethasone. Cerebrospinal fluid and bone marrow taps were performed and a port catheter was implanted into the right internal jugular vein. The procedures took 2 hours. On the evening of the day of the operation, the patient's lactate dehydrogenase (LDH) increased to 3220 U/L, his phosphate levels increased from 1.93 mmol/L to 2.22 mmol/L. On the first postoperative day his LDH peaked at 3486 U/L. Creatinine values increased twofold from baseline. The leukocyte count dropped from 58.800/µl to 10.400/µl. This was interpreted as a sign of tumour lysis. The patient showed no signs of cardiac arrhythmias or seizures. To protect the kidneys, the patient had received IV fluids, allopurinol and furosemide since the day before surgery. The patient recovered. On postoperative day 8, chemotherapy was started (vincristine, prednisone and daunorubicin), in which the initial prednisone dose was reduced in response to his previous tumour lysis syndrome under dexamethasone. The chemotherapy was well tolerated.

Reporter's comment:

Serious unexpected reaction. Causality assessment: possible

Hormosan Pharma GmbH, 20.05.2008 DE-HORMOSAN PHARMA GMBH-951L

Sender diagnosis: not reported

Narrative

A case of an 8-year-old male patient with tumour lysis syndrome was reported. The patient had a history of pre B-ALL with 72% leukemic cells and hepatosplenomegaly. He presented for implantation of a central venous port catheter for chemotherapy. The patient was intubated orotracheally and ventilated with 30% oxygen. He received propofol, alfentanil, desflurane (6-7%), paracetamol (500 mg) and remifentanil. Because of nausea and vomiting following previous anaesthesia, the patient also received 4 mg of dexamethasone. Cerebrospinal fluid and bone marrow taps were performed and a port catheter was implanted into the right internal jugular vein. The procedures took 2 hours. On the evening of the day of the operation, the patient's lactate dehydrogenase (LDH) increased to 3220 U/L, his phosphate levels increased from 1.93 mmol/L to 2.22 mmol/L. On the first postoperative day his LDH peaked at 3486 U/L. Creatinine values increased twofold from baseline. The leukocyte count dropped from 58.800/µl to 10.400/µl. This was interpreted as a sign of tumour lysis. The patient showed no signs of cardiac arrhythmias or seizures. To protect the kidneys, the patient had received IV fluids, allopurinol and furosemide since the day before surgery. The patient recovered. On postoperative day 8, chemotherapy was started (vincristine, prednisone and daunorubicin), in which the initial prednisone dose was reduced in response to his previous tumour lysis syndrome under dexamethasone. The chemotherapy was well tolerated.

Hormosan Pharma GmbH, 20.05.2008 DE-HORMOSAN PHARMA GMBH-951L

Sender diagnosis: not reported

Narrative

A case of an 8-year-old male patient with tumour lysis syndrome was reported. The patient had a history of pre B-ALL with 72% leukemic cells and hepatosplenomegaly. He presented for implantation of a central venous port catheter for chemotherapy. The patient was intubated orotracheally and ventilated with 30% oxygen. He received propofol, alfentanil, desflurane (6-7%), paracetamol (500 mg) and remifentanil. Because of nausea and vomiting following previous anaesthesia, the patient also received 4 mg of dexamethasone. Cerebrospinal fluid and bone marrow taps were performed and a port catheter was implanted into the right internal jugular vein. The procedures took 2 hours. On the evening of the day of the operation, the patient's lactate dehydrogenase (LDH) increased to 3220 U/L, his phosphate levels increased from 1.93 mmol/L to 2.22 mmol/L. On the first postoperative day his LDH peaked at 3486 U/L. Creatinine values increased twofold from baseline. The leukocyte count dropped from 58.800/µl to 10.400/µl. This was interpreted as a sign of tumour lysis. The patient showed no signs of cardiac arrhythmias or seizures. To protect the kidneys, the patient had received IV fluids, allopurinol and furosemide since the day before surgery. The patient recovered. On postoperative day 8, chemotherapy was started (vincristine, prednisone and daunorubicin), in which the initial prednisone dose was reduced in response to his previous tumour lysis syndrome under dexamethasone. The chemotherapy was well tolerated.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08022443

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19710506	Age:	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting					Unknown
2 Overdose					Unknown
3 Drug abuse					Unknown
4 Suicide attempt					Unknown
5 Slow response to stimuli					Unknown
6 Hepatic insufficiency					Unknown
7 Somnolence					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALPRAZOLAM	Oral	0.5 Mg milligram(s)/	-	
alprazolam		Tablet	-	
2 ass	Oral	500 Mg milligram(s)/	-	
acetylsalicylic acid		Tablet	-	
3 grippostad c	Oral	/	-	
caffeine; chlorphenamine maleate; paracetamol		Tablet	-	
4 LOPERAMID	Oral	2 Mg milligram(s)/	-	
loperamide hydrochloride		Tablet	-	
5 NOVAMINSULFON	Oral	500 Mg milligram(s)/	-	
metamizole		Tablet	-	
6 PARACETAMOL 500	Oral	500 Mg milligram(s)/	-	
paracetamol		Tablet	-	
7 Silomat Dragees gegen Reizhusten	Oral	40 Mg milligram(s)/	-	
clobutinol hydrochloride		Coated tablet	-	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 25.02.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01081DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 36-year-old female patient took 1x21 dragees Silomat Dragees gegen Reizhusten (clobutinol-HCL), 1x10 tablets alprazolam, 1x10 tablets ASS (acetylsalicylic acid),

1x20 tablets Grippostad C (paracetamol, ascorbic acid, coffein, chlorphenaminmaleat), 1x30 tablets Loperamid (loperamid-HCL), 1x10 tablets Novaminsulfon (metamizol-sodium 1H2O) and 1x80 tablets paracetamol 500 in suicidal intention and hospitalisation was recommended. Suicide attempt, drug abuse and overdose with the mild symptoms vomiting substance and decelerated were reported. Therapy before consultation was ACC schema. Stomach protection and symptomatic therapy were recommended.

Therapy is unknown. Outcome was reported as unknown.

Lab data: blood level of paracetamol: 213mg/l

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

PILZDEYN

Case assessment created by PL on 22 Feb 2008

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 27.03.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01081DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 36-year-old female patient took 1x21 dragees Silomat Dragees gegen Reizhusten (clobutinol-HCL), 1x10 tablets alprazolam, 1x10 tablets ASS (acetylsalicylic acid),

1x20 tablets Grippostad C (paracetamol, ascorbic acid, coffein, chlorphenaminmaleat), 1x30 tablets Loperamid (loperamid-HCl), 1x10 tablets Novaminsulfon (metamizol-sodium 1H₂O) and 1x80 tablets paracetamol 500 in suicidal intention and was hospitalised. Suicide attempt, drug abuse and overdose with the severe symptoms vomiting substance, somnolence and hepatic insufficiency and mild decelerated were reported. Therapy before consultation was ACC schema.

Patient was treated with coal and ACC scheme.

The patient's outcome is unknown.

Lab data: blood level of paracetamol: 213mg/l

Boehringer Ingelheim GmbH, Germany, 28.03.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-01081DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 36-year-old female patient took 1x21 dragees Silomat Dragees gegen Reizhusten (clobutinol-HCL), 1x10 tablets alprazolam, 1x10 tablets ASS (acetylsalicylic acid),

1x20 tablets Grippostad C (paracetamol, ascorbic acid, coffein, chlorphenaminmaleat), 1x30 tablets Loperamid (loperamid-HCl), 1x10 tablets Novaminsulfon (metamizol-sodium 1H₂O) and 1x80 tablets paracetamol 500 in suicidal intention and was hospitalised. Suicide attempt, drug abuse and overdose with the severe symptoms vomiting substance, somnolence and hepatic insufficiency and mild decelerated were reported. Therapy before consultation was ACC schema.

Patient was treated with coal and ACC scheme.

The patient's outcome is unknown.

Lab data: blood level of paracetamol: 213mg/l

Hepatic insufficiency was not further specified but might be rather due to the paracetamol overdose.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

PILZDEYN

Case assessment created by PL on 27 Mar 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08050915

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	200804			28 Month	Unknown
2 Transaminase NOS increased	200804			28 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 hypericum perforatum	Oral	/	200803-	
hypericum perforatum			-	
2 Paracetamol	Oral	/	200803-	
paracetamol			-	
3 TREVILOR RETARD	Oral	75 Mg milligram(s)/Day	200511-	
venlafaxine hydrochloride		Prolonged-release capsule	-	

Drugs (concomitant)

4 ENALAPRIL	Oral	/	-	
enalapril			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma GT	200804	948	IU/L	

Narratives (nur bei elektronischem Eingang)

Wyeth-Pharma GmbH, 01.05.2008 DE-WYE-G01421708

Sender diagnosis: not reported

Narrative

This case was considered medically important.

Information was received from a healthcare professional regarding a female patient who received Trevilor Retard (venlafaxine hydrochloride capsule, extended release) therapy and experienced a severe increase of her gamma GT value and an increase of transaminases.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

Indication for Trevilor Retard was not provided. Therapy began in Nov-2005 and continued. Dose regimen was 75 mg 1 time per day (oral).

Additional suspect medication included Hypericum Perforatum and Acetaminophen.

CONCOMITANT THERAPY:

Concomitant therapy included Enalapril.

EVENT DETAILS:

Since her last medical check-up (5-6 month prior to the report) the patient's gamma GT value severely increased (gamma-glutamyltransferase increased). In Apr-2008 it was found to be 948 IU/L. Further increased transaminases (transaminases increased) were detected in Apr-2008. Three weeks before this diagnosis the patient started to take acetaminophen and hypericum perforatum extract which were planned to be discontinued temporarily. Trevilor Retard therapy continued. The outcome was unknown.

TEST RESULTS:

Transaminases (results: increased) and gamma-glutamyltransferase (results: 948 IU/L) were done in Apr-2008.

Birte Kruse

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08052712

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Unknown cause of death
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					fatal
2 Hepatic encephalopathy					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Hadem J,Stiefel P,Bahr M,Tillmann H,Rifai K,Klempnauer J,et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol 2008 Mar;6:3:339-345.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 06.05.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-02564GD

Sender diagnosis: not reported

Narrative

Cumulative literature report:

A retrospective study identified 210 patients with acute liver failure (ALF) treated at an intensive care unit between 1996 and 2005. Of those, 102 patients fulfilled the diagnostic criteria of ALF as initially published by Trey and Davidson as well as those recently defined by the American Association for the Study of Liver Diseases (ie, hepatic encephalopathy, acute-onset increase of INR >1.5, and absence of signs of chronic liver disease in clinical and ultrasound examination).

All parameters except the maximum encephalopathy grade were documented at admission to intensive care unit (ICU). Outcome end points included liver transplantation, death, and survival without transplantation for at least 8 weeks after admission at ICU. Hepatic encephalopathy was graded on a standard scale of I-IV as described previously.

Of the 102 patients with acute liver failure, 72 were women. The median age of the group was 38 years (range, 16-74 years).

16 patients had ALF due to acetaminophen ingestions (age range 20 - 52 years, 9 females, maximum encephalography grade II). 8-week survival without transplantation was 12 of 16 patients and liver transplantation was performed in 12% in this group. 3 patients in this group died.

Case assessment created by DP on 05 May 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055792

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001969

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

Toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors. Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055793

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001971

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors. Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055794

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001972

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055795

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001976

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-08055796

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes Yes

Patient

Initials: Sex: Female Birth: Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification: Physician observestudytype:
Literature: HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001973

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055797

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001974

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055798

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001977

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055799

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001979

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055800

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001978

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055801

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001981

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055802

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001980

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055803

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001982

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

we consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055804

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001983

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a male patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055748

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001975

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors. Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055805

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J ET AL. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001970

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

We consider these cases as serious and probably related to the drug substance paracetamol. The adverse event of ALF is expected because the liver toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors.

Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055806

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	HADEM J, STIEFEL P, BAHR MJ, TILLMANN HL, RIFAI K, KLEMPNAUER J et al. PROGNOSTIC IMPLICATIONS OF LACTATE, BILIRUBIN, AND ETIOLOGY IN GERMAN PATIENTS WITH ACUTE LIVER FAILURE. CLIN GASTROENTEROL HEPATOL. 2008 MAR;6(3): 339-45	

Narratives (nur bei elektronischem Eingang)

Dr. Mann Pharma/Bausch & Lomb Incorporated, 13.05.2008 DE-BAUSCH-2008BL001914

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a female patient experienced acute hepatic failure after Paracetamol ingestion. The patient was inpatient hospitalized. No further information is available.

In this article 16 cases (9 female and 7 male patients) of acute hepatic failure after Paracetamol ingestion are mentioned. All other available data are not assignable to the single cases. The median age of these patients is 37 years. Two of the 16 patients received transplant, three of the 16 patients died. The following characteristics of the 16 patients at admission are medians: maximum encephalopathy grade: II, Prothrombin time 24 %, WBC 11.6 ul, Hemoglobin 13.0 g/dl, coagulation factor V 19 %, Sodium 138 mmol/l, Creatinine 87 umol/l, ALT 3497 U/L, AST 2474 U/L, Alkaline phosphatase 179 U/L, Bilirubin 58 umol/l, Ammonia 83 umol/l, Lactate 3.2 mmol/l, C-reactive protein 38 mg/l, MELD score 27, KCC fulfilled: 4 (25 %), BILE score: 1.9, BILE score >=6.9: 3 (19 %).

Source: Hadem J, Stiefel P, Bahr MJ, Tillmann HL, Rifai K, Klempnauer J et al. Prognostic implications of lactate, bilirubin, and etiology in German patients with acute liver failure. Clin Gastroenterol Hepatol. 2008 Mar;6(3): 339-45

Toxicity of paracetamol is the most well-known side effect of paracetamol in case of overdose or combination with other risk factors like chronic alcohol consumption, concomitant medications. The risk of liver toxicity and the consequences is explicitly described in the Summary of Product Characteristics (SPC) of our paracetamol-containing product (Vivimed N gegen Fieber und Kopfschmerzen). In addition it is stated that the maximum daily dose of 4g paracetamol should not be exceeded or should be reduced in case of other risk factors. Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08055749

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose			9 Day		recovered/resolved
2 Intentional overdose					recovered/resolved
3 Acute liver failure					recovering/resolving
4 Drug toxicity due to intentional overdose					recovered/resolved
5 Cell death			9 Day		Unknown
6 International normalized ratio increased			9 Day		Unknown
7 Bilirubin increased			9 Day		Unknown
8 Liver function test abnormal			9 Day		Unknown
9 Aspartate aminotransferase increased			9 Day		Unknown
10 Hepatomegaly			9 Day		Unknown
11 Alanine aminotransferase increased			9 Day		Unknown
12 Jaundice			9 Day		Unknown
13 Fatigue			9 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol paracetamol	Oral	30 G gram(s)/Day	- -	Intentional overdose

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Jaundice			Unknown	
Fatigue			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A, et al. Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure. Liver Int 2008; 28: 713-6	
Qualification:	Physician	observestudytype:
Literature:	Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A. Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure. Liver International 2008;28:713-716	
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ALT increased		> 9000	U/L	
Alanine aminotransferase		175	U/L	
Aspartate aminotransferase		103	U/L	
Conjugated bilirubin level increased		2.9	mg/dL	
Bilirubin		0.3	mg/dL	

	DE-BFARM-08055749	
International normalized ratio	1.9	NA
International normalized ratio	0.93	NA
INR increased	1.9	NA
Ultrasound scan	slight hepatomegaly	
Immunology test	12 050	U/L
Immunology test	190	U/L

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 14.05.2008 DE-GlaxoSmithKline-B0520024A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of intentional overdose in a 18-year-old male patient who received Paracetamol (acetaminophen - AAP) (brand name/trade name unknown) for an unknown drug indication.

On an unknown date, the patient took an intentional overdose of Paracetamol (oral) at 30 gram(s). At an unknown time after starting Paracetamol, the patient experienced acute hepatic failure indicated by elevated alanine aminotransferase (ALT) levels, greater than 9,000 U/L, bilirubin: 2.9 mg/dL and international normalized ratio (INR) of 1.9 on presentation. Clinically, the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy. On admission, patient had high levels of M65 immunoreactivity (12,050 U/L; 30 × higher than normal) and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death but a low rate of apoptosis. Over the next 2 days M65 and M30 immunoreactivity rapidly returned to normal levels while the liver function test (LFT) levels decreased with a latency of 2 days. The patient was treated with acetylcysteine and Vitamin K. The patient improved quickly and lab values returned to normal, detected by the coagulatory studies. The patient was discharged after 9 days with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L. At the time of reporting, the events were resolved.

The author's considered the events were related to treatment with Paracetamol. The author's commented that "the paracetamol-induced liver injury in our first case is mostly because of necrosis."

Boehringer Ingelheim GmbH, Germany, 20.05.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-02875GD

Sender diagnosis: not reported

Narrative

Patient 1 is an 18-year-old male, who was admitted with acute liver failure after intended intoxication with approximately 30 g of acetaminophen. Alanine aminotransferase (ALT) levels by that time exceeded 9000 U/L, with bilirubin levels of 2.9 mg/dL and an international normalized ratio (INR) of 1.9. Clinically, the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy.

By the time of admission, high levels of M65 immunoreactivity (12 050 U/L; 30 x higher than normal) correlated with high ALT and aspartate aminotransferase (AST) levels, and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death but a low rate of apoptosis. M65 and M30 immunoreactivity rapidly returned to normal levels while the liver function test levels decreased with a latency of 2 days. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. By these measures, the patient improved quickly and lab values returned to normal, detected by the coagulatory studies. Thus, the patient was discharged 9 days after admission with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

Bayer Vital GmbH, 21.05.2008 DE-BAYER-200818196GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 08 May 2008 and describes the occurrence of acute liver failure after intended intoxication with approximately 30 g of acetaminophen (AAP) in an 18-year-old male.

Abstract:

Cell death is known to play a crucial role in liver diseases. We compared the clinical courses of two cases of liver failure of different origins correlated with the extent of cell death (apoptosis and/or necrosis), measured in the patient's blood. Patient 1 was admitted with acute liver failure following acetaminophen intoxication. Intravenous therapy with acetylcysteine was performed. The percentage of apoptotic cell death measured by M30 was three times higher than normal and the overall cell death measured by M65 was more than 30 x higher than normal. High levels of aminotransferases were detected, indicating high rates of necrosis. M30 and M65 values rapidly returned to normal levels, while the liver function test (LFT) levels decreased with latency. Patient 2 was admitted with acute-on-chronic liver failure with known liver fibrosis. M30 values were 16 x higher than normal. In contrast, M65 values were lower than in the first case. This time, no movement in M30 levels was seen until the M30 levels rapidly increased indicating the inevitable death of the patient, while LFTs did not change. These results indicate the role of M30 and M65 immunoexpression as markers for functioning liver cell mass, capacity for recovery and therefore as predictive markers in acute liver failure. Irrespective of the initial cause, acute liver failure (ALF) is the final common pathway of severe hepatocyte injury. ALF is one of the most dangerous complications in hepatology, correlating with a high rate of mortality. According to the accepted definition of ALF by O'Grady, seemingly healthy individuals may suddenly develop severe liver dysfunction with coagulopathy. The onset of hepatic encephalopathy is observed within 8 weeks of the initial symptoms. However, in case of ALF or insufficiency, patients with unknown, but pre-existing liver disease cannot currently be clinically distinguished from those without prior liver damage. Hence, establishing a measure for distinguishing patients with and without pre-existing liver disease would appear to be very useful as a prognostic tool for deciding which patients are likely to survive on supportive measures alone, as opposed to the necessity of receiving a liver graft. Therefore, there is an increasing demand for clinical predictors of the patients' outcomes, as liver donors are rare and the optimal time for listing the patient is often hard to determine. The measurement of cell death by apoptosis and/or necrosis as a predictive marker for the clinical course of ALF might be an emerging way to ease this process of evaluation.

Case report:

Patient 1 is an 18-year-old male, who was admitted with ALF after intended intoxication with approximately 30 g of acetaminophen. Alanine aminotransferase (ALT) levels by that time exceeded 9000 U/L, with bilirubin levels of 2.9 mg/dL and an international normalized ratio (INR) of 1.9. Clinically, the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy. By the time of admission, high levels of M65 immunoreactivity (12050 U/L; 30 x higher than normal) correlated with high ALT and aspartate aminotransferase (AST) levels, and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death but a low rate of apoptosis. M65 and M30 immunoreactivity rapidly returned to normal levels while the liver function test (LFT) levels decreased with a latency of 2 days. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. By these measures, the patient improved quickly and lab values returned to normal, detected by the coagulatory studies. Thus, the patient was discharged 9 days after admission with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L).

Discussion:

Acute liver failure is a devastating clinical condition with a high rate of mortality. Even today, the only treatment option is an emergency liver transplantation, although some patients with ALF may have the possibility for spontaneous recovery. Although several clinical and laboratory criteria (e.g. King's College criteria) for urgent liver transplantation (LTx) have been developed, it still remains a clinical challenge to identify those with the capacity to survive without LTx. In patient 1, apoptosis caused by AAP-induced liver injury most probably did not account for a significant loss of hepatic parenchyma. One can speculate that in fatal cases with massive liver damage and without any capacity to survive, the circulus vitiosus process orchestrated by non-parenchymal cell (e.g. Kupffer cells, hepatic stellate cells) activation with cytokine production leads to the self-ignition of round after round of cell death by apoptosis. Therefore, the low percentage of apoptosis in the overall rate of free CK18 indicates that the AAP-induced liver injury in our first case is mostly because of necrosis. Interestingly, low rates of hepatocyte apoptosis and high M65 levels were associated with strikingly good regenerative capacity and a better prognosis.

Boehringer Ingelheim GmbH, Germany, 21.05.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-02875GD

Sender diagnosis: not reported

Narrative

Patient 1 is an 18-year-old male, who was admitted with acute liver failure after intended intoxication with approximately 30 g of acetaminophen.

18.03.2013

BfArM - Pharmacovigilance

291 / 640

Alanine aminotransferase (ALT) levels by that time exceeded 9000 U/L, with bilirubin levels of 2.9 mg/dL and an international normalized ratio (INR) of 1.9. Clinically, the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy.

By the time of admission, high levels of M65 immunoreactivity (12 050 U/L; 30 x higher than normal) correlated with high ALT and aspartate aminotransferase (AST) levels, and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death but a low rate of apoptosis. M65 and M30 immunoreactivity rapidly returned to normal levels while the liver function test levels decreased with a latency of 2 days. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. By these measures, the patient improved quickly and lab values returned to normal, detected by the coagulatory studies. Thus, the patient was discharged 9 days after admission with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

Comment on case 2008-DE-02875GD / PARACETAMOL

Acute liver failure and paracetamol intoxication are unlisted for the product in question. Acute liver failure is a known and well described consequence of paracetamol intoxication. No information whether the patient took paracetamol on a regular basis or information on the patient's medical history regarding depression or suicidal ideation was provided. There is no reasonable evidence in favour of a causal association between the reported adverse event(s) and the product in question.

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

PILZDEYN

Case assessment created by PL on 20 May 2008

Reporter's comment:

MENARINIP, 21.05.2008 DE-MEN-12157

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 18-year-old male patient, who was admitted with acute liver failure after intentionally ingesting about 30 g of paracetamol [time to reaction onset not stated]. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9 mg/dL, an INR of 1.9, high levels of M65 immunoreactivity (12050 U/L) and slightly elevated M30 levels (190 U/L), indicating a high rate of cell death, but a low rate of apoptosis. He had jaundice and fatigue. An ultrasound revealed slight hepatomegaly. He received cetylcysteine and vitamin K and he quickly improved and his laboratory values returned to normal. Nine days after admission, he was discharged.

Intake of paracetamol. No measures are deemed necessary.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 21.05.2008 DE-PFLEGER-000058

Sender diagnosis: not reported

Narrative

The patient was admitted with acute liver failure (ALF) after intended intoxication with approximately 30g of acetaminophen. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9mg/dl, an INR of 1.9, high levels of M65 immunoreactivity (12050U/L) and slightly elevated M30 levels (190U/L), indicating a high rate of cell death, but a low rate of apoptosis. He had jaundice and fatigue. An ultrasound revealed slight hepatomegaly. He received acetylcysteine and vitamine K and he quickly improved and his laboratory values returned to normal. Nine days after admission he was discharged with an INR of 0.93, bilirubin of 0.3mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

The reporters main reason for writing this article was to show the difference between liver cell apoptosis and necrosis and if this difference can be

associated with special enzyme levels measured in the blood.

The causal relationship between the intake of paracetamol and the reported adverse reaction is judged by the MAH as probable.

Reporter's comment:

Molimin Arzneimittel GmbH, 21.05.2008 DE-MOLIMIN-000015

Sender diagnosis: not reported

Narrative

The patient was admitted with acute liver failure (ALF) after intended intoxication with approximately 30g of acetaminophen. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9mg/dl, an INR of 1.9, high levels of M65 immunoreactivity (12050U/L) and slightly elevated M30 levels (190U/L), indicating a high rate of cell death, but a low rate of apoptosis. He had jaundice and fatigue. An ultrasound revealed slight hepatomegaly. He received acetylcysteine and vitamine K and he quickly improved and his laboratory values returned to normal. Nine days after admission he was discharged with an INR of 0.93, bilirubin of 0.3mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

The reporters main reason for writing this article was to show the difference between liver cell apoptosis and necrosis and if this difference can be associated with special enzyme levels measured in the blood.

The causal relationship between the intake of paracetamol and the reported adverse reaction is judged by the MAH as probable.

Reporter's comment:

Bristol-Myers Squibb Company, 23.05.2008 DE-Bristol-Myers Squibb Company-14195648

Sender diagnosis: not reported

Narrative

This case was derived from a scientific literature and described about an 18-year-old male patient who had an intentional overdose of acetaminophen and was hospitalised for medically significant acute liver failure. The patient had an intended intoxication with approximately 30 g of acetaminophen and experienced acute liver failure. At the time of admission, alanine aminotransferase (ALT) levels had exceeded 9000 U/L, with bilirubin levels of 2.9 mg/dL and an international normalized ratio (INR) of 1.9. Clinically, the patient was jaundiced and fatigued, but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy. By the time of admission, high levels of M65 immunoreactivity (12050 U/L; 30 times higher than normal) correlated with high ALT and aspartate aminotransferase (AST) levels, and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death, but a low rate of apoptosis. M65 and M30 immunoreactivity, used for assessing liver cell death, rapidly returned to normal levels while the liver function test (LFT) levels decreased with a latency of 2 days. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. By these measures, the patient improved quickly and lab values returned to normal, detected by the coagulatory studies. The patient was discharged 9 days after admission with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

Author comment: In patient 1, apoptosis caused by AAP-induced liver injury most probably did not account for a significant loss of hepatic parenchyma. One can speculate that in fatal cases with massive liver damage and without any capacity to survive, the circulus vitiosus process orchestrated by non-parenchymal cell (e.g. Kupffer cells, hepatic stellate cells) activation with cytokine production leads to the self-ignition of round after round of cell death by apoptosis. Therefore, the low percentage of apoptosis in the overall rate of free CK18 indicates that the AAP-induced liver injury in our first case is mostly because of necrosis. Interestingly, low rates of hepatocyte apoptosis and high M65 levels were associated with strikingly good regenerative capacity and a better prognosis.

Literary Citation: Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A. Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure. Liver International 2008 May 1;28:713-6.

SANOFI-SYNTHELABO, 23.05.2008 DE-SANOFI-SYNTHELABO-A01200805471

Sender diagnosis: not reported

Narrative

Reaction as reported: acute liver failure (ALT exceeded 9,000 U/L, bilirubin at 2.9 mg/dL, INR of 1.9, jaundice, fatigue) in a context of intentional overdose with 30 g of paracetamol

LLT: ACUTE LIVER FAILURE / 10049844

PT: ACUTE HEPATIC FAILURE / 10000804

Reaction as reported: acute liver failure (ALT exceeded 9,000 U/L, bilirubin at 2.9 mg/dL, INR of 1.9, jaundice, fatigue) in a context of intentional overdose with 30 g of paracetamol

LLT: INTENTIONAL OVERDOSE / 10022523

PT: INTENTIONAL OVERDOSE / 10022523

Narrative:

Case received from a literature article (full-text) on 13-May-2008 and from our local affiliate (Lit PCM 01/08) on 15-May-2008, processed together.

Case reports

In this report, we present the clinical courses of two patients with liver failures.

Patient 1 is an 18-year-old male, who was admitted with ALF after intended intoxication with approximately 30 g of acetaminophen. Alanine aminotransferase (ALT) levels by that time exceeded 9,000 U/L, with bilirubin levels of 2.9 mg/dL and an international normalized ratio (INR) of 1.9. Clinically, the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Abdominal ultrasound revealed a slight hepatomegaly without signs of chronic hepatopathy.

By the time of admission, high levels of M65 immunoreactivity (12,050 U/L; 30x higher than normal) correlated with high ALT and aspartate aminotransferase (AST) levels, and only slightly elevated levels of M30 (190 U/L), indicating a high rate of overall cell death but a low rate of apoptosis. M65 and M30 immunoreactivity rapidly returned to normal levels while the liver function test (LFT) levels decreased with a latency of 2 days. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. By these measures, the patient improved quickly and lab values returned to normal, detected by the coagulatory studies. Thus, the patient was discharged 9 days after admission with an INR of 0.93, bilirubin of 0.3 mg/dL, AST of 103 U/L and ALT levels of 175 U/L.

In sum, we described two patients with liver failure: one ALF with restitutio ad integrum and another fatal course of AOC liver failure. In the beginning, both patients had high serum levels of free CK18 as an indicator of dying cells. In the first case, the overall cell death (M65) and M30 rapidly decreased to normal levels, correlating with the clinical course. In the second patient, free CK18 levels were 16x higher at the beginning with a four-fold increase shortly before the death of the patient.

Krewel Meuselbach GmbH, 29.05.2008 DE-KREWEL-000702

Sender diagnosis: not reported

Narrative

An 18-year-old man was admitted with acute liver failure after intentionally ingesting about 30g of paracetamol [time to reaction onset not stated]. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9 mg/dL, an INR of 1.9, high levels of M65 immunoreactivity (12 050 U/L) and slightly elevated M30 levels (190 U/L), indicating a high rate of cell death, but a low rate of apoptosis. He had jaundice and fatigue. An ultrasound revealed slight hepatomegaly. He received acetylcysteine and vitamin K and he quickly improved and his laboratory values returned to normal. Nine days after admission, he was discharged.

This German literature case report describes an acute liver failure after intended intoxication with paracetamol. Acute liver failure is not expressis verbis listed in the SPC of our paracetamol containing medicinal product Captin Zäpfchen 125/250/500 mg, and paracetamol and metoclopramide containing medicinal product Migräne Neuridal, but necrosis of liver cells and hepatocellular insufficiency after overdose are listed in section overdose of the SPCs. Further measures are not necessary.

Reporter's comment:STADA Arzneimittel AG, 30.05.2008 DE-STADA-010675

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Reactions Weekly: Volume (1202) 17 May 2008 p 18
HMG-CoA reductase inhibitors/paracetamol: Liver failure: 2 case reports

Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A. Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure. Liver International 28: 713-716, No. 5, May 2008 - Germany

Summary:

One patient developed acute liver failure following ingestion of a paracetamol [acetaminophen] overdose and another patient experienced acute deterioration of acute-on-chronic liver failure associated with a HMG-CoA reductase inhibitors and subsequently died [specific drug, dosage, duration of therapy to reaction onset not stated].

An 18-year-old man was admitted with acute liver failure after intentionally ingesting about 30 g of paracetamol [time to reaction onset not stated]. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9 mg/dL, an INR of 1.9, high levels of M65 immunoreactivity (12 050 U/L) and slightly elevated M30 levels (190 U/L), indicating a high rate of cell death, but a low rate of apoptosis. He had jaundice and fatigue. An ultrasound revealed slight hepatomegaly. He received acetylcysteine and vitamin K and he quickly improved and his laboratory values returned to normal. Nine days after admission, he was discharged.

Patient 2: No Stada drug.<dbtext>To_Char_NARRATIVE_2</dbtext>

BETA, 03.06.2008 DE-BETA-8202/08L

Sender diagnosis: not reported

Narrative

This is a report from literature. An 18-year-old male experienced acute liver failure after an intentional overdose of approximately 30 g paracetamol. The patient was admitted with jaundice and fatigue without any significant encephalopathy or significant bleeding. Abdominal ultrasound showed mild hepatomegaly without signs of chronic hepatomegaly. ALT peaked at more than 9000 U/l and AST was approximately 1000 U/l; bilirubin was measured at 2.9 mg/dl and international normalised ratio (INR) at 1.9. The patient's condition improved rapidly on corrective therapy. On discharge after nine days the INR level was 0.93, bilirubin 0.3 mg/dl, AST 103 U/l and ALT 175 U/l.

ASSESSMENT FOR PARACETAMOL: Serious/ intentional overdose. Not applicable/ Acute liver failure. unexpected/ MAIN measures to reduce drug risk. Increased monitoring

Reporter's comment:

ratiopharm GmbH, 03.06.2008 DE-RATIOPH-2008R054187

Sender diagnosis: not reported

Narrative

An 18-year-old male experienced acute liver failure after an intentional overdose of approximately 30 g paracetamol. The patient was admitted with jaundice and fatigue without any significant encephalopathy or significant bleeding. Abdominal ultrasound showed mild hepatomegaly without signs of chronic hepatomegaly. ALT peaked at more than 9000 U/l and AST was approximately 1000 U/l; bilirubin was measured at 2.9 mg/dl and international normalised ratio (INR) at 1.9. The patient's condition improved rapidly on corrective therapy. On discharge after nine days the INR level was 0.93, bilirubin 0.3 mg/dl, AST 103 U/l and ALT 175 U/l. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: Yes, Pregnancy / Lactation tracking: No

Reporter's comment:

AbZ Pharma GmbH, 03.06.2008 DE-ABZ-2008A054209

Sender diagnosis: not reported

Narrative

An 18-year-old male experienced acute liver failure after an intentional overdose of approximately 30 g paracetamol. The patient was admitted with jaundice and fatigue without any significant encephalopathy or significant bleeding. Abdominal ultrasound showed mild hepatomegaly without signs of chronic hepatomegaly. ALT peaked at more than 9000 U/l and AST was approximately 1000 U/l; bilirubin was measured at 2.9 mg/dl and international normalised ratio (INR) at 1.9. The patient's condition improved rapidly on corrective therapy. On discharge after nine days the INR level was 0.93, bilirubin 0.3 mg/dl, AST 103 U/l and ALT 175 U/l. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: Yes, Pregnancy / Lactation tracking: No

Reporter's comment:

Temmler Pharma GmbH & Co. KG, 17.07.2008 DE-TEMMLER06-000117

Sender diagnosis: not reported

Narrative

An 18-year-old male experienced acute liver failure after an intentional overdose of approximately 30 g paracetamol. The patient was admitted with jaundice and fatigue without any significant encephalopathy or significant bleeding. Abdominal ultrasound showed mild hepatomegaly without signs of chronic hepatomegaly. ALT peaked at more than 9000 U/l and AST was approximately 1000 U/l; bilirubin was measured at 2.9 mg/dl and international normalised ratio (INR) at 1.9. The patient's condition improved rapidly on corrective therapy. On discharge after nine days the INR level was 0.93, bilirubin 0.3 mg/dl, AST 103 U/l and ALT 175 U/l.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08058506

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth:	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose					recovering/resolving
2 Liver failure					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL (NGX)	Unknown	/	-	
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ultrasound kidney abnormal			Unknown	slight hepatomegaly

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Liver International; Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A; 28; 2008; 713-716; Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 20.05.2008 DE-SZ09-GXKR2008DE04385

Sender diagnosis: not reported

Narrative

Initial literature case received on 12 May 2008. A younger patient with an unknown medical history was admitted with acute liver failure after intentionally ingesting about 30 g of paracetamol. He had an ALT level of > 9000 U/L, a bilirubin level of 2.9 mg/dL, an INR of 1.9, high levels of M65 immunoreactivity (12 050 U/L) and slightly elevated M30 levels (190 U/L), indicating a high rate of cell death, but a low rate of apoptosis. He presented with jaundice and fatigue. An ultrasound revealed slight hepatomegaly. The patient received acetylcysteine and vitamin K and he quickly improved and his laboratory values returned to normal. Nine days after admission, he was discharged.

Serious literature report (prophylactically) already noted in the Core Data Sheet information.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08061220

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug intoxication					Unknown
2 Acute liver failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Liver International, (May 2008) Vol. 28, No. 5, pp. 713-716.	

Narratives (nur bei elektronischem Eingang)

Fair Med Healthcare GmbH, 27.05.2008 DE-Fairmed-20080526001

Sender diagnosis: not reported

Narrative

Report from literature: A physician reported about a patient who was admitted hospital with acute liver failure following acetaminophen intoxication. Intravenous therapy with acetylcysteine was performed. The percentage of apoptotic cell death measured by M30 was three times higher than normal and the overall cell death measured by M65 was more than 30 x higher than normal. High levels of aminotransferases were detected, indicating high rates of necrosis. M30 and M65 values rapidly returned to normal levels, while the liver function test (LFT) levels decreased with latency.

No further information provided.

No information of drug brand name provided.

BASED ON BfArM ADVICE FROM MAY 2008.

The risk benefit ratio of Paracetamol Fair Med Tabletten is not changed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08065877

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19500408	Age:	weight (kg): 80	BMI_int: 31.64	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN Infusionsloesung paracetamol	Intravenous (not otherwis	4 G gram(s)/	20080531-20080602	Analgesia
			3 Day	

Drugs (concomitant)

2 dipidolor piritramide	Intravenous (not otherwis	30 Mg milligram(s)/	20080530-	Analgesia
			-	
3 zienam cilastatin sodium; imipenem	Intravenous (not otherwis	1.5 G gram(s)/	20080530-	Peritonitis suppurative
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastroenteritis			Unknown	
Ileus			No	
Liver fatty change			Yes	
Intestinal perforation			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
LDH	20080603	3800	NA	
SGOT	20080603	2604	NA	
SGPT	20080603	1430	NA	
Gamma GT	20080603	64	NA	
Alkaline phosphatase	20080603	171	NA	
Bilirubin direct	20080603	4.1	NA	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08064173

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19610915	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombocyte count decreased					Unknown
2 Liver failure					Unknown
3 Coagulation time abnormal					Unknown
4 Liver enzyme abnormal					Unknown
5 Coffee ground vomiting					Unknown
6 Icterus					Unknown
7 Overdose					Unknown
8 Medication error					Unknown
9 Stomach discomfort					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 pain killer	Oral	/	-	
paracetamol		Tablet	-	
2 thomapyrin	Oral	/	-	Headache
acetylsalicylic acid; caffeine; paracetamol		Tablet	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 04.06.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-03177DE

Sender diagnosis: not reported

Narrative

Information has been received from the poisoning information centre at the university hospital Mainz.

A 46 -years old male patient took, for the treatment of recurrent headache accidentally up to 25 tablets of Thomapyrin (acetylsalicylic acid, paracetamol) per day, for approximately 2-3 weeks, last 4 days, and a lot of pain killers from USA. Hospitalisation was recommended.

Medication error and overdose with the severe symptoms vomiting coffee ground form, liver values pathological, discomfort upper abdomen, thrombocytes decreased, inchoate liver failure, coagulation values pathological and mild icterus. Symptomatic therapy, ACC schema, silibinin and liver transplantation was recommended.

Laboratory values: Tas:>20.000; Thromboz.<1; Quick 25%; Bili 4,2, Krea 1,4, CRP 45, yGT 246

Remarks: since four days pain in upper adomen, since two days mild sclera icterus, yesterday vomiting coffee ground, therefore effected hospitalisation, original tablets will be provided.

Therapy is unknown. Outcome was reported as unknown.
 the available evidence on an ongoing basis. no change in the company core safety information or other measures are recommended at this time.

DUEMPELM

Case assessment created by DP on 03 Jun 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08066542

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 00058 Year	weight (kg): 00089	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	00000010 G gram(s)/	-	
paracetamol	Intravenous infusion		00002 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Postoperative ileus			Unknown	postoperative state after ileus with perforation and proliferation on the liver.

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	
Coagulation time		: Low	NA	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 09.06.2008 DE-Bristol-Myers Squibb Company-14216881

Sender diagnosis: not reported

Narrative

Serious criteria: Important Medical Event

A physician reported via BMS Germany (local file number 2008060504) and via Poison Control Center Mainz (PCCM) (local file number AZ2008011723) that a 58-year-old female patient experienced liver failure with increased liver values and decreased coagulation values while on therapy with acetaminophen 10 mg/ml infusion solution 100ml. Four days ago, the patient received intravenous acetaminophen 10 grams for over 2.5 days. The patient received treatment for the events with acetylcysteine scheme and she was transferred for liver transplantation. At the time of the report, the outcome was unknown.

The patient's medical history included postoperative state after ileus with perforation and proliferation on the liver.

BMS Medical Evaluation Comment:

This 58-year-old patient experienced liver failure with increased liver values and decreased coagulation values while on therapy with intravenous acetaminophen received in the dose of 10 grams over 2.5 days. The patient was treated with acetylcysteine scheme and transferred for liver transplantation. Significant medical history included postoperative state after ileus with perforation and proliferation on the liver that could have contributed to the event. Considering the temporal relationship of the event, a causal role of acetaminophen could not be excluded.

Comments: Hepatic failure is listed for PERFALGAN only in context of overdose. Additionally, significant medical history included postoperative state after ileus with perforation and proliferation on the liver which might have contributed to the event.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08073350

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Male Birth: 19411209 Age: 66 Year weight (kg): 00054 BMI_int: 19.83 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20080614				recovering/resolving
2 Pneumonia					Unknown
3 Respiratory insufficiency	200806				Unknown
4 Transient psychosis	200806				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AVALOX moxifloxacin hydrochloride		G gram(s)/001 Day	20080613-	Pneumonia
2 PERFALGAN IV paracetamol	Intravenous (not otherwis	00000001 Df dosage form Intravenous infusion	20080611-20080615 5 Day	Postoperative pain
3 PERFALGAN IV paracetamol	Intravenous (not otherwis	00000001 Df dosage form Intravenous infusion	20080611-20080615 5 Day	Analgesia

Drugs (concomitant)

4 atosil promethazine hydrochloride		/	-	Sedation
5 CEFAZOLINE cefazolin sodium	Intravenous (not otherwis	6 G gram(s)/Day	-	Antibiotic therapy
6 CLEXANE enoxaparin	Subcutaneous	00000040 Mg milligram(s)	-	Thrombosis prophylaxis
7 clonidine clonidine		/	-	Sedation
8 dipidolor piritramide		/	-	Pain
9 gentamicin gentamicin		00000320 Mg milligram(s)	20080614-	
10 HALOPERIDOL haloperidol		00000001 Df dosage form	-	
11 metamizole metamizole sodium		00000005 G gram(s)/024	-	Pain
12 midazolam midazolam		/	-	Sedation

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Peripheral occlusive disease			Unknown	
Aortic bi-femoral bypass	20080611		Unknown	Implantation of Aortic bi-femoral bypass
Femoropopliteal artery bypass	20080611		Unknown	Femoro popliteal bypass
Tobacco abuse			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		nicht vorhanden	NA	absent
Bilirubin	200806	14	MG/DL	
Bilirubin	200806	22	MG/DL	
Blood pressure systolic	200806	140	MM HG	
Body temperature	200806	38.8	DEG C	
Thorax X-ray	20080615	nicht vorhanden	NA	absent
Computerised tomogram abdomen	200806	nicht vorhanden	NA	absent

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 25.06.2008 DE-Bristol-Myers Squibb Company-14235162

Sender diagnosis: not reported

Narrative

A physician reported via BMS Germany (Cross reference no: 2008062002) that a 55-year-old male patient experienced life threatening liver failure while on therapy with acetaminophen for analgesia and moxifloxacin hcl for pneumonia. The patient had been in hospital for vascular operation. After surgery patient experienced symptomatic transitory psychotic syndromes and then pneumonia which was treated with moxifloxacin hcl and acetaminophen (intravenous, 9g over 5 days) since 12-Jun-2008. The patient had to be ventilated and could be extubated again in-between therapy for pneumonia. Then, patient experienced increase of bilirubin greater than 22 and liver failure. At the time of report patient had to be artificially ventilated and outcome was uncertain. A planned transfer to a different hospital for liver transplantation was refused due to shortage of space. According to the physician a concrete causality was not assessable and a previous hepatic damage had not been demonstrated.

BMS medical evaluation comment:

This patient developed life threatening liver failure while on therapy with acetaminophen and moxifloxacin. Limited information precludes a meaningful assessment of the case.

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: 'Elevation of hepatic transaminases' is listed in the current SPC of PERFALGAN. Besides, 'hepatic cell necrosis', which can lead to a total irreversible necrosis and later hepatocellular insufficiency, is mentioned in the context of overdose, but 'hepatic failure' is not listed. However, limited information precludes a meaningful assessment of the case.

Reporter's comment:

Bristol-Myers Squibb Company, 26.06.2008 DE-Bristol-Myers Squibb Company-14235162

Sender diagnosis: not reported

Narrative

A physician reported via BMS Germany (Cross reference no: 2008062002) that a 55-year-old male patient experienced life threatening liver failure while on therapy with acetaminophen for analgesia and moxifloxacin hcl for pneumonia. The patient had been in hospital for vascular operation. After surgery patient experienced symptomatic transitory psychotic syndromes and then pneumonia which was treated with moxifloxacin hcl and acetaminophen (intravenous, 9g over 5 days) since 12-Jun-2008. The patient had to be ventilated and could be extubated again in-between therapy for pneumonia. Then, patient experienced increase of bilirubin greater than 22 and liver failure. At the time of report patient had to be artificially ventilated and outcome was uncertain. A planned transfer to a different hospital for liver transplantation was refused due to shortage of space. According to the physician a concrete causality was not assessable and a previous hepatic damage had not been demonstrated.

BMS medical evaluation comment:

This patient developed life threatening liver failure while on therapy with acetaminophen and moxifloxacin. Limited information precludes a meaningful assessment of the case.

Supplemental information received from the physician on 20-Jun-2008 added additional events of pneumonia left side, medically significant respiratory insufficiency and non serious events of post operative symptomatic transitory psychotic syndrome. The patient was admitted to hospital on 11-Jun-2008 to the surgery intensive care ward for a vascular surgery with initially hypertensive blood pressure values. After a bolus administration of urapidil 12.5mg and continuous intravenous infusion of 2mg nitrate/hour systolic blood pressure was about 140mmHg. Both legs were warm and well supplied with blood. Pain therapy was performed with fractionated piritramide, dipyrone and acetaminophen infusion. Antibiotic therapy due to plastic prosthesis was done with cefazoline 2g thrice daily intravenously and thrombosis prophylaxis was done with enoxaparin sodium 40mg subcutaneous injection. On the morning of the first day post-operative the patient experienced a severe post-operative symptomatic transitory psychotic syndrome. Due to sedation with promethazine hydrochloride, clonidine and midazolam the patient became calm but additionally he experienced respiratory insufficiency with the need for intubation and controlled ventilation for 2 days. After extubation the patient was further confused but could be guided under therapy with haloperidol up to 3 ampoules per day. Due to temperature increase up to 38.8 degree Celsius, increasing C-reactive protein and leukocytes the antibiotics was switched to moxifloxacin 0.4 g/d on 13-Jun-2008 and on 14-Jun-2008 gentamicin 320mg single dose was added, with plasma level determination. Since 14-Jun-2008 bilirubin values continuously increased, but other liver values only moderately increased. Sonographic control of the abdomen revealed no congestion in the liver parenchyma or cholecystolithiasis and only little sludge in the gall bladder. The Computed Tomogram of the abdomen revealed no retro-peritoneal hematoma. The patient had purged mucous twice and post-operative 2 red blood cell concentrates were transfused. The thoracic x-ray on 15-Jun-2008 revealed a basal bilateral opacity. A control on 16-Jun-2008 revealed an infiltration at left basal region and moderate signs of congestion. A further control on 19-Jun-2008 revealed no more signs of congestion but further left basal infiltration. Due to a suspected diagnosis of a drug/toxic-induced hepatitis the antibiotics was switched from moxifloxacin to ceftriaxone 2g/d. The acetaminophen infusions were stopped on 16-Jun-2008 (in total the patient had received acetaminophen 9g within 5 days, at one of these days he had received 3 infusions). Due to the suspected diagnosis of drug-induced liver function disorder, acetylcysteine 300mg/kg /24 hour was administered. Since 17-Jun-2008 the patient again was controlled ventilated due to a new respiratory insufficiency. There was an increase of bilirubin from 14 mg/dl to 22 mg/dl within 24 hours on 19-Jun-2008. The values of the hepatitis serology had still been outstanding at the time of this report.

The patient's medical history included peripheral occlusive disease stage III, implantation of an aorto-bi-femoral and femoral popliteal bypass on the right side on 11-Jun-2008 and nicotine abuse.

BMS medical evaluation comment :

This patient who was admitted for vascular surgery experienced the reported events after receiving therapy with multiple medications including acetaminophen. Although therapy with multiple sedatives could have contributed to respiratory insufficiency, considering the temporal relationship a causal role of acetaminophen to the reported events could not be excluded . Pneumonia and transient psychotic syndrome could be a post-operative complications.

Bristol-Myers Squibb Company, 10.07.2008 DE-Bristol-Myers Squibb Company-14235162

Sender diagnosis: not reported

Narrative

A physician reported via BMS Germany (Cross reference no: 2008062002) that a 55-year-old male patient experienced life threatening liver failure while on therapy with intravenous acetaminophen for analgesia and moxifloxacin hcl for pneumonia. The patient had been in hospital for vascular operation. After surgery patient experienced symptomatic transitory psychotic syndromes and then pneumonia which was treated with moxifloxacin hcl and acetaminophen (intravenous, 9g over 5 days) since 12-Jun-2008. The patient had to be ventilated and could be extubated again in-between therapy for pneumonia. Then, patient experienced increase of bilirubin greater than 22 and liver failure. At the time of report patient had to be artificially ventilated and outcome was uncertain. A planned transfer to a different hospital for liver transplantation was refused due to shortage of space. According to the physician a concrete causality was not assessable and a previous hepatic damage had not been demonstrated.

BMS medical evaluation comment:

This patient developed life threatening liver failure while on therapy with acetaminophen and moxifloxacin. Limited information precludes a meaningful assessment of the case.

Supplemental information received from the physician on 20-Jun-2008 added additional events of pneumonia left side, medically significant respiratory insufficiency and non serious events of post operative symptomatic transitory psychotic syndrome. The patient was admitted to hospital on 11-Jun-2008 to the surgery intensive care ward for a vascular surgery with initially hypertensive blood pressure values. After a bolus administration of urapidil 12.5mg and continuous intravenous infusion of 2mg nitrate/hour systolic blood pressure was about 140mmHg. Both legs were warm and well supplied with blood. Pain therapy was performed with fractionated piritramide, dipyrone and acetaminophen infusion. Antibiotic therapy due to plastic prosthesis was done with cefazoline 2g thrice daily intravenously and thrombosis prophylaxis was done with enoxaparin sodium 40mg subcutaneous injection. On the morning of the first day post-operative the patient experienced a severe post-operative symptomatic transitory psychotic syndrome. Due to sedation with promethazine hydrochloride, clonidine and midazolam the patient became calm but additionally he experienced respiratory insufficiency with the need for intubation and controlled ventilation for 2 days. After extubation the patient was further confused but could be guided under therapy with haloperidol up to 3 ampoules per day. Due to temperature increase up to 38.8 degree Celsius, increasing C-reactive protein and leukocytes the antibiotics was switched to moxifloxacin 0.4 g/d on 13-Jun-2008 and on 14-Jun-2008 gentamicin 320mg single dose was added, with plasma level determination. Since 14-Jun-2008 bilirubin values continuously increased, but other liver values only moderately increased. Sonographic control of the abdomen revealed no congestion in the liver parenchyma or cholecystolithiasis and only little sludge in the gall bladder. The Computed Tomogram of the abdomen revealed no retro-peritoneal hematoma. The patient had purged mucous twice and post-operative 2 red blood cell concentrates were transfused. The thoracic x-ray on 15-Jun-2008 revealed a basal bilateral opacity. A control on 16-Jun-2008 revealed an infiltration at left basal region and moderate signs of congestion. A further control on 19-Jun-2008 revealed no more signs of congestion but further left basal infiltration. Due to a suspected diagnosis of a drug/toxic-induced hepatitis the antibiotics was switched from moxifloxacin to ceftriaxone 2g/d. The

acetaminophen infusions were stopped on 16-Jun-2008 (in total the patient had received acetaminophen 9g within 5 days, at one of these days he had received 3 infusions). Due to the suspected diagnosis of drug-induced liver function disorder, acetylcysteine 300mg/kg /24 hour was administered. Since 17-Jun-2008 the patient again was controlled ventilated due to a new respiratory insufficiency. There was an increase of bilirubin from 14 mg/dl to 22 mg/dl within 24 hours on 19-Jun-2008. The values of the hepatitis serology had still been outstanding at the time of this report.

The patient's medical history included peripheral occlusive disease stage III, implantation of an aorto-bi-femoral and femoral popliteal bypass on the right side on 11-Jun-2008 and nicotine abuse.

BMS medical evaluation comment :

This patient who was admitted for vascular surgery experienced the reported events after receiving therapy with multiple medications including acetaminophen. Although therapy with multiple sedatives could have contributed to respiratory insufficiency, considering the temporal relationship a causal role of acetaminophen to the reported events could not be excluded . Pneumonia and transient psychotic syndrome could be a post-operative complications.

Supplemental information received on 07-Jul-2008 from the physician via BMS Germany noted that the patient had experienced the beginning of liver failure (cholestatic hepatosis) since 14-Jun-2008 while under therapy with intravenous acetaminophen, short-time infusion (daily dose 1-3 g) from 11-Jun-2008 until 15-Jun-2008 for post operative pain therapy after vessel operation. The patient had elevated glutamate oxalate transaminase (GOT), glutamate pyruvate transaminase (GPT), lactate dehydrogenase (LDH) , gamma-glutamyl transferase (gamma-GT), alkaline phosphatase and lowered cholinesterase (CHE) values. The event was ongoing (at the time of the report 22nd day). However, at the time of the report the patient was on the way to recovery, but he still had to be ventilated. Furthermore, the physician stated that the patient had not received acetaminophen previously and he was not re-exposed. The physician considered the event liver failure as possibly related to acetaminophen therapy.

Bristol-Myers Squibb Company, 05.08.2008 DE-Bristol-Myers Squibb Company-14235162

Sender diagnosis: not reported

Narrative

A physician reported via BMS Germany (Cross reference no: 2008062002) that a 55-year-old male patient experienced life threatening liver failure while on therapy with intravenous acetaminophen for analgesia and moxifloxacin hcl for pneumonia. The patient had been in hospital for vascular operation. After surgery patient experienced symptomatic transitory psychotic syndromes and then pneumonia which was treated with moxifloxacin hcl and acetaminophen (intravenous, 9g over 5 days) since 12-Jun-2008. The patient had to be ventilated and could be extubated again in-between therapy for pneumonia. Then, patient experienced increase of bilirubin greater than 22 and liver failure. At the time of report patient had to be artificially ventilated and outcome was uncertain. A planned transfer to a different hospital for liver transplantation was refused due to shortage of space. According to the physician a concrete causality was not assessable and a previous hepatic damage had not been demonstrated.

BMS medical evaluation comment:

This patient developed life threatening liver failure while on therapy with acetaminophen and moxifloxacin. Limited information precludes a meaningful assessment of the case.

Supplemental information received from the physician on 20-Jun-2008 added additional events of pneumonia left side, medically significant respiratory insufficiency and non serious events of post operative symptomatic transitory psychotic syndrome. The patient was admitted to hospital on 11-Jun-2008 to the surgery intensive care ward for a vascular surgery with initially hypertensive blood pressure values. After a bolus administration of urapidil 12.5mg and continuous intravenous infusion of 2mg nitrate/hour systolic blood pressure was about 140mmHg. Both legs were warm and well supplied with blood. Pain therapy was performed with fractionated piritramide, dipyrone and acetaminophen infusion. Antibiotic therapy due to plastic prosthesis was done with cefazoline 2g thrice daily intravenously and thrombosis prophylaxis was done with enoxaparin sodium 40mg subcutaneous injection. On the morning of the first day post-operative the patient experienced a severe post-operative symptomatic transitory psychotic syndrome. Due to sedation with promethazine hydrochloride, clonidine and midazolam the patient became calm but additionally he experienced respiratory insufficiency with the need for intubation and controlled ventilation for 2 days. After extubation the patient was further confused but could be guided under therapy with haloperidol up to 3 ampoules per day. Due to temperature increase up to 38.8 degree Celsius, increasing C-reactive protein and leukocytes the antibiotics was switched to moxifloxacin 0.4 g/d on 13-Jun-2008 and on 14-Jun-2008 gentamicin 320mg single dose was added, with plasma level determination. Since 14-Jun-2008 bilirubin values continuously increased, but other liver values only moderately increased. Sonographic control of the abdomen revealed no congestion in the liver parenchyma or cholecystolithiasis and only little sludge in the gall bladder. The Computed Tomogram of the abdomen revealed no retro-peritoneal hematoma. The patient had purged mucous twice and post-operative 2 red blood cell concentrates were transfused. The thoracic x-ray on 15-Jun-2008 revealed a basal bilateral opacity. A control on 16-Jun-2008 revealed an infiltration at left basal region and moderate signs of congestion. A further control on 19-Jun-2008 revealed no more signs of congestion but further left basal infiltration. Due to a suspected diagnosis of a drug/toxic-induced hepatosis the antibiotics was switched from moxifloxacin to ceftriaxone 2g/d. The acetaminophen infusions were stopped on 16-Jun-2008 (in total the patient had received acetaminophen 9g within 5 days, at one of these days he had received 3 infusions). Due to the suspected diagnosis of drug-induced liver function disorder, acetylcysteine 300mg/kg /24 hour was administered. Since 17-Jun-2008 the patient again was controlled ventilated due to a new respiratory insufficiency. There was an increase of bilirubin from 14 mg/dl to 22 mg/dl within 24 hours on 19-Jun-2008. The values of the hepatitis serology had still been outstanding at the time of this report.

The patient's medical history included peripheral occlusive disease stage III, implantation of an aorto-bi-femoral and femoral popliteal bypass on the right side on 11-Jun-2008 and nicotine abuse.

BMS medical evaluation comment :

This patient who was admitted for vascular surgery experienced the reported events after receiving therapy with multiple medications including

acetaminophen. Although therapy with multiple sedatives could have contributed to respiratory insufficiency, considering the temporal relationship a causal role of acetaminophen to the reported events could not be excluded. Pneumonia and transient psychotic syndrome could be a post-operative complications.

Supplemental information received on 07-Jul-2008 from the physician via BMS Germany noted that the patient had experienced the beginning of liver failure (cholestatic hepatitis) since 14-Jun-2008 while under therapy with intravenous acetaminophen, short-time infusion (daily dose 1-3 g) from 11-Jun-2008 until 15-Jun-2008 for post operative pain therapy after vessel operation. The patient had elevated glutamate oxalate transaminase (GOT), glutamate pyruvate transaminase (GPT), lactate dehydrogenase (LDH), gamma-glutamyl transferase (gamma-GT), alkaline phosphatase and lowered cholinesterase (CHE) values. The event was ongoing (at the time of the report 22nd day). However, at the time of the report the patient was on the way to recovery, but he still had to be ventilated. Furthermore, the physician stated that the patient had not received acetaminophen previously and he was not re-exposed. The physician considered the event liver failure as possibly related to acetaminophen therapy.

Supplemental information received from the physician on 01-Aug-2008 noted that the patient need not be ventilated anymore and that he was transferred to the internistic ward.

BMS medical evaluation comment remains the same.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08073645

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19500408	Age: 00058 Year	weight (kg): 00080	BMI_int: 31.64	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fulminant hepatic failure	20080602			3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan IV 100 ML paracetamol	Intravenous (not otherwis	00000010 G gram(s)/ Intravenous infusion	20080531-20080602 00002 Day	Postoperative analgesia

Drugs (concomitant)

2 dipidolor piritramide	Intravenous (not otherwis	00000030 Mg milligram(s)	20080530- -	Analgesia
3 ZIENAM cilastatin sodium; imipenem	Intravenous (not otherwis	00000002 G gram(s)/001	20080530- -	Peritonitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastritis			Unknown	
Diarrhea			Unknown	
Meteorism			Unknown	
Hypokalemia			Unknown	
Gastroenteritis			Unknown	
Esophagitis			Unknown	purulent
Hepatic steatosis			Unknown	80 per cent hitologically
Peritonitis			Unknown	Peritonitis upon mechanical ileus with intestinal perforation
Postoperative ileus			Unknown	postoperative state after ileus with perforation and proliferation on the liver.

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver function tests		: High	NA	
Coagulation time		: Low	NA	
Lactate dehydrogenase	20080603	3800	U/L	
Glutamic-oxaloacetic transferase	20080603	2600	U/L	
Glutamic-pyruvate transaminase	20080603	1400	U/L	
Gamma-glutamyltransferase	20080603	64	U/L	
Investigation NOS	20080603	171	U/L	

Bilirubin	20080603	4.1	MG/DL
Acetaminophen	20080603	16.6	NA
Hemoglobin	20080603	9.9	G/DL
Quick's test	20080603	40	%
Quick's test	20080603	24	%
Partial prothrombin time	20080603	39.3	S
Partial prothrombin time	20080603	47.7	S
Thrombin time	20080603	19.6	S
Thrombin time	20080603	25.7	S
Fibrinogen	20080603	345	MG/DL
Lactate dehydrogenase	20080603	2133	U/L
Lactate dehydrogenase	20080603	3769	U/L
Glutamic-oxaloacetic transferase	20080603	1364	U/L
Glutamic-oxaloacetic transferase	20080603	2604	U/L
Glutamic-pyruvate transaminase	20080603	762	U/L
Glutamic-pyruvate transaminase	20080603	1430	U/L
Gamma-glutamyltransferase	20080603	61	U/L
Gamma-glutamyltransferase	20080603	64	U/L
Urea	20080603	17	MG/DL
Creatinine	20080603	0.75	MG/DL
Bilirubin direct	20080603	2.05	MG/DL
Blood pancreatic amylase	20080603	45	U/L
Lipase	20080603	6	%
Protein total	20080603	40.8	G/L
Computerised tomogram abdomen		nicht vorhanden	NAabsent
Histology NOS		nicht vorhanden	NAabsent
Liver biopsy		nicht vorhanden	NAabsent
Anti-HBs antibody		245.9	NA
Bilirubin total		3.26	MG/DL
Cholinesterase	20080603	4059	NA
Cholinesterase	20080603	4058	NA

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 26.06.2008 DE-Bristol-Myers Squibb Company-14216881

Sender diagnosis: not reported

Narrative

Serious criteria: Important Medical Event

A physician reported via BMS Germany (local file number 2008060504) and via Poison Control Center Mainz (PCCM) (local file number AZ2008011723) that a 58-year-old female patient experienced liver failure with increased liver values and decreased coagulation values while on therapy with acetaminophen 10 mg/ml infusion solution 100ml. Four days ago, the patient received intravenous acetaminophen 10 grams for over 2.5 days. The patient received treatment for the events with acetylcysteine scheme and she was transferred for liver transplantation. At the time of the report, the outcome was unknown.

The patient's medical history included postoperative state after ileus with perforation and proliferation on the liver.

BMS Medical Evaluation Comment:

This 58-year-old patient experienced liver failure with increased liver values and decreased coagulation values while on therapy with intravenous

acetaminophen received in the dose of 10 grams over 2.5 days. The patient was treated with acetylcysteine scheme and transferred for liver transplantation. Significant medical history included postoperative state after ileus with perforation and proliferation on the liver that could have contributed to the event. Considering the temporal relationship of the event, a causal role of acetaminophen could not be excluded.

Supplemental information received from the physician via German Health Authority BfArM on 16-Jun-2008 reported that the patient experienced life threatening fulminant liver failure while under therapy with intravenous acetaminophen, from 31-May-2008 to 02-Jun-2008 with daily dose of 4g for postoperative analgesia. The patient had presented to the hospital with abdominal complaints and already had complained of upper abdominal pains for six months. Infectious parameters were massively increased and also coagulation was already impaired. A computer tomography (CT) scan of abdomen showed free air and long-stretched stenosis in terminal ileum with prestenotic signs of small intestinal ileus. Stabilization of coagulation by administration of 4 FFP (fresh frozen plasma) was done. Then an extended ileocaecal resection with ileoascendostomy and liver sample excision of segment V and III upon mechanical ileus with ileal perforation by a stenosing ileal tumor with purulent 4-quadrant peritonitis were performed on 30-May-2008. Patient recovered well at first. The patient received cilastatine ± imipenem 3 x 0.5g/day as antibiotic. Pain therapy was performed with acetaminophen 4 x 1g/day and piritramide due to known intolerance of various analgesics. The patient received in total 10 x 1g acetaminophen as short time infusion at six hour intervals. In the course infectious parameters decreased further. Patient experienced liver failure on 02-Jun-2008 and the same evening a massively impaired coagulation was found in the laboratory control. A distinct increase of lactate dehydrogenase (LDH) and all other liver enzymes was observed, which tripled until transfer to a different hospital. Upon patient's admission all values had been in normal range. Poison control centre Mainz (PCCM) was consulted upon suspicion of liver failure by acetaminophen administration upon already preexisting steatosis hepatitis. Antidote therapy with high dosed acetylcysteine was started. The patient's laboratory investigations on 03-Jun-2008 included LDH at 3800 U/L (normal range 0 to 246), Serum glutamic oxaloacetic transaminase (GOT) at 2600 U/L (normal range 0 to 31), Serum glutamic pyruvic transaminase (GPT) at 1400 U/L (normal range 0 to 34), gamma-glutamyl transpeptidase (GGT) at 64 U/L (normal range 0 to 37), alkaline phosphatase (AP) at 171 U/L (normal range 30-120), bilirubin at 4.1 mg/dL (normal range 0.00 to 0.25), acetaminophen level in serum was at 16.6 (normal range 10-20), hemoglobin (Hb) was at 9.9 g/dL (normal range 12-15), quick test was 40% and 24% (normal range 70 to 100) at 05:17 and 15:15 hrs, partial thromboplastin time (PTT) at 39.3 S (seconds) and 47.7 S at 05:17 and 15:15 hrs (normal range 22.0 to 35.0 S), thrombin time at 05:17 and 15:15 hrs was 19.6 S and 25.7 S (normal range 9.0 to 12.0), fibrinogen was 345 mg/dL (normal range 180 to 350). The laboratory values taken on the same day at 5:17 and 11:58 hours included LDH at 2133 and 3769 U/L, GOT at 1364 and 2604 U/L, GPT at 762 and 1430 U/L, GGT at 61 and 64, urea at 17 mg/dL (normal range 17 to 43), creatinine was 0.75 mg/dL (normal range 0.66 to 1.09), bilirubin direct at 2.05 mg/dL (normal range 0.00 to 0.25), pancreatic amylase was 45 U/L (normal range 8 to 53), lipase was 6 % (normal range 21 to 67) and total protein at 40.8 g/L (normal range 6 to 33), cholinesterase was 4059 and 4058 U/L.

The patient's histology revealed :1. Partial resection of ileum with caecum with severe chronic and florid, focal-shaped purulent abscess-forming, lumen stenosing inflammatory reaction with fibrinous-purulent peritonitis upon perforation of terminal ileum and severe fibrinous-purulent peritonitis of whole ileal and caecal resectate. 12 hyperplastic lymph nodes in co-resected fatty tissue.2. Liver tissue with circumscribed chronic-granulating, calcificating inflammatory reaction and pronounced steatosis of liver cells was observed.3. The liver punch biopsy revealed pronounced steatosis of liver cells, affecting 80 percent of the hepatocytes, besides a portal, non-septal fibrosis and a distinct unspecific mesenchymal inflammation.4. Fatty tissue of omentum with fibrinous-purulent serositis.

The patient received therapy with combination solution of amino acids, glucose, electrolytes, 120 milliliter/ per hour, heparin 5000 international units/50 ml/hour, piritramide 30 mg/250ml 15 ml per hour, acetylcysteine 300 mg intravenously twice daily, pantaprazole 40 mg intravenously, twice daily, cilastatine + imipenem 0.5 g thrice daily and acetylcysteine 12750 mg hourly, 4250 mg/ every 4 hours and 8500 mg/6 every 16 hours on 03-Jun-2008. The patient's anti-HBS was 245.9 and bilirubin total was 3.26 mg/dL (normal range 0.00 to 1.00). The patient was transferred for further therapy and potential liver transplantation to another hospital. The reporter stated that the therapy with acetaminophen was withdrawn. However the event of liver failure was ongoing at the time of the report.

The patient's medical history included peritonitis and "proliferation of the liver" upon mechanical ileus with intestinal perforation, steatosis hepatitis (80 percent histology), gastroenteritis presumably of infective origin, purulent esophagitis, pangastritis, diarrhea, meteorism, hypokalemia and stenosing ileal tumor. The patient was intolerant to metamizole and acetylsalicylic acid and had underwent ileocaecal resection with ileoascendostomy.

Reporter's causality: The physician considered the fulminant liver failure as probably related to acetaminophen.

BMS Medical Evaluation Comment:

This patient with significant medical history experienced fulminant liver failure shortly after postoperative analgesic therapy with intravenous acetaminophen 10 grams over 2.5 days. She had undergone an ileocaecal resection for ileal tumor, and a hepatic biopsy for investigation of a liver condition following prior surgery for perforated ileus. The administration of acetaminophen in this patient with a pre-existing liver condition (pronounced steatotic hepatitis and portal fibrosis) could have contributed to a severe hepatic dysfunction. Hence, a causal role of acetaminophen in the event could not be excluded.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08080095

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug intoxication					Unknown
2 Abnormal liver function tests					
3 Acute liver failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Liver international : official journal of the International Association for the Study of the Liver, (2008 May) Vol. 28, No. 5, pp. 713-6.	

Narratives (nur bei elektronischem Eingang)Fair Med Healthcare GmbH, 11.07.2008 DE-Fairmed-20080707001

Sender diagnosis: not reported

Narrative

Report from literature:

A patient of unknown age and gender was admitted with acute liver failure following paracetamol intoxication (dose unknown). Intravenous therapy with acetylcysteine was performed. The percentage of apoptotic cell death measured by M30 was three times higher than normal and the overall cell death measured by M65 was more than 30 x higher than normal. High levels of aminotransferases were detected, indicating high rates of necrosis. M30 and M65 values rapidly returned to normal levels, while the liver function test (LFT) levels decreased with latency.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08084104

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Poisoning					Unknown
2 Acute liver failure					Unknown
3 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Volkman X, Anstaett M, Hadem J, Stiefel P, Bahr M, Lehner F, et al. Caspase activation is associated with spontaneous recovery from acute liver failure. Hepatology 2008 May;47:5:1624-1633.	

Narratives (nur bei elektronischem Eingang)Boehringer Ingelheim GmbH, Germany, 22.07.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-04169GD

Sender diagnosis: not reported

Narrative

Cumulative literature report:

In this study, the role of apoptosis and caspases was investigated in acute liver failure (ALF) patients using novel biomarkers that allow the detection of caspase activation in serum samples.

Sera from 70 ALF patients (18 men, 52 women; mean age, 43 +/- 1.8 years; range, 16-77 years) were investigated. ALF was defined as acute-onset deterioration of liver function without signs of chronic liver disease resulting in jaundice, decrease in coagulation factors (international normalized ratio >1.5) and development of encephalopathy within 8 weeks (King's College Hospital criteria and model of end-stage liver disease score were applied).

In 24 cases, ALF was caused by toxic liver injury (overdose). ALF patients with toxic liver injury included nine patients with acetaminophen (AAP), 12 patients with other drug or Amanita mushroom poisoning (non-acetaminophen, NAAP), and three patients with mixed poisoning.

In patients with toxic liver injury, spontaneous recovery occurred in 14 patients (including 7 patients of the AAP group).

This case has been upgraded to serious based on BI internal medical judgement.

Case assessment created by DP on 21 Jul 2008

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08089666

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19400323	Age: 68 Year	weight (kg): 00046	BMI_int: 17.31	Age fetus:
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Patientdeath

deathdate: 20080801	deathcause: Multiple organ failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver function tests	20080731			3 Day	fatal
2 Coagulation disorder					fatal
3 Multiple organ failure					fatal
4 Overdose					fatal
5 Vigilance decreased					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CEFOTAXIME cefotaxime sodium		/	-	
2 metronidazole metronidazole		/	-	
3 PANTOPRAZOLE pantoprazole sodium		/	-	
4 PERFALGAN IV paracetamol	Intravenous (not otherwise specified)	00000007 G gram(s)/Day Intravenous infusion	20080729-20080731 3 Day	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adenocarcinoma			Unknown	
NIDDM			Unknown	
COPD			Unknown	
Mitral insufficiency			Unknown	
Hip operation NOS			No	
Radiotherapy			No	
Pain			Unknown	
Alcohol abuse			Unknown	
Sepsis			Unknown	septic constellation at recurrent anastomosis insufficiency with peritonitis.
Resection of rectum			No	
Anastomotic complication			Unknown	
Peritonitis			Unknown	
Surgery			Unknown	adenocarcinoma surgery
Drug allergy			Unknown	novalgin

Primary source

Qualification: Physician	observestudytype:
Literature:	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Alanine aminotransferase	20080730	15	U/L
Alanine aminotransferase	20080731	2384	U/L
Alanine aminotransferase	20080729	83	U/L
Alkaline phosphatase	20080728	278	U/L
Alkaline phosphatase	20080731	198	U/L
Aspartate aminotransferase	20080731	5586	U/L
Aspartate aminotransferase	20080730	15	U/L
Aspartate aminotransferase	20080729	74	U/L

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 06.08.2008 DE-Bristol-Myers Squibb Company-14285480

Sender diagnosis: not reported

Narrative

A physician reported via Drug Poisson Control Center Mainz (PCCM # AZ2008016310) and BMS-Germany (Local file #: 2008073111) that a 68-year-old female patient was hospitalized due to increase of liver values and decrease of coagulation parameters while under therapy with acetaminophen. The patient was also hospitalized due to an overdose with acetaminophen. The hospital physician reported that the patient was iatrogenic administered in total 7 sacs of intravenous acetaminophen (10 milligram per milliliter in 100 milliliter infusion solution, in total 7.000 milligram), within 2 days. The patient suffered from pain which was already pre-existing and experienced increased liver values and decreased coagulation parameters. The Poisson Control Center Mainz recommended determination of plasma level of paracetamol and liver values and in-patient treatment with prolonged ACC scheme, silibinin and if necessary HD haemodialysis. It was reported that plasma level of paracetamol and liver values were measured (values not provided) and that the Quick value was 13. The patient was treated with the ACC scheme, which was prolonged. Furthermore it was reported that the patient had no symptoms and that the outcome of the events is unknown.

The patient's past medical history included pain.

NO, not applicable

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: The occurrence of coagulopathy could be seen in the context of thrombocytopenia which is expected according to the current SPC of PERFALGAN.

Reporter's comment:

Bristol-Myers Squibb Company, 20.08.2008 DE-Bristol-Myers Squibb Company-14285480

Sender diagnosis: not reported

Narrative

A physician reported via Drug Poisson Control Center Mainz (PCCM # AZ2008016310) and BMS-Germany (Local file #: 2008073111) that a 68-year-old female patient was hospitalized due to increase of liver values and decrease of coagulation parameters while under therapy with acetaminophen. The patient was also hospitalized due to an overdose with acetaminophen. The hospital physician reported that the patient was iatrogenic administered in total 7 sacs of intravenous acetaminophen (10 milligram per milliliter in 100 milliliter infusion solution, in total 7.000 milligram), within 2 days. The patient suffered from pain which was already pre-existing and experienced increased liver values and decreased coagulation parameters. The Poisson Control Center Mainz recommended determination of plasma level of paracetamol and liver values and in-patient treatment with prolonged ACC scheme, silibinin and if necessary HD haemodialysis. It was reported that plasma level of paracetamol and liver values were measured (values not provided) and that the Quick value was 13. The patient was treated with the ACC scheme, which was prolonged. Furthermore it was reported that the patient had no symptoms and that the outcome of the events is unknown.

The patient's past medical history included pain.

Supplemental information received on 12-Aug-2008 from the physician noted that the female patient had died due to raised liver values, coagulation disorders, overdose and reduced vigilance. The patient (weight about 40kg) was under reduced vigilance prolonging her hospitalization and

subsequently died after she was initially hospitalized for iatrogenically administered 7 grams of intravenous acetaminophen within 48 hours. The date of intoxication was 31-Jul-2008 and the latency to administration was 48 hours (not 13 hours as previously reported). The symptoms were coagulation disorder, Quick's value low < 6%, elevated liver values with PTT(plasma thromboplastin time) high 150 sec, GOT (glutamate oxalate transaminase) high 5945 U/L, GPT(glutamate pyruvate transaminase) high 2530 U/L, bilirubin 5.5 mg/dL, ammonia 78 micromol/L, reduced vigilance requiring intubation on 01-Aug-2008. The blood level of acetaminophen was measured 38.2 mg/l. The patient was treated in the ICU (intensive care unit) with hemodialysis, the molecular adsorbent recirculating system (MARS) on 01-Aug-2008 and received prolonged ACC scheme with acetylcysteine 100mg/kg over 16 hours starting on 31-Jul-2008. She also was treated with silibinin 250mg 4 times a day since 01-Aug-2008. The Poisson Control Center Mainz (PCCM) rated the symptoms as severe according to the Severity Score.

The patient's medical history included adenocarcinoma surgery.

Reporter's comment:

Bristol-Myers Squibb Company, 29.08.2008 DE-Bristol-Myers Squibb Company-14285480

Sender diagnosis: not reported

Narrative

A physician reported via Drug Poisson Control Center Mainz (PCCM # AZ2008016310) and BMS-Germany (Local file #: 2008073111) that a 68-year-old female patient was hospitalized due to increase of liver values and decrease of coagulation parameters while under therapy with acetaminophen. The patient was also hospitalized due to an overdose with acetaminophen. The hospital physician reported that the patient was iatrogenically administered in total 7 sacs of intravenous acetaminophen (10 milligram per milliliter in 100 milliliter infusion solution, in total 7.000 milligram), within 2 days. The patient suffered from pain which was already pre-existing and experienced increased liver values and decreased coagulation parameters. The Poisson Control Center Mainz recommended determination of plasma level of paracetamol and liver values and in-patient treatment with prolonged ACC scheme, silibinin and if necessary HD haemodialysis. It was reported that plasma level of paracetamol and liver values were measured (values not provided) and that the Quick value was 13. The patient was treated with the ACC scheme, which was prolonged. Furthermore it was reported that the patient had no symptoms and that the outcome of the events is unknown.

The patient's past medical history included pain.

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The patient's medical history included adenocarcinoma surgery.

Supplemental information received from the physician on 27-Aug-2008 stated that this case with fatal outcome had been very distinctive because the female patient did not show a liver-impairment until 3 days after the acetaminophen-infusion and then quickly developed a complete liver failure. Previously, the liver-values had been normal. Furthermore, the physician reported that the patient could possibly also have developed a sepsis which could also have contributed to the fatal outcome. The physician regretted that it was impossible to clarify this matter, because the patient's relatives did not give their consent to an obduction.

Bristol-Myers Squibb Company, 09.09.2008 DE-Bristol-Myers Squibb Company-14285480

Sender diagnosis: not reported

Narrative

A physician reported via Drug Poisson Control Center Mainz (PCCM # AZ2008016310) and BMS-Germany (Local file #: 2008073111) that a 68-year-old female patient was hospitalized due to increase of liver values and decrease of coagulation parameters while under therapy with acetaminophen. The patient was also hospitalized due to an overdose with acetaminophen. The hospital physician reported that the patient was iatrogenically administered in total 7 sacs of intravenous acetaminophen (10 milligram per milliliter in 100 milliliter infusion solution, in total 7.000 milligram), within 2 days. The patient suffered from pain which was already pre-existing and experienced increased liver values and decreased coagulation parameters. The Poisson Control Center Mainz recommended determination of plasma level of paracetamol and liver values and in-patient treatment with

prolonged ACC scheme, silibinin and if necessary HD haemodialysis. It was reported that plasma level of paracetamol and liver values were measured (values not provided) and that the Quick value was 13. The patient was treated with the ACC scheme, which was prolonged. Furthermore it was reported that the patient had no symptoms and that the outcome of the events is unknown.

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The patient's medical history included adenocarcinoma surgery.

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Supplemental information received on 02-Sep-2008 from the hospital physician noted that the patient received acetaminophen for post operative pain from 29-Jul-2008. On 31-Jul-2008, the laboratory values showed signs of the hepatic failure as lowered coagulation values, elevated live enzymes and elevated LDH values followed by cardio respiratory insufficiency, necessity of artificial respiration, increased need of catecholamines and acute renal failure. On 01-Aug-2008, the patient died due to multiple organ failure. According to physician, the medications received by the patient such as pantoprazole, cefotaxime and metronidazole could also have contributed to the events. As of 31-Jul-2008 lab data included sodium-144 mmol/l, potassium-3.9 mmol/l, calcium-1.7 mmol/l, chloride-114 mmol/l, AST- GOT (glutamate oxalate transaminase)-774U/L, ALT- GPT(glutamate pyruvate transaminase)-425 U/L, Alkaline phosphatase-77U/L, GGT- (Gamma-Glutamyl Transpeptidase)-24U/L, LDH (Lactate dehydrogenase)-1192U/L, creatinine-.64mg/dl, urea-31mg/dl, INR (international normalized ratio)-6.4, PTT(Partial thromboplastin time)- 130 sec.

The patient's medical history includes septic constellation at recurrent anastomosis insufficiency with peritonitis and anamnestic alcohol abuse.

Correction to file made on 08-Sep-2008 based on the supplemental information received on 02-Sep-2008 from the hospital physician.

The physician reported that the patient's laboratory values showed the signs of elevated liver enzymes (not elevated live enzymes as previously reported) on 31-Jul-2008. Laboratory data on 28-Jul-2008 included GGT (gamma glutamyl transferase) 118 U/L and alkaline phosphatase-278 U/L (Normal range: 38-126). Laboratory data on 29-Jul-2008, procalcitonine level was 0.54 microgram/L. On 30-Jul-2008, lab data included AST-GOT (glutamate oxalate transaminase)- 15U/L, ALT-GPT (glutamate pyruvate transaminase)- 15 U/L, LDH (Lactate dehydrogenase)- 110 U/L, procalcitonine 6.63 microgram/L and Quick-Wert value was 58%. Laboratory data taken at 04:45 on 31-Jul-2008 included potassium 4.7 mmol/L, AST-GOT: 774 U/L, ALT-GPT: 425 U/L, Alkaline phosphatase-77 U/L, LDH-1192 U/L, procalcitonine 10.81 microgram/L and bilirubin 1.6 mg/dl. Laboratory data taken at 18:48 on 31-Jul-2008, included potassium- 4.2 mmol/l, AST-GOT: 5586 U/L, ALT-GPT: 2384 U/L, Alkaline phosphatase-198 U/L, LDH-9091 U/L, bilirubin 3.9 mg/dl. Quick-Wert value was less than 6% at 04:30 on 31-Jul-2008 and procalcitonine level was 12.82 microgram/L at 13:46 on 31-Jul-2008.

case: DE-BFARM-08103536

Patient

Initials: Sex: Female Birth: 19630913 Age: 44 Year weight (kg): 60 BMI int: 22.58 Age fetus:

Reactions

MedDRA LLT		Begin	End	duration	TtO	outcome
1	Weight increase	20080816	200808	10 Day	34 Day	recovered/resolved
2	Leg oedema	20080816			1 Month	recovered/resolved
3	Edema legs	20080816		10 Day	34 Day	recovered/resolved
4	Transaminases increased	20080816		10 Day	34 Day	recovered/resolved
5	Hypokalemia	20080816		10 Day	34 Day	recovered/resolved
6	Clotting disorder	20080816		10 Day	34 Day	recovered/resolved
7	Headache	20080816		10 Day	34 Day	recovered/resolved
8	Gut pain	20080816		10 Day	34 Day	recovered/resolved
9	Abdominal pain	20080816		10 Day	34 Day	recovered/resolved
10	Fibrin D dimer increased	20080816		10 Day	34 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	1.5 G gram(s)/Day	200808-	Headache
paracetamol		245	-	
2 cimicifuga racemosa root	Oral	2 Df dosage form/Day	20080714-20080817	Climacteric discomfort
		245	35 Day	

Drugs (concomitant)

3	mestinon pyridostigmine bromide	Oral	60 Mg milligram(s)/Day 9	2000- -	Myasthenia gravis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Myasthenia gravis	2000		Yes	
Factor V deficiency			Yes	
Allergy	2006		Yes	Allergic to comfrey.

Primary source

Qualification:	Physician	observestudytype:
Literature:	R. Zimmermann, A. Witte, R.E. Voll, J. Strobel and M. Frieser. Coagulation activation and fluid retention associated with the use of black cohosh: a case study. Climacteric 2009; 1-5, iFirst article	
Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ALT increased	20080817	104	U/l	
AST increased	20080817	66	U/l	
Coagulation factor VIII level increased	20080817	223	%	
Fibrin D dimer	20080817	1304	ng/ml	
Antinuclear antibody positive	20080817	320	U	

Narratives (nur bei elektronischem Eingang)

Sender diagnosis: not reported

Narrative

2 September 2008: The physician sends Schaper & Brümmer in advance by e-mail a report of a 45-years-old female patient, who had taken Remifemin tablets (2 x 1 tablet/day) from 14 July until 17 August 2008 for treating menopausal symptoms. During the last two weeks of the Remifemin intake the patient gained weight (6,5 kgs). On 17 August 2008 the patient has been admitted to emergency room, because deep vein thrombosis or dextrocardia insufficiency were suspected. The patient was examined in the emergency room and was discharged the same day. The following reactions were reported: Weight gain, leg oedema, transaminases increased (AST and ALT), hypokalaemia, coagulation-disorder, headache, and abdominal pain. A sonography showed oedema in the lower legs, however, the veins did not show a thrombosis. All of the serological analysis regarding hepatitis and other infections were negative.

After withdrawal of Remifemin the complaints abated and all pathological values have normalized. From the anamnesis it is known, that the patient has a myasthenia gravis since 2000 and, therefore, she takes Mestinon (60 mg/d). Furthermore, she sporadically takes Paracetamol in case of headaches. In the last few days she took up to 2 x 1 g/day. Two years ago she had an anaphylactic reaction when taking Kytta balsam (comfrey). It is also known, that the patient suffers from a factor V Leiden. Up to now she did not have any thrombo-embolic events. The physician was requested to provide more detailed information regarding this case.

24 October 2008: The physician reported this case to the Arzneimittelkommission der Deutschen Ärzteschaft and to the BfArM. He supposed, that Remifemin has caused the reactions. In particular he finds it very interesting, that the laboratory parameters (D-Dimere) have changed and he assumes, that Remifemin might have caused liver toxicity.

4 August 2009 - 1. Follow-up case report information (from literature - Zimmermann et al, 2009):

A 44-year-old woman presented with a 1-week history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Her past history was significant for myasthenia gravis with predominant involvement of ocular muscles which had been diagnosed 8 years before. A thoracoscopic thymectomy had been performed within the first year. Since then, the patient had taken pyridostigmine at dosages between 60 and 120 mg/day but no immunosuppressive drugs. Additionally, a previous diagnostic procedure had revealed a heterozygous factor V Leiden mutation. However, until today the patient has not experienced a thromboembolic event. Three years before presentation, the patient suffered from an acute anaphylactic reaction caused by a first-time local administration of a comfrey root extract ointment (Extr. Rad. Symphyti; Kytta-Balsam). Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock (Remifemin, Schaper & Brümmer GmbH) at a daily dose of 5 mg. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The patient reported to be a vegetarian and to have a minimal alcohol intake.

Physical examination of the patient revealed bilateral lower leg edema with a 1.5-cm difference in the magnitude of the shank circumference between the left and the right sides. There was a mild right upper quadrant tenderness. Vital signs were blood pressure 139/74 mmHg, pulse rate 71 beats/min, respiratory rate 15 breaths/min, and temperature 37.7 °C.

Laboratory evaluation revealed a white blood cell count of 5.3E+9/l, a hemoglobin level of 123 g/l, and a platelet count of 181E+9/l. Creatine kinase was 83 U/l (reference value <145 U/l). Serum creatinine was 57.5 µmol/dl. Coagulation values were prothrombin time international normalized ratio (INR) 0.98, activated partial thromboplastin time 31.4 s (reference value 26-38 s), and D-dimers of 1304 mg/ml (reference value <500 mg/ml).

Because of the difference between the activation of the coagulation cascade and the activation of the complement system, which can indicate liver injury, however, there was only a transient increase of transaminases, which often can occur in "healthy population". In addition, bilirubin value and other coagulation values- like INR, partial thromboplastin time remained at normal range.

Since many years the patient takes pyridostigmin and has been treated with acetaminophen on occurrence of the clinical symptoms. She also sporadically used paracetamol in a dose of up to 2 grams per day which may explain the slight elevation of transaminases.

There is also only a weak evidence for an autoimmune hepatitis. The positive antinuclear antibodies were only marginally increased and may be associated with the patient's other diseases like myasthenia gravis.

The patient is a vegetarian and has a minimal alcohol intake. The alcohol consumption can explain the pathological finding on transaminases. She also suffers from allergy, as she had an anaphylactic shock after the administration of a comfrey root extract (Kyttä-Balsam) three years before presentation.

Furthermore, the exact indication for Remifemin has not been reported. The patient is 44 years old and might not be in menopause, yet.

In summary, the diagnosis is unclear. It seems that the authors are constructing a diagnosis of "liver damage", where neither the symptoms (headache, abdominal pain, and weight gain) nor the diagnostic findings (liver values slightly increased) are not in line with it. An autoimmune hepatitis is also unlikely. The female might have suffered from a transient thromboembolic event. Only a temporary coincidence with the intake of Remifemin is plausible. Therefore, a causal relationship between the occurrence of the reactions and the intake of the preparation seems to be unlikely.

Sender diagnosis: not reported

Narrative

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Remifemin tablets (2 x 1 tablet/day) from 14 July until 17 August 2008 for treating menopausal symptoms. During the last two weeks of the Remifemin intake the patient gained weight (6,5 kgs). On 17 August 2008 the patient has been admitted to emergency room, because deep vein thrombosis or dextrocardia insufficiency were suspected. The patient was examined in the emergency room and was discharged the same day. The following reactions were reported: Weight gain, leg oedema, transaminases increased (AST and ALT), hypokalaemia, coagulation-disorder, headache, and abdominal pain. A sonography showed oedema in the lower legs, however, the veins did not show a thrombosis. All of the serological analysis regarding hepatitis and other infections were negative.

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24 October 2008: The physician reported this case to the Arzneimittelkommission der Deutschen Ärzteschaft and to the BfArM. He supposed, that Remifemin has caused the reactions. In particular he finds it very interesting, that the laboratory parameters (D-Dimere) have changed and he assumes, that Remifemin might have caused liver toxicity.

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Physical examination of the patient revealed bilateral lower leg edema with a 1.5-cm difference in the magnitude of the shank circumference between the left and the right sides. There was a mild right upper quadrant tenderness. Vital signs were blood pressure 139/74 mmHg, pulse rate 71 beats/min, respiratory rate 15 breaths/min, and temperature 37.7 °C.

Laboratory evaluation revealed a white blood cell count of $5.3 \times 10^9/l$, a hemoglobin level of 123 g/l, and a platelet count of $181 \times 10^9/l$. Creatine kinase was 83 U/l (reference value ≤ 145 U/l). Serum creatinine was $57.5 \mu\text{mol/dl}$. Coagulation values were prothrombin time international normalized ratio (INR) 0.98, activated partial thromboplastin time 31.4 s (reference value 26-38 s), and D-dimers of 1304 mg/ml (reference value ≤ 500 mg/ml).

Because of the difference in driving is a well-known event.

Certainly, coagulation activation can indicate liver injury, however, there was only a transient increase of transaminases, which often can occur in "healthy population". In addition, bilirubin value and other coagulation values- like INR, partial thromboplastin time remained at normal range.

Since many years the patient takes pyridostigmin and has been treated with acetaminophen on occurrence of the clinical symptoms. She also sporadically used paracetamol in a dose of up to 2 grams per day which may explain the slight elevation of transaminases.

There is also only a weak evidence for an autoimmune hepatitis. The positive antinuclear antibodies were only marginally increased and may be associated with the patient's other diseases like myasthenia gravis.

The patient is a vegetarian and has a minimal alcohol intake. The alcohol consumption can explain the pathological finding on transaminases.

She also suffers from allergy, as she had an anaphylactic shock after the administration of a comfrey root extract (Kytt-Balsam) three years before presentation.

Furthermore, the exact indication for Remifemin has not been reported. The patient is 44 years old and might not be in menopause, yet.

In summary, the diagnosis is unclear. It seems that the authors are constructing a diagnosis of "liver damage", where neither the symptoms (headache, abdominal pain, and weight gain) nor the diagnostic findings (liver values slightly increased) are not in line with it. An autoimmune hepatitis is also unlikely. The female might have suffered from a transient thromboembolic event. Only a temporary coincidence with the intake of Remifemin is plausible. Therefore, a causal relationship between the occurrence of the reactions and the intake of the preparation seems to be unlikely.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08109294

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	No			No		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fulminant hepatic failure					Unknown
2 Hepatic encephalopathy					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diphenhydramine		/	-	
diphenhydramine			-	
2 PARACETAMOL		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Leifeld L, Merk P, Schmitz V, Nattermann J, Kalff J, Hirner A, et al. Course and therapy of acute liver failure. Eur J Med Res 2008 Feb 25;13:2:87-91.	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 18.09.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-05419GD

Sender diagnosis: not reported

Narrative

A patient who had taken paracetamol in combination with diphenhydramine experienced fulminant hepatic failure (FHF) accompanied by hepatic encephalopathy (see below for definition).

This case is from an analysis of the follow-up of all patients, who had been treated for FHF (n=35, aged 40 +/- 14, 12 males) or acute hepatitis (n=56) at the University of Bonn between January 1998 and December 2004. FHF was defined by the presence of severe acute liver disease with jaundice and coagulopathy accompanied by a hepatic encephalopathy in a previously healthy person. Time between jaundice and hepatic encephalopathy had to be shorter than 8 weeks. For comparison, patients with acute hepatitis were included into the analysis. These patients had also severe acute hepatitis and imminent FHF but did not fulfill the criteria for FHF.

The case above belonged to a group of 9 patients with FHF caused by amanita or drugs (4 had an indication for liver transplant (LTX); 1 had improvement despite former indication for LTX; LTX was performed in 2; 1-year survival was 7/9).

It was outlined in general that one patient with FHF who had taken paracetamol received treatment with N-acetylcysteine and could be withdrawn from the transplantation list.

A cumulative literature report is also available:

2008-DE-05421GD

reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08109296

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	No			No		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute					Unknown
2 Hepatic encephalopathy					Unknown
3 Fulminant hepatic failure					Unknown
4 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL		/	-	
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Leifeld L, Merk P, Schmitz V, Nattermann J, Kalff J, Hirner A, et al. Course and therapy of acute liver failure. Eur J Med Res 2008 Feb 25;13:2:87-91.	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 18.09.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-05421GD

Sender diagnosis: not reported

Narrative

Cumulative literature report:

An analysis of the follow-up of all patients, who had been treated for fulminant hepatic failure (FHF) (n=35, aged 40 +/- 14, 12 males) or acute hepatitis (n=56) at the University of Bonn between January 1998 and December 2004 was performed. FHF was defined by the presence of severe acute liver disease with jaundice and coagulopathy accompanied by a hepatic encephalopathy in a previously healthy person. Time between jaundice and hepatic encephalopathy had to be shorter than 8 weeks. For comparison, patients with acute hepatitis were included into the analysis. These patients had also severe acute hepatitis and imminent FHF but did not fulfill the criteria for FHF.

2 patients experienced FHF with paracetamol.

Of the 56 patients with severe acute hepatitis, 5 had taken paracetamol.

It was outlined in general that excellent results were obtained in the paracetamol group due to N-acetylcysteine treatment. In 6 patients a diagnosis of paracetamol overdose was made, five patients developed acute hepatitis and 1 patient developed FHF. All of them were treated with N-acetylcysteine and none of 5 patients with severe hepatitis developed fulminant hepatitis and one patient with FHF could be withdrawn from the transplantation list.

A single case (FHF with paracetamol in combination with diphenhydramine) is also available:

2008-DE-05419GD

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08124816

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Male Birth: 19511125 Age: 56 Year weight (kg): 74 BMI_int: 23.62 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20080805	20080818	14 Day	8 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ciprobay, Bayer, 250mg, Tabl ciprofloxacin	Oral	500 Mg milligram(s)/Day 90	20080729-20080807 10 Day	Drug use for unknown indication
2 Paracetamol, NN, 500mg, Tabl paracetamol	Oral	500 Mg milligram(s)/Day 245	20080725-20080805 12 Day	Drug use for unknown indication
3 Remergil Soltab, 15/30 mg, Organon, Tabl mirtazapine	Oral	/Day 171	20080723-20080807 16 Day	Depressive episode

Drugs (concomitant)

4 Enahexal, Hexal, 10 mg, Tabl enalapril	Oral	10 Mg milligram(s)/Day 245	20080723- -	Drug use for unknown indication
5 Fluoxetin, NN, 20 mg, Tabl fluoxetine	Oral	20 Mg milligram(s)/Day	20080723-20080729 7 Day	Depressive episode
6 Ibuhexal, Hexal, 400 mg, Tabl ibuprofen	Oral	400 Mg milligram(s)/ 82	20080726-20080815 21 Day	Lumbago
7 Omic Ocas, Astellas, 0.4 mg, Tabl tamsulosin	Oral	400 µg microgram(s)/Day 212	20080730- -	Prostatic hypertrophy
8 Pantozol, Nycomed, 40 mg, Tabl pantoprazole		40 Mg milligram(s)/Day	20080723- -	
9 Requip, GSK, 0.25 mg, Tabl ropinirole	Oral	/Day 82	20080725- -	Parkinson's disease
10 Tavor/Tavor expidet, Wyeth, Tabl/Täfelchen lorazepam	Oral	/	20080722- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Parkinson's disease			Unknown	
Insomnia			Unknown	insomnia
Depressive episode			Unknown	
Acute gastritis	200807		Unknown	
Hypotestosteronism			Unknown	idiopathic hypotestosteronemia
Anxiety disorder			Unknown	anxiety disorder
Transaminases increased			Unknown	increase in transaminases
Nycturia			Unknown	nycturia
Nausea			Unknown	nausea
Micturition disorder			Unknown	bladder inflammation (disturbed micturition)
Hypertension arterial			Unknown	arterial hypertension
Gastritis erosive	200807		No	acute erosive gastritis
Fruit allergy			Unknown	allergy to strawberries
Ex-smoker		1970	No	ex-smoker
Cervical spine degeneration			Unknown	degeneration cervica spine
Prostatic hypertrophy			Yes	
Exhaustion			Unknown	felt increasingly exhausted
Lumbago			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20080805	113	U/l	
Alanine aminotransferase increased	20080812	59	U/l	
Alanine aminotransferase increased	20080808	130	U/l	
Alanine aminotransferase increased	20080723	50	U/l	
Alanine aminotransferase normal	20080818	27	U/l	
Aspartate aminotransferase increased	20080805	63	U/l	
Aspartate aminotransferase increased	20080808	60	U/l	
Aspartate aminotransferase normal	20080723	42	U/l	
Aspartate aminotransferase normal	20080812	21	U/l	
Aspartate aminotransferase normal	20080818	13	U/l	
Bilirubin normal	20080723	0.49	mg/dl	
Bilirubin normal	20080818	0.37	mg/dl	
Bilirubin total	20080818	0.37	MG/DL	
Bilirubin total	20080723	0.49	MG/DL	
Cholesterol total	20080818	186	MG/DL	
Cholesterol total	20080812	207	MG/DL	
Cholesterol total	20080808	200	MG/DL	
Gamma-glutamyltransferase	20080818	141	U/L	
Gamma-glutamyltransferase	20080812	257	U/L	
Gamma-glutamyltransferase	20080808	317	U/L	
Gamma-glutamyltransferase	20080805	207	U/L	

SGOT	20080818	13	U/L
SGOT	20080812	21	U/L
SGOT	20080808	60	U/L
SGOT	20080805	63	U/L
SGOT	20080723	42	U/L
SGPT	20080818	27	U/L
SGPT	20080812	59	U/L
SGPT	20080808	130	U/L
SGPT	20080805	113	U/L
SGPT	20080723	50	U/L

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 24.10.2008 DE-FAKOS-170228

Sender diagnosis: not reported

Narrative**Sender's comment:**

initial report (more info follows)

Reporter's comment:

Bayer Vital GmbH, 31.10.2008 DE-BAYER-200829275GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 by BFARM, the regulatory authority of Germany, referring to a male patient whose age is unknown.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprobay (ciprofloxacin hydrochloride), Paracetamol Ratiopharm (paracetamol), and Remergil (mirtazapine) for unknown indications. Administration dates unknown, total daily doses were not reported.

On an unknown date, patient experienced ACUTE HEPATITIS considered serious due to important medical event. Outcome unknown at time of report.

The reporter considered the diagnosis ACUTE HEPATITIS to be probably related.

PVZ-FAKOS, Klinische Pharmakologie, 18.11.2008 DE-FAKOS-170228

Sender diagnosis: not reported

Narrative

Die UAW hat sich im Krankenhaus entwickelt. Ende der Gabe von Remergil und Ciprobay am 07.08.08 sowie von Paracetamol am 05.08.08. Im weiteren Verlauf Rückgang und Normalisierung der erhöhten Transaminasen.

Remergil Soltab Tabl, Organon, ist unspezifisch gelabelt: "Erhöhung der Serumtransaminasen", Stand: 02/2007.

Ciprobay Tabl, Bayer, ist spezifisch gelabelt: "Hepatitis", Stand: 07/2007.

Paracetamol Tabl ist unspezifisch gelabelt bei Ratiopharm: "Anstieg der Lebertransaminasen", Stand: 02/2008.

Reporter's comment:

Bayer Vital GmbH, 24.11.2008 DE-BAYER-200829275GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 by BFARM, the regulatory authority of Germany, referring to a male patient whose age is unknown.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprobay (ciprofloxacin hydrochloride), Paracetamol Ratiopharm (paracetamol), and Remergil (mirtazapine) for unknown indications. Administration dates unknown, total daily doses were not reported.

On an unknown date, patient experienced ACUTE HEPATITIS considered serious due to important medical event. Outcome unknown at time of report.

The reporter considered the diagnosis ACUTE HEPATITIS to be probably related.

****Follow up information received by the Federal Institute for Drugs and Medical Devices on 18-Nov-2008:**

Patient had concomitant conditions of anxiety disorder, acute gastritis, allergy to strawberries, and hypotestosteronemia. Patient's relevant medical history includes ex-smoker (up to 1970, at last 4 cigarettes daily). Past drug therapy includes Tamsulosin (tamsulosin) for pain prostatic. Concomitant drugs are Enahexal, Fluoxetin, Ibuthexal, Omnic Ocas, Pantozol, Requip, and Tavor. For more details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin hydrochloride). Administration from 29-Jul-2008, for a duration of 10 days at a total daily dose of 500 MG orally with lot UNK.
- Paracetamol Ratiopharm (paracetamol). Administration from on 25-Jul-2008 at a total daily dose of 1500 MG orally with lot UNK, and also administered on 26-Jul-2008, for a duration of 11 days at a total daily dose of 2000 MG orally with lot UNK.
- Remergil (mirtazapine) for depressive episode. Administration from 23-Jul-2008 at a total daily dose of 15 MG orally with lot UNK, and also administered on 24-Jul-2008, for a duration of 15 days at a total daily dose of 30 MG orally with lot UNK.

On 05-Aug-2008, patient experienced ACUTE HEPATITIS during hospitalisation. On 18-Aug-2008 the patient recovered from the event.

PVZ-FAKOS, Klinische Pharmakologie, 27.11.2008 DE-FAKOS-170228

Sender diagnosis: not reported

Narrative

Die UAW hat sich im Krankenhaus entwickelt. Ende der Gabe von Remergil und Ciprobay am 07.08.08 sowie von Paracetamol am 05.08.08. Im weiteren Verlauf Rückgang und Normalisierung der erhöhten Transaminasen.

Remergil Soltab Tabl, Organon, ist unspezifisch gelabelt: "Erhöhung der Serumtransaminasen", Stand: 02/2007.

Ciprobay Tabl, Bayer, ist spezifisch gelabelt: "Hepatitis", Stand: 07/2007.

Paracetamol Tabl ist unspezifisch gelabelt bei Ratiopharm: "Anstieg der Lebertransaminasen", Stand: 02/2008.

complete

Reporter's comment:

Bayer Vital GmbH, 05.02.2009 DE-BAYER-200829275GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 by BFARM, the regulatory authority of Germany, referring to a male patient whose age is unknown.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprobay (ciprofloxacin hydrochloride), Paracetamol Ratiopharm (paracetamol), and Remergil (mirtazapine) for unknown indications. Administration dates unknown, total daily doses were not reported.

On an unknown date, patient experienced ACUTE HEPATITIS considered serious due to important medical event. Outcome unknown at time of report.

The reporter considered the diagnosis ACUTE HEPATITIS to be probably related.

****Follow up information received by the Federal Institute for Drugs and Medical Devices on 18-Nov-2008:**

Patient had concomitant conditions of anxiety disorder, acute gastritis, allergy to strawberries, and hypotestosteronemia. Patient's relevant medical history includes ex-smoker (up to 1970, at last 4 cigarettes daily). Past drug therapy includes Tamsulosin (tamsulosin) for pain prostatic. Concomitant drugs are Enahexal, Fluoxetin, Ibuhexal, Omnic Ocas, Pantozol, Requip, and Tavor. For more details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin hydrochloride). Administration from 29-Jul-2008, for a duration of 10 days at a total daily dose of 500 MG orally with lot UNK.
- Paracetamol Ratiopharm (paracetamol). Administration from on 25-Jul-2008 at a total daily dose of 1500 MG orally with lot UNK, and also administered on 26-Jul-2008, for a duration of 11 days at a total daily dose of 2000 MG orally with lot UNK.
- Remergil (mirtazapine) for depressive episode. Administration from 23-Jul-2008 at a total daily dose of 15 MG orally with lot UNK, and also administered on 24-Jul-2008, for a duration of 15 days at a total daily dose of 30 MG orally with lot UNK.

On 05-Aug-2008, patient experienced ACUTE HEPATITIS during hospitalisation. On 18-Aug-2008 the patient recovered from the event.

****Follow-up (additional information) received from a psychiatrist on 29-Jan-2009:**

This spontaneous case report refers to a 56 year-old male (birth date 25-Nov-1951). Additional Lab values of GGT and cholesterol were reported.

Patient had newly reported concomitant conditions of sleeping disorder due to nycturia, increase in transaminases, nausea, arterial hypertension, and bladder inflammation (disturbed micturition). New concomitant drug Sertralin (sertraline) was reported and administration details of the previously reported concomitant medications were updated. For details see section "Concomitant Drugs" at the end of this report.

The patient was hospitalized at the psychiatric ward between 22 Jul 2008 and 21 Aug 2008.

Two weeks prior to this hospitalization, he was treated at the internal ward of the same hospital for a suspected depression with somatoform disorders. He was recommended to visit the psychiatric ward if the acute pain symptoms worsened. A beginning Parkinson's disease was diagnosed due to the patient's posture. The Parkinson symptoms subsided swiftly after the treatment with the dopamine antagonist ropinirol was started. The depressive mood and the sleeping disorder as well as non-specific pain symptoms persisted however, requiring treatment with anti-depressives and mirtazapin (Remergil) at 30 mg, allowing the patient to feel more detached from pain. Additionally, paracetamol was given, provoking an increase in transaminases and causing nausea. Sonography of the upper abdomen revealed no findings, therefore mirtazapin and paracetamol were withdrawn and the treatment with sertraline was started which at time of report was given at a daily dose of 100 mg. Under treatment with this drug, the patient experienced a mood improvement as well as an increased detachment of the vague pain symptoms. A condition of sleep interruptions due to nycturia persisted. Apart from nycturia, the patient was suffering from arterial hypertension where drug adjustment had proved difficult. Therefore, he was recommended to present to a cardiologist. For treatment of a pre-existing lower back pain, ibuprofen was administered successfully.

On 29 Jul 2008, a urological examination was performed revealing a prostrate hypertrophy and a bladder inflammation (disturbed micturition). Sonography showed residual urine amounting to 80 ml. Kidney sonography revealed no findings. For treatment, Ciprobay (ciprofloxacin) at a dose of 2 x 250 ml daily, and Omnic (tamsulosin) 0.4, evening administration, was recommended. In case of a urine suppression, the insertion of a suprapubic catheter was recommended.

The persisting condition of hypogonadism called for a thorough tumour search which showed no clear results.

Occasionally, the patient participated in occupational therapy. He proved capable of socializing to some extent. Due to his lower back pain, and the conditions of Parkinson's disease and arterial hypertension, his participation in exercise therapy was limited. He somewhat lacked initiative as well. After the medication had been adjusted and the patient's affective condition had improved (no risk for committing suicide), he was discharged on his own demand.

PVZ-FAKOS, Klinische Pharmakologie, 13.04.2010 DE-FAKOS-170228

Sender diagnosis: not reported

Narrative

Aufnahme des Patienten in ein Krankenhaus am 22.07.2008. Seit 6 Monaten fühlte sich der Patient zunehmend abgeschlagen und litt unter

18.03.2013

BfArM - Pharmacovigilance

327 / 640

Durchschlafstörungen. Die Transaminasenwerte betrugen am 23.07.2008: ALAT: 50 U/l (Normbereich: <45 U/l) und ASAT: 42 U/l (Normbereich: <50 U/l). Am 05.08.2008 kam es zu einem Anstieg der Transaminasen (ALAT: 113 U/l und ASAT: 63 U/l). Die ALAT erreichte einen Maximalwert am 08.08.2008: 130 U/l. Serologisch erfolgte der Ausschluss einer viralen Hepatitis. Im weiteren stationären Verlauf kam es zu einer Normalisierung der Laborparameter (18.08.2008: ALAT: 27 U/l, ASAT: 13 U/l). Am 21.08.2008 wurde der Patient aus dem Krankenhaus entlassen.

Als Auslöser der Hepatitis werden verdächtigt:

Paracetamol Tabl. (Einnahme: 25.07. - 05.08.2008),

Ciprobay Tabl. (Einnahme: 29.07. - 07.08.2008) und

Remergil Soltab Tabl. (Einnahme: 23.07. - 07.08.2008).

Die Hepatitis wird mit "wahrscheinlich" Arzneimittel-induziert bewertet. Nach Absetzen der Medikamente zeigt sich eine Normalisierung der Transaminasen (Dechallenge). Aufgrund der zeitlichen Überlappung ist insgesamt eine Differenzierung zwischen den Arzneimitteln nicht möglich. Die UAW hat sich im Krankenhaus entwickelt.

Remergil Soltab Tabl, Organon, ist unspezifisch gelabelt: "Erhöhung der Serumtransaminasen", Stand: 02/2007.

Reporter's comment:

sanofi-aventis, 20.04.2010 DE-SANOFI-AVENTIS-A01200815026

Sender diagnosis: not reported

Narrative

Case received from the website of the Health Authorities (DE-BfArM-08124816) via PVZFAKOS, klinische Pharmakologie (DE-FAKOS-170228) through our local affiliate (PCM 04/08) on 19-Nov-2008.

A male patient (age unknown) with a history of Parkinson's disease, depressive episode, anxiety disorder, hypotestosteronemia, lumbago and acute gastritis since Jul-2008, was treated with paracetamol tablets (once 4 x 500 mg tablet /day and later 3x 500 mg/day, brand unknown) for unknown indication for 11 days (25-Jul-2008 to 05-Aug-2008). On 05-Aug-2008 ALAT was elevated at 2.5ULN, ASAT 1.2ULN. An "acute hepatitis" was diagnosed by the reporter. Paracetamol was withdrawn. Under therapy in hospital, patient's transaminases levels were normalizing. On 18-Aug-2008 patient recovered completely.

Further suspected drugs were: Ciprofloxacin with treatment days from 29-Jul-2008 to 07-Aug-2008, Mirtazapine treatment days from 23-Jul-2008 to 07-Aug-2008.

The patient was also receiving enalapril, fluoxetine, ibuprofen, tamsulosin, pantoprazol, ropinirol, and lorazepam.

No etiologic investigations were performed.

The reporter's causality assessment was "probable" for ciprofloxacin, paracetamol and mirtazapine.

Additional information extracted from the website of the German health authority on 15-Apr-2010: The patient was hospitalised on 22-Jul-2008 after six months of increasing asthenia and sleep-maintenance insomnia. ALAT was already slightly increased at time of admission. On 05-Aug-2008, transaminases increased (ALAT 2.51 ULN, ASAT 1.26 ULN). Viral hepatitis was ruled out by serology. After discontinuation of paracetamol, mirtazapine, and ciprofloxacin, transaminases normalised (dechallenge according to information on CIOMS form); a differentiation between these medications was not possible due to temporal overlap. The patient was discharged from hospital on 21-Aug-2008. Causality assessment by FAKOS: "probable/likely" for all suspect drugs.

Bayer Vital GmbH, 20.04.2010 DE-BAYER-200829275GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 by BFARM, the regulatory authority of Germany, referring to a male patient whose age is unknown.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprobay (ciprofloxacin hydrochloride), Paracetamol Ratiopharm (paracetamol), and Remergil (mirtazapine) for unknown indications. Administration dates unknown, total daily doses were not reported.

On an unknown date, patient experienced ACUTE HEPATITIS considered serious due to important medical event. Outcome unknown at time of report.

The reporter considered the diagnosis ACUTE HEPATITIS to be probably related.

**Follow up information received by the Federal Institute for Drugs and Medical Devices on 18-Nov-2008:

Patient had concomitant conditions of anxiety disorder, acute gastritis, allergy to strawberries, and hypotestosteronemia. Patient's relevant medical

history includes ex-smoker (up to 1970, at last 4 cigarettes daily). Past drug therapy includes Tamsulosin (tamsulosin) for pain prostatic. Concomitant drugs are Enahexal, Fluoxetin, Ibuhexal, Omnic Ocas, Pantozol, Requip, and Tavor. For more details see section "Concomitant Drugs" at the end of this report.

Patient received:

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- Remergil (mirtazapine) for depressive episode. Administration from 23-Jul-2008 at a total daily dose of 15 MG orally with lot UNK, and also administered on 24-Jul-2008, for a duration of 15 days at a total daily dose of 30 MG orally with lot UNK.

On 05-Aug-2008, patient experienced ACUTE HEPATITIS during hospitalisation. On 18-Aug-2008 the patient recovered from the event.

****Follow-up (additional information) received from a psychiatrist on 29-Jan-2009:**

This spontaneous case report refers to a 56 year-old male (birth date 25-Nov-1951). Additional Lab values of GGT and cholesterol were reported.

Patient had newly reported concomitant conditions of sleeping disorder due to nycturia, increase in transaminases, nausea, arterial hypertension, and bladder inflammation (disturbed micturition). New concomitant drug Sertraline (sertraline) was reported and administration details of the previously reported concomitant medications were updated. For details see section "Concomitant Drugs" at the end of this report.

The patient was hospitalized at the psychiatric ward between 22 Jul 2008 and 21 Aug 2008.

Two weeks prior to this hospitalization, he was treated at the internal ward of the same hospital for a suspected depression with somatoform disorders. He was recommended to visit the psychiatric ward if the acute pain symptoms worsened. A beginning Parkinson's disease was diagnosed due to the patient's posture. The Parkinson symptoms subsided swiftly after the treatment with the dopamine antagonist ropinirol was started. The depressive mood and the sleeping disorder as well as non-specific pain symptoms persisted however, requiring treatment with anti-depressives and mirtazapin (Remergil) at 30 mg, allowing the patient to feel more detached from pain. Additionally, paracetamol was given, provoking an increase in transaminases and causing nausea. Sonography of the upper abdomen revealed no findings, therefore mirtazapin and paracetamol were withdrawn and the treatment with sertraline was started which at time of report was given at a daily dose of 100 mg. Under treatment with this drug, the patient experienced a mood improvement as well as an increased detachment of the vague pain symptoms. A condition of sleep interruptions due to nycturia persisted. Apart from nycturia, the patient was suffering from arterial hypertension where drug adjustment had proved difficult. Therefore, he was recommended to present to a cardiologist. For treatment of a pre-existing lower back pain, ibuprofen was administered successfully.

On 29 Jul 2008, a urological examination was performed revealing a prostrate hypertrophy and a bladder inflammation (disturbed micturition). Sonography showed residual urine amounting to 80 ml. Kidney sonography revealed no findings. For treatment, Ciprobay (ciprofloxacin) at a dose of 2 x 250 ml daily, and Omnic (tamsulosin) 0.4, evening administration, was recommended. In case of a urine suppression, the insertion of a suprapubic catheter was recommended.

The persisting condition of hypogonadism called for a thorough tumour search which showed no clear results.

Occasionally, the patient participated in occupational therapy. He proved capable of socializing to some extent. Due to his lower back pain, and the conditions of Parkinson's disease and arterial hypertension, his participation in exercise therapy was limited. He somewhat lacked initiative as well. After the medication had been adjusted and the patient's affective condition had improved (no risk for committing suicide), he was discharged on his own demand.

****Follow-up received on 14-Apr-2010:**

Patient had newly reported relevant medical history of feeling increasingly exhausted and suffering from insomnia. Transaminases on 23-JUL-2008 were ALAT 50 U/l, ASAT 42 U/l. On 05-AUG-2008 transaminases were increased to 113 U/l (ALAT) and 63 U/l (ASAT). Maximum level of ALAT was 130 U/l on 08-AUG-2008. Viral hepatitis could be excluded. During hospital course lab parameters normalized. Hepatitis was assessed to be "likely" drug induced by Paracetamol, Ciprobay and Remergil. After withdrawal of suspect drugs transaminases normalized (dechallenge). Due to temporal overlap further differentiation between the drugs is not possible. SAE developed during hospital stay.

***** BAYER Schering Global Comment - PartI *****

ACUTE HEPATITIS, considered medically important and therefore serious, is listed in the International Product Information for CIPROBAY (CIPROFLOXACIN MONOHYDROCHLORIDE). A causal relationship cannot be properly assessed due to missing information concerning temporal relationship. Paracetamol might have caused or contributed to the event. Case is under national follow-up.

****Follow-up received on 18-NOV-2008 included concomitant and suspect drug details and lab values((ALT,ASTand bilirubin).**

Based on the information now received a causal relationship between the event reported and the treatment with Ciprobay (ciprofloxacin cannot be excluded due to a compatible temporal relationship. Referring to the information on concomitant drugs the moderately increased transaminases in this patient cannot be attributed to Ciprobay alone as the patient received several drugs simultaneously which are also known to cause increased transaminases (paracetamol, fluoxetin, mirtazapine, pantoprazol). An additive effect of all prescribed medication may be discussed in this context as well.

Worthy of note is that the patient's ALT was already slightly above the ULN 6 days prior to start with Ciprobay treatment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08135416

Serious Yes	Death	Lifethr. Yes	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19160608	Age: 92 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20081112				not recovered/not resolved
2 Renal failure	20081112				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan	Intravenous (not otherwis	3 G gram(s)/Day	-	Postoperative pain
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pelvic discomfort			Unknown	Beckenprellung nach Sturz
Dementia			Yes	
Insulin dependent diabetic			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08137035

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19830926	Age: 24 Year	weight (kg): 65	BMI_int: 22.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukocyte count increased	20080929	20081013	15 Day	15 Day	recovered/resolved
2 Thrombocytopenia	20080929	20081017	19 Day	15 Day	recovered/resolved
3 Hemoglobin decreased	20080929			15 Day	recovering/resolving
4 Headache aggravated	200809			4 Month	Unknown
5 Anemia	20080929			4 Month	recovering/resolving
6 Blood bilirubin increased	20080929	20081002	4 Day	4 Month	recovered/resolved
7 LDH increased	20080929	20081027	29 Day	4 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Unknown	/	-	Headache aggravated
2 paracetamol paracetamol	Unknown	/	-	Headache aggravated
3 Valette dienogest; ethinylestradiol	Oral	1 Df dosage form/Day Film-coated tablet	200806-20080928 3 Month	Oral contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			Unknown	smoker

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hemoglobin	200809	6.9	G/DL	
Bilirubin total	20080929	1.5	MG/DL	
C-reactive protein	20081027	0.2	MG/DL	
C-reactive protein	20080929	1.5	MG/DL	
Fibrin D dimer	20080929	0.33	MG/L	
Hemoglobin	20081027	11.2	G/DL	
Hemoglobin	20081017	9.0	G/DL	
Hemoglobin	20081003	7.9	G/DL	
Hemoglobin	20080929	9.6	G/DL	
Lactate dehydrogenase	20081027	189	IU/L	
Lactate dehydrogenase	20081001	492	IU/L	
Lactate dehydrogenase	20080929	602	IU/L	
Platelet count	20080929	6	GIGA/L	

Platelet count	20081005	33	GIGA/L
Platelet count	20081015	157	GIGA/L
Platelet count	20081014	135	GIGA/L
Platelet count	20081020	177	GIGA/L
Platelet count	20081013	126	GIGA/L
Platelet count	20081017	177	GIGA/L
Platelet count	20081004	32	GIGA/L
Platelet count	20081003	31	GIGA/L
Platelet count	20081002	22	GIGA/L
Platelet count	20081001	18	GIGA/L
Platelet count	20080929	37	GIGA/L
Red blood cell count	20081023	3.3	10E9/MM3
Red blood cell count	20081020	2.9	10E9/MM3
Red blood cell count	20081027	3.4	10E9/MM3
Red blood cell count	20081004	2.7	10E9/MM3
Red blood cell count	20081003	2.6	10E9/MM3
Red blood cell count	20080929	3.2	10E9/MM3
Red blood cell count	20080929	3.0	10E9/MM3
Red blood cell count	20081001	3.0	10E9/MM3
Red blood cell count	20081002	2.7	10E9/MM3
Red blood cell count	20081017	2.7	10E9/MM3
Red blood cell count	20081005	2.4	10E9/MM3
Red blood cell count	20081013	2.5	10E9/MM3
Red blood cell count	20081014	2.5	10E9/MM3
Red blood cell count	20081015	2.5	10E9/MM3
SGPT	20081002	45	IU/L
White blood cells	20081013	7.6	GIGA/L
White blood cells	20081005	19.4	GIGA/L
White blood cells	20081004	12.7	GIGA/L

Narratives (nur bei elektronischem Eingang)

JENAPHARM DEUTSCHLAND, 24.11.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

**Follow-up information received on 18-Nov-2008 from the physician:

This case has been upgraded to SERIOUS due to life threatening.

The event "platelets incresed" changed to PLATELETS DROP (UNSPECIFIED).

Patient's demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m². Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

JENAPHARM DEUTSCHLAND, 05.12.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

****Follow-up information received on 18-Nov-2008 from the physician:**

This case has been upgraded to SERIOUS due to life threatening.

The event "platelets incresed" changed to PLATELETS DROP (UNSPECIFIED).

Patient's demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m². Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

The correct date of Valette start was Jun-2008.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography.

There were no ECG or chest X-ray abnormalities.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. A minimal elevation of D-dimers on 29-Sep-2008 was not commented upon.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state.

Treatment with 100 mg/day prednisone was started after the infusion of a platelet concentrate on the assumption of an autoimmune disorder, however treatment ended on 08-Oct-2008 without success.

A blood transfusion was refused by the patient due to the relatively good subjective general state.

The patient was then discharged in an improved status after the coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor and moderate penetrance was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned. After

discharge, daily out-patient blood checks showed a gradual, spontaneous improvement of the hemoglobin levels and of the platelet count. Thus, the recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events. A pill change was recommended, as well as a strict avoidance of anti-pain medication.

****Bayer-Shering Global Comment (Part I):**

PLATELETS INCREASED, preliminary considered non-serious, is not listed in the International Product Information for VALETTE (DIENOGEST + ETHINYLESTRADIOL). A causal relationship cannot be excluded due to a compatible temporal relationship. Development of malignancy cannot be ruled out. Case is under national follow-up.

****Follow-up information received on 18-Nov-2008 from the physician:**

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a 25-years old female who developed PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED during the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) for oral contraception.

This case has been upgraded to SERIOUS due to life threatening of the reported events.

The reported events - PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED - are unlisted in the CCDS for VALETTE (DIENOGEST + ETHINYLESTRADIOL). Based on the information received for this case, a causal relationship between the reported events and the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) cannot be excluded. This view is supported by a positive dechallenge for VALETTE (DIENOGEST + ETHINYLESTRADIOL). The information about an alternative explanation is not available for this case. Case in Follow-up.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal. (continued)

JENAPHARM DEUTSCHLAND, 15.12.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

****Follow-up information received on 18-Nov-2008 from the physician:**

This case has been upgraded to SERIOUS due to life threatening.

The event "platelets incresed" changed to PLATELETS DROP (UNSPECIFIED).

Patient`s demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m2. Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

The correct date of Valette start was Jun-2008.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography.

There were no ECG or chest X-ray abnormalities.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. A minimal elevation of D-dimers on 29-Sep-2008 was not commented upon.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state.

Treatment with 100 mg/day prednisone was started after the infusion of a platelet concentrate on the assumption of an autoimmune disorder, however treatment ended on 08-Oct-2008 without success.

A blood transfusion was refused by the patient due to the relatively good subjective general state.

The patient was then discharged in an improved status after the coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor and moderate penetrance) was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned. After discharge, daily out-patient blood checks showed a gradual, spontaneous improvement of the hemoglobin levels and of the platelet count.

Thus, the recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events. A pill change was recommended, as well as a strict avoidance of anti-pain medication.

****Follow up information received on 05-Dec-2008:**

A PTC number was provided.

Co-suspect drugs were included with aspirin and paracetamol.

****Bayer-Shering Global Comment (Part I):**

PLATELETS INCREASED, preliminary considered non-serious, is not listed in the International Product Information for VALETTE (DIENOGEST + ETHINYLESTRADIOL). A causal relationship cannot be excluded due to a compatible temporal relationship. Development of malignancy cannot be ruled out. Case is under national follow-up.

****Follow-up information received on 18-Nov-2008 from the physician:**

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a 25-years old female who developed PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED during the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) for oral contraception.

This case has been upgraded to SERIOUS due to live threatening of the reported events.

The reported events - PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED - are unlisted in the CCDS for VALETTE (DIENOGEST + ETHINYLESTRADIOL). Based on the information received for this case, a causal relationship between the reported events and the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) cannot be excluded. This view is supported by a positive dechallenge for VALETTE (DIENOGEST + ETHINYLESTRADIOL). The information about an alternative explanation is not available for this case. Case in Follow-up.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Valette (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal. (continued)

Bayer Vital GmbH, 15.12.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is

apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

****Follow-up information received on 18-Nov-2008 from the physician:**
This case has been upgraded to SERIOUS due to life threatening.
The event "platelets incresed" changed to PLATELETS DROP (UNSPECIFIED).

Patient`s demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m2. Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral. The correct date of Valette start was Jun-2008.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol. In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography.

There were no ECG or chest X-ray abnormalities.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. A minimal elevation of D-dimers on 29-Sep-2008 was not commented upon.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state.

Treatment with 100 mg/day prednisone was started after the infusion of a platelet concentrate on the assumption of an autoimmune disorder, however treatment ended on 08-Oct-2008 without success.

A blood transfusion was refused by the patient due to the relatively good subjective general state.

The patient was then discharged in an improved status after the coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor and moderate penetrance) was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned. After discharge, daily out-patient blood checks showed a gradual, spontaneous improvement of the hemoglobin levels and of the platelet count.

Thus, the recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events. A pill change was recommended, as well as a strict avoidance of anti-pain medication.

****Follow up information received on 05-Dec-2008:**

A PTC number was provided.

Co-suspect drugs were included with aspirin and paracetamol.

****Bayer-Shering Global Comment (Part I):**

PLATELETS INCREASED, preliminary considered non-serious, is not listed in the International Product Information for VALETTE (DIENOGEST + ETHINYLESTRADIOL). A causal relationship cannot be excluded due to a compatible temporal relationship. Development of malignancy cannot be ruled out. Case is under national follow-up.

****Follow-up information received on 18-Nov-2008 from the physician:**

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a 25-years old female who developed PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED during the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) for oral contraception.

This case has been upgraded to SERIOUS due to life threatening of the reported events.

The reported events - PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED - are unlisted in the CCDS for VALETTE (DIENOGEST + ETHINYLESTRADIOL). Based on the information received for this case, a causal relationship between the reported events and the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) cannot be excluded. This view is supported by a positive dechallenge for VALETTE (DIENOGEST + ETHINYLESTRADIOL). The information about an alternative explanation is not available for this case. Case in Follow-up.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks

before referral.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal. (continued)

JENAPHARM DEUTSCHLAND, 16.12.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

****Follow-up information received on 18-Nov-2008 from the physician:**

This case has been upgraded to SERIOUS due to life threatening.

The event "platelets incresed" changed to PLATELETS DROP (UNSPECIFIED).

Patient`s demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m2. Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

The correct date of Valette start was Jun-2008.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography.

There were no ECG or chest X-ray abnormalities.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. A minimal elevation of D-dimers on 29-Sep-2008 was not commented upon.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryiopoiesis and erythropoiesis, typical of a post-consumption state.

Treatment with 100 mg/day prednisone was started after the infusion of a platelet concentrate on the assumption of an autoimmune disorder, however treatment ended on 08-Oct-2008 without success.

A blood transfusion was refused by the patient due to the relatively good subjective general state.

The patient was then discharged in an improved status after the coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor and moderate penetrance) was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned. After discharge, daily out-patient blood checks showed a gradual, spontaneous improvement of the hemoglobin levels and of the platelet count.

Thus, the recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events. A pill change was recommended, as well as a strict avoidance of anti-pain medication.

****Follow up information received on 05-Dec-2008:**

A PTC number was provided.

Co-suspect drugs were included with aspirin and paracetamol.

****Result and assessment of the product technical complaint investigation was received on 09-Dec-2008:**

Complaint as reported:

DEJ2008266 "gefährliche Thrombozytopenie + Anämie 4 Monate nach Valette-Start"

Final assessment:

The recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events.

Medical assessment:

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral. In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities. The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol. In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal. A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state. The coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned (alternative explanation for the reported events). The recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events.

****Bayer-Shering Global Comment (Part I):**

PLATELETS INCREASED, preliminary considered non-serious, is not listed in the International Product Information for VALETTE (DIENOGEST + ETHINYLESTRADIOL). A causal relationship cannot be excluded due to a compatible temporal relationship. Development of malignancy cannot be ruled out. Case is under national follow-up.

****Follow-up information received on 18-Nov-2008 from the physician:**

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a 25-years old female who developed PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED during the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) for oral contraception.

This case has been upgraded to SERIOUS due to live threatening of the reported events.

The reported events - PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED - are unlisted in the CCDS for VALETTE (DIENOGEST + ETHINYLESTRADIOL). Based on the information received for this case, a causal relationship between the reported events and the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) cannot be excluded. This view is supported by a positive dechallenge for VALETTE (DIENOGEST + ETHINYLESTRADIOL). The information about an alternative explanation is not available for this case. Case in Follow-up.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state. (continued)

Bayer Vital GmbH, 16.12.2008 DE-BAYER-200828297GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a female patient of unknown age.

Details regarding patient's past medical history and concurrent medical conditions were not reported. It is unknown whether any concomitant drugs have been given.

On an unspecified date, patient received Valette at 1 tablet/day. Administration lasting a few years, the indication for use was not specified.

In 2008, patient experienced PLATELETS INCREASED, considered non-serious. Outcome unknown at time of report. The platelet increase is apparently conspicuous (reportedly 6000, but units were not specified), so that a malignancy is being considered in the differential diagnosis.

No assessment was given.

****Follow-up information received on 18-Nov-2008 from the physician:**

This case has been upgraded to SERIOUS due to life threatening.

The event "platelets increased" changed to PLATELETS DROP (UNSPECIFIED).

Patient's demographics added. The consumer is smoker and has a normal BMI of 22.2 kg/m². Concomitant diseases were reported as absent. The intake of Valette started approximately on 15-Sep-2008 for the indication oral contraception. Valette had been taken before and well tolerated.

In contrast to the first report, the platelets were reported as decreased (drop of unspecified extent). In addition, increased leucocytes (unspecified extent) and decreased hemoglobin to 6.9 g/dl were now reported. The reported hemoglobin value was nearly half the lower limit of the generally known normal range (12 g/dl).

On an unspecified date, Valette was discontinued.

The consumer has not recovered, but her conditions improved.

The gynecologist reported the causality as possible.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

The correct date of Valette start was Jun-2008.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography.

There were no ECG or chest X-ray abnormalities.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. A minimal elevation of D-dimers on 29-Sep-2008 was not commented upon.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state.

Treatment with 100 mg/day prednisone was started after the infusion of a platelet concentrate on the assumption of an autoimmune disorder, however treatment ended on 08-Oct-2008 without success.

A blood transfusion was refused by the patient due to the relatively good subjective general state.

The patient was then discharged in an improved status after the coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor and moderate penetrance) was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned. After discharge, daily out-patient blood checks showed a gradual, spontaneous improvement of the hemoglobin levels and of the platelet count.

Thus, the recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events. A pill change was recommended, as well as a strict avoidance of anti-pain medication.

****Follow up information received on 05-Dec-2008:**

A PTC number was provided.

Co-suspect drugs were included with aspirin and paracetamol.

****Result and assessment of the product technical complaint investigation was received on 09-Dec-2008:**

Complaint as reported:

DEJ2008266 "gefährliche Thrombozytopenie + Anämie 4 Monate nach Valette-Start"

Final assessment:

The recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events.

Medical assessment:

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks

before referral. In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities. The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol. In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal. A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state. The coagulation tests excluded a thrombotic thrombocytopenic purpura (Moskowitz syndrome). A von Willebrand-Jürgens disease Type I with quantitative defect of the Willebrand factor was suspected but other differential diagnoses, i.e., a transient autoimmune-mediated thrombopenia and autoimmune-hemolytic anemia, were also mentioned (alternative explanation for the reported events). The recent change of oral contraceptive combined with the intake of aspirin and paracetamol were reported as possible triggers of the patient's events.

****Bayer-Shering Global Comment (Part I):**

PLATELETS INCREASED, preliminary considered non-serious, is not listed in the International Product Information for VALETTE (DIENOGEST + ETHINYLESTRADIOL). A causal relationship cannot be excluded due to a compatible temporal relationship. Development of malignancy cannot be ruled out. Case is under national follow-up.

****Follow-up information received on 18-Nov-2008 from the physician:**

This spontaneous case report was received on 07-Oct-2008 from a gynecologist in Germany, referring to a 25-years old female who developed PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED during the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) for oral contraception.

This case has been upgraded to SERIOUS due to live threatening of the reported events.

The reported events - PLATELETS DROP, LEUCOCYTES INCREASED and HEMOGLOBIN DECREASED - are unlisted in the CCDS for VALETTE (DIENOGEST + ETHINYLESTRADIOL). Based on the information received for this case, a causal relationship between the reported events and the treatment with VALETTE (DIENOGEST + ETHINYLESTRADIOL) cannot be excluded. This view is supported by a positive dechallenge for VALETTE (DIENOGEST + ETHINYLESTRADIOL). The information about an alternative explanation is not available for this case. Case in Follow-up.

****Follow-up received on 01-Dec-2008 from the gynecologist:**

The patient was hospitalized from 29-Sep-2008 to 08-Oct-2008 (further reason for seriousness) to clarify the genesis of thrombocytopenia and anemia, in the presence of tiredness and exhaustion, mild dyspnea, and blue spots in the extremities with hematomas and petechiae in the 1-2 weeks before referral.

In the hospital no abnormalities of liver, biliary tracts, pancreas, spleen, kidneys, urinary bladder, abdominal vessels, gastrointestinal tract, pleura, and pericardium could be found at physical examination and/or ultrasonography. There were no ECG or chest X-ray abnormalities.

The hospital discharge letter mentioned that the patient had switched from another pill (unspecified) 4 months before the events and that a progressively increasing headache (listed event) had developed before hospitalization, leading to the intake of large amounts of aspirin and paracetamol.

In addition to the already reported events, mildly elevated bilirubin levels and elevated lactic dehydrogenase (LDH) levels were also reported. The reported events are not listed in the International Product Information for Vallete (dienogest + ethinylestradiol), however, it mentioned that acute or chronic disturbances of liver function may necessitate the discontinuation of the drug use until markers of liver function return to normal.

A bone marrow biopsy was carried out and histology and cytology examinations excluded a malignancy and showed a regenerative-activated pattern of megakaryopoiesis and erythropoiesis, typical of a post-consumption state. (continued)

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08141206

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20080805				recovering/resolving
2 Abdominal pain					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol paracetamol	Oral	1500 Mg milligram(s)/Day	20080802-20080805 4 Day	
2 Remergil mirtazapine		15 Mg milligram(s)/Day	20080723- -	
3 Remergil mirtazapine		30 Mg milligram(s)/Day	20080724-20080807 15 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal cyst			Unknown	
Parkinson's disease			Unknown	
Depressive episode			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20080805	63	NA	
SGPT	20080805	113	NA	
Gamma GT	20080805	207	NA	
SGOT	20080808	60	NA	
SGPT	20080808	130	NA	
Gamma GT	20080808	317	NA	
SGOT	20080812	21	NA	
SGPT	20080812	59	NA	
Gamma GT	20080812	257	NA	
SGOT	20080818	13	NA	
SGPT	20080818	27	NA	
Gamma GT	20080818	141	NA	

Narratives (nur bei elektronischem Eingang)

ORGANON, 02.12.2008 DE-ORGANON-2008-187381-NL

Sender diagnosis: not reported

Narrative

Case report 2008-187381-NL is a spontaneous case report sent by a health professional. This case was reported within the framework of the AMSP

project.

An approximately 57-year-old male patient had used Remeron since 23 July 2008 and paracetamol since 2 August 2008. He experienced an unspecific abdominal pain on an unspecified date and increased transaminase values on 5 August 2008.

Laboratory values:

5 August 2008: GOT 63 (normal < 50), GPT 113 (normal < 45), gGT 207 (normal <55).

8 August 2008: GOT 60, GPT 130, gGT 317

12 August 2008: GOT 21, GPT 59, gGT 257

18 August 2008: GOT 13, GPT 27, gGT 141

An abdomen sonography on 15 August 2008 was inconspicuous, but showed a small renal cyst on the right side as side finding. Remeron was discontinued on 7 August 2008 and paracetamol on 5 August 2008. Treatment with Sertralin (sertraline hydrochloride) and ibuprofen was started. Ibuprofen was discontinued when the pain abated. Sertralin prescription was continued.

Prior exposition: no. Reexposition: no. No possible risk factors.

At the time of reporting, the transaminase levels were normalising, the abdominal pain had abated on an unspecified date.

Concomitantly the patient suffered from a severe depressive episode and Parkinson's disease.

Outcome: RECOVERING

NV Organon does not consider the included Organon assessment to be an admission that REMERGIL either caused or contributed to the described adverse event(s)

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08142489

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 00045 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Liver failure
deathdate:	deathcause: Septic shock
deathdate:	deathcause: Renal failure
deathdate:	deathcause: Disorder coagulation

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Septic shock					fatal
2 Disorder coagulation					fatal
3 Renal failure					fatal
4 Liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 parfalgan paracetamol		00000001 G gram(s)/	-	Pain
2 parfalgan paracetamol		00000001 G gram(s)/	-	Cholangitis

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 05.12.2008 DE-Bristol-Myers Squibb Company-14425649

Sender diagnosis: not reported

Narrative

A hospital physician reported via BMS Germany (local file number 2008120104 and TRECnet 1-14524741) that 45-year-old female patient experienced septic shock, liver failure, renal failure and abnormal coagulation 6 to 7 hours after a single administration of 1 gram acetaminophen for pain-treatment of abdominal symptomatology and suspected cholangitis. The patient died in the afternoon of the next day. According to the physician, the patient had been admitted to hospital due to abdominal symptomatology and suspected cholangitis.

BMS Medical Evaluation Comment:

This patient experienced fatal septic shock, liver failure, renal failure and abnormal coagulation 6 to 7 hours after a single administration of acetaminophen for pain treatment of abdominal symptomatology and suspected cholangitis. Suspected underlying disease could have led to the reported events. However limited information precludes a meaningful assessment of this case.

#1; unexpected; PSB
 #2; unexpected; PSB
 #3; unexpected; PSB
 #4; unexpected; PSB

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: Suspected underlying disease could have led to the reported events. However limited information precludes a meaningful assessment of

this case.

Reporter's comment:

Bristol-Myers Squibb Company, 02.02.2009 DE-Bristol-Myers Squibb Company-14425649

Sender diagnosis: not reported

Narrative

A hospital physician reported via BMS Germany (local file number 2008120104 and TRECnet 1-14524741) that 45-year-old female patient experienced septic shock, liver failure, renal failure and abnormal coagulation 6 to 7 hours after a single administration of 1 gram acetaminophen for pain-treatment of abdominal symptomatology and suspected cholangitis. The patient died in the afternoon of the next day. According to the physician, the patient had been admitted to hospital due to abdominal symptomatology and suspected cholangitis.

BMS Medical Evaluation Comment:

This patient experienced fatal septic shock, liver failure, renal failure and abnormal coagulation 6 to 7 hours after a single administration of acetaminophen for pain treatment of abdominal symptomatology and suspected cholangitis. Suspected underlying disease could have led to the reported events. However limited information precludes a meaningful assessment of this case.

Supplemental information received on 21-Jan-2009 from the physician reported that the patient also experienced an allergic reaction after administration of acetaminophen.

The patient underwent emergency operation on 01-Dec-2008 at about 6:45 am for diagnostic laparoscopy and open cholecystectomy. The patient was pre-operational tachypnoeic, SpO2 was 75%, was hypertensive with 170/80 mmHg, heart rate was 125/minute in sinus rhythm, marbling at the extremities. Despite pre-oxygenation with 16 L/min, only very hesitant oxygen (O2) increase. Immediately after start of anesthesia, a massive circulatory break down occurred, the patient had to be treated with norepinephrine.

The same day, the septic patient (suspected meningococcal sepsis) was administered acetaminophen 500mg i.v. from 8:15 to 8:30 am for the treatment of fever. At 8:30 the patient experienced the allergic reaction with blood pressure decrease from 110/70 mm Hg to about 70/40 mmHg and an increase of heart rate from 110 /min to about 135 /min. The patient was taken over at the intensive ward at about 9:00 am intubated and circulatory instable (blood pressure: 70/50 mmHg, heart rate about 110 /minute). The patient was treated with prednisolone 250mg intravenously (iv) and the shock symptoms of the patient improved and resolved (back to baseline values) at about 10:00 am.

Due to the sepsis a substantial coagulation disorder occurred (besides others decrease of thrombocytes to 27.000, Quick <5%, Partial thromboplastin time (PTT) not measurable, decrease of coagulation factors, antithrombin (AT) III: 23%), which could not be controlled after administration of proconvertin or F VII, prothrombin, Stuart factor or Factor X and antihaemophilic factor B or Factor IX (PPSB), fresh frozen plasma (FFP) and AT III.

Despite maximal therapy the patient expired at about 15:25 pm.

The anaerobic blood culture revealed gram positive cocci, the result was provided by phone at 17:40 pm.

The physician stated that blood pressure decrease after acetaminophen 500mg could have contributed to the events. The physician reported that he was informed by the relatives of the patient afterwards that the patient was allergic to acetaminophen, but he was not aware of this when he decided to administer acetaminophen 500mg for the treatment of fever.

The physician considered the allergic reaction non-serious and assessed the allergic reaction probably related to acetaminophen, septic shock, liver failure, renal failure and abnormal coagulation were not likely related to acetaminophen.

Patient medical history included acute cholecystitis, pneumonia infrahilar left side, pleural effusion right side, lupus erythematosus , polycystic kidneys, meningitis in 1996, nasal septum plasty in Jan-2008 - chronic sinusitis ethmoidalis and maxillaris both sides, sinusitis sphenoidalis left side.

BMS Medical Evaluation Comment:

This patient with history of allergy to acetaminophen and underlying sepsis suffered a massive circulatory breakdown during emergency surgery for cholecystitis. Later that day, the patient died of septic shock with DIC and multiorgan failure. Although hypovolemia occurred immediately after start of anesthesia, prior to acetaminophen infusion, an allergic reaction to acetaminophen could have contributed to further vascular decompensation. The fatal outcome was considered most likely related to the natural course of the underlying sepsis.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08148370

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 18 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	30 G gram(s)/	-	Suicide attempt
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Bechmann LP, Marquitan G, Jochum C, Saner F, Gerken G, Canbay A. Apoptosis versus necrosis rate as a predictor in acute liver failure following acetaminophen intoxication compared with acute-on-chronic liver failure. Liver International 2008; 28(5): 713-716	

Narratives (nur bei elektronischem Eingang)

Woerwag Pharma GmbH, 18.12.2008 DE-WOERWAG PHARMA GMBH-085180

Sender diagnosis: not reported

Narrative

A case of paracetamol-induced acute liver failure is reported:

A 18-year-old woman was admitted with acute liver failure after intended intoxication with approximately 30 g of acetaminophen. ALT levels by that time exceeded 9000 U/L, with bilirubin levels of 2.9 mg/dL and an INR of 1.9. Clinically the patient was jaundiced and fatigued but did not show signs of higher degrees of encephalopathy or significant bleeding. Conservative treatment with acetylcysteine was initiated and a supportive therapy with vitamin K was conducted. The patient improved quickly and lab values returned to normal, detected by the coagulatory studies. The patient was discharged 9 days after admission with an INR of 0.93, bilirubin 0.3 mg/dL, AST of 102 U/L and ALT levels of 175 U/L. paracetamol and the SPC to Sinpro. No actions are required.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08148882

Serious Yes	Death Yes	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19411201	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20060610	deathcause: Stevens Johnson syndrome
deathdate: 20060610	deathcause: Sepsis
deathdate: 20060610	deathcause: Hepatic failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20060514			29 Day	fatal
2 Sepsis					fatal
3 Liver failure					fatal
4 Eye redness	20060516			31 Day	fatal
5 Esophageal erosions	20060519			34 Day	fatal
6 Mucosal erosion	20060516			31 Day	fatal
7 Blister	20060515			30 Day	fatal
8 Stevens Johnson syndrome	20060514			29 Day	fatal
9 Fever	20060508			25 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Oral	/	20060416-20060513 Respiratory disorder 28 Day
2	ACERBON lisinopril	Oral	/	20060424-20060513 Hypertension 20 Day
3	ANAESTHETIKA, Lokal	Subcutaneous	/	20060508- -
4	augmentan i.v. amoxicillin sodium; clavulanate potassium		/	20060508- Prophylaxis -
5	BISOHEXAL bisoprolol hemifumarate	Oral	/	20060424-20060513 Hypertension 20 Day
6	BONDIOL alfacalcidol	Oral	/	20060416-20060513 Hyperparathyroidism secondary 28 Day
7	calcimagon calcium carbonate	Oral	/	20060416-20060513 Hyperparathyroidism secondary 28 Day
8	LASIX furosemide	Oral	/	20060416-20060513 Renal failure 28 Day
9	lonolox minoxidil	Oral	/	20060416-20060422 Hypertension 7 Day
10	OXAZEPAM oxazepam	Oral	/	20060508- Sedation -
11	PANTOZOL pantoprazole sodium	Oral	/	20060416-20060513 Prophylaxis 28 Day
12	PERFALGAN paracetamol		/	20060508- Fever -
13	SANDOSTATIN octreotide acetate	Subcutaneous	/	20060420-20060503 Pancreatitis 14 Day
14	thyronajod levothyroxine sodium; potassium iodide	Oral	/	20060416-20060513 28 Day
15	TRANXILIUM clorazepate dipotassium	Oral	/	20060508- Sedation -
16	XIPAMIDE xipamide	Oral	/	20060417-20060513 Hypertension 27 Day

Drugs (concomitant)					
17	AQUAPHOR xipamide	/CAN/	Oral	/	20060416-20060417 Hypertension arterial 2 Day
18	BICANORM sodium carbonate anhydrous	/01764101/	Oral	/	20060416-20060514 29 Day
19	clont metronidazole			/	20060411-20060425 Sepsis NOS 15 Day
20	CONCOR bisoprolol fumarate		Oral	/	20060416-20060423 Hypertension arterial 8 Day
21	dreisavit ascorbic acid; calcium pantothenate; folic acid; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride		Oral	/	20060417-20060514 28 Day
22	HYDROCORTISON hydrocortisone	/CZE/		/	20060412-20060430 19 Day
23	LISIHEXAL lisinopril		Oral	/	20060416-20060423 Hypertension arterial 8 Day
24	MERONEM meropenem			/	20060411-20060424 Sepsis NOS 14 Day
25	nacl sodium chloride			/	20060411-20060417 7 Day
26	nephro-vite ascorbic acid; folic acid; vitamin b nos			/	20060424-20060503 10 Day
27	VANCOMYCIN vancomycin			/	20060420- Sepsis NOS -
28	VANCOMYCIN vancomycin			/	20060425- -
29	VANCOMYCIN vancomycin			/	20060430- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal failure			Unknown	
Hyperparathyroidism secondary			Unknown	
Renal insufficiency			Unknown	
Respiratory disorder			Yes	
Prophylaxis			Unknown	
Hypertension arterial			Yes	
Difficulty breathing			Unknown	
Sepsis	200804	200804	No	
Fever			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.12.2008 DE-002147023-PHHY2008DE31655

Sender diagnosis: not reported

Narrative

Physician report originating from a non-Novartis registry, "RegiScar Project, Documentation Centre for Severe Skin Reactions in Germany", case ref-no. 9170408, received on 11 Dec 2008: This patient with a history of arterial hypertension and renal insufficiency had been treated with multiple drugs. He had been hospitalized in a surgery ward since 10 Apr 2006. On 08 May 2006 and 11 May 2006 he underwent surgery for shunt placements. He had received Lonolox 5 mg daily (minoxidil) from 16 Apr 2006 to 22 Apr 2006, acetylcysteine 600 mg daily from 16 Apr to 13 May 2006, Lasix (furosemide) 40 mg daily from 16 Apr to 13 May 2006, Pantozol (pantoprazole) 80 mg daily from 16 Apr to 13 May 2006, Calcimagon (calcium carbonate) 1500 mg daily from 16 Apr to 13 May 2006, Bondiol (alfacalcidol) 0.25 mg daily from 16 Apr to 13 May 2006, Thyronajod (levothyroxine, potassium iodide) from 16 Apr to 13 May 2006, xipamid 20 mg daily from 17 Apr to 13 May 2006, Sandostatin s.c. injections 3 times daily for treatment

of a pancreatitis from 20 Apr 2006 to 03 May 2006, Acerbon (lisinopril) 10 mg daily from 24 Apr to 13 May 2006, Bisohexal (bisoprolol) 5 mg daily from 24 Apr to 13 May 2006, an unknown local anaesthetic substance on 08 May 2006, oxazepam 10 mg on 08 May 2006, Tranxilium (dipotassium chlorazepate) 10 mg on 08 May 2006, Perfalgan (propacetamol) 1 g injection on 08 May 2006 and Augmentan (amoxicilline, clavulanic acid) 2.2 g injection on 08 May 2006. On 08 and 09 May 2006 he showed fever. On 14 May 2006, he again experienced fever, on 15 May 2006 he showed spots and cocards on his skin, as well as skin blistering. On this day, he was transferred from surgery to an intensive care unit. On 16 May 2006 he showed eye redness and erosions at lips and oral mucosa. On 18 May 2006, he also showed erosions at genital mucosa. On 19 May 2006, Nikolski's sign was positive, there were also erosive oesophagus lesions. In the further course he developed Stephens-Johnson-syndrome / Toxic epidermal necrolysis, transitional form. He developed sepsis and liver failure and finally passed away due to the events. End date of the events (presumably date of death) was provided as 10 Jun 2006. For Lonolox, acetylcysteine, Lasix, Pantozol, Calcimagon, Bondiol, Thyronajod, xipamid, Sandostatin, Acerbon, Bisohexal, the unknown local anaesthetic substance, oxazepam, tranxilium and Perfalgan, causality was reported as possible. For Augmantan causality was reported as probable.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003267

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19750626	Age: 33 Year	weight (kg): 97	BMI_int: 35.63	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20080726				recovering/resolving
2 Hepatocellular injury	20080726				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 METAMIZOLE	Oral	500 Mg milligram(s)/82	20080725-20080729	Fever
metamizole sodium			5 Day	
2 PARACETAMOL	Oral	500 Mg milligram(s)/245	20080722-20080727	Fever
paracetamol			6 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pyrexial cold	20080722	20080726	No	Concomitant disease
Multiple allergies			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080727	5.75	µmol/l*s	
Alanine aminotransferase	20080829	0.76	µmol/l*s	
Alanine aminotransferase	20080729	7.86	µmol/l*s	
Alkaline phosphatase	20080727	3.64	µmol/l*s	
Alkaline phosphatase	20080806	3.81	µmol/l*s	
Alkaline phosphatase	20080805	3.7	µmol/l*s	
Alkaline phosphatase	20080729	4.82	µmol/l*s	
Aspartate aminotransferase	20080805	4.6	µmol/l*s	
Aspartate aminotransferase	20080829	0.49	µmol/l*s	
Aspartate aminotransferase	20080806	4.39	µmol/l*s	
Aspartate aminotransferase	20080727	4.0	µmol/l*s	
Aspartate aminotransferase	20080801	5.5	µmol/l*s	
Aspartate aminotransferase	20080729	5.7	µmol/l*s	
C-reactive protein	20080804	9.2	mg/l	
C-reactive protein	20080727	53.5	mg/l	
Creatinine	20080727	76	µmol/l	
Gamma GT	20080806	4.44	µmol/l*s	

Gamma GT	20080727	4.54	μmol/l*s
Gamma GT	20080829	0.67	μmol/l*s
Gamma GT	20080801	5.87	μmol/l*s
Gamma GT	20080805	4.3	μmol/l*s
LDH	20080805	8.6	μmol/l*s
LDH	20080727	9.0	μmol/l*s
Urea	20080727	2.1	mmol/l
Glutamate dehydrogenase	20080801	930	μmol/l*s
Glutamate dehydrogenase	20080727	867	μmol/l*s
Glutamate dehydrogenase	20080729	965	μmol/l*s
Total white blood cell count	20080806	11.8	Gpt/l
Total white blood cell count	20080804	15.7	Gpt/l
Total white blood cell count	20080802	16	Gpt/l
Total white blood cell count	20080731	13.1	Gpt/l
Total white blood cell count	20080727	6.5	Gpt/l

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 12.01.2009 DE-PVZIBET-JENA-10006

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 22.01.2009 DE-AVENTIS-200910120DE

Sender diagnosis: not reported

Narrative

Initial report: This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09003267); it was reported by a regional pharmacovigilance centre (identification number DE-PVZIBET-JENA-10006). The case involves a 33-year-old female patient with a medical history of pyrexial cold (from 22-Jul- to 26-Jul-2008). No information was provided on concomitant medication. The patient started treatment with paracetamol on 22-Jul-2008 for fever ("intake max. 2g/d; 500 mg, 1 separate dose"). On 25-Jul-2008 she added metamizole (no trade name provided) "500mg, 1 separate dose". On 26-Jul-2008 the patient developed (or was diagnosed to suffer from) increase in liver values. Paracetamol was discontinued on 27-Jul-2008, metamizole on 29-Jul-2008. At time of report (06-Aug-2008) outcome was reported as "recovering/resolving".

Causality assessment by reporter (method: Begaud Algorithm):

"possible" for metamizole and paracetamol

No drug interaction was reported.

Laboratory (no quantitative values provided):

27-Jul-2008: aspartate aminotransferase (ASAT) increased, alanine aminotransferase (ALAT) increased, glutamate dehydrogenase (GLUD) increase, gamma-GT (GGT) increased, alkaline phosphatase (AP) increased, LDH increased, urea decreased, creatinine normal, total white blood cell count increased, C-reactive increased

06-Aug-2008: ASAT increased, GGT increased, AP increased, total white blood cell count increased.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 17.03.2009 DE-AVENTIS-200910120DE

Sender diagnosis: not reported

Narrative

Initial report: This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09003267); it was reported by a regional pharmacovigilance centre (identification number DE-PVZIBET-JENA-10006). The case involves a 33-year-old female patient with a medical history of pyrexial cold (from 22-Jul- to 26-Jul-2008). No information was provided on concomitant medication. The patient started treatment with paracetamol on 22-Jul-2008 for fever ("intake max. 2g/d; 500 mg, 1 separate dose"). On 25-Jul-2008 she added metamizole (no trade name provided) "500mg, 1 separate dose". On 26-Jul-2008 the patient developed (or was diagnosed to suffer from) increase in liver values. Paracetamol was discontinued on 27-Jul-2008, metamizole on 29-Jul-2008. At time of report (06-Aug-2008) outcome was reported as "recovering/resolving".

Causality assessment by reporter (method: Begaud Algorithm):

"possible" for metamizole and paracetamol

No drug interaction was reported.

Laboratory (no quantitative values provided):

27-Jul-2008: aspartate aminotransferase (ASAT) increased, alanine aminotransferase (ALAT) increased, glutamate dehydrogenase (GLDH) increase, gamma-GT (GGT) increased, alkaline phosphatase (AP) increased, LDH increased, urea decreased, creatinine normal, total white blood cell count increased, C-reactive increased

06-Aug-2008: ASAT increased, GGT increased, AP increased, total white blood cell count increased.

Additional information received on 09-MAR-2009

Information by pharmacovigilance centre: On 22-Jul-2008 the patient developed feeling of illness with increased temperature of 37.5 degrees Celsius. She took 500mg paracetamol. The next day, she took 1500mg paracetamol, on 24-Jul-2008 1000mg, on 25-Jul-2008 500mg. On 25-Jul-2008 she was prescribed metamizole, leading to an improvement of her condition. In the night of 26/27-Jul-2008 the patient developed upper abdominal pain. On 27-Jul-2008, she was hospitalised; bed rest, infusion therapy, diet, metamizole (Novalgin) as needed (twice on 28-Jul-2008, once on 29-Jul-2008), paracetamol discontinued on 28-Jul-2008, treatment with ceftriaxone from 28-Jul-2008 to 03-Aug-2008 due to suspicion of bacterial cholangitis. On 29-Jul-2008 ERCP without pathologic findings. On 01-Aug-2008 liver biopsy. Tenderness on pressure in right upper abdomen until 29-Jul-2008, general malaise, improved

considerably since 30-Jul-2008. 28-Jul-2008 serology: CMV negative, EBV negative. 29-Jul-2008 ERCP: papille and duodenum unremarkable, intrahepatic bile ducts subtle, no caliber leaps, no concrements, biliary flow good, gallbladder free of stones. 05-Aug-2008 histopathology/diagnosis: the morphologic picture speaks for a chronic hepatitis with moderate inflammatory activity (grade 3 according to Desmet) and mild portal fibrosis (grade 1 according to Desmet). The material was processed immunohistochemically. Regular bile duct structures could be detected. No ductopenia, no destructing cholangitis; summarized, these findings (taking in consideration the clinical description) speak for a drug-toxic liver damage. Wilson's disease was ruled out, autoimmune antibodies negative; control examination planned for 22-Aug-2008.

Laboratory:

27-Jul-2008: urea 2.1 mmol/l (normal 2.6-6.7), C-reactive protein 53.5 mg/l (< 7.5), ASAT not done, ALAT 5.75 umol/l.s (< 0.56), GLDH not done, gamma-GT 4.54 umol/l.s (< 0.63), alkaline phosphatase 3.64 umol/l.s (< 2.0), LDH 9.08 umol/l.s (< 4.12), leukocytes 6.5 Gpt/l (4.4-11.3), MCH 1.75 fmol (1.74-2.05), thrombocytes 115 Gpt/l (150-360), protein in urine 149 mg/l (< 150)

28-Jul-2009: C-reactive protein 48.5 mg/l, ASAT not done, ALAT 6.47 umol/l.s, GLDH 796 nmol/l.s, gamma-GT 5.35 umol/l.s, leukocytes 7.6 Gpt/l, MCH 1.71 fmol, thrombocytes 120 Gpt/l, ferritin 419 ug/l (10-291)

29-Jul-2008: protein in urine 262mg

04-Aug-2008: C-reactive protein 9.2 mg/l, ASAT 2.47 umol/l.s (< 0.52), GLDH 287 nmol/l.s, LDH 8.63 umol/l.s

06-Aug-2008: ALAT 4.39 umol/l.s, gamma-GT 4.44 umol/l.s, alkaline phosphatase 3.81 umol/l.s, leukocytes 11.8 Gpt/l, MCH 1.69 fmol, thrombocytes 280 Gpt/l

Explanatory notes for causality assessment (see above): paracetamol was discontinued, metamizole continued as needed, last intake on 29-Jul-2008, afterwards decrease in liver values (increasing until 29-Jul-2008). Liver toxicity unknown for metamizole, but temporal relationship most likely, paracetamol not taken in overdose (normal up to 4800mg in this patient), taken previously and tolerated well, other alternative causes for increase in liver enzymes could not be found. Histopathological findings could be reconciled with clinical diagnosis. Relationship possible from clinico-pharmacological view, other caused could not be definitely excluded.

HELIOS Klinikum Wuppertal, 05.05.2010 DE-PVZIBE-JENA-10006

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberwerterhöhung unter Paracetamol, Metamizol

KURZZUSAMMENFASSUNG:

Leberwerterhöhung unter Paracetamol, Metamizol

BESCHREIBUNG:

stationäre Aufnahme wegen Oberbauchbeschwerden rechts. Seit 22.07.2008 Krankheitsgefühl mit erhöhter Temperatur von 37,5°C. Am 22.07. einmalig Paracetamol 500. Am 23.07.2008 1500 mg Paracetamol, am 24.07.2008 1000 mg, am 25.07.2008 500 mg. Am 25.07.2008 zum HA, Metamizol verordnet, daraufhin Fieber und Krankheitssymptome besser. In der Nacht vom 26/27.07.2008 Oberbauchschmerzen --->. Paracetamol schon früher genommen und vertragen. Sonst werden keine Medikamente eingenommen.

THERAPIE:

stationär seit 27.07. Bettruhe und Infusionstherapie, Schonkost

Novalgine bei Bedarf (28.07. 2x; 29.07. 1x)

27.07.2008 Paracetamol abgesetzt

28.07.2008 - 03.08.08 Rocephin 2g (V.a. bakterielle Cholangitis)

29.07.08 ERCP (ohne patholog. Befund)

01.08.08 Leberbiopsie

VERLAUF:

ZNA: Druckschmerz re Oberbauch

bis 29.07. allg. Krankheitsgefühl, seit 30.07. viel besser!

28.07.08 Serologie: CMV-neg; EBV-neg

29.07.08 ERCP: Papille und Duodenum unauff., intrahep. Gallengänge zart, keine Kalibersprünge. Keine Konkremente. Gallenabfluß gut. Gallenblase steinfrei.

05.08.2008 Histopatho: Diagnose: Das morphologische Bild entspricht einer chronischen Hepatitis mit mittelgradig entzündlicher Aktivität (Grad 3 nach Desmet) und geringer portaler Fibrose (Grad 1 nach Desmet). Ankündigungsgemäß wurde das Material immunhistochemisch aufgearbeitet. Unter Verwendung von Antikörpern gegenüber CK19 ließen sich regelrechte Gallengangsstrukturen nachweisen. Keine Ductopenie. Keine destruierende Cholangitis. Insgesamt entspricht der Befund in Kenntnis der klinischen Angaben für eine medikamentös-toxische Leberschädigung. Lt Arztbrief M. Wilson ausgeschlossen, Autoimmunantikörper neg.

Verlaufskontrolle für 22.08.2008 geplant.

ERLÄUTERUNGEN ZUR UAW:

33-jährige Patientin kommt bei akuten Oberbauchbeschwerden m. V.a. bakterielle Cholangitis (die sich nicht bestätigte) zur stat. Aufnahme. Die akuten Oberbauchbeschwerden rührten von einer Leberentzündung her, die klin. am ehesten mit Medikamenteneinnahme in Zusammenhang gebracht wurde.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Leberenzyme erhöht

ERLÄUTERUNG: 33-jährige Patientin kommt bei akuten Oberbauchbeschwerden m. V.a. bakterielle Cholangitis (die sich nicht bestätigte) zur stat. Aufnahme. Paracetamol wurde als lebertoxisch sofort abgesetzt, Novalgine b.B. weitergegeben. 29.07. letzte Novalgine-Einnahme, daraufhin begannen die Leberwerte zu sinken, während sie bis zum 29.07.08 steigend waren. Paracetamol war nicht überdosiert, sowie früher schon eingenommen (und vertragen) worden. Andere Ursachen für die Leberfermenterhöhung fanden sich nicht, histopathologisch konnte Befund mit klin. Diagnose in Einklang gebracht werden. Aus klin.-pharmakolog. Sicht ist Zusammenhang möglich, aber auch andere Ursachen nicht sicher ausgeschlossen. (S2)

MEDIKAMENT: METAMIZOLE

BETREFFENDE UAW: Leberenzyme erhöht

ERLÄUTERUNG: NW: keine Leberfunktionsstörungen in FI beschrieben + Micromedex. Obwohl von Klinikern nicht verdächtigt (Bedarfsmedikation stationär) muss das Metamizol aufgrund des zeitlichen Zusammenhangs verdächtigt werden. Erst nach Nicht-Mehr-Einnahme des Medikaments kam es zum Abfall der Leberwerte. Verlaufskontrolle ca drei Wochen nach Entlassung zeigt normale Leberwerte. Lt Pathologen ist in der Gesamtschau aller Befunde das morphologische Bild mit medikamentös-toxischer Leberschädigung vereinbar. <dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 18.05.2010 DE-SANOFI-AVENTIS-200910120DE

Sender diagnosis: not reported

Narrative

Initial report: This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09003267); it was reported by a regional pharmacovigilance centre (identification number DE-PVZIBET-JENA-10006). The case involves a 33-year-old female patient with a medical history of pyrexial cold (from 22-Jul- to 26-Jul-2008). No information was provided on concomitant medication. The patient started treatment with paracetamol on 22-Jul-2008 for fever ("intake max. 2g/d; 500 mg, 1 separate dose"). On 25-Jul-2008 she added metamizole (no trade name provided) "500mg, 1 separate dose". On 26-Jul-2008 the patient developed (or was diagnosed to suffer from) increase in liver values. Paracetamol was discontinued on 27-Jul-2008, metamizole on 29-Jul-2008. At time of report (06-Aug-2008) outcome was reported as "recovering/resolving".

Causality assessment by reporter (method: Begaud Algorithm):

"possible" for metamizole and paracetamol

No drug interaction was reported.

Laboratory (no quantitative values provided):

27-Jul-2008: aspartate aminotransferase (ASAT) increased, alanine aminotransferase (ALAT) increased, glutamate dehydrogenase (GLUD) increase,

gamma-GT (GGT) increased, alkaline phosphatase (AP) increased, LDH increased, urea decreased, creatinine normal, total white blood cell count increased, C-reactive increased

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28-Jul-2009: C-reactive protein 48.5 mg/l, ASAT not done, ALAT 6.47 umol/l.s, GLDH 796 nmol/l.s, gamma-GT 5.35 umol/l.s, leukocytes 7.6 Gpt/l, MCH 1.71 fmol, thrombocytes 120 Gpt/l, ferritin 419 ug/l (10-291)

29-Jul-2008: protein in urine 262mg

04-Aug-2008: C-reactive protein 9.2 mg/l, ASAT 2.47 umol/l.s (< 0.52), GLDH 287 nmol/l.s, LDH 8.63 umol/l.s

06-Aug-2008: ALAT 4.39 umol/l.s, gamma-GT 4.44 umol/l.s, alkaline phosphatase 3.81 umol/l.s, leukocytes 11.8 Gpt/l, MCH 1.69 fmol, thrombocytes 280 Gpt/l

Explanatory notes for causality assessment (see above): paracetamol was discontinued, metamizole continued as needed, last intake on 29-Jul-2008, afterwards decrease in liver values (increasing until 29-Jul-2008). Liver toxicity unknown for metamizole, but temporal relationship most likely, paracetamol not taken in overdose (normal up to 4800mg in this patient), taken previously and tolerated well, other alternative causes for increase in liver enzymes could not be found. Histopathological findings could be reconciled with clinical diagnosis. Relationship possible from clinico-pharmacological view, other caused could not be definitely excluded.

Additional information extracted from the website of the German health authority on 07-May-2010. Method of causality assessment was changed from Begaud algorithm to WHO score, but remained "possible" for metamizole sodium and paracetamol. Still neither trade name was provided for metamizole nor for paracetamol.

Case was closed, because further information was not expected by PV centre.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09011635

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19430828	Age: 65 Year	weight (kg): 92	BMI_int: 27.47	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 INR increased	20090112		1 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 600 acetylcysteine	Oral	600 Mg milligram(s)/Day	20090111-20090111	Cough
			1 Day	
2 Paracetamol-ratiopharm paracetamol	Oral	/	20090111-20090111	Fever
			1 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Artificial heart device user			Yes	
Common cold			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 29.01.2009 DE-RATIOPH-2009R0100502

Sender diagnosis: not reported

Narrative

Reported terms: INR increased(International normalised ratio increased)

Case reference number 2009R0100502 is a spontaneous case report sent by a pharmacist which refers to a male of unknown age.

There is no information about risk factors, concomitant diseasesn or prior drugs the patient has taken.

The patient started taking NAC-ratiopharm (ACETYLCYSTEINE), indication Common cold on an unknown date and Paracetamol-ratiopharm (PARACETAMOL), indication Common cold on an unknown date. The patient reported INR increased (International normalised ratio increased)(start date ?).

The following action was taken: for NAC-ratiopharm (ACETYLCYSTEINE) - Drug withdrawn for Paracetamol-ratiopharm (PARACETAMOL) - Drug withdrawn.

The outcome for INR increased (International normalised ratio increased) is Unknown.

Rechallenge information for NAC-ratiopharm is not applicable. Rechallenge information for Paracetamol-ratiopharm is not applicable. Dechallenge information for NAC-ratiopharm is not applicable. Dechallenge information for Paracetamol-ratiopharm is not applicable.

The reporter's assessment for drug Paracetamol-ratiopharm related with INR increased is as follows: Unclassifiable. The reporter's assessment for drug NAC-ratiopharm related with INR increased is as follows: Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

ratiopharm GmbH, 18.03.2009 DE-RATIOPH-2009R0100502

Sender diagnosis: not reported

Narrative

Reported terms: Rapid decrease of INR(International normalised ratio increased)

Case reference number 2009R0100502 is a spontaneous case report sent by a pharmacist which refers to a male aged 65 years.

The patient's past medical history included Artificial heart implant (Artificial heart device user). There is no information about risk factors. Concomitant diseases include Common cold with fever (Nasopharyngitis). No prior drugs are known to have been taken by the patient.

The patient started taking ACC 600 (ACETYLCYSTEINE), indication Cough on 11-Jan-2009 and Paracetamol-ratiopharm (PARACETAMOL), indication Fever on 11-Jan-2009. The patient developed Rapid decrease of INR (International normalised ratio increased)(start date 12-Jan-2009).

The following action was taken: for ACC 600 (ACETYLCYSTEINE) - Drug withdrawn for Paracetamol-ratiopharm (PARACETAMOL) - Drug withdrawn.

The outcome for Rapid decrease of INR (International normalised ratio increased) is Recovered / Resolved.

Rechallenge information for ACC 600 is not applicable. Rechallenge information for Paracetamol-ratiopharm is not applicable. Dechallenge information for ACC 600 is not applicable. Dechallenge information for Paracetamol-ratiopharm is not applicable.

The reporter's causality for the case is Unknown. The reporter's assessment for drug ACC 600 related with Rapid decrease of INR is as follows: Not applicable. The reporter's assessment for drug Paracetamol-ratiopharm related with Rapid decrease of INR is as follows: Not applicable.

CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09036964

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Female Birth: 19771226 Age: 31 Year weight (kg): 175 BMI_int: 280.4 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alanine aminotransferase increased	20090113				not recovered/not resolved
2 Alkaline phosphatase increased	20090113				not recovered/not resolved
3 Hepatic enzymes increased	20090114	20090119	6 Day	11 Day	recovering/resolving
4 Lactate dehydrogenase increased	20090113	20090119	7 Day		recovered/resolved
5 Transaminases increased	20090114	20090119	6 Day		recovering/resolving
6 Aspartate aminotransferase increased	20090113				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass "hexal" acetylsalicylic acid	Oral	100 Mg milligram(s)/ Day 245	200803-20090115 -	Coronary artery disease
2 EZETROL ezetimibe	Oral	10 Mg milligram(s)/Day 245	200803-20090114 -	Dyslipoproteinaemia
3 PARACETAMOL ABZ paracetamol	Oral	500 Mg milligram(s)/ 245	200808-20090114 -	Pain
4 ZOSTEX brivudine	Oral	125 Mg milligram(s)/Day 245	20090104-20090110 7 Day	Herpes zoster

Drugs (concomitant)

5 actrapid penfill insulin human	Subcutaneous	/ Day 120	1980- -	Type 1 diabetes mellitus
6 BLOPRESS candesartan cilexetil	Oral	16 Mg milligram(s)/Day 245	2000-20090114 -	Hypertension arterial
7 Calciumacetat Nefro calcium acetate	Oral	3*1400 Mg milligram(s)/ D 245	2008- -	Renal insufficiency
8 doss alfacalcidol	Oral	1 µg microgram(s)/ Week 5	2008- -	Osteoporosis
9 FOSRENOL SHIRE PHARMACEUTICALS lanthanum carbonate	Oral	3* /Day 8	200805- -	Renal insufficiency
10 PROTAPHAN PENFILL insulin human injection, isophane	Subcutaneous	2* /Day 120	1980- -	Type 1 diabetes mellitus
11 TORASEMID AAA-PHARMA torasemide	Oral	200 Mg milligram(s)/Day 245	2008- -	Renal insufficiency
12 XIPAMID STADA xipamide	Oral	20 Mg milligram(s)/Day 245	20080801- -	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial	2006		Yes	
Osteonecrosis			Yes	Beginn unbekannt; rechts
Osteoporosis			Yes	Beginn unbekannt
Pain NOS			Yes	Beginn unbekannt
Renal insufficiency	200711		Yes	dialysepflichtig
Smoker	1990		Yes	3 Zigaretten/Tag
Psoriasis vulgaris	200803		Yes	
Focal nodular hyperplasia	200809		Yes	Leber, OP 06/2008 und 12/2008
Osteosynthesis	20070601		Yes	bei Korrekturarthrodese Lisfranc I/II rechts
Renal anemia			Yes	Beginn unbekannt
Diabetic nephropathy	2007		Yes	
Dyslipoproteinaemia	2007		Yes	
Type 1 diabetes mellitus	1978		Yes	
Foot arthrodesis			Yes	
Liver fatty			Yes	
Transaminases increased			Yes	
Renal anaemia			Yes	
Herpes zoster		20090110	Yes	Beginn unbekannt
Drug allergy			Yes	Ciprobay; Beginn unbekannt
Coronary artery disease			Yes	Beginn unbekannt; 03/2008 Herzkatheter: 3 unkritische Stenosen
Charcot's syndrome			Yes	Beginn unbekannt; mit Osteonekrosen
Dialysis			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080915	0.73	NA	
Alanine aminotransferase	20090115	242	U/L	
Alanine aminotransferase	20090119	170	U/L	
Alanine aminotransferase	20090114	210	U/L	
Alanine aminotransferase	20081201	0.73	NA	
Alanine aminotransferase	20090116	273	U/L	
Alanine aminotransferase	20080211	0.68	NA	
Alanine aminotransferase	20080929	57.2	U/L	
Alkaline phosphatase	20090114	242	U/L	
Alkaline phosphatase	20090116	193	U/L	
Alkaline phosphatase	20090115	229	U/L	
Alkaline phosphatase	20090119	174	U/L	
Alkaline phosphatase	20080929	246	U/L	
Aspartate aminotransferase	20080915	0.84	NA	
Aspartate aminotransferase	20090114	307	U/L	
Aspartate aminotransferase	20090119	130	U/L	
Aspartate aminotransferase	20090115	357	U/L	

Aspartate aminotransferase	20081201	0.25	NA
Aspartate aminotransferase	20080929	83.7	U/L
Aspartate aminotransferase	20080211	0.51	NA
Aspartate aminotransferase	20090116	405	U/L
Bilirubin total	20090116	13,6	UMOL/L
Bilirubin total	20090119	9,4	UMOL/L
Bilirubin total	20080929	18.7	UMOL/L
Bilirubin total	20080211	4.2	UMOL/L
Blood pressure	20090114	96/39	MMHG
Body temperature	20090114	36.2	C
C-reactive protein	20090115	20.1	MG/L
C-reactive protein	20090116	17.7	MG/L
Fibrin D dimer	20090114	0.79	UG/L
Gamma GT	20090115	281	U/L
Gamma GT	20090114	317	U/L
Gamma GT	20080915	2.01	NA
Haematocrit	20090115	0.46	NA
Haemoglobin	20090115	9.8	MMOL/L
LDH	20090115	463	U/L
LDH	20090114	486	U/L
LDH	20090116	395	U/L
LDL	20090119	232	U/L
Oxygen saturation	20090114	97	%
Pulse rate	20090114	100	NA

Narratives (nur bei elektronischem Eingang)

Medizinische Fakultät der Universität Rostock, 24.03.2009 DE-PVZIBE-ROSTOCK-10118

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Transaminasenanstieg

KURZZUSAMMENFASSUNG:

Transaminasenanstieg unter Brivudin, Paracetamol, Ezetimib, ASS 100, (Candesartan)

BESCHREIBUNG:

seit gestern (13.01.09) Übelkeit, 1xErbrechen, Schwindel, Luftnot, Engegefühl;
heute (14.01.09) bei Dialyse Verschlechterung der Symptome, Einweisung durch Nephrologen bei Verdacht auf Pneumonie;
bei Aufnahme: Blutdruck 96/39mmHg, Puls 100/min, SpO2 97%, Temperatur 36.2°C, erhöhte Transaminasen und D-Dimer, daraufhin stationäre Aufnahme;
anamnestisch: diskreter Transaminasenanstieg seit 1 Jahr bekannt, Hepatitis wurde ausgeschlossen

THERAPIE:

derzeit pausiert: Paracetamol, Ezetrol, Brivudin

VERLAUF:

14.01.09 Röntgen-Thorax: kein Hinweis für Pneumonie;
14.01.09 Lungenszintigraphie: kein Nachweis einer Lungenembolie;
15.01.09 Sonografie Abdomen: chronische Nierenparenchymschädigung, Zustand nach Cholezystektomie, Hepatosplenomegalie mit Leberparenchymveränderungen wie bei Steatosis hepatis, bekannte fokale noduläre Hyperplasie;
16.01.09 ausführliche Immundiagnostik: keine Hinweise auf autoimmunologische Genese;
16.01.09 ausführliche mikrobiologische-/virologische Diagnostik: kein Hinweis auf infektiöse Genese

ERLÄUTERUNGEN ZUR UAW:

Die sttionäre Einweisung erfolgte bei allgemeiner Zustandsverschlechterung mit dem zunächst geäußerten Verdacht auf Pneumonie (konnte nicht bestätigt werden); auffällig waren aber fast 10 fach erhöhte Transaminasen
Krankenhausaufenthalt: 14.-21.01.2009;

Nach Ausschluss einer infektiösen und autoimmunologischen Genese des seit Januar 2009 extremen Anstieges der Transaminasen, steht der Verdacht auf eine arzneimitteltoxische Genese unter Brivudin (zeitlicher Zusammenhang am deutlichsten). Aber auch für Paracetamol, Ezetimib, ASS und Candesartan (zeitlich am unwahrscheinlichsten) lässt sich ein Summationseffekt nicht ausschließen. Als Risikofaktor besteht bei der Patientin eine Steatosis hepatis mit bekannter fokaler Hyperplasie, sowie eine kürzlich durchgemachte Herpes zoster Erkrankung.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: BLOPRESS

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Das Auftreten von Leberfunktionsstörungen ist unter Candesartan beschrieben. Die zeitliche Komponente spricht in diesem Fall eher dagegen, obwohl man einen Zusammenhang nicht völlig ausschließen kann. Insgesamt wird die Einnahme mehrerer potentiell leberschädigender Arzneimittel als ursächlich bewertet.

MEDIKAMENT: ASS "HEXAL"

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Fachinfo: Erhöhung der Transaminasen ist beschrieben. Eine Mitbeteiligung an der Leberschädigung lässt sich nicht ausschließen. Insgesamt wird die Einnahme mehrerer potentiell leberschädigender Arzneimittel als ursächlich bewertet. Nach Absetzen fielen die Transaminasen deutlich.

MEDIKAMENT: XIPAMID STADA

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Fachinfo: Ikterus; zeitlicher Zusammenhang hier unwahrscheinlich

MEDIKAMENT: EZETROL

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Die Beeinträchtigung der Leberfunktion ist eine beschriebene UAW. Insgesamt wird aber die Einnahme mehrerer potentiell leberschädigender Arzneimittel als ursächlich bewertet. Nach Absetzen fielen die Transaminasen deutlich.

MEDIKAMENT: PARACETAMOL ABZ

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Die Beeinträchtigung der Leberfunktion ist eine beschriebene UAW. Insgesamt wird aber die Einnahme mehrerer potentiell leberschädigender Arzneimittel als ursächlich bewertet. Nach Absetzen fielen die Transaminasen deutlich.

MEDIKAMENT: ZOSTEX

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Der zeitliche Verlauf zwischen der Einnahme von Zostex und dem Auftreten der UAW scheint am wahrscheinlichsten. Die Beeinträchtigung der Leberfunktion ist eine beschriebene UAW. Insgesamt wird aber die Einnahme mehrerer potentiell leberschädigender Arzneimittel als ursächlich bewertet. Nach Absetzen fielen die Transaminasen deutlich.

009507513, 31.03.2009 DE-MERCK-0903DEU00187

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 31-year-old female patient with liver fatty, coronary artery disease, pain, type 1 diabetes mellitus, hypertension arterial, renal insufficiency, osteoporosis, smoker, diabetic nephropathy, renal anaemia, charcot's syndrome, osteonecrosis, focal nodular hyperplasia, psoriasis vulgaris and dialysis and a history of herpes zoster and liver operation who, in March 2008, was placed on therapy with ezetimibe, 10 mg tablet, 10 mg, once a day for the treatment of dyslipoproteinaemia (exact duration not reported). Concomitant suspect therapy included aspirin (ASS HEXAL), 100 mg tablet, 100 mg daily for the treatment of coronary artery disease (therapy initiated in March 2008); acetaminophen, 500 mg tablet for the treatment of pain (total daily dose not reported - therapy initiated in August 2008); brivudine (ZOSTEX), 125 mg tablet, 125 mg daily for the treatment of herpes zoster (therapy initiated on 04-JAN-2009 until 10-JAN-2009). Other concomitant therapy included insulin, neutral (ACTRAPID), candesartan cilexetil (BLOPRESS), Ca acetate, alfacalcidol (DOSS (alfacalcidol)), lanthanum carbonate (FOSRENOL), insulin human, isophane (PROTAPHAN), torsemide and xipamide.

On 13-JAN-2009, the patient developed nausea, vomiting, dizziness, dyspnoea and chest tightness. On 14-JAN-2009, during dialysis, the symptoms got worse and the patient was hospitalized by the nephrologist due to suspicion of pneumonia. Upon admission, chest x-ray showed no evidence of pneumonia. Laboratory analysis revealed increased AST 307 U/l; increased ALT 210 U/l; increased AP 242 U/l and increased LDH 486 U/l. On 14-JAN-2009, therapies with ezetimibe and acetaminophen were discontinued. On 15-JAN-2009, therapy with aspirin (ASS HEXAL) was discontinued. On 16-JAN-2009, AST was 405 U/l and ALT was 273 U/l, no evidence of autoimmune genesis and no evidence of infectious genesis was found. Medicamentous toxic genesis in the context of brivudine (ZOSTEX) intake was suspected. On 19-JAN-2009, AST was 130 U/l, ALT was 170 U/l and

AP was 174 U/l. On 19-JAN-2009, the patient recovered from increased LDH (232 U/l). On 11-FEB-2009, AST was 0.51 mcmol/sl and ALT was 0.68 mcmol/sl. The patient's increased AST, ALT and AP persisted.

The reporting physician felt that increased ALT, AST, AP and LDH were possibly related to therapy with ezetimibe and/or aspirin (ASS HEXAL) and/or acetaminophen and likely related to therapy with brivudine (ZOSTEX).

Additional information is not expected.

009507513, 03.04.2009 DE-MERCK-0903DEU00187

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 31-year-old female patient with liver fatty, coronary artery disease, pain, type 1 diabetes mellitus, hypertension arterial, renal insufficiency, osteoporosis, smoker, diabetic nephropathy, renal anaemia, charcot's syndrome, osteonecrosis, focal nodular hyperplasia, psoriasis vulgaris and dialysis and a history of herpes zoster and liver operation who, in March 2008, was placed on therapy with ezetimibe, 10 mg tablet, 10 mg, once a day for the treatment of dyslipoproteinaemia (exact duration not reported). Concomitant suspect therapy included aspirin (ASS HEXAL), 100 mg tablet, 100 mg daily for the treatment of coronary artery disease (therapy initiated in March 2008); acetaminophen, 500 mg tablet for the treatment of pain (total daily dose not reported - therapy initiated in August 2008); brivudine (ZOSTEX), 125 mg tablet, 125 mg daily for the treatment of herpes zoster (therapy initiated on 04-JAN-2009 until 10-JAN-2009). Other concomitant therapy included insulin, neutral (ACTRAPID), candesartan cilexetil (BLOPRESS), Ca acetate, alfacalcidol (DOSS (alfacalcidol)), lanthanum carbonate (FOSRENOL), insulin human, isophane (PROTAPHAN), torsemide and xipamide.

On 11-FEB-2008 (prior to therapy start with ezetimibe), AST was 0.51 mcmol/sl and ALT was 0.68 mcmol/sl.

On 13-JAN-2009, the patient developed nausea, vomiting, dizziness, dyspnoea and chest tightness. On 14-JAN-2009, during dialysis, the symptoms got worse and the patient was hospitalized by the nephrologist due to suspicion of pneumonia. Upon admission, chest x-ray showed no evidence of pneumonia. Laboratory analysis revealed increased AST 307 U/l; increased ALT 210 U/l; increased AP 242 U/l and increased LDH 486 U/l. On 14-JAN-2009, therapies with ezetimibe and acetaminophen were discontinued. On 15-JAN-2009, therapy with aspirin (ASS HEXAL) was discontinued. On 16-JAN-2009, AST was 405 U/l and ALT was 273 U/l, no evidence of autoimmune genesis and no evidence of infectious genesis was found. Medicamentous toxic genesis in the context of brivudine (ZOSTEX) intake was suspected. On 19-JAN-2009, AST was 130 U/l, ALT was 170 U/l and AP was 174 U/l. On 19-JAN-2009, the patient recovered from increased LDH (232 U/l). The patient's increased AST, ALT and AP persisted.

The reporting physician felt that increased ALT, AST, AP and LDH were possibly related to therapy with ezetimibe and/or aspirin (ASS HEXAL) and/or acetaminophen and likely related to therapy with brivudine (ZOSTEX).

Additional information is not expected.

HELIOS Klinikum Wuppertal, 06.11.2009 DE-PVZIBE-ROSTOCK-10118

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Transaminasenanstieg

KURZZUSAMMENFASSUNG:
Transaminasenanstieg unter Brivudin, Paracetamol, Ezetimib, ASS 100, (Candesartan)

BESCHREIBUNG:
seit gestern (13.01.09) Übelkeit, 1x Erbrechen, Schwindel, Luftnot, Engegefühl;
heute (14.01.09) bei Dialyse Verschlechterung der Symptome, Einweisung durch Nephrologen bei Verdacht auf Pneumonie;
bei Aufnahme: Blutdruck 96/39 mmHg, Puls 100/min, SpO2 97%, Temperatur 36.2°C, erhöhte Transaminasen und D-Dimer, daraufhin stationäre Aufnahme;
anamnestisch: diskreter Transaminasenanstieg seit 1 Jahr bekannt, Hepatitis wurde ausgeschlossen (zuletzt am 01.12.2008 HBs AG negativ, Anti HBs <10 IE/l, Hepatitis C AK negativ)

THERAPIE:
derzeit pausiert: Paracetamol, Ezetrol, Brivudin

VERLAUF:
14.01.09 Röntgen-Thorax: kein Hinweis für Pneumonie;
14.01.09 Lungenszintigraphie: kein Nachweis einer Lungenembolie;

15.01.09 Sonografie Abdomen: chronische Nierenparenchymschädigung, Zustand nach Cholezystektomie, Hepatosplenomegalie mit Leberparenchymveränderungen wie bei Steatosis hepatis, bekannte fokale noduläre Hyperplasie;
 16.01.09 ausführliche Immundiagnostik: keine Hinweise auf autoimmunologische Genese, 16.01.09 ausführliche mikrobiologische-/virologische Diagnostik: kein Hinweis auf infektiöse Genese;
 (negativ: Ro-52-Antikörper, Antikörper gegen AMA-M2, 3E(BPO) Antikörper, SP100 Antikörper, PML Antikörper, gp210 Antikörper, LKM-1 (Cytochrom P450 2D6) Antikörper, Lebercytosol Antikörper, SLA/LP (lösliches Leber-Pankreas-Antigen), Parietalzell Antikörper, Intrinsic Faktor; unauffälliger Befund: Borrelien-Antikörper, Toxoplasmose-Antikörper, Chlamydia psittaci-Antikörper, Mycoplasma pneumoniae-Antikörper, Legionella pneumophila-Antikörper, Bordetella pertussis-Antikörper, Yersinien-Antikörper, Leptospiren-Antikörper, Brucellen-Antikörper)

Die Entlassung erfolgte bei gebesserem Allgemeinzustand und rückläufigen Transaminasen. Eine Rückmeldung des weiteren ambulanten Verlaufs war nicht erhältlich.

ERLÄUTERUNGEN ZUR UAW:

Die stationäre Einweisung erfolgte bei allgemeiner Zustandsverschlechterung mit dem zunächst geäußerten Verdacht auf Pneumonie (konnte nicht bestätigt werden, Diagnose: Hyperventilationssyndrom wurde gestellt); auffällig waren aber fast 10-fach erhöhte Transaminasen, die die tatsächliche stationäre Aufnahme bedingten.

Krankenhausaufenthalt: 14.-21.01.2009;

Nach weitgehendem Ausschluss einer infektiösen und autoimmunologischen Genese des seit Januar 2009 extremen Anstieges der Transaminasen, steht der Verdacht auf eine arzneimitteltoxische Genese unter Brivudin (zeitlicher Zusammenhang am deutlichsten). Brivudin war bis wenige Tage vor der stationären Aufnahme wegen eines Zosters eingenommen worden. Aber auch für Paracetamol, Ezetimib, ASS und Candesartan (zeitlich am unwahrscheinlichsten) lässt sich ein Summationseffekt nicht ausschließen. Als Risikofaktor besteht bei der Patientin eine Steatosis hepatis mit bekannter fokaler Hyperplasie, sowie eine kürzlich durchgemachte Herpes zoster Erkrankung. Varicella Zoster Virus, Zytomegalie Virus und Epstein Barr Virus als hepatotrope Viren wurden im Rahmen der umfangreichen Diagnostik nicht bestimmt und stellen somit potentiell eine Alternativursache dar. Die Zoster-Erkrankung, die zur Einnahme von Brivudin geführt hatte, war zum Zeitpunkt der stationären Aufnahme weitgehend abgeheilt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: BLOPRESS

BETREFFENDE UAW: Transaminasen erhöht

ERLÄUTERUNG: Das Auftreten von Leberfunktionsstörungen ist unter Candesartan beschrieben. Die zeitliche Komponente spricht in diesem Fall eher dagegen, obwohl man einen Zusammenhang nicht völlig ausschließen kann. Candesartan wurde nicht abgesetzt und unter weiter erfolgter Gabe trat eine Besserung ein. Aus diesem Grunde ist eine Beteiligung von Candesartan zwar prinzipiell möglich, wird in diesem Fall aber als unwahrscheinlich angesehen. Insgesamt wird die Einnahme mehrerer potentiell toxischer Substanzen im Zusammenhang mit der stationären Aufnahme gesehen.

009507513, 13.11.2009 DE-MERCK-0903DEU00187

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 31-year-old female patient (smoker) under dialysis and with fatty liver, coronary artery disease, pain, type 1 diabetes mellitus, arterial hypertension, renal insufficiency, osteoporosis, diabetic nephropathy, renal anemia, Charcot's syndrome, osteonecrosis, osteosynthesis, foot arthrodesis, ciprofloxacin allergy, psoriasis vulgaris, herpes zoster (onset date unknown) and discretely increased transaminases (since approximately January 2008; hepatitis virus infection ruled out) who, in March 2008, was placed on therapy with ezetimibe, 10 mg tablet, 10 mg, once a day for the treatment of dyslipoproteinemia (exact duration not reported). Concomitant suspect therapy included aspirin (ASS HEXAL), 100 mg tablet, 100 mg daily for the treatment of coronary artery disease (therapy started in March 2008; exact duration not reported), acetaminophen, tablet, 500 mg, once a day for the treatment of pain (therapy started in August 2008; exact duration not reported), brivudine (ZOSTEX), 125 mg tablet, 125 mg, once a day for the treatment of herpes zoster (therapy started on 04-JAN-2009). Other concomitant therapy included insulin, neutral (ACTRAPID), candesartan cilexetil (BLOPRESS), Ca acetate, alfacalcidol (DOSS), lanthanum carbonate (FOSRENOL), insulin human isophane (PROTAPHAN), torsemide and xipamide.

In June 2008 and December 2008, the patient underwent liver operation in the context of focal nodular hyperplasia (onset date reported as "September 2008").

On 10-JAN-2009, therapy with brivudine was stopped as the patient's herpes zoster was extensively healed.

On 13-JAN-2009, the patient experienced nausea, dizziness and chest tightness and developed vomiting and dyspnea. On 14-JAN-2009, during dialysis, the patient's symptoms worsened and she was sent to hospital by the nephrologist due to deteriorated general condition and suspicion of pneumonia. At hospital, chest X-ray showed no evidence of pneumonia. Pulmonary radionuclear scan showed no evidence of pulmonary embolism. The suspected pneumonia was not confirmed and the diagnosis of hyperventilation syndrome was made. Laboratory analysis revealed AST (307 U/l; previous value on 11-FEB-2008: 0.51 mcml/sl), ALT (210 U/l; previous value on 11-FEB-2008: 0.68 mcml/sl), alkaline phosphatase (AP) 242 U/l and LDH 486 U/l. The almost 10-fold increase of transaminases was conspicuous and the reason the patient was actually hospitalized. That same day, therapy with ezetimibe and acetaminophen was discontinued. On 15-JAN-2009, therapy with aspirin was discontinued.

On 16-JAN-2009, laboratory analysis revealed AST 405 U/l and ALT 273 U/l. Thorough immunologic diagnostic work-up revealed no evidence of an autoimmunologic origin. Thorough microbiologic/virologic diagnostic work-up (exclusive of tests for varicella zoster virus, cytomegalia virus and Epstein Barr virus) revealed no evidence of an infectious origin. A drug-toxic origin in the context of brivudine intake was suspected. On 19-JAN-2009,

laboratory analysis revealed AST 130 U/l , ALT 170 U/l,AP 174 U/l and LDH 232 U/l. The patient was recovering from increase of transaminases. On 21-JAN-2009, the patient was discharged from hospital.

The reporting physician felt that increase of transaminases was possibly related to therapy with ezetimibe and/or aspirin and/or acetaminophen and likely related to therapy with brivudine. Since tests for varicella zoster virus, cytomegaly virus and Epstein Barr virus as hepatotropic viruses were not included in the thorough diagnostic work-up they were considered as a potential alternative cause. Increase of transaminases was the only event coded by the agency.

Additional information is not expected.

MENARINIP, 20.11.2009 DE-MEN-13706

Sender diagnosis: not reported

Narrative

This case, initially reported directly to BfArM and then published as an ICH-E2B-compliant ICSR on its website, involved a 31-year-old female patient, who developed transaminases increased. The patient started treatment with 125 mg/day brivudin on 04-Jan-2009 for a normal treatment cycle of 7 days.

Concomitantly, she received acetylsalicylic acid (ASS 100 Hexal), ezetimibe (Ezetrol), paracetamol (PCM ABZ), Calciumacetat Nefro, vitamin D derivative (DOSS), lanthancarbohydrazat (Fosrenol Shire Pharmaceuticals), insulin human injection, Isophane (Protaphan Penfill), torasemide (Torasemid AAA-Pharma), xipamide (Xipamid stada), insulin human (Actrapid Penfill) and candesartan cilexetil (Blopess). The patient's medical history included type 1 diabetes mellitus since 1978, diabetic nephropathy since 2007, renal insufficiency requiring dialysis since Nov-2007, renal anemia, hypertension arterial, coronary artery disease, charcot's syndrom, osteonecrosis, focal nodular hyperplasia since Sep-2008, dyslipoproteinaemia since 2007, psoriasis vulgaris since Mar-2008, osteoporosis and pain.

On 14-Jan-2009 her general condition got worse during dialysis and she was hospitalised due to suspicion of pneumonia. A laboratory test showed a tenfold increase of transaminases. A discrete increase of transaminases was still known for a year. Hepatitis was excluded. Thoracic radiographs were made on 14-Jan-2009 and showed no evidence of pneumonia. A pulmonary scintigraphy substantiated that no pulmonary embolism was present. Additionally a steatosis hepatis diagnosed, but no hepatitis. Acetylsalicylic acid, ezetimibe and paracetamol were discontinued. The patient was discharged on 21-Jan-2009, but the event was not resolved. The reporter stated the event as probable related to the intake of brivudin and as possible related to the use of ASA, paracetamol and ezetimibe.

Follow-up information received on 09-NOV-2009:

Trought a Quality Control Review, based on the collected data showing, other than the already reported increase of transaminases, also the increase of GGT, LDH and Alkaline phosphatase, the previous event "transaminases increased" was changed in "hepatic enzymes increased".

The outcome of the event was reported as resolving: the end date of the event was reported to be 19-JAN-2009 with a duration of five days.

The patient was discharged in improved general status with declining transaminases.

Information regarding outpatient follow up of the event was not available.

Additional laboratory findings were also available.

Osteosynthesis since 01-Jun-2007 and drug allergy to ciprofloxacin (Ciprobay) was included in the patient's medical history.

The reporter stated that potential infections with hepatotropic viruses, e.g. Cytomegalo-, Epstein-Barr- or reactivated Varicella-Zoster-Virus, had not been excluded within diagnostic evaluation and therefore considered as possible alternative cause of the event.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09051313

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19520702	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic necrosis	20090330			1 Month	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 FLUPIRTINE flupirtine	Oral	800 Mg milligram(s)/Day	-	-
2 PARACETAMOL paracetamol	Oral	/	200902-	-

Drugs (concomitant)

3 captopril captopril	Oral	50 Mg milligram(s)/Day	-
4 celecoxib celecoxib	Oral	/Day	-
5 levothyroxine levothyroxine	Oral	75 µg microgram(s)/Day	-
6 tilidine hydrochloride tilidine hydrochloride	Oral	2* /Day	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Common cold	200902		Unknown	
Chronic pain			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Liver biopsy		NA	NA	

Narratives (nur bei elektronischem Eingang)

TEVA, 21.04.2009 DE-TEVA-191998ISR

Sender diagnosis: not reported

Narrative

16-Apr-2009, Spontaneous Serious Report, Forwarded by Teva Germany(Local #FLPDS-09-25).

A Physician reported the case of a patient (sex and age not specified)who experienced TOXIC HEPATIC NECROSIS while on FLUPIRTINE (KATADOLON).

A physician reported to the company that a patient (currently no further information available) developed a toxic hepatic necrosis. The patient has been taken Katadolon for a longer time (not further specified) and Paracetamol since 3 days. Further information will be requested.

This case was considered serious based on the following criteria:Medically significant

Teva Comment: Case unassessable due to insufficient information.

Changes in the statements of the German SmPC and PL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:TEVA, 11.05.2009 DE-TEVA-191998ISR

Sender diagnosis: not reported

Narrative

16-Apr-2009, Spontaneous Serious Report, Forwarded by Teva Germany(Local#FLPDS-09-25).

A Physician reported the case of a patient (sex and age not specified)who experienced TOXIC HEPATIC NECROSIS while on FLUPIRTINE (KATADOLON).

A physician reported to the company that a patient (currently no further information available) developed a toxic hepatic necrosis. The patient has been taken Katadolon for a longer time (not further specified) and Paracetamol since 3 days. Further information will be requested.

This case was considered serious based on the following criteria:Medically significant

Teva Comment: Case unassessable due to insufficient information.

30-Apr-2009:

Paracetamol was changed to be a suspect drug (Teva product can not be excluded).

Upon request the physician updated the company with patient's data (initials, date of birth, age, sex) and gave further information on the medication and underlying disease (chronic pain syndrome of the lumbar spine). In the course of a hospital stay for multi-modal pain therapy noticeable liver values were detected. Histology after a liver biopsy showed a finding of 3 necrotic areas that most likely might be toxic drug-induced.

In -FEB-2009 (date in conflict to initial report) the patient took Paracetamol because of a common cold. The physician suspects Katadolon and Paracetamol to have caused the AE and assessed the causality as probably related to both drugs. The patient's condition did not improve after discontinuation of Katadolon and the symptoms are still ongoing. Paracetamol had already been discontinued some time ago. Further information was not provided.

This case was considered serious based on the following criteria:Hospitalization

Teva Comment: Possibly related to katadolone. Paracetamol unassessable, no details given on its dose.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09074418

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute generalized exanthematous pustulosis	2009		6 Week	5 Day	recovered/resolved
2 Acute generalised exanthematous pustulosis			6 Week	3 Day	recovered/resolved
3 Hepatomegaly	2009		6 Week	3 Day	recovered/resolved
4 Fever					recovered/resolved
5 Mucosal erosion			6 Week	3 Day	recovered/resolved
6 C-reactive protein increased	2009			3 Day	recovered/resolved
7 Exanthema				3 Day	recovered/resolved
8 Leukocytosis	2009			3 Day	recovered/resolved
9 Neutrophilia				3 Day	recovered/resolved
10 Hypersensitivity reaction					recovered/resolved
11 Liver disorder					recovered/resolved
12 Alanine aminotransferase increased	2009			3 Day	recovered/resolved
13 GGT increased	2009				recovered/resolved
14 Acanthosis				3 Day	Unknown
15 Gamma-glutamyltransferase increased				3 Day	Unknown
16 Pustule				3 Day	Unknown
17 Skin lesion				3 Day	Unknown
18 ALT increased					recovered/resolved
19 GGTP increase					recovered/resolved
20 Elevated liver enzyme levels	2009		6 Week	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN amoxicillin	Unknown	/	- 3 Day	Otitis media
2 Paracetamol paracetamol	Unknown	/	- 5 Day	Otitis media

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Otitis media			Unknown	
Psoriasis			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Derm Venereol 2009; 89: 314-5	
Qualification:	Other Health Professional	observestudytype:
Literature:	Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Derm Venereol 2009 Jan 1;89:314-5.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 2009 Jan 1;89:314-5.	
Qualification:	Physician	observestudytype:
	Treudler R. et al.	

Literature: Prolonged Course of Acute Generalised Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Dermato-Venereologica 2009; 89: 314-315.

Qualification: Other Health Professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		2 - 3 fold	NA	
Body temperature		more than 39	C	
C-reactive protein		250	mg/l	
Eosinophils		NA	NA	
GGT		2 - 3 fold	NA	
Hepatic enzyme		NA	NA	
Histology		NA	NA	
Interleukin level		less than 2.5	ng/l	
Neutrophils		87	%	
Skin patch test		5	%	
Sonogram		NA	NA	
Virus serology test		NA	NA	
WBC count		25	10 ⁹ /l	
Leukocyte count NOS		25/nl	NA	
ALANINE AMINOTRANSFERASE INCRE		NA	NA	
c-reactive protein		250	MG/L	
GGT INCREASED		NA	NA	
INTERLEUKIN LEVEL		<2.5	PG/ML	
LEUKOCYTOSIS		25	NL	
neutrophilia		87	%	
Body temperature		>39	degC	
C-reactive protein		250	mg/L	
Leukocyte count NOS		25	1/nl	
Neutrophils		87	Percent	
C-reactive protein		250	MG/L	
Neutrophils		87	%	
Leukocyte count NOS		25	/NL	
Patch test positive		positive		
Patch test positive		positive		
Prick test		positive		

Narratives (nur bei elektronischem Eingang)

JUTA, 11.06.2009 DE-JUTA-1103-09

Sender diagnosis: not reported

Narrative

A 48-year-old male patient developed acute generalized exanthematous pustulosis and raised liver function tests during treatment with amoxicillin and paracetamol. 5 days after the start of paracetamol and 3 days after the start of amoxicillin the patient developed exanthema, which was progressive despite immediate cessation of both drugs. Histology revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate. Skin lesions were accompanied by fever, leukocytosis, neutrophilia and increased C-reactive protein. After further 3 days GGT and ALT increased two- to three-fold. Ultrasound revealed mild hepatomegaly. Patch tests for amoxicillin and paracetamol were positive. With medical treatment skin lesions, together with hepatomegaly and laboratory abnormalities improved.

explanation. No further measures are considered to be necessary.

Reporter's comment:

BETA, 16.06.2009 DE-BETA-19465-09L

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 16.06.2009 DE-PFLEGER-000067

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.<dbtext>To_Char_NARRATIVE_2</dbtext>

Dr. R. Pflieger Chemische Fabrik GmbH, 16.06.2009 DE-PFLEGER-000067

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment.

Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

CT-Arzneimittel GmbH, 17.06.2009 DE-CT-2009CT0605704

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement.

The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Cases concerning AGEF associated with Paracetamol use are available in the literature. In most of these cases, Paracetamol was the only suspected drug. AGEF is characterized by fever and generalized scarlatiniform erythema with small, sterile, nonfollicular pustules. "Hypersensitivity reactions, ranging from erythema to urticaria and anaphylactic shock" is listed as adverse effect of Paracetamol in the RSI. No actions concerning the wording of the RSI are regarded to be necessary based on current data.

Reporter's comment:

MENARINIP, 17.06.2009 DE-MEN-14305

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 48-year-old male patient with otitis media, who was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Reporter's comment:

Molimin Arzneimittel GmbH, 18.06.2009 DE-MOLIMIN-000017

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: not probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: co-suspect drug amoxicillin, REACTION ABATE AFTER STOPPING DRUG: Not applicable.<dbtext>To_Char_NARRATIVE_2</dbtext>

Krewel Meuselbach GmbH, 18.06.2009 DE-KREWEL-001510

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

NOTE: Other risk factor: co-suspect drug amoxicillin.<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

MEDA, 19.06.2009 DE-MEDA-V-DE-2009-5216

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-401590)- literature search project VIGILIT on 05-Jun-2009.

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged Course of Acute Generalized Exanthematous Pustulosis with Liver Involvement due to Sensitization to Amoxicillin and Paracetamol. Acta Derm Venereol 2009; 89: 314-5

CAUSALITY ASSESSMENT FOR AMOXICILLIN.

Temporal Relationship: plausible

Pharmacological Relationship: plausible

Observed reaction is a Sign/Symptom of Underlying Disease: not probable

History of Observed Event without drug involvement: no
 Other Risk Factors: probable
 Sufficiency of Documentation: sufficient
 OVERALL CAUSALITY ASSESSMENT: possible
 NOTE: Other risk factor: co-suspect drug paracetamol
 REACTION ABATE AFTER STOPPING DRUG: Not applicable

Reporter's comment:

The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

AbZ Pharma GmbH, 19.06.2009 DE-ABZ-2009A0605705

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

cases concerning AGEP associated with Paracetamol use are available in the literature. In most of these cases, Paracetamol wasn't the only suspected drug. AGEP is characterized by fever and generalized scarlatiniform erythema with small, sterile, nonfollicular pustules. "Hypersensitivity reactions, ranging from erythema to urticaria and anaphylactic shock" is listed as adverse effect of Paracetamol in the RSI. No actions concerning the wording of the RSI are regarded to be necessary based on current data.

Reporter's comment:

ratiopharm GmbH, 19.06.2009 DE-RATIOPH-2009R0605642

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

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Reporter's comment:

SANOFI-SYNTHELABO, 22.06.2009 DE-SANOFI-SYNTHELABO-A04200900810

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthematous pustulosis
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: elevated liver enzyme levels (2 to 3-fold)
LLT: ELEVATED LIVER ENZYME LEVELS / 10014480
PT: HEPATIC ENZYME INCREASED / 10060795

Reaction as reported: hepatomegaly
LLT: HEPATOMEGALY / 10019842
PT: HEPATOMEGALY / 10019842

Reaction as reported: oral erosions
LLT: ORAL ULCERATION / 10031027
PT: MOUTH ULCERATION / 10028034

Reaction as reported: fever (>39 degrees C)
LLT: FEVER / 10016558
PT: PYREXIA / 10037660

Narrative:

Initial report:

This serious case is a literature report about acute generalized exanthematous pustulosis under amoxicillin and paracetamol in a male.
Case report: A 48-year-old male patient with no history of psoriasis was treated with paracetamol and amoxicillin (no trade names provided) due to otitis media and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with elevated liver enzyme levels (nos) and hepatomegaly. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever >39 degrees C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The adverse reactions reappeared after reintroduction (details were not provided). The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

Addendum for follow-up received 15-Jun-2009

The full article was received and the case report could be amended as follows:

The patient was admitted to hospital on day six after start of therapy and a therapy with prednisolone-succinate (250 mg/day for two days) was initiated. On day seven he was referred to the reporters university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes.

On day 8, eosinophils and liver enzymes were within normal range but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to threefold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative.

Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids.

The positive patch tests with both drugs (5% in petrolatum) revealed papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution).

The patient was advised to avoid both drugs in the future.

Temmler Pharma GmbH & Co. KG, 23.06.2009 DE-TEMMLER06-003457

Sender diagnosis: not reported

Narrative

A 48-year-old male patient with otitis media was treated with paracetamol and amoxicillin and developed a prolonged course of acute generalised

exanthematous pustulosis (AGEP) with liver involvement. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol.

Sender's comment:

Co-suspected Drug is amoxicillin

Reporter's comment:

The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

SANOFI-SYNTHELABO, 24.06.2009 DE-SANOFI-SYNTHELABO-A04200900810

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthematous pustulosis
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: elevated liver enzyme levels (2 to 3-fold)
LLT: ELEVATED LIVER ENZYME LEVELS / 10014480
PT: HEPATIC ENZYME INCREASED / 10060795

Reaction as reported: hepatomegaly
LLT: HEPATOMEGALY / 10019842
PT: HEPATOMEGALY / 10019842

Reaction as reported: oral erosions
LLT: ORAL ULCERATION / 10031027
PT: MOUTH ULCERATION / 10028034

Reaction as reported: fever (>39 degrees C)
LLT: FEVER / 10016558
PT: PYREXIA / 10037660

Narrative:

Initial report:

This serious case is a literature report about acute generalized exanthematous pustulosis under amoxicillin and paracetamol in a male.
Case report: A 48-year-old male patient with no history of psoriasis was treated with paracetamol and amoxicillin (no trade names provided) due to otitis media and developed a prolonged course of acute generalised exanthematous pustulosis (AGEP) with elevated liver enzyme levels (nos) and hepatomegaly. The patient was treated with paracetamol for otitis media, and in addition, amoxicillin was started 2 days later. Within 3 days he developed exanthema, which was progressive despite immediate cessation of both drugs. The patient was hospitalised with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever >39 degrees C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The skin lesions healed with desquamation. Hepatomegaly and laboratory abnormalities as well as the skin lesions showed a prolonged clinical course over a period of 6 weeks on administration of corticosteroid treatment. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. The adverse reactions reappeared after reintroduction (details were not provided). The authors state that they ascribe the patient's symptoms to both amoxicillin and paracetamol because of the clear history in combination with positive skin tests.

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acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative.

Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids.

The positive patch tests with both drugs (5% in petrolatum) revealed papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

Boehringer Ingelheim GmbH, Germany, 13.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04771GD

Sender diagnosis: not reported

Narrative

A case of severe acute generalized exanthematous pustulosis (AGEP) is reported that was elicited by a combined sensitization to amoxicillin and paracetamol. The subject showed unusual liver involvement and prolonged course of the disease (complete recovery after 6 weeks).

The 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to an university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever >39 degC, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-gluta-myl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinik-freiburg.de/hautklinik/live/dzh.html>) and was classified as 'probable' AGEP by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulo pustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

It is concluded that the AGEP in this patient was unique because of: (i) the prolonged course of skin and liver involvement; and (ii) the combined sensitization to two drugs, including (iii) paracetamol, which was recently assigned as having no significant association with AGEP.

GRUNENTHAL, 21.07.2009 DE-GRUNENTHAL-2009-12149

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

Manufacturer Control Number: 2009-12149

ADVERSE EVENT DETAILS

This literature case from GERMANY was reported by a physician via other and was initially received on 10-Jul-2009.

It concerns a 48 year-old male subject/patient (height unknown / weight unknown).

The relevant medical history included otitis media.

Other suspected medication paracetamol.

The patient with no history of psoriasis was treated with paracetamol because of otitis media, in addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone- succinate (250 mg/day for 2 days). On day 7 he was referred to hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. Skin lesions were accompanied by fever, more than 39 C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Bart virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B 19). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. Treatment included high dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch test for both amoxicillin and paracetamol.

Third party case numbers

GRT LitDB AccNo: 624033

GlaxoSmithKline, 21.07.2009 DE-GlaxoSmithKline-B0584328A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of acute generalized exanthematous pustulosis (AGEP) in a 48-year-old male patient who received amoxicillin trihydrate (Amoxicillin; brand name/trade name unknown) for an otitis media.

Co-suspect medication included Paracetamol. The patient had no history of psoriasis.

On an unknown date, the patient started paracetamol and approximately 2 days later started amoxicillin trihydrate (both unknown dosage regimen). Approximately 3 days after starting amoxicillin trihydrate, the patient developed exanthema. Treatment with paracetamol and amoxicillin trihydrate was discontinued immediately and the following day the patient was hospitalised. Despite treatment with prednisolone succinate at 2 mg daily for 2 days the exanthema became progressively worse. On the same day, the patient was treated with oral methylprednisolone, starting at 160 mg with decreasing dosages, in combination with topical corticosteroids.

The following day, the patient was referred to another hospital with pustular suberythroderma exanthema and oral erosions. Histology from the trunk revealed slight acanthiosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by a fever of greater than 39 °C, leukocytosis (25 /nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The following day, eosinophils and liver enzymes were within normal range, but GGT and alanine aminotransferase increased two to three fold respectively. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein Barr virus, cytomegalovirus, human herpes virus-6, parovirus B19). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml) and ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile and blood culture was negative. The patient was discharged on day 21 after starting paracetamol.

The skin lesions healed with desquamation and showed a prolonged clinical course over the period of 6 weeks together with hepatomegaly and laboratory abnormalities. 4 months later, skin testing revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol. At the time of reporting, the events were resolved.

The author considered the events were probably related to treatment with the suspect drugs commenting "The case was reported to the German Registry of Severe Skin Reactions and was classified as probable AGEF by an associated expert panel" and "because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol".

NOVARTIS, 22.07.2009 DE-SZ09-GXKR2009DE07972

Sender diagnosis: not reported

Narrative

Initial literature report received on 13 Jul 2009. A patient developed acute generalized exanthematous pustulosis (AGEP) due to sensitization to amoxicillin and paracetamol, with an unusual liver involvement and prolonged course of the disease. The patient with no history of psoriasis received treatment with paracetamol for otitis media. Two days later, amoxicillin was started and within more 3 days patient developed exanthema, which progressed despite cessation of both drugs on day 5 after the start of drug therapy. Patient was hospitalised on day 6 and received prednisolone-succinate for 2 days. On day 7, the patient was referred to university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. Skin lesions were accompanied by fever >39 C, leukocytosis, neutrophilia and increased C-reactive protein. On day 8, two- to three-fold increase in gamma-glutamyl transferase and alanine aminotransferase was detected, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly and no lymphadenopathy. Treatment included high-dose oral methylprednisolone (with slowly decreasing dosages) in combination with topical corticosteroids. Skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. The patient needed prolonged corticosteroid treatment until complete recovery after 6 weeks. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEF lesions. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol. The patient was advised to avoid both drugs in the future. The authors state that AGEF was unique in this patient because of the prolonged course of skin and liver involvement, combined sensitization to two drugs.

Bristol-Myers Squibb Company, 03.08.2009 DE-Bristol-Myers Squibb Company-14716021

Sender diagnosis: not reported

Narrative

This case was derived from a literature abstract which described about acute generalised exanthematous pustulosis (AGEF) with liver involvement, due to sensitization to acetaminophen and amoxicillin in a male patient.

A 48-year-old male patient was hospitalized for acute generalised exanthematous pustulosis (AGEF) with liver involvement, due to sensitization to acetaminophen and amoxicillin (dosages not stated). He also developed hepatomegaly, alanine aminotransferase (ALT) increased and gamma-glutamyl transferase (GGT) increased.

The man was commenced on acetaminophen for otitis media, and after 2 days, amoxicillin was initiated (therapeutic indication not stated). Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalized and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two - to three-fold increase in ALT and GGT levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEF by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Author comment: "Because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol. Definitive confirmation as the cause of AGEF could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Literature citation: Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 2009 Jan 1;89:314-5.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

RANBAXY, 05.08.2009 DE-RANBAXY-2009RR-25985

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2009RR-25985 from GERMANY refers to a 48 year old male who developed acute generalised exanthematous pustulosis with liver involvement following treatment with amoxicillin and paracetamol that required hospitalisation.

This case was reported in the literature.

The subject received the following suspect products:

Amoxicillin: patch test and prick test positive. The drug was withdrawn.

Paracetamol: for otitis media. Patch test positive, prick test negative. The drug was withdrawn.

The patient received the following treatment products:

Prednisolone succinate: 2 days.

Methylprednisolone: high-dose.

Topical corticosteroids.

The patient developed acute generalised exanthematous pustulosis with liver involvement.

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

The outcome of the events was reported as "healed" at the time of this report.

The reporter considered the case serious because the patient was hospitalised.

Author's Comment: "Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Company comment:

The case is rated as serious and unexpected and possible.

Acute generalized exanthematous pustulosis (AGEP) is an uncommon cutaneous eruption that is most often provoked by drugs, by acute infections with enteroviruses, or by mercury. It is characterized by acute, extensive formation of nonfollicular sterile pustules on an erythematous background, fever and elevated numbers of blood neutrophils. Involvement of T cells in drug-induced AGEP was suggested by positive patch tests and lymphocyte transformation tests. Moreover, drug-specific CD4+ and CD8+ T cells could be isolated and propagated in vitro from patch test sites and blood from AGEP patients.

Most cases of AGEP are drug induced, particularly by antibiotics and mainly beta-lactams.

Because of the clear history in combination with positive skin tests, both amoxicillin and paracetamol contributed to this patient's symptoms. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

Reference:

Britschgi M, Pichler WJ. Acute generalized exanthematous pustulosis, a clue to neutrophil-mediated inflammatory processes orchestrated by T cells. Curr Opin Allergy Clin Immunol 2002 Aug;2(4):325-31.

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to

sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 89: 314-315, No. 3, 2009.

Generics [UK] Ltd, 07.08.2009 DE-MYLANLABS-2009S1013253

Sender diagnosis: not reported

Narrative

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta-Derm-Venereol 2009; 89(3) 314-315

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated]. The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and gamma-glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Generics [UK] Ltd, 07.08.2009 DE-MYLANLABS-2009S1013253

Sender diagnosis: not reported

Narrative

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta-Derm-Venereol 2009; 89(3) 314-315

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated]. The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and gamma-glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

STADA Arzneimittel AG, 07.08.2009 DE-STADA-019143

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Reactions Weekly 1263 pp 6-7; 1 August 2009

Amoxicillin/paracetamol: Acute generalised exanthematous pustulosis with liver involvement: case report

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. Acta Dermato-Venereologica 89: 314-315, No. 3, 2009 - Germany

Summary:

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days

later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and γ -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a 'probable' AGEF by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

Bayer Vital GmbH, 10.08.2009 DE-BAYER-200927350GPV

Sender diagnosis: not reported

Narrative

This report derived from medical literature on 29-Jul-2009 and describes the occurrence of acute generalized exanthematous pustulosis with liver involvement (hepatomegaly and elevation of liver enzymes) in a 48-year-old male who was treated with amoxicillin and paracetamol for otitis media.

****Case Report:**

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to the university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever $>39^{\circ}\text{C}$, leukocytosis (25/nL), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions and was classified as "probable" acute generalized exanthematous pustulosis (AGEP) by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulopustular infiltrates reminiscent of typical AGEF lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

****Authors' Comments:**

The authors report the case of a patient for whom they have good evidence (i.e. by skin testing) that AGEF (acute generalized exanthematous pustulosis) was elicited by a combination of amoxicillin and paracetamol and who showed unusual liver involvement and prolonged course of the disease... Because of the clear history in combination with positive skin tests, the authors attributed the patient's symptoms to both amoxicillin and paracetamol. Positive patch tests in the diagnosis of AGEF have been reported for these two as well as for several other drugs. In this patient, patch testing was more sensitive than prick testing, where paracetamol remained negative. Definitive confirmation as the cause of AGEF could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here. In conclusion, the AGEF in this patient was unique because of: (i) the prolonged course of skin and liver involvement; and (ii) the combined sensitization to two drugs, including (iii) paracetamol, which was recently assigned as having no significant association with AGEF.

Dolorgiet GmbH, 11.08.2009 DE-DOLORGIET-2009paracetamol01411

Sender diagnosis: not reported

Narrative

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEF) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and γ -glutamyl transferase (GGT) levels, respectively; this peaked on day 11. An

ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol. A prick test was positive for amoxicillin at day 2, but not positive for paracetamol.

RANBAXY, 12.08.2009 DE-RANBAXY-2009RR-25985

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2009RR-25985 from GERMANY refers to a 48 year old male who developed acute generalised exanthematous pustulosis with liver involvement following treatment with amoxicillin and paracetamol that required hospitalisation.

This case was reported in the literature.

The subject received the following suspect products:

Amoxicillin: patch test and prick test positive. The drug was withdrawn.

Paracetamol: for otitis media. Patch test positive, prick test negative. The drug was withdrawn.

The patient received the following treatment products:

Prednisolone succinate: 250 mg daily, 2 days.

Methylprednisolone: 160 mg, high-dose. The drug dose was decreased.

Topical corticosteroids.

The patient developed acute generalised exanthematous pustulosis with liver involvement.

A 48-year-old man developed acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitisation to paracetamol [acetaminophen] and amoxicillin [dosages not stated].

The man with no history of psoriasis was commenced on paracetamol for otitis media, and after 2 days, amoxicillin was initiated [therapeutic indication not stated]. Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalised and started treatment with prednisolone succinate 250 mg/day for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions.

Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two- to three-fold increase in ALT and -glutamyl transferase (GGT) levels, respectively; this peaked on day 11.

Serology was negative for acute viral infections (hepatitis a, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV-6), parvovirus B19.) An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions.

Treatment with high-dose of 160 mg methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation.

Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and paracetamol (both in 5% in petroleum). A prick test was positive for amoxicillin at day 2, but not positive for paracetamol. The patient was advised to avoid both drugs in the future.

The outcome of the events was reported as "healed" at the time of this report.

The reporter considered the case serious because the patient was hospitalised.

Author's Comment: "Because of the clear history in combination with positive skin tests, we contributed [sic] our patient's symptoms to both amoxicillin and paracetamol. . . Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Follow up report received on 30-JUL-2009 containing further information regarding:

Laboratory data.

Treatment products: dosages.

Reaction text.

Company comment:

The case is rated as serious and unexpected and possible.

Acute generalized exanthematous pustulosis (AGEP) is an uncommon cutaneous eruption that is most often provoked by drugs, by acute infections with enteroviruses, or by mercury. It is characterized by acute, extensive formation of nonfollicular sterile pustules on an erythematous background, fever and elevated numbers of blood neutrophils. Involvement of T cells in drug-induced AGEP was suggested by positive patch tests and lymphocyte transformation tests. Moreover, drug-specific CD4+ and CD8+ T cells could be isolated and propagated in vitro from patch test sites and blood from AGEP patients.

Most cases of AGEP are drug induced, particularly by antibiotics and mainly beta-lactams.

Because of the clear history in combination with positive skin tests, both amoxicillin and paracetamol contributed to this patient's symptoms. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here.

Reference:

Britschgi M, Pichler WJ. Acute generalized exanthematous pustulosis, a clue to neutrophil-mediated inflammatory processes orchestrated by T cells. *Curr Opin Allergy Clin Immunol* 2002 Aug;2(4):325-31.

Treudler R, Grunewald S, Gebhardt C, Simon J-C. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. *Acta Dermato-Venereologica* 89: 314-315, No. 3, 2009.

Bristol-Myers Squibb Company, 13.08.2009 DE-Bristol-Myers Squibb Company-14716021

Sender diagnosis: not reported

Narrative

This case was derived from a literature abstract which described about acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to acetaminophen and amoxicillin in a male patient.

A 48-year-old male patient was hospitalized for acute generalised exanthematous pustulosis (AGEP) with liver involvement, due to sensitization to acetaminophen and amoxicillin (dosages not stated). He also developed hepatomegaly, alanine aminotransferase (ALT) increased and gamma-glutamyl transferase (GGT) increased.

The man was commenced on acetaminophen for otitis media, and after 2 days, amoxicillin was initiated (therapeutic indication not stated). Three days later, he developed exanthema. Both drugs were ceased on the same day. The next day, he was hospitalized and started treatment with prednisolone succinate for 2 days. On day 7, he was transferred to another hospital with pustular suberythrodermic exanthema and oral erosions. Histology showed slight acanthosis and subcorneal pustules containing neutrophils. Perivascular and interstitial infiltrate made up of neutrophils, eosinophils and lymphocytes was present in the papillary dermis. He had a fever, and displayed neutrophilia, leucocytosis and an elevated C-reactive protein level. After day 8, there was a two - to three-fold increase in ALT and GGT levels, respectively; this peaked on day 11. An ultrasound revealed mild hepatomegaly. The reaction was classified as a "probable" AGEP by an expert panel of the German Registry of Severe Skin Reactions. Treatment with high-dose methylprednisolone was started on day 7 and slowly tapered. Topical corticosteroids were also administered. The man was discharged on day 21; at that time his GGT and ALT levels had only slightly improved. The hepatomegaly and laboratory abnormalities healed after a period of 6 weeks, as did the skin lesions, with desquamation. Four months later, skin testing yielded positive patch test results on day 3 for both amoxicillin and acetaminophen. A prick test was positive for amoxicillin at day 2, but not positive for acetaminophen.

Author comment: "Because of the clear history in combination with positive skin tests, we contributed our patient's symptoms to both amoxicillin and paracetamol. Definitive confirmation as the cause of AGEP could only be obtained by re-challenging the patient, which is, however, ethically not justifiable in severe cases like the one reported here."

Literature citation: Treudler R, Grunewald S, Gebhardt C, Simon JC. Prolonged course of acute generalized exanthematous pustulosis with liver involvement due to sensitization to amoxicillin and paracetamol. *Acta Dermato-Venereologica* 2009 Jan 1;89:314-5.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis after therapy with amoxicillin and acetaminophen. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, a causal role of these medications remains possible.

Supplemental information was received on 05-Aug-2009 as full text article. The event of hepatomegaly was deleted and seriousness criterion of hospitalization was added for alanine aminotransferase increased and gamma-glutamyltransferase increased.

Skin lesions were accompanied by fever more than 39 degree celsius, leucocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). The serum level of interleukin-17 was below the detection level (less than 2.5 pg/ml). Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The patient was advised to avoid both acetaminophen and amoxicillin in future.

BMS medical evaluation comment:

This patient developed acute generalised exanthematous pustulosis (AGEP), increased alanine aminotransferase (ALT) and increased gamma-glutamyl transferase (GGT) after therapy with amoxicillin and acetaminophen. As part of concurrent pharmacotherapy, the cosuspect acetaminophen could cause increased ALT and increased GGT. In the clinical context of symptom evolution and positive patch test results for both amoxicillin and acetaminophen, these medications possibly resulted in AGEP.

Dr. August Wolff Arzneimittel, 21.09.2009 DE-DRWOLFF-2009-029

Sender diagnosis: not reported

Narrative

A case of acute generalized exanthematous pustulosis was reported in a 48-year-old male patient. The patient had a past medical history of otitis media for which he received treatment with paracetamol and two days later with amoxicillin. He developed a progressive erythema within three more days (day 5) and discontinued both drugs. He was hospitalised on day 6 and received prednisolone-succinate. On day 7 he was diagnosed with a pustular suberythrodermic exanthema, oral erosions and fever of 39 degree Celsius. Ultrasound demonstrated mild hepatomegaly. Histological examination of the skin revealed slight acanthosis and subcorneal pustules filled with neutrophils and the papillary dermis showed mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Relevant laboratory test results included leukocytes 25/nl, neutrophils 87%, C-reactive protein 250 mg/l and GGT and ALT two- and threefold increased. Skin lesions healed with desquamation and showed a prolonged clinical course over 6 weeks. Skin testing performed 4 months later revealed positive patch and prick test for amoxicillin and a positive patch but negative prick test for paracetamol.

Reporter's comment:

Bausch & Lomb Incorporated, 22.09.2009 DE-BAUSCH-2009BL004524

Sender diagnosis: not reported

Narrative

A report was identified in the literature indicating that a 48-year-old male patient with no history of psoriasis experienced acute generalized exanthematous pustulosis (AGEP) (ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS) with liver involvement after therapy with PARACETAMOL (route of administration, dose and manufacturer unknown) for otitis media and AMOXICILLIN (route of administration, dose and manufacturer unknown) that was added two days later. Within three more days the patient experienced exanthema. Despite stopping both drugs on day 5 after start of drug therapy, the exanthema was progressive and the patient was hospitalized on day 6. Therapy with prednisolone-succinate (250 mg/day for two days) was started. On day 7 he was referred to a university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever (PYREXIA), leukocytosis (LEUKOCYTOSIS) (25/nl), neutrophilia (NEUTROPHILIA) (87 %) and increased C-reactive protein (CRP) level (C-REACTIVE PROTEIN INCREASED) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) (GAMMA-GLUTAMYLTRANSFERASE INCREASED) and alanine aminotransferase (ALT) (ALANINE AMINOTRANSFERASE INCREASED) then increased two-to three-fold, respectively, with a maximum on day 11 and only slight improvement by discharge on day 21. Acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19) could be excluded serologically. The serum level of interleukin 17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly (HEPATOMEGALY) but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. On day 7 treatment with 160 mg high-dose oral methylprednisolone with slowly decreasing dosages in combination with topical corticosteroids were started. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of six weeks. The patient made a complete recovery.

Skin testing performed four months later revealed positive patch tests for both, amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol. The patient was advised to avoid both drugs in future.

Source: TREUDLER R, GRUNEWALD S, GEBHARDT C, SIMON JC. PROLONGED COURSE OF ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS WITH LIVER INVOLVEMENT DUE TO SENSITIZATION TO AMOXICILLIN AND PARACETAMOL. ACTA DERM VENEROL 2009; 89 (3): 314-5

Bausch & Lomb Incorporated, 12.10.2009 DE-BAUSCH-2009BL004524

Sender diagnosis: not reported

Narrative

A report was identified in the literature indicating that a 48-year-old male patient with no history of psoriasis experienced acute generalized exanthematous pustulosis (AGEP) (ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS) with liver involvement after therapy with PARACETAMOL (route of administration, dose and manufacturer unknown) for otitis media and AMOXICILLIN (route of administration, dose and manufacturer unknown) that was added two days later. Within three more days the patient experienced exanthema. Despite stopping both drugs on day 5 after start of drug therapy, the exanthema was progressive and the patient was hospitalized on day 6. Therapy with prednisolone-succinate (250 mg/day for two days) was started. On day 7 he was referred to a university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever (PYREXIA), leukocytosis (LEUKOCYTOSIS) (25/nl), neutrophilia (NEUTROPHILIA) (87 %) and increased C-reactive protein (CRP) level (C-REACTIVE PROTEIN INCREASED) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) (GAMMA-GLUTAMYLTRANSFERASE INCREASED) and alanine aminotransferase (ALT) (ALANINE AMINOTRANSFERASE INCREASED) then increased two-to three-fold, respectively, with a maximum on day 11 and only slight improvement by discharge on day 21. Acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19) could be excluded serologically. The serum level of interleukin 17 was below the detection level (<2.5 pg/ml). Ultrasound revealed mild hepatomegaly (HEPATOMEGALY) but no lymphadenopathy. The content of

the pustules was sterile, and blood culture was negative. On day 7 treatment with 160 mg high-dose oral methylprednisolone with slowly decreasing dosages in combination with topical corticosteroids were started. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of six weeks. The patient made a complete recovery.

Skin testing performed four months later revealed positive patch tests for both, amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol. The patient was advised to avoid both drugs in future.

Source: TREUDLER R, GRUNEWALD S, GEBHARDT C, SIMON JC. PROLONGED COURSE OF ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS WITH LIVER INVOLVEMENT DUE TO SENSITIZATION TO AMOXICILLIN AND PARACETAMOL. ACTA DERM VENEROL 2009; 89 (3): 314-5

Upon further file review it was noticed that the company's causality assessment at the event level for the reported event gamma-glutamyl transferase increased (LLT: GGT increased and PT: Gamma-glutamyltransferase increased) was entered as "possible related" to the product. Changes were made to the report reflecting the company's causality assessment as "probable related" for the reported event GGT increased.

MIP International Pharma Research GmbH, 24.11.2009 DE-MIP-hamxpo161809-hpaopo160109

Sender diagnosis: not reported

Narrative

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to university hospital with pustular suberythrodermic exanthema and oral erosions. Skin lesions were accompanied by fever, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but GGT and ALT then increased two- to three-fold, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections, serum level of interleukin-17 was below the detection level. Ultrasound revealed mild hepatomegaly but no lymphadenopathy. Treatment included high-dose oral methylprednisolone in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol. A prick test was positive at late readings for amoxicillin but not indicative for paracetamol.

as well as Paracetamol-Saft Tabletten und Paracetamol-Saft.

Reporter's comment:

The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinik-freiburg.de/hautklinik/live/dzh.html>) by the authors and was classified as probable acute generalized exanthematous pustulosis by an associated expert panel. Because of the clear history in combination with positive skin tests, the authors assume that the symptoms were released by both amoxicillin and paracetamol.

AXCOUNT, 07.09.2010 DE-AXCOUNT-2010-013

Sender diagnosis: not reported

Narrative

A 48-year-old man was treated with paracetamol and amoxicillin because of otitis media. Amoxicillin was started 2 days after treatment start with paracetamol and within 3 more days, the man developed exanthema, which was progressive despite immediate cessation of both drugs. The man was hospitalised on day 6 after start of drug therapy and treated with prednisolone-succinate (250 mg/day for 2 days). The man had pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophiles. Skin lesions were accompanied by fever ($>39^{\circ}\text{C}$), leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (250 mg/l). On day 8, the patient's GGT and ALT levels increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for viral infections. The content of the pustules was sterile, and blood culture was negative. Acute generalised exanthematous pustulosis (AGEP) was diagnosed and the patient was treated with oral methylprednisolone in combination with topical corticosteroids. The lesions healed within 6 weeks and the laboratory abnormalities resolved. A skin testing was performed 4 months later and revealed positive patch tests for both amoxicillin and paracetamol with papulopustular infiltrates reminiscent of typical AGEP lesions. The patient was advised to avoid both drugs in the future.

The authors stated that because of the clear history in combination with positive skin tests, the patient's symptoms are attributed to amoxicillin and paracetamol.

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

A 48-year-old man with no history of psoriasis was treated with paracetamol because of otitis media. In addition, amoxicillin was started 2 days later and within 3 more days he developed exanthema, which was progressive despite immediate cessation of both drugs on day 5 after start of drug therapy, hospital admission on day 6 and therapy with prednisolone-succinate (250 mg/day for 2 days). On day 7 he was referred to our university hospital with pustular suberythrodermic exanthema and oral erosions. Histology from the trunk revealed slight acanthosis and subcorneal pustules filled with neutrophils. In the papillary dermis there was a mixed perivascular and interstitial infiltrate of neutrophils, eosinophils and lymphocytes. Skin lesions were accompanied by fever > 39°C, leukocytosis (25/nl), neutrophilia (87%) and increased C-reactive protein level (CRP) (250 mg/l). On day 8, eosinophils and liver enzymes were within normal range, but gamma-glutamyl transferase (GGT) and alanine aminotransferase (ALT) then increased two- to three-fold, respectively, with a maximum on day 11 and only slight improvement until discharge on day 21. Serology was negative for acute viral infections (hepatitis A, B, C, Epstein-Barr virus, cytomegalovirus, human herpes virus (HHV)-6, parvovirus B19). The serum level of interleukin-17 was below the detection level (< 2.5 pg/ml). Ultrasound revealed mild hepatomegaly but no lymphadenopathy. The content of the pustules was sterile, and blood culture was negative. The case was reported to the German Registry of Severe Skin Reactions (<http://www.uniklinikfreiburg.de/hautklinik/live/dzh.html>) and was classified as "probable" AGEP by an associated expert panel. Treatment included high-dose oral methylprednisolone (starting on day 7 with 160 mg, with slowly decreasing dosages) in combination with topical corticosteroids. The skin lesions healed with desquamation and showed a prolonged clinical course over a period of 6 weeks, together with hepatomegaly and laboratory abnormalities. Skin testing performed 4 months later revealed positive patch tests for both amoxicillin and paracetamol (both 5% in petrolatum, day 3) with papulopustular infiltrates reminiscent of typical AGEP lesions. A prick test was positive at late readings for amoxicillin (papular infiltrate of 5 mm at day 2) but not indicative for paracetamol (both pure in aqueous solution). The patient was advised to avoid both drugs in the future.

To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09089434

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19580218	Age: 50 Year	weight (kg): 73	BMI_int: 25.56	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090209	20090401	52 Day	15 Day	recovered/resolved
2 Carpal tunnel decompression	20090203			9 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Lormetazepam-Ratiopharm, Tabl, Ratiopharm	Oral	1 Mg milligram(s)/ Day	20090126-20090207	
lormetazepam		245	13 Day	
2 Novaminsulfon-Ratiopharm, Trpf, Ratiopharm	Oral	750 Mg milligram(s)/ Day	20090127-20090217	
metamizole		158	22 Day	
3 Pantozol, 20 mg, Tabl (mr), Nycomed	Oral	20 Mg milligram(s)/Day	20090126-20090212	
pantoprazole		94	18 Day	
4 Paracetamol-Ratiopharm, 500 mg, Tabl, Ratiopharm	Oral	1500 Mg milligram(s)/Day	20090207-20090210	
paracetamol		245	4 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Neurolysis	200812		Unknown	state after neurolysis of the right median nerve
Wound curettage			Unknown	
Carpal tunnel syndrome			Unknown	
Hypertension arterial			Unknown	
Tumour excision			No	excision of skin tumour of the right knee
Sudeck's atrophy			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20090126	31	U/l	
Alanine aminotransferase increased	20090209	276	U/l	
Alanine aminotransferase increased	20090216	376	U/l	
Alanine aminotransferase increased	20090218	317	U/l	
Alanine aminotransferase increased	20090223	149	U/l	
Alanine aminotransferase increased	20090401	36	U/l	
Aspartate aminotransferase normal	20090126	32	U/l	
Aspartate aminotransferase increased	20090209	93	U/l	
Aspartate aminotransferase increased	20090214	181	U/l	
Aspartate aminotransferase increased	20090216	164	U/l	
Aspartate aminotransferase increased	20090218	107	U/l	
Aspartate aminotransferase increased	20090223	55	U/l	

Aspartate aminotransferase normal	20090401	27	U/l
Bilirubin increased	20090211	4.91	mg/dl
Bilirubin increased	20090213	7.05	mg/dl
Bilirubin increased	20090216	9.11	mg/dl
Bilirubin increased	20090218	5.94	mg/dl
Bilirubin normal	20090401	0.9	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 13.07.2009 DE-FAKOS-170254

Sender diagnosis: not reported

Narrative

Aufnahme der Patientin am 26.01.2009 nach Einweisung durch den behandelnden ambulanten Chirurgen. Grund war eine notwendige Operation bei Karpaltunnelsyndrom. Am 03.02.2009 erfolgte die Operation. Am 09.02.2009 wurde im Labor eine Erhöhung der Transaminasen festgestellt (ALAT 276 U/l (Normbereich: <36 U/l), ASAT 93 U/l (Normbereich: <36 U/l)). Am 11.02.2009 wurde ein Bilirubin von 4,91 mg/dl (Normbereich: <1 mg/dl) festgestellt. Am 16.02.2009 wurde ein Peak erreicht (ALAT 376 U/l, ASAT 164 U/l, Bili 9,11 mg/dl), wonach sich eine spontane und progrediente Erholung zeigte. Serologisch wurde eine virusassoziierte Hepatitis ausgeschlossen. Ein Test auf EBV und CMV wurde nicht durchgeführt. Entlassung der Patientin am 25.02.2009. Vollständige Normalisierung der Transaminasen vom Hausarzt gemessen am 01.04.2009: (ALAT 36 U/l, ASAT 27 U/l, Bili 0,9 mg/dl).

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Lormetazepam 1 mg Ratiopharm Tabl. (Einnahme: 26.01. - 07.02.2009 (Pause am 31.01.2009)),

Novaminsulfon Ratiopharm Trpf. (Einnahme: 27.01. - 17.02.2009 (mit Pausen)),

Paracetamol Ratiopharm Tabl. (Einnahme: 07.02. - 10.02.2009), Pantozol Tabl. (Einnahme: 26.01. - 12.02.2009).

Wegen der Leberwerterhöhung wurden diese zuvor neu verabreichten Arzneimittel wieder abgesetzt. Insofern ist eine eindeutige Zuordnung bezüglich Kausalität nicht möglich.

reporter's comment:

NOVARTIS, 29.07.2009 DE-002147023-PHHY2009DE30079

Sender diagnosis: not reported

Narrative

Initial Health Authority report (reference no # DE-BFARM-09089434) received on 14 Jul 2009 and forwarded by Nycomed (reference no # 0206563) on 22 Jul 2009: This patient started Pantozol (pantoprazole), 20 mg daily, (the day of her hospitalization for carpal tunnel syndrome surgery) on 26 Jan 2009, for an unspecified indication. Co-suspect medications were Lormetazepam, Novaminsulfon and paracetamol, initiated on 26 Jan 2009, 27 Jan 2009 and 07 Feb 2009, respectively. The patient had a medical history of Sudeck's atrophy and hypertension as well as state after curettage, state after excision of skin tumour of the right knee and state after neurolysis of the right median nerve in Dec 2008 and carpal tunnel syndrome. The patient did not have any concomitant medications. She was previously treated with acetaminophen/dimenhydrinate. The carpal tunnel syndrome surgery was successfully performed on 03 Feb 2009. On 09 Feb 2009, 14 days after the first dose of Pantozol, her lab work-up revealed an increase in transaminase levels ((ALAT 276 U/l (normal range: <36 U/l), ASAT 93 U/l (normal range: <36 U/l)). On 11 Feb 2009, bilirubin was found to be 4.91 mg/dl (normal range: <1 mg/dl). On 16 Feb 2009 all lab results reached their peak values (ALAT 376 U/l, ASAT 164 U/l, bilirubin 9.11 mg/dl). The patient was diagnosed with acute hepatitis. Serological examinations excluded a virus-associated hepatitis. Epstein-Barr virus and cytomegalovirus were not tested. Therapy with Pantozol was discontinued on 12 Feb 2009 due to the adverse event. Lormetazepam, Novaminsulfon and paracetamol were discontinued on 07 Feb 2009, 17 Feb 2009 and 10 Feb 2009, respectively. From 18 Feb 2009, liver values started decreasing. The patient was discharged on 25 Feb 2009. A complete normalization of liver function enzymes was observed by the family doctor on 01 Apr 2009 (ALAT 36 U/l, ASAT 27 U/l, Bilirubin 0.9 mg/dl). The patient completely recovered on 04 Apr 2009. The German Health Authority assessed causality as "possible" for Pantozol and other suspect medications.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09090323

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Male Birth: 19400222 Age: 69 Year weight (kg): 84 BMI_int: 27.12 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Amylase increased	20090302			6 Day	Unknown
2 Edematous pancreatitis	20090227	20090304	6 Day	6 Day	recovered/resolved
3 Abdominal pain					Unknown
4 Lipase increased	20090302			6 Day	Unknown
5 Epigastric pain	20090227			3 Day	recovering/resolving
6 AST increased	20090302			6 Day	Unknown
7 ALT increased					Unknown
8 Abdominal pain NOS	20090301			5 Day	recovering/resolving
9 Pancreatitis	20090227	20090304	6 Day	2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buprenorphine buprenorphine	Transdermal	/	20090224-20090302 7 Day	Postoperative pain
2 IBUPROFEN ibuprofen	Oral	600 Mg milligram(s)/Day 82	2007-20090220 -	Coxarthrosis
3 OMEP omeprazole	Oral	40 Mg milligram(s)/Day 245	20090225- -	Gastric ulcer prophylaxis
4 PARACETAMOL paracetamol	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
5 RAMIPRIL ramipril	Oral	2* /Day 245	2000- -	Hypertension arterial
6 VOLTAREN RESINAT diclofenac resinate	Oral	150 Mg milligram(s)/Day 6	20090225-20090227 3 Day	Postoperative pain

Drugs (concomitant)

7 BERLOSIN metamizole	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
8 CEFUROXIME cefuroxime	Intravenous drip	1.5 Mg milligram(s)/Day 293	20090224-20090224 1 Day	Infection prophylaxis
9 FLOTRIN terazosin hydrochloride	Oral	2 Mg milligram(s)/Day 245	2005- -	Benign prostatic hyperplasia
10 norspan buprenorphine	Transdermal	/Day 250	20090224-20090302 7 Day	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Prosthesis implantation	200807		Unknown	
Prosthesis implantation	20090224		Unknown	
Total hip replacement	20090224		No	
Total hip replacement	200807		No	
Alcohol abuse, unspecified drinking behavior			Yes	
Ex-smoker	1980		Yes	1955-1980 1 Packung/Woche
Alcohol use			Yes	Beginn unbekannt, 3 Bier+1 Flasche Rotwein in der Woche
Coxarthrosis			Yes	Beginn unbekannt, TEP links 07/2008, rechts 24.02.2009
Pancreatitis	200807	200807	No	
Benign prostatic hyperplasia	200807		Yes	Beginn unbekannt
Hypertension arterial	1990		Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090302	89	µmol/l	
Calcium	20090223	2.31	mmol/l	
Ultrasonography	20090302	nicht vorhanden	NA	Absent
Ultrasonography	20090223	nicht vorhanden	NA	Absent
Lipase	20090302	1429	NA	
C-reactive protein	20090302	24.9	mg/l	
Aspartate aminotransferase	20090302	52.4	NA	
Aspartate aminotransferase	20090223	26	NA	
Amylase	20090302	1495	NA	
Amylase	20090223	57.1	NA	
Alanine aminotransferase	20090302	55.1	NA	
Alanine aminotransferase	20090223	21	NA	
Sodium	20090302	138	mmol/l	
Potassium	20090302	4.1	mmol/l	
Lipase	20090302	1429	U/l	
Amylase	20090302	1495	U/l	
Amylase	20090223	57.1	U/l	
C-reactive protein	20090302	24.9	mg/l	
Gamma GT	20090302	32.0	U/l	
Gamma GT	20090223	14.0	U/l	
Alkaline phosphatase	20090223	57.0	U/l	
Alanine aminotransferase	20090302	55.1	U/l	
Alanine aminotransferase	20090223	21.0	U/l	
Aspartate aminotransferase	20090302	52.4	U/l	

Aspartate aminotransferase	20090223	26.0	U/l
Creatinine clearance	20090302	>1.0	ml/s

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal / Universität Witten, 14.07.2009 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor oder Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis;

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber, Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur.

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:**MEDIKAMENT: RAMIPRIL**

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher wird Ramipril hier nicht verdächtig.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous (health professional confirmed) report from the German Health Authority via Web-Tool.

On 16Jul09 information has been received concerning a 69-years old male patient who suffered from pancreatitis from 27Feb09 to 04Mar09 while on therapy with paracetamol 3 tablets per day from 26Feb09 to 27Feb09. The reporting physician assumed the relationship between paracetamol and the event as likely.

Relevant medical history included coxarthrosis, benign prostatic hyperplasia and arterial hypertension. The patient is alcohol user (3 bottles of beer and 1 bottle of red wine in the week) and is ex-smoker.

The patient suffered from pancreatitis in Jul08. Concomitant medication included Berlosin 500mg (metamizole sodium), 3 tablets daily for postoperative pain; Cefuroxime (cefuroxime) 1,5 mg daily for infection prophylaxis; Voltaren Resinat 75 mg (diclofenac resinate), daily 2 capsules for postoperative pain; Diclofenac Resinate (diclofenac resinate) for an unknown indication; Flotrin 2 mg (terazosin hydrochloride), daily 1 tablet for benign prostatic hyperplasia; Ibuprofen 600 mg (ibuprofen) daily 1 film-coated tablet for coxarthrosis; Norspan (buprenorphine) daily 1 transdermal patch for postoperative pain; Omeprazol 40 mg (omeprazole) daily 1 tablet for gastric ulcer prophylaxis and Ramipril (ramipril) daily 2 tablets (2,5-0-5 mg) for hypertension arterial.

Tracking:

16Jul09 - initial information via Web-Tool

SANOFI-SYNTHELABO, 28.07.2009 DE-SANOFI-SYNTHELABO-A04200901023

Sender diagnosis: not reported

Narrative

Reaction as reported: pancreatitis (lipase 25 xULN, amylase 14 xULN)

LLT: PANCREATITIS / 10033645

PT: PANCREATITIS / 10033645

Narrative:

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

NOVARTIS, 29.07.2009 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematic pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

HELIOS Klinikum Wuppertal, 26.01.2010 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor (Tramadol, Metamizol) und Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis (es wurde ein regelmäßiger Alkoholkonsum angegeben, allerdings zur Zeit wegen OP kein Alkohol);

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber,

Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden auch in der Fachinfo beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur (aber nicht eindeutig in der Fachinfo).

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: RAMIPRIL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher ist Ramipril hier unwahrscheinlich.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, darunter Symptombesserung - es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben. Die Einnahme wurde 1 Woche vor Auftreten der Symptome beendet und über einen mehrjährigen Zeitraum gut vertragen.

NOVARTIS, 03.02.2010 DE-NOVNCHHQ-GXKR2009DE08058

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 15 Jul 2009 from the German Health Authority (DE-BfArM-09090323), provided by a pharmacovigilance center, reported by a physician: This multimorbid patient developed pancreatitis during treatment with paracetamol tablets (MAH unknown) for postoperative pain. Voltaren resinate (diclofenac resinate) was also suspected. Several other drugs were taken concomitantly. The outcome of the adverse event was reported as resolved. For paracetamol causality was assessed as likely.

Significant follow-up received from health authority (BfArM homepage, DE-BFARM-09090323) on 27 Jan 2010 changing several concomitant drugs into suspected drugs and changing causality assessment: Ramipril (MAH unknown), Omeprazole (omeprazole) and ibuprofen (MAH unknown) were also suspected. Causality for ramipril, Omeprazole and ibuprofen was assessed as unlikely. For paracetamol causality was assessed as possible. Spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 04.02.2010 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report health care professional confirmed report from the German Health Authority BfArM.

18.03.2013

BfArM - Pharmacovigilance

392 / 640

On 28Jan2010 follow-up information was received from a physician in Germany via the German Health Authority BfArM concerning a 69-year-old male patient, who was on paracetamol 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain, on ramipril 2,5-0-5 mg/day since 200 and continuing for arterial hypertension, on Voltaren Resinat (diclofenac resinate) 2x75 mg/day from 25Feb2009 to 27Feb2009 for postoperative pain, Omep (omeprazole) 1x40 mg/day since 25Feb2009 and continuing for gastric ulcer prophylaxis and on ibuprofen 1x600 mg/day from 2007 to 20Feb2009 for coxarthrosis. The event was considered as serious due to the statement of the reporter. Ramipril and Omep were continued. Paracetamol, Voltaren Resinat and ibuprofen were discontinued. The patient has recovered.

From 27Feb2009 to 04Mar2009 the patient suffered from pancreatitis.

Comments to the following event:

Pancreatitis

Short summary:

acute pancreatitis due to diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

Description:

since 27Feb2009 mild epigastric pain,

since 01Mar2009 severe stabbing stomach pain, emissive into the lower abdomen, condition after hip-TEP-operation on 24Feb2009,

during accommodation 12-point increased lipase value,

anamnesic: Jul2008 identical event after first hip-TEP-operation (medicament anamnesis not known, operation was in Saxony)

Therapy:

initial abstinence from food, liquid substitution (Sterofundin)

analgesics discontinued

Würzburger perfusion (tramadol, metamizol) and Haldol,

Clexane 40 mg sc

Course:

no typical existent of risk factors for a pancreatitis (regular intake of alcohol was stated, but at the time of the operation no alcohol),

fast improvement of the symptomatology (no further laboratory test has been carried out)

02Mar2009 sonography abdomen: oedematous pancreatitis, pancreatic duct not expanded, splenoportal vascular system without pathological findings,

liver, gallbladder and bile duct without pathological findings

explanation of the event:

hospitalisation from 02Mar2009 - 04Mar2009

reason for hospitalisation: acute pancreatitis, condition after total endoprosthesis of hip joint on 24Feb2009,

in chronological sequence Diclofenac, Paracetamol, Buprenorphin and Cefuroxim are suspected. For NSAR are pancreatitis also described in the

product information. For Paracetamol, Buprenorphin and Cefuroxim there are references in the literature (but not definite in the product information)

After the first hip-TEP in Jul2008 a pancreatitis also occurred. At that time a medicament anamnesis is not ascertainable. As alternative reason for the pancreatitis could be discussed a reaction due to the anaesthesia, as well due to the surgical intervention. However until now only abdominal surgical interventions are associated with pancreatitis in literature.

The bile duct were sonographic without pathological findings. Against acute intake of alcohol as reason speaks the normal value of Gamma-GT (glutamyl transpeptidase).

Triglycerides were not calculated.

Concomitant medications included Flotrin (terazosin hydrochloride) 1x2 mg/day since 2005 and continuing for benign prostatic hyperplasia, Cefuroxime (cefuroximaxetil) 1x1,5 mg/day on 24Feb2009 for infection prophylaxis, Norspan (buprenorphine) 1x1 plaster/day from 24Feb2009 to 02Mar2009 for postoperative pain and Berlosin (metamizole sodium) 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain.

Concomitant diseases included ex-smoker since -1980 (1955-1980 1 package/week) and continuing, alcohol use (beginning unknown, 3 beer + 1 bottle wine a week) and continuing, coxarthrosis (beginning unknown) and continuing, benign prostatic hyperplasia (beginning unknown) and continuing and arterial hypertension sine 1990 and continuing.

Past diseases included TEP (total endoprosthesis) left in Jul2008, TEP (total endoprosthesis) right on 24Feb2009 and pancreatitis in Jul2008.

The reporter rated the causal relationship between the event and Paracetamol and Voltaren Resinat as possible and rated the causal relationship between the event and Ramipril, Omep and Ibuprofen as unlikely.

Tracking of Changes:

16Jul2009 - initial information from physician via the German Health Authority BfArM

28Jan2010 - follow-up information from physician via the German Health Authority BfArM

sanofi-aventis, 04.02.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority

(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

Additional information extracted from the website of the German health authority on 28-Jan-2010. Information by PV centre: The patient had reported regular alcohol consumption, but around the time of operation the patient did not consume alcohol.

The following drugs were switched from concomitant to suspect medication: ramipril (trade name not provided) 7.5mg/day from 2000 onwards for arterial hypertension, ibuprofen (trade name not provided) 600mg/day from 2007 to 20-Feb-2009 for coxarthrosis, and omeprazole 1x40mg/day from 25-Feb-2009 onwards as gastric ulcer prophylaxis.

Causality assessment was changed by reporter to WHO score:

"possible" for paracetamol, diclofenac,

"unlikely" for ramipril, ibuprofen, omeprazole.

Comments regarding causality of new suspect drugs:

Pancreatitis is described in physicians' information for ramipril as very rare; the drug had been taken for several years, the symptoms improved despite continuation of treatment, therefore ramipril is rather unlikely in this case. Pancreatitis is described in physicians' information for ibuprofen as very rare; the drug was stopped one week before onset of symptoms and had been tolerated over a longer period of time. Pancreatitis is described for omeprazole; symptoms improved despite continuation of treatment with omeprazole, therefore no temporal relationship exists in this case.

NOVARTIS, 05.02.2010 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematous pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

Follow-up information received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 27 Jan 2010: Ramipril (manufacturer unknown) 2.5 mg and 5 mg two times a day for hypertension arterial from 2000 and Ibuprofen (manufacturer unknown) 600 mg per day for coxarthrosis from 2007 to 20 Feb 2009 were added as co-suspect in this case. Norspan (buprenorphine) and Cefuroxim was changed from suspect drug to concomitant medications. The onset date for pancreatitis was reported as 27 Feb 2009.

sanofi-aventis, 07.04.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority

18.03.2013

BfArM - Pharmacovigilance

394 / 640

(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

Additional information extracted from the website of the German health authority on 28-Jan-2010. Information by PV centre: The patient had reported regular alcohol consummation, but around the time of operation the patient did not consume alcohol.

The following drugs were switched from concomitant to suspect medication: ramipril (trade name not provided) 7.5mg/day from 2000 onwards for arterial hypertension, ibuprofen (trade name not provided) 600mg/day from 2007 to 20-Feb-2009 for coxarthrosis, and omeprazole 1x40mg/day from 25-Feb-2009 onwards as gastric ulcer prophylaxis.

Causality assessment was changed by reporter to WHO score:

"possible" for paracetamol, diclofenac,

"unlikely" for ramipril, ibuprofen, omeprazole.

Comments regarding causality of new suspect drugs:

Pancreatitis is described in physicians' information for ramipril as very rare; the drug had been taken for several years, the symptoms improved despite continuation of treatment, therefore ramipril is rather unlikely in this case. Pancreatitis is described in physicians' information for ibuprofen as very rare; the drug was stopped one week before onset of symptoms and had been tolerated over a longer period of time. Pancreatitis is described for omeprazole; symptoms improved despite continuation of treatment with omeprazole, therefore no temporal relationship exists in this case.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09090287

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19610617	Age: 47 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Overdose	20090331			2 Day	recovering/resolving
2 Loose stools					Unknown
3 Nausea					Unknown
4 Right upper quadrant pain					Unknown
5 Vomiting					Unknown
6 Acute hepatic failure	20090331	20090407	8 Day	2 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	20*500 Mg milligram(s)/ D 245	20090330-20090330	Toothache
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Toothache			Unknown	Beginn unbekannt
Hypokalaemia			Yes	Beginn unbekannt
Thrombocytopenia			Yes	Beginn unbekannt
Alcohol abuse chronic			Yes	Beginn und Menge unbekannt
Smoker			Yes	Beginn unbekannt

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090402	77.8	µmol/l	
Creatinine	20090403	67.6	µmol/l	
Creatinine clearance	20090403	>1.0	ml/s	
Urea	20090403	3.3	mmol/l	
Aspartate aminotransferase	20090402	7837	U/l	
Aspartate aminotransferase	20090403	4410	U/l	
Aspartate aminotransferase	20090404	817	U/l	
Aspartate aminotransferase	20090405	291	U/l	
Aspartate aminotransferase	20090406	131	U/l	
Alanine aminotransferase	20090402	9462	U/l	
Alanine aminotransferase	20090403	7114	U/l	
Alanine aminotransferase	20090404	4107	U/l	
Alanine aminotransferase	20090405	2881	U/l	

DE-BFARM-09090287			
Alanine aminotransferase	20090406	1819	U/l
Bilirubin total	20090402	47.7	µmol/l
Bilirubin total	20090403	55.7	µmol/l
Bilirubin total	20090404	59.3	µmol/l
Bilirubin total	20090405	40.4	µmol/l
Bilirubin total	20090406	23.8	µmol/l
Alkaline phosphatase	20090402	48	U/l
Alkaline phosphatase	20090403	53	U/l
Gamma GT	20090403	207	U/l
Quick's test	20090402	36	Prozent
Quick's test	20090403	42	Prozent
Quick's test	20090404	69	Prozent
Quick's test	20090405	86	Prozent
Partial prothrombin time	20090402	40.2	sec
INR	20090402	2.04	Physiologis
INR	20090403	1.69	Physiologis
Thrombocyte count	20090402	12	Gpt/l
Thrombocyte count	20090403	19	Gpt/l
Thrombocyte count	20090405	84	Gpt/l
Thrombocyte count	20090406	129	Gpt/l
LDH	20090402	4314	U/l
Haemoglobin	20090402	8.2	mmol/l
Haemoglobin	20090403	7.3	mmol/l
Haemoglobin	20090405	7.7	mmol/l
Haemoglobin	20090406	7.5	mmol/l
Haematocrit	20090403	0.34	SI-Einheit
Haematocrit	20090405	0.36	SI-Einheit
Haematocrit	20090406	0.36	SI-Einheit
Total white blood cell count	20090402	7.09	Gpt/l
C-reactive protein	20090402	36.2	mg/l
C-reactive protein	20090403	40.5	mg/l
Potassium	20090402	3.6	mmol/l
Potassium	20090403	3.4	mmol/l
Sodium	20090402	130	mmol/l
Sodium	20090403	133	mmol/l
Calcium	20090402	2.1	mmol/l
Calcium	20090403	2.07	mmol/l
Coagulation factor V level	20090402	37	Prozent
Coagulation factor V level	20090403	35	Prozent
Coagulation factor V level	20090405	111	Prozent
Coagulation factor V level	20090406	153	Prozent
Blood pH	20090402	7.43	Bitte füllen
PCO2	20090402	5.5	kPa
Bicarbonate	20090402	26.8	mmol/l

Base excess	20090402	2.9	mmol/l
O2 saturation	20090402	92	%
Ammonia	20090402	54.7	µmol/l
Ammonia	20090403	35.4	µmol/l
Ammonia	20090406	52	µmol/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal / Universität Witten, 14.07.2009 DE-PVZIBE-02-10164

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akutes Leberversagen

KURZZUSAMMENFASSUNG:

akutes Leberversagen nach Einnahme von Paracetamol

BESCHREIBUNG:

bei Aufnahme: Blutdruck 135/70mmHg, Puls 69/min, leicht reduzierter Allgemeinzustand, gräuliches Hautkolorit, Druckschmerz im rechten Ober- und Mittelbauch, Transaminasen deutlich erhöht, deutliche Hepatomegalie nach Einnahme von 20 Tabletten Paracetamol á 500mg innerhalb von 8 Stunden wegen Zahnschmerzen;

nach Therapie mit Fluimucil Antidot sowie Transfusion von 1 Thrombozytenkonzentrat Abfall der Transaminasen und Normalisierung der Gerinnungsparameter, Verbesserung der Hepatomegalie

THERAPIE:

02.04.2009 bis 4.04.2009 Intensivstation

02.04.2009: Transfusion von 1 Thrombozytenkonzentrat, Infusion von 60ml Fluimucil 20% in 500ml G 5% (um 13:00 Uhr)

VERLAUF:

31.03.2009 (einweisende Klinik): stark erhöhte Transaminasen und beginnendes Gerinnungsversagen, rechtsseitige Oberbauchschmerzen, Übelkeit, Erbrechen, Durchfall;

02.04.2009: Druckschmerz im rechten Ober- und Mittelbauch, deutliche Hepatomegalie, Nierenlager indolent;

02.04.2009 18:45 Uhr: kein Ethanol und kein Paracetamol im Serum nachweisbar (3 Tage nach Einnahme);

06.04.2009 Sonographie Abdomen: leichtgradige Hepatomegalie, kein Anhalt für chronische Parenchymschädigung, leichtgradiges Gallenblasenwandödem als Folge des Leberversagens, Pankreas mit Zeichen der Parenchymfibrose, Verdacht auf umschriebene kleinzystische Pankreasgangektasien, alkoholtoxische Fibrose, anamnestisch kein Hinweis für Pankreaserkrankung;

06.04.2009: Rückgang der Transaminasen, Normalisierung der Gerinnung;

07.04.2009 Entlassung im gebesserten Allgemeinzustand

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-07.04.2009;

bei Aufnahme Verdacht auf akutes Leberversagen nach Einnahme von Paracetamol (anamnestisch Einnahme von 10g in 8 Stunden am 30.03.2009) wegen Zahnschmerzen in Selbstmedikation

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: akutes Leberversagen

ERLÄUTERUNG: Absolute Überdosierung: 20 Tabletten á 500mg in 8 Stunden

Als zusätzlicher Risikofaktor für eine erhöhte Paracetamoltoxizität besteht eine mögliche Enzyminduktion durch chronischen Alkoholabusus.
Rückläufige Werte unter Antidotgabe.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 20.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04917DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report health care professional confirmed report from the German Health Authority BfArM.

On 16July2009 information has been received from the German Health Authority BfArM via web tool report.

A 47-year-old male patient suffered from acute liver failure from 31March 2009 until 07April 2009 after administration of 20 tablets Paracetamol 500 mg within 8 hours on 30 March 2009 in the indication toothache. This is considered to be an absolute overdose. The patient was admitted to the intensive care unit from 02April 2009 until 04April2009. The reporting source rated the case to be life threatening and to be most likely related to Paracetamol overdosing. The event was treated with antidote: Fluimucil and thrombocyte concentrate. The patient has recovered.

Lab data elevation (only pathological) in the consens of liver failure after hospitalization:

Asparate aminotransferasis 7837 U/l; Bilirubine total 47,7 micromol/l; Gamma GT 207 U/l; ammonia 54,7 micromol/l; C-reactive protein 36,2 mg/l. elevated blood coagulation parameter: Quick's test 36 %; partial thrombine time 40,2 sec; INR 2,04; thrombocytes count 12 Gpt/l; LDH 4314 U/l; haemoglobine 8,2 mmol/l; haematocrite 0,34 SI Units, coagulation factor V level 37 %; elevated values acid base balance: bicarbonate 26,8 mmol/l; base excess 2,9 mmol/l;

The patient has a medical history of alcohol abusos and he is a smoker. Thrombocytopenia and hyperkalaemia are reported as concomitant diseases. Co-medication was not reported.

Tracking:

16July2009 - initial information via report from the German Health Authority BfArM via report.

SANOFI-SYNTHELABO, 23.07.2009 DE-SANOFI-SYNTHELABO-A04200901005

Sender diagnosis: not reported

Narrative

Reaction as reported: acute hepatic failure (ASAT 156 xULN, ALAT 189 xULN, total bilirubin 2x ULN, beginning failure of coagulation)

LLT: ACUTE HEPATIC FAILURE / 10000804

PT: ACUTE HEPATIC FAILURE / 10000804

Reaction as reported: overdose (20 tablets paracetamol a 500mg in 8 hours)

LLT: OVERDOSE / 10033295

PT: OVERDOSE / 10033295

Narrative:

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090287).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

The case involves a 47-year-old male patient (smoker) suffering from chronic alcohol abuse, thrombocytopenia, and hypokalemia. On 30-Mar-2009, the patient took 20 tablets of paracetamol (no trade name provided) 500mg over a period of eight hours, indication: toothache.

On 31-Mar-2009, the patient was hospitalised with severely increased transaminases, beginning failure of coagulation, right-sided upper abdominal pain, nausea, vomiting, and diarrhoea.

On 02-Apr-2009, the patient was transferred to reporting hospital (intensive care unit) in a mildly reduced general health condition, greyish skin colouration, tenderness of right upper and middle abdomen, pronounced hepatomegaly, blood pressure of 135/70 mmHg, and pulse of 69/min; renal bed was indolent; laboratory showed severely increased transaminases; ethanol and paracetamol were not detectable in serum. The patient received transfusion of one thrombocyte concentrate, and infusion of 60 ml acetylcysteine 20% in 500 ml glucose 5%.

On 04-Apr-2009, the patient could be discharged from intensive care unit.

On 06-Apr-2009, transaminases were resolving, coagulation was normalised.

On 07-Apr-2009, the patient was discharged from hospital in improved general health condition. Outcome was reported as "recovering / resolving".

Causality determined by PV centre: very likely.

Explanatory notes on causality: Absolute overdose (20 tablets a 500mg in eight hours). As additional risk factor for increased paracetamol toxicity, possible enzyme induction by chronic alcohol abuse could be considered. Values resolving under administration of antidote.

HELIOS Klinikum Wuppertal, 26.01.2010 DE-PVZIBE-02-10164

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

akutes Leberversagen

KURZZUSAMMENFASSUNG:

akutes Leberversagen nach Einnahme von Paracetamol

BESCHREIBUNG:

30.03.09 Zahnarztbesuch, zuvor in der Nacht Einnahme von 20 Tabletten Paracetamol á 500mg innerhalb 8 Stunden, kein Alkoholexzess
seit 31.03.09 Übelkeit, mehrmaliges Erbrechen, breiiger Stuhlgang gefolgt von rechtsseitigen Oberbauchschmerzen

02.04.09 Vorstellung wegen dieser Beschwerden im Krankenhaus

bei Aufnahme: Blutdruck 135/70mmHg, Puls 69/min, leicht reduzierter Allgemeinzustand, gräuliches Hautkolorit, Druckschmerz im rechten Ober- und Mittelbauch, Transaminasen deutlich erhöht, deutliche Hepatomegalie nach Einnahme von 20 Tabletten Paracetamol á 500mg innerhalb von 8 Stunden wegen Zahnschmerzen;

am gleichen Tag Verlegung ins tertiäre Krankenhaus
nach Therapie mit Flumucil Antidot sowie Transfusion von 1 Thrombozytenkonzentrat Abfall der Transaminasen und Normalisierung der Gerinnungsparameter, Verbesserung der Hepatomegalie

THERAPIE:

02.04.2009 bis 4.04.2009 Intensivstation

02.04.2009: Transfusion von 1 Thrombozytenkonzentrat, Infusion von 60ml Flumucil 20% in 500ml G 5% (um 13:00 Uhr)

VERLAUF:

02.04.2009 (einweisende Klinik): stark erhöhte Transaminasen und beginnendes Gerinnungsversagen, rechtsseitige Oberbauchschmerzen, Übelkeit, Erbrechen, Durchfall;

02.04.2009: Druckschmerz im rechten Ober- und Mittelbauch, deutliche Hepatomegalie, Nierenlager indolent;

02.04.2009 18:45 Uhr: kein Ethanol und kein Paracetamol im Serum nachweisbar (3 Tage nach Einnahme);

Konsil Chirurgie: Abwarten, zunächst keine Lebertransplantations-Indikation

06.04.2009 Sonographie Abdomen: leichtgradige Hepatomegalie, kein Anhalt für chronische Parenchymschädigung, leichtgradiges Gallenblasenwandödem als Folge des Leberversagens, Pankreas mit Zeichen der Parenchymfibrose, Verdacht auf umschriebene kleinzystische Pankreasgangektasien, alkoholtoxische Fibrose, anamnestisch kein Hinweis für Pankreaserkrankung;

06.04.2009: Rückgang der Transaminasen, Normalisierung der Gerinnung;

07.04.2009 Entlassung im gebessertem Allgemeinzustand

07.04. Hepatitis-Serologie: HBs-Antigen negativ, HBc-IgG und -IgM negativ; HCV-IgM und IgG negativ

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-07.04.2009;

bei Aufnahme Verdacht auf akutes Leberversagen nach Einnahme von Paracetamol (anamnestisch Einnahme von 10g in 8 Stunden am 30.03.2009) wegen Zahnschmerzen in Selbstmedikation.

Der Verlauf der Symptomentwicklung ist vereinbar mit einer Paracetamolintoxikation. Als zusätzlicher Faktor kommt ein bekannter chronischer Alkoholkonsum hinzu, welcher durch Enzyminduktion die toxische Paracetamolwirkung unterstützt. Ein akuter Alkoholexzess wurde anamnestisch verneint. Da der Patient sich erst 3 Tage nach Einnahme im Krankenhaus vorstellte, ließ sich im Serum schon aus pharmakokinetischen Gründen kein Paracetamol mehr nachweisen. Alkohol war zu diesem Zeitpunkt ebenfalls negativ, was zumindest ein chronisches "Pegeltrinken" als Alternativursache für die akute Leberschädigung ausschließt. Ein akuter Konsum, vor mehreren Tagen bleibt theoretisch denkbar, ist aber aufgrund der doch noch geringfügigen chronischen Leberparenchymveränderungen und fehlender akuter Pankreatitischer Symptome (hier sonographisch stärkere chronische Schädigung) eher unwahrscheinlich.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: akutes Leberversagen

ERLÄUTERUNG: Absolute Überdosierung: 20 Tabletten á 500mg in 8 Stunden

Als zusätzlicher Risikofaktor für eine erhöhte Paracetamoltoxizität besteht eine mögliche Enzyminduktion durch chronischen Alkoholabusus. Rückläufige Werte unter Antidotgabe. Da sich der Patient 3 Tage nach erfolgter Einnahme erst im Krankenhaus vorstellte, war es aus pharmakokinetischen Gründen (Paracetamol-Halbwertszeit ca. 4 Stunden) zu erwarten, dass der dann gemessene Paracetamolspiegel bereits negativ war.

Eine akute Hepatitis B und C wurden

sanofi-aventis, 04.02.2010 DE-SANOFI-AVENTIS-A04200901005

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090287).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

The case involves a 47-year-old male patient (smoker) suffering from chronic alcohol abuse, thrombocytopenia, and hypokalemia. On 30-Mar-2009, the patient took 20 tablets of paracetamol (no trade name provided) 500mg over a period of eight hours, indication: toothache.

On 31-Mar-2009, the patient was hospitalised with severely increased transaminases, beginning failure of coagulation, right-sided upper abdominal pain, nausea, vomiting, and diarrhoea.

On 02-Apr-2009, the patient was transferred to reporting hospital (intensive care unit) in a mildly reduced general health condition, greyish skin colouration, tenderness of right upper and middle abdomen, pronounced hepatomegaly, blood pressure of 135/70 mmHg, and pulse of 69/min; renal bed was indolent; laboratory showed severely increased transaminases; ethanol and paracetamol were not detectable in serum. The patient received transfusion of one thrombocyte concentrate, and infusion of 60 ml acetylcysteine 20% in 500 ml glucose 5%.

On 04-Apr-2009, the patient could be discharged from intensive care unit.

On 06-Apr-2009, transaminases were resolving, coagulation was normalised.

On 07-Apr-2009, the patient was discharged from hospital in improved general health condition. Outcome was reported as "recovering / resolving".

Causality determined by PV centre: very likely.

Explanatory notes on causality: Absolute overdose (20 tablets a 500mg in eight hours). As additional risk factor for increased paracetamol toxicity,

possible enzyme induction by chronic alcohol abuse could be considered. Values resolving under administration of antidote.

Additional information extracted from the website of the German health authority on 27-Jan-2010. Information by PV centre: The patient went to the dentist on 30-Mar-2009; the night before, he had taken 20 tablets of 500mg paracetamol each within eight hours; alcohol excess was negated. Since 31-Mar-2009, the patient was suffering from nausea, several times of vomiting, and mushy stools, followed by right-sided upper abdominal pain. On 02-Apr-2009, the patient presented himself in hospital due to these symptoms. On the same day, he was transferred to another hospital. After transfusion of thrombocytes and application of flumucil antidote, the patient's transaminases decreased, his coagulation parameters normalised, and hepatomegaly improved. The consulted surgeon recommended to wait (no indication for liver transplantation).

Comments concerning adverse effect and causality: The course of development of symptoms is conformable with a paracetamol intoxication. A known chronic alcohol consume acts as additive factor, that promotes the toxic paracetamol effect by enzyme induction. Acute alcohol excess was negated by the patient. As the patient showed up in hospital only three days after intake, paracetamol could not be detected in serum anymore; alcohol was also negative, so that at least a chronic "level drinking" could be ruled out as alternative cause of acute liver damage. An acute consume a few days prior to the event could theoretically be taken into account, but is rather unlikely considering the still marginal chronic changes in liver parenchyma and the missing pancreatic symptoms (here sonographically more pronounced chronic damage). Acute hepatitis B and C were excluded as alternative explanation.

Causality assessment was changed by reporter to WHO score "probable" for paracetamol.

Further information is not anticipated by the PV centre.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09095148

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19401026 Age: 68 Year weight (kg): 00066 BMI_int: 21.31 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Glomerulonephritis					Unknown
2 Systemic inflammatory response syndrome					Unknown
3 Disseminated intravascular coagulation					Unknown
4 Acute agranulocytosis	20090419	20090423	5 Day		recovered/resolved
5 Cytomegalovirus infection reactivation					Unknown
6 Function liver decreased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DIFLUCAN fluconazole	Intravenous (not otherwis	00000400 Mg milligram(s) 293	20090410-20090415 6 Day	
2 NOVAMINSULFON metamizole	Intravenous (not otherwis	500 Mg milligram(s)/Day 293	20090416-20090416 -	
3 PARACETAMOL-RATIOPHARM paracetamol	Oral	00000500 Mg milligram(s) 245	20090409-20090414 6 Day	
4 PERFALGAN IV paracetamol	Intravenous (not otherwis	1000 Mg milligram(s)/Day 124	20090402-20090415 14 Day	
5 ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis	2000 Mg milligram(s)/Day 293	20090403-20090416 14 Day	
6 VANCO-CELL vancomycin	Intravenous (not otherwis	00000001 Df dosage form 293	20090406-20090416 11 Day	
7 Vanco-cell, 500 mg, Lsg, Cell Pharm vancomycin	Oral	/Day 169	20090416-20090417 2 Day	

Drugs (concomitant)				
8	ANTITHROMBIN III antithrombin iii	Intravenous (not otherwis	00000001 Df dosage form	20090417-
9	ARIXTRA fondaparinux sodium	Subcutaneous	00000002 Mg milligram(s)	20090418-
10	baldrian valeriana officinalis root	Oral	00000450 Mg milligram(s)	20090408-
11	BISOPROLOL bisoprolol fumarate	Oral	00000002 Mg milligram(s)	20090402-
12	Blutzellkonzentrat, human blood cells, packed human	Intravenous (not otherwis	00000001 Df dosage form	20090416-
13	CALCIUM EFFERVESCENT TABS calcium	Oral	/001 Day 245	20090402-
14	clont metronidazole	Oral	1200 Mg milligram(s)/Day	20090406-
15	CONCOR bisoprolol fumarate	Oral	00000002 Mg milligram(s)	20090412-
16	DIGITOXIN digitoxin	Oral	00000001 Df dosage form	20090402-
17	FRAXIPARINE nadroparin calcium	Subcutaneous	ml millilitre(s)/001 Day	20090402-
18	FURO furosemide	Intratumor	40 Mg milligram(s)/Day	20090416-
19	FURORESE furosemide	Oral	00000040 Mg milligram(s)	20090419-
20	FUROSEMIDE furosemide		00000020 Mg milligram(s)	20090409-
21	gentamicin gentamicin	Intravenous (not otherwis	00000080 Mg milligram(s)	20090414-
22	jonosteril calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis	2000 ml millilitre(s)/Day	20090410-
23	kalinor potassium chloride	Oral	/001 Day	20090402-
24	lemon ascorbic acid; mepyramine maleate; paracetamol; phenylephrine hydrochloride	Nasal	/001 Day	20090402-
25	NEUPOGEN	Subcutaneous	00000001 Df dosage form	20090421-
26	OCUTEARS		/001 Day	20090402-
27	omniflora escherichia coli, lyophilized; lactobacillus acidophilus, lyophilized; lactobacillus bifidus lyophilized	Oral	00000002 Df dosage form	20090405-
28	PANTOZOL pantoprazole sodium	Oral	00000040 Mg milligram(s)	20090402-
29	prednisolone prednisolone	Oral	00000010 Mg milligram(s)	20090402-
30	thomaejonin calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwis	00000001 Df dosage form	20090417-
31	TOREM torasemide	Oral	00000010 Mg milligram(s)	20090412-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
T-cell lymphoma			Unknown	
Ulcus ventriculi			Unknown	
Systemic inflammatory response syndrome			Unknown	
Bladder disorder	1997		Unknown	Neurogene Blasenentleerungsstörung
Critical illness polyneuropathy	2007		Unknown	
Atrial fibrillation	2002		Unknown	Arrhythmia absoluta with atrial fibrillation
Osteoporosis	200903		Unknown	
Urosepsis	200903		Unknown	
Acute renal failure			Unknown	
Bladder neurogenic	1997		Unknown	
Endocarditis infective			Unknown	Infectious culture-negative endocarditis
Fever			Unknown	
Hypertension			Unknown	
Pneumonia			Unknown	
Polyneuropathy	2007		Unknown	critical illness

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
White blood cell increased	20090302	26.6	Gpt/l	
White blood cell increased	20090323	14.1	Gpt/l	
White blood cell count decreased	20090323	2.49	Gpt/l	
White blood cell count decreased	20090410	3.65	Gpt/l	
White blood cell count decreased	20090421	3.16	Gpt/l	
White blood cell count decreased	20090422	3.55	Gpt/l	
White blood cell count normal	20090423	7.33	Gpt/l	
Neutrophil percentage increased	20090302	95.7	%	
Neutrophil percentage decreased	20090410	30	%	
Neutrophil percentage decreased	20090419	7	%	
Neutrophil percentage	20090423	41.7	%	
Neutrophil count decreased	20090419	0.32	Gpt/l	
Neutrophil count normal	20090423	3.06	Gpt/l	
Activated partial thromboplastin time	20090419	43.3: High	SEC	
Activated partial thromboplastin time	20090420	32.2: Normal	SEC	
Alanine aminotransferase	20090420	46: High	U/L	
Albumin	20090420	46.0: Low	%	
Alkaline phosphatase	20090420	209: High	U/L	
Aspiration bone marrow	20090414			
C-reactive protein	20090420	3.21: High	MG/DL	
C-reactive protein	20090419	4.10: High	MG/DL	
C-reactive protein	20090422	8.51: High	MG/DL	
C-reactive protein	20090421	4.61: High	MG/DL	
Creatinine	20090421	0.68: Normal	MG/DL	

Creatinine	20090422	0.65: Normal	MG/DL
Creatinine	20090419	0.70: Normal	MG/DL
Creatinine	20090420	0.65: Normal	MG/DL
Erythrocyte sedimentation rate	20090422	42	MM/H
Fibrin D dimer	20090419	6.93	MG/L
Fibrinogen	20090419	64: Low	MG/DL
Gamma-glutamyltransferase	20090420	213: High	U/L
International normalised ratio	20090420	1.11: Normal	%
International normalised ratio	20090419	1.30: High	%
Lactate dehydrogenase	20090420	446: High	U/L
Monocytes	20090420	0.65: Normal	/NL
Monocytes	20090421	11.0: High	%
Monocytes	20090420	16.0: High	%
Monocytes	20090419	5.0: Normal	%
Monocytes	20090419	6.0: Normal	%
Neutrophils	20090423	3.06	GPT/L
Neutrophils	20090420	0.03: Low	/NL
Neutrophils	20090410	30	%
Neutrophils	20090419	7	%
Neutrophils	20090302	95.7	%
Potassium	20090420	3.7: Normal	MMOL/L
Potassium	20090422	3.4: Normal	MMOL/L
Potassium	20090421	3.2: Low	MMOL/L
Protein	20090420	6.1: Low	G/DL
Sodium	20090422	128: Low	MMOL/L
Sodium	20090420	132: Low	MMOL/L
Sodium	20090421	130: Low	MMOL/L
Thrombocyte count	20090421	118: Low	/NL
Thrombocyte count	20090421	116: Low	/NL
Thrombocyte count	20090420	81: Low	/NL
Thrombocyte count	20090420	87: Low	/NL
Thrombocyte count	20090419	64: Low	/NL
Thrombocyte count	20090419	67: Low	/NL
Thrombocyte count	20090422	163	/NL
Urea	20090420	32: Normal	MG/DL
Leukocyte count NOS	20090302	26.6	GPT/L
Leukocyte count NOS	20090419	4.51: Normal	/NL
Leukocyte count NOS	20090420	3.56: Low	/NL
Leukocyte count NOS	20090407	2.49	GPT/L
Leukocyte count NOS	20090410	3.65	GPT/L
Leukocyte count NOS	20090423	7.33	GPT/L
Leukocyte count NOS	20090422	3.55: Low	/NL
Leukocyte count NOS	20090419	4.60: Normal	/NL
Leukocyte count NOS	20090323	14.1	GPT/L

Leukocyte count NOS	20090421	3.27: Low	/NL
Leukocyte count NOS	20090421	3.16: Low	/NL
Leukocyte count NOS	20090420	3.61: Low	/NL
Antithrombin III	20090419	82: Normal	%
Drug level NOS	20090422	98	SEC
Drug level NOS	20090420	0.3	MG/L
Drug level NOS	20090421	94	SEC
Drug level NOS	20090420	82	SEC
Quick's test	20090419	69: Low	%
Quick's test	20090420	85: Normal	%
Computerised tomogram thorax	20090421		

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 23.07.2009 DE-FAKOS-111092

Sender diagnosis: not reported

Narrative

Die Patientin war im März (02.-23.03.2009) mit einer Sepsis bei unklarem Fokus und einer Pleuritis exsudativa stationär behandelt worden (Labor am 02.03.2009: Leukozyten 26,6 Gpt/l (Normbereich: 3,8-10 Gpt/l), Neutrophile 95,7 % (Normbereich: 40-70 %)). Verlegung am 23.03.2009 in ein Krankenhaus der Maximalversorgung (Leukozyten bei Verlegung: 14,1 Gpt/l). Weiterer Abfall der Leukozyten am 07.04.2009: 2,49 Gpt/l, am 10.04.2009: 3,65 Gpt/l, Neutrophile 30 %). Im Verlauf kam es zu einer Agranulozytose (19.04.2009: Neutrophile 0,3 Gpt/l bzw. 7 %). Knochenmarkuntersuchung (14.04.2009): deutlich reduzierte Granulopoese. Am 15.04.2009 wurde die Patientin mit einem SIRS (Systemic Inflammatory Response Syndrome) auf die Intensivstation innerhalb des Hauses verlegt. Hier wurden unter anderem eine Glomerulonephritis mit Proteinurie, eine eingeschränkte Leberfunktion, eine Hypoalbuminämie, eine Reaktivierung einer persistierenden CMV (Cytomegalievirus)-Infektion, eine Verbrauchskoagulopathie, sowie eine Pleuritis exsudativa mit Verklebungen diagnostiziert. Es wurde der Verdacht einer autoimmunologischen Grunderkrankung geäußert. Gabe von 2 Erythrozytenkonzentraten am 16.04.2009. Nach Gabe von Neupogen 300 (21.-23.04.2009) Erholung der Neutrophilen (23.04.2009: 3,06 Gpt/l). Leukozytenwert vom 23.04.2009: 7,33 Gpt/l. Verlegung am 23.04.2009 in eine hämatologische Klinik. Es bestand in der Woche vor dem Indexdatum eine komplexe Arzneimitteltherapie. Suspekt sind (mindestens ein Fallbericht in Pubmed) und damit als mögliche Ursache kommen in Frage: Ceftriaxon, Vancomycin, Fluconazol, Paracetamol, Metamizol. Einnahmezeitraum der verdächtigen Arzneimittel: Rocephin (Ceftriaxon): 03.04.- 16.04.2009, Vanco-cell (Vancomycin): 06.- 17.04.2009, Diflucan (Fluconazol): 10.- 15.04.2009, Perfalgan (Paracetamol): 02.04., 12.- 15.04.2009, Paracetamol Ratiopharm: 09.04., 11.04., 13.- 14.04.2009, Novaminsulfon-Ratiopharm (Metamizol): am 16.04.2009.<dbtext>To_Char_NARRATIVE_2</dbtext>

Bristol-Myers Squibb Company, 21.08.2009 DE-Bristol-Myers Squibb Company-14714109

Sender diagnosis: not reported

Narrative

This case was received from German health authority (local file # DE-FAKOS-111092 and 09095148) and BMS Germany (local file # 2009072304).

A physician reported that a 68-year-old female patient was hospitalized for medically significant and life threatening acute agranulocytosis from 19-Apr-2009 until 23-Apr-2009 after receiving intravenous acetaminophen (Perfalgan). The patient received acetaminophen (Perfalgan) ampoules, 1x1 infusions of 1000mg daily on 02-Apr-2009 and from 12-Apr-2009 to 15-Apr-2009. The patient was also on therapy with fluconazole, dipyrone, acetaminophen (Paracetamol-ratiopharm), ceftriaxone sodium and vancomycin hcl.

The patient was admitted to the hospital from 02-Mar-2009 to 23-Mar-2009 due to a sepsis with unclear focus and a pleuritis exsudativa. Lab values on 02-Mar-2009: leukocytes 26.6 Gpt/l (normal value 3.8 to 10 Gpt/l), neutrophils 95.7% (normal value 40 to 70%). On 23-Mar-2009 the patient was transferred to another hospital (leukocytes at transferral: 14.1 Gpt/l). The leukocytes decreased further: Value on 07-Apr-2009: 2.49 Gpt/l, values on 10-Apr-2009: 3.65 Gpt/l, neutrophils: 30%. Consecutively, the patient developed an agranulocytosis (19-Apr-2009: neutrophils 0.3 Gpt/l = 7%).

Bone marrow examination on 14-Apr-2009 showed distinctly reduced granulopoiesis. On 15-Apr-2009, the patient was transferred to the intensive care unit within the hospital due to a systemic inflammatory response syndrome (SIRS). There she was diagnosed with a glomerulonephritis with proteinuria, a reduced liver function, a hypoalbuminemia, a reactivation of a persistent CMV (cytomegalovirus) infection, a disseminated intravascular coagulation as well as a pleuritis exsudativa with adhesion. An underlying auto immunologic disease was suspected. On 16-Apr-2009 the patient was administered 2 erythrocyte concentrates. After administration of filgrastim 300 from 21-Apr-2009 to 23-Apr-2009 the neutrophil values normalized (lab

values on 23-Apr-2009: neutrophils: 3.06 Gpt/l; leukocytes: 7.33 Gpt/l).

On 23-Apr-2009, the patient was transferred to a hematological clinic. In the week prior to the index date the patient was under a complex drug therapy. Administration dates of the suspected drugs: ceftriaxone sodium; 1x1 infusion of 2000mg daily IV 03-Apr-2009 to 16-Apr-2009; vancomycin hcl 500mg or 1000mg ampoules (single dose different); IV, daily application; cumulative dose number (to first reaction): 8500 mg: 06-Apr-2009 to 16-Apr-2009; vancomycin hcl 500mg oral solution: 16-Apr-2009: 750 mg, orally; 17-Apr-2009: 500 mg, orally; fluconazole; IV; 400mg ampoules; (single dose different, daily administration) cumulative dose number (to first reaction): 3200mg: 10-Apr-2009 to 15-Apr-2009; acetaminophen (Perfalgan) ,IV,1x1 infusion of 1000mg daily:02-Apr-2009, 12-Apr-2009 to 15-Apr-2009; acetaminophen (Paracetamol ratiopharm) 1x1 tablet of 500mg; orally: 09-Apr-2009, 11-Apr-2009, 13-Apr-2009 to 14-Apr-2009; dipyrone 1000 mg ampoules; administered dosage: 1x 500mg IV: on 16-Apr-2009. All the drugs were discontinued. At the time of this report the event had resolved and the patient had recovered.

Results of tests and procedures relevant to the investigation of the patient: from 22-Apr-2009 the leukocyte-values increased again.

The following lab-values were provided by the reporting physician (presented as: date/test/result/unit/Normal low range/normal high range):

02-Mar-2009/ White blood cell increased/ 26.6/ Gpt/l /3.8/ 10;
 23-Mar-2009/ White blood cell increased/ 14.1/ Gpt/l /4.5/ 11.0;
 23-Mar-2009/ White blood cell count/ 2.49/ Gpt/l /4.5 /11.0;
 10-Apr-2009/ White blood cell count/ 3.65/ Gpt/l/ 4.5/ 11.0;
 21-Apr-2009/ White blood cell count/ 3.16/ Gpt/l /4.5 /11.0;
 22-Apr-2009/ White blood cell count/ 3.55/ Gpt/l /4.5 /11.0;
 23-Apr-2009/ White blood cell count normal/ 7.33/ Gpt/l / 4.5 /11.0;
 02-Mar-2009/ Neutrophil percentage/ 95.7/ %/ 40/ 70;
 10-Apr-2009/ Neutrophil percentage/ 30/ %/ 40 /70;
 19-Apr-2009/ Neutrophil percentage/ 7/ %/ 40/ 70;
 23-Apr-2009/ Neutrophil percentage/ 41.7/ %/ 40/ 70;
 19-Apr-2009/ Neutrophil count decreased/ 0.32/ Gpt/l / 1.8 /7.7;
 23-Apr-2009/ Neutrophil count normal/ 3.06/ Gpt/l /1.8/ 7.7;

According to the reporting physician the patient received 19 concomitant drugs (not further specified).

The patient's medical history included a neurogenic bladder disorder (from 1997), a critical illness polyneuropathy (from 2007), atrial fibrillation (Arrhythmia absoluta with atrial fibrillation; from 2002), osteoporosis and urosepsis (from Mar-2009).

The reporting physician considered the event of acute agranulocytosis to be possibly related to fluconazole, dipyrone, acetaminophen (Paracetamol-ratiopharm and Perfalgan), ceftriaxone sodium, vancomycin hcl (IV application and oral solution).

BMS medical evaluation comment:

This patient with sepsis on ceftriaxone, vancomycin, fluconazole, dipyrone and acetaminophen developed agranulocytosis and was diagnosed with systemic inflammatory response syndrome, glomerulonephritis, reduced liver function, reactivation of cytomegalovirus infection and disseminated intravascular coagulation. Sepsis and multiple medication use most likely rendered this patient susceptible to agranulocytosis and the reported events.

Supplemental information received from the physician in the form of hospital discharge summary on 17-Aug-2009 indicated the patient was hospitalized from 19-Apr-2009 to 23-Apr-2009 and again from 23-Apr-2009 to 30-Apr-2009. The patient's immune status was assessed on 20-Apr-2009 and showed distinct granulopenia, mild monocytosis and CD8+ T-cell-expansion, corresponding inverse CD4/CD8 ratio. The patient's immune status was assessed on 21-Apr-2009 and showed signs of cytomegalovirus (CMV) reactivation, fever of unknown origin and agranulocytosis. On 21-Apr-2009, Computed Tomography (CT) thorax with contrast medium revealed systemic inflammatory response syndrome (SIRS). The patient was diagnosed with status post urosepsis with concomitant CMV reactivation during hospitalization from 23-Apr-2009 to 30-Apr-2009.

The patient was currently under granulocyte-colony stimulating factor (G-CSF) therapy left shift with maturing granulopoiesis and leukocytosis in the peripheral blood, no hints of a hematological systemic disease in the bone marrow. The diagnosis was considerably augmented, left-shifted granulopoiesis, maturing erythropoiesis and megakaryopoiesis in a hypercellular iliac crest biopsy specimen with marginal reactive interstitial lymphocytosis. It was reported that there was no need for action from hematological perspective. The patient did not consider herself to be discharged home and she will be retransitioned. At the time of report, therapy with acetaminophen (Perfalgan) was not restarted.

Laboratory tests on 19-Apr-2009 included Creatinine (normal range: less than 0.1): 0.70 mg/dl, C-reactive protein (CRP) (normal range: less than 0.5): 4.10 mg/dl, leukocytes (normal range: 4.5 to 11.0): 4.51 and 4.60 /nl, thrombocytes (normal range: 150 to 400): 67 and 64 /nl, monocytes (normal range: 2 to 8): 5 and 6 %, quick test value (normal range: 70-130) was 69%, Quick International Normalized Ratio (INR) (normal range: 0.9 to 1.25): 1.30, Activated partial thromboplastin time (APTT) (normal range: 26 to 40): 43.3 seconds, fibrinogen(normal range: 150 to 40): 64 mg/dl, D-dimer (normal range: less than 0.5): 6.93 mg/l.

Laboratory tests on 20-Apr-2009 included creatine: 0.65, urea (normal range: 14 to 46): 32 mg/dl, protein (normal range: 6.5 to 8.7): 6.1 g/dl, albumin (normal range: 55.8 to 66.1): 46.0%, CRP: 3.21 mg/dl, alanine aminotransferase (ALT) at 37 degree celsius (normal range: less than 34): 46 U/l, alkaline phosphatase (normal range: 35 to 104): 209 U/l, gamma-glutamyl transferase (GGT) (normal range: less than 38): 213 U/l, lactate dehydrogenase (LDH) (normal value: less than 247): 446 U/l, leukocytes: 3.56 and 3.61 /nl, thrombocytes: 87 and 81 /nl, neutrophil: 0.03 /nl, monocytes: 16% , monocytes: 0.65 /nl. Quick test value was 85, Quick INR was 1.11, aPTT: 32.2 sec, heparin time: 82 sec. gentamicin level was 0.3 mg/l.

Laboratory tests on 21-Apr-2009 included creatine: 0.68 mg/dl, CRP: 4.61 mg/dl, leukocytes: 3.16 and 3.27 /nl, thrombocytes: 116 and 118 /nl,

monocytes: 11%, heparin time: 94 sec. Laboratory tests on 22-Apr-2009 included creatine: 0.65 mg/dl, CRP: 8.51 mg/dl, leukocytes: 3.55 /nl, thrombocytes: 163 /nl, heparin time: 98 sec, erythrocyte sedimentation rate: 42 mm/h.

The patient's medical history included T-cell lymphoma, acute renal failure with electrolyte derailment and respiratory partial insufficiency, ulcer ventriculi, febrile temperature and hypertension.

BMS Medical Evaluation Comment:

This patient with sepsis, T-cell lymphoma and acute renal failure was on ceftriaxone, vancomycin, fluconazole, dipyrone and acetaminophen and developed agranulocytosis and was diagnosed with systemic inflammatory response syndrome, glomerulonephritis, reduced liver function, reactivation of cytomegalovirus infection and disseminated intravascular coagulation. sepsis, T-cell lymphoma and treatment with ceftriaxone, vancomycin, fluconazole, dipyrone was most likely rendered this patient susceptible to agranulocytosis and other reported events. Hence the reported events were not likely due to the acetaminophen.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09104979

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19150321	Age: 94 Year	weight (kg): 60	BMI_int: 24.97	Age fetus:
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Patientdeath

deathdate: 20090901	deathcause: Death NOS
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Death NOS	20090901	20090901		18 Day	fatal
2 Upper abdominal pain					Unknown
3 Icterus					Unknown
4 Hepatitis acute	20090720	2009		8 Day	recovering/resolving
5 Hepatitis acute	20090710	2009			recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Arcoxia, 90 mg, Filmtabl, MSD Sharp & Dohme etoricoxib	Oral	90 Mg milligram(s)/Day 82	20090706-20090713 8 Day	
2 Baypen 2g i.v., Bayer mezlocillin sodium	Intravenous (not otherwis	2 G gram(s)/Day 120	20090713-20090724 12 Day	
3 Cipramil 10 mg Tabl., Lundbeck citalopram	Oral	10 Mg milligram(s)/Day 245	20090714-20090722 9 Day	
4 Pantozol, 20 mg, Tabl (mr), Nycomed pantoprazole	Oral	20 Mg milligram(s)/Day 94	20090703-20090713 11 Day	
5 Paracetamol, 500 mg, Tabl, Ratiopharm paracetamol	Oral	1500 Mg milligram(s)/Day 245	20090703-20090713 11 Day	

Drugs (concomitant)

6 Clexane, 4000 IE, Amp, Sanofi-Aventis enoxaparin	Subcutaneous	4000 Iu international unit(s) 120	20090704-20090710 7 Day	
7 nitro glyceryl trinitrate	Respiratory (inhalation)	400 µg microgram(s)/Day 109	20090707-20090707 -	
8 Novalgin, Trpf, Sanofi-Aventis metamizole	Oral	/ 158	20090704-20090716 13 Day	
9 Votum, 10 mg, Filmtabl, Berlin-Chemie olmesartan medoxomil	Oral	/Day 82	2003-20090722 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac failure			Yes	
Hypertension arterial			Yes	
Coxarthrosis			Yes	Koxarthrose rechts
Lumboischialgia			Yes	
Hospitalisation			Yes	
Papilla of Vater stenosis			Unknown	papillary stenosis

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20090713	215	U/l	
Alanine aminotransferase increased	20090716	109	U/l	
Aspartate aminotransferase increased	20090713	196	U/l	
Aspartate aminotransferase increased	20090716	153	U/l	
Bilirubin increased	20090713	5.4	mg/dl	
Bilirubin increased	20090716	2.1	mg/dl	
Alkaline phosphatase increased	20090713	377	U/l	
Alkaline phosphatase increased	20090716	355	U/l	
Alanine aminotransferase increased	20090724	207	U/l	
Alanine aminotransferase increased	20090819	68	U/l	
Alanine aminotransferase increased	20090824	38	U/l	
Alanine aminotransferase increased	20090720	297	U/l	
Alkaline phosphatase increased	20090824	274	U/l	
Alkaline phosphatase increased	20090724	483	U/l	
Alkaline phosphatase increased	20090819	373	U/l	
Alkaline phosphatase increased	20090720	512	U/l	
Aspartate aminotransferase increased	20090819	106	U/l	
Aspartate aminotransferase increased	20090824	62	U/l	
Aspartate aminotransferase increased	20090724	156	U/l	
Aspartate aminotransferase increased	20090720	249	U/l	
Bilirubin increased	20090824	4.5	mg/dl	
Bilirubin increased	20090716	2.1	mg/dl	
Bilirubin increased	20090819	7.8	mg/dl	
Bilirubin increased	20090724	1.3	mg/dl	
Bilirubin increased	20090720	2.4	mg/dl	
Bilirubin increased	20090713	5.4	mg/dl	
Lipase increased	20090819	3598	U/l	
Lipase increased	20090824	68	U/l	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 12.08.2009 DE-FAKOS-170280

Sender diagnosis: not reported

Narrative**Sender's comment:**

initial report (more info follows).

Reporter's comment:

009507513, 18.08.2009 DE-MERCK-0908DEU00022

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 94-year-old female patient who, on 06-JUL-2009 was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day (indication not reported). Secondary suspect therapy included acetaminophen (PARACETAMOL RATIOPHARM), 500 mg tablet, 500 mg three times a day taken from 03-JUL-2009 until 13-JUL-2009 (indication not reported).

On 10-JUL-2009, the patient was diagnosed with acute hepatitis and was hospitalized. On 13-JUL-2009, laboratory analysis revealed serum alanine aminotransferase 215 U/l, serum alkaline phosphatase 377 U/l, total serum bilirubin 5.4 mg/dl and serum aspartate aminotransferase 196 U/l. That same day, therapy with etoricoxib was discontinued. Subsequently, the patient's acute hepatitis improved. On 16-JUL-2009, laboratory analysis revealed serum alanine aminotransferase 109 U/l, serum alkaline phosphatase 355 U/l, total serum bilirubin 2.1 mg/dl and serum aspartate aminotransferase 153 U/l, i.e. the patient was recovering from acute hepatitis.

The reporting pharmacovigilance center/physician felt that acute hepatitis was related to therapy with etoricoxib and/or acetaminophen.

Acute hepatitis was considered to be an Other Important Medical event by the agency/physician.

Additional information has been requested.

009507513, 20.08.2009 DE-MERCK-0908DEU00022

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 94-year-old female patient who, on 06-JUL-2009 was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day (indication not reported). Secondary suspect therapy included acetaminophen (PARACETAMOL RATIOPHARM), 500 mg tablet, 500 mg three times a day taken from 03-JUL-2009 until 13-JUL-2009 (indication not reported).

On 10-JUL-2009, the patient was diagnosed with acute hepatitis and was hospitalized. On 13-JUL-2009, laboratory analysis revealed serum alanine aminotransferase 215 U/l, serum alkaline phosphatase 377 U/l, total serum bilirubin 5.4 mg/dl and serum aspartate aminotransferase 196 U/l. That same day, therapy with etoricoxib was discontinued. Subsequently, the patient's acute hepatitis improved. On 16-JUL-2009, laboratory analysis revealed serum alanine aminotransferase 109 U/l, serum alkaline phosphatase 355 U/l, total serum bilirubin 2.1 mg/dl and serum aspartate aminotransferase 153 U/l, i.e. the patient was recovering from acute hepatitis.

The reporting pharmacovigilance center/physician felt that acute hepatitis was related to therapy with etoricoxib and/or acetaminophen.

Acute hepatitis was considered to be an Other Important Medical event by the agency/physician.

Additional information has been requested.

PVZ-FAKOS, Klinische Pharmakologie, 05.10.2009 DE-FAKOS-170280

Sender diagnosis: not reported

Narrative

Aufnahme der Patientin in ein Krankenhaus am 03.07.2009. Grund waren immobilisierende Lumbalgien und eine Koxarthrose. Es erfolgte eine Schmerztherapie im Krankenhaus mit Paracetamol Ratiopharm und Arcoxia. Darunter kam es zu Oberbauchschmerzen und Ikterus, woraufhin die Patientin am 10.07.2009 innerhalb des Hauses verlegt wurde. Der Laborbefund vom 13.07.2009 zeigt den über dreifach erhöhten Anstieg der Leberwerte: ALAT 215 U/l (Normbereich: <35 U/l), ASAT 196 U/l (Normbereich: <32 U/l), Alkalische Phosphatase (AP) 377 U/l (Normbereich: 35-104 U/l), Bilirubin 5,4 mg/dl (Normbereich: <1,2 mg/dl). Nach Absetzen der Schmerzmedikation rückläufige Transaminasen am 16.07.2009: ALAT 109 U/l, ASAT 153 U/l, AP 355 U/l, Bilirubin 2,1 mg/dl.

Ab 13.07.2009 Beginn einer Antibiose mit Baypen 2g i.v. und ab 14.07.2009 Gabe von Cipramil. Wiederanstieg der Leberwerte am 20.07.2009: ALAT 297 U/l, ASAT 249 U/l, AP 512 U/l, Bilirubin 2,4 mg/dl, die am 24.07.2009 wieder rückläufig waren: ALAT 207 U/l, ASAT 156 U/l, AP 483 U/l, Bilirubin 1,3 mg/dl. Baypen und Cipramil werden verdächtigt, diesen erneuten Anstieg der Leberenzyme verursacht zu haben.

Serologisch erfolgte der Ausschluss einer viralen Hepatitis. Am 31.07.2009 erfolgte die Entlassung aus der stationären Behandlung in eine Kurzzeitpflege. Am 19.08.2009 nach Erbrechen und Exsikkose Wiederaufnahme ins Krankenhaus. Laborwerte vom 19.08.2009: ALAT 68 U/l, ASAT 106 U/l, AP 373 U/l, Bilirubin 7,8 mg/dl, Lipase 3598 U/l. Am 21.08.2009 Feststellung einer Papillenstenose durch ERCP mit anschließender

Papillotomie. Nach kurzer Besserung verstarb die Patientin am 01.09.2009 im Krankenhaus.

Die UAW hat sich im Krankenhaus entwickelt.

Als Auslöser des ersten Ereignisses (Hepatitis Index am 10.07.2009) werden folgende Arzneimittel verdächtigt:

Paracetamol Ratiopharm Tabl, Einnahmezeitraum: 03. - 13.07.2009,

Arcoxia 90 mg Filmtabl, Einnahmezeitraum: 06. - 13.07.2009,

Pantozol 20mg Tabl, Einnahmezeitraum: 03. - 13.07.2009.

Als Auslöser des zweiten Ereignisses (Hepatitis Index am 20.07.2009) werden folgende Arzneimittel verdächtigt:

Baypen 2g i.v., Bayer, Einnahmezeitraum: 13. - 24.07.2009 (1 x täglich)

Cipramil 10 mg Tabl., Lundbeck, Einnahmezeitraum: 14. - 22.07.2009 (1x1 tägl.)

Die Kausalitäten werden jeweils mit „möglich“ bewertet.“ Eine am 21.08.2009 festgestellte Papillenstenose muss als alternative Ursache angesehen werden.<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 12.10.2009 DE-MERCK-0908DEU00022

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 94-year-old female patient with lumboischialgia, coxarthrosis, arterial hypertension and cardiac failure.

On 03-JUL-2009, the patient was hospitalized due to lumboischialgia and coxarthrosis. That day, the patient was placed on therapy with acetaminophen (PARACETAMOL RATIOPHARM), 500 mg tablet, 500 mg three times a day for the treatment of lumboischialgia and coxarthrosis, and pantoprazole sodium (PANTOZOL), 20 mg tablet, 20 mg once a day (indication not reported).

On 06-JUL-2009, the patient was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day for the treatment of lumboischialgia and coxarthrosis.

Concomitant therapy included enoxaparin sodium (CLEXANE), nitroglycerin (NITRO), dipyrrone (NOVALGIN) and olmesartan medoxomil (VOTUM).

On 10-JUL-2009, the patient was transferred to an other ward due to upper abdominal pain and icterus.

On 13-JUL-2009, laboratory analysis revealed that ALT was 215 U/l, alkaline phosphatase was 377 U/l, total serum bilirubin was 5.4 mg/dl and AST was 196 U/l. The patient was diagnosed with acute hepatitis. That same day, therapy with etoricoxib, acetaminophen and pantoprazole was discontinued. On 13-JUL-2009, therapy with mezlocillin sodium (BAYPEN), 2 gram iv, 2 gram daily, was started and, on 14-JUL-2009, therapy with citalopram hydrobromide (CIPRAMIL), 10 mg tablet, 10 mg once a day, both given as antibiotics.

On 16-JUL-2009, ALT was 109 U/l, AP was 355 U/l, total serum bilirubin 2.1 mg/dl and AST was 153 U/l, i.e. the patient was recovering from acute hepatitis.

On 20-JUL-2009, the patient was diagnosed with a second episode of acute hepatitis: ALT was 297 U/l, AST was 249 U/l, AP was 512 U/l and bilirubin was 2.4 mg/dl. On 22-JUL-2009, therapy with citalopram was discontinued and, on 24-JUL-2009, therapy with mezlocillin was stopped.

On 24-JUL-2009, ALT was 207 U/l, AST was 156 U/l, AP was 483 U/l and bilirubin was 1.3 mg/dl. The patient was recovering from this second episode of acute hepatitis.

Clinical serology test (date not provided) was performed and viral hepatitis was ruled out.

On 31-JUL-2009, the patient was discharged from hospital.

On 19-AUG-2009, the patient was re-hospitalized due to exsiccosis and vomiting. ALT was 68 U/l, AST was 106 U/l, AP was 373 U/l, bilirubin was 7.8 mg/dl and lipase was 3598 U/l. On 21-AUG-2009, endoscopic retrograde cholangiopancreatography revealed ampulla of Vater stenosis and an endoscopic retrograde sphincterotomy was performed. The patient's condition improved but on 01-SEP-2009 the patient died at hospital. Cause of death was neither reported by the agency nor coded as an AE.

The reporting pharmacovigilance center felt that the first episode of acute hepatitis was possibly related to therapy with etoricoxib and/or acetaminophen and/or pantoprazol and that the second episode was possibly related to therapy with citalopram and/or mezlocillin and not to therapy with etoricoxib. Ampulla of Vater stenosis was discussed as alternative cause for acute hepatitis.

Both episodes of acute hepatitis prolonged hospitalization and were considered as Other Important Medical events by the pharmacovigilance center/agency/physician.

Additional information is not expected.

NOVARTIS, 13.10.2009 DE-002147023-PHHY2009DE43469

Sender diagnosis: not reported

Narrative

Initial case from a healthcare professional reported to BfArM (ref. no. DE-BFARM-09104979) and forwarded by BfArM to Nycomed on 06 Oct 2009 and further forwarded by Nycomed on 08 Oct 2009. The patient started therapy with pantoprazole, 20 mg once daily, orally, on 03 Jul 2009, due to an unknown indication. Co-suspect medications included paracetamol and etoricoxib, started on 03 Jul 2009 and 06 Jul 2009, respectively, after the patient was hospitalized due to lumbar pain and coxarthrosis. The patient's current condition consisted of lumboischialgia, arterial hypertension and, cardiac failure and coxarthrosis. On 10 Jul 2009 (one week after the first dose of pantoprazole and paracetamol and 4 days after the first dose of etoricoxib), the patient developed upper abdominal pain and icterus, which led to the patient's transfer in the hospital (NOS). On 13 Jul 2009, lab work-up revealed ALAT 215 IU/L (normal range (NR): < 35 IU/L), ASAT 196 IU/L (NR: < 32 IU/L), alkaline phosphatase (AP) 377 IU/L (NR: 35 - 104 IU/L) and bilirubin 5.4 mg/dL (NR: < 1.2 mg/dL). An acute hepatitis was diagnosed on an unknown date. Pantoprazole, paracetamol and etoricoxib were discontinued on 13 Jul 2009. The patient was started on mezlocilline on that day, and, on citalopram, the following day. On 16 Jul 2009, the patient's hepatic values had decreased (ALAT: 109 IU/L, ASAT: 153 IU/L, AP: 355 IU/L and bilirubin: 2.1 mg/dL). However, on 20 Jul 2009, the hepatic values increased again (ALAT: 297 IU/L, ASAT: 249 IU/L, AP: 512 IU/L and bilirubin: 2.4 mg/dL). Both citalopram and mezlocilline were suspected and discontinued on 22 Jul 2009 and 24 Jul 2009, respectively. On 31 Jul 2009, the patient was discharged with a short-term treatment. On 19 Aug 2009, the patient was hospitalized again due to vomiting and dehydration. On that date, hepatic values were: ALAT 68 IU/L, ASAT 106 IU/L, AP 373 IU/L and bilirubin 7.8 mg/dL. On 21 Aug 2009, papillary stenosis was detected by endoscopic retrograde cholangiopancreatography, leading to papillotomy. After a short improvement, the patient eventually died on 01 Sep 2009 in the hospital from an unspecified cause.

LUNDBECK, 14.10.2009 DE-LUNDBECK-DKLU1040098

Sender diagnosis: not reported

Narrative

Case reference number DKLU1040098, is a spontaneous case received from the Regional Pharmacovigilance Center PVZ-FAKOS (ref no.: DE-FAKOS-170280) via the BfArM (ref no.: DE-BFARM-09104979):

The patient was hospitalized on 03/Jul/2009 due to immobilizing lumbago and coxarthrosis.

Pain therapy was initiated and the patient was administered etoricoxib, orally, 90 mg/day, from 06/Jul/2009 to 13/Jul/2009, paracetamol, orally, 1500 mg/day, from 03/Jul/2009 to 13/Jul/2009 and pantoprazole, orally, 20 mg/day, from 03/Jul/2009 to 13/Jul/2009.

On 10/Jul/2009 the patient experienced a first episode of ACUTE HEPATITIS with pain in the upper abdomen and icterus.

On 13/Jul/2009 laboratory values showed alanine aminotransferase of 215 U/l (normal range: 0-34 U/l), aspartate aminotransferase of 196 U/l (normal range: 0-31 U/l), bilirubin of 5.4 mg/dl (normal range: < 1.1 mg/dl) and alkaline phosphatase of 377 U/l (normal range: 35-104 U/l). After discontinuation of pain therapy laboratory values improved with alanine aminotransferase of 109 U/l, aspartate aminotransferase of 153 U/l, bilirubin of 2.1 mg/dl and alkaline phosphatase of 355 U/l on 16/Jul/2009.

Etoricoxib, paracetamol and pantoprazole were discontinued on 13/Jul/2009. The patient was transferred to another ward within the hospital and was recovering from acute hepatitis.

The reporter assessed the causal relationship to etoricoxib, paracetamol and pantoprazole as possibly related according to WHO Global Introspection Score, since etoricoxib is labeled with 'Hepatitis', paracetamol is labeled with 'Increase of transaminases' and pantoprazole is labeled with 'Severe liver cell disorder involving icterus and liver failure'.

Afterwards the patient was administered Cipramil (Generic name: citalopram, strength: 10 mg), orally, 10 mg/day, from 14/Jul/2009 to 22/Jul/2009.

Co-suspected medication included mezlocilline, 2 g as intravenous injection, from 13/Jul/2009 to 24/Jul/2009.

On 20/Jul/2009 the patient experienced a second episode of ACUTE HEPATITIS.

On 20/Jul/2009 laboratory values showed alanine aminotransferase of 297 U/l, aspartate aminotransferase 249 U/l, bilirubin of 2.4 mg/dl and alkaline phosphatase of 512 U/l. After discontinuation of citalopram and mezlocilline laboratory values improved with alanine of 207 U/l, aspartate aminotransferase of 156 U/l, bilirubin of 1.3 mg/dl and alkaline phosphatase of 274 U/l on 24/Jul/2009. Viral hepatitis was excluded by serology.

Citalopram was discontinued on 22/Jul/2009 and the co-suspected mezlocilline was discontinued on 24/Jul/2009. The patient was recovering from acute hepatitis and was discharged from hospital into short term care on 31/Jul/2009.

The reporter assessed the causal relationship to citalopram and mezlocilline as possibly related according to WHO Global Introspection Score, since Cipramil is labeled with 'Hepatitis' and mezlocilline is labeled with 'Increased bilirubin, temporarily increased SGOT and SGPT, increased alkaline phosphatase'.

On 19/Aug/2009 the patient experienced vomiting and exsiccosis and was again hospitalized. Laboratory values showed alanine aminotransferase of 68 U/l, aspartate aminotransferase of 106 U/l, bilirubin of 7.8 mg/dl, alkaline phosphatase of 373 U/l and lipase of 3598 U/l (normal range: < 60 U/l).

On 21/Aug/2009 an endoscopic retrograde cholangiopancreatography showed papillae stenosis. Papillotomy was performed. The patient improved temporarily, but died on 01/Sep/2009 in hospital.

Papillae stenosis was stated as possible alternative reason for the event.

The patient's medical history comprised lumboischialgia, arterial hypertension, cardiac failure and coxarthrosis right.

Concomitant medication included enoxaparin, 4000 IU as subcutaneous injection, from 04/Jul/2009 to 10/Jul/2009 and glyceril trinitrate, 400 mcg as inhalation on 07/Jul/2009. Concomitant medication further included metamizole, orally, 1500 mg/day, from 04/Jul/2009 to 09/Jul/2009 and 2250 mg/day on 16/Jul/2009 and olmesartan, orally, 30 mg/day, from 2003 to 02/Jul/2009 and 10 mg/day, from 03/Jul/2009 to 22/Jul/2009. Past medication included paracetamol.

No further information is expected and the case is considered closed.

Bayer Vital GmbH, 14.10.2009 DE-BAYER-200934953GPV

Sender diagnosis: not reported

Narrative

This solicited report submitted by a health professional in Germany from the FAKOS- Project (systematically screens for adverse drug reactions in certain hospitals) was identified on 06-Oct-2009 in an internet-based pool of cases of BfArM, the regulatory health authority in Germany, reported from a physician via local pharmacovigilance center in Germany, referring to a 94 year-old female (height: 155 cm, weight: 60 kg).

Medical and drug history:

Patient's medical history included lumboischialgia, arterial hypertension, cardiac failure and coxarthrosis.

Patient received concomitant drugs (see relevant section in this report).

Patient received the following drugs, reported as 'suspect drugs' (indications unspecified):

- intravenous Baypen (mezlocillin) 2 g injection once daily from 13-Jul-2009 until being withdrawn on 24-Jul-2009,
- oral Cipramil (citalopram) 10 mg tablet once daily from 14-Jul-2009 until being withdrawn on 22-Jul-2009,
- oral Pantozol (pantoprazole) 20 mg gastro-resistant tablet once daily from 03-Jul-2009 until being withdrawn on 13-Jul-2009,
- oral Paracetamol Ratiopharm 500 mg tablet thrice daily from 03-Jul-2009 until being withdrawn on 13-Jul-2009,
- oral Arcoxia (etoricoxib) 90 mg film-coated tablet once daily from 06-Jul-2009 until being withdrawn on 13-Jul-2009.

Details surrounding the onset, course and outcome of the adverse reaction:

On 10-Jul-2009, patient experienced ACUTE HEPATITIS, considered serious by the reporter as medical important event, requiring hospitalization/prolongation of existing hospitalization. The event was resolving.

On 20-Jul-2009, patient again experienced ACUTE HEPATITIS, considered serious by the reporter as medical important event, requiring hospitalization/prolongation of existing hospitalization. The event was resolving at the time of the report.

Further details:

Patient was admitted to hospital on 03-Jul-2009 for lumbalgias which severely inhibited the mobility of the patient, as well as for coxarthrosis. In hospital, patient underwent pain therapy with Paracetamol and Arcoxia (etoricoxib) causing upper abdominal pain and icterus.

Thus on 10-Jul-2009, patient was referred to another ward.

On 13-Jul-2009, laboratory findings of showed a more than threefold increased rise in liver values (see 'Lab test' section).

From 13-Jul-2009 onward, the patient was treated with Baypen / mezlocillin intravenously, at a dose of 2 g, and from 14-Jul-2009 onward, with Cipramil (citalopram).

On 16-Jul-2009, after withdrawal of the pain medication, examination of transaminases showed decreasing results (see 'Lab test' section).

On 20-Jul-2009, again increase in liver values (see 'Lab test' section). Examination on 24 Jul 2009 revealed

decreasing values again:(see 'Lab test' section).

Baypen (mezlocillin) and Cipramil (citalopram) were suspected having caused the relapse in increase in liver enzymes. Serological exclusion of a viral hepatitis.

On 31-Jul-2009, patient was discharged from hospital and was referred to a short-term care institution.

On 19-Aug-2009, patient was hospitalised again for vomiting and exsiccosis. Liver values were increased again (see 'Lab test' section).

On 21-Aug-2009, papillary stenosis was revealed by endoscopic retrograde cholangiopancreatography with a subsequent papillotomy. The patient's condition improved for a short time, however, patient died (DEATH NOS) on 01-Sep-2009 in the hospital.

Reporter's clinical assessment:

The reporter assessed the diagnosis ACUTE HEPATITIS (onset 10-Jul-2009) to be possibly associated with Pantozol (pantoprazole), Paracetamol and Arcoxia (etoricoxib).

The reporter assessed the diagnosis ACUTE HEPATITIS (onset 20-Jul-2009) to be possibly associated with Baypen (mezlocillin) and Cipramil (citalopram).

A papillary stenosis, established on 21 Aug 2009, has been assessed as alternative cause.

009507513, 15.10.2009 DE-MERCK-0908DEU00022

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 94-year-old female patient with lumboischialgia, coxarthrosis, arterial hypertension and cardiac failure.

On 03-JUL-2009, the patient was hospitalized due to lumboischialgia and coxarthrosis. That day, the patient was placed on therapy with acetaminophen (PARACETAMOL RATIOPHARM), 500 mg tablet, 500 mg three times a day for the treatment of lumboischialgia and coxarthrosis, and pantoprazole sodium (PANTOZOL), 20 mg tablet, 20 mg once a day (indication not reported).

On 06-JUL-2009, the patient was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day for the treatment of lumboischialgia and coxarthrosis.

Concomitant therapy included enoxaparin sodium (CLEXANE), nitroglycerin (NITRO), dipyrrone (NOVALGIN) and olmesartan medoxomil (VOTUM).

On 10-JUL-2009, the patient was transferred to an other ward due to upper abdominal pain and icterus.

On 13-JUL-2009, laboratory analysis revealed that ALT was 215 U/l, alkaline phosphatase was 377 U/l, total serum bilirubin was 5.4 mg/dl and AST was 196 U/l. The patient was diagnosed with acute hepatitis. That same day, therapy with etoricoxib, acetaminophen and pantoprazole was discontinued. On 13-JUL-2009, therapy with mezlocillin sodium (BAYPEN), 2 gram iv, 2 gram daily, was started and, on 14-JUL-2009, therapy with citalopram hydrobromide (CIPRAMIL), 10 mg tablet, 10 mg once a day, both given as antibiotics.

On 16-JUL-2009, ALT was 109 U/l, AP was 355 U/l, total serum bilirubin 2.1 mg/dl and AST was 153 U/l, i.e. the patient was recovering from acute hepatitis.

On 20-JUL-2009, the patient was diagnosed with a second episode of acute hepatitis: ALT was 297 U/l, AST was 249 U/l, AP was 512 U/l and bilirubin was 2.4 mg/dl. On 22-JUL-2009, therapy with citalopram was discontinued and, on 24-JUL-2009, therapy with mezlocillin was stopped.

On 24-JUL-2009, ALT was 207 U/l, AST was 156 U/l, AP was 483 U/l and bilirubin was 1.3 mg/dl. The patient was recovering from this second episode of acute hepatitis.

Clinical serology test (date not provided) was performed and viral hepatitis was ruled out.

On 31-JUL-2009, the patient was discharged from hospital.

On 19-AUG-2009, the patient was re-hospitalized due to exsiccosis and vomiting. ALT was 68 U/l, AST was 106 U/l, AP was 373 U/l, bilirubin was 7.8 mg/dl and lipase was 3598 U/l. On 21-AUG-2009, endoscopic retrograde cholangiopancreatography revealed ampulla of Vater stenosis (not coded by agency) and an endoscopic retrograde sphincterotomy was performed. The patient's condition improved but on 01-SEP-2009 the patient died at hospital. Cause of death was neither reported by the agency nor coded as an AE.

The reporting pharmacovigilance center felt that the first episode of acute hepatitis was possibly related to therapy with etoricoxib and/or acetaminophen and/or pantoprazole and that the second episode was possibly related to therapy with citalopram and/or mezlocillin and not to therapy with etoricoxib. Ampulla of Vater stenosis was discussed as alternative cause for acute hepatitis.

Both episodes of acute hepatitis prolonged hospitalization and were considered as Other Important Medical events by the pharmacovigilance center/agency/physician.

Additional information is not expected.

This is an amended report: The narrative was changed to reflect the fact that the agency did not code ampulla of Vater stenosis as AE.

case: DE-BFARM-09109244

Patient

Initials: Sex: Female Birth: Age: 44 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leg edema					recovered/resolved
2 Fluid retention				4 Week	recovered/resolved
3 Weight gain					Unknown
4 Coagulation factor increased				4 Week	recovered/resolved
5 Abdominal discomfort					
6 Drug interaction					recovered/resolved
7 Autoimmune hepatitis				4 Week	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol paracetamol		/	-	Headache
2 Remifemin cimicifuga racemosa root	Unknown	5 Mg milligram(s)/Day	-	Drug use for unknown indication

Drugs (concomitant)

3	Pyridostigmine pyridostigmine	/	- -	Drug use for unknown indication
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Myasthenia gravis			Unknown	
Factor V Leiden heterozygote			k.A.	
Thymectomy			Unknown	thymectomy
Acute anaphylactic reaction			k.A.	acute anaphylactic reaction
Haemangioma			Yes	haemangioma

Primary source

Qualification:	Physician	observestudytype:
Literature:	Zimmermann R, Witte A, Voll RE, Strobel J, Frieser M. Coagulation activation and fluid retention associated with the use of black cohosh: a case study. Climacteric 2009; DOI: 10.1080/13697130902939921	
Qualification:	Physician	observestudytype:
Literature:	Zimmermann R, Witte A, Voll RE, Strobel J, Frieser M. Coagulation activation and fluid retention associated with the use of black cohosh: a case study. Climacteric 2009; DOI: 10.1080/13697130902939921	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time		0.98	/l	
Creatine kinase		83	U/L	
Creatinine		57.5	MCMOL/L	
Hemoglobin		123	G/L	
Platelet count		181	GIGA/L	
Potassium		3.5	MVAL/L	

	DE-BFARM-09109244	
SGOT	66	U/L
SGPT	104	IU/L
Sodium	138	MVAL/L
White blood cells	5.3	GIGA/L
Activated partial thromboplastin time	31.4	sec.
Antinuclear antibody	320	
Coagulation factor VIII level	223	Percent
Creatine kinase	83	U/L
Fibrin D dimer	1304	mg/mL
Hemoglobin	123	g/L
International normalized ratio	0.98	
Platelet count	181	x10E9/L
Pulmonary arterial systolic pressure	28	mmHg

Narratives (nur bei elektronischem Eingang)

BETA, 20.08.2009 DE-BETA-20200/09L

Sender diagnosis: not reported

Narrative

A 44-year-old female patient developed transient autoimmune hepatitis after concomitant use of Remifemin® (cimicifuga) and paracetamol. The patient presented with a 1-week-history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Four weeks earlier she started to take cimicifuga. When the above named symptoms had already developed, the patient took two doses of paracetamol for headache. Physical examination revealed bilateral lower leg oedema with a 1.5-cm difference in the magnitude of the shank circumference between left and right side. Laboratory tests showed coagulation time activation. Abdominal sonography showed a liver of normal size and structure. Duplex sonography of both legs revealed remarkable fluid accumulation in the subcutaneous tissue. Laboratory tests performed one day later showed elevated liver enzymes and demonstrated transient autoantibodies that indicated the classification of hepatotoxicity as transient autoimmune hepatitis. The acute-phase protein factor 8 was also increased. Further tests showed a weak reaction in test for antibodies against SSA/Ro-52. Cimicifuga was discontinued. The patient recovered completely within four weeks. Use of the Naranjo ADR probability scale indicates a probable relationship between the observed symptom and the treatment with cimicifuga. The authors state that a drug interaction between cimicifuga and paracetamol cannot be completely ruled out.

Krewel Meuselbach GmbH, 25.08.2009 DE-KREWEL-001717

Sender diagnosis: not reported

Narrative

A 44-year-old female patient developed transient autoimmune hepatitis after concomitant use of Remifemin® (cimicifuga) and paracetamol. The patient presented with a 1-week-history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Four weeks earlier she started to take cimicifuga. When the above named symptoms had already developed, the patient took two doses of paracetamol for headache. Physical examination revealed bilateral lower leg oedema with a 1.5-cm difference in the magnitude of the shank circumference between left and right side. Laboratory tests showed coagulation time activation. Abdominal sonography showed a liver of normal size and structure. Duplex sonography of both legs revealed remarkable fluid accumulation in the subcutaneous tissue. Laboratory tests performed one day later showed elevated liver enzymes and demonstrated transient autoantibodies that indicated the classification of hepatotoxicity as transient autoimmune hepatitis. The acute-phase protein factor 8 was also increased. Further tests showed a weak reaction in test for antibodies against SSA/Ro-52. Cimicifuga was discontinued. The patient recovered completely within four weeks. Use of the Naranjo ADR probability scale indicates a probable relationship between the observed symptom and the treatment with cimicifuga. The authors state that a drug interaction between cimicifuga and paracetamol cannot be completely ruled out.

NOTE: Other risk factor: co-suspect drug.<dbtext>To_Char_NARRATIVE_2</dbtext>

ratiopharm GmbH, 26.08.2009 DE-RATIOPH-2009R0807600

Sender diagnosis: not reported

Narrative

A 44-year-old female patient developed transient autoimmune hepatitis after concomitant use of Remifemin® (cicicifuga) and paracetamol. The patient presented with a 1-week-history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Four weeks earlier she started to take cicicifuga. When the above named symptoms had already developed, the patient took two doses of paracetamol for headache. Physical examination revealed bilateral lower leg oedema with a 1.5-cm difference in the magnitude of the shank circumference between left and right side. Laboratory tests showed coagulation time activation. Abdominal sonography showed a liver of normal size and structure. Duplex sonography of both legs revealed remarkable fluid accumulation in the subcutaneous tissue. Laboratory tests performed one day later showed elevated liver enzymes and demonstrated transient autoantibodies that indicated the classification of hepatotoxicity as transient autoimmune hepatitis. The acute-phase protein factor 8 was also increased. Further tests showed a weak reaction in test for antibodies against SSA/Ro-52. Cicicifuga was discontinued. The patient recovered completely within four weeks. Use of the Naranjo ADR probability scale indicates a probable relationship between the observed symptom and the treatment with cicicifuga. The authors state that a drug interaction between cicicifuga and paracetamol cannot be completely ruled out. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: Yes, Abuse / Misuse: No, Pregnancy / Lactation tracking: No accumulates in overdose. It is therefore not clear why the authors suspect a contribution of normal dosed paracetamol. Also an interaction between Paracetamol and Cicicifuga causing immunogenic reactions has not been reported before.

In conclusion an interaction between the two drugs is assessed as unlikely by ratiopharm as well as a contribution of Paracetamol to autoimmune hepatitis.

Actions with regard to the labelling are not necessary.

Reporter's comment:

MENARINIP, 28.08.2009 DE-MEN-14813

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, involved a 44-year-old female patient who developed transient autoimmune hepatitis after concomitant use of Remifemin® (cicicifuga) and paracetamol. The patient presented with a 1-week-history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Four weeks earlier she started to take cicicifuga. When the above named symptoms had already developed, the patient took two doses of paracetamol for headache. Physical examination revealed bilateral lower leg oedema with a 1.5-cm difference in the magnitude of the shank circumference between left and right side. Laboratory tests showed coagulation time activation. Abdominal sonography showed a liver of normal size and structure. Duplex sonography of both legs revealed remarkable fluid accumulation in the subcutaneous tissue. Laboratory tests performed one day later showed elevated liver enzymes and demonstrated transient autoantibodies that indicated the classification of hepatotoxicity as transient autoimmune hepatitis. The acute-phase protein factor 8 was also increased. Further tests showed a weak reaction in test for antibodies against SSA/Ro-52. Cicicifuga was discontinued. The patient recovered completely within four weeks. Use of the Naranjo ADR probability scale indicates a probable relationship between the observed symptom and the treatment with cicicifuga. The authors state that a drug interaction between cicicifuga and paracetamol cannot be completely ruled out.

Temmler Pharma GmbH & Co. KG, 14.09.2009 DE-TEMMLER06-003589

Sender diagnosis: not reported

Narrative

A 44-year-old female patient developed transient autoimmune hepatitis after concomitant use of Remifemin® (cicicifuga) and paracetamol. The patient presented with a 1-week-history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Four weeks earlier she started to take cicicifuga. When the above named symptoms had already developed, the patient took two doses of paracetamol for headache. Physical examination revealed bilateral lower leg oedema with a 1.5-cm difference in the magnitude of the shank circumference between left and right side. Laboratory tests showed coagulation time activation. Abdominal sonography showed a liver of normal size and structure. Duplex sonography of both legs revealed remarkable fluid accumulation in the subcutaneous tissue. Laboratory tests performed one day later showed elevated liver enzymes and demonstrated transient autoantibodies that indicated the classification of hepatotoxicity as transient autoimmune hepatitis. The acute-phase protein factor 8 was also increased. Further tests showed a weak reaction in test for antibodies against SSA/Ro-52. Cicicifuga was discontinued. The patient recovered completely within four weeks. Use of the Naranjo ADR probability scale indicates a probable relationship between the observed symptom and the treatment with cicicifuga.

Reporter's comment:

The authors state that a drug interaction between cimicifuga and paracetamol cannot be completely ruled out.

Bayer Vital GmbH, 24.03.2010 DE-BAYER-201019240GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 12-MAR-2010 and describes the occurrence of coagulation activation, fluid retention and transient autoimmune hepatopathy in a 44 year old female patient who was with Remifemin (Cimicifuga rhizome) for unknown indication. A drug interaction between black cohosh on the one hand and acetaminophen on the other could not be ruled out completely.

Case Report:

A 44-year-old woman presented with a 1-week history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Her past history was significant for myasthenia gravis with predominant involvement of ocular muscles which had been diagnosed 8 years before. A thorascopic thymectomy had been performed within the first year. Since then, the patient had taken pyridostigmine at dosages between 60 and 120 mg/day but no immunosuppressive drugs. Additionally, a previous diagnostic procedure had revealed a heterozygous factor V Leiden mutation. However, until today the patient has not experienced a thromboembolic event. Three years before presentation, the patient suffered from an acute anaphylactic reaction caused by a first-time local administration of a comfrey root extract ointment (Extr. Rad. Symphyti; Kytta-Balsam). Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock (Remifemin, Schaper and Brummer GmbH, Salzgitter, Germany) at a daily dose of 5 mg. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The patient reported to be a vegetarian and to have a minimal alcohol intake. Physical examination of the patient revealed bilateral lower leg edema with a 1.5-cm difference in the magnitude of the shank circumference between the left and the right sides. There was a mild right upper quadrant tenderness. Vital signs were blood pressure 139/74 mmHg, pulse rate 71 beats/min, respiratory rate 15 breaths/min, and temperature 37.78°C. Laboratory evaluation revealed a white blood cell count of $5.3 \times 10^9/l$, a hemoglobin level of 123 g/l, and a platelet count of $181 \times 10^9/l$. Creatine kinase was 83 U/l (reference value <145 U/l). Serum creatinine was 57.5 µmol/dl. Coagulation values were prothrombin time international normalized ratio (INR) 0.98, activated partial thromboplastin time 31.4 s (reference value 26-38 s), and D-dimers of 1304 mg/ml (reference value <500 mg/ml). Because of the difference between the two limbs in lower leg edema, the weight gain and the obvious coagulation activation, further diagnostic procedures were performed. An abdominal sonographic evaluation showed a liver of normal size and structure. There was one hemangioma with a diameter of 18 mm. The portal vein and the liver veins were normal. The other abdominal organs were normal, too. A duplex sonography of the veins of both legs revealed no signs of thrombosis. However, there was a remarkable fluid accumulation in the subcutaneous tissue. An echocardiographic examination showed normal atrial and ventricle chambers. The left ventricular ejection fraction was 60%. There was a hemodynamically irrelevant mild insufficiency of the mitral valve. The pulmonary arterial systolic pressure was 28 mmHg (reference value 536 mmHg). One day later, further laboratory investigations showed an alanine transaminase level of 104 U/l (reference value 534 U/l) and an aspartate transaminase level of 66 U/l (reference value 531 U/l). Total bilirubin and creatinine levels were normal. The C-reactive protein concentration was 4 mg/l (reference value <5 mg/l). Extended coagulation measurements gave normal activities for factors 1,2, 5, 7, 9, 11, 13 and antithrombin. Platelet function tests were normal. The acute-phase protein factor 8 was increased to 223% (reference value for blood group AB 65-165%). The serum concentrations of sodium and potassium were 138 mval/l and 3.5 mval/l, respectively. Serologic evaluations showed IgG antibodies to hepatitis A virus and hepatitis B virus surface antigen after vaccination. The other serologic tests for hepatitis viruses A, B and C, and cytomegalovirus were negative. Using polymerase chain reactions, there was no evidence of infection with hepatitis B virus, hepatitis C virus, hepatitis G virus, adenovirus, picorna virus and parvovirus B19. An antinuclear antibody screening was positive; the titer was 320 (reference value <100). Further tests revealed a weak reaction in a test for antibodies against SSA/Ro-52 (a 52 kDa protein associated with Sjogren's syndrome). The black cohosh was discontinued. Complete resolution of symptoms occurred within 1 week, and resolution of the abnormal liver enzymes and the increased D-dimers occurred within 2 weeks. At 4 weeks, the antinuclear and anti-SSA/Ro-52 antibodies had disappeared.

Author's Comments: In summary, despite the absence of a rechallenge trial, several lines of evidence support black cohosh to be probably the causative agent in this case of coagulation activation, fluid retention and transient autoimmune hepatopathy. Nevertheless, a drug interaction between black cohosh on the one hand and acetaminophen on the other cannot be ruled out completely.

Initial received on 12-MAR-2010:

Although it was not a company product, potentially important new safety information emerging from this report might have implications for company products. Therefore the case was entered in the company's database.

The authors describe the occurrence of coagulation activation, fluid retention and transient autoimmune hepatopathy in a 44 year old female patient who was with Remifemin (Cimicifuga rhizome) for unknown indication. Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock at a daily dose of 5 mg. Three weeks later the woman suffered from remarkable weight gain of 6.5 kg, headache and abdominal discomfort. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The authors diagnosed coagulation activation, fluid retention and transient autoimmune hepatopathy. A heterocygous factor V Leiden mutation was known in this patient. The

Authors mentioned that several lines of evidence support black cohosh to be probably the causative agent in this case. A drug interaction between black cohosh on the one hand and acetaminophen on the other could not be ruled out completely.

JENAPHARM DEUTSCHLAND, 25.03.2010 DE-BAYER-201019240GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 12-MAR-2010 and describes the occurrence of coagulation activation, fluid retention and transient autoimmune hepatopathy in a 44 year old female patient who was with Remifemin (Cimicifuga rhizome) for unknown indication. A drug interaction between black cohosh on the one hand and acetaminophen on the other could not be ruled out completely.

Case Report:

A 44-year-old woman presented with a 1-week history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Her past history was significant for myasthenia gravis with predominant involvement of ocular muscles which had been diagnosed 8 years before. A thorascopic thymectomy had been performed within the first year. Since then, the patient had taken pyridostigmine at dosages between 60 and 120 mg/day but no immunosuppressive drugs. Additionally, a previous diagnostic procedure had revealed a heterozygous factor V Leiden mutation. However, until today the patient has not experienced a thromboembolic event. Three years before presentation, the patient suffered from an acute anaphylactic reaction caused by a first-time local administration of a comfrey root extract ointment (Extr. Rad. Symphyti; Kytta-Balsam). Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock (Remifemin, Schaper and Brummer GmbH, Salzgitter, Germany) at a daily dose of 5 mg. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The patient reported to be a vegetarian and to have a minimal alcohol intake. Physical examination of the patient revealed bilateral lower leg edema with a 1.5-cm difference in the magnitude of the shank circumference between the left and the right sides. There was a mild right upper quadrant tenderness. Vital signs were blood pressure 139/74 mmHg, pulse rate 71 beats/min, respiratory rate 15 breaths/min, and temperature 37.78°C. Laboratory evaluation revealed a white blood cell count of $5.3 \times 10^9/l$, a hemoglobin level of 123 g/l, and a platelet count of $181 \times 10^9/l$. Creatine kinase was 83 U/l (reference value <145 U/l). Serum creatinine was 57.5 µmol/dl. Coagulation values were prothrombin time international normalized ratio (INR) 0.98, activated partial thromboplastin time 31.4 s (reference value 26-38 s), and D-dimers of 1304 mg/ml (reference value <500 mg/ml). Because of the difference between the two limbs in lower leg edema, the weight gain and the obvious coagulation activation, further diagnostic procedures were performed. An abdominal sonographic evaluation showed a liver of normal size and structure. There was one hemangioma with a diameter of 18 mm. The portal vein and the liver veins were normal. The other abdominal organs were normal, too. A duplex sonography of the veins of both legs revealed no signs of thrombosis. However, there was a remarkable fluid accumulation in the subcutaneous tissue. An echocardiographic examination showed normal atrial and ventricle chambers. The left ventricular ejection fraction was 60%. There was a hemodynamically irrelevant mild insufficiency of the mitral valve. The pulmonary arterial systolic pressure was 28 mmHg (reference value 536 mmHg). One day later, further laboratory investigations showed an alanine transaminase level of 104 U/l (reference value 534 U/l) and an aspartate transaminase level of 66 U/l (reference value 531 U/l). Total bilirubin and creatinine levels were normal. The C-reactive protein concentration was 4 mg/l (reference value <5 mg/l). Extended coagulation measurements gave normal activities for factors 1, 2, 5, 7, 9, 11, 13 and antithrombin. Platelet function tests were normal. The acute-phase protein factor 8 was increased to 223% (reference value for blood group AB 65-165%). The serum concentrations of sodium and potassium were 138 mval/l and 3.5 mval/l, respectively. Serologic evaluations showed IgG antibodies to hepatitis A virus and hepatitis B virus surface antigen after vaccination. The other serologic tests for hepatitis viruses A, B and C, and cytomegalovirus were negative. Using polymerase chain reactions, there was no evidence of infection with hepatitis B virus, hepatitis C virus, hepatitis G virus, adenovirus, picorna virus and parvovirus B19. An antinuclear antibody screening was positive; the titer was 320 (reference value <100). Further tests revealed a weak reaction in a test for antibodies against SSA/Ro-52 (a 52 kDa protein associated with Sjogren's syndrome). The black cohosh was discontinued. Complete resolution of symptoms occurred within 1 week, and resolution of the abnormal liver enzymes and the increased D-dimers occurred within 2 weeks. At 4 weeks, the antinuclear and anti-SSA/Ro-52 antibodies had disappeared.

Author's Comments: In summary, despite the absence of a rechallenge trial, several lines of evidence support black cohosh to be probably the causative agent in this case of coagulation activation, fluid retention and transient autoimmune hepatopathy. Nevertheless, a drug interaction between black cohosh on the one hand and acetaminophen on the other cannot be ruled out completely.

Initial received on 12-MAR-2010:

Although it was not a company product, potentially important new safety information emerging from this report might have implications for company products. Therefore the case was entered in the company's database.

The authors describe the occurrence of coagulation activation, fluid retention and transient autoimmune hepatopathy in a 44 year old female patient who was with Remifemin (Cimicifuga rhizome) for unknown indication. Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock at a daily dose of 5 mg. Three weeks later the woman suffered from remarkable weight gain of 6.5 kg, headache and abdominal discomfort. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The authors diagnosed coagulation activation, fluid retention and transient autoimmune hepatopathy. A heterozygous factor V Leiden mutation was known in this patient. The Authors mentioned that several lines of evidence support black cohosh to be probably the causative agent in this case. A drug interaction between black cohosh on the one hand and acetaminophen on the other could not be ruled out completely.

Boehringer Ingelheim GmbH, Germany, 25.05.2010 DE-Boehringer Ingelheim GmbH, Germany-2010-DE-02789GD

Sender diagnosis: not reported

Narrative

P10-05858/S10-2117

The case of a woman with coagulation activation, fluid retention and transient autoimmune hepatitis associated with the use of black cohosh is reported. The patient was also using paracetamol.

A 44-year-old woman presented with a 1-week history of remarkable weight gain of 6.5 kg, headache and abdominal discomfort. Her past history was significant for myasthenia gravis with predominant involvement of ocular muscles which had been diagnosed 8 years before. A thoracoscopic thymectomy had been performed within the first year. Since then, the patient had taken pyridostigmine at dosages between 60 and 120 mg/ day but no immunosuppressive drugs. Additionally, a previous diagnostic procedure had revealed a heterozygous factor V Leiden mutation. However, until today the patient has not experienced a thromboembolic event. Three years before presentation, the patient suffered from an acute anaphylactic reaction caused by a first-time local administration of a comfrey root extract ointment (Extr. Rad. Symphyti). Four weeks before presentation, a gynecologist prescribed a dry extract of Cimicifuga rootstock (Remifemin) at a daily dose of 5 mg. When symptoms had already developed, the patient took two doses of 1 g acetaminophen because of headache. The patient reported to be a vegetarian and to have a minimal alcohol intake.

Physical examination of the patient revealed bilateral lower leg edema with a 1.5-cm difference in the magnitude of the shank circumference between the left and the right sides. There was a mild right upper quadrant tenderness. Vital signs were blood pressure 139/74 mmHg, pulse rate 71 beats/min, respiratory rate 15 breaths/min, and temperature 37.7 degC.

Laboratory evaluation revealed a white blood cell count of $5.3 \times 10^9/\text{l}$, a hemoglobin level of 123 g/l, and a platelet count of $181 \times 10^9/\text{l}$. Creatine kinase was 83 U/l (reference value $<145 \text{ U/l}$). Serum creatinine was 57.5 mmol/dl. Coagulation values were prothrombin time international normalized ratio (INR) 0.98, activated partial thromboplastin time 31.4 s (reference value 26-38 s), and D-dimers of 1304 mg/ml (reference value $<500 \text{ mg/ml}$). Because of the difference between the two limbs in lower leg edema, the weight gain and the obvious coagulation activation, further diagnostic procedures were performed. An abdominal sonographic evaluation showed a liver of normal size and structure. There was one hemangioma with a diameter of 18 mm. The portal vein and the liver veins were normal. The other abdominal organs were normal, too. A duplex sonography of the veins of both legs revealed no signs of thrombosis. However, there was a remarkable fluid accumulation in the subcutaneous tissue. An echocardiographic examination showed normal atrial and ventricle chambers. The left ventricular ejection fraction was 60%. There was a hemodynamically irrelevant mild insufficiency of the mitral valve. The pulmonary arterial systolic pressure was 28 mmHg (reference value $<36 \text{ mmHg}$).

Oneday later, further laboratory investigations showed an alanine transaminase level of 104 U/l (reference value $<34 \text{ U/l}$) and an aspartate transaminase level of 66 U/l (reference value $<31 \text{ U/l}$). Total bilirubin and creatinine levels were normal. The C-reactive protein concentration was 4 mg/l (reference value $<5 \text{ mg/l}$). Extended coagulation measurements gave normal activities for factors 1, 2, 5, 7, 9, 11, 13, and antithrombin. Platelet function tests were normal. The acute-phase protein factor 8 was increased to 223% (reference value for blood group AB 65-165%). The serum concentrations of sodium and potassium were 138 mval/l and 3.5 mval/l, respectively. Serologic evaluations showed IgG antibodies to hepatitis A virus and hepatitis B virus surface antigen after vaccination. The other serologic tests for hepatitis viruses A, B and C, and cytomegalovirus were negative. Using polymerase chain reactions, there was no evidence of infection with hepatitis B virus, hepatitis C virus, hepatitisGvirus, adenovirus, picorna virus and parvovirus B19. An antinuclear antibody screening was positive; the titer was 320 (reference value <100). Further tests revealed a weak reaction in a test for antibodies against SSA/Ro-52 (a 52 kDa protein associated with Sjogren's syndrome).

The black cohosh was discontinued. Complete resolution of symptoms occurred within 1 week, and resolution of the abnormal liver enzymes and the increased D-dimers occurred within 2 weeks. At 4 weeks, the antinuclear and anti-SSA/Ro-52 antibodies had disappeared.

The authors state: 'The adverse event appeared 3 weeks after black cohosh was administered and disappeared when the drug was stopped. Alternative causes were excluded as far as possible by objective evidence. There were no signs of viral infection. There were no concomitant drugs or harmful agents that could explain the adverse events. ... High doses of acetaminophen are hepatotoxic in themselves. There were no other signs of acetaminophen toxicity in our case in the form of nephrotoxicity or bleeding. ... Our patient, however, had taken only two single doses of 1 g acetaminophen per day on two different days. Such low doses of acetaminophen usually do not cause liver damage. Nevertheless, a drug interaction between black cohosh on the one hand and acetaminophen on the other cannot be ruled out completely. Also the transient occurrence of anti-nuclear antibodies and the transient weak reaction in a test for antibodies against SSA/Ro-52 do not allow the ruling out of a contribution of acetaminophen in this case, since a weak formation of other autoantibodies in cases of acetaminophen-induced liver damage has been seen occasionally.'

Due to the medical circumstances the case was rated serious: seriousness criterion 'other' (medically significant).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09109630

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19661215	Age: 42 Year	weight (kg): 00068	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug abuse					Unknown
2 Drug overdose					Unknown
3 Vomiting					Unknown
4 Raised liver function tests					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL TABS 500 MG		00007000 Mg milligram(s) -		
paracetamol		245 -		
2 perfalgan		00001000 Mg milligram(s) -		
paracetamol		-		

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 20.08.2009 DE-Bristol-Myers Squibb Company-14742712

Sender diagnosis: not reported

Narrative

This report was received from Poison Control Center Mainz (PCCM, local file no.AZ2009017964) and BMS Germany (local file no.2009081501).

A hospital physician reported that a 42-year-old female patient had received acetaminophen (Perfalgan) 1000mg and had administered 14 tablets of acetaminophen (paracetamol) 500mg (in total 7000mg) within 24 hours. The patient suffered from vomiting and increased liver values. Drug abuse with acetaminophen (paracetamol) for a longer time was suspected.

BMS medical evaluation comment:

This patient was suspected to have drug abuse with acetaminophen for a long time. The information available in this report is limited and does not allow a medically meaningful assessment of the case.

#1; unexpected; PSB

#2; unexpected; PSB

#3; non-serious

#4; non-serious

* according to current Summary of Product Characteristics

** DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no

Comments: This patient was suspected to have drug abuse with acetaminophen for a long time. The information available in this report is limited and does not allow a medically meaningful assessment of the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09119465

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg): 00054	BMI_int: 18.25	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 perfalgan paracetamol	Intravenous (not otherwis	00000003 G gram(s)/001	-	Pain
2 perfalgan paracetamol	Intravenous (not otherwis	00000003 G gram(s)/001	-	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rectal cancer			Unknown	
Fever			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 10.09.2009 DE-Bristol-Myers Squibb Company-14723282

Sender diagnosis: not reported

Narrative

This case was received via a BMS sales representative and BMS Germany (local file number 2009072904).

A physician reported that a female patient (not a patient of the reporting physician) experienced elevated liver values while under therapy with acetaminophen.

Supplemental information received on 08-Sep-2009 from the physician via BMS Germany reported that the patient was hospitalized due to life threatening acute liver failure while under therapy with intravenous acetaminophen (3 g/day) for pain and fever.

Due to acute liver failure the patient was treated in the intensive care unit of the hospital. At the time of this report the patient had recovered. The physician reported that the following liver values were elevated: glutamate pyruvate transaminase (GPT), glutamate oxaloacetate transaminase (GOT), gamma glutamyl transferase (GGT).

Furthermore, the physician reported that the patient had received acetaminophen therapy before and had tolerated it then. De-challenge with acetaminophen was positive. The physician suspected a causal relationship between the event of acute liver failure and acetaminophen therapy.

The patient's medical history included cancer of the rectum.

Comment: Elevated liver transaminases are noted in the current event but ALT, AST, GGT, however, acute hepatic failure is not mentioned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09130880

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19711031	Age: 37 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestatic hepatitis	200906			13 Year	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 avalox moxifloxacin hydrochloride		/	200906-	Respiratory infection
2 cephalosporin c cephalosporin c		/	200906-	Respiratory infection
3 MARCUMAR phenprocoumon	Oral	/	1996-20091012	Aortic valve replacement
4 PARACETAMOL paracetamol		/	200906-	Respiratory infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diverticulitis			Unknown	
Diverticulitis	200905		No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hepatic enzyme		Increased	n.a.	
Alanine aminotransferase	2009	maximum value	n.a.	
Alanine aminotransferase	20091012	85	n.a.	
Gamma-glutamyltransferase	2009	maximum value	n.a.	
Gamma-glutamyltransferase	20091012	118	n.a.	

Narratives (nur bei elektronischem Eingang)

MEDA, 30.09.2009 DE-MEDA-M-EU-2009090114

Sender diagnosis: not reported

Narrative

A report was received from a physician on 24-Sep-2009.

A physician reported that a patient who had been treated with Marcumar (phenprocoumon) for years, developed cholestatic hepatitis on an unspecified date. The suspected drug paracetamol and a further (not specified) drug were discontinued because of the event. At the time of the report the increased liver values had not normalised.

MEDA comment:

Serious case report from physician

Cholestatic hepatitis is considered listed for Marcumar (phenprocoumon). However, the patient had been treated with Marcumar for years before the event occurred. Concomitant medication consisted of paracetamol that may increase liver enzymes in therapeutic dosages and may lead to severe hepatic damage when overdosed. The patient had also taken at least one further concomitant drug that was not specified. No information on therapy dates and dosages of concomitant medication or patient's medical history was provided. Liver values were not reported either. With the available

information the cause of the event cannot be clarified. The causality to Marcumar cannot be assessed.

Reporter's comment:

MEDA, 20.10.2009 DE-MEDA-M-EU-2009090114

Sender diagnosis: not reported

Narrative

A report was received from a physician on 24-Sep-2009.

A physician reported that a patient who had been treated with Marcumar (phenprocoumon) for years, developed cholestatic hepatitis on an unspecified date. The suspected drug paracetamol and a further (not specified) drug were discontinued because of the event. At the time of the report the increased liver values had not normalised.

Additional information was received from the physician on 13-Oct-2009.

Information concerned the patient's age, medical history, medication and laboratory values were provided and updated.

The patient was a 37-year-old male with a BMI of about 29. He was treated with Marcumar (phenprocoumon) for aortic valve replacement since 1996. The dosage of phenprocoumon was adjusted according to his INR value. In June 2009 the patient was treated with co-suspect drugs cephalosporin, Avalox (moxifloxacin hydrochloride) and paracetamol for a respiratory infection.

Thereafter he felt malaise and his transaminases were increased. Cholestatic hepatitis developed in June 2009. Subsequently all drugs, apart for Marcumar, were discontinued on unknown date.

On 12-Oct-2009, the date of the follow-up report, Marcumar was discontinued and the event cholestatic hepatitis was resolving.

The patient's medical history and drugs included sigma diverticulitis, surgery for sigma diverticulitis followed by the treatment with piperacillin and Combactam (sulbactam sodium) in May-2009.

Laboratory values:

Alanine aminotransferase (GPT) with maximum value of 321. On 12-Oct-2009 the value was 85.

Gamma-glutamyltransferase (GGT) with maximum value of 929. On 12-Oct-2009 the value was 118 .

The physician assessed the causality between Marcumar and the event cholestatic hepatitis as possible.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130726

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	200909				Unknown
2 Drug toxicity due to intentional overdose	200909				Unknown
3 Intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004750

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was >= 1.5 due to acute severe liver injury with and without encephalopathy >= grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly (p < 0.001) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130730

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	200909				Unknown
2 Drug toxicity due to intentional overdose	200909				Unknown
3 Intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004743

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130731

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	200909				Unknown
2 Drug toxicity	200909				Unknown
3 Accidental overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004775

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130732

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004749

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130735

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Accidental overdose	200909				Unknown
2 Drug toxicity	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004779

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776 and 2009BL004777.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130737

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004745

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130739

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004758

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130740

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity	200909				Unknown
2 Accidental overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004773

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130741

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity due to intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004751

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130743

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity due to intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004752

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130746

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity	200909				Unknown
2 Accidental overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004776

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130748

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004748

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130749

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Accidental overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004777

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130751

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity due to intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

[Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004742](#)

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130752

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004760

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130753

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004739

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130755

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Accidental overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004774

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to accidental acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for an unknown indication. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130756

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity due to intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004755

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130758

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Drug toxicity due to intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004753

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130760

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	200909				Unknown
2 Acute liver failure	200909				Unknown
3 Drug toxicity due to intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004746

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

134 adult patients (41 +/- 16 years of age, median: 38 years; 63% females/37% males) were initially enrolled in this study at participating hospitals and were referred to a liver transplant centre between January 2002 and April 2008. Patients presented within four weeks of disease onset without apparent preexisting liver disease at admission according to the criteria established by the United States Acute Liver Study Group. Their international normalized ratio (INR) was ≥ 1.5 due to acute severe liver injury with and without encephalopathy $\geq$ grade 1. Diagnosis of acetaminophen intoxication was based on a toxic serum level of acetaminophen and/or its metabolic products of 10 - 30 ug/mL, and/or on a history of uptake of this drug in excess of the therapeutic dosage. Upon admission and first examination, the presence of ascites was excluded ultrasonographically. Thereupon, the patients' body weights were determined, and the BMI was calculated according to BMI. Four weeks after enrolment, patients were identified as survivors (i) who had recovered spontaneously; (ii) who had deceased; or (iii) who had received a liver transplant. The etiology of ALF was eventually identified according to standard diagnostic criteria; thorough review of the patients' own records; the anamneses (including clinical data recorded prior to ALF); medication histories; and the diagnostic findings (including ultrasound imaging as well as laboratory data recorded prior to ALF). Among all drug toxicity cases, acetaminophen overdosing was identified as the most predominant etiology of ALF. Sixteen patients (72.7% of these cases or 11.9% of all patients, respectively) ingested a median amount of 22 g acetaminophen with suicidal intent. Six patients (i. e., 27.3% of this group) accidentally intoxicated themselves by ingesting 3 - 5 g of acetaminophen on a given day. Importantly, patients with acetaminophen-induced liver failure were significantly ($p < 0.001$) younger when compared to patients suffering from ALF due to other etiologies (30 years vs. 43 years). Patients with acetaminophen-induced ALF revealed a shorter duration of disease, higher serum AST than ALT values, as well as lower bilirubin levels when compared to those with other etiologies. During the four-week enrolment period, 89 (66.4%) of the patients recovered spontaneously under conservative therapy, while 45 patients (33.6) deceased or underwent transplantation. Under conservative therapy, patients intoxicated with acetaminophen or with HAV-related ALF revealed a four-week survival of 82%. Among the 30 patients (22.4%) listed for transplantation, four (3% of all patients) deceased either while awaiting the graft or due to their severely deteriorated condition, which excluded them from receiving a transplant. Of the 26 transplanted patients (19.4%), 20 (i. e., 14.9% of all patients or 77% of the transplant recipients, respectively) survived, while six (4.5% of all patients) died postoperatively.

Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130762

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	200909				Unknown
2 Drug toxicity due to intentional overdose	200909				Unknown
3 Intentional overdose	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004757

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004747, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-09130763

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug toxicity due to intentional overdose	200909				Unknown
2 Intentional overdose	200909				Unknown
3 Acute liver failure	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	-	Suicidal ideation
paracetamol			-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:	CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA IN GERMANY: A RETROSPECTIVE STUDY (2002-2008). Z GASTROENTEROL 2009 SEP; 47 (9): 807-13. EPUB 2009 SEP 11		

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 06.10.2009 DE-BAUSCH-2009BL004747

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that an adult patient experienced drug toxicity due to intentional acetaminophen overdose (DRUG TOXICITY) and acute liver failure (ALF) (ACUTE HEPATIC FAILURE) following PARACETAMOL (manufacturer unknown) ingestion for suicidal intention. No further specific information regarding this single case is available.

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Q. v. related cases 2009BL004739, 2009BL004742, 2009BL004743, 2009BL004745, 2009BL004746, 2009BL004748, 2009BL004749, 2009BL004750, 2009BL004751, 2009BL004752, 2009BL004753, 2009BL004755, 2009BL004757, 2009BL004758, 2009BL004760, 2009BL004773, 2009BL004774, 2009BL004775, 2009BL004776, 2009BL004777 and 2009BL004779.

Source:
CANBAY A, JOCHUM C, BECHMANN LP, FESTAG S, GIESELER RK, YUEKSEL Z ET AL. ACUTE LIVER FAILURE IN A METROPOLITAN AREA

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09139514

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19560302	Age: 53 Year	weight (kg): 79	BMI_int: 28.33	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090816				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ASS plus C-ratiopharm, Brtbl. acetylsalicylic acid; ascorbic acid	Oral	2*1 Df dosage form/Month 59	2007-20090820 -	Head pain
2 Paracetamol, 500 mg, Tabl, Ratiopharm paracetamol	Oral	3*500 Mg milligram(s)/Moi 245	1999-200906 -	Migraine
3 Sumatriptan Stada, 50 mg, Filmtabl. sumatriptan	Oral	3*50 Mg milligram(s)/Moni 82	2008-20090818 -	Migraine

Drugs (concomitant)

4 ALDI Magnesiumtabletten stoffangabe (teilweise) unklar	Oral	1 Df dosage form/Day 245	-20090826 -	
5 Ibufexal, 400 bzw. 600 mg, Filmtabl ibuprofen	Oral	3*1 Df dosage form/Month 82	200807-20090906 -	Head pain
6 Metohexal-Succ, 47,5 mg, Retardtabl, Hexal metoprolol	Oral	23750 µg microgram(s)/D 212	200611-20090826 -	Hypertension arterial
7 Zink, Tabl., NN zinc sulfate	Oral	1 Df dosage form/Day 245	200708-20090826 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Migraine	1976		k.A.	
Hypertension arterial	200611		k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20090826	2988	U/l	
Alanine aminotransferase increased	20090831	2666	U/l	
Alanine aminotransferase increased	20090912	940	U/l	
Alanine aminotransferase increased	20091008	209	U/l	
Aspartate aminotransferase increased	20090826	1773	U/l	
Aspartate aminotransferase increased	20090830	2057	U/l	
Aspartate aminotransferase increased	20090831	1986	U/l	
Aspartate aminotransferase increased	20090912	717	U/l	
Aspartate aminotransferase increased	20091008	164	U/l	
Bilirubin increased	20090826	7.71	mg/dl	
Bilirubin increased	20090831	17.26	mg/dl	

Bilirubin increased	20090912	31	mg/dl
Bilirubin increased	20091008	7	mg/dl
Alkaline phosphatase increased	20090826	343	U/l
Alkaline phosphatase increased	20090912	248	U/l
Alanine aminotransferase increased	20091015	190	U/l
Aspartate aminotransferase increased	20091015	156	U/l
Bilirubin increased	20090908	41	mg/d
Bilirubin increased	20091015	5	mg/dl
Bilirubin increased	20091029	2.2	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 21.10.2009 DE-FAKOS-170290

Sender diagnosis: not reported

Narrative

Sender's comment:

initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 24.11.2009 DE-FAKOS-170290

Sender diagnosis: not reported

Narrative

Wenige Tage nach einer Urlaubsreise in Mallorca (05.08. - 13.08.2009) kam es zunehmend zum unklaren Krankheitsgefühl mit Unwohlsein, Schwäche und subfebrilen Temperaturen. Zur Linderung der Beschwerden nahm die Patientin am 17.08.2009 1 Tbl. Ibuprofen (Ibuhexal) 600 mg, am 18.08.2009 Sumatriptan (Stada) 50 mg sowie am 19.08. und 20.08.2009 1 Tbl. ASS plus C 500 mg (Ratiopharm) ein. Bei dunklem, rot-braunem Urin und Verdacht auf Harnwegsinfekt erfolgte eine Antibiose mit Cotrim forte (Heumann) 2x1 Tbl. über 3 Tage (24.-26.08.2009). Der Urin blieb weiterhin dunkel. Am 26.08.2009 veranlasste die Hausärztin die stationäre Einweisung wegen eines neu aufgetretenen Sklerenikterus. Die Laborwerte bei Aufnahme betrugen: ALAT (Alaninaminotransferase) 2988 U/l (Normbereich: <31 U/l), ASAT 1773 U/l (Normbereich: <31 U/l, AP (Alkalische Phosphatase) 343 U/l (Normbereich: 35-200 U/l), Gesamtbilirubin 7,71 mg/dl (Normbereich: <1,2 mg/dl). Im stationären Verlauf kam es innerhalb von 3 Tagen zu einem beginnenden Abfall der ALAT und AP bei Zunahme des Ikterus und massivem Anstieg des Bilirubins mit einem Maximum am 08.09.2009 (Gesamtbilirubin 41 mg/dl). Danach fielen die Leberenzymwerte stetig ab, auch das Bilirubin. Die letzten Werte vor Entlassung waren noch deutlich erhöht (Laborwerte vom 12.09.2009: ALAT 940 U/l, ASAT 717 U/l, AP 248 U/l, Bilirubin 31 mg/dl). Am 17.09.2009 erfolgte die Entlassung aus dem Krankenhaus. Auch danach zeigte sich weiterhin ein stetiger Abfall der Transaminasen und des Bilirubins (15.10.2009: ALAT 190 U/l, ASAT 156 U/l, Bilirubin 5,0 mg/dl). Im Rahmen der Hepatitis kam es zur Lebersynthesestörung mit Verminderung des Quick und Verlängerung der PTT (Partielle Thromboplastinzeit).

Trotz einer umfangreichen Diagnostik blieb die Ursache der Hepatitis unklar. Es bestehen keine Grunderkrankungen, die eine Leberbeteiligung verursachen könnten. Sonographisch waren Leber und Gallengänge unauffällig, dopplersonographisch fand sich kein Hinweis auf eine Durchblutungsstörung der Leber. In der Leberbiopsie am 03.09.2009 wurde ein ausgeprägter Parenchymschaden und eine mäßige periportal betonte Entzündung mit lymphoplasmazellulärem Infiltrat und zahlreichen Eosinophilen (periportale Hepatitis, zonale Nekrose) festgestellt. Diese Veränderungen wurden am ehesten auf eine akut-toxische Schädigung zurückgeführt.

Die UAW hat sich ambulant entwickelt.

Die Arzneimittelanamnese (Auswahl suspekter Arzneimittel) zeigt eine chronische Einnahme von Paracetamol, Sumatriptan, ASS (Acetylsalicylsäure) und Ibuprofen gegen Migräne.

Unmittelbar nach stationärer Aufnahme wurden Paracetamol, Sumatriptan und ASS nicht mehr eingenommen, Ibuprofen wurde bei Bedarf weiter verabreicht.

Sowohl die Krankheits- und Medikationsanamnese, die Leberhistologie, sowie die Besserung der Transaminasen nach Absetzen der ambulanten Arzneimittel (außer Ibuprofen), unterstützen sehr die Vermutung einer medikamentös-toxischen Ursache, auch wenn differenzialdiagnostisch eine Hepatitis im Rahmen einer Infektion (Urlaubsreise) nicht sicher auszuschließen ist.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Paracetamol-ratiopharm Tbl., Einnahmezeitraum: mindestens seit 1999- etwa Juni oder Juli 2009.

Sumatriptan Stada Filmtbl., Einnahmezeitraum: 2008- 18.08.2009,

ASS plus C-ratiopharm Brtbl., Einnahmezeitraum: 2007- 20.08.2009, zuletzt 1 Tbl. am 19. und 20.08.2009.

Aufgrund der bekannten starken Hepatotoxizität, langjähriger Einnahme und teilweise widersprüchlicher Angaben sowohl über die Einnahme überhaupt, als auch eingenommenen Mengen, kann Paracetamol als die wahrscheinlichste Ursache der Hepatitis vermutet werden. Da der Zeitpunkt der letzten Einnahme nicht genau bekannt ist (nach Angaben der Patientin Juni oder Juli 2009), wird Paracetamol als möglich bewertet.

Sumatriptan Stada und ASS plus C-ratiopharm werden ebenfalls mit möglich bewertet. Acetylsalicylsäure in hohen Dosen kann auch eine akute Hepatitis mit dem o.g. histologi<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09174736

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age: 65 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests raised					Unknown
2 Migraine					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol, Koffein	Oral	/	2009-2009	Migraine
paracetamol		245	183 Day	

Drugs (concomitant)

2 OMEPRAZOL "ACYFABRIK"	/	-
omeprazole		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthrosis			k.A.	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.12.2009 DE-NOVNCHHQ-CHPA2009DE23187

Sender diagnosis: not reported

Narrative

Report from consumer with scanty information received on 08 DEC 2009. This patient used Optalidon Schmerztabletten mit Paracetamol und Coffein to treat migraine because Optalidon N Zäpfchen was not available any more and was diagnosed with 'elevated liver test values' by her physician in 2009. Optalidon Schmerztabletten mit Paracetamol und Coffein was taken for approx 6 months in 2009. Optalidon Schmerztabletten mit Paracetamol und Coffein and also the concomittant treatment with Omeprazol was stopped 'because a hint for the liver was contained in the patient leaflets of both compounds'. The patient reported that she was currently under treatment with Aspirin effervescent tablets. Outcome: not reported. Follow-up information requested in written form in week 50, 2009.

by a health professional. The event, liver function test abnormal, was assessed as unlabeled according to the product information. The available information was considered inadequate to fully assess the case. The other events were considered non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09171903

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19580925	Age: 00051 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20091214	deathcause: Acute liver failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bucaine bupivacaine hydrochloride		00000002 ml millilitre(s)/0l	-	Postoperative care
2 perfalgan IV paracetamol		00000001 G gram(s)/ 124	-	Postoperative care
3 PROPOFOL propofol		00000200 Mg milligram(s)	-	Anaesthesia
4 SEVORANE sevoflurane		/	-	Anaesthesia
5 SULFENTANYL fentanyl		00000020 µg microgram(s)	-	Anaesthesia

Drugs (concomitant)

6 L-THYROXINE levothyroxine sodium	/	-	Hypothyroidism
7 remifemin cimicifuga racemosa root	/	-	Menopausal symptoms
8 tranxilium clorazepate dipotassium	00000020 Mg milligram(s)	-	Premedication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypothyroidism			Unknown	
Menopausal symptoms			Unknown	
Nicotine abuse			Unknown	6 Cigarettes/D
Alcohol abuse			Unknown	Not regularly
Arthroscopy			Unknown	Ambulatory shoulder arthroscopy
Impingement syndrome			Unknown	
Impingement syndrome			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase		66	U/L	

Narratives (nur bei elektronischem Eingang)Bristol-Myers Squibb Company, 22.12.2009 DE-Bristol-Myers Squibb Company-14899355

Sender diagnosis: not reported

Narrative

This report was received from Trec (cross reference number 1-24236553) via BMS-Germany (local file number 2009121705).

A physician reported that a 51-year-old female patient was hospitalized for liver failure and subsequently died after therapy with intravenous acetaminophen, bucaïne, propofol, sulfentanyl and sevoflurane.

She was administered 2 X 1g acetaminophen and 2ml/hour bucaïne 0.5% over 24 hours, post operatively, after an ambulatory shoulder arthroscopy. Furthermore, the patient had received 200mg propofol, 20 microgram sulfentanyl, oxygen-air-mixture and sevoflurane as anaesthetics. As presurgery medication, the patient had received 20mg clorazepate dipotassium. Before the ambulatory operation, gamma-GT value had been 66 U/L. Four days after the operation the patient experienced upper abdominal disorders and consulted a general practitioner. Subsequently she was admitted to hospital with acute liver failure. The patient died on 14-Dec-2009.

Patient medical history: The patient's medical history included impingement syndrome and ambulatory shoulder arthroscopy 14 days prior to this report.

BMS Medical evaluation comment:

Although administration of multiple anesthetics could have contributed to fatal liver failure in this patient, based on available information, a participatory role of acetaminophen could not be ruled out.

participatory role of acetaminophen could not be ruled out.

Reporter's comment:Bristol-Myers Squibb Company, 11.01.2010 DE-Bristol-Myers Squibb Company-14899355

Sender diagnosis: not reported

Narrative

This report was received from Trec (cross reference number 1-24236553) via BMS-Germany (local file number 2009121705).

A physician reported that a 51-year-old female patient was hospitalized for liver failure and subsequently died after therapy with intravenous acetaminophen, bucaïne, propofol, sulfentanyl and sevoflurane.

She was administered 2 X 1g acetaminophen and 2ml/hour bucaïne 0.5% over 24 hours, post operatively, after an ambulatory shoulder arthroscopy. Furthermore, the patient had received 200mg propofol, 20 microgram sulfentanyl, oxygen-air-mixture and sevoflurane as anaesthetics. As presurgery medication, the patient had received 20mg clorazepate dipotassium. Before the ambulatory operation, gamma-GT value had been 66 U/L. Four days after the operation the patient experienced upper abdominal disorders and consulted a general practitioner. Subsequently she was admitted to hospital with acute liver failure. The patient died on 14-Dec-2009.

Patient medical history: The patient's medical history included impingement syndrome and ambulatory shoulder arthroscopy 14 days prior to this report.

BMS Medical evaluation comment:

Although administration of multiple anesthetics could have contributed to fatal liver failure in this patient, based on available information, a participatory role of acetaminophen could not be ruled out.

Supplemental information received from the physician via BMS Germany on 07-Jan-2010 stated that the patient was given anaesthesia on 25-Nov-2009 (day of surgery). It was unknown to the reporter whether the patient had received any acetaminophen before or after the surgery. At the time of report, the physician did not have the results of the autopsy.

BMS medical evaluation comment remains unchanged based on follow-up information.

Sender diagnosis: not reported

Narrative

This report was received from Trec (cross reference number 1-24236553) via BMS-Germany (local file number 2009121705).

A physician reported that a 51-year-old female patient was hospitalized for liver failure and subsequently died after therapy with intravenous acetaminophen, bucaïne, propofol, sulfentanyl and sevoflurane.

She was administered 2 X 1g acetaminophen and 2ml/hour bucaïne 0.5% over 24 hours, post operatively, after an ambulatory shoulder arthroscopy. Furthermore, the patient had received 200mg propofol, 20 microgram sulfentanyl, oxygen-air-mixture and sevoflurane as anaesthetics. As presurgery medication, the patient had received 20mg clorazepate dipotassium. Before the ambulatory operation, gamma-GT value had been 66 U/L. Four days after the operation the patient experienced upper abdominal disorders and consulted a general practitioner. Subsequently she was admitted to hospital with acute liver failure. The patient died on 14-Dec-2009.

Patient medical history: The patient's medical history included impingement syndrome and ambulatory shoulder arthroscopy 14 days prior to this report.

BMS Medical evaluation comment:

Although administration of multiple anesthetics could have contributed to fatal liver failure in this patient, based on available information, a participatory role of acetaminophen could not be ruled out.

Supplemental information received from the physician via BMS Germany on 07-Jan-2010 stated that the patient was given anaesthesia on 25-Nov-2009 (day of surgery). It was unknown to the reporter whether the patient had received any acetaminophen before or after the surgery. At the time of report, the physician did not have the results of the autopsy.

BMS medical evaluation comment remains unchanged based on follow-up information.

Supplemental information received via BMS-Germany on 11-Jan-2010.

The physician reported that the patient had received acetaminophen 2.0 g on 25-Nov-2009 from 12:00 hrs till 17:00 hrs for postoperative pain therapy.

The patient had received levothyroxine as long-term medication for the treatment of hypothyroidism and cimicifuga rhizome for the treatment of menopausal complaints. It was unknown to the physician whether the patient had taken any other formulations of acetaminophen. It was also unknown to the physician if the patient had received acetaminophen for a longer period of time. The physician stated that the patient had not received an overdose of acetaminophen.

The physician did not know the patient's cause of death because the autopsy results were not available. Before the surgery the patient had been in a good general condition. Laboratory test revealed the gamma-GT level had been 69 U/l (normal value was less than 39 U/l). The other laboratory values had been without pathological findings.

The patient's medical history included nicotine abuse (6 cigarettes per day), hypothyroidism and menopausal complaints. The patient had not consumed alcohol regularly.

According to the physician the causality between the liver failure with fatal outcome and acetaminophen-therapy was not assessable.

No change in medical evaluation comment as per the follow up information received.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09174811

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19801029	Age: 28 Year	weight (kg): 77	BMI_int: 21.56	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea	20090922			30 Day	recovered/resolved
2 Gastritis	20090922			3 Day	Unknown
3 General physical health deterioration	20090922			30 Day	recovered/resolved
4 Alanine aminotransferase increased	20090922			30 Day	recovered/resolved
5 Aspartate aminotransferase increased	20090922			30 Day	recovered/resolved
6 Transaminases increased	20090922			30 Day	recovered/resolved
7 GGT increased	20090922			30 Day	recovered/resolved
8 Elevated liver enzymes	20090922			3 Day	Unknown
9 Allergic reaction	20090919			26 Day	Unknown
10 Abdominal pain	20090922			30 Day	recovered/resolved
11 Hepatitis acute	20090922			30 Day	recovered/resolved
12 Vomiting	20090922			30 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Grippostad C, Hartkps, STADA	Oral	6 Df dosage form/Day	20090902-20090905	Flu symptoms
caffeine; chlorphenamine maleate; paracetamol		6	4 Day	
2 IBUPROFEN	Oral	600 Mg milligram(s)/Day	20090824-20090831	Flu symptoms
ibuprofen		245	8 Day	
3 Ibuprofen, 200 mg, Tabl, NN in Spanien	Oral	600 Mg milligram(s)/Day	20090824-20090831	
ibuprofen			8 Day	
4 PANTOZOL		/	-	Gastritis
pantoprazole			-	
5 Paracetamol, Tabl, NN in Spanien	Oral	/	20090824-20090831	Flu symptoms
paracetamol			8 Day	
6 Reactine, 10 mg, Filmtabl, Mc Neil	Oral	30 Mg milligram(s)/Day	20090919-20090921	Allergic to cats
cetirizine		82	3 Day	

Drugs (concomitant)

7 SYMBICORT TURBUHALER "DRACO"	Respiratory (inhalation)	2 Df dosage form/Day	20090701-	Allergic asthma
budesonide; formoterol		110	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic asthma			Unknown	
Allergy			Unknown	animal hair
Gastritis erosive			k.A.	
Pollinosis			k.A.	
House dust allergy			k.A.	

Primary source

Qualification:	Physician	observestudytype:
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Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Alanine aminotransferase increased	20090922	305	U/l
Alanine aminotransferase increased	20090923	264	U/l
Alanine aminotransferase increased	20091001	241	U/l
Alanine aminotransferase increased	20091029	337	U/l
Alanine aminotransferase increased	20091118	59	U/l
Aspartate aminotransferase increased	20090922	99	U/l
Aspartate aminotransferase increased	20090923	83	U/l
Aspartate aminotransferase increased	20091001	60	U/l
Aspartate aminotransferase increased	20091029	463	U/l
Aspartate aminotransferase normal	20091118	41	U/l
Bilirubin normal	20090922	0.53	mg/dl
Bilirubin normal	20090923	0.62	mg/dl
Alkaline phosphatase normal	20090922	116	U/l
Alkaline phosphatase normal	20090923	102	U/l
Alkaline phosphatase normal	20091001	77	U/l
Alkaline phosphatase normal	20091118	84	U/l
Alanine aminotransferase	20091001	241	U/L
Aspartate aminotransferase	20091118	41	U/L
GGT	20090922	63	U/L
Laboratory test	20090922	116	U/L
Laboratory test	20091118	84	U/L

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 30.12.2009 DE-FAKOS-170273

Sender diagnosis: not reported

Narrative**Sender's comment:**

initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 09.02.2010 DE-FAKOS-170273

Sender diagnosis: not reported

Narrative

Während eines Aufenthaltes in Spanien (bis Ende August 2009) waren am 20.08.2009 grippale Symptome mit subfebrilen Temperaturen und Gliederschmerzen, sowie eine allgemeine Schwäche aufgetreten. Es erfolgte eine Selbstmedikation mit Paracetamol, Ibuprofen sowie Grippostad C. Die grippalen Erscheinungen waren Anfang September 2009 wieder abgeklungen. Dann traten am 19.09.2009 allergische Symptome in den oberen Luftwegen auf, weswegen der Patient Reactine für 3 Tage einnahm. Am 22.09.2009 wurde er stationär aufgenommen wegen Verschlechterung des Allgemeinzustandes mit Übelkeit, Erbrechen und Bauchschmerzen. Laborchemisch konnten deutlich erhöhte Transaminasen festgestellt werden (ALAT 305 U/l (Normbereich: <41 U/l), ASAT 99 U/l (Normbereich: <37 U/l). Weitere Laborwerte vom 22.09.2009: GGT (Gamma-Glutamyl-Transferease) 63 U/l (Normbereich: 8-61 U/l), AP (Alkalische Phosphatase) 116 U/l (Normbereich: 40-260 U/l), Bilirubin gesamt 0,53 mg/dl (Normbereich: <1,2 mg/dl). Am 23.09.2009 wurde der Patient bei Wohlbefinden aus dem Krankenhaus entlassen. Im weiteren Verlauf fielen die

Transaminasen zunächst wieder ab (Laborwerte vom 01.10.2009: ALAT 241 U/l, ASAT 60 U/l). Unter einer ambulanten Medikation mit Pantozol zur Behandlung einer gastroscopisch nachgewiesenen erosiven Gastritis kam es zu einem erneuten Anstieg der Aminotransferasen (Laborwerte vom 29.10.2009: ALAT 337 U/l, ASAT 463 U/l, außerdem GGT 44 U/l. Nach dem Absetzen von Pantozol besserten sich die Werte wieder (18.11.2009: ALAT 59 U/l, ASAT 41 U/l, GGT 30 U/l, AP 84 U/l). Patient war beschwerdefrei und in gutem Allgemeinzustand.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Reactine 10 mg Filmtabl, Mc Neil, Einnahmezeitraum: 19.-21.09.2009,
Grippostad C Kps, STADA, Einnahmezeitraum: 02.-05.09.2009 sowie vorher
Paracetamol Tabl NN (in Spanien), Einnahmezeitraum: 24.-31.08.2009 und
Ibuprofen 200 mg Tabl, NN (in Spanien), Einnahmezeitraum: 24.-31.08.2009.

Gesamtbeurteilung:

Eine medikamenteninduzierte Genese der Hepatitis ist möglich.

Möglicherweise erlitt der Patient Ende August 2009 einen viralen Infekt mit den oben genannten Symptomen, in dessen Rahmen es auch zu einer Leberbeteiligung gekommen sein könnte. Entsprechende Infektionsserologie wurde aktuell nicht durchgeführt, eine Leberpunktion ebenfalls nicht. Sonographisch war die Leber unauffällig. Dennoch könnte auch eine medikamentenverursachte Hepatitis vorgelegen haben. Hier kommt am ehesten Reactine in Frage, aber aufgrund des zeitlichen Zusammenhangs auch Ibuprofen und Paracetamol bzw. Grippostad C.

Die Arzneimittelkausalität der genannten Arzneistoffe wird mit "möglich" bewertet.

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 19.02.2010 DE-SANOFI-AVENTIS-2010SA009043

Sender diagnosis: not reported

Narrative

Initial report:

This serious case was extracted from the website of the German Health Authorities (id no. DE-BFARM-09174811). Initially the case was reported from a German physician via a Regional pharmacovigilance centre (id no. DE-FAKOS-170273). The case involves a 28-year-old male patient with an allergy due to house dust, animal hair and pollinosis (gras) as well as allergic asthma. On 20-Aug-2009 the patient suffered from influenza-like symptoms with subfebrile temperature and joint pain as well as general malaise during a detention in Spain. Therefore he started self-medication on 24-Aug-2009 with ibuprofen (no trade name provided) 600 mg daily and paracetamol (no trade name provided). Therapy with ibuprofen and paracetamol was stopped on 31-Aug-2009 and the patient started therapy with caffeine/chlorphenamine maleate/paracetamol six dosage forms daily on 02-Sep-2009. Influenza-like symptoms were recovered at the beginning of September 2009. Therefore therapy with caffeine/chlorphenamine maleate/paracetamol was stopped on 05-Sep-2009. On 19-Sep-2009 the patient developed an allergic reaction of the upper respiratory tract. He received cetirizine 30 mg daily from 19-Sep-2009 to 21-Sep-2009. On 22-Sep-2009 he was hospitalized due to aggravation of general condition with nausea and vomiting as well as abdominal pain. During hospitalization increased values for transaminases were mentioned. Diagnosis: acute hepatitis. On 23-Sep-2009 the patient was discharged from the hospital in good general condition. In the further course transaminases decreased. During an ambulant treatment with pantoprazole sodium (from another company) due to erosive gastritis (proven in a gastroscopy) aminotransferases increased again. Therefore pantoprazole sodium was withdrawn and values decreased. Outcome of acute hepatitis: recovering/resolving.

Causality assessment according Sender (method: Global introspection (WHO GI)): possible for all suspected products.

NOVARTIS, 25.02.2010 DE-002147023-CHPA2010DE03101

Sender diagnosis: not reported

Narrative

Initial health authority report received 12 Feb 2010 from a physician, which was forwarded to the Regional Pharmacovigilance Center (case number: DE-BFARM-09174811). This 28 year-old male patient, with a history of allergies to house dust, animal hair, and grass, developed flu-like symptoms on 20 Aug 2009, with subfebrile temperature, limb pain and general impairment. He received treatment with the following medications: Paracetamol (acetaminophen) from 24 to 31 Aug 2009 (unknown dose), Ibuprofen (ibuprofen) from 24 to 31 Aug 2009, 200mg, orally, 1 tablet 3 times daily, and Grippostad C (paracetamol, caffeine, chlorphenamine maleate) 02 to 05 Sep 2009, orally, 2 DF three times daily. The flu-like symptoms abated completely at the beginning of Sep 2009. On 19 Sep 2009, the patient developed allergic symptoms of the upper respiratory tract, due to an allergy to cats, and therefore used Reactine (cetirizine), 10mg, 1 DF, three times daily, orally, for 3 days. On 22 Sep 2009, the patient was hospitalized due to aggravation of general condition, with nausea, vomiting, and abdominal pain, and acute hepatitis. The clinical chemistry showed significantly increased transaminases (ALAT: 305 U/l, normal range: < 41 U/l; ASAT 99 U/l, normal range: <37 U/l). Additional lab findings on 22 Sep 2009: GGT (gamma-glutamyltransferase) 63 U/l, normal range: 8-61 U/l; AP (Alkaline phosphatase) 116 U/l, normal range: 40-260 U/l; Bilirubin total 0.53 mg/dl, normal range: <1.2 mg/dl. On 23 Sep 2009, the patient was discharged from the hospital in good condition. The transaminases dropped down (lab values from 01 Oct 2009: ALAT 241 U/l, ASAT 60 U/l). During an ambulant treatment with Pantozol (pantoprazole) (date unknown), dosage unspecified, to treat erosive gastritis, as diagnosed by a gastroscopy (date unspecified), the patient again developed increased transaminase (lab values from 29 Oct 2009: ALAT 337 U/l, ASAT 463 U/l, additionally GGT 44 U/l). Lab values resolved (lab values from 18 Nov 2009: ALAT 59 U/l, ASAT 41 U/l, GGT 30 U/l, AP 84 U/l) after treatment with Pantozol was discontinued. The patient was symptom free and in good general condition. As trigger for the hepatitis, the following medicinal products were suspected: Reactine, Grippostad C, Paracetamol, and Ibuprofen.

The available information was considered inadequate to fully assess the case.

Reporter's comment:GlaxoSmithKline, 26.02.2010 DE-GlaxoSmithKline-B0636094A

Sender diagnosis: not reported

Narrative

This case was reported by a regulatory authority, via UCB and described the occurrence of acute hepatitis in a 28-year-old male patient who received Cetirizine hydrochloride (Reactine) over a period of 4 days for allergy.

Co-suspect medication included Paracetamol, Ibuprofen and Grippostad.

On 19 September 2009 the patient started Cetirizine hydrochloride (oral). Approximately 3 days later, on 22 September 2009, the patient was hospitalized due to nausea, vomiting, abdominal pain and decreased general health. On the same day, increased liver enzyme activity was found: Alanine transaminase (ALAT) 305 U/L (normal range less than 41 U/L), aspartate aminotransferase (ASAT) 99 U/L (normal range less than 37 U/L) and additional laboratory results were also noted as: gamma-glutamyl transpeptidase (gGT) 63 U/L (normal range 8-61 U/L), alkaline phosphatase (aP) 116 U/L (normal range less than 40-260 U/L), total bilirubin 0.53 mg/dl (normal range less than 1.2 mg/dl). Treatment with Cetirizine hydrochloride was discontinued on 22 September 2009. It was unspecified whether cetirizine hydrochloride was discontinued due to the events. On 23 September 2009, the patient was discharged in good health. On 01 October 2009, laboratory results noted ALAT 241 U/L, ASAT 60 U/L. The patient received outpatient treatment with pantoprazol due to gastritis. Liver enzymes again increased. On 29 October 2009, laboratory values were reported as ALAT 337 U/L, ASAT 463 U/L, gGT 44 U/L. After pantoprazol was stopped, the liver enzyme activity decreased again. On 18 November 2009, laboratory results noted: ALAT 59 U/L, ASAT 41 U/L, gGT 30 U/L, aP 84 U/L. The patient recovered on an unknown date and was in good health. On an unknown date, a liver ultrasound had a normal appearance. The regulatory authority reported that the events were clinically significant (or requiring intervention). At the time of reporting, the events were resolved. The regulatory authority reported that the events were possibly related to treatment with Cetirizine hydrochloride. It was reported that the causal relationship of the event to the four suspect drugs was determined to be possibly.

GlaxoSmithKline, 01.03.2010 DE-GlaxoSmithKline-B0636094A

Sender diagnosis: not reported

Narrative

This case was reported by a regulatory authority, via UCB and described the occurrence of acute hepatitis in a 28-year-old male patient who received Cetirizine hydrochloride (Reactine) over a period of 4 days for allergy.

Co-suspect medication included Paracetamol, Ibuprofen and Grippostad.

On 19 September 2009 the patient started Cetirizine hydrochloride (oral). Approximately 3 days later, on 22 September 2009, the patient was hospitalized due to nausea, vomiting, abdominal pain and decreased general health. On the same day, increased liver enzyme activity was found: Alanine transaminase (ALAT) 305 U/L (normal range less than 41 U/L), aspartate aminotransferase (ASAT) 99 U/L (normal range less than 37 U/L) and additional laboratory results were also noted as: gamma-glutamyl transpeptidase (gGT) 63 U/l (normal range 8-61 U/l), alkaline phosphatase (aP) 116 U/l (normal range less than 40-260 U/l), total bilirubin 0.53 mg/dl (normal range less than 1.2 mg/dl). Treatment with Cetirizine hydrochloride was discontinued on 22 September 2009. It was unspecified whether cetirizine hydrochloride was discontinued due to the events. On 23 September 2009, the patient was discharged in good health. On 01 October 2009, laboratory results noted ALAT 241 U/l, ASAT 60 U/l. The patient received outpatient treatment with pantoprazol due to gastritis. Liver enzymes again increased. On 29 October 2009, laboratory values were reported as ALAT 337 U/l, ASAT 463 U/l, gGT 44 U/l. After pantoprazol was stopped, the liver enzyme activity decreased again. On 18 November 2009, laboratory results noted: ALAT 59 U/l, ASAT 41 U/l, gGT 30 U/l, aP 84 U/l. The patient recovered on an unknown date and was in good health. On an unknown date, a liver ultrasound had a normal appearance. The regulatory authority reported that the events were clinically significant (or requiring intervention). At the time of reporting, the events were resolved. The regulatory authority reported that the events were possibly related to treatment with Cetirizine hydrochloride. It was reported that the causal relationship of the event to the four suspect drugs was determined to be possibly.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10002875

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials: Sex: Female Birth: 19580203 Age: 51 Year weight (kg): 75 BMI_int: 26.57 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	20090930	20091022	23 Day		not recovered/not resolved
2 Drug abuse					Unknown
3 Gallbladder enlargement					recovered/resolved with sequel
4 Metabolic disorder					recovered/resolved with sequel
5 Scleral icterus	20090930				recovered/resolved with sequel
6 Skin coloring yellow	20090930				recovered/resolved with sequel
7 Hepatomegaly					recovered/resolved with sequel
8 Hepatitis cholestatic	20090930	20091022	23 Day		recovered/resolved with sequel
9 Fatty liver					recovered/resolved with sequel
10 Drug interaction					Unknown
11 Icterus					Unknown
12 Pruritus					Unknown
13 Reduced general condition	20090930				recovered/resolved with sequel
14 Scleral icterus					Unknown
15 Liver cirrhosis					Unknown
16 Cirrhosis of liver					recovered/resolved with sequel
17 Drug interaction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-ratiopharm 500 lysinat schmerztabletten ibuprofen	Oral	3000 Mg milligram(s)/Day 82	20090924-20091001 8 Day	Intervertebral disc disorder
2 PARACETAMOL paracetamol	Oral	6*500 Mg milligram(s)/4 D 245	1999-20090923 10 Year	Intervertebral disc disorder
3 RASILEZ HCT aliskiren fumarate; hydrochlorothiazide	Oral	1 Df dosage form/Day 245	-20091001 -	Hypertension arterial

Drugs (concomitant)

4 PANTOZOL pantoprazole sodium	Oral	80 Mg milligram(s)/Day 94	2000- -	Gastric ulcer prophylaxis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Helicobacter gastritis			Unknown	chronic active helicobacter-pylori-gastritis
Smoker			Yes	Beginn unbekannt, 25py, seit 3 Jahren 13 Zigaretten pro Tag
Alcohol use	2002		Yes	gelegentlich (zu Feiern) 1 Glas Wein
Alcohol abuse	1991	2002	Yes	alcohol abuse
Polydrug abuse	1999		Yes	Paracetamol (seit 1999 alle 4h 1 Tbl. á 500mg), Ibuprofen (seit 09/09: 40 Tbl. in 7 Tagen á 500mg)
Hypertension arterial			Yes	Beginn unbekannt
Duodenal ulcer	2000	2000	Unknown	Beginn unbekannt
Reflux esophagitis			Unknown	peptic esophagitis II
Axial hiatal hernia			Unknown	axial hiatal hernia
Intervertebral disc disorder	1999		Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			
Qualification:	Pharmacist	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20091006	0.83 umol/*s	NA	
Alanine aminotransferase	20091026	49.8 U/l	NA	
Alanine aminotransferase	20091022	46.5 U/l	NA	
Alanine aminotransferase	20091014	41.6 U/l	NA	
Alanine aminotransferase	20091006	45.4 U/l	NA	
Alanine aminotransferase	20091001	0.93 umol/*s	NA	
Alanine aminotransferase	20091005	0.77 umol/*s	NA	
Alanine aminotransferase	20091004	0.75 umol/*s	NA	
Alanine aminotransferase	20091003	0.77 umol/*s	NA	
Alanine aminotransferase	20091002	0.79 umol/*s	NA	
Alkaline phosphatase	20091022	160 U/l	NA	
Alkaline phosphatase	20091014	202 U/l	NA	
Alkaline phosphatase	20091006	205 U/l	NA	
Alkaline phosphatase	20091026	143 U/l	NA	
Alkaline phosphatase	20091003	NA	NA	
Alkaline phosphatase	20091002	NA	NA	
Alkaline phosphatase	20091001	NA	NA	
Alkaline phosphatase	20091001	3.50 SI-Einheit	NA	
Alkaline phosphatase	20091002	3.03 SI-Einheit	NA	
Alkaline phosphatase	20091003	NA	NA	
Alkaline phosphatase	20091005	3.61 SI-Einheit	NA	
Alkaline phosphatase	20091006	4.03 SI-Einheit	NA	
Alkaline phosphatase	20091004	3.29 SI-Einheit	NA	

DE-BFARM-10002875			
Ammonia	20091001	66 umol/l	NA
Aspartate aminotransferase	20091005	2.69 umol/*s	NA
Aspartate aminotransferase	20091006	204 U/l	NA
Aspartate aminotransferase	20091014	145 U/l	NA
Aspartate aminotransferase	20091022	211 U/l	NA
Aspartate aminotransferase	20091026	231 U/l	NA
Aspartate aminotransferase	20091006	3.26 umol/*s	NA
Aspartate aminotransferase	20091004	2.53 umol/*s	NA
Aspartate aminotransferase	20091003	2.12 umol/*s	NA
Aspartate aminotransferase	20091002	1.98 umol/*s	NA
Aspartate aminotransferase	20091001	2.29 umol/*s	NA
Bilirubin direct	20091003	155.7 umol/l	NA
Bilirubin direct	20091001	182.1umol/l	NA
Bilirubin direct	20091002	146.1 umol/l	NA
Bilirubin direct	20091006	281.2 umol/l	NA
Bilirubin direct	20091005	214.9 umol/l	NA
Bilirubin direct	20091004	151.9 umol/l	NA
Bilirubin total	20091007	NA	NA
Bilirubin total	20091013	NA	NA
Bilirubin total	20091006	298.6 ?mol/l	NA
Bilirubin total	20091020	198 ?mol/l	NA
Bilirubin total	20091022	164 ?mol/l	NA
Bilirubin total	20091026	119 ?mol/l	NA
Bilirubin total	20091014	329 ?mol/l	NA
Bilirubin total	20091005	230.4 ?mol/l	NA
Bilirubin total	20091004	193.6 ?mol/l	NA
Bilirubin total	20091003	161.9 ?mol/l	NA
Bilirubin total	20091002	161.8 ?mol/l	NA
Bilirubin total	20091001	193.1 ?mol/l	NA
Biopsy liver	20091013	NA	NA
Blood bilirubin indirect	20091006	126 umol/l	NA
Chest X-ray	20091001	NA	NA
C-reactive protein	20091026	22.2 mg/L	NA
C-reactive protein	20091022	28.2 mg/L	NA
C-reactive protein	20091020	33.9 mg/L	NA
C-reactive protein	20091001	2.3 mg/L	NA
C-reactive protein	20091016	56.7 mg/L	NA
CT scan	20091022	NA	NA
ECG	20091001	NA	NA
Gamma GT	20091022	268 U/l	NA
Gamma GT	20091004	NA	NA
Gamma GT	20091001	11.66 umol/l	NA
Gamma GT	20091026	272 U/l	NA
Gamma GT	20091003	NA	NA

Gamma GT	20091014	243 U/l	NA
Gamma GT	20091006	363 U/l	NA
Gamma GT	20091007	NA	NA
Gamma GT	20091006	6.37 umol/l	NA
Gamma GT	20091005	6.06 umol/l	NA
Gamma GT	20091002	9.73 umol/l	NA
Gastroscopy	20091007	NA	NA
LDH	20091006	282 U/L	NA
Ultrasonography	20091001	NA	NA
Ultrasonography	20091007	NA	NA

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 13.01.2010 DE-PVZIBE-02-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau

KURZZUSAMMENFASSUNG:

alkohol- und medikamententoxische cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau durch Paracetamol- und Ibuprofenabusus, Rasilez und HCT könnten interagierend gewirkt haben

BESCHREIBUNG:

seit 1999 bis 23.09.2009 am Tag alle 4 Stunden 1 Tablette Paracetamol á 500mg wegen Bandscheibenproblemen, danach 40 Tabletten Ibuprofen á 500mg in 7 Tagen;

am 30.09.2009 Gelbfärbung der Haut, die bis 01.10.2009 zugenommen hatte, daraufhin Hausarzt aufgesucht, Klinikeinweisung (01.-06.10.2009 peripheres Krankenhaus) mit reduziertem Allgemeinzustand und deutlichem Haut- und Sklerenikterus

THERAPIE:

zuweisende Klinik: Vit.-B-duo, Vit.-K. intravenös, fettfreie Kost, daraufhin sanken die Leberwerte, seit 04.10.2009 stiegen diese jedoch wieder (daraufhin Verlegung ins tertiäre Krankenhaus);

tertiäres Krankenhaus: Vit.-K, Vit.-B1 und Vit.-B6, Fenistil und Cholestyramin gegen Juckreiz;

Paracetamol abgesetzt am 23.09.09; Ibuprofen abgesetzt am 01.10.09

unter der Therapie sinkende Leber- und Bilirubinwerte

VERLAUF:

01.10.2009 EKG: Sinustachykardie, unauffälliger Erregungsablauf;

01.10.2009 Röntgen-Thorax: unauffällig;

Sonographie Abdomen: massive Hepatomegalie mit Zirrhosezeichen, kein Aszitis, keine Cholestasezeichen;

07.10.2009 Autoimmunodiagnostik negativ: ANA=1:320, AMA=1:320, AMA-M2 grenzwertig, 3E(BPO) AK positiv (als Abbaustörung aufgrund der Leberinsuffizienz interpretiert), negativ: Ros2-AK, SP-100-AK, PML-AK, gp210-AK, LKM1-AK, LC1-AK, SLA/LP, PCA-AK; intrinsikfaktor-AK, IgA-AK (ASCA), IgG-AK (ASCA);

07.10.2009 Virologie: HBS-AK und HCV-IgG/M: keine Infektion; HA-IgM/G: entweder vor Jahren durchgemachte Erkrankung oder Antikörper nach Impfung;

07.10.2009 Sonographie Abdomen: Hepatomegalie mit Leberparenchymveränderungen wie bei Fettleberhepatitis, Gallenblase vergrößert mit betonter Gallenblasenwand als Begleitödem bei Hepatitis;

07.10.2009 Gastroskopie: Refluxösophagitis II° bei axialer Hiatushernie, Gastritis, Histologie: geringgradige chronisch aktive Helicobacter-pylori-Gastritis von Antrum und Korpus, kein Anhalt für Malignität;

13.10.2009 Biopsie rechter Leberlappen: Histologie: mäßig aktive (alkoholische) Fettleberhepatitis mit beginnendem zirrrogenem Umbau und Cholestasezeichen, kombinierte toxische Leberschädigung mit zusätzlichen medikamentösen Noxen, kein Anhalt für Malignität und Spezifität

16.10.2009 Röntgen-Thorax: unauffällig;

22.10.2009 CT-Thorax: vergrößerte Leber, ansonsten unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 06.-27.10.2009;

Krankenseinweisung aufgrund von Ikterus und reduziertem Allgemeinzustand bei hepatitischen Veränderungen mit beginnendem zirrhotischem Umbau;

Differentialdiagnostisch: Autoimmunodiagnostik negativ, Gallenblase unauffällig, Virologie unauffällig;

Entlassungsbrief: "Diagnose einer mäßig aktiven alkoholischen Fettleberhepatitis mit beginnendem zirrrogenen Umbau- und Cholestasezeichen, eventuell auch mit zusätzlicher Schädigung durch medikamentöse Noxen, so dass in Zusammenschau aller Befunde, der chronische Alkoholabusus der Patientin, sowie die zuletzt massive Einnahme von Schmerzmedikamenten als Ursachen für die beginnende Leberzirrhose anzusehen sind." ; kein Anhalt für akuten Alkoholexzeß und Atemwegsinfekt;

In der Zusammenschau ist eine chronische Leberparenchymschädigung auf dem Boden eines chronischen Alkoholkonsums (anamnestisch bis 2002) sowie 10jähriger chronischer Paracetamoleinnahme als möglich zu erachten. Beide Noxen stellen auch für sich alleine eine mögliche Ursache dar -

die Toizität von Paracetamol ist bei begleitendem Alkoholkonsum erhöht. Langjährige hepatitische Schäden könnten den beginnenden zirrhotischen Umbau bedingt haben. Akuter Auslöser könnte möglicherweise die erhöhte Ibuprofeneinnahme darstellen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: rasilez hct

BETREFFEN<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 18.01.2010 DE-SANOFI-AVENTIS-2010SA002246

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10002875).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: alcohol- and drug-toxic cholestatic hepatitis with beginning cirrhotic conversion by paracetamol and ibuprofen abuse.

Aliskiren+hydrochlorothiazide could have an interactive effect.

This case involves a 51-year-old female (smoker, body weight 75 kg) with a past medical history of alcohol abuse from 1999 to 2002 (since 2002 only occasionally one glass of wine) and duodenal ulcer; at time of event, the patient was suffering from polydrug abuse (paracetamol and ibuprofen), arterial hypertension, and intervertebral disc disorder. The following drug was reported to have been taken concomitantly: pantoprazole.

The patient started treatment with paracetamol (trade name not ascertainable) 500mg every four hours in 1999 for intervertebral disc disorder. In Sep.2009, she additionally initiated treatment with ibuprofen (trade name not ascertainable) 40 tablets in seven days.

On 30-Sep-2009, the patient observed yellow colouration of her skin that increased on the following day, so that the patient went to her family doctor.

The patient was hospitalised in a peripheral hospital with reduced general condition and pronounced icterus of skin and sclerae. She was treated with vitamin B1+B6, intravenous vitamin K, and fat-free diet, leading to a decrease in liver values. On 04-Oct-2009, the values started to increase again, so that the patient was transferred to a tertiary hospital on 06-Oct-2009. Here she was treated with vitamin K, vitamin B1 and B6, and dimetindene maleate and colestyramine for pruritus. Under treatment, liver and bilirubin values decreased. Paracetamol was discontinued on 23-Sep-2009, ibuprofen was discontinued on 01-Oct-2009. Autoimmune diagnostics were negative, virology was unremarkable, gallbladder was unremarkable.

Outcome was reported as "recovered/resolved with sequel" on 22-Oct-2009. The patient was discharged from hospital on 27-Oct-2009. Discharge letter read: "diagnosis of a moderately active alcoholic fatty liver hepatitis with signs of beginning cirrhotic conversion and cholestasis, maybe also with additional damage by medicamentous noxae, so that, as summary of all findings, the patient's chronic alcohol abuse as well as the at last massive intake of analgesics could be considered as causes of the beginning liver cirrhosis." There was no indication of acute alcoholic excess or respiratory tract infection. As synopsis, a chronic damage of liver parenchyma on the base of chronic alcohol consumption and 10 years of chronic paracetamol intake has to be considered possible. Each noxa by itself can also be regarded as possible cause - toxicity of paracetamol is increased under additional intake of alcohol. Long years of hepatic damage could have led to beginning cirrhotic conversion. Acute catalyst could have possibly been the increased ibuprofen intake.

Causality determined by PV centre:

"possible" for paracetamol, ibuprofen, and aliskiren+hydrochlorothiazide (interacting).

Comments concerning causality: Increased liver enzymes are a described adverse effect of paracetamol. In connection with alcohol, liver damage can occur. The long-term, regular high-dose intake could have caused chronic hepatic damages, that could have led to beginning cirrhotic conversion.

Liver function disorders, liver damage, and liver failure (especially under long-term therapy) and acute hepatitis are described adverse effects of ibuprofen. Drugdex:cases of icterus are described after oral treatment, hepatotoxicity after three days, at the earliest. Icterus is described as adverse effect of aliskiren+hydrochlorothiazide, the chronologic element is remote, there is a possibility of interaction with paracetamol and ibuprofen.

Possible alternative cause: chronic alcohol consumption in past medical history (acute alcohol excess was negated by patient).

Bayer Vital GmbH, 21.01.2010 DE-BAYER-201011670GPV

Sender diagnosis: not reported

Narrative

This solicited report was forwarded to the German Health Authority from a pharmacist in Germany via a pharmacovigilance center, which systematically screens for adverse drug reactions in certain hospitals. The report was identified on the ADR-internet page of the German Health Authority (BfArM) on 20-Nov-2009, referring to a 51 year-old female (height: 168 cm, weight: 75 kg).

Medical and drug history:

Patient's concomitant conditions included smoking, alcohol abuse until 2002, then alcohol use continuing, and arterial hypertension. Patient had a duodenal ulcer until 2000. Concomitant drugs have been given. For details see section "Concomitant Drugs".

Patient received the following drugs, reported as 'suspect drugs':

-oral Paracetamol (trade name unknown) 500 mg was taken every 4 hours at a total daily dose of 3000 mg (6 tablets) a day for intervertebral disc disorder from 1999 to 23-Sep-2009,

-oral Ibuprofen (trade name unknown) 500 mg film-coated tablet was taken thrice daily at a total dose of 40 tablets within 7 days for intervertebral disc disorder from 24-Sep-2009 until 01-Oct-2009,

-oral Rasilez (aliskiren/ hydrochlorothiazide) 40 mg film-coated tablet was taken twice daily for arterial hypertension from unspecified start date until 01-Oct-2009.

Onset, progression and outcome of the event:

On 30-Sep-2009 skin icterus, increasing until 01-Oct-2009. Consultation of family physician; hospitalization (01-Oct-2009 to 06-Oct-2009, and 06-Oct-2009 to 27-Oct-2009) with reduced general condition and significant scleral and skin icterus. Tests showed CHOLESTATIC HEPATITIS WITH BEGINNING CIRRHOTIC ALTERATIONS (see 'Other Tests' and 'Lab Test' sections).

Therapy: Vitamin B duo, Vitamin K intravenously, greaseless nutrition. Laboratory values dropped, but raised again on 04-Oct-2009. Therapy was continued with Vitamin K, B1 and B6, Fenistil/ dimetindenmaleate and cholestyramin against pruritus. Paracetamol was withdrawn on 23-Sep-2009, Ibuprofen on 01-Oct-2009. Undertherapy dropping liver and BILI values. At the time of the report, patient has recovered with sequelae.

Reporter's causality assessment:

Alcohol- and drug induced CHOLESTATIC HEPATITIS WITH BEGINNING CIRRHOTIC ALTERATIONS caused by Paracetamol- and Ibuprofen abuse. Rasilez HCT (aliskiren/ hydrochlorothiazide) could have been interacting.

Comment: Chronic abuse of alcohol and intake of pain-reliever were considered as cause of beginning livercirrhosis. No signs of acute alcohol excess nor respiratory tract infection. In synopsis, chronic hepatosis was possibly caused by alcohol abuse (until 2002) and 10 years of intake of paracetamol. However, both noxa could be causes on their own.

Toxicity of paracetamol is increased with concomitant consumption of alcohol. Long- time damage of liver could have caused cirrhotic alteration. Acute trigger could have been intake of ibuprofen.

HELIOS Klinikum Wuppertal, 22.01.2010 DE-PVZIBE-02-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau

KURZZUSAMMENFASSUNG:

alkohol- und medikamententoxische cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau durch Paracetamol- und Ibuprofenabusus (Aliskiren/HCT; Pantoprazol)

BESCHREIBUNG:

seit 1999 bis 23.09.2009 am Tag alle 4 Stunden 1 Tablette Paracetamol á 500mg wegen Bandscheibenproblemen, danach 40 Tabletten Ibuprofen á 500mg in 7 Tagen; bis 2002 chronischer Alkoholkonsum

am 30.09.2009 Gelbfärbung der Haut, die bis 01.10.2009 zugenommen hatte, daraufhin Hausarzt aufgesucht, Klinikeinweisung (01.-06.10.2009 peripheres Krankenhaus; 06.10. bis 27.10.2009 tertiäres Krankenhaus) mit reduziertem Allgemeinzustand und deutlichem Haut- und Sklerenikterus

THERAPIE:
zuweisende Klinik: Vit.-B-duo, Vit.-K. intravenös, fettfreie Kost, daraufhin sanken die Leberwerte, seit 04.10.2009 stiegen diese jedoch wieder (daraufhin Verlegung ins tertiäre Krankenhaus);

tertiäres Krankenhaus: Vit.-K, Vit.-B1 und Vit.-B6, Fenistil und Cholestyramin gegen Juckreiz;

Paracetamol abgesetzt am 23.09.09; Ibuprofen abgesetzt am 01.10.09

unter der Therapie sinkende Cholestasewerte

VERLAUF:

01.10.2009 EKG: Sinustachykardie, unauffälliger Erregungsablauf;

01.10.2009 Röntgen-Thorax: unauffällig;

Sonographie Abdomen: massive Hepatomegalie mit Zirrhosezeichen, kein Aszitis, keine Cholestasezeichen;

07.10.2009 Autoimmundiagnostik negativ: ANA=1:320, AMA=1:320, AMA-M2 grenzwertig, 3E(BPO) AK positiv (als Abbaustörung aufgrund der Leberinsuffizienz interpretiert), negativ: Ros2-AK, SP-100-AK, PML-AK, gp210-AK, LKM1-AK, LC1-AK, SLA/LP, PCA-AK; intrinsikfaktor-AK, IgA-AK (ASCA), IgG-AK (ASCA);

07.10.2009 Virologie: HBS-AK und HCV-IgG/M: keine Infektion; HA-IgM/G: entweder vor Jahren durchgemachte Erkrankung oder Antikörper nach Impfung;

07.10.2009 Sonographie Abdomen: Hepatomegalie mit Leberparenchymveränderungen wie bei Fettleberhepatitis, Gallenblase vergrößert mit betonter Gallenblasenwand als Begleitödem bei Hepatitis;

07.10.2009 Gastroskopie: Refluxösophagitis II° bei axialer Hiatushernie, Gastritis, Histologie: geringgradige chronisch aktive Helicobacter-pylori-Gastritis von Antrum und Korpus, kein Anhalt für Malignität;

13.10.2009 Biopsie rechter Leberlappen: Histologie: mäßig aktive (alkoholische) Fettleberhepatitis mit beginnendem zirrhogenem Umbau und Cholestasezeichen, kombinierte toxische Leberschädigung mit zusätzlichen medikamentösen Noxen, kein Anhalt für Malignität und Spezifität

16.10.2009 Röntgen-Thorax: unauffällig;

22.10.2009 CT-Thorax: vergrößerte Leber, ansonsten unauffällig

unter der Therapie sinkende Cholestaseparameter, Transaminasen noch unverändert

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 01.-27.10.2009;

Krankenseinweisung aufgrund von Ikterus und reduziertem Allgemeinzustand bei hepatitischen Veränderungen mit beginnendem zirrhotischem Umbau;

Differentialdiagnostisch: Autoimmundiagnostik negativ, Gallenblase unauffällig, Virologie unauffällig;

Entlassungsbrief: "Diagnose einer mäßig aktiven alkoholischen Fettleberhepatitis mit beginnendem zirrhogenen Umbau- und Cholestasezeichen, eventuell auch mit zusätzlicher Schädigung durch medikamentöse Noxen, so dass in Zusammenschau aller Befunde, der chronische Alkoholabusus

der Patientin, sowie die zuletzt massive Einnahme von Schmerzmedikamenten als Ursachen für die beginnende Leberzirrhose anzusehen sind." ; kein Anhalt für akuten Alkoholexzeß und Atemwegsinfekt;

In der Zusammenfassung ist eine chronische Leberparenchymschädigung auf dem Boden eines chronischen Alkoholkonsums (anamnestisch bis 2002) sowie 10jähriger chronischer Paracetamoleinnahme als möglich zu erachten. Beide Noxen stellen auch für sich alleine eine mögliche Ursache dar - die Toxizität von Paracetamol ist bei begleitendem Alkoholkonsum erhöht. Langjährige hepatitische Schäden könnten den beginnenden zirrhotischen Umbau bedingt haben. Akuter Auslöser könnte möglicherweise die erhöhte Ibu<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 25.01.2010 DE-002147023-PHHY2010DE02669

Sender diagnosis: not reported

Narrative

Initial report received from received from a regional pharmacovigilance center from a physician (intensive monitoring project) via HA on 13 Jan 2010: This patient had a history of alcohol abuse, smoker, polydrug abuse (ibuprofen, paracetamol, Tegen), duodenal ulcer and intervertebral disc disorder. The patient started treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), 150/12.5 mg for hypertension arterial on an unspecified date. She was also on treatment with paracetamol, 500 mg for intervertebral disc disorder since 1999 (unknown date). On 23 Sep 2009 treatment with paracetamol was stopped. On 24 Sep 2009 treatment with Ibuprofen, 1000 mg for intervertebral disc disorder was initiated. On 30 Sep 2009 she experienced alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse. On 30 Sep 2009, it was seen yellow coloration of the skin, which had increased up to 01 Oct 2009. On 01 Oct 2009 treatment with Rasilez HCT and Ibuprofen were stopped. She was admitted to hospital, with impaired general condition and with pronounced skin and scleral jaundice on 01 Oct 2009. Following lab abnormalities were seen: On 01 Oct 2009, Abdominal sonography showed massive hepatomegaly with signs of cirrhosis, no ascites, no signs of cholestasis. On 07 Oct 2009, 3E(BPO) antibodies positive (interpreted as metabolic disorder due to the hepatic insufficiency) was seen, abdominal sonography showed hepatomegaly with liver parenchymal changes of the kind associated with fatty liver hepatitis, gallbladder enlarged with accentuated gallbladder wall as accompanying edema in hepatitis. On 13 Oct 2009 biopsy of right lobe of liver; histology: moderately active (alcoholic) fatty liver hepatitis with incipient cirrhotic remodelling and signs of cholestasis, combined toxic liver damage with additional medicinal drug related injurious factors, no evidence of malignancy and specificity. On 22 Oct 2009 Thoracic computed tomography (CT) showed enlarged liver, otherwise no abnormalities. She was treated with vitamin B duo, vitamin K intravenous, fat-free diet, vitamin K, vitamin B1 and vitamin B6, Fenistil and cholestyramine for pruritus. On the treatment, decreasing liver and bilirubin values. On 22 Oct 2009, the patient was resolved/ recovered with sequel. The reporter suspected the relationship between the events and Rasilez HCT, paracetamol and Ibuprofen commenting Rasilez HCT, could have had an interacting effect.

sanofi-aventis, 29.01.2010 DE-SANOFI-AVENTIS-2010SA002246

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10002875).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: alcohol- and drug-toxic cholestatic hepatitis with beginning cirrhotic conversion by paracetamol and ibuprofen abuse.

Aliskiren+hydrochlorothiazide could have an interactive effect.

This case involves a 51-year-old female (smoker, body weight 75 kg) with a past medical history of alcohol abuse from 1999 to 2002 (since 2002 only occasionally one glass of wine) and duodenal ulcer; at time of event, the patient was suffering from polydrug abuse (paracetamol and ibuprofen), arterial hypertension, and intervertebral disc disorder. The following drug was reported to have been taken concomitantly: pantoprazole.

The patient started treatment with paracetamol (trade name not ascertainable) 500mg every four hours in 1999 for intervertebral disc disorder. In Sep.2009, she additionally initiated treatment with ibuprofen (trade name not ascertainable) 40 tablets in seven days.

On 30-Sep-2009, the patient observed yellow colouration of her skin that increased on the following day, so that the patient went to her family doctor. The patient was hospitalised in a peripheral hospital with reduced general condition and pronounced icterus of skin and sclerae. She was treated with vitamin B1+B6, intravenous vitamin K, and fat-free diet, leading to a decrease in liver values. On 04-Oct-2009, the values started to increase again, so that the patient was transferred to a tertiary hospital on 06-Oct-2009. Here she was treated with vitamin K, vitamin B1 and B6, and dimetindene maleate and colestyramine for pruritus. Under treatment, liver and bilirubin values decreased. Paracetamol was discontinued on 23-Sep-2009, ibuprofen was discontinued on 01-Oct-2009. Autoimmune diagnostics were negative, virology was unremarkable, gallbladder was unremarkable. Outcome was reported as "recovered/resolved with sequel" on 22-Oct-2009. The patient was discharged from hospital on 27-Oct-2009. Discharge letter read: "diagnosis of a moderately active alcoholic fatty liver hepatitis with signs of beginning cirrhotic conversion and cholestasis, maybe also with additional damage by medicamentous noxae, so that, as summary of all findings, the patient's chronic alcohol abuse as well as the at last massive intake of analgesics could be considered as causes of the beginning liver cirrhosis." There was no indication of acute alcoholic excess or respiratory tract infection. As synopsis, a chronic damage of liver parenchyma on the base of chronic alcohol consumption and 10 years of chronic paracetamol intake has to be considered possible. Each noxa by itself can also be regarded as possible cause - toxicity of paracetamol is increased under additional intake of alcohol. Long years of hepatic damage could have led to beginning cirrhotic conversion. Acute catalyst could have possibly been the increased ibuprofen intake.

Causality determined by PV centre:

"possible" for paracetamol, ibuprofen, and aliskiren+hydrochlorothiazide (interacting).

Comments concerning causality: Increased liver enzymes are a described adverse effect of paracetamol. In connection with alcohol, liver damage can occur. The long-term, regular high-dose intake could have caused chronic hepatic damages, that could have led to beginning cirrhotic conversion. Liver function disorders, liver damage, and liver failure (especially under long-term therapy) and acute hepatitis are described adverse effects of

ibuprofen. Drugdex: cases of icterus are described after oral treatment, hepatotoxicity after three days, at the earliest. Icterus is described as adverse effect of aliskiren+hydrochlorothiazide, the chronologic element is remote, there is a possibility of interaction with paracetamol and ibuprofen. Possible alternative cause: chronic alcohol consumption in past medical history (acute alcohol excess was negated by patient).

Additional information extracted from the website of the German health authority on 25-Jan-2010. Information by PV centre: the patient was treated with ibuprofen manufactured by Ratiopharm. Pantoprazole was changed to "suspect drug". The patient's cholestasis parameters decreased under treatment, transaminases remained unchanged.

Causality assessment by PV centre (method: WHO score):

"possible" for paracetamol and ibuprofen

"unlikely" for aliskiren+hydrochlorothiazide and pantoprazole

Comment concerning pantoprazole: liver damage and icterus are described under treatment with pantoprazole; cholestasis improved under continuation of treatment with pantoprazole.

NOVARTIS, 03.02.2010 DE-002147023-PHHY2010DE02669

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center from a physician (intensive monitoring project) via HA on 13 Jan 2010: This patient had a history of alcohol abuse, smoker, polydrug abuse (ibuprofen, paracetamol, Tegen), duodenal ulcer and intervertebral disc disorder. The patient started treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), 150/12.5 mg for hypertension arterial on an unspecified date. She was also on treatment with paracetamol, 500 mg for intervertebral disc disorder since 1999 (unknown date). On 23 Sep 2009 treatment with paracetamol was stopped. On 24 Sep 2009 treatment with Ibuprofen, 1000 mg for intervertebral disc disorder was initiated. On 30 Sep 2009 she experienced alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse. On 30 Sep 2009, it was seen yellow coloration of the skin, which had increased up to 01 Oct 2009. On 01 Oct 2009 treatment with Rasilez HCT and Ibuprofen were stopped. She was admitted to hospital, with impaired general condition and with pronounced skin and scleral jaundice on 01 Oct 2009. Following lab abnormalities were seen: On 01 Oct 2009, Abdominal sonography showed massive hepatomegaly with signs of cirrhosis, no ascites, no signs of cholestasis. On 07 Oct 2009, 3E(BPO) antibodies positive (interpreted as metabolic disorder due to the hepatic insufficiency) was seen, abdominal sonography showed hepatomegaly with liver parenchymal changes of the kind associated with fatty liver hepatitis, gallbladder enlarged with accentuated gallbladder wall as accompanying edema in hepatitis. On 13 Oct 2009 biopsy of right lobe of liver; histology: moderately active (alcoholic) fatty liver hepatitis with incipient cirrhotic remodelling and signs of cholestasis, combined toxic liver damage with additional medicinal drug related injurious factors, no evidence of malignancy and specificity. On 22 Oct 2009 Thoracic computed tomography (CT) showed enlarged liver, otherwise no abnormalities. She was treated with vitamin B duo, vitamin K intravenous, fat-free diet, vitamin K, vitamin B1 and vitamin B6, Fenistil and cholestyramine for pruritus. On the treatment, decreasing liver and bilirubin values. On 22 Oct 2009, the patient was resolved/ recovered with sequel. The reporter suspected the relationship between the events and Rasilez HCT, paracetamol and Ibuprofen commenting Rasilez HCT, could have had an interacting effect.

Follow-up information was received from a regional pharmacovigilance center (intensive monitoring project) via HA on 22 Jan 2010: This patient had chronic alcohol abuse until 2002. Alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse (aliskiren/hydrochlorothiazide; pantoprazole). Cholestasis value was decreasing during therapy, transaminases were unchanged. Hospitalisation in a peripheral hospital from 01 Oct until 06 Oct 2009 and in a tertiary hospital from 06 Oct until 27 Oct 2009. To the explanations of causalities Pantozol (pantoprazole) was added regarding the ADR cholestatic hepatitis with the following comment: Hepatocellular damages and icterus are described for pantoprazole. There was an improvement of the cholestasis while pantoprazole was continued. Symptomatically the symptoms were explainable by the chronic intake of paracetamol and high doses of ibuprofen. Alternative cause: chronic ethanol toxic damage. Outcome of the patient's condition was reported as not recovered. The causal relationship for Rasilez HCT was changed from 'possible' to unlikely and the causal relationship to Pantozol was noted as unlikely.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10004588

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 51 Year	weight (kg): 75	BMI_int: 26.57	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	20090930	20091022	23 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	20090924-20091001	Intervertebral disc disorder
ibuprofen		82	8 Day	
2 PANTOZOL	Oral	80 Mg milligram(s)/Day	2000-	Gastric ulcer prophylaxis
pantoprazole sodium			-	
3 PARACETAMOL	Oral	/	1999-20090923	Intervertebral disc disorder
paracetamol		245	-	
4 rasilez hct		/	-20091001	Hypertension arterial
aliskiren fumarate; hydrochlorothiazide		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol use	2002		Unknown	1 glas of wein occasionally
Duodenal ulcer		2000	No	
Hypertension arterial			Unknown	
Polydrug abuse	1999		Unknown	Paracetamol 500 mg and ibuprofene
Alcohol abuse	1991	2002	No	
Intervertebral disc disorder	1999		Unknown	
Smoker			Unknown	25 py; 13 cigarettes per day for about 3 years

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.01.2010 DE-NOVNCHHQ-GXKR2010DE00650

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 13 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with paracetamol (MAH unknown), ibuprofen (MAH unknown) and Rasilez HCT (aliskiren/hydrochlorothiazide). Rasilez was considered as interacting drug. The patient received Pantozol (pantoprazole) concomitantly. The outcome was reported as resolved with sequel. The causality was assessed as possible.

Serious spontaneous report (nosptalisation) or nepatitis cholestatic, already listed in the Core Data Sheet information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 26.01.2010 DE-NOVNCHHQ-GXKR2010DE00650

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 13 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with paracetamol (MAH unknown), ibuprofen (MAH unknown) and Rasilez HCT (aliskiren/hydrochlorothiazide). Rasilez was considered as interacting drug. The patient received Pantozol (pantoprazole) concomitantly. The outcome was reported as resolved with sequel. The causality was assessed as possible.

Follow-up report received on 22 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), Pantozol (pantoprazole), paracetamol (MAH unknown) and Ibuprofen Ratiopharm 500 lysinat (ibuprofen lysinate). In this report, Pantozol was no longer considered as concomitant drug and Rasilez was not classified as interacting drug. The outcome was reported as not resolved. For paracetamol and ibuprofen, the causality was assessed as possible. The causality was assessed as unlikely for Rasilez and Pantozol. No more information was expected. Therefore, the case was closed by the Regional Pharmacovigilance Center.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10020659

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19481004	Age: 61 Year	weight (kg): 68.5	BMI_int: 32.58	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea					Unknown
2 Liver failure	20100119				recovering/resolving
3 Hypertransaminaemia	20100119				recovering/resolving
4 Hypertransaminasemia	20100119	20100127	9 Day		recovering/resolving
5 Deranged liver function tests	20100119	20100127	9 Day		recovering/resolving
6 Weight loss					Unknown
7 Upper abdominal discomfort					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 falithrom phenprocoumon	Oral	3.0 Mg milligram(s)/ Day 245	2008-20100117 -	Atrial fibrillation
2 inegy ezetimibe; simvastatin	Oral	1 Mg milligram(s)/Day 245	2008-20100117 -	Hyperlipidaemia
3 paracetamol paracetamol	Oral	1000 Mg milligram(s)/ Day 245	20091213-20100117 36 Day	Pains in legs

Drugs (concomitant)

4 concor bisoprolol fumarate	Oral	2*7.5 Mg milligram(s)/ Day 245	2008- -	Atrial fibrillation
5 digitoxin digitoxin	Oral	0.07 Mg milligram(s)/ Day 245	2008- -	Atrial fibrillation
6 pantozol pantoprazole sodium	Oral	2*40 Mg milligram(s)/ Day 90	2008- -	Gastric ulcer prophylaxis
7 ramipril ramipril	Oral	5 Mg milligram(s)/ Day 245	2008- -	Hypertension
8 spiriva tiotropium bromide	Respiratory (inhalation)	18 µg microgram(s)/ Day 5	2008- -	COPD

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gastric ulcer prophylaxis			Yes	
Alcohol use			Yes	
Pain in leg			Yes	
Overweight			Yes	
Hypertension arterial			Yes	Beginn unbekannt
COPD			Yes	Beginn unbekannt
Hepatic steatosis			Yes	Beginn unbekannt
Hypertension pulmonary			Yes	Beginn unbekannt
Infarct myocardial	200802	200802	No	mit PTCA + Stentimplantation
Coronary disease	2008		Yes	
Atrial fibrillation	200806		Yes	
Percutaneous transluminal coronary angioplasty	200802	200802	No	
Coronary stent placement	200802	200802	No	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20100119	98	µmol/l	
Aspartate aminotransferase	20090502	1.27	µmol/l*s	
Aspartate aminotransferase	20100119	10.4	µmol/l*s	
Aspartate aminotransferase	20100120	6	µmol/l*s	
Aspartate aminotransferase	20100127	1.13	µmol/l*s	
Alanine aminotransferase	20090502	1.81	µmol/l*s	
Alanine aminotransferase	20091212	2	µmol/l*s	
Alanine aminotransferase	20100119	4.81	µmol/l*s	
Alanine aminotransferase	20100120	3.48	µmol/l*s	
Alanine aminotransferase	20100122	2.63	µmol/l*s	
Alanine aminotransferase	20100127	1.42	µmol/l*s	
Bilirubin total	20100122	14	µmol/l	
Alkaline phosphatase	20091212	1.42	µmol/l*s	
Alkaline phosphatase	20100120	2.23	µmol/l*s	
Gamma GT	20090502	3.06	µmol/l*s	
Gamma GT	20091212	5.38	µmol/l*s	
Gamma GT	20100119	22.5	µmol/l*s	
Gamma GT	20100120	17.74	µmol/l*s	
Gamma GT	20100122	16.68	µmol/l*s	
Gamma GT	20100127	7.45	µmol/l*s	
Glutamate dehydrogenase	20090502	351	µmol/l*s	
Glutamate dehydrogenase	20100119	2763	µmol/l*s	
Glutamate dehydrogenase	20100120	932	µmol/l*s	
Quick's test	20091212	28	therapeutis	
Quick's test	20100117	17	therapeutis	
Quick's test	20100127	99	therapeutis	
INR	20100119	4.2	therapeutis	
Total white blood cell count	20100119	3.8	Gpt/l	
C-reactive protein	20100119	2	mg/l	
Ammonia	20100122	42	µmol/l	
Blood triglycerides	20100119	69.78	mmol/l	
Blood triglycerides	20100122	2.97	mmol/l	

Narratives (nur bei elektronischem Eingang)HELIOS Klinikum Wuppertal, 18.02.2010 DE-PVZIBE-04-10173

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
 Leberwerterhöhung/ Transaminasenerhöhung

KURZZUSAMMENFASSUNG:

Leberwerterhöhung/ Transaminasenerhöhung unter Paracetamol, Falithrom und Inegy

BESCHREIBUNG:

AD: V.a. akutes Leberversagen, a.e. medikamentös-toxisch. Die Patientin wurde aufgrund steigender Leberfermente seit Januar 2010 unter einem V.a. ein Leberversagen eingewiesen. Es bestehen OB-Beschwerden, v.a. nach Mahlzeiten und Tabletteneinnahme, sowie Übelkeit und 3 kg Gewichtsverlust in letzten 4 Wochen. Fieber, Erbrechen, Durchfälle und Ikterus bestehen nicht. Patientin trinkt gewohnheitsmäßig ein Glas Sekt am Tag. In letzter Zeit hat sie vermehrt Paracetamol wegen Schmerzen eingenommen.

THERAPIE:

18.01.2010: Paracetamol abgesetzt, Falithrom abgesetzt, Inegy abgesetzt, Monoembolox zur Antikoagulation, Volumentherapie. 26.01.2010: ASS 100 unter PPI-Prophylaxe.

VERLAUF:

Ambulante Hepatitis-Serologie (A,B,C) war negativ. 19.01.2010 Abdomen-Sono: Leber: abgerundeter Unterrand, Oberfläche glatt. Gallenblase steinfrei, regelrechte Wandarchitektur. Gallenwege nicht erweitert. Pankreas soweit erkennbar unauffällig. Beurteilung: Deutliche Fettleber, Hepatomegalie. Laborverlauf: Leberwerte im Vorfeld 2-3 fach erhöht, deutliche Zunahme von Dezember 2009 zu Januar 2010 auf das bis zu 20 fache. Seit stationärer Aufnahme kontinuierliches Absinken der erhöhten Leberwerte auf Ausgangsniveau (ausgenommen GGT, hier deutlich Rückbildung aber weiterhin Werte oberhalb des Ausgangsniveaus).

ERLÄUTERUNGEN ZUR UAW:

Einweisung wegen akuter starker Verschlechterung der Leberwerte, klinisch nahezu asymptomatisch, zur Abklärung. Kliniker sahen den Zusammenhang a.e. toxisch bei Falithromtherapie und Alkoholkonsum. Bei näherer Betrachtung scheint die verstärkte Einnahme von Paracetamol durch die Patientin ausschlaggebend gewesen zu sein.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol

BETREFFENDE UAW: entgleiste Leberwerte

ERLÄUTERUNG: Fachinformation: Intoxikationsrisiko besteht v.a. bei Personen mit Lebererkrankungen, chronischem Alkoholmissbrauch, UAW: selten: Anstieg Lebertransaminasen. Dosierung lt FI (Paracetamol): Einzeldosis max. 10-15 mg/kgKG [=990], Tagesgesamtdosis 60 mg/kgKG [= 3960/d], bei Leberinsuffizienz muss Dosis vermindert oder Intervall verlängert werden. 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebervorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

MEDIKAMENT: falithrom

BETREFFENDE UAW: entgleiste Leberwerte

ERLÄUTERUNG: 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebervorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

MEDIKAMENT: inegy

BETREFFENDE UAW: entgleiste Leberwerte

ERLÄUTERUNG: Fachinformation: Leberwerterhöhungen beschrieben (1,7% in Studie häufig), Leberversagen beschrieben. 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten

aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebertorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

009507513, 25.02.2010 DE-MERCK-1002DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 61-year-old overweight female patient with pain in leg, atrial fibrillation, coronary artery disease, pulmonary hypertension, chronic obstructive pulmonary disease, arterial hypertension, hepatic steatosis, regular but moderate alcohol consumption (1 glass of sparkling wine per day) and a history of myocardial infarction with percutaneous transluminal coronary angioplasty and coronary stent placement who, in 2008, was placed on therapy with ezetimibe (+) simvastatin, 10-80 mg tablet, 10-80 mg, once a day for the treatment of hyperlipidaemia (duration not reported).

Suspect secondary therapy included phenprocoumon (FALITHROM), 3 mg tablet, 3 mg once a day for the treatment of atrial fibrillation (therapy start in 2008).

Concomitant therapy included bisoprolol fumarate (CONCOR), pantoprazole sodium (PANTOZOL), ramipril, digitoxin and tiotropium bromide (SPIRIVA).

On 02-MAY-2009, laboratory analysis revealed that AST was 1.27 mcmol/l*s, ALT was 1.81 mcmol/l*s, GGT was 3.06 mcmol/l*s and serum glutamate dehydrogenase was 351 mcmol/l*s.

On 12-DEC-2009, AP was 1.42 mcmol/l*s, GGT was 5.38 mcmol/l*s, ALT was 2 mcmol/l*s.

On 13-DEC-2009, therapy with acetaminophen, tablet, 1000 mg taken as needed (1-2 x day) for the treatment of pain in legs.

On an unknown date, ambulatory performed serology for hepatitis A, B and C was negative.

On 18-JAN-2010, the patient was hospitalized with suspected acute liver failure. The patient complained about upper abdominal discomfort (particularly after meals and intake of tablets), nausea and weight loss (3 kg within the last four weeks). There was no fever, vomiting, diarrhea or icterus. The patient reported that she "recently" had taken acetaminophen more often than usual due to leg pain. That same day, therapy with ezetimibe (+) simvastatin, acetaminophen and phenprocoumon was discontinued (last intake on 17-JAN-2010). Therapy with certoparin sodium and fluid replacement therapy were initiated.

On 19-JAN-2010 (reported as SAE onset date), AST was 10.4 mcmol/l*s, ALT was 4.81 mcmol/l*s, GGT was 22.5 mcmol/l*s, glutamate dehydrogenase was 2763 mcmol/l*s, triglyceride test was 69.78 mmol/l. Abdominal ultrasound showed fatty liver, hepatomegaly, a regular pancreas, no cholelithiasis, bile ducts were not dilated.

Serology diagnosis of viral infections was negative (no date/details provided).

On 20-JAN-2010, AST was 6 mcmol/l*s, ALT was 3.48 mcmol/l*s, AP was 2.23 mcmol/l*s, GGT was 17.74 mcmol/l*s and glutamate dehydrogenase was 932 mcmol/l*s.

On 22-JAN-2010, total bilirubin was 14 mcmol/l*s, ALT was 2.63 mcmol/l*s, GGT was 16.68 mcmol/l*s and triglyceride test was 2.97 mmol/l.

On 26-JAN-2010, therapy with acetylsalicylic acid, 100 mg, was initiated.

On 27-JAN-2010, AST was 1.13 mcmol/l*s, ALT was 1.42 mcmol/l*s and GGT was 7.45 mcmol/l*s.

On an unknown date, the patient was discharged from hospital. At that time, liver values had again decreased to the already slightly increased base level with the exception of GGT which had clearly decreased, however, was still distinctly above base level. The patient was reported as recovering.

The reporting regional pharmacovigilance center felt that the acute hepatic decompensation was most likely caused by combination of alimentary predisposition, chronic alcohol consumption and medication. The PRN medication with acetaminophen was considered as the most likely final trigger due to high single dose of 1000 mg, the increased intake of acetaminophen in the past months and the patient's pre-existing liver damage (hepatic steatosis). The regional pharmacovigilance center felt that increased liver values / transaminases were possibly related to therapy with ezetimibe (+) simvastatin and probably related to therapy with falithrom; a drug interaction between phenprocoumon, acetaminophen and ezetimibe (+) simvastatin (documented as interacting drug) was considered as possible, however, drug interaction was not coded as adverse experience by the agency.

Additional information is not expected.

sanofi-aventis, 01.03.2010 DE-SANOFI-AVENTIS-2010SA009983

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10020659) on 22-Feb-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: increase in liver values / increase in transaminases under treatment with paracetamol, phenprocoumon, and simvastatin+ezetimib. The case involves a 61-year-old female patient with a past history of myocardial infarction in Feb. 2008 (percutaneous transluminal coronary angioplasty and stent placement). At the time of event, the patient was suffering from atrial fibrillation, coronary disease, pulmonary hypertension, arterial hypertension, chronic obstructive pulmonary disease, and hepatic steatosis. The following drugs are reported to have been taken concomitantly (not suspected): bisoprolol fumarate, pantoprazole, ramipril, digitoxin, and tiotropium bromide. Simvastatin+ezetimib (start of treatment in 2008, indication: hyperlipidaemia) is reported as "interacting drug". The following drugs are reported as "suspect": paracetamol (name of authorization holder not reported) as needed 1000mg once or twice daily from 13-Dec-2009 to 17-Jan-2010 for pains in legs, and phenprocoumon 1x3 mg/day from 2008 to 17-Jan-2010 for atrial fibrillation.

The patient was admitted to hospital under the suspicion of liver failure, due to increasing liver enzymes since Jan 2010 (onset of event was reported as 19-Jan-2010). The patient complained of upper abdominal discomfort, especially after meals and tablet intake, as well as about nausea and weight loss of three kg within the last four weeks. Fever, vomiting, diarrhoea, and icterus were not present. The patient usually drank one glass of sparkling wine each day. Recently, she had increased the intake of paracetamol due to pain. Her liver values had been increased two- to three-fold before, but from Dec. 2009 to Jan. 2010 they increased to 20 times the upper limit of normal. Hepatitis serology before hospitalisation was negative; abdominal sonography revealed a fatty liver and hepatomegaly. On 18-Jan-2010, paracetamol, phenprocoumon, and simvastatin+ezetimib were withdrawn; the patient instead was treated with certoparin for anticoagulation and received volume substitution. On 26-Jan-2010, treatment with acetylsalicylic acid 100mg was started under proton-pump inhibitor prophylaxis. Outcome was reported as "recovering/resolving" on 27-Jan-2010.

Causality determined by PV centre:

"probable" for paracetamol

"possible" for phenprocoumon and simvastatin+ezetimib ("interacting")

PV centre's comment on causality: A risk of intoxication with paracetamol is present especially in patients with liver diseases or chronic alcohol misuse. Increase in liver transaminases is known as rare adverse effect of paracetamol. According to the summary of product information for paracetamol, single doses should not exceed 10 to 15 mg/kg body weight ($\Rightarrow$ 990), total daily dose should not exceed 60 mg/kg body weight (= 3960/day), in patients with liver insufficiency, dose has to be decreased or the administration interval has to be increased. After withdrawal of the liver-stressing drugs (paracetamol, simvastatin+ezetimib) and phenprocoumon, and under pausing of alcohol consumption, the patient's liver values decreased spontaneously, down to pre-existing slightly pathological levels. A viral infection could be excluded. As alternative explanation, an exacerbation in the course of nutritive or alcoholic consumption, was discussed. From a clinical-pharmacological point of view, the acute hepatic decompensation (clinically nearly asymptomatic) was caused most likely by the interaction of alimentary predisposition, chronic alcohol use, and the medication. Final trigger was probably the acute medication with paracetamol, that showed the tightest chronologic connection, taking into account the single dose of 1000mg in a patient with hepatic pre-damage, and increased intake during the last period before onset of events. The hospital physicians regarded phenprocoumon as the most suspect drug, an opinion that was not shared by the PV centre. A possible interaction with simvastatin+ezetimib (a product, that can often lead to increase in liver values, and whose intake is not recommended in connection with chronic alcohol consumption), was also discussed by the PV centre.

HELIOS Klinikum Wuppertal, 07.04.2010 DE-PVZIBE-04-10173

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberwerterhöhung/ Transaminasenerhöhung

KURZZUSAMMENFASSUNG:

Leberwerterhöhung/ Transaminasenerhöhung unter Paracetamol, Falithrom und Inegy

BESCHREIBUNG:

AD: V.a. akutes Leberversagen, a.e. medikamentös-toxisch. Die Patientin wurde aufgrund steigender Leberenzyme seit Januar 2010 unter einem V.a. ein Leberversagen eingewiesen. Es bestehen OB-Beschwerden, v.a. nach Mahlzeiten und Tabletteneinnahme, sowie Übelkeit und 3 kg Gewichtsverlust in letzten 4 Wochen. Fieber, Erbrechen, Durchfälle und Ikterus bestehen nicht. Patientin trinkt gewohnheitsmäßig ein Glas Sekt am Tag. In letzter Zeit hat sie vermehrt Paracetamol wegen Schmerzen eingenommen.

THERAPIE:

18.01.2010: Paracetamol abgesetzt, Falithrom abgesetzt, Inegy abgesetzt, Monoembolox zur Antikoagulation, Volumentherapie. 26.01.2010: ASS 100 unter PPI-Prophylaxe.

VERLAUF:

Ambulante Hepatitis-Serologie (A,B,C) war negativ. 19.01.2010 Abdomen-Sono: Leber: abgerundeter Unterrand, Oberfläche glatt. Gallenblase steinfrei, regelrechte Wandarchitektur. Gallenwege nicht erweitert. Pankreas soweit erkennbar unauffällig. Beurteilung: Deutliche Fettleber, Hepatomegalie. Laborverlauf: Leberwerte im Vorfeld 2-3 fach erhöht, deutliche Zunahme von Dezember 2009 zu Januar 2010 auf das bis zu 20 fache. Seit stationärer Aufnahme kontinuierliches Absinken der erhöhten Leberwerte auf Ausgangsniveau (ausgenommen GGT, hier deutlich Rückbildung aber weiterhin Werte oberhalb des Ausgangsniveaus).

ERLÄUTERUNGEN ZUR UAW:

Einweisung wegen akuter starker Verschlechterung der Leberwerte, klinisch nahezu asymptomatisch, zur Abklärung. Kliniker sahen den Zusammenhang a.e. toxisch bei Falithromtherapie und Alkoholkonsum. Bei näherer Betrachtung scheint die verstärkte Einnahme von Paracetamol durch die Patientin ausschlaggebend gewesen zu sein.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Hypertransaminasaemie

ERLÄUTERUNG: Fachinformation: Intoxikationsrisiko besteht v.a. bei Personen mit Lebererkrankungen, chronischem Alkoholmissbrauch, UAW: selten: Anstieg Lebertransaminasen. Dosierung lt FI (Paracetamol): Einzeldosis max. 10-15 mg/kgKG [=990], Tagesgesamtdosis 60 mg/kgKG [= 3960/d], bei Leberinsuffizienz muss Dosis vermindert oder Intervall verlängert werden. 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebervorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

MEDIKAMENT: falithrom

BETREFFENDE UAW: Hypertransaminasaemie

ERLÄUTERUNG: 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebervorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

MEDIKAMENT: inegy

BETREFFENDE UAW: Hypertransaminasaemie

ERLÄUTERUNG: Fachinformation: Leberwerterhöhungen beschrieben (1,7% in Studie häufig), Leberversagen beschrieben. 61-jährige, übergewichtige Patientin mit regelmäßigem, nicht - übermäßigem Alkoholkonsum (1 Glas Sekt/d) kommt wegen akut stark verschlechterter Leberwerte zur stationären Abklärung und Behandlung. Sofort Pausieren der leberbelastenden Medikamente (Paracetamol, Inegy) und Falithrom bei V.a. medikamentös-toxische Genese. Es ist davon auszugehen, dass auch der Alkoholgenuß stationär ausblieb. Darunter fielen die erhöhten Werte spontan ab, zur Entlassung auf das leicht-erhöhte Ausgangsniveau. Virusserologie konnte Virusinfekt ausschließen, auch fehlende Infektzeichen lassen Virusinfekt unwahrscheinlich erscheinen. Als Alternativursache wird eine Exacerbation im Rahmen nutritivem oder alkoholischem Genuß berücksichtigt. Aus klinisch-pharmakologischer Sicht kam die akute hepatische Dekompensation (klinisch nahezu asymptomatisch) am ehesten aufgrund des Zusammenspiels der alimentären Vorbelastung, des chronischen Alkoholgenußes und der Medikation zustande. Endgültiger Auslöser scheint hier sehr wahrscheinlich die Bedarfsmedikation mit Paracetamol zu sein, die bei Lebervorschädigung mit 1000 mg pro Einzeldosis und zuletzt von Patientin verstärkt eingesetzt, den engsten chronologischen Zusammenhang zeigt und deshalb die höchste Bewertung erfährt. Die Kliniker sahen den Zusammenhang (lt. AB) am stärksten mit Falithrom, was uns allenfalls "möglich" scheint. Eventuell fand sich eine Interaktion mit Inegy, unter dem es häufig zu Leberwerterhöhungen kommen kann, weshalb in der Fachinformation für Inegy vor Einsatz bei chronischem Alkoholgenuß gewarnt wird.

009507513, 09.04.2010 DE-MERCK-1002DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 61-year-old overweight female patient with pain in leg, atrial fibrillation, coronary artery disease, pulmonary hypertension, chronic obstructive pulmonary disease, arterial hypertension, hepatic steatosis, regular but moderate alcohol consumption (1 glass of sparkling wine per day) and a history of myocardial infarction with percutaneous transluminal coronary angioplasty and coronary stent placement who, in 2008, was placed on therapy with ezetimibe (+) simvastatin, 10-80 mg tablet, 10-80 mg, once a day for the treatment of hyperlipidaemia (duration not reported).

Suspect secondary therapy included phenprocoumon (FALITHROM), 3 mg tablet, 3 mg once a day for the treatment of atrial fibrillation (therapy start in 2008).

Concomitant therapy included bisoprolol fumarate (CONCOR), pantoprazole sodium (PANTOZOL), ramipril, digitoxin and tiotropium bromide (SPIRIVA).

On 02-MAY-2009, laboratory analysis revealed that AST was 1.27 mcmol/l*s, ALT was 1.81 mcmol/l*s, GGT was 3.06 mcmol/l*s and serum glutamate dehydrogenase was 351 mcmol/l*s.

On 12-DEC-2009, AP was 1.42 mcmol/l*s, GGT was 5.38 mcmol/l*s, ALT was 2 mcmol/l*s.

On 13-DEC-2009, therapy with acetaminophen, tablet, 1000 mg taken as needed (1-2 x day) for the treatment of pain in legs.

On an unknown date, ambulatory performed serology for hepatitis A, B and C was negative.

On 18-JAN-2010, the patient was hospitalized with suspected acute liver failure. The patient complained about upper abdominal discomfort (particularly after meals and intake of tablets), nausea and weight loss (3 kg within the last four weeks). There was no fever, vomiting, diarrhea or icterus. The patient reported that she "recently" had taken acetaminophen more often than usual due to leg pain. That same day, therapy with ezetimibe (+) simvastatin, acetaminophen and phenprocoumon was discontinued (last intake on 17-JAN-2010). Therapy with certoparin sodium and fluid replacement therapy were initiated.

On 19-JAN-2010 (reported as SAE onset date), AST was 10.4 mcmol/l*s, ALT was 4.81 mcmol/l*s, GGT was 22.5 mcmol/l*s, glutamate dehydrogenase was 2763 mcmol/l*s, triglyceride test was 69.78 mmol/l. Abdominal ultrasound showed fatty liver, hepatomegaly, a regular pancreas, no cholelithiasis, bile ducts were not dilated.

Serology diagnosis of viral infections was negative (no date/details provided).

On 20-JAN-2010, AST was 6 mcmol/l*s, ALT was 3.48 mcmol/l*s, AP was 2.23 mcmol/l*s, GGT was 17.74 mcmol/l*s and glutamate dehydrogenase was 932 mcmol/l*s.

On 22-JAN-2010, total bilirubin was 14 mcmol/l*s, ALT was 2.63 mcmol/l*s, GGT was 16.68 mcmol/l*s and triglyceride test was 2.97 mmol/l.

On 26-JAN-2010, therapy with acetylsalicylic acid, 100 mg, was initiated.

On 27-JAN-2010, AST was 1.13 mcmol/l*s, ALT was 1.42 mcmol/l*s and GGT was 7.45 mcmol/l*s.

On an unknown date, the patient was discharged from hospital. At that time, liver values had again decreased to the already slightly increased base level with the exception of GGT which had clearly decreased, however, was still distinctly above base level. The patient was reported as recovering.

The reporting regional pharmacovigilance center felt that the acute hepatic decompensation was most likely caused by combination of alimentary predisposition, chronic alcohol consumption and medication. The PRN medication with acetaminophen was considered as the most likely final trigger due to high single dose of 1000 mg, the increased intake of acetaminophen in the past months and the patient's pre-existing liver damage (hepatic steatosis). The regional pharmacovigilance center felt that increased liver values / transaminases were possibly related to therapy with ezetimibe (+) simvastatin and probably related to therapy with falithrom; a drug interaction between phenprocoumon, acetaminophen and ezetimibe (+) simvastatin (documented as interacting drug) was considered as possible, however, drug interaction was not coded as adverse experience by the agency.

Additional information is not expected.

sanofi-aventis, 16.04.2010 DE-SANOFI-AVENTIS-2010SA009983

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10020659) on 22-Feb-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: increase in liver values / increase in transaminases under treatment with paracetamol, phenprocoumon, and simvastatin+ezetimibe. The case involves a 61-year-old female patient with a past history of myocardial infarction in Feb. 2008 (percutaneous transluminal coronary angioplasty and stent placement). At the time of event, the patient was suffering from atrial fibrillation, coronary disease, pulmonary hypertension, arterial hypertension, chronic obstructive pulmonary disease, and hepatic steatosis. The following drugs are reported to have been taken concomitantly (not suspected): bisoprolol fumarate, pantoprazole, ramipril, digitoxin, and tiotropium bromide. Simvastatin+ezetimibe (start of treatment in 2008, indication: hyperlipidaemia) is reported as "interacting drug". The following drugs are reported as "suspect": paracetamol (name of authorization holder not reported) as needed 1000mg once or twice daily from 13-Dec-2009 to 17-Jan-2010 for pains in legs, and phenprocoumon 1x3 mg/day from 2008 to 17-Jan-2010 for atrial fibrillation.

The patient was admitted to hospital under the suspicion of liver failure, due to increasing liver enzymes since Jan 2010 (onset of event was reported as 19-Jan-2010). The patient complained of upper abdominal discomfort, especially after meals and tablet intake, as well as about nausea and weight loss of three kg within the last four weeks. Fever, vomiting, diarrhoea, and icterus were not present. The patient usually drank one glass of sparkling

wine each day. Recently, she had increased the intake of paracetamol due to pain. Her liver values had been increased two- to three-fold before, but from Dec. 2009 to Jan. 2010 they increased to 20 times the upper limit of normal. Hepatitis serology before hospitalisation was negative; abdominal sonography revealed a fatty liver and hepatomegaly. On 18-Jan-2010, paracetamol, phenprocoumon, and simvastatin+ezetimib were withdrawn; the patient instead was treated with certoparin for anticoagulation and received volume substitution. On 26-Jan-2010, treatment with acetylsalicylic acid 100mg was started under proton-pump inhibitor prophylaxis. Outcome was reported as "recovering/resolving" on 27-Jan-2010.

Causality determined by PV centre:

"probable" for paracetamol

"possible" for phenprocoumon and simvastatin+ezetimib ("interacting")

PV centre's comment on causality: A risk of intoxication with paracetamol is present especially in patients with liver diseases or chronic alcohol misuse. Increase in liver transaminases is known as rare adverse effect of paracetamol. According to the summary of product information for paracetamol, single doses should not exceed 10 to 15 mg/kg body weight ($\Rightarrow$ 990), total daily dose should not exceed 60 mg/kg body weight (= 3960/day), in patients with liver insufficiency, dose has to be decreased or the administration interval has to be increased. After withdrawal of the liver-stressing drugs (paracetamol, simvastatin+ezetimib) and phenprocoumon, and under pausing of alcohol consumption, the patient's liver values decreased spontaneously, down to pre-existing slightly pathological levels. A viral infection could be excluded. As alternative explanation, an exacerbation in the course of nutritive or alcoholic consumption, was discussed. From a clinical-pharmacological point of view, the acute hepatic decompensation (clinically nearly asymptomatic) was caused most likely by the interaction of alimentary predisposition, chronic alcohol use, and the medication. Final trigger was probably the acute medication with paracetamol, that showed the tightest chronologic connection, taking into account the single dose of 1000mg in a patient with hepatic pre-damage, and increased intake during the last period before onset of events. The hospital physicians regarded phenprocoumon as the most suspect drug, an opinion that was not shared by the PV centre. A possible interaction with simvastatin+ezetimib (a product, that can often lead to increase in liver values, and whose intake is not recommended in connection with chronic alcohol consumption), was also discussed by the PV centre.

Additional information extracted from the website of the German health authority on 08-Apr-2010: coding of event was changed from "deranged liver function tests" to "hypertransaminasemia". Assessment remained unchanged.

sanofi-aventis, 26.05.2010 DE-SANOFI-AVENTIS-2010SA009983

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10020659) on 22-Feb-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: increase in liver values / increase in transaminases under treatment with paracetamol, phenprocoumon, and simvastatin+ezetimib. The case involves a 61-year-old female patient with a past history of myocardial infarction in Feb. 2008 (percutaneous transluminal coronary angioplasty and stent placement). At the time of event, the patient was suffering from atrial fibrillation, coronary disease, pulmonary hypertension, arterial hypertension, chronic obstructive pulmonary disease, and hepatic steatosis. The following drugs are reported to have been taken concomitantly (not suspected): bisoprolol fumarate, pantoprazole, ramipril, digitoxin, and tiotropium bromide. Simvastatin+ezetimib (start of treatment in 2008, indication: hyperlipidaemia) is reported as "interacting drug". The following drugs are reported as "suspect": paracetamol (name of authorization holder not reported) as needed 1000mg once or twice daily from 13-Dec-2009 to 17-Jan-2010 for pains in legs, and phenprocoumon 1x3 mg/day from 2008 to 17-Jan-2010 for atrial fibrillation.

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Additional information extracted from the website of the German health authority on 08-Apr-2010: coding of event was changed from "deranged liver function tests" to "hypertransaminasemia". Assessment remained unchanged.

Significant correction on 21-May-2010: After internal review the MedDRA code "hypertransaminasaemia" was changed to "liver failure".

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10025212

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19480823	Age: 61 Year	weight (kg): 118	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20091122				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-Saar, 600 mg, Filmtabl., MIP clindamycin	Oral	1800 Mg milligram(s)/Day 82	20091019-20091025 7 Day	Tooth extraction
2 Ibu ratiopharm, 800 mg, Filmtabl ibuprofen	Oral	400 Mg milligram(s)/Day 82	2004-20091020 -	Pain in spine
3 Paracetamol, 500 mg, Tabl, Ratiopharm paracetamol	Oral	2*500 Mg milligram(s)/We 245	2004-20091208 -	Pain in spine

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertensive heart disease			k.A.	
Hypertension arterial	199812		k.A.	arterielle Hypertonie mit rezidivierenden Entgleisungen
Atrial fibrillation	200902		k.A.	absolute Arrhythmie bei Vorhofflimmern
Type 2 diabetes mellitus	200411		k.A.	
Adipositas per magna	197412		k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20091208	599	U/l	
Alanine aminotransferase increased	20091209	1046	U/l	
Alanine aminotransferase increased	20091211	1584	U/l	
Alanine aminotransferase increased	20091222	943	U/l	
Alanine aminotransferase increased	20100104	79	U/l	
Aspartate aminotransferase increased	20091208	525	U/l	
Aspartate aminotransferase increased	20091210	1087	U/l	
Aspartate aminotransferase increased	20091211	940	U/l	
Aspartate aminotransferase increased	20091222	364	U/l	
Aspartate aminotransferase normal	20100104	28	U/l	
Bilirubin normal	20091209	0.99	mg/dl	
Bilirubin increased	20091210	1.8	mg/dl	
Bilirubin normal	20091221	0.96	mg/dl	
Alkaline phosphatase normal	20091208	81	U/l	
Alkaline phosphatase normal	20091214	111	U/l	

Narratives (nur bei elektronischem Eingang)PVZ-FAKOS, Klinische Pharmakologie, 25.02.2010 DE-FAKOS-170303

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:PVZ-FAKOS, Klinische Pharmakologie, 25.03.2010 DE-FAKOS-170303

Sender diagnosis: not reported

Narrative

Am 09.12.2009 erfolgte die stationäre Aufnahme des Patienten wegen seit 2 Wochen bestehender Übelkeit mit Diarrhoen sowie wegen der neu aufgetretenen, starken Erhöhung der Transaminasen. Vorausgegangen war eine Einnahme von Clindamycin in der Zeit vom 19.10. - 25.10.2009 nach Zahnextraktion.

Bei Aufnahme waren die Transaminasen stark erhöht (ALAT 1046 U/l (Normbereich: <41 U/l); ASAT 731 U/l (Normbereich: <37 U/l); außerdem: GGT (Gamma-Glutamyltransferase) am 08.12.2009: 85 U/l (Normbereich: 8-61 U/l); AP (Alkalische Phosphatase) am 08.12.2009: 81 U/l (Normbereich: 40-260 U/l), Gesamtbilirubin 0,99 mg/dl (Normbereich: <1,2 mg/dl). Es kam zu einem weiteren Anstieg in den ersten Tagen der stationären Behandlung (Maximum ALAT am 11.12.2009: 1584 U/l, Maximum ASAT am 10.12.2009: 1087 U/l; geringer Anstieg des Bilirubins mit 1,8 mg/dl (10.12.2009). Im Verlauf rasche Rückbildungstendenz der erhöhten Transaminasen (am 04.01.2010 ALAT 79 U/l; ASAT 28 U/l; GGT 85 U/l) bei klinischer Besserung. Eine umfangreiche Erregerdiagnostik (Cytomegalie, EBV (Epstein-Barr-Virus), Hepatitis A, B und C) war unauffällig. Leber, Gallenblase, Gallengänge und Pankreas waren sowohl sonographisch als auch computertomographisch unauffällig. Die Leberpunktion am 11.12.2009 ergab frische Nekrosen und epitheloidzellige Granulome - Bild wie bei einer toxischen Leberschädigung. Auch laborchemisch keine Hinweise auf eine Autoimmunhepatitis (Auto-Antikörper ANA, AMA, LKM, SLA-LP, SMA negativ). Am 21.12.2009 wurde der Patient aus dem Krankenhaus entlassen.

In der Arzneimittelanamnese auffällig ist eine Selbstmedikation mit verschiedenen Analgetika (Paracetamol, ASS und Ibuprofen) seit mehreren Jahren bei chronischen Wirbelsäulenschmerzen. ASS wurde vom Patienten nach Entlassung aus der stationären Behandlung wieder eingenommen und gut vertragen.

Eine medikamenteninduzierte Genese der Hepatitis ist wahrscheinlich.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Clinda-Saar Filmtabl., Einnahmezeitraum: 19.-25.10.2009,

Paracetamol ratiopharm Tabl., Einnahmezeitraum: 2004-08.12.2009 und

Ibu-ratiopharm Filmtabl., Einnahmezeitraum: 2004-ca. 20.10.2009.

Möglicherweise spielen mehrere Arzneistoffe gleichzeitig eine Rolle in der Entstehung der Hepatitis, da in der Histologie neben Granulomen auch frische Nekrosen gesehen wurden. Die Arzneimittelkausalität von Clindamycin und Paracetamol wird mit "wahrscheinlich", die von Ibuprofen mit "möglich" bewertet. Die frischen Nekrosen könnten im Zusammenhang mit der Paracetamoleinnahme stehen.

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10025902

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19420806	Age: 67 Year	weight (kg): 78	BMI_int: 26.37	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20100105	20100118	14 Day	9 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clarithromycin clarithromycin	Oral	500 Mg milligram(s)/ Day 82	20091227-20100102 7 Day	Bronchitis
2 paracetamol paracetamol	Oral	1000 Mg milligram(s)/ Day 245	20091227-20100102 7 Day	Bronchitis

Drugs (concomitant)

3 aarane cromoglicate sodium; reproterol hydrochloride	Respiratory (inhalation)	1 Df dosage form/ Day 204	-	COPD
4 karvezide hydrochlorothiazide; irbesartan	Oral	50 Mg milligram(s)/ Day 82	2004-	Hypertension
5 nifurantin nitrofurantoin	Oral	100 Mg milligram(s)/ Day 9	2004-20100105	Bladder inflammation
6 tamsulosin tamsulosin	Oral	0.4 Mg milligram(s)/ Day 208	2009-	BPH
7 viani fluticasone propionate; salmeterol xinafoate	Respiratory (inhalation)	1 Df dosage form/ Day 204	-	COPD

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bronchitis	200912	201001	No	
Chronic cystitis	2004		Unknown	
Drug allergy			Yes	Beginn unbekannt, Salicyte
COPD			Yes	Beginn unbekannt
BPH	2009		Yes	
Hypertension arterial	2009		Yes	
Hip replacement	200802	200802	No	
Hepatitis B	1982	1982	No	
Bladder inflammation			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20100105	85	µmol/l	
Aspartate aminotransferase	20100105	12.52	µmol/l*s	
Aspartate aminotransferase	20100106	10.3	µmol/l*s	
Aspartate aminotransferase	20100111	7.44	µmol/l*s	
Aspartate aminotransferase	20100112	5.1	µmol/l*s	

Aspartate aminotransferase	20100115	2.21	µmol/l*s
Aspartate aminotransferase	20100118	0.87	µmol/l*s
Alanine aminotransferase	20090224	2.15	µmol/l*s
Alanine aminotransferase	20100105	23.55	µmol/l*s
Alanine aminotransferase	20100106	19.49	µmol/l*s
Alanine aminotransferase	20100111	15.1	µmol/l*s
Alanine aminotransferase	20100112	12.23	µmol/l*s
Alanine aminotransferase	20100115	6.59	µmol/l*s
Alanine aminotransferase	20100118	3.44	µmol/l*s
Bilirubin total	20100105	50	µmol/l
Bilirubin total	20100106	33	µmol/l
Bilirubin direct	20010105	39.7	µmol/l
Bilirubin direct	20100106	25	µmol/l
Alkaline phosphatase	20100106	11.37	µmol/l*s
Alkaline phosphatase	20100115	9.03	µmol/l*s
Gamma GT	20090224	5.85	µmol/l*s
Gamma GT	20100105	11.3	µmol/l*s
Gamma GT	20100106	9.11	µmol/l*s
Gamma GT	20100111	6.91	µmol/l*s
Gamma GT	20100112	6.04	µmol/l*s
Gamma GT	20100115	4.75	µmol/l*s
Gamma GT	20100118	3.89	µmol/l*s
Glutamate dehydrogenase	20100105	1660	µmol/l*s
Quick's test	20100112	65	therapeutis
Quick's test	20100115	75	therapeutis
Total white blood cell count	20100105	11.7	Gpt/l
C-reactive protein	20090224	38.5	mg/l
C-reactive protein	20100105	41.8	mg/l
C-reactive protein	20100106	34	mg/l
C-reactive protein	20100112	9	mg/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 26.02.2010 DE-PVZIBE-04-10178

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberversagen

KURZZUSAMMENFASSUNG:

Leberversagen unter Paracetamol und Clarithromycin

BESCHREIBUNG:

Patient (ehemaliger ärztlicher Kollege) hustet bereits seit einem halben Jahr, in letzter Zeit mit Auswurf, behandelt sich selbst deswegen mit Clarithromycin 500mg 1xd und Paracetamol 1000mg 1xd über 7 Tage. Keine Besserung der respiratorischen Situation, zunehmende Belastungsdyspnoe führt zum Aufsuchen des Hausarztes am 04.01.2010. Am 05.01.2010 hausärztliche Einweisung des Patienten wegen stark erhöhten Leberwerten. In Zentraler Notaufnahme heller Stuhlgang, stark erhöhte Leberwerte, stationäre Aufnahme zur Überwachung.

THERAPIE:

Zentrale Notaufnahme: ACC 600 mg als KI. Ab 06.01.2010 Rocephin 2g 1xd bis 16.01.2010. Am 08.01.2010 Tazobac 4,5 g i.v., Therapie wurde abgebrochen wegen Hitzewallungen und Kribbeln in Händen und Füßen. Ab 12.01.2010 Sinupret forte 1xd, Bifiteral 20ml 1xd. Ab 16.01.2010 Urbason 20 mg 1xd (für 4 Wochen).

VERLAUF:

Zentrale Notaufnahme 05.01.2010 gelber Stuhlgang, Sono unauffällig. Am 06.01.2010 gelblicher Stuhlgang, 07.01. braun. 07.01.2010 Mycoplasmen negativ, Legionellen negativ, Chlamydien (trachomatis+pneumoniae)negativ. 08.01.2010 Röntgen-Thorax: streifige Verdichtungen in Oberfeldern -> interstitielle Infiltrate bei atypischer Pneumonie. 11.01.2010 starker Nachtschweiß. 12.01.2010 Serologie: HIV negativ, EBV negativ, Hepatitisserologie: kein Anhalt für akute Infektionen. 12.01.2010 keine Luftnot mehr, Rückläufige Leberwerte und Entzündungswerte.

ERLÄUTERUNGEN ZUR UAW:

Patient sucht ambulant Hausarzt auf, da bronchialer Infekt trotz Therapie nicht besser wird. Einweisung erfolgt, nachdem Ergebnisse der Blutentnahme vorhanden sind, wegen drastisch erhöhten Leberwerten. Stationäre Aufnahme bei V.a. Leberversagen (Hauptdiagnose). V.a. BOOP ist Nebendiagnose.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Leberversagen

ERLÄUTERUNG: Nach Therapie mit Clarithromycin und Paracetamol kam es zu einem drastischen Leberfermentanstieg, der die stationäre Aufnahme nötig machte. Während des knapp zweiwöchigen stationären Aufenthalts waren die Leberwerte spontan rückläufig. Die Kombination aus zwei leberbelastenden Medikamenten bei vorbestehender leichter Leberinsuffizienz (Vorwerte erhöht) scheint hier Ausschlag für die hepatische Dekompensation gewesen zu sein. Für andere Ursachen fand sich kein Anhalt, können aber dennoch nicht gänzlich ausgeschlossen werden.

MEDIKAMENT: clarithromycin

BETREFFENDE UAW: Leberversagen

ERLÄUTERUNG: Fachinformation: häufig hep. Dysfunktion, Hepatitis, Cholestase

sanofi-aventis, 09.03.2010 DE-SANOFI-AVENTIS-2010SA012202

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10025902) on 02-Mar-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This case involves a 67-year-old male (retired physician), whose medical history included hepatitis B in 1982 and hip replacement in February 2008.

The following drugs are known to have been taken by the patient: fluticasone propionate/salmeterol xinafoate and cromoglicate sodium for COPD, hydrochlorothiazide/irbesartan for arterial hypertension, nitrofurantoin for bladder inflammation as well as tamsulosin hydrochloride for benign prostatic hyperplasia (BPH). Furthermore the patient suffered from chronic cystitis and drug allergy against salicyte.

The patient suffered from cough for a half year, which worsened to productive cough. Therefore he performed a self medication with clarithromycin 500 mg daily and paracetamol 1000 mg daily from 27-Dec-2009 to 02-Jan-2010.

Due the situation had not improved the patient visited his treating physician on 04-Jan-2010. On the next day he was hospitalised due to severe increased liver values.

Corrective treatment included acetylcysteine 600 mg (duration of therapy was not mentioned) and ceftriaxone sodium 2 g from 06-Jan-2010 to 16-Jan-2010. On 08-Jan-2010 intravenous piperacillin sodium/tazobactam sodium 4.5 g was administered and stopped due to hot flushes and prickling in the hands and feet on the same day. Furthermore ascorbic acid/gentiana lutea root/primula spp. flower/rumex acetosa/sambucus nigra flower/verbena officinalis herb and lactulose 20 ml daily were given. Methylprednisolone 20 mg daily was started on 16-Jan-2010 and continued for four weeks.

Subsequently the tests for mycoplasma, legionella and chlamydia (trachomatis+pneumoniae) were negative on 07-Jan-2010. On the next day the thoracic x-ray revealed interstitial infiltration (sign for atypical pneumonia). On 11.01.2010 the patient presented severe night sweat. On 12.01.2010 the serology resulted in HIV negative and EBV negative. The hepatitis serology did not find any signs for acute infection; no dyspnoea. The liver values and inflammation parameters were decreased.

The patient was considered as recovered on 18-Jan-2010. The reporter specified bronchiolitis obliterans organizing pneumonia (BOOP) as secondary diagnosis.

Reporter causality: probable for paracetamol and clarithromycin

HELIOS Klinikum Wuppertal, 07.04.2010 DE-PVZIBE-04-10178

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberversagen

KURZZUSAMMENFASSUNG:

Leberversagen unter Paracetamol und Clarithromycin

BESCHREIBUNG:

Patient (ehemaliger ärztlicher Kollege) hustet bereits seit einem halben Jahr, in letzter Zeit mit Auswurf, behandelt sich selbst deswegen mit Clarithromycin 500mg 1xd und Paracetamol 1000mg 1xd über 7 Tage. Keine Besserung der respiratorischen Situation, zunehmende Belastungsdyspnoe führt zum Aufsuchen des Hausarztes am 04.01.2010. Am 05.01.2010 hausärztliche Einweisung des Patienten wegen stark erhöhten Leberwerten. In Zentraler Notaufnahme heller Stuhlgang, stark erhöhte Leberwerte, stationäre Aufnahme zur Überwachung.

THERAPIE:

Zentrale Notaufnahme: ACC 600 mg als KI. Ab 06.01.2010 Rocephin 2g 1xd bis 16.01.2010. Am 08.01.2010 Tazobac 4,5 g i.v., Therapie wurde abgebrochen wegen Hitzewallungen und Kribbeln in Händen und Füßen. Ab 12.01.2010 Sinupret forte 1xd, Bifiteral 20ml 1xd. Ab 16.01.2010 Urbason 20 mg 1xd (für 4 Wochen).

VERLAUF:

Zentrale Notaufnahme 05.01.2010 gelber Stuhlgang, Sono unauffällig. Am 06.01.2010 gelblicher Stuhlgang, 07.01. braun. 07.01.2010 Mycoplasmen negativ, Legionellen negativ, Chlamydien (trachomatis+pneumoniae)negativ. 08.01.2010 Röntgen-Thorax: streifige Verdichtungen in Oberfeldern -> interstitielle Infiltrate bei atypischer Pneumonie. 11.01.2010 starker Nachtschweiß. 12.01.2010 Serologie: HIV negativ, EBV negativ, Hepatitisserologie: kein Anhalt für akute Infektionen. 12.01.2010 keine Luftnot mehr, Rückläufige Leberwerte und Entzündungswerte.

ERLÄUTERUNGEN ZUR UAW:

Patient sucht ambulant Hausarzt auf, da bronchialer Infekt trotz Therapie nicht besser wird. Einweisung erfolgt, nachdem Ergebnisse der Blutentnahme vorhanden sind, wegen drastisch erhöhten Leberwerten. Stationäre Aufnahme bei V.a. Leberversagen (Hauptdiagnose). V.a. BOOP ist Nebendiagnose.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Leberversagen

ERLÄUTERUNG: Nach Therapie mit Clarithromycin und Paracetamol kam es zu einem drastischen Leberfermentanstieg, der die stationäre Aufnahme nötig machte. Während des knapp zweiwöchigen stationären Aufenthalts waren die Leberwerte spontan rückläufig. Die Kombination aus zwei leberbelastenden Medikamenten bei vorbestehender leichter Leberinsuffizienz (Vorwerte erhöht) scheint hier Ausschlag für die hepatische Dekompensation gewesen zu sein. Für andere Ursachen fand sich kein Anhalt, können aber dennoch nicht gänzlich ausgeschlossen werden.

MEDIKAMENT: clarithromycin

BETREFFENDE UAW: Leberversagen

ERLÄUTERUNG: Fachinformation: häufig hep. Dysfunktion, Hepatitis, Cholestase

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10027654

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19581025	Age: 51 Year	weight (kg): 83	BMI_int: 30.86	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20100121			31 Day	recovering/resolving
2 Hepatitis acute	20100121				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Citalopram, 10 mg, Filmtabl, 1A-Pharma	Oral	10 Mg milligram(s)/Day	20091123-20091222	Recurrent depressive disorder
citalopram		82	30 Day	
2 Paracetamol, 500 mg, Tabl, Ratiopharm	Oral	500 Mg milligram(s)/	2006-	Head pain
paracetamol		245	-	
3 Remergil Soltab, 15/30 mg, Schmelztabl, Esse	Oral	/Day	20091222-20100120	Recurrent depressive disorder
mirtazapine		171	30 Day	

Drugs (concomitant)

4 Trevilor retard, 75 mg, Retardkps, Wyeth	Oral	/	20091222-20100120
venlafaxine		208	30 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Recurrent depressive disorder			Unknown	
Hypertensive heart disease			Unknown	
Hypertension arterial			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20091223	61	U/l	
Alanine aminotransferase increased	20100121	168	U/l	
Alanine aminotransferase increased	20100122	122	U/l	
Alanine aminotransferase increased	20100203	85	U/l	
Aspartate aminotransferase normal	20100122	32	U/l	
Aspartate aminotransferase increased	20100126	61	U/l	
Aspartate aminotransferase increased	20100203	39	U/l	
Bilirubin normal	20100122	0.7	mg/dl	
Bilirubin normal	20100203	0.8	mg/dl	
Alkaline phosphatase normal	20091223	101	U/l	
Alkaline phosphatase increased	20100201	137	U/l	
Alanine aminotransferase increased	20100218	38	U/l	
Alkaline phosphatase normal	20100218	98	U/l	
Aspartate aminotransferase normal	20100218	26	U/l	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 03.03.2010 DE-FAKOS-170314

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 13.04.2010 DE-FAKOS-170314

Sender diagnosis: not reported

Narrative

Am 22.12.2009 erfolgte die stationäre Aufnahme der Patientin in eine psychiatrische Abteilung wegen einer depressiven Episode. Vorausgegangen war nach Angaben der Patientin eine chronische Paracetamoleinnahme seit mehreren Jahren wegen Kopfschmerzen; seit 10/2009 wurden deutlich höhere Dosen eingenommen (ca. 3-4 Tbl./die Paracetamol-ratiopharm 500 mg). Am 23.11.2009 wurde ambulant mit der Behandlung mit Citalopram 1A-Pharma 10 mg/die begonnen. Da keine Besserung der depressiven Symptomatik unter dieser Medikation eintrat, wurde die Patientin stationär aufgenommen, das Citalopram abgesetzt und die antidepressive Medikation umgestellt. Während der stationären Behandlung kam es zu einem deutlichen Anstieg (am 21.01.2010: ALAT 168 U/l (Normbereich: <36 U/l); außerdem Gamma-GT 565 U/l (Normbereich: <39 U/l)) der bereits bei Aufnahme gering erhöhten Transaminasen (am 23.12.2009: ALAT 61 U/l; außerdem Gamma-GT 134 U/l; AP (Alkalische Phosphatase) 101 U/l (Normbereich: 36-132 U/l); Ausgangswert: am 15.10.2009 ASAT 37 U/l (Normbereich: <31 U/l)). Im weiteren Verlauf Rückgang der erhöhten Aminotransferasen (am 22.01.2010: ALAT 122 U/l; ASAT 32 U/l; außerdem GGT 473 U/l; Bili gesamt 0,7 mg/dl (Normbereich: <1 mg/dl); am 26.01.2010: ALAT 105 U/l; ASAT 61 U/l; außerdem Gamma-GT 532 U/l; am 18.02.1010: ALAT 38 U/l; ASAT 26 U/l; Gamma-GT 170 U/l; AP 98 U/l). Anschließend waren nur unbedeutende Schwankungen der Transaminasen bei Fettleber und bekannter Adipositas festzustellen (ALAT maximal 56 U/l; ASAT maximal 41 U/l; Gamma-GT maximal 143 U/l).

Sonographisch Befund wie bei Fettleber mit massiver Hepatomegalie (ca. 20 cm große Leber). Die Leberverfettung wurde dann am 04.02.2010 durch eine Leberbiopsie histologisch gesichert. In der Histologie massive grobtropfige Parenchymverfettung (>90%) mit Neutralfetteinlagerungen ohne Leberumbauzeichen und ohne Hinweis auf alkohol-toxischen Leberschaden, kein Hinweis auf Virushepatitis, primär biliäre Zirrhose, primär sklerosierende Cholangitis oder Autoimmunhepatitis. Allerdings laborchemisch gering bis mäßig erhöhter ANA-Titer mit ringförmigem Fluoreszenzmuster wie bei autoimmunen Lebererkrankungen; Befund mit derzeit eher geringer Signifikanz. TSH basal normal, C- und p-ANCA negativ, unauffällige Hepatitisserologie (A, B, C), desweiteren unauffällige Serologie auf Lues und CMV (Cytomegalie-Virus). EBV (Epstein-Barr-Virus)-Serologie wie bei lange zurückliegender Primärinfektion (IgG pos, IgM neg.). Laborchemisch geringgradige Hypercholesterinämie und Hypertriglyzeridämie.

In Zusammenschau aller Befunde und des Verlaufs der Transaminasen kann bei vorbestehendem Leberschaden aktuell von einem zusätzlichen Akutereignis ausgegangen werden, das wahrscheinlich medikamentös bedingt ist.

Die UAW hat sich im Krankenhaus entwickelt.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Citalopram 10 mg Filmtabl, 1A-Pharma, Einnahmezeitraum: 23.11.2009-22.12.2009,

Paracetamol-ratiopharm 500 mg Tabl, Einnahmezeitraum: 2006-weiterhin,

Remergil SolTab 15/30 mg Schmelztabl, Essex Pharma, Einnahmezeitraum: 22.12.2009-20.01.2010.

Da eine weitere genauere Zuordnung nicht möglich ist, wird die Arzneimittelkausalität jeweils mit "möglich" bewertet.

Der anfängliche Verdacht einer durch Trevilor ausgelösten Hepatitis hat sich nach weiterer Evaluation nicht bestätigt, da zu einem späteren Zeitpunkt dieser Arzneistoff wieder eingesetzt und von der Patientin gut vertragen wurde.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10031822

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. No
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Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quick's value decreased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol-ratiopharm 500 mg Tabletten	Oral	/	-	Cold
paracetamol		245	1 Week	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test		41	%	
Quick pulse		22	%	

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 10.03.2010 DE-RATIOPH-2010R0301519

Sender diagnosis: not reported

Narrative

Reported terms: Quick value decreased(Prothrombin time prolonged)

Case reference number 2010R0301519 is a spontaneous case report sent by a consumer which refers to a female of unknown age. Case was not confirmed by a health professional.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL), indication Cold on an unknown date. The patient developed Quick value decreased (Prothrombin time prolonged)(start date ?).

The following action was taken: for Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL) - Drug withdrawn

The outcome for Quick value decreased (Prothrombin time prolonged) is unknown

Rechallenge information for Paracetamol-ratiopharm 500 mg Tabletten is not applicable. Dechallenge information for Paracetamol-ratiopharm 500 mg Tabletten is not applicable.

The reporter's assessment for drug Paracetamol-ratiopharm 500 mg Tabletten related with Quick value decreased is as follows: Not applicable.

CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10084485

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19490402	Age: 61 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20100602			1 Day	recovered/resolved
2 Gasping	20100602			1 Day	recovered/resolved
3 Drug overdose deliberate self-inflicted	20100601				recovered/resolved
4 Suicide attempt	20100601				recovered/resolved
5 CK increased	20100602			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMITRIPTYLINE amitriptyline	Oral	/	20100601-20100601 1 Day	
2 ethanol ethanol		/	20100601-20100601 1 Day	
3 PARACETAMOL paracetamol	Oral	/	20100601-20100601 1 Day	
4 TAVOR lorazepam	Oral	/ 245	20100601-20100601 1 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20100602	150	IU/L	
CK	20100602	4000	IU/L	
GGT	20100602	110	IU/L	
GPT	20100602	40	IU/L	

Narratives (nur bei elektronischem Eingang)

Wyeth-Pharma GmbH, 11.06.2010 DE-WYE-G06229810

Sender diagnosis: not reported

Narrative

Information was received from a clinician via the Poison Information Center in Mainz regarding a 61-year-old male patient who took a drug overdose for unknown reason and experienced hepatic enzymes increased, CK increased and stertorous breathing.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

The patient was suspected of having taken 9 tablets of Tavor (lorazepam unspec) (overdose amount 9mg) (oral), an unknown overdose amount of Paracetamol (oral) and an unknown amount of beer/wine (unknown amount of ethanol) (oral) on 01-Jun-2010.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient took a drug overdose for unknown reason (overdose) on 01-Jun-2010 and experienced hepatic enzymes increased (hepatic enzyme increased), CK increased (blood creatine phosphokinase increased) and stertorous breathing (dyspnoea). It is unknown if the patient was receiving daily therapy with Tavor. Initially the Poison Information Center recommended to hospitalize the patient for monitoring (ICU), to perform a drug

screening and to check the blood alcohol level and drug level of paracetamol. Advised treatment included ACC scheme, administration of silibinin and volume replacement. The clinician was further advised to call back after done recommended diagnostics. The outcome was unknown.

TEST RESULTS:

In Jun-2010 test results were: blood creatine phosphokinase (results: 4000); alanine aminotransferase (results: 40); gamma-glutamyltransferase (results: 110); and aspartate aminotransferase (results: 150).

Arzneimittel in suizidaler Absicht bekannt. Die übrigen Ereignisse werden als unbekannt eingestuft. Jedoch ist eine Erhöhung der Lebertransaminasen, Atemdepression und Apnoe in der FI gelistet. Ein möglicher Zusammenhang kann nicht ausgeschlossen werden. Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Jutta Kleinbunttemeyer

Reporter's comment:

Wyeth-Pharma GmbH, 02.07.2010 DE-WYE-G06229810

Sender diagnosis: not reported

Narrative

Additional information was received that indicated the overdose as suicide attempt and added the patient's weight, Paracetamol overdose amount, Amitriptyline as additional suspect product, treatment and outcome. In addition the event onset dates have been clarified.

Information was received from a clinician via the Poison Information Center in Mainz regarding a 61-year-old male patient who took a drug overdose deliberately self-inflicted as a suicide attempt and experienced hepatic enzymes increased, CK increased and stertorous breathing.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

The patient was suspected of having taken 9 tablets of Tavor (lorazepam tablet) (overdose amount 9mg) (oral), 20 tablets of Paracetamol (overdose amount 10 g) (oral), an unknown amount of beer/wine (unknown amount of ethanol) (oral) and 30 tablets of Amitriptyline (overdose amount 1500 mg) (oral) on 01-Jun-2010. The drug intake was considered questionable for all suspected drugs.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient took a drug overdose deliberately self-inflicted (intentional overdose) on 01-Jun-2010 and was found with hepatic enzymes increased (hepatic enzyme increased), CK increased (blood creatine phosphokinase increased) and stertorous breathing (dyspnoea) on 02-Jun-2010. The overdose was classified as suicide attempt (suicide attempt). It is unknown if the patient was receiving daily therapy with Tavor. Initially the Poison Information Center recommended to hospitalize the patient to ICU for monitoring, to perform a drug screening and to check the blood alcohol level and drug level of paracetamol. Advised treatment included ACC scheme, administration of silibinin and volume replacement. The clinician was further advised to call back after done recommended diagnostics. The patient was subsequently monitored on ICU and a drug screen has been performed (result unknown). The patient recovered without specific treatment.

TEST RESULTS:

Drug screen (results: unknown) was done in Jun-2010. On 02-Jun-2010 test results were: aspartate aminotransferase (results: 150 IU/L); alanine aminotransferase (results: 40 IU/L); blood creatine phosphokinase (results: 4000 IU/L); and gamma-glutamyltransferase (results: 110 IU/L).

weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. L. Kuphal

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10098420

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19820706	Age: 27 Year	weight (kg): 55	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20100530	deathcause: Fulminant hepatic failure
deathdate: 20100530	deathcause: Hepatorenal syndrome
deathdate: 20100530	deathcause: Acute renal failure
deathdate: 20100530	deathcause: Hepatic encephalopathy

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute renal failure	20100526				fatal
2 Hepatorenal syndrome	20100526				fatal
3 Fulminant hepatic failure	20100526				fatal
4 Subarachnoid haemorrhage					fatal
5 Abdominal pain					not recovered/not resolved
6 Cerebral edema					fatal
7 Hepatic encephalopathy	20100526				fatal
8 Vomiting					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	201005-201005	Fever
paracetamol		245	6 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Glutamic-oxaloacetic transferase	20100527	8581	U/l	
GPT	20100526	6950	U/l	
Paracetamol	20100526	7.6	g/ml	
Aspartate transferase	20100527	8581	u/L	
GPT	20100527	6950	u/L	
Paracetamol	20100527	7.6	g/mL	
APTT	20100527	53	S	
APTT	20100526	47	S	
APTT	20100529	57	S	
Bilirubin	20100526	7.70	MG/DL	
Bilirubin	20100529	5.60	MG/DL	
Bilirubin	20100528	2.70	MG/DL	
Bilirubin	20100527	6.40	MG/DL	
Bilirubin total	20100528	1.99	MG/DL	
Bilirubin total	20100527	4.88	MG/DL	

Creatinine	20100527	2.09	MG/DL
Creatinine	20100529	5.26	MG/DL
Creatinine	20100526	1.47	MG/DL
Gamma GT	20100526	73	U/L
Gamma GT	20100528	84	U/L
Gamma GT	20100529	111	U/L
Gamma GT	20100527	75	U/L
GPT	20100527	6124	U/I
GPT	20100526	6950	U/I
GPT	20100529	3094	U/I
INR	20100529	4.64	NA
INR	20100527	4.0	NA
INR	20100526	6.07	NA
Prothrombin time	20100529	12	%
Prothrombin time	20100527	15	%
Prothrombin time	20100526	<8.0	%
SGOT	20100528	2450	U/I
SGOT	20100529	1391	U/I
Lipase	20100527	127	U/L
Lipase	20100526	97	U/L
Lipase	20100529	323	U/L

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 15.07.2010 DE-GlaxoSmithKline-B0665385A

Sender diagnosis: not reported

Narrative

This case was reported by a physician via the German Regulatory Authority (BfArM) and described the occurrence of fulminant hepatic failure in a 27-year-old female patient who received Paracetamol over a period of 6 days for fever.

On an unknown date, the patient started Paracetamol. On 26 May 2010, the patient experienced fulminant hepatic failure, hepatorenal syndrome and acute renal failure. At the time of reporting, the events were fatal. The health professional considered the events were possibly related to treatment with Paracetamol.

The patient died, cause of was fulminant hepatic failure, hepatorenal syndrome and acute renal failure. It is unknown whether an autopsy was

performed.

sanofi-aventis, 19.07.2010 DE-SANOFI-AVENTIS-2010SA038765

Sender diagnosis: not reported

Narrative

Initial report:

On 06-Jul-2010 this case was extracted from the website of the German health authority (authority number DE-BFARM-10098420); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association (DCGMA) about a 27-year-old female patient, whose medical history, concomitant disease(s) and drug(s) were not provided.

At unspecified time point the intake of paracetamol (total dosage: 9 - 18 g) for fever was started and continued for six days.

On 26-May-2010 the patient suffered fulminant hepatic failure, hepatorenal syndrome and acute renal failure. The outcome of all events was specified with fatal.

Causality by DCGMA (WHO GI): possible for all events

19-Jul-2010 Sanofi-Aventis Germany

This case report includes clinical diagnoses, which are considered locally unlabelled for one (or more) suspected product(s). The medical evaluation however does not imply a change in the state of scientific knowledge for the suspected product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 05.01.2011 DE-SANOFI-AVENTIS-2010SA038765

Sender diagnosis: not reported

Narrative

Initial report:

On 06-Jul-2010 this case was extracted from the website of the German health authority (authority number DE-BFARM-10098420); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association (DCGMA) about a 27-year-old female patient, whose medical history, concomitant disease(s) and drug(s) were not provided.

At unspecified time point the intake of paracetamol (total dosage: 9 - 18 g) for fever was started and continued for six days.

On 26-May-2010 the patient suffered fulminant hepatic failure, hepatorenal syndrome and acute renal failure. The outcome of all events was specified with fatal.

Causality by DCGMA (WHO GI): possible for all events

Additional information received on 03-Jan-2011:

From the Drug Committee of the German Physicians Association we received the ADR forms and hospital reports documenting the course of patient's events until her death. Due to feeling of fever the patient took paracetamol in the initial reported dosage and duration. On 26-May-2010 the patient visited the central emergency room, and presented recurrent vomiting and diffuse abdominal pain.

The fulminant hepatic failure with massive increased transaminases (GPT 6950 U/L from 26-May-2010, SGOT 8581 U/L from 27-May-2010 and gammaGT 84 U/L from 28-May-2010), was revealed by lab investigation. Consequently the patient was admitted to the gastro-enterological ward on 27-May-2010 and corrective treatment with acetylcysteine (firstly according to a scheme and continuing with 150 mg/kg/24 h) was started. The extracorporeal detoxification with bilirubin apheresis was performed on the same and the next day. Due to worsening of patient's condition a liver transplantation was discussed. Furthermore the patient developed hepatorenal syndrome with acute renal failure and she was transferred to a gastroenterological intensive unit on 29-May-2010. The counter measurement with haemodialysis was started. In the evening of the same day a CT was performed and disclosed the hepatic encephalopathy with massive cerebral edema with secondary subarachnoidal bleeding.

Consequently a hepatic encephalopathy was diagnosed. A neurosurgical intervention was not more realisable at this time point. In the noon of the next day, the brain dead was confirmed by transcranial and cervical Doppler sonography, pathologic apnoe test and not measurable brain stem reflexes. Cause of death: hepatic encephalopathy.

Reporter causality; possible

Bristol-Myers Squibb Company, 01.02.2011 DE-Bristol-Myers Squibb Company-15177801

Sender diagnosis: not reported

Narrative

This case was received via the Drug Commission of German Physicians (cross reference number: 10153232) and the German health authority (cross reference number: 10098420) in BMS Germany (local file number: 2010070502)

A physician reported that a 27-year-old female patient experienced fulminant hepatic failure, hepatorenal syndrome and acute renal failure and subsequently died while on therapy with acetaminophen for the treatment of fever.

The patient had received acetaminophen therapy for 6 days (total dose: 9 to 18 g). According to the reporting physician the start date of all 3 events (acute renal failure, fulminant hepatic failure and hepatorenal syndrome) was 26-May-2010, the outcome of all 3 events was 'fatal'.

The physician considered the events acute renal failure, fulminant hepatic failure and hepatorenal syndrome were possibly related to acetaminophen therapy.

GPT (glutamic pyruvic transaminase) was at 6950 U/L, Acetaminophen was at 7.6 g/ml on 26-May-2010. Glutamic-oxaloacetic transaminase was at 8581 U/L on 27-May-2010.

It was unknown whether an autopsy was done at the time of this report.

BMS Medical Evaluation Comment:

This patient developed fatal fulminant hepatic failure, acute renal failure and hepatorenal syndrome while on acetaminophen therapy. Fulminant hepatic failure could have resulted in acute renal failure secondary to hepatorenal syndrome in this patient. Although information regarding concomitant medication and medical history was lacking, a contributory role of acetaminophen was considered possible.

Supplemental information received from the physician via the Drug Commission of German Physicians and BMS Germany on 25-Jan-2011 provided the addition of a fatal event subarachnoid hemorrhage.

The patient was presented with recurrent vomiting and diffused abdominal pain. Due to "feeling of fever" she took acetaminophen tablets within the last days. It was not possible to determine the exact dose of acetaminophen. The total dose was most likely 9 to 18 g acetaminophen during the last 6 days.

Laboratory investigations showed liver failure with massively increased transaminases and a distinct coagulopathy as well as acute renal failure. Due to suspected acetaminophen intoxication the patient received acetylcysteine therapy according to scheme. After completion of this scheme she was continued therapy with acetylcysteine 150 mg/kg/24 hours.

On 27-May-2010 the patient was admitted to the internistic intensive care unit via the central emergency department. On 27-May-2010 she was started with extracorporeal detoxication using a bilirubin apheresis. For this purpose a Shaldon's catheter was placed at right jugular after previous administration of PPSB (prothrombin concentrate) as well as antithrombin and performed a bilirubin apheresis on 27-May-2010 and on 28-May-2010 respectively.

The patient's clinical state worsened in the course and progressive renal failure became apparent. After consultation with gastroenterological-nephrological physicians, they planned the transfer of the patient to a liver center as liver transplantation might be due. On 29-May-2010 she was transferred to the gastroenterological intensive care unit of the university clinic.

Electrocardiogram (EC) on 26-May-2010: sinus rhythm, vertical type, heart rate 110/min, no repolarization disorder and on 28-May-2010: Sinus rhythm, vertical type, heart rate 119/min, no repolarization disorder.

X-Ray diagnosis of thorax on 27-May-2010 and 28-May-2010 showed mediastinum in a middle position and normally wide. Heart normal in size and regularly configured without signs of congestion. Thick-lumen jugularis catheter of the right in unchanged position. Mediastinum in middle position.

She was transferred to the second hospital and was hospitalized from 29-May-2010 until 30-May-2010. At takeover she was comatose, still spontaneously breathing in a massively reduced general condition. The patient was not awakened by painful stimuli; in addition protective reflexes were missing. Shaldon's catheter left vena jugularis.

After hospital admission of the patient an application of a central venous catheter and an arterial cannula were performed. Immediately after admission arrangements for HU (highly urgent)-liver transplantation were made, so that the patient was actively listed at 18:00 hrs on 29-May-2010. In the further course optimization of the coagulation status was performed using transfusion of 4 FFP (fresh frozen plasma), 1 THK (thrombocyte concentrate, highly concentrated) as well as 2 erythrocytes concentrates.

In the evening of the day of admission starting at 21:00 hrs recurrent severe bradycardic episodes occurred which were alternating with sinus tachycardia and were accompanied by severe blood pressure fluctuations requiring the administration of both, noradrenaline and antihypertensive agents.

Upon persisting hemodynamic instability in the further course the pupils unrounded and were no more responded to light, so that they immediately arranged a cerebral computed tomography which revealed a massive cerebral edema with transforaminal herniation and ubiquitous subarachnoid amount of blood. In view of the underlying disease most likely an acute hepatic encephalopathy with secondary subarachnoid bleeding had been assumed. Due to the presumably for an advanced time persisting complete constriction there was unfortunately no option for an emergency neurosurgical intervention.

On 30-May-2010 at noon in consultant collaboration with the colleagues from the neurologic department the proper ascertainment of the cerebral death was determined. The cerebral death was at 13:20 hrs and had to be verified by a transcranial and cervical Doppler sonography, a pathologic apnea test and defuncted brain stem reflexes.

The hospital concluded with

- cerebral death due to massive cerebral edema with transforaminal herniation with secondary subarachnoid hemorrhage in severe hepatic encephalopathy
- Fulminant liver failure upon accidental intoxication of acetaminophen
- Hepatorenal syndrome with acute renal failure
- Hemorrhagic diathesis with severe coagulopathy.

BMS Medical Evaluation Comment:

This patient developed fatal fulminant hepatic failure, acute renal failure, hepatorenal syndrome and subarachnoid hemorrhage while on acetaminophen therapy. Acetaminophen is known to be associated with hepatotoxicity and intoxication could have led to fulminant hepatitis. Fulminant hepatic failure could have led to subarachnoid hemorrhage and acute renal failure secondary to hepatorenal syndrome and subsequent fatal outcome in this patient.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10129565

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19561210	Age: 53 Year	weight (kg): 55	BMI_int: 21.76	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatopathy	20100705	20100816	43 Day	19 Day	recovered/resolved
2 Appetite lost					recovering/resolving
3 Dizziness					recovering/resolving
4 Tiredness					recovering/resolving
5 Transaminases increased					recovering/resolving
6 Physical disability					recovering/resolving
7 Weakness					recovering/resolving
8 Vomiting					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 mabthera rituximab	Intravenous drip	6*1000 Mg milligram(s)/14293	2009-20100630 14 Month	Arthritis rheumatoid
2 mtx methotrexate sodium	Oral	15 Mg milligram(s)/Week245	2000-20100705 10 Year	Arthritis rheumatoid
3 paracetamol paracetamol	Oral	1000 Mg milligram(s)/Day245	20100616-20100616 15 Day	Fever
4 PARACETAMOL paracetamol	Oral	1000 Mg milligram(s)/Day245	20100630-20100630 2 Day	Fever
5 PARACETAMOL paracetamol	Oral	1000 Mg milligram(s)/245	20100630-20100630 2 Day	Fever

Drugs (concomitant)

6 prednisolone prednisolone	Oral	20 Mg milligram(s)/Day245	20100705- -	Exanthema
7 prednisolone prednisolone	Oral	5 Mg milligram(s)/Day245	1997-20100704 -	Arthritis rheumatoid
8 prednisolone prednisolone	Oral	5 Mg milligram(s)/Day245	1997-20100704 -	Exanthema
9 prednisolone prednisolone	Oral	20 Mg milligram(s)/Day245	20100705- -	Arthritis rheumatoid

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatic cirrhosis	2010		Yes	ED 7/2010 bei neu diagnostizierter Hämochromatose
Osteopenia	2010		Yes	Beginn unbekannt
Arthritis rheumatoid	1978		Yes	bisherige Therapien: MTX; Adalimumab + MTX; seit 2009 Rituximab + MTX
Iron overload	20100716		Yes	ED Hämochromatose mit heterozygoter Mutation

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	200603	1.42	UNK	
Alanine aminotransferase	20100716	3.58	UNK	
Alanine aminotransferase	20100709	18.4	UNK	
Alanine aminotransferase	20100707	32.3	UNK	
Alanine aminotransferase	20100706	46.6	UNK	
Albumin	20100706	55	g/l	
Albumin	20100709	32	g/l	
Aspartate aminotransferase	20100716	0.37	UNK	
Aspartate aminotransferase	20100709	1.64	UNK	
Aspartate aminotransferase	20100707	6.27	UNK	
Aspartate aminotransferase	20100706	18.2	UNK	
Aspartate aminotransferase	200603	0.63	UNK	
Biopsy of liver	20100820	UNCHARACTERISTIC FINDINGS		
C-reactive protein	20100707	56.9	mg/l	
C-reactive protein	20100706	85.3	mg/l	
C-reactive protein	20100716	3.3	mg/l	
Creatinine	20100607	72	MCMOL/L	
Creatinine	20100716	69	MCMOL/L	
Gamma GT	20100716	3.36	UNK	
Gamma GT	20100709	8.39	UNK	
Gamma GT	20100706	7.26	UNK	
Gamma GT	200603	0.84	UNK	
Gene mutation identification test		HETEROZYGOUS MUTATION A63C		

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 26.08.2010 DE-PVZIBE-04-10209

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Hepatopathie

KURZZUSAMMENFASSUNG:

Hepatopathie mit Transaminasenanstieg unter Paracetamol bei Therapie mit Rituximab und MTX sowie vorbestehender Leberzirrhose

BESCHREIBUNG:

AD.: Leberschädigung mit V.a. medikamentös-tox. Genese. Die stat. Aufnahme erfolgt am 5.7. in Heimatkrankenhaus nach Einweisung durch den Hausarzt aufgrund massiv erhöhter Transaminasen. Die Patientin hatte sich dort aufgrund von Müdigkeit, Leistungsminderung, Appetitlosigkeit seit dem 2.7. und einmaligem Erbrechen am 4.7. vorgestellt. Am 30.06. hatte die Patientin die 6. Infusion Rituximab (= 2. Infusion des 3. Zyklus) sowie 1 g Paracetamol erhalten. Am 6.7. wurde die Patientin bei massivem Transaminasenanstieg und mässig erhöhten Entzündungsparametern in Zentrumsklinik verlegt; bei Übernahme klagte sie über Schwindel und Schwäche, Schmerzen und Infektgefühl wurden verneint.

THERAPIE:

6.7. Absetzen aller pot. hepatotox. Medikamente (Paracetamol, MTX abgesetzt). 10.7. ACC, Legalon, Reduktion der Prednisolondosis (schrittweise)

VERLAUF:

7.7. Rö-Thorax unauffällig, keine Infiltrate, Virus-Serologie: durchgemachte CMV/EBV-Infektion; keine akute Infektion; Hepatitisserologie neg. 8.7. Ausschluss Kupferspeicherkrankheit; Autoimmundiagnostik negativ. 9.7. Sonographie: Leber normal groß, regelrechte Gefäßkonfiguration, Gallenwege nicht erweitert, Gallenblase steinfrei, keine freie Flüssigkeit. 10.7. CT Thorax/Abdomen: Leber: mikronoduläre Zirrhose. 16.7. Hämochromatose-Gen: positiv.

ERLÄUTERUNGEN ZUR UAW:

Bei der Patientin liegt zwar eine hepatische Grunderkrankung vor, dennoch handelt es sich bei der aktuellen Symptomatik sehr wahrscheinlich um einen akuten Prozeß, welcher medikamentös-toxisch verursacht sein könnte.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: mtx

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: bei Langzeiteinnahme von MTX kann es zu Leberschädigung und Leberzirrhose kommen, insbesondere bei Vorbelastung (wie Diabetes, Adipositas) und Einnahme weiterer potentiell hepatotoxischer Medikamente. Im Falle der Patienten sind die gelegentlichen Einnahmen von NSAR und regelmäßige Intervall-Therapien mit Prednisolon bekannt. Die Hauptursache für die Leberzirrhose muss in diesem Fall allerdings in der neu diagnostizierten Hämochromatose gesehen werden. Dennoch oder gerade deswegen kann eine akute Verschlechterung unter MTX nicht ausgeschlossen werden, zumal auch Leberfermenterhöhungen und akute Hepatitiden vereinzelt beobachtet wurden. Allerdings spricht die Chronologie eher für Paracetamol.

MEDIKAMENT: mabthera

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: Eine Leberschädigung ist für Rituximab nicht beschrieben, der zeitliche Zusammenhang lässt eine Beteiligung dennoch möglich erscheinen. Eventuell war auch die Begünstigung eines infektiösen Geschehens durch Rituximab für die Hepatopathie mit verantwortlich (ein Erreger konnte jedoch nicht nachgewiesen werden).

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: Eine dosisabhängige Leberschädigung ist für Paracetamol bekannt, bei Vorschädigung der Leber kann es auch schon bei geringer Dosierung zu Hepatopathie mit Transaminasenanstieg kommen. Aufgrund der Paracetamol-Einnahme von 1 g am 16. und 30. Juni zur Prämedikation (bei Rituximab) ist ein Zusammenhang wahrscheinlich (C3). Das gute Ansprechen auf die Therapie mit ACC und das spontane Dechallenge etwa 7 Tage nach der letzten Einnahme sprechen ebenfalls dafür. Die Leberzirrhose wurde höchstwahrscheinlich durch die Hämochromatose hervorgerufen, die akute Verschlechterung kann aktuell jedoch im Zusammenhang mit der Paracetamol-Einnahme gesehen werden. (Aufgrund der möglichen Alternativursachen hier jedoch S2).

sanofi-aventis, 06.09.2010 DE-SANOFI-AVENTIS-2010SA051596

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10129565).

Short summary: hepatopathy with increase in transaminases under treatment with paracetamol, rituximab, and methotrexate, as well as in the course of pre-existing liver cirrhosis.

The case involves a 53-year-old female patient suffering from rheumatoid arthritis, osteopenia, hepatic cirrhosis (diagnosed in Jul. 2010 in the course of newly diagnosed haemochromatosis). Concomitant medication included prednisolone. The patient was treated with paracetamol (trade name not provided) 1000mg orally on 16-Jun- and 30-Jun-2010 as pre-medication ("indication: fever"). Treatment with methotrexate had been started in 2000 (dose 15mg/week, indication: rheumatoid arthritis), treatment with rituximab was added in 2009 (dose 2x 1000mg with a time-lag of 14 days = one cycle; indication: rheumatoid arthritis). The patient had received the second infusion of the third cycle with rituximab on 30-Jun-2010, last dose of methotrexate was on 05-Jul-2010.

On 05-Jul-2010, the patient was admitted to hospital by her family doctor due to massively increased transaminases. She had called on her physician on 04-Jul-2010 because of tiredness, decrease of physical ability, loss of appetite since 02-Jul-2010, and recurrent vomiting. On 06-Jul-2010, the patient was transferred to another hospital due to the severe increase in transaminases and a moderate increase in inflammation parameters. At admission to this hospital, the patient complained of dizziness and weakness. Admission diagnosis of liver damage with suspicion of drug-toxic aetiology was established (either in first or in second hospital).

On 06-Jul-2010, all potentially hepatotoxic drugs (paracetamol, methotrexate) were discontinued. On 10-Jul-2010, treatment with acetylcysteine and silybum marianum was initiated as corrective therapy, prednisolone dose was slowly reduced. Outcome was reported as "recovering/resolving".

Causality determined by PV centre:

"probable" for paracetamol,

"possible" for methotrexate,

"unlikely" for rituximab.

Comment on causality: Dose-dependent liver damage is known for paracetamol; if the liver is already damaged, even a lower dose could lead to hepatopathy with increase in transaminases; because of intake of 1g paracetamol on 16- and 30-Jun-2010, respectively, a relationship is probable; this is also backed-up by the good response to the treatment with acetylcysteine, and the spontaneous dechallenge about seven days after last intake. The liver cirrhosis had, without much doubt, been caused by haemochromatosis, but the acute progression can be regarded in relationship to the intake of paracetamol.

Long-term intake of methotrexate can lead to liver damage and liver cirrhosis, especially in pre-stressed patients (diabetes, obesity), and with concomitant intake of other potentially hepatotoxic drugs. In the case of this patient, an infrequent intake of non-steroidal anti-rheumatic drugs and regular interval treatment with prednisolone are known. However, the newly diagnosed haemochromatosis must be considered the main cause of liver cirrhosis in this patient. Nevertheless, an acute progression under treatment with methotrexate cannot be excluded, as there have been single incidences of increase in liver enzymes and acute hepatitis under methotrexate. The chronology of events rather speaks for paracetamol as cause of

the observed hepatopathy.

Liver damage is not described for rituximab, but the temporal relationship points to a possible involvement. Maybe, a promotion of an infectious process by rituximab was jointly responsible for the hepatopathy (however, a pathogen could not be verified).

HELIOS Klinikum Wuppertal, 22.11.2010 DE-PVZIBE-04-10209

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Hepatopathie

KURZZUSAMMENFASSUNG:

Hepatopathie bei Transaminasenanstieg unter Paracetamol sowie Therapie mit Rituximab und MTX auf dem Boden einer Hämochromatose.

BESCHREIBUNG:

AD.: Leberschädigung mit V.a. medikamentös-tox. Genese. Die stationäre Aufnahme erfolgte am 5.7. in Heimatkrankenhaus nach Einweisung durch den Hausarzt aufgrund massiv erhöhter Transaminasen. Die Patientin hatte sich dort aufgrund von Müdigkeit, Leistungsminderung, Appetitlosigkeit seit dem 2.7. und einmaligem Erbrechen am 4.7. vorgestellt. Am 30.06. hatte die Patientin die 6. Infusion Rituximab (= 2. Infusion des 3. Zyklus) sowie 1 g Paracetamol erhalten. Am 6.7. wurde die Patientin bei massivem Transaminasenanstieg und mässig erhöhten Entzündungsparametern in Zentrumklinik verlegt; bei Übernahme klagte sie über Schwindel und Schwäche, Schmerzen und Infektgefühl wurden verneint.

THERAPIE:

6.7. Absetzen aller pot. hepatotox. Medikamente (Paracetamol, MTX, Mabthera). 10.7. ACC, Legalon, Reduktion der Prednisolondosis (schrittweise)

VERLAUF:

7.7. Rö-Thorax unauffällig, keine Infiltrate, Virus-Serologie: durchgemachte CMV/EBV-Infektion; keine akute Infektion; Hepatitisserologie neg. 8.7. Ausschluss Kupferspeicherkrankheit; Autoimmundiagnostik negativ. 9.7. Sonographie: Leber normal groß, regelrechte Gefäßkonfiguration, Gallenwege nicht erweitert, Gallenblase steinfrei, keine freie Flüssigkeit. 10.7. CT Thorax/Abdomen: Leber: mikronoduläre Zirrhose. Labor: Transferrinsättigung mit 94% sowie Eisen im Plasma mit 34 µmol/l stark erhöht, 16.7. Hämochromatose-Gen: positiv.

Stationärer Aufenthalt vom 16.8. bis 23.8.:

Die Pat. wurde im August nochmals stationär aufgenommen um den weiteren Verlauf abzuklären. Bei der Leber-Stanzbiopsie (patholog. Befund vom 20.8.) wurde keine Zirrhose festgestellt, auch Veränderungen im Sinne einer Hämochromatose waren nicht nachweisbar, kein Malignomverdacht, insgesamt uncharakteristische Veränderungen mit V.a. medikamentös-tox. Genese;

Die Genanalyse ergab eine heterozygote Mutation A63D;

Sowohl Paracetamol, MTX als auch Mabthera wurde von den behandelnden Ärzten in Betracht gezogen, seit dem Absetzen der Medikamente am 6.7. hatte sich Patientin erstaunlich gut erholt, so dass von der Diagnose einer Leberzirrhose vorerst wieder Abstand genommen wurde. Die Patientin wurde ausdrücklich auf die Meidung hepatotoxischer Noxen hingewiesen;

Zur Weitertherapie der Rheumatoiden Arthritis wurden Abatacept oder Tocilizumab empfohlen. Das Ende der UAW wurde mit dem 16.8. definiert, da zu diesem Zeitpunkt die Transaminasen wieder normwertig waren.

ERLÄUTERUNGEN ZUR UAW:

Die Patientin wurde aufgrund eines massiven Transaminasenanstiegs stationär eingewiesen, bei am ehesten medikamentös-toxischer Genese auf dem Boden einer heterozygoten Form der Hämochromatose.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: mtx

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: bei Langzeiteinnahme von MTX kann es zu Leberschädigung und Leberzirrhose kommen, insbesondere bei Vorbelastung (wie Diabetes, Adipositas) und Einnahme weiterer potenziell hepatotoxischer Medikamente, auch Leberfermenterhöhungen und akute Hepatitiden sind in Einzelfällen bekannt. Die neu diagnostizierte Hämochromatose stellt vermutlich einen Risikofaktor dar. Dennoch oder gerade deswegen kann eine akute Verschlechterung unter MTX nicht ausgeschlossen werden. Allerdings spricht die Chronologie eher für Paracetamol und Mabthera. Die zügige und bisher anhaltende Normalisierung der Transaminasen nach Absetzen der Arzneimittel spricht gegen eine fortgeschrittene hepatische Erkrankung und für eine medikamentöse Schädigungskomponente.

MEDIKAMENT: mabthera

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: Eine Leberschädigung ist für Rituximab nicht beschrieben, der zeitliche Zusammenhang lässt eine Beteiligung dennoch möglich erscheinen. Eventuell war auch die Begünstigung eines infektiösen Geschehens durch Rituximab

sanofi-aventis, 02.12.2010 DE-SANOFI-AVENTIS-2010SA051596

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10129565).

Short summary: hepatopathy with increase in transaminases under treatment with paracetamol, rituximab, and methotrexate, as well as in the course of pre-existing liver cirrhosis.

The case involves a 53-year-old female patient suffering from rheumatoid arthritis, osteopenia, hepatic cirrhosis (diagnosed in Jul. 2010 in the course of newly diagnosed haemochromatosis). Concomitant medication included prednisolone. The patient was treated with paracetamol (trade name not provided) 1000mg orally on 16-Jun- and 30-Jun-2010 as pre-medication ("indication: fever"). Treatment with methotrexate had been started in 2000 (dose 15mg/week, indication: rheumatoid arthritis), treatment with rituximab was added in 2009 (dose 2x 1000mg with a time-lag of 14 days = one cycle; indication: rheumatoid arthritis). The patient had received the second infusion of the third cycle with rituximab on 30-Jun-2010, last dose of methotrexate was on 05-Jul-2010.

On 05-Jul-2010, the patient was admitted to hospital by her family doctor due to massively increased transaminases. She had called on her physician on 04-Jul-2010 because of tiredness, decrease of physical ability, loss of appetite since 02-Jul-2010, and recurrent vomiting. On 06-Jul-2010, the patient was transferred to another hospital due to the severe increase in transaminases and a moderate increase in inflammation parameters. At admission to this hospital, the patient complained of dizziness and weakness. Admission diagnosis of liver damage with suspicion of drug-toxic aetiology was established (either in first or in second hospital).

On 06-Jul-2010, all potentially hepatotoxic drugs (paracetamol, methotrexate) were discontinued. On 10-Jul-2010, treatment with acetylcysteine and silybum marianum was initiated as corrective therapy, prednisolone dose was slowly reduced. Outcome was reported as "recovering/resolving".

Causality determined by PV centre:

"probable" for paracetamol,

"possible" for methotrexate,

"unlikely" for rituximab.

Comment on causality: Dose-dependent liver damage is known for paracetamol; if the liver is already damaged, even a lower dose could lead to hepatopathy with increase in transaminases; because of intake of 1g paracetamol on 16- and 30-Jun-2010, respectively, a relationship is probable; this is also backed-up by the good response to the treatment with acetylcysteine, and the spontaneous dechallenge about seven days after last intake. The liver cirrhosis had, without much doubt, been caused by haemochromatosis, but the acute progression can be regarded in relationship to the intake of paracetamol.

Long-term intake of methotrexate can lead to liver damage and liver cirrhosis, especially in pre-stressed patients (diabetes, obesity), and with concomitant intake of other potentially hepatotoxic drugs. In the case of this patient, an infrequent intake of non-steroidal anti-rheumatic drugs and regular interval treatment with prednisolone are known. However, the newly diagnosed haemochromatosis must be considered the main cause of liver cirrhosis in this patient. Nevertheless, an acute progression under treatment with methotrexate cannot be excluded, as there have been single incidences of increase in liver enzymes and acute hepatitis under methotrexate. The chronology of events rather speaks for paracetamol as cause of the observed hepatopathy.

Liver damage is not described for rituximab, but the temporal relationship points to a possible involvement. Maybe, a promotion of an infectious process by rituximab was jointly responsible for the hepatopathy (however, a pathogen could not be verified).

Additional information extracted from the website of the German health authority on 24-Nov-2010. Additional information by PV centre:

New concomitant disease: iron overload since 16-Jul-2010.

The patient was hospitalised again from 16- to 23-Aug-2008 to evaluate the further course. Liver core needle biopsy (pathologic findings from 20-Aug-2008) did not reveal cirrhosis, changing in the sense of haemochromatosis, or signs of malignancy; changes were, all in all, uncharacteristic with suspicion of drug-toxic aetiology. Genetic analysis revealed a heterozygotic mutation A63D. The physicians suspected paracetamol, methotrexate, as well as rituximab. After withdrawal of these medications, the patient recovered astonishingly well, so that the diagnosis of liver cirrhosis was ruled out for the moment. End of adverse drug reaction was reported as 16-Aug-2008, since transaminases were normal again by this date.

PV centre's comment on adverse drug reaction: Patient was referred to hospital due to massive increase in transaminases, most likely of drug-toxic aetiology as a result of a heterozygotic form of haemochromatosis. Causality assessment by PV centre was changed to "possible" for paracetamol; it was not changed for the other drugs.

Case was considered closed by the PV centre, as no further information was expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10146828

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19221028	Age: 87 Year	weight (kg): 52	BMI_int: 22.51	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20100630			18 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol, 500 mg, Tabl, Ratiopharm paracetamol	Oral	/Day 245	20100613-20100630 18 Day	

Drugs (concomitant)

2 ASS, 100 mg, Tabl, Hexal acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	1985- -	
3 Atehexal bzw. Atenolol, 50 mg, Filmtabl, Hexal atenolol	Oral	25 Mg milligram(s)/Day 82	1985- -	
4 Durogesic, 25 µg/h, Pflaster, Janssen-Cilag fentanyl	Cutaneous	600 µg microgram(s)/Day 390	20100623- -	
5 Enahexal, 10 mg, Tabl, Hexal enalapril	Oral	30 Mg milligram(s)/Day 245	1985- -	
6 Fragmin, 0,5 ml, Amp, Pfizer dalteparin	Subcutaneous	5000 Iu international unit(s) 120	20100627- -	
7 Movicol, Btl, Norgine macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral	1 Df dosage form/ 168	20100618-20100629 12 Day	
8 Simvahexal, 40 mg, Filmtabl, Hexal simvastatin	Oral	40 Mg milligram(s)/Day 82	1985-20100630 -	
9 Torasemid, 10 mg, Tabl, Hexal torasemide	Oral	/Day 245	20100613- -	
10 Valoron N, Trpf, Pfizer naloxone hydrochloride; tilidine hydrochloride	Oral	20 Gtt drop(s)/Day 158	20100621-20100623 3 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acetabulum fracture	20100612		k.A.	Acetabulumfraktur links
Coxarthrosis			Yes	beidseitige Coxarthrose
Hypertension arterial			Yes	
Coronary disease	1980		Yes	
Hyperlipidemia			Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20100630	469	U/l	
Alanine aminotransferase increased	20100701	240	U/l	
Alanine aminotransferase increased	20100705	81	U/l	

Alanine aminotransferase normal	20100713	19	U/l
Aspartate aminotransferase increased	20100630	585	U/l
Aspartate aminotransferase increased	20100701	128	U/l
Aspartate aminotransferase normal	20100705	32	U/l
Aspartate aminotransferase normal	20100713	20	U/l
Alkaline phosphatase increased	20100713	156	U/l
Bilirubin normal	20100630	0.4	mg/dl
Bilirubin normal	20100713	0.4	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 24.09.2010 DE-FAKOS-170333

Sender diagnosis: not reported

Narrative

Am 12.06.2010 erfolgte die stationäre Aufnahme der Patientin wegen einer Beckenringfraktur links, die konservativ behandelt wurde. Am 13.06.2010 wurde eine Therapie mit Paracetamol 500 mg Tabl. begonnen. Dann am 30.06.2010 wurden stark erhöhte Transaminasen festgestellt (ALAT 469 U/l Normbereich: <32 U/l), ASAT 585 U/l (Normbereich: 10-35 U/l), außerdem Gamma-GT 1528 U/l (Normbereich: <40 U/l), Bili gesamt 0,4 mg/dl (Normbereich: <1 mg/dl)). Nach Absetzen des Paracetamols (letzte Gabe am 30.06.2010) besserten sich die Laborparameter wieder (am 05.07.2010: ALAT 81 U/l, ASAT 32 U/l, Gamma-GT 739 U/l; am 13.07.2010: ALAT 19 U/l, ASAT 20 U/l, Gamma-GT 293 U/l, AP (Alkalische Phosphatase) 156 U/l (Normbereich: 35-105 U/l), Bili 0,4 mg/dl).

Die Hepatitisserologie (B, C) sowie die Leberultraschalluntersuchung waren unauffällig.

Am 14.07.2010 wurde die Patientin aus der stationären Behandlung entlassen.

Am wahrscheinlichsten handelte es sich um eine durch Paracetamol verursachte Hepatitis. Die UAW hat sich im Krankenhaus entwickelt.

Als Auslöser wird folgendes Arzneimittel verdächtigt:

Paracetamol-ratiopharm 500 mg Tabl, Einnahmezeitraum: 13.-30.06.2010.

Die Arzneimittelkausalität wird mit "wahrscheinlich" bewertet.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10157275

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19390116	Age: 71 Year	weight (kg): 65	BMI_int: 25.08	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased					Unknown
2 Hepatitis	20100712			1 Day	recovering/resolving
3 Decreased INR					Unknown
4 Hepatic failure					recovering/resolving
5 Hepatic enzymes increased					recovering/resolving
6 Hepatopathy					Unknown
7 Hypokalemia					Unknown
8 Hepatic steatosis					Unknown
9 Vertigo					Unknown
10 Difficulty in walking					Unknown
11 Dysesthesia					Unknown
12 Quick's value decreased					Unknown
13 GOT increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin cardio acetylsalicylic acid	Unknown	/	-	Chronic back pain
2 avalox moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day 82	20100712-20100722 11 Day	Ankle ulcer
3 AVALOX moxifloxacin		/	-	Septic joint
4 brufen ibuprofen	Oral	3000 Mg milligram(s)/Day	-	Chronic back pain
5 carbasalate calcium carbasalate calcium	Unknown	1660 Mg milligram(s)/Day	-	Chronic back pain
6 NOVALGIN metamizole	Unknown	/	-	Chronic back pain
7 PARACETAMOL paracetamol	Unknown	/	-	Chronic back pain
8 PRAVALOTIN pravastatin sodium	Oral	2*20 Mg milligram(s)/Wee	-	Ill-defined disorder
9 RISPERDAL risperidone	Oral	1 Mg milligram(s)/Day	-	Ill-defined disorder

Drugs (concomitant)

10 Beloc Zok mite metoprolol succinate		1 Df dosage form/Day	-	
11 Marcumar phenprocoumon		/	20100701-20100722 22 Day	
12 simvastatin simvastatin		10 Mg milligram(s)/Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Venous insufficiency			Unknown	venous insufficiency
Coronary heart disease			Unknown	CHD

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time		7	%	
Alkaline phosphatase		133	U/L	
Blood amylase		110	U/L	
Blood bilirubin direct		106	mg/dL	
Blood bilirubin total	20100712	1.72	mmol/L	
GGT	20101123	53	U/L	
GGT	20100819	173	U/L	
GGT		168	U/L	
SGOT	20100913	125	U/L	
SGOT	20101123	102	U/L	
SGOT	20100817	462	U/L	
SGOT	20100712	462	U/L	
SGOT	20080812	38	U/L	
SGPT	20101123	67	U/L	
SGPT	20100913	77	U/L	
SGPT	20100816	401	U/L	
SGPT	20100712	401	U/L	
SGPT	20081216	18	U/L	
Lipase		92	U/L	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 14.10.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient developed MASSIVE HEPATITIS.
At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

Bayer Vital GmbH, 21.10.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient developed MASSIVE HEPATITIS.

At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

** Follow-up on 4-Oct-2010: Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiotics, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

Janssen-Cilag, Germany Local Safety Officers, 16.11.2010 CH-JNJFOC-20101101051

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (Swiss Agency for therapeutic products (Switzerland) - 2010-03148), and concerns a 77 year old female patient from Switzerland: NW-11.10-331-S.

The patient's weight was 88 kilograms and height was not reported. The patient's medical history and concurrent conditions included: chronic back pain and motion independent pain in right lateral upper leg. The patient had applied high doses of different analgesic drugs due to motion independent pain in right lateral upper leg and chronic back pain which included ibuprofen 600 mg 2 to 6 times, acetyl salicylate upto 1100 mg, metamizole sodium several times daily and paracetamol (unspecified, unknown) initiated on an unspecified date. The patient was treated with risperidone (unspecified, oral) 1 mg once a day, initiated on an unspecified date for unknown indication. Non-company suspect drugs also included: Alcacyl* (carbasalate calcium) for motion independent pain in right lateral upper leg and chronic back pain and pravastatin sodium for unknown indication. All drugs were applied for several weeks and it was suspected that the patient applied them for several months. Concomitant medications were not reported. The patient was admitted to a hospital due to a significant increase of liver values and hypokalemia on an unspecified date. The patient showed up at general practitioner due to vertigo, aggravation of walking disorder and dysesthesia of the legs. The right hypogastric zone was pressure sensitive and there was a labor-chemical finding of decreased spontaneous quick of 32 %, a remarkable increase of transaminases (Serum glutamate oxaloacetate transaminase: GOT 11869 U/L and serum glutamate pyruvate transaminase: GPT 2424 U/L), a clear increase of gamma glutamyl transpeptidase (gamma-GT) and creatine kinase (CK), a slight increase of inflammatory parameters and a hypokalemia of 2.5 mmol/L. The abdominal sonography showed an enlarged hepatopathy without intra and extrahepatic cholestasis. The abdominal computed tomography (CT) showed a remarkable liver steatosis. These findings were interpreted in the hospital as acute/subacute liver failure with a probable medical-toxic etiology. Treatment with risperidone, paracetamol, ibuprofen, carbasalate calcium and pravastatin sodium was withdrawn and action taken with metamizole sodium, acetylsalicylic acid and metamizole was not reported. The patient was recovering from hepatic failure and hepatic enzymes increased and outcome was not reported for hepatopathy, hypokalemia, liver steatosis, vertigo, aggravation of walking disorder and dysesthesia.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Bayer Vital GmbH, 24.11.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient experienced MASSIVE HEPATITIS.

At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** Follow-up on 4-Oct-2010:** Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiotics, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** FU 19-Nov-2010:**

Comments on the moxifloxacin questionnaire:

Drugs were administered from 12-Jul-2010 to 22-Jul-2010, once daily. Laboratory testing revealed infected crural ulcer. Investigations (ultrasound): no abnormal findings.

Hepatitis marker (A, B, C, D, E) were negative. The patient had no liver specific symptoms.

No specific concomitant drugs associated with liver events or other drugs were given. Medical history and concomitant diseases - no findings.

Quick's value dropped from 26% to 7% under Avalox.

Smear of foot ulcer revealed the presence of pseudomonas aeruginosa, staphylococcus aureus, peptostreptococcus anaerobius.

Further lab values were provided.

*****Bayer Schering Global comment (part one):**

Initial 08-Oct-2010: MASSIVE HEPATITIS (considered serious due to medical relevance by the company) is considered listed according to the CCDS of Avalox (moxifloxacin). Based on the limited provided information, a plausible causality assessment cannot be made. However, due to the fact that the event was reported to have occurred during treatment, a temporal relationship was implied and therefore, due to the lack of another explanation, a role of Avalox cannot be excluded. Further information has been requested (i.e. completion of Liver questionnaire).

****FU 14-Oct-2010:** Physician provided further details including increased ALT and AST values, and reported as onset date of the event, the day that Avalox treatment was started for an lateral malleolus ulcer with intermalleolar sepsis. An improvement was reported (no lab values provided) but it is unclear whether this was during continuing treatment or after completion of the Avalox therapy. Therefore the company assessment remains unchanged for the massive hepatitis. Additionally an INR DROP was reported to have occurred during antibiotics (retrieved from unstructured information by company). No INR lab values were provided. The patient was using Marcumar (phenprocoumon) concomitantly. Purely due to the statement that the event occurred during antibiotics, a temporal relationship is given and a role of Avalox cannot be excluded. However, antibiotics normally lead to an increase in INR, therefore a causal role of Avalox seems less likely. The most likely explanation of this patient's decreased INR might be an unreliable intake/dose change of his Marcumar or the underlying infection. Further follow-up request still pending (i.e. completion of questionnaire) and clarification of INR drop requested.

(cont.)

Bayer Vital GmbH, 19.01.2011 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

serious, related, listed

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient experienced MASSIVE HEPATITIS. At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** Follow-up on 4-Oct-2010:** Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiosis, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** FU 19-Nov-2010:**

Comments on the moxifloxacin questionnaire:

Drugs were administered from 12-Jul-2010 to 22-Jul-2010, once daily. Laboratory testing revealed infected crural ulcer. Investigations (ultrasound): no abnormal findings.

Hepatitis marker (A, B, C, D, E) were negative. The patient had no liver specific symptoms.

No specific concomitant drugs associated with liver events or other drugs were given. Medical history and concomitant diseases - no findings.

Quick's value dropped from 26% to 7% under Avalox.

Smear of foot ulcer revealed the presence of pseudomonas aeruginosa, staphylococcus aureus, peptostreptococcus anaerobius.

Further lab values were provided.

[Addendum <28-NOV-2010>: This case was converted from the Clintrace database into Argus. The medical content of the case was not changed.]

****Follow-up information was provided by the attending physician on 12-Jan-2011:**

Regarding the course of the adverse reaction, the physician reported about declining but still elevated transaminases. The female patient had not developed a sepsis, white blood count revealed 3.8, CRP normal findings. No statement was provided regarding the erythrocyte sedimentation rate. Marcumar / phenprocoumon treatment between 1 and 22-Jul-2010 followed the regimen 1/4, 1/4, 1/2, 1/4, 1/2, 1/2, 1/4.

The reporter considered the elevated liver values and the declining Quick's value as associated to the intake of Avalox/ moxifloxacin.

For further case information refer to section LAB DATA

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10163776

Serious Yes	Death Yes	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19241125	Age: 85 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20100802	deathcause: Liver failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20100802			22 Day	fatal
2 Reduced general condition	20100720			9 Day	recovering/resolving
3 Hepatocellular damage	20100720			9 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CITALOPRAM citalopram		/	-	Depressive disorder
2 DOMINAL /00018902/ prothipendyl hydrochloride		/	-	Drug use for unknown indication
3 FOSAMAX alendronate sodium		/	-	Osteoporosis postmenopausal
4 PARACETAMOL paracetamol		/	-	Chronic pain
5 Targin 5mg/2.5mg Retardtabletten naloxone hydrochloride; oxycodone hydrochloride	Oral	5 Mg milligram(s)/ 212	20100712- -	Pain

Drugs (concomitant)

6 Alendronsaeure alendronic acid	/	-
7 BISOPROLOL bisoprolol	/	-
8 FUROSEMID /00032601/ furosemide	/	-
9 MARCUMAR phenprocoumon	/	-
10 NOVAMINSULFON metamizole	/	-
11 NOVODIGAL /00017701/ digoxin	/	-
12 PRAVASTATIN pravastatin	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Yes	
Renal insufficiency			Yes	
Polyneuropathy			Yes	
Hyperuricaemia			Yes	
Hyperlipidaemia			Yes	
Senile dementia with depressive features			Yes	
Elastoplast allergy			Yes	
Allergic reaction to wasp sting			Yes	
Living in nursing home	2006		Yes	
International normalised ratio abnormal			Yes	
Depressive disorder			Yes	
Cholecystectomy	200802	200802	No	
Chronic pain			Yes	
Thoracic vertebral fracture T6	20100712	20100712	No	
Osteoporosis postmenopausal			Yes	
Chronic atrial fibrillation			Yes	
Heart insufficiency			Yes	
Coronary heart disease			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20091214	15	U/L	
Alanine aminotransferase	20100526	17	U/L	
Alanine aminotransferase	20100720	98.0	U/L	
Cholesterol total	20100526	210	mg/dL	
Cholesterol total	20100720	202	mg/dL	
Carcinoembryonic antigen	20100720	7.1	ng/ml	
C-reactive protein	20100720	18.7	mg/L	
Gamma-glutamyltransferase	20091214	64	U/L	
Gamma-glutamyltransferase	20100526	66	U/L	
Gamma-glutamyltransferase	20100720	1027.0	U/L	
GFR	20100720	40.4	ml/min	
INR	20100720	5.02	N/A	
LDL	20100720	125	mg/dL	
Sonogram	20100722	See text	N/A	

Narratives (nur bei elektronischem Eingang)

NAPPMUNDI, 25.10.2010 DE-NAPPMUNDI-DEU-2010-0006635

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2010-0006635 was received on 14OCT2010 from a physician in Germany. This spontaneous case relates to an 85 year-old female patient.

The patient's current conditions included chronic pain syndrome due to thoracic spine fracture 6, postmenopausal osteoporosis, chronic atrial fibrillation, cardiac failure, coronary disease, hypertension, chronic renal impairment, peripheral polyneuropathy, hyperuricaemia, hyperlipidaemia,

dementia with depressive mood, elaprepallergy, allergic reaction to wasp sting, living in nursing home, and INR [international normalised ratio] lapse due to administration of Marcumar during hepatic dysfunction. Her concomitant medication included Marcumar (phenprocoumon), Novodigal (digoxin), bisoprolol, pravastatin, Furosemid (furosemide), citalopram, Fosamax (alendronate), paracetamol, Dominal (prothipendyl), and Novaminsulfon (metamizole). The patient's medical history included cholecystectomy and slightly increased gamma-glutamyl transaminase. Historical medication was not reported.

The patient received TARGIN 5/2.5mg (oxycodone/naloxone, 5mg twice a day, started on 12JUL2010, stopped on an unspecified date) for her pain. On 20JUL2010 the patient experienced worsening of general condition. The patient was sent from nursing home to first aid at hospital with poor laboratory results, particularly liver values and worsening of general condition. She was hospitalised on 21JUL2010. Laboratory tests showed the following results: INR [international normalized ratio] of 5.02, CRP [C-reactive protein] of 18.7mg/L, GGT [gamma-glutamyltransferase] of 1027U/L, and GPT [Alanine aminotransferase] of 98U/L. The patient was accepted at hospital with reduced general condition, disoriented concerning time and place however not concerning people. A sonography of the abdomen on 22JUL2010 showed liver without cholestasis syndrome, without mass, no cirrhosis, normal findings. Condition after CCE [cholecystectomy]. The right adrenal gland strikingly enhanced with 3.5cm. Inpatient hospitalisation happened emergency through first aid department because of pathologic liver values with GGT greater than 1000U/L. Negative hepatitis serology, no ANA [antinuclear antibodies] and no AMA [anti-mitochondrial antibodies] as well as no evidence of hemochromatosis were found. Histological pathology at the hospital on 27JUL2010 showed liver values with minor to moderate hydropic-granular swelling of epithelia. No indication of fatty degeneration, hepatitis, cholangitis, cell atypia nor malignity. Sonography of hepatocellular damage was performed for further clarification with no findings. Hence a needle biopsy of the liver was done, the suspicion of a drug-induced hepatocellular damage was confirmed. After initial suspension of all medication with hepatic metabolism, liver values regressed over time. Medication with Fosamax was stopped completely. Also Dominal and citalopram, where hepatic dysfunction already has been described, could also be seen as reasonable for liver impairment. It was recommended to stop both drugs for the moment. Furthermore therapy with Paracetamol and TARGIN was stopped. Hypokalaemia was treated with dispersible tablets. The patient was treated with Clexane (enoxaparin), Concor (bisoprolol), Omeprazol (omeprazole), Kalinor-Brausetabletten [dispersible tablets] (potassium carbonate, potassium citrate) and Eunerpan (melperone), and was discharged from hospital on 29JUL2010, after improvement of her general condition. According to the hospital discharge report diagnosis of liver cell damage, most likely drug induced was made. The patient died on 02AUG2010. According to the physicians liver failure was assumedly the cause for the death. An autopsy was not planned.

The reporting physician found the adverse events to be probably related to the treatment with TARGIN because all other drugs were administered for quite a while before the adverse reaction happened and were well tolerated.

The doctors at hospital found the adverse events to be possibly related to the treatment with TARGIN but also possible related to the treatment with Fosamax, Dominal, citalopram, and paracetamol which could be the cause for the adverse events also.

This case was rated to be serious because of the fatal outcome and because of the life-threatening nature of all adverse events which had led to hospitalisation before.

No additional information is expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10173664

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19900827	Age: 20 Year	weight (kg): 62	BMI_int: 20.96	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic damage	20101012			3 Month	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 INTERFERON BETA interferon beta	Subcutaneous	3*44 µg microgram(s)/We	201007-20101012 3 Month	Multiple sclerosis
2 paracetamol paracetamol	Oral	/ 245	201007-20101012 3 Month	Flu like symptoms
3 rebif 44 interferon beta-1a	Subcutaneous	3*44 µg microgram(s)/We 120	201007-20101012 -	Multiple sclerosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Flu like symptoms	201007		Yes	known side effect when on rebif
Multiple sclerosis	201007		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin	20101015	9.87	mmol/l	
Haematocrit	20101015	0.46		
Mean corpuscular haemoglobin	20101015	89.8	fl	
Total white blood cell count	20101015	6.49	Gpt/l	
Total white blood cell count	20101022	6	Gpt/l	
Thrombocyte count	20101015	180	Gpt/l	
Quick's test	20101015	77.8	Prozent	
INR	20101015	1.12		
Activated partial thromboplastin time	20101015	33.4	sec	
Sodium	20101015	140	mmol/l	
Potassium	20101015	4.21	mmol/l	
Calcium	20101015	2.43	mmol/l	
Creatinine	20101015	65.4	µmol/l	
Urea	20101015	4.8	mmol/l	
Aspartate aminotransferase	20101015	16.06	µmol/s.l	
Aspartate aminotransferase	20101017	12.99	µmol/s.l	
Aspartate aminotransferase	20101022	11.92	µmol/s.l	
Alanine aminotransferase	20101015	34.42	µmol/s.l	

Alanine aminotransferase	20101017	29.67	µmol/s.l
Alanine aminotransferase	20101022	30.76	µmol/s.l
Alkaline phosphatase	20101015	2.15	µmol/l
Alkaline phosphatase	20101017	1.97	µmol/l
Alkaline phosphatase	20101022	2.09	µmol/l
Gamma GT	20101015	3.23	µmol/l.s
Gamma GT	20101017	3.19	µmol/l.s
Gamma GT	20101022	3.79	µmol/l.s
Bilirubin direct	20101015	10.3	µmol/l
Bilirubin direct	20101017	11.6	µmol/l
Bilirubin direct	20101022	20.9	µmol/l
C-reactive protein	20101015	2.1	mg/l
C-reactive protein	20101022	1.5	mg/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 10.11.2010 DE-PVZIBE-07-10407

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschaden, medikamentös-toxisch

KURZZUSAMMENFASSUNG:

Leberschaden unter Interferon- und Paracetamoleinnahme

BESCHREIBUNG:

Stationäre Einweisung wegen Leberschaden bei V.a. Nebenwirkung von Rebif 44. Seit Juli 2010 Therapie mit Rebif 44 wegen neu aufgetretener Multipler Sklerose. Im Rahmen der Rebif-Therapie erfolgte lt. anamnestischen Angaben wegen Verminderung der dafür bekannten grippeähnlichen Symptome die Anwendung von (wahrscheinlich!) Paracetamol.

THERAPIE:

- Rebif am 12.10.10 durch Hausarzt bereits abgesetzt
- klinische Überwachung

VERLAUF:

- Nur leichter Rückgang der Leberwerte bei klinischer Unauffälligkeit.
- Oberbauchsonographie: Normalbefund
- Ausschluss Hepatitis A, B, C, EBV negativ, Autoimmundiagnostik negativ.
- Entlassung mit Bitte um weitere Verlaufskontrollen und Meidung potentiell toxischer Genussmittel/Medikamente.

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief liegt ein medikamentös-toxischer Leberschaden unter Interferontherapie vor. In der Fachinformation werden für das verdächtige Medikament Rebif 44 Leberschädigungen nicht ausgeschlossen, die meisten Fälle schwerer Leberfunktionsstörungen traten lt. Fachinformation während der ersten sechs Behandlungsmonate auf. Relevante Alternativursachen (s. Medikamente) bzgl. des Leberschadens werden beim Patienten ausgeschlossen. Suspekt wird auch Paracetamol eingestuft. Leberschädigungen hierunter sind bekannt. Allerdings besteht keine absolute Sicherheit, ob Paracetamoleinnahme tatsächlich vorlag. Insgesamt ist für beide Medikamente Chronologie und Symptomatologie plausibel.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: rebif 44

BETREFFENDE UAW: Leberschaden

ERLÄUTERUNG: - Symptomatologie/Chronologie plausibel

- challenge: compatible
- dechallenge: suggestive

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Leberschaden

ERLÄUTERUNG: - Symptomatologie/Chronologie plausibel

- challenge: compatible
- dechallenge: suggestive

Einnahme erfolgte wahrscheinlich gelegentlich (lt. Empfehlung Fachinformation Rebif 44, um grippeähnliche Symptomatik zu vermindern). Lt. Angaben aus Anamnese keine Einnahme toxischer Dosierungen. Lt. Entlassungsbrief kann die gleichzeitige Einnahme Paracetamol/Rebif verstärkt zur Leberschädigung beigetragen haben.<dbtext>To_Char_NARRATIVE_2</dbtext>

TEVA, 19.11.2010 DE-TEVA-254997GER

Sender diagnosis: not reported

Narrative

10-Nov-2010, Spontaneous Serious report,

Case was received via the BFARM (DE-PVZIBE-07-10407;DE-BFARM-10173664). A Physician reported the case of a Male patient 20 Years of age who experienced TOXIC HEPATIC DAMAGE DRUG RELATED while on PARACETAMOL (Teva product(s) cannot be excluded) and REBIF 44 . At the time of the report the outcome of the AEs were: TOXIC HEPATIC DAMAGE DRUG RELATED : recovering/resolving.

NOTES TO THE FOLLOWING ADR: Toxic liver damage, drug-induced

EXECUTIVE SUMMARY: Liver damage due to interferon and paracetamol therapy

DESCRIPTION: Hospital admissions due to liver damage, suspected side effect of Rebif 44.

Since Jul-2010 therapy with Rebif 44 because of re-occurred Multiple sclerosis.

As part of Rebif therapy, according to medical history information to reduce known flu-like symptoms, the application of paracetamol is probable.

THERAPY: Rebif discontinued on 12-Oct-2010 due to general practitioner, clinical monitoring.

COURSE: Only slight decrease in liver enzymes while clinically without findings. - Upper abdominal ultrasonography: normal findings - excluding Hepatitis A, B, C, EBV negative, negative autoimmune diagnostics. Dismissal by requesting further follow-ups and avoiding potential toxic tobacco / drugs.

NOTES TO THE ADR: According dismissal letter a drug-toxic liver damage, among interferon therapy is indicated. In the SmPC for the drug Rebif 44 liver damage cannot be excluded, most cases of severe liver dysfunction were seen according to SmPC during the first six months of treatment.

Relevant alternative explanations (see medication) in terms of liver damage are excluded in the patient. Paracetamol is also suspected. Liver damages are known among paracetamol. However, it is not definitely certain that paracetamol was used by the patient. In summary, for both of the used drugs chronology and symptomatic are considered plausible.

NOTES TO THE CAUSALITY: Rebif 44 ADR: Liver damage NOTE: Symptomatology and chronology are plausible. Challenge compatible - dechallenge: suggestive intake probably as needed (according to SmPC to avoid flu-like symptoms). According to anamnesis no intake of toxic dosages. According to dismissal letter the concomitant use of Rebif and paracetamol can lead to increased liver damage, respectively

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: As contribution of Paracetamol cannot be excluded there is a possible causal relationship of Paracetamol and Toxic Liver damage.

This is a spontaneous case and therefore considered and handled as a related case.

sanofi-aventis, 24.11.2010 DE-SANOFI-AVENTIS-2010SA068427

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10173664) on 11-Nov-2010.

Short summary: liver damage under treatment with interferon and paracetamol.

The case involves a 20-year-old male patient suffering from multiple sclerosis and interferon-associated flu like symptoms. The patient started treatment with paracetamol (trade name not provided) in Jul. 2010 to decrease the flu like symptoms that are known to occur during treatment with interferon; paracetamol was probably taken irregularly in an unknown dose. Start of treatment with interferon beta 3x44 mcg/week for multiple sclerosis was in Jul. 2010, as well. No information was provided on concomitant medication.

On 12-Oct-2010, the patient was diagnosed to suffer from liver damage; treatment with interferon was discontinued by the patient's family doctor, the patient was admitted to hospital for monitoring. Paracetamol was withdrawn. Liver values decreased slightly; the patient showed no clinical symptoms. Outcome was reported as "recovering/resolving".

Causality determined by PV centre: "possible" for paracetamol and for interferon.

PV centre's comment on causality concerning paracetamol: "symptomatology/chronology plausible; challenge compatible, dechallenge suggestive; intake probably was infrequent; according to patient's medical history no intake of toxic doses. According to hospitalisation, concurrent intake of paracetamol/interferon could have increasingly contributed to the liver damage".

reporter's comment:

HELIOS Klinikum Wuppertal, 25.11.2010 DE-PVZIBE-07-10407

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberschaden, medikamentös-toxisch

KURZZUSAMMENFASSUNG:

Leberschaden unter Interferon beta-1a.

BESCHREIBUNG:

Stationäre Einweisung wegen Leberschaden bei V.a. Nebenwirkung von Rebif. Seit Juli 2010 Therapie mit Rebif wegen neu aufgetretener Multipler Sklerose. Im Rahmen der Rebif-Therapie erfolgte lt. anamnestischer Angaben wegen Verminderung der dafür bekannten grippeähnlichen Symptome die Anwendung von (wahrscheinlich!) Paracetamol.

THERAPIE:

- Rebif am 12.10.10 durch Hausarzt bereits abgesetzt
- klinische Überwachung

VERLAUF:

- Nur leichter Rückgang der Leberwerte bei klinischer Unauffälligkeit.
- Oberbauchsonographie: Normalbefund
- Ausschluss Hepatitis A, B, C, EBV negativ, Autoimmundiagnostik negativ.
- Entlassung mit Bitte um weitere Verlaufskontrollen und Meidung potenziell toxischer Genussmittel/Medikamente.
- der Pat. sollte am 25.10.10 nochmals zur poststationären Lborwertkontrolle vorstellig werden - ist aber nicht erschienen

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief liegt ein (Zitat:) "medikamentös-toxischer Leberschaden unter Interferontherapie" vor. In der Fachinformation werden für dies verdächtige Medikament Rebif Leberschädigungen nicht ausgeschlossen, die meisten Fälle schwerer Leberfunktionsstörungen traten lt. Fachinformation während der ersten sechs Behandlungsmonate auf. Relevante Alternativursachen (s. Medikamente) bzgl. des Leberschadens wurden beim Patienten ausgeschlossen. Problematisch ist die Beurteilung von Paracetamol, da keine sichere Einnahme angenommen werden kann (unterschiedliche anamnestische Aussagen). Leberschädigungen hierunter sind bekannt. Allerdings besteht keine absolute Sicherheit, ob Paracetamoleinnahme tatsächlich vorlag. Insgesamt sind zumindest für das Medikament Rebif die Chronologie und Symptomatologie plausibel.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: rebif 44

BETREFFENDE UAW: Leberschaden

ERLÄUTERUNG: - Symptomatologie/Chronologie plausibel

- challenge: compatible
- dechallenge: suggestive

MEDIKAMENT: paracetamol

BETREFFENDE UAW: Leberschaden

ERLÄUTERUNG: - Symptomatologie/Chronologie plausibel

- challenge: compatible
- dechallenge: suggestive

Einnahme erfolgte - wenn überhaupt - wahrscheinlich gelegentlich (lt. Empfehlung Fachinformation Rebif 44, um grippeähnliche Symptomatik zu vermindern). Lt. Angaben aus Anamnese keine Einnahme toxischer Dosierungen. Lt. Entlassungsbrief kann die gleichzeitige Einnahme Paracetamol/Rebif verstärkt zur Leberschädigung beigetragen haben. Aber insgesamt gibt es bei den anamnestischen Ausführungen widersprüchliche Aussagen des Pat., ob das Medikament tatsächlich eingenommen wurde.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11000454

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19760414	Age: 34 Year	weight (kg): 81	BMI_int: 28.36	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus					recovering/resolving
2 Hepatic encephalopathy	20100821			52 Day	recovered/resolved
3 Acute liver failure	20100821			52 Day	Unknown
4 Coagulopathy	20100821			52 Day	recovering/resolving
5 Nausea					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	/	20100701-20100812	Wound pain
paracetamol		245	43 Day	

Drugs (concomitant)

2 Homoeopathische Zubereitung	Unknown	/	-	Drug use for unknown indication
3 ibuprofen	Oral	/	20100701-20100812	Wound pain
ibuprofen		245	43 Day	
4 L-THYROXIN		/	-	Hypothyroidism
levothyroxine sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tonsillectomy			No	
Angiomyolipoma	201008		Unknown	
Hypothyroidism			Unknown	
Cholecystolithiasis			Unknown	
Cesarean section	20100701	20100701	No	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20100902	94	U/L	
Alkaline phosphatase	20100831	106	U/L	
Alkaline phosphatase	20100901	106	U/L	
Antimitochondrial antibody	2010	1:160	NA	
Bilirubin total	20100902	11.00	MG/DL	
Bilirubin total	20100901	12.40	MG/DL	
Bilirubin total	20100831	11.60	MG/DL	
Echocardiogram	2010	UNREMARKABLE	NA	
Gamma GT	20100831	89	U/L	
Gamma GT	20100901	105	U/L	

DE-BFARM-11000454			
Gamma GT	20100902	86	U/L
Liver biopsy	20100902	RESULTS NOT AVAILABLE	
SGOT	20100902	84	U/L
SGOT	20100901	96	U/L
SGOT	20100831	83	U/L
SGPT	20100902	138	U/L
SGPT	20100901	178	U/L
SGPT	20100831	189	U/L
Smooth muscle antibody	2010	SLIGHTLY INCREASED	
Ophthalmological examination	20100826	NO SIGNS OF WILSON'S DISEASE	
Ceruloplasmin	2010	0.16	G/L
Cytomegalovirus serology NOS	2010	NEGATIVE	NA
Epstein-Barr virus serology	2010	NEGATIVE	NA
Quick's test	20100902	62.1	%
Quick's test	20100901	59.9	%
Quick's test	20100831	56.0	%
Oesophagogastroduodenoscopy	20100824	REFLUX OESOPHAGITIS, GASTROPATHY 1ST DEG	
Urine copper	2010	SLIGHTLY INCREASED	
Hepatitis E antigen	2010	NEGATIVE	NA
Human herpes virus 6 serology	2010	NEGATIVE	NA
Herpes simplex serology	2010	NEGATIVE	NA
Hepatitis B virus test	2010	NEGATIVE	NA
Hepatitis C virus test	2010	NEGATIVE	NA
Hepatitis A virus test	2010	NEGATIVE	NA
Varicella virus test	2010	NEGATIVE	NA

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 14.01.2011 DE-SANOFI-AVENTIS-2011SA000522

Sender diagnosis: not reported

Narrative

Initial report:

On 05-Jan-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11000454); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 34-year-old female patient. The medical history included tonsillectomy, cholecystolithiasis and angiomyolipoma developed in Aug-2010. Concomitantly the patient used homeopathics. For wound pain the patient took paracetamol >2-3 g/day and ibuprofen until 800 mg/day from 01-Jul-2010 to 12-Aug-2010. On 21-Aug-2010 the patient suffered acute hepatic failure, coagulopathy and encephalopathy and was hospitalised. Corrective treatment was not provided and outcomes were unknown.

Only paracetamol was specified as suspected drug and all events were assessed with possible by the Drug Commission.

14-Jan-11 Sanofi-Aventis Germany

The reported diagnoses are considered locally unlabeled. As only limited information has been obtained so far, it is difficult to assess a cause and effect relationship for the suspected drug. To date, measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 15.02.2011 DE-SANOFI-AVENTIS-2011SA000522

Sender diagnosis: not reported

Narrative

Initial report:

On 05-Jan-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11000454); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 34-year-old female patient. The medical history included tonsillectomy, cholecystolithiasis and angiomyolipoma developed in Aug-2010. Concomitantly the patient used homeopathics. For wound pain the patient took paracetamol >2-3 g/day and ibuprofen until 800 mg/day from 01-Jul-2010 to 12-Aug-2010. On 21-Aug-2010 the patient suffered acute hepatic failure, coagulopathy and encephalopathy and was hospitalised. Corrective treatment was not provided and outcomes were unknown.

Only paracetamol was specified as suspected drug and all events were assessed with possible by the Drug Commission.

Additional information received from the DCGMA on 08-Feb-2011 (adverse reaction report form and hospital discharge report): The patient had a medical history of cholecystolithiasis and hypothyroidism. She had delivered a healthy female by Caesarean section on 01-Jul-2010 without complications. After the section, on 01-Jul-2010, she started treatment with paracetamol (trade name not provided) at least two to three grams daily, and sometimes additionally with ibuprofen up to 1600 mg/daily due to postoperative wound pain. Concomitant medication included unspecified homeopathic drugs and levothyroxine. The patient breastfed her child for about six weeks under medical treatment (see case number 2011SA007872).

After seven weeks of treatment with paracetamol, on 21-Aug-2010, the patient was hospitalised due to icterus and nausea; acute liver failure was diagnosed and the patient was transferred to the reporting hospital. Examination at time of admission showed cardiorespiratorily stable patient with hepatic encephalopathy of first degree. Serology was negative for CMV, EBV, HAV, HBV, HCV, HEV, HSV, HHV6, and VZV. Laboratory revealed a minimally increased titre for smooth muscles antibodies and anti-mitochondrial antibodies (to 1:160, normal range < 1:40), and negative result for AMA M2 (a biopsy for confirmation was not performed due to coagulopathy), decreased ceruloplasmin and twice slightly increased copper in urine (Wilson's disease was excluded by ophthalmologic examination); therefore, these values were seen as being caused by the acute liver failure. Upper abdominal sonographies on 26-Aug-2010 and 31-Aug-2010 showed a normal liver perfusion and homogenous structure, the assessment read "hepatomegaly with sonographic signs of hepatic parenchymal damage". Oesophagogastroduodenoscopy on 24-Aug-2010 revealed a reflux oesophagitis as well as a portal gastropathy of first degree. Echocardiography was unremarkable. During hospitalisation, liver transaminases regressed (GPT from 189 U/l on 31-Aug-2010 to 138 U/l on 02-Sep-2010, normal range up to 35, and GOT from 83 U/l on 31-Aug-2010 and 96 U/l on 01-Sep-2010 to 84 U/l on 02-Sep-2010, normal range up to 35), alkaline phosphatase decreased from 106 U/l on 31-Aug-2010 to 94 U/l on 02-Sep-2010, normal range 55 to 105), gamma GT from 89 U/l on 31-Aug-2010 and 105 U/l on 01-Sep-2010 to 96 U/l on 02-Sep-2010, normal range up to 40,. Bilirubin decreased from 11.60 mg/dl on 31-Aug-2010 and 12.40 mg/dl on 01-Sep-2010 to 11.00 mg/dl on 02-Sep-2010 (normal range up to 1.0). Quick, as sign of liver synthesis, began to increase from 56.0 % on 31-Aug-2010 to 62.1 % on 02-Sep-2010 (normal range 70 to 125). Results of liver puncture on 02-Sep-2010 was not available at time of discharge report. The patient underwent intensive medical care; corrective therapy consisted of supportive laxation, and an antidote treatment with acetylcysteine. Treatment with paracetamol, ibuprofen, and homeopathic drugs was discontinued. Breastfeeding was stopped upon admission to hospital. The patient had no further episode of encephalopathy during hospitalisation. She was discharged from hospital on 02-Sep-2010. The hospital physicians assumed a causal relationship with paracetamol, but not with ibuprofen or with the homeopathic medication. Outcome was reported as "improvement after discontinuation" and "worsening after renewed administration" (there was no elaboration as to which medication was re-introduced).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11011791

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Redness of legs	2010				recovered/resolved
2 Gamma-GT increased	201005				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Antidepressiva		/	-	Multiple sclerosis
2 ibuprofen		/	-	Pain
ibuprofen			-	
3 PARACETAMOL		/	-	Pain
paracetamol			-	
4 SIRDALUD		/	-	Multiple sclerosis
tizanidine hydrochloride			-	
5 Targin 20/10 mg Retardtabletten	Oral	20 Mg milligram(s)/	200911-201005	Pain
naloxone hydrochloride; oxycodone hydrochloride		212	6 Month	

Drugs (concomitant)

6 LAXATIVES		/	-
7 OMEPRAZOL	/00661201/	/	-
omeprazole			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Multiple sclerosis			Yes	
Pain			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase	200911	142	unk	
Gamma-glutamyltransferase	201005	1055	unk	
Gamma-glutamyltransferase	201007	422	unk	
Gamma-glutamyltransferase	201101	See notes	unk	

Narratives (nur bei elektronischem Eingang)

NAPPMUNDI, 24.01.2011 DE-NAPPMUNDI-DEU-2011-0006884

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2011-0006884 was received on 13JAN2011 from a physician in Germany via a company representative and additional information on 17JAN2011 from the initially reporting physician via a company representative. The new information received on 17JAN2011 made this case serious. This spontaneous report relates to a 46 year-old female patient.

The patient's current conditions included pain due to multiple sclerosis. Her concomitant medication included laxatives, paracetamol, omeprazole, and several other unspecified medication. The medical history was not reported. Her historical medication included Durogesic (fentanyl). In NOV2009 the patient was switched from Durogesic to Targin 20/10mg (oxycodone/naloxone, 20mg twice daily) due to increased constipation under Durogesic therapy. On an unspecified date she experienced red spots on her legs and her gamma-glutamyltransferase was at 142 [unit not reported] in NOV2009. In MAY2010 another blood analyses showed gamma-glutamyltransferase at 1055 [unit not reported]. The treatment with Targin, paracetamol, and omeprazole was discontinued. In JUL2010 her gamma-glutamyltransferase was at 422 [unit not reported] and the red spots were disappearing. At the time of this report the gamma-glutamyltransferase was normal and she had recovered from her red spots. The patient had been switched back to Durogesic.

The reporting physician found the increased gamma-glutamyltransferase to be possibly related to the treatment with Targin. A detailed causality assessment between the red spots and the treatment with Targin was not given.

This case was rated to be serious because of the medical significance of the adverse event "Gamma GT increased up to 1055 [unit not reported]". Additional information has been requested. The case will be updated accordingly should more information become available.

NAPPMUNDI, 27.05.2011 DE-NAPPMUNDI-DEU-2011-0006884

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2011-0006884 was received on 13JAN2011 from a physician in Germany via a company representative and additional information on 17JAN2011 from the initially reporting physician via a company representative. The new information received on 17JAN2011 made this case serious. This spontaneous report relates to a 46 year-old female patient.

The patient's current conditions included pain due to multiple sclerosis. Her concomitant medication included laxatives, paracetamol, omeprazole, and several other unspecified medication. The medical history was not reported. Her historical medication included Durogesic (fentanyl). In NOV2009 the patient was switched from Durogesic to Targin 20/10mg (oxycodone/naloxone, 20mg twice daily) due to increased constipation under Durogesic therapy. On an unspecified date she experienced red spots on her legs and her gamma-glutamyltransferase was at 142 [unit not reported] in NOV2009. In MAY2010 another blood analyses showed gamma-glutamyltransferase at 1055 [unit not reported]. The treatment with Targin, paracetamol, and omeprazole was discontinued. In JUL2010 her gamma-glutamyltransferase was at 422 [unit not reported] and the red spots were disappearing. At the time of this report the gamma-glutamyltransferase was normal and she had recovered from her red spots. The patient had been switched back to Durogesic.

The reporting physician found the increased gamma-glutamyltransferase to be possibly related to the treatment with Targin. A detailed causality assessment between the red spots and the treatment with Targin was not given.

This case was rated to be serious because of the medical significance of the adverse event "Gamma GT increased up to 1055 [unit not reported]".

***Additional information was received on 17MAY2011 from the initially reporting physician.

Accordingly the patient had an isolated gamma-glutamyltransferase increase which had normalised again. The patient was on a variety of concomitant medications also including Sirdalud (tizanidine), ibuprofen and antidepressants for her multiple sclerosis. All medications including Targin had been discontinued permanently.

The reporter found that the cause of the isolated gamma-glutamyltransferase increase was most likely due to the number of medications used by the patient.

No additional information is expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11034329

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19660316	Age: 44 Year	weight (kg): 65	BMI_int: 23.88	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	201012			1 Month	recovering/resolving
2 Hepatitis	201012			1 Month	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol-ratiopharm 500 mg Tabletten	Oral	500 Mg milligram(s)/ 245	201011-201012 31 Day	Postoperative pain
paracetamol				

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cholecystectomy	201011		No	
Biliary fistula	201012		No	
Smoker			Yes	
Choledocholithiasis	201012		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20101129	181		
Alkaline phosphatase	20110216	141		
Alkaline phosphatase	20110211	115		
Alkaline phosphatase	20101118	111		
Alkaline phosphatase	20110118	118		
Alkaline phosphatase	20110223	108		
Alkaline phosphatase	20101122	951		
Bilirubin	20110216	1,21		
Bilirubin	20101118	1,26		
Bilirubin	20101122	2,27		
Bilirubin	20110211	1,19		
Bilirubin	20110118	0.53		
Bilirubin	20110122	0,51		
Bilirubin	20101129	0.34		
Bilirubin	20110223	1,89		
GGT	20101118	258		
GGT	20110211	249		
GGT	20110216	281		
GGT	20110223	188		

GGT	20110118	182
GGT	20110122	154
GGT	20101129	333
GGT	20101122	1408
Glutamic-oxaloacetic transferase	20110118	339
Glutamic-oxaloacetic transferase	20101122	109
Glutamic-oxaloacetic transferase	20110216	1146
Glutamic-oxaloacetic transferase	20101129	17
Glutamic-oxaloacetic transferase	20110223	591
Glutamic-oxaloacetic transferase	20110211	691
Glutamic-oxaloacetic transferase	20110122	434
Glutamic-oxaloacetic transferase	20101118	24
GPT	20101122	73
GPT	20110223	320
GPT	20110122	463
GPT	20101129	27
GPT	20110216	1045
GPT	20101118	54
GPT	20110118	441
GPT	20110211	672

Narratives (nur bei elektronischem Eingang)

TEVA, 22.02.2011 DE-TEVA-267854GER

Sender diagnosis: not reported

Narrative

18-Feb-2011, Spontaneous Serious Report

A physician reported the case of a female patient 49 years of age who experienced TRANSAMINASES INCREASED while on Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL).

The patient was hospitalized in November 2010 due to increased AP, gamma-GT and malaise. Cholecystectomy was performed after ERCP revealed no stone formation. A biliary fistula occurred and was treated with stent insertion. After healing up the stent was removed in the mid of December 2010. At that time transaminases increased up to 600. It was concluded that transaminases increased because of the parallel existing choledocholithiasis. The patient was taking paracetamol 4-5 g daily after the cholecystectomy, however, the dosage was decreased considerably (exact dose not stated) in the mid of Dec-2010. At the time of this report the general practitioner detected a severe increase of transaminases (1000) again. It is not clearly stated if the patient is hospitalized again due to this event.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: contribution of Paracetamol cannot be excluded, nevertheless other reasons might have contributed to development of symptoms reported: Transaminases increased are possibly related to Paracetamol use.

This is a spontaneous case and therefore considered and handled as a related case.

TEVA, 21.03.2011 DE-TEVA-267854GER

Sender diagnosis: not reported

Narrative

18-Feb-2011, Spontaneous Serious Report

A physician reported the case of a female patient 49 years of age who experienced TRANSAMINASES INCREASED while on Paracetamol-ratiopharm 500 mg Tabletten (PARACETAMOL).

The patient was hospitalized in November 2010 due to increased AP, gamma-GT and malaise. Cholecystectomy was performed after ERCP revealed no stone formation. A biliary fistula occurred and was treated with stent insertion. After healing up the stent was removed in the mid of December

2010. At that time transaminases increased up to 600. It was concluded that transaminases increased because of the parallel existing choledocholithiasis. The patient was taking paracetamol 4-5 g daily after the cholecystectomy, however, the dosage was decreased considerably (exact dose not stated) in the mid of Dec-2010. At the time of this report the general practitioner detected a severe increase of transaminases (1000) again. It is not clearly stated if the patient is hospitalized again due to this event.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events))

Teva Comment: contribution of Paracetamol cannot be excluded, nevertheless other reasons might have contributed to development of symptoms reported: Transaminases increased are possibly related to Paracetamol use.

This is a spontaneous case and therefore considered and handled as a related case.

17-Mar-2011

Additional information including detailed lab data was received from physician. HEPATITIS was added as event. Event was declared as life-threatening by physician. Patient is recovering.

TEVA comment: Hepatitis and Transaminases increased are possibly related to Paracetamol.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11083498

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19681009	Age: 42 Year	weight (kg): 100	BMI_int: 29.54	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute hepatic failure	20110412				not recovered/not resolved
2 Liver function tests abnormal					not recovered/not resolved
3 Hepatotoxic effect	20110412				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	2*500 Mg milligram(s)/ Day	20110414-20110417	Pain NOS
paracetamol		245	4 Day	
2 ramipril	Oral	10 Mg milligram(s)/Day	20110325-20110418	STEMI
ramipril		245	25 Day	
3 simvastatin	Oral	40 Mg milligram(s)/Day	20110325-20110415	STEMI
simvastatin		82	22 Day	

Drugs (concomitant)

4 ass 100	Oral	100 Mg milligram(s)/ Day	20110325-	Stent insertion NOS
acetylsalicylic acid		245	-	
5 metoprolol	Oral	47.5 Mg milligram(s)/ Day	20110325-	STEMI
metoprolol		212	-	
6 plavix 75mg filmtabletten	Oral	75 Mg milligram(s)/ Day	20110325-	Stent insertion NOS
clopidogrel sulfate		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Stent insertion NOS	20110325	20110325	No	bei Z.n. STEMI
STEMI	20110325	20110325	No	Vorderwand mit Kammerflimmern und Reanimation
Pain			Yes	
Resuscitation	20110325	20110325	No	
Cholecystolithiasis			Yes	
Rehabilitation therapy			Yes	
Biliary sludge			Yes	
Cholecystitis			Yes	
Ventricular fibrillation	20110325	20110325	No	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20110419	71	µmol/l	
Creatinine	20110420	87	µmol/l	
Urea	20110419	5.6	mmol/l	
Urea	20110420	5.1	mmol/l	

Alanine aminotransferase	20110412	1.35	µkatal/l
Alanine aminotransferase	20110418	9.16	µkatal/l
Alanine aminotransferase	20110419	8.9	µkatal/l
Alanine aminotransferase	20110420	7.6	µkatal/l
Alanine aminotransferase	20110423	4.5	µkatal/l
Aspartate aminotransferase	20110412	0.64	µkatal/l
Aspartate aminotransferase	20110418	3.64	µkatal/l
Aspartate aminotransferase	20110419	2.8	µkatal/l
Aspartate aminotransferase	20110420	2.2	µkatal/l
Aspartate aminotransferase	20110423	1.2	µkatal/l
Bilirubin total	20110412	6.61	µmol/l
Bilirubin total	20110418	5.57	µmol/l
Bilirubin total	20110419	10.1	µmol/l
Bilirubin total	20110420	12.2	µmol/l
Alkaline phosphatase	20110418	2.86	µkatal/l
Alkaline phosphatase	20110420	2.7	µkatal/l
Alkaline phosphatase	20110423	2.4	µkatal/l
Gamma-glutamyltransferase	20110412	8.23	µkatal/l
Gamma-glutamyltransferase	20110418	15.02	µkatal/l
Gamma-glutamyltransferase	20110419	16	µkatal/l
Gamma-glutamyltransferase	20110420	16	µkatal/l
Gamma-glutamyltransferase	20110423	14	µkatal/l
Quick's test	20110419	122	Prozent
INR	20110419	1.0	NA
Haemoglobin	20110419	9.1	mmol/l
C-reactive protein	20110419	<3.1	mg/l
Glucose	20110419	5.1	mmol/l
Potassium	20110419	4.7	mmol/l
Potassium	20110420	4.6	mmol/l
Sodium	20110419	141	mmol/l
Sodium	20110420	139	mmol/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 09.05.2011 DE-PVZIBE-06-10442

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akutes, anikteres Leberversagen

KURZZUSAMMENFASSUNG:
akutes, anikteres Leberversagen unter Ramipril-, Simvastatin- und Paracetamol-Medikation

BESCHREIBUNG:

Der Patient befand sich vom 19.-23.04.2011 aufgrund eines #ED: akuter am ehesten medikamentös toxischer Leberschaden# in stationärer Behandlung. Die Aufnahme erfolgte am 19.04.2011 als Übernahme aus einer Rehabilitationsklinik wegen eines kontinuierlichen Anstiegs der Leberwerte ab dem 12.04.2011.

THERAPIE:

Absetzen von Ramipril und Simvastatin ab dem 19.04.2011

VERLAUF:

Sonographie vom 19.04.2011: leichte Hepatomegalie, sonographisch unauffälliges Leberparenchym; Schrumpfgallenblase bei chronischer

Cholezystitis bei Cholezystolithiasis und Gallenblasensludge.

Weitere Diagnostik: kein Anhalt für infektiöse Genese; Autoantikörper, AFP, Coeruloplasmin im Urin ohne positiven Befund; Hepatitis A+B+C-Sero negativ; CMV- und EBV-IgM negativ; IgG positiv; HIV 1/2-Serologie negativ

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 13.05.2011 DE-MERCK-1105DEU00039

Sender diagnosis: not reported

Narrative

Information has been received from a pharmacist via the German agency and a Regional Pharmacovigilance Center concerning a 42 year old male patient with pain and a history of stent placement after STEMI with ventricular fibrillation and resuscitation (on 25-MAR-2011) who, on 25-MAR-2011, was placed on therapy with simvastatin (manufacturer unknown), 40 mg tablet, 40 mg, once a day for the treatment of STEMI. Concomitant suspect therapy included ramipril, 5mg tablet, 5mg daily for the treatment of STEMI, therapy initiated on 25-MAR-2011 and acetaminophen, 500mg tablet, 500mg twice daily for the treatment of pain, therapy initiated on 14-APR-2011. Other concomitant therapy included aspirin, clopidogrel bisulfate (PLAVIX) and metoprolol.

On 12-APR-2011, during the stay in a rehabilitation facility, the patient developed an continuously increase of liver values. Values were reported as followed: ALT 1.35 mckatal/l; AST 0.64 mckatal/l; total bilirubin 6.61 mcmol/l; GGT 8.23 mckatal/l.

On 15-APR-2011 (also reported as on 19-APR-2011), therapy with simvastatin was discontinued. On 17-APR-2011, therapy with acetaminophen was discontinued. On 18-APR-2011 (also reported as on 19-APR-2011), therapy with ramipril was discontinued.

On 18-APR-2011, laboratory analysis revealed: ALT 9.16 mckatal/l; AST 3.64 mckatal/l; total bilirubin 5.57 mcmol/l; GGT 15.02 mckatal/l; AP 2.86 mckatal/l.

On 19-APR-2011, he was transferred to a hospital for clarification of increased liver values. On 19-APR-2011, laboratory analysis revealed: creatinine 71 mcmol/l; urea 5.6 mmol/l; ALT 8.9 mckatal/l; AST 2.8 mckatal/l; total bilirubin 10.1 mcmol/l; GGT 16 mckatal/l; Quick's test 122 %; INR 1.0; hemoglobin 9.1 mmol/l; CRP < 3.1 mg/l; Glucose 5.1 mmol/l; potassium 4.7 mmol/l; sodium 141 mmol/l. That same day, ultrasound showed slight hepatomeglia, acute liver failure, hepatic parenchyma without findings, contracted gallbladder with chronic cholecystitis at cholecystolithiasis and gallbladder sludge. On approximately 19-APR-2011, further diagnostic revealed: no evidence of an infection; autoantibodies and ceruloplasmin in urine within normal limits; Hepatitis A+B+C negative, CMV negative, HIV negative, EBV-IgM negative, IgG positive and AFP negative. The patient was diagnosed with acute anicteric liver failure.

On 20-APR-2011, laboratory analysis revealed: creatinine 87 mcmol/l; urea 5.1 mmol/l; ALT 7.6 mckatal/l; AST 2.2 mckatal/l; total bilirubin 12.2 mcmol/l; GGT 16 mckatal/l; AP 2.7 mckatal/l; potassium 4.6 mmol/l; sodium 139 mmol/l.

On 23-APR-2011, laboratory analysis revealed: ALT 4.5 mckatal/l; AST 1.2 mckatal/l; GGT 14 mckatal/l; AP 2.4 mckatal/l. That same day, the patient was discharged from hospital. He was reported as not recovered from acute anicteric liver failure.

The reporting pharmacist felt that acute anicteric liver failure was probably related to therapy with simvastatin (manufacturer unknown) and/or ramipril and/or acetaminophen.

Additional information has been requested.

sanofi-aventis, 13.05.2011 DE-SANOFI-AVENTIS-2011SA029109

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-11083498) on 11-May-2011.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case report refers to a 42-year-old male patient, whose past medical history included STEMI (of anterior wall with atrial fibrillation and reanimation) with subsequent stent insertion NOS on 25-Mar-2011. On the same day therapy was started with acetylsalicylic acid 100 mg/day, clopidogrel bisulfate 75 mg/day and metoprolol tartrate 47.5 mg/day (considered as non-suspected) as well as ramipril 5 mg twice daily and simvastatin 40 g/day on 25-Mar-2011 (suspected drugs). Since 12-Apr-2011 the increase of liver values was observed during the patients' stay in a rehabilitation center. There, additionally paracetamol 500 mg twice daily for pain NOS was started two days later. Due to further increase of the liver values simvastatin, paracetamol and ramipril were discontinued on 15-Apr-2011, 17-Apr-2011 and on 18-Apr-2011, respectively and the patient was hospitalised. Acute, anicteric hepatic failure caused by drug induced toxic liver damage was diagnosed on 19-Apr-2011. The sonography, performed on the same day, presented mild hepatomegaly, unsuspicious liver parenchyma. Concomitantly scleroatrophic cholecystitis due to cholecystolithiasis and biliary sludge were detected. Further corrective treatment was not mentioned; the patient had not yet recovered at date of report.

Reporter causality: probable for ramipril, simvastatin and paracetamol

009507513, 17.06.2011 DE-MERCK-1105DEU00039

Sender diagnosis: not reported

Narrative

Information has been received from a pharmacist via the German agency and a Regional Pharmacovigilance Center concerning a 42 year old male patient with pain and a history of stent placement after STEMI with ventricular fibrillation and resuscitation (on 25-MAR-2011) who, on 25-MAR-2011, was placed on therapy with simvastatin (manufacturer unknown), 40 mg tablet, 40 mg, once a day for the treatment of STEMI. Concomitant suspect therapy included ramipril, 5mg tablet, 5mg daily for the treatment of STEMI, therapy initiated on 25-MAR-2011 and acetaminophen, 500mg tablet, 500mg twice daily for the treatment of pain, therapy initiated on 14-APR-2011. Other concomitant therapy included aspirin, clopidogrel bisulfate (PLAVIX) and metoprolol.

On 12-APR-2011, during the stay in a rehabilitation facility, the patient developed an continuously increase of liver values. Values were reported as followed: ALT 1.35 mckatal/l; AST 0.64 mckatal/l; total bilirubin 6.61 mcmol/l; GGT 8.23 mckatal/l.

On 15-APR-2011 (also reported as on 19-APR-2011), therapy with simvastatin was discontinued. On 17-APR-2011, therapy with acetaminophen was discontinued. On 18-APR-2011 (also reported as on 19-APR-2011), therapy with ramipril was discontinued.

On 18-APR-2011, laboratory analysis revealed: ALT 9.16 mckatal/l; AST 3.64 mckatal/l; total bilirubin 5.57 mcmol/l; GGT 15.02 mckatal/l; AP 2.86 mckatal/l.

On 19-APR-2011, he was transferred to a hospital for clarification of increased liver values. On 19-APR-2011, laboratory analysis revealed: creatinine 71 mcmol/l; urea 5.6 mmol/l; ALT 8.9 mckatal/l; AST 2.8 mckatal/l; total bilirubin 10.1 mcmol/l; GGT 16 mckatal/l; Quick's test 122 %; INR 1.0; hemoglobin 9.1 mmol/l; CRP < 3.1 mg/l; Glucose 5.1 mmol/l; potassium 4.7 mmol/l; sodium 141 mmol/l. That same day, ultrasound showed slight hepatomeglia, acute liver failure, hepatic parenchyma without findings, contracted gallbladder with chronic cholecystitis at cholecystolithiasis and gallbladder sludge. On approximately 19-APR-2011, further diagnostic revealed: no evidence of an infection; autoantibodies and ceruloplasmin in urine within normal limits; Hepatitis A+B+C negative, CMV negative, HIV negative, EBV-IgM negative, IgG positive and AFP negative. The patient was diagnosed with acute anicteric liver failure.

On 20-APR-2011, laboratory analysis revealed: creatinine 87 mcmol/l; urea 5.1 mmol/l; ALT 7.6 mckatal/l; AST 2.2 mckatal/l; total bilirubin 12.2 mcmol/l; GGT 16 mckatal/l; AP 2.7 mckatal/l; potassium 4.6 mmol/l; sodium 139 mmol/l.

On 23-APR-2011, laboratory analysis revealed: ALT 4.5 mckatal/l; AST 1.2 mckatal/l; GGT 14 mckatal/l; AP 2.4 mckatal/l. That same day, the patient was discharged from hospital. He was reported as not recovered from acute anicteric liver failure.

The reporting pharmacist felt that acute anicteric liver failure was probably related to therapy with simvastatin (manufacturer unknown) and/or ramipril and/or acetaminophen.

Follow-up received on 14-JUN-2011. The hospital physicians reported that the manufacturer of simvastatin was not known. No additional information was provided.

Additional information is not expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11135358

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19610811	Age: 49 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Intentional overdose	201106				Unknown
2 Coma	201107				Unknown
3 Respiratory insufficiency	201107				Unknown
4 Renal function test abnormal	201107				Unknown
5 Quick's value decreased	201107				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 MARCUMAR phenprocoumon	Oral	/	-	Suicide attempt
2 PARACETAMOL paracetamol	Oral	/	-	Suicide attempt

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood carbon monoxide		1.6	%	
Carbon dioxide		33	%	
Quick's test	20110709	55	%	

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 13.07.2011 DE-MEDA-M-EU-2011070098

Sender diagnosis: not reported

Narrative

A report was received by the GIFTINFORMATIONSZENTRALE (German Drug and Poison Information Call Centre, reference no. AZ2011014911) on 07-Jul-2011.

A 49-year-old male patient attempted suicide by taking orally an unknown amount of Marcumar (phenprocoumon) along with an unknown amount of paracetamol, various unspecified substances, and he exposed himself to car exhausts. The patient developed coma, respiratory insufficiency, increased renal values and decreased Quick values. The symptoms were described as severe. At the hospital, the patient was intubated, ventilated, and monitored. Laboratory examinations showed the following: COHb: 1.6%, blood gases: pCO₂ 33%, and Quick value 55%. Transaminases were normal and drug screening was negative. The following diagnostics and therapies were recommended by the GIFTINFORMATIONSZENTRALE: further monitoring, ACC scheme, administration of vitamin K and, in case of increased transaminases administration of silybinin, and intensive care treatment. At the time of reporting, the outcome was unknown.

Increased renal values were probably caused by paracetamol and other substances. Regarding the events coma, respiratory insufficiency, and increased renal values the causality to Marcumar is assessed as unlikely. The causality with regard to intentional overdose is not assessed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11138447

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		

Patient

Initials: Sex: Female Birth: 19540724 Age: 56 Year weight (kg): 80 BMI_int: 29.74 Age fetus:

Patientdeath

deathdate: 20101216 deathcause: Acute hepatic failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pulmonary function test abnormal	20101201				Unknown
2 Encephalopathy	20101201				Unknown
3 Kidney failure	20101201				Unknown
4 Creatinine increased	20101201				Unknown
5 Prothrombin increased	20101201				Unknown
6 Bilirubinaemia	20101201				Unknown
7 SGPT increased	20101201				Unknown
8 SGOT increased	20101201				Unknown
9 Pruritus	20101201				Unknown
10 Exanthem	20101201				Unknown
11 Liver function test abnormal	20101201				Unknown
12 Acute hepatic failure	20101201	20101216	16 Day		fatal
13 Acidosis	20101201				Unknown
14 Multiorgan failure	20101201				Unknown
15 Vascular disorder NOS	20101201				Unknown
16 Consumptive coagulopathy	20101201				Unknown
17 DIC	20101201				Unknown
18 Septic shock	20101201				Unknown
19 Purpura	20101201				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMITRIPTYLIN amitriptyline	Oral	25 Mg milligram(s)/Day	-20101209	Intervertebral disc degeneration
2 KATADOLON flupirtine	Oral	1 Df dosage form/	20101101-20101206 36 Day	Sciatica
3 NAPROXEN naproxen	Oral	/	2008-	Intervertebral disc degeneration
4 PARACETAMOL paracetamol	Oral	1 G gram(s)/	20101201-20101206 6 Day	Pyrexia
5 Saroten Tabs 50mg Tabletten amitriptyline	Oral	25 Mg milligram(s)/Day	20101101-20101209 39 Day	Degenerative disc disease

Drugs (concomitant)

6 MetoHEXAL Succ 47,5 mg Retardtabletten metoprolol succinate	Oral	2 Df dosage form/	-	Hypertension arterial
7 novaminsulfon metamizole sodium	Oral	500 Mg milligram(s)/	20101207-20101209 3 Day	Ache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Shoulder operation			Unknown	Z.n. Schulteroperation rechts
Intervertebral disc protrusion			Unknown	
Spinal disorder			Unknown	
Lumboischialgia	201004		Unknown	links betonte Lumboischialgie nach Heben einer Patientin; seit Jahren Schmerzen in beiden Schultern
Pain NOS			Unknown	chronisches Schmerzsyndrom
Intervertebral disc degeneration			Unknown	
Sciatica			Unknown	
Pyrexia			Unknown	
Gastric ulcer			Unknown	
Ulcus ventriculi			Unknown	
Hypertension			Unknown	
Pain of extremities			Unknown	witterungsabhängige Svhmerzen in Fingern und Zehen

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20101212	1405	U/l	
GPT	20101212	1572	U/l	
Bilirubin	20101212		14.23 mg/l	
Albumin	20101212	23	g/l	
Quick's test	20101212	45.7	%	
INR	20101212	1.60	INR	
Haemoglobin	20101212	9.0	g/dl	
Alanine aminotransferase	20101212	1572	UNIT: IU	UNITLESS
Albumin	20101212	23	g/L	
Aspartate aminotransferase	20101212	1405	UNIT: IU	UNITLESS
Bilirubin	20101212	14.23	mg/dL	
Haemoglobin	20101212	9	g/dL	
International normalised ratio	20101212	1.6	UNITLESS	
Prothrombin time	20101212	45.7	%	

Narratives (nur bei elektronischem Eingang)

ROCHE, 27.07.2011 DE-ROCHE-791973

Sender diagnosis: not reported

Narrative

THIS CASE WAS RECEIVED BY GRUENENTHAL GMBH (GERMANY, 201113174) ON 22 JULY 2011

INITIAL INFORMATION FOR THIS SPONTANEOUS CASE WAS RECEIVED ON 19 JULY 2011 FROM A PHYSICIAN VIA REGULATORY AUTHORITY AND CONCERNS A 56 YEARS OLD FEMALE PATIENT OF WHO EXPERIENCED THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WHILE TREATED WITH NAPROXEN, AMITRIPTYLINE (AMITRIPTYLIN) FOR INTERVERTEBRAL DISC DEGENERATION, PARACETAMOL FOR PYREXIA AND FLUPIRTINE (KATADOLON) FOR SCIATICA.

NO CONCURRENT CONDITIONS WERE PROVIDED. THE PATIENT HAD HISTORY OF HYPERTENSION AND GASTRIC ULCER. NO CONCOMITANT MEDICATIONS AND PAST DRUGS WERE REPORTED.

ON AN UNSPECIFIED DATE OF 2008, THE PATIENT STARTED ORAL NAPROXEN TWICE DAILY (DOSE, FROM NOT REPORTED). 01 NOVEMBER 2010, THE PATIENT STARTED ORAL FLUPIRTINE (DOSE AND FREQUENCY NOT REPORTED). ON AN UNSPECIFIED DATE, THE PATIENT STARTED AMITRIPTYLINE 25 MG DAILY. ON 01 DECEMBER 2010, THE PATIENT STARTED ORAL PARACETAMOL 1GRAM. ON SAME DAY, THE PATIENT EXPERIENCED ACUTE HEPATIC FAILURE RESULTING IN HOSPITALISATION. ON 06 DECEMBER 2010, THERAPY WITH FLUPIRTINE AND PARACETAMOL WAS DISCONTINUED. ON 09 DECEMBER 2010, THERAPY WITH AMITRIPTYLINE WAS DISCONTINUED. ON 12 DECEMBER 2010, BLOOD TEST WAS DONE ALANINE AMINOTRANSFERASE 1572 IU/L (NORMAL RANGE: 35), ASPARTATE AMINOTRANSFERASE 1405 IU/L (NORMAL RANGE: 35), ALBUMIN 23 G/L (NORMAL RANGE: 35 TO 52), BLOOD BILIRUBIN 14.23 MG/DL (NORMAL RANGE: 1.20), HAEMOGLOBIN 9 G/DL (NORMAL RANGE: 12 TO 16), INTERNATIONAL NORMALIZED RATIO 1.60 AND PROTHROMBIN TIME 45.7 PERCENT (NORMAL RANGE: 70). ON 16 DECEMBER 2010, THE PATIENT DIED DUE TO ACUTE HEPATIC FAILURE. IT WAS REPORTED THAT IT WAS NOT KNOWN WHETHER AUTOPSY WAS DONE.

THERE WAS INSUFFICIENT INFORMATION REGARDING THERAPY ONGOING STATUS OF NAPROXEN AT THE TIME OF DEATH

THE PHYSICIAN DID NOT PROVIDE THE CAUSAL RELATIONSHIP FOR THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WITH NAPROXEN.

THE COMPANY ASSESSED THE EVENT ACUTE HEPATIC FAILURE TO BE POSSIBLY RELATED TO THE ADMINISTRATION OF NAPROXEN.

NO FURTHER INFORMATION WAS PROVIDED.

Bayer Vital GmbH, 28.07.2011 DE-BAYER-2011-065178

Sender diagnosis: not reported

Narrative

serious, unlisted/listed, related

This spontaneous case report received via license partner (Roche), on 22-Jul-2011, from a physician in Germany referring to a 56 year old female patient. The initial report was provided via regulatory authority to Gruenenthal GMBH which informed the case to Roche.

Patient's concurrent condition included hypertension, gastric ulcer, intervertebral disc degeneration, pyrexia and sciatica.

In 2008 she received NAPROXEN SODIUM for intervertebral disc degeneration and no additional product information was provided. On an unspecified date she also took AMITRIPTYLIN (amitriptyline) for the same indication. Drug was administered at a total daily dose of 25 mg oral. On 01-Dec-2010 the patient started PARACETAMOL at a daily dose of 1 g oral for pyrexia and she also administered KATADOLON (flupirtine maleate), since 01-Nov-2010 for sciatica.

On 01-Dec-2010, on the same day she started Paracetamol, the patient experienced ACUTE HEPATIC FAILURE and was hospitalized (laboratory tests were provided). On 06-Dec-2010 she discontinued the therapy with Katadolon and Paracetamol. On 09-Dec-2010 the Amitriptylin was withdrawn and on 16-Dec-2010 the patient died due to the acute hepatic failure. It was reported that it was not known whether autopsy was done.

The physician did not provide the causal relationship but Roche assessed the event to be possibly related to the administration of naproxen.

sanofi-aventis, 28.07.2011 DE-SANOFI-AVENTIS-2011SA046468

Sender diagnosis: not reported

Narrative

Initial report extracted from the web site of the German health authority (identification number DE-BFARM-11138447) on 20-Jul-2011. A physician reported via the Drug Commission of the German Medical Association (DCGMA) on a 56-year-old female patient with a history of arterial hypertension and ventricular ulcer.

The patient started treatment with amitriptyline (trade name not provided) 25 mg orally at nights on an unspecified date for degenerative disc disease. Also suspected were paracetamol (trade name not provided) 1g orally for fever (start of treatment on 01-Dec-2010), oral flupirtine daily for lumboschialgia (start of treatment on 01-Nov-2010), and naproxen 2x1 dosage form orally for degenerative disc disease (for three years). On 01-Dec-2010, the patient developed acute hepatic failure (no values were provided for that date); laboratory on 12-Dec-2010 revealed SGOT of 1405 U/l (normal ranges less than 35 U/l), GPT of 1572 U/l (normal ranges less than 35 U/l), bilirubin of 14.23 mg/dl (normal ranges less than 1.20 mg/dl), albumin of 23 g/l (normal ranges 35 to 52 g/l), Quick's test of 45.7 % (normal ranges greater than 70 %), INR of 1.60 (no normal ranges provided), and haemoglobin of 9.0 g/dl (normal ranges 12.0 to 16.0 g/dl).

Paracetamol and flupirtine were withdrawn on 06-Dec-2010, amitriptyline on 09-Dec-2010; action taken concerning naproxen was unknown.

On 16-Dec-2010, the patient died of acute hepatic failure; it was unknown whether an autopsy was performed.

Causality determined by DCGMA (method: WHO Global Introspection): "possible" for amitriptyline, paracetamol, and flupirtine, "unlikely" for naproxen. No further information, including concomitant medication, and corrective treatment, was provided.

Reporter's comment:

LUNDBECK, 29.07.2011 DE-LUNDBECK-DKLU1072189

Sender diagnosis: not reported

Narrative

Case reference number DKLU1072189, is a spontaneous case received from a health professional/pharmaceutical company bayer.

This case concerns a 56-year-old female administered Amitriptyline-Form-Unknown (amitriptyline hydrochloride) 25 mg/d from an unspecified date to 09/Dec/2010 for intervertebral disc degeneration.

Co-suspected medication included paracetamol 1 gr/d from 01/Dec/2010 for pyrexia, naproxen sodium unknown daily dosage from an unspecified date in 2008 for intervertebral disc degeneration and Katadolon (flupirtine maleate) unknown daily dosage from 01/Nov/2010 to 06/Dec/2010 for sciatica.

On 01/Dec/2010 after initiation of amitriptyline the patient developed acute hepatic failure and was hospitalised.

Investigations and laboratory findings showed: Albumin decreased, 12/Dec/2010: 23 g/dL (reference range 35-52); Bilirubin, 12/Dec/2010: 14.23 mg/dL (reference range 1.2); Hemoglobin decreased, 12/Dec/2010: 9 g/dL (reference range 12-16) PT, 12/Dec/2010: 45.7 % (normal reference 70); INR, 12/Dec/2010, 1.6; SGOT, 12/Dec/2010, 1405 IU/L (reference range 35); SGPT, 12/Dec/2010<. 1572 (reference range 35).

Amitriptyline was discontinued on 09/Dec/2010 eight days after event onset.

Treatment of the event was not given.

The patient died on 16/Dec/2010 of acute hepatic failure. It is unknown if an autopsy was done.

The patient's medical history comprised hypertension, gastric ulcer, intervertebral disc degeneration, pyrexia and sciatica.

Concomitant medication was not reported.

Additional information has been requested and if new relevant information is received the case will be re-submitted.

sanofi-aventis, 07.10.2011 DE-SANOFI-AVENTIS-2011SA046468

Sender diagnosis: not reported

Narrative

Initial report extracted from the web site of the German health authority (identification number DE-BFARM-11138447) on 20-Jul-2011. A physician reported via the Drug Commission of the German Medical Association (DCGMA) on a 56-year-old female patient with a history of arterial hypertension, ventricular ulcer, medial prolapse of lumbar disc L1-2, L2-3, L4-5 with chronic pain due to lumboischialgia and past history of surgery of right shoulder.

The patient started treatment with amitriptyline 25 mg orally at night on an unspecified date for degenerative disc disease. The patient started treatment with paracetamol (trade name not provided) 1g orally for fever on 01-Dec-2010 by herself.

She also took oral flupirtine daily for lumboischialgia (start of treatment on 01-Nov-2010), and naproxen 2x1 dosage form orally for degenerative disc disease (for three years); both drugs were also suspected. Concomitant medication included metoprolol (no therapy dates provided) and metamizole (as needed from Apr-2010, last intake from 07-Dec-2010 to 09-Dec-2010).

According to hospital report, the patient had her blood tested in Oct. 2010; at that time, her values had been unremarkable (results were not available to hospital physicians). In Apr. 2010, the patient had stayed on the Philippines for vacation without any events. From time to time, she had drunken natural diuretic tea bought in an Asian shop.

On 01-Dec-2010, the patient developed fever up to 40 degrees Celsius, initially chills, dizziness, headache and pain in extremities, and dysuria; fever was regressing in the further course.

On 07-Dec-2010, the patient was admitted to the hospital with a suspected diagnosis of spondylodiscitis.

Examination at admission to hospital did not reveal a definite icterus, nor did the patient show other dermatologic symptoms of liver disorder; liver and spleen were not palpably enlarged; the patient complained of decent tenderness in epigastrium, no foetor was present.

Laboratory values provided for 07-Dec-2010 concerned only urinalysis with picture of urinary tract infection.

One day later (08-Dec-2010), the patient developed generalised exanthema (starting on her trunk).

Abdominal sonography on 08-Dec-2010 led to the assessment: liver sonomorphologically unremarkable.

Dermatological consultation on 09-Dec-2010 led to the findings of maculopapular exanthema with (questionably) beginning cockades, especially on the anterior trunk, with massive pruritus.

Laboratory on 12-Dec-2010 revealed SGOT of 1405 U/l (normal ranges less than 35 U/l), GPT of 1572 U/l (normal ranges less than 35 U/l), bilirubin of 14.23 mg/dl (normal ranges less than 1.20 mg/dl), albumin of 23 g/l (normal ranges 35 to 52 g/l), Quick's test of 45.7 % (normal ranges greater than 70 %), INR of 1.60 (no normal ranges provided), and haemoglobin of 9.0 g/dl (normal ranges 12.0 to 16.0 g/dl).

SGOT increased up to 2215 U/l on 13-Dec-2010, SGPT up to 1821 U/l, bilirubin up to 16.95 (units not provided), leukocytes up to 18.7/nl, Quick's test from initially 91.3 down to 35.1 %. The patient's vigilance decreased (patient was tired, arousable, suffered from nausea with one-time vomiting, but showed no tremor). Additionally, the patient's renal function deteriorated with an increase in creatinine up to 1.93 mg/dl.

Marker for hepatitis A, B, and C were negative; HBV-DNA was not yet available at time of hospital discharge report.

Magnetic resonance tomography of lumbar spine (date not provided) did not confirm spondylodiscitis.

Peripheral blood slide on 13-Dec-2010 revealed erythrocytes of moderate anisocytosis with detection of several target cells as sign of liver cell damage; assessment: strong suspicion of liver cell alteration due to viral infection.

Paracetamol and flupirtine were withdrawn on 06-Dec-2010, amitriptyline on 09-Dec-2010, naproxen on an unknown date in Dec. 2010.

Corrective treatment included parenteral substitution with vitamin K, intravenous acetylcysteine, antibiotic therapy with ciprofloxacin and clarithromycin (due to C-reactive protein increase up to 200 mg/l. On 13-Dec-2010, intravenous treatment with terlipressin was started due to developing hepatorenal syndrome.

On 13-Dec-2010, the patient was transferred to intensive care unit (ICU) with a diagnosis of acute liver failure of unknown origin (differential diagnoses: drug-toxic (naproxen) or unclear (maybe viral) infection).

Examination at admission to ICU showed a patient with icteric sclerae and skin, very dry oral mucosa and lips, generalised maculopapular exanthema, no oedema.

In the further course, the patient showed progressive hepatic encephalopathy in the course of increasing lactate, so that she was put on the list for high-urgency liver transplantation. Under increasing level of arterial ammoniac, the patient developed a hepatic encephalopathy grade 3 with pronounced somnolence, necessitating a protective intubation on 14-Dec-2010. In the further course, the patient developed acute renal failure and metabolic acidosis, leading to continuous haemodialysis on 15-Dec-2010. The patient additionally suffered from pronounced purpura and severe catecholamine-dependent distributive shock leading to suspicion of accompanying sepsis with initially unknown focus. Laboratory tests showed severe acidosis, hyperlactataemia, and consumptive coagulopathy as signs of acute liver failure and secondary sepsis.

On 16-Dec-2010, the patient underwent laparotomy due to availability of a transplant; during laparotomy severe intestinal ischemia (possibly due to consumptive coagulopathy) was diagnosed as focus of septic shock. Therefore, liver transplantation was considered unpromising, operation was aborted, and the - at that time already very unstable - patient was transferred back to ICU. The patient died in the early afternoon of that day.

Main diagnosis as reported in ICU discharge letter: acute liver failure, most likely drug-induced (flupirtine, naproxen, paracetamol); secondary diagnoses: septic shock due to pronounced small intestinal ischemia, acute pulmonary failure, acute renal failure, consumptive intravascular coagulation with consumptive coagulopathy, hepatic encephalopathy.

As cause of death, acute liver failure was reported. No information was provided whether autopsy was performed.

Causality determined by DCGMA (method: WHO Global Introspection): "possible" for amitriptyline, paracetamol, and flupirtine, "unlikely" for naproxen.

Additional information received from the DCGMA by letter on 29-Sep-2011 (including two hospital reports).

Patient's birth date and initials were added. Medical history and concomitant medication were amended; course of event with examination results and corrective treatment was added. Amitriptyline was changed to non-company drug (Saroten).

Bayer Vital GmbH, 09.11.2011 DE-BAYER-2011-094580

Sender diagnosis: not reported

Narrative

serious/ related/ unlisted

This case is a spontaneous case report received from a physician via regulatory authority (case# DE-BFARM-11138447) from GERMANY on 04-OCT-2011 and refers to a 53 year old female patient.

The patient's medical history included chronic pain syndrome, lumboschialgia (more pronounced on the left side), degenerative damage of the vertebral column, intervertebral disk protrusion, arterial hypertension.

On 01-NOV-2010 the patient started SAROTEN (AMITRIPTYLINE) at 25 mg oral for chronic pain. It was unknown whether SAROTEN was used previously.

The patient received co-suspect medications: On 01-NOV-2010 the patient started FLUPIRTINE MALEATE (KATADOLON) for pain., PARACETAMOL for fever. On an unspecified date the patient started NAPROXEN (Naproxen) for pain and METAMIZOLE SODIUM (Metamizol). On 07-DEC-2010 the patient started METAMIZOLE SODIUM (Novaminsulfon) at 500 mg, PRN for pain.

Due to significantly elevated inflammation parameters the patient was admitted to hospital on a not reported date. At clinic, the increased transaminases and cholestasis parameters. One day after admission the patient developed MACULOPAPULAR EXANTHEMA and GENERALIZED PRURITUS (initially expanding over the abdomen, later on generalized with massive itching). The entire medication was discontinued except for metoprolol. In the further course the patient developed LIVER FAILURE. The patient received vitamin K and acetylcysteine. The development of HEPATORENAL SYNDROME was treated with Terlipressin iv. Furthermore, due to a suspicion of acute infection with increased CRP levels up to 200 mg/l antibiotic treatment was initiated. A aggravating HEPATIC ENCEPHALOPATHY with rising lactate levels provided the indication of liver transplantation. In the further course the patient became somnolent requiring preventive intubation. Acute pulmonary failure, acute renal failure and metabolic acidosis occurred. The further therapy included continual hemodialysis. Marked purpura ,severe shock (with suspicion of accompanying

sepsis), DISSEMINATED INTRAVASCULAR COAGULATION and acute liver failure occurred. During laparotomy for liver transplantation the patient developed marked INTESTINAL ISCHEMIA ,SEPTIC SHOCK. On 16-DEC-2010 the patient died a few hours after discontinuation of surgery.

The patient died on 16-DEC-2010. The reported cause of death was SEPTIC SHOCK, MULTI-ORGAN FAILURE, HEPATIC ENCEPHALOPATHY AND LIVER FAILURE.

SAROTEN was discontinued on 09-DEC-2010. KATADOLON was discontinued on 06-DEC-2010. PARACETAMOL was discontinued on 06-DEC-2010. Novaminsulfon was discontinued on 09-DEC-2010.

No assessment was given.

** Internal correction done on 24-Oct-2011:
Age of patient was corrected to 56 years old.

Bayer Vital GmbH, 09.11.2011 DE-BAYER-2011-065178

Sender diagnosis: not reported

Narrative

serious, unlisted, related

This spontaneous case report received via license partner (Roche), on 22-Jul-2011, from a physician in Germany referring to a 56 year old female patient. The initial report was provided via regulatory authority to Gruenthal GMBH which informed the case to Roche.

Patient's concurrent condition included hypertension, gastric ulcer, intervertebral disc degeneration, pyrexia and sciatica.

In 2008 she received NAPROXEN SODIUM for intervertebral disc degeneration and no additional product information was provided. On an unspecified date she also took AMITRIPTYLIN (amitriptyline) for the same indication. Drug was administered at a total daily dose of 25 mg oral. On 01-Dec-2010 the patient started PARACETAMOL at a daily dose of 1 g oral for pyrexia and she also administered KATADOLON (flupirtine maleate), since 01-Nov-2010 for sciatica.

On 01-Dec-2010, on the same day she started Paracetamol, the patient experienced ACUTE HEPATIC FAILURE and was hospitalized (laboratory tests were provided). On 06-Dec-2010 she discontinued the therapy with Katadolon and Paracetamol. On 09-Dec-2010 the Amitriptylin was withdrawn and on 16-Dec-2010 the patient died due to the acute hepatic failure.
It was reported that it was not known whether autopsy was done.

The physician did not provide the causal relationship but Roche assessed the event to be possibly related to the administration of naproxen.

**Follow-up information received on 28-Jul-2011
Lundbeck reference number received: DKLU1072189

Addendum on 09-Oct-2011 During an internal review case 2011-094580 was identified as a duplicate to this case. All additional information has been transferred from 2011-094580 to this case. 2011-094580 will be deleted:

**Follow-up received on 04-Oct-2011, from a physician via regulatory authority (case# DE-BFARM-11138447). Additional information was received on 17, 20 and 21-Oct-2011.

Patient details were updated. Chronic pain syndrome, lumboschialgia (more pronounced on the left side) and intervertebral disk protrusion were added as medical history. Additionally, Metohexal Succ 47.5 mg retard was added as concomitant medication.

Pain was added as indication for NAPROXEN SODIUM.

The suspect drug's name was revised from "amitriptyline" to SAROTEN (amitriptyline), started on 01-Nov-2010 for chronic pain.

The dose of Katadolon was updated to 1 dose form daily. Patient also received Novaminsulfon on 07-Dec-2010, at a total dose of 500 mg as needed, for pain and Metamizol since Apr-2010.

Due to significantly elevated inflammation parameters the patient was admitted to hospital. At clinic, there were increased transaminases and cholestasis parameters (Lab tests of Billirubi, C-reactive protein, Creatinine, PT level (Quick) provided). One day after admission the patient developed MACULOPAPULAR EXANTHEMA and GENERALIZED PRURITUS (initially expanding over the abdomen, later on generalized with massive itching). The entire medication was discontinued except for metoprolol. In the further course the patient developed ACUTE HEPATIC FAILURE on 01-Dec-2010. The patient received vitamin K and acetylcysteine. The development of HEPATORENAL SYNDROME was treated with Terlipressin iv. Furthermore, due to a suspicion of acute infection with increased CRP levels up to 200 mg/l antibiotic treatment was initiated. An aggravating HEPATIC ENCEPHALOPATHY with rising lactate levels provided the indication of liver transplantation. In the further course the patient became somnolent requiring preventive intubation. Acute pulmonary failure, acute renal failure and metabolic acidosis occurred. The further therapy included continual hemodialysis. Marked purpura, severe shock (with suspicion of accompanying sepsis) and DISSEMINATED INTRAVASCULAR

COAGULATION occurred. During laparotomy for liver transplantation the patient developed marked INTestinal ISCHEMIA, SEPTIC SHOCK. On 16-Dec-2010 the patient died a few hours after discontinuation of surgery.

The reported cause of death was SEPTIC SHOCK, MULTI-ORGAN FAILURE, HEPATIC ENCEPHALOPATHY AND ACUTE HEPATIC FAILURE.

No assessment was given.

ROCHE, 01.12.2011 DE-ROCHE-791973

Sender diagnosis: not reported

Narrative

THIS CASE WAS RECEIVED BY GRUENENTHAL GMBH (GERMANY, 201113174) ON 22 JULY 2011

INITIAL INFORMATION FOR THIS SPONTANEOUS CASE WAS RECEIVED ON 19 JULY 2011 FROM A PHYSICIAN VIA REGULATORY AUTHORITY AND CONCERNS A 56 YEARS OLD FEMALE PATIENT OF WHO EXPERIENCED THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WHILE TREATED WITH NAPROXEN, AMITRIPTYLINE (AMITRIPTYLIN) FOR INTERVERTEBRAL DISC DEGENERATION, PARACETAMOL FOR PYREXIA AND FLUPIRTINE (KATADOLON) FOR SCIATICA.

NO CONCURRENT CONDITIONS WERE PROVIDED. THE PATIENT HAD HISTORY OF HYPERTENSION AND GASTRIC ULCER. NO CONCOMITANT MEDICATIONS AND PAST DRUGS WERE REPORTED.

ON AN UNSPECIFIED DATE OF 2008, THE PATIENT STARTED ORAL NAPROXEN TWICE DAILY (DOSE, FROM NOT REPORTED). 01 NOVEMBER 2010, THE PATIENT STARTED ORAL FLUPIRTINE (DOSE AND FREQUENCY NOT REPORTED). ON AN UNSPECIFIED DATE, THE PATIENT STARTED AMITRIPTYLINE 25 MG DAILY. ON 01 DECEMBER 2010, THE PATIENT STARTED ORAL PARACETAMOL 1GRAM. ON SAME DAY, THE PATIENT EXPERIENCED ACUTE HEPATIC FAILURE RESULTING IN HOSPITALISATION. ON 06 DECEMBER 2010, THERAPY WITH FLUPIRTINE AND PARACETAMOL WAS DISCONTINUED. ON 09 DECEMBER 2010, THERAPY WITH AMITRIPTYLINE WAS DISCONTINUED. ON 12 DECEMBER 2010, BLOOD TEST WAS DONE ALANINE AMINOTRANSFERASE 1572 IU/L (NORMAL RANGE: 35), ASPARTATE AMINOTRANSFERASE 1405 IU/L (NORMAL RANGE: 35), ALBUMIN 23 G/L (NORMAL RANGE: 35 TO 52), BLOOD BILIRUBIN 14.23 MG/DL (NORMAL RANGE: 1.20), HAEMOGLOBIN 9 G/DL (NORMAL RANGE: 12 TO 16), INTERNATIONAL NORMALIZED RATIO 1.60 AND PROTHROMBIN TIME 45.7 PERCENT (NORMAL RANGE: 70). ON 16 DECEMBER 2010, THE PATIENT DIED DUE TO ACUTE HEPATIC FAILURE. IT WAS REPORTED THAT IT WAS NOT KNOWN WHETHER AUTOPSY WAS DONE.

THERE WAS INSUFFICIENT INFORMATION REGARDING THERAPY ONGOING STATUS OF NAPROXEN AT THE TIME OF DEATH

THE PHYSICIAN DID NOT PROVIDE THE CAUSAL RELATIONSHIP FOR THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WITH NAPROXEN.

THE COMPANY ASSESSED THE EVENT ACUTE HEPATIC FAILURE TO BE POSSIBLY RELATED TO THE ADMINISTRATION OF NAPROXEN.

NO FURTHER INFORMATION WAS PROVIDED.

Additional information was received at Bayer pharma on 04 October 2011 and at Roche on 09 November 2011.

Follow-up received on 04-Oct-2011, from a physician via regulatory authority (case# DE-BFARM-11138447). Additional information was received on 17, 20 and 21 October 2011.

Patient details were updated. Chronic pain syndrome, lumboschialgia (more pronounced on the left side) and intervertebral disc protrusion were added as medical history. Concurrent conditions included intervertebral disc degeneration, pyrexia, sciatica, hypertension (previously reported as past history) and gastric ulcer (previously reported as past history). Additionally, metoprolol tartarate 47.5 mg retard was added as concomitant medication. Pain was added as indication for naproxen sodium.

The suspect drugs name was revised from amitriptyline to Saroten (amitriptyline), started on 01-Nov-2010 for chronic pain. On 09 December 2010, amitriptyline was stopped. The dose of Katadolon was updated to 1 dose form daily. On an unspecified date in April 2010, the patient received metamizole sodium (metamizol). On 06 December 2010, flupirtine maleate and paracetamol was stopped. On 07 December 2010, patient also received metamizole sodium (Novaminsulfon) at a total dose of 500 mg as needed, for pain. On 09 December 2010, metamizole sodium (Novaminsulfon) was stopped. Due to significantly elevated inflammation parameters the patient was admitted to hospital. At clinic, there were increased transaminases and cholestasis parameters (Lab tests of Billirubin, C-reactive protein, Creatinine, PT level (Quick) provided). One day after admission the patient developed maculopapular exanthema and generalised pruritus (initially expanding over the abdomen, later on generalized with massive itching). The entire medication was discontinued except for metoprolol. In the further course the patient developed acute hepatic failure on 01-Dec-2010. The patient received vitamin K and acetylcysteine. Sonography showed homogenous liver without space occupying lesion and no dilated bile ducts. Peripheral blood smear showed erythrocytes featuring moderate anisocytosis and numerous target cells indicating liver cell damage (unless no thalassemia exists). Serology was negative for markers of Hepatitis A, B and C. The development of hepatorenal syndrome was treated with intravenous Terlipressin. Furthermore, due to a suspicion of acute infection with increased CRP levels up to 200 mg/l antibiotic treatment was initiated. An aggravating hepatic encephalopathy with rising lactate levels provided the indication of liver transplantation. In the further course the patient became somnolent requiring preventive intubation. Acute pulmonary failure, acute renal failure and metabolic acidosis occurred. The further therapy included continual hemodialysis. Marked purpura, severe shock (with suspicion of accompanying sepsis) and disseminated intravascular

coagulation occurred. During laparotomy for liver transplantation the patient developed marked intestinal ischemia and septic shock. On 16-Dec-2010 the patient died a few hours after discontinuation of surgery.

The reported cause of death were septic shock, multi-organ failure, hepatic encephalopathy and acute hepatic failure.

The outcome of hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia was not reported.

The reporter assessed events of hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia as medically significant and life threatening.

The reporter did not provide the causal relationship between the events of fatal septic shock, fatal multi-organ failure, fatal hepatic encephalopathy, hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia with the drugs, naproxen, amitriptyline, flupirtine maleate, paracetamol and metamizole sodium.

ROCHE, 07.12.2011 DE-ROCHE-791973

Sender diagnosis: not reported

Narrative

THIS CASE WAS RECEIVED BY GRUENENTHAL GMBH (GERMANY, 201113174) ON 22 JULY 2011

INITIAL INFORMATION FOR THIS SPONTANEOUS CASE WAS RECEIVED ON 19 JULY 2011 FROM A PHYSICIAN VIA REGULATORY AUTHORITY AND CONCERNS A 56 YEARS OLD FEMALE PATIENT OF WHO EXPERIENCED THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WHILE TREATED WITH NAPROXEN, AMITRIPTYLINE (AMITRIPTYLIN) FOR INTERVERTEBRAL DISC DEGENERATION, PARACETAMOL FOR PYREXIA AND FLUPIRTINE (KATADOLON) FOR SCIATICA.

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THE PHYSICIAN DID NOT PROVIDE THE CAUSAL RELATIONSHIP FOR THE FATAL AND LIFE-THREATENING EVENT OF ACUTE HEPATIC FAILURE WITH NAPROXEN.

THE COMPANY ASSESSED THE EVENT ACUTE HEPATIC FAILURE TO BE POSSIBLY RELATED TO THE ADMINISTRATION OF NAPROXEN.

NO FURTHER INFORMATION WAS PROVIDED.

Additional information was received at Bayer pharma on 04 October 2011 and at Roche on 09 November 2011.

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01-Dec-2010. The patient received vitamin K and acetylcysteine. Sonography showed homogenous liver without space occupying lesion and no dilated bile ducts. Peripheral blood smear showed erythrocytes featuring moderate anisocytosis and numerous target cells indicating liver cell damage (unless no thalassemia exists). Serology was negative for markers of Hepatitis A, B and C. The development of hepatorenal syndrome was treated with intravenous Terlipressin. Furthermore, due to a suspicion of acute infection with increased CRP levels up to 200 mg/l antibiotic treatment was initiated. An aggravating hepatic encephalopathy with rising lactate levels provided the indication of liver transplantation. In the further course the patient became somnolent requiring preventive intubation. Acute pulmonary failure, acute renal failure and metabolic acidosis occurred. The further therapy included continual hemodialysis. Marked purpura, severe shock (with suspicion of accompanying sepsis) and disseminated intravascular coagulation occurred. During laparotomy for liver transplantation the patient developed marked intestinal ischemia and septic shock. On 16-Dec-2010 the patient died a few hours after discontinuation of surgery.

The reported cause of death were septic shock, multi-organ failure, hepatic encephalopathy and acute hepatic failure.

The outcome of hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia was not reported.

The reporter assessed events of hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia as medically significant and life threatening.

The reporter did not provide the causal relationship between the events of fatal septic shock, fatal multi-organ failure, fatal hepatic encephalopathy, hepatorenal syndrome, disseminated intravascular coagulation and intestinal ischemia with the drugs, naproxen, amitriptyline, flupirtine maleate, paracetamol and metamizole sodium.

Following review on 05/Dec/2011, following information, originally received on 04/Oct/2011, was corrected: The events maculopapular exanthema and generalized pruritus was added in event tab and causality and labelling part was completed.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-11144001

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19750415	Age: 36 Year	weight (kg): 68	BMI_int: 19.66	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rash	20110520	20110605	17 Day		recovered/resolved
2 Hepatotoxicity	20110531				recovering/resolving
3 Hepatic failure	20110530				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BEVACIZUMAB bevacizumab	Intravenous (not otherwis	730 Mg milligram(s)/	20110518-20110518	Metastatic renal cell carcinoma
2 BEVACIZUMAB bevacizumab	Intravenous (not otherwis	730 Mg milligram(s)/	20110504-20110504	Metastatic renal cell carcinoma
3 BEVACIZUMAB bevacizumab	Intravenous (not otherwis	730 Mg milligram(s)/	20110420-20110420	Metastatic renal cell carcinoma
4 BEVACIZUMAB bevacizumab	Intravenous (not otherwis	780 Mg milligram(s)/2 We	20110406-20110406	Metastatic renal cell carcinoma
5 INTERFERON ALFA-2A interferon alfa-2a	Subcutaneous	3 Miu iu(1,000,000s)/2 Da	20110406-20110529	Metastatic renal cell carcinoma
6 PARACETAMOL paracetamol	Unknown	/	20110406- 54 Day	Drug use for unknown indication

Drugs (concomitant)

7 INTRONA interferon alfa-2b		/	20110404- -	
8 kortikosteroide	Oral	/	- -	
9 NOVALGIN metamizole sodium		/	20110404- -	
10 TAVEGIL clemastine		/	- -	
11 ZOMETA zoledronic acid	Intravenous (not otherwis	4 Mg milligram(s)/4 Week	20110228- -	Bone metastases

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bone metastases			Unknown	
Renal tumour excision	20110318		Unknown	
Hand repair operation	2009		Unknown	
Liver metastases			Unknown	
Renal cell carcinoma			Unknown	
Wasting			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Consumer or other non health professional	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time	20110531	42	sec	
Activated partial thromboplastin time	20110601	42	sec	
Alanine aminotransferase	20110228	24	U/L	
Alanine aminotransferase	20110518	112	U/L	
Alanine aminotransferase	20110531	2445	U/L	
Alanine aminotransferase	20110602	1670	U/L	
Alanine aminotransferase	20110606	577	U/L	
Alanine aminotransferase	20110610	388	U/L	
Alkaline phosphatase	20110228	235	U/L	
Alkaline phosphatase	20110518	148	U/L	
Alkaline phosphatase	20110531	644	U/L	
Alkaline phosphatase	20110602	508	U/L	
Alkaline phosphatase	20110606	376	U/L	
Alkaline phosphatase	20110610	460	U/L	
ANA	20110601	< 1:80 (NORMAL)	UNITLESS	
ANCA	20110601	C-ANCA < 1:10 (NORMAL)	UNITLESS	
ANCA	20110601	P-ANCA < 1:10 (NORMAL)	UNITLESS	
Aspartate aminotransferase	20110228	25	U/L	
Aspartate aminotransferase	20110518	94	U/L	
Aspartate aminotransferase	20110531	3541	U/L	
Aspartate aminotransferase	20110602	1498	U/L	
Aspartate aminotransferase	20110606	203	U/L	
Aspartate aminotransferase	20110610	248	U/L	
Bilirubin total	20110531	16.9	mg/dL	
Bilirubin total	20110602	18.5	mg/dL	
Bilirubin total	20110606	13.2	mg/dL	
Bilirubin total	20110610	11.5	mg/dL	
C-reactive protein	20110601	3.3	mg/dL	
Creatinine	20110228	1.04	mg/dL	
Creatinine	20110518	1.15	mg/dL	
Creatinine	20110601	.8	mg/dL	
Fibrin D dimer	20110531	1176	ng/mL	
GGT	20110228	39	U/L	
GGT	20110518	155	U/L	
GGT	20110531	843	U/L	
GGT	20110602	514	U/L	
GGT	20110606	653	U/L	
GGT	20110610	583	U/L	
Haematocrit	20110531	35.6	%	
Haematocrit	20110610	36.2	%	
Haemoglobin	20110531	12.1	g/dL	

	DE-BFARM-11144001		
Haemoglobin	20110610	12.3	g/dL
INR	20110531	1.6	UNITLESS
INR	20110601	2.4	UNITLESS
LDH	20110228	189	U/L
LDH	20110518	301	U/L
LDH	20110601	2621	U/L
Lipase	20110601	1491	U/L
Lipase	20110610	78	U/L
Liver-kidney microsomal antibody	20110601	< 1:40 (NORMAL RANGE)	
MRI	20110623	SEE LAB RESULTS	
Protein total	20110531	5.8	g/dL
Ribonucleoprotein antibody	20110601	1.5 ANTI-U1-RNP	
SM antibody	20110601	.3	U/mL
Sonogram	20110531	NO REASON FOR THIS LIVER VALUES FOUND	
Sonogram	20110601	NO REASON FOR THIS LIVER VALUES FOUND	
Thrombocyte count	20110531	207	x10E3/uL
Thrombocyte count	20110610	272	x10E3/uL
Urea	20110228	6.4	mg/dL
Urea	20110518	7.7	mg/dL
White blood cell count	20110531	9.65	x10E3/uL
White blood cell count	20110610	15.17	x10E3/uL

Narratives (nur bei elektronischem Eingang)

ROCHE, 25.07.2011 DE-ROCHE-780801

Sender diagnosis: not reported

Narrative

INITIAL INFORMATION FOR THIS SOLICITED CASE WAS RECEIVED ON 31 MAY 2011 FROM AN ONCOLOGIST HIMSELF AND VIA A ROCHE SALES REPRESENTATIVE AND CONCERNS A 36-YEAR-OLD MALE PATIENT WHO DEVELOPED HEPATIC FAILURE WHILE TREATED WITH BEVACIZUMAB (AVASTIN) AND INTERFERON ALFA-2A (ROFERON-A) FOR METASTATIC RENAL CELL CARCINOMA. THE PATIENT (# 1094) WAS ENROLLED IN THE GERMAN NON-INTERVENTIONAL STUDY ML21519: AVASTIN FIRST LINE IN METASTATIC RENAL CELL CARCINOMA.

THE PATIENT HAD MEDULLAR HEPATIC RENAL CARCINOMA WITH BONE METASTASES WHICH WERE TREAETED WITH ZOLEDRONIC ACID AND LIVER METASTASES. CONCURRENT CONDITION INCLUDED WASTING SYNDROME. CONCOMITANT MEDICATION ALSO INCLUDED DIPYRONE AND PARACETAMOL. NO PAST MEDICAL HISTORY AND NO PAST MEDICATION WAS PROVIDED.

THE PATIENT STARTED INTRAVENOUS BEVACIZUMAB AT 10 MG/KG EVERY 14 DAYS AND SUBCUTANEOUS INTERFERON ALFA-2A AT 9 MIU ON DAY 1, 3, 5, 8, 10, 12, 15, 17, 19, 22, 24, AND 26 OF A CYCLE OF 4 WEEKS. LAST APPLICATION OF BEVACIZUMAB WAS ON 18 MAY 2011 AND LAST APPLICATION OF INTERFERON ALFA-2A ON 30 MAY 2011. ON 30 MAY 2011 THE PATIENT DEVELOPED HEPATIC FAILURE FOR WHICH HE WAS HOSPITALIZED ON THE SAME DAY. TREATMENT WITH BEVACIZUMAB AND INTERFERON ALFA-2A WAS INTERRUPTED AND THE PATIENT WAS TREATED WITH 50 MG PREDNISOLONE DAILY DUE TO WASTING SYNDROME. THE OUTCOME OF THE EVENT WAS NOT REPORTED.

THE REPORTER ASSESSED THE EVENT OF HEPATIC FAILURE AS RELATED TO BEVACIZUMAB, INTERFERON ALFA-2A AND THE PATIENT'S LIVER METASTASES.

ADDITIONAL INFORMATION WAS RECEIVED ON 11 JUL 2011 FROM THE SAME PHYSICIAN AS BEFORE:

PARACETAMOL WAS ADDED AS SUSPECT DRUG. TOXIC LIVER DAMAGE WAS ADDED AS EVENT.

THE PATIENT WAS CONCOMITANTLY TREATED WITH CORTICOSTEROID NOS, INTERFERON ALFA-2B AND CLEMASTINE. THE PATIENT HAD A LUMBAL TUMORNEPHRECTOMY RIGHT AND A HAND SURGERY DUE TO CUT INJURY IN THE PAST. THE PATIENT DID NOT HAVE THE RISK FACTORS OF PRE-EXISTING HEPATOBILIARY DISORDER, PANCREATIC DISORDER, DRUG ALLERGY, B

LOOD TRANSFUSION, ALCOHOL USE, TATTOO, ACUPUNCTURE, INTRAVENOUS DRUG ABUSE, DIABETES MELLITUS, OBESITY, NON-ALCOHOLIC STEATOHEPATITIS, VIRAL HEPATITIS, FAMILY HISTORY OF LIVER DISEASE, RECENT TRAVEL TO ENDEMIC AREAS OF VIRAL HEPATITIS OR CONGENITAL HEPATIC FIBROSIS. THE PATIENT HAD NO HYPERTENSION, THYROID DISORDER, NO THROMBOSIS, NO MYOCARDIAL INFARCTION AND NO BLOOD COAGULATION DISORDER.

THE PATIENT STARTED BEVACIZUMAB AT 780 MG EVERY TWO WEEKS AND INTERFERON ALFA-2A AT 3 MIO UNITS EVERY SECOND DAY ON 6 APR 2011. SINCE 20 APR 2011 DOSAGE OF BEVACIZUMAB WAS REDUCED TO 730 MG DUE TO THE PATIENT'S LOWER WEIGHT. LAST APPLICATION OF BEVACIZUMAB WAS ON 18 MAY 2011, LAST APPLICATION OF INTERFERON ALFA-2A WAS ON 30 MAY 2011. ON 20 MAY 2011 THE PATIENT DEVELOPED RASH GRADE 2 WHICH LASTED UNTIL 5 JUN 2011. APPROXIMATELY ON 28 JUN 2011 THE PATIENT DEVELOPED NAUSEA, VOMITING AND IKTERUS (CO-MANIFESTATIONS) AND HEPATIC FAILURE (PREVIOUSLY REPORTED) WAS DIAGNOSED ON 30 MAY 2011. THE PATIENT WAS HOSPITALISED ON 31 MAY 2011. THE PATIENT'S ALANINE AMINOTRANSFERASE AND ASPARTATE AMINOTRANSFERASE WERE INCREASED FOR MORE THAN 3 TIMES THE UPPER LIMIT OF NORMAL AND TOTAL BILIRUBIN FOR MORE THAN 2 TIMES THE UPPER LIMIT OF NORMAL FROM 31 MAY 2011 UNTIL 10 JUN 2011 AND AGAIN ON 15 JUN 2011 (SEE LAB VALUES). SONOGRAM ON 31 MAY 2011 AND 1 JUN 2011 SHOWED NO REASON FOR THE HIGH LIVER VALUES. AS CAUSE FOR THE HEPATIC FAILURE TOXIC LIVER DAMAGE WAS DETERMINED. THE EVENTS IMPROVED ON AN UNREPORTED DATE. ON 10 JUN 2011 THE PATIENT WAS DISCHARGED FROM HOSPITAL. MAGNETIC RESONANCE IMAGING ON 23 JUN 2011 SHOWED THAT ONE TUMOUR IN LOWER LOBE HAD SLIGHTLY DECREASED.

THE REPORTER ASSESSED THE EVENTS OF HEPATIC FAILURE AND TOXIC LIVER DAMAGE AS POSSIBLY RELATED TO PARACETAMOL AND INTERFERON ALFA-2A AND DID NOT PROVIDE A DRUG/EVENT RELATIONSHIP FOR BEVACIZUMAB. THE REPORTER DID ALSO NOT PROVIDE ANY SERIOUSNESS CRITERIA OR CAUSALITY ASSESSMENT FOR THE EVENT OF RASH. THE COMPANY ASSESSED THE EVENT OF TOXIC LIVER DAMAGE AS RELATED TO BEVACIZUMAB AND THE EVENT OF RASH AS NON SERIOUS AND AS RELATED TO BEVACIZUMAB AND INTERFERON ALFA-2A.

ROCHE, 15.08.2011 DE-ROCHE-780801

Sender diagnosis: not reported

Narrative

INITIAL INFORMATION FOR THIS SOLICITED CASE WAS RECEIVED ON 31 MAY 2011 FROM AN ONCOLOGIST HIMSELF AND VIA A ROCHE SALES REPRESENTATIVE AND CONCERNS A 36-YEAR-OLD MALE PATIENT WHO DEVELOPED HEPATIC FAILURE WHILE TREATED WITH BEVACIZUMAB (AVASTIN) AND INTERFERON ALFA-2A (ROFERON-A) FOR METASTATIC RENAL CELL CARCINOMA. THE PATIENT (# 1094) WAS ENROLLED IN THE GERMAN NON-INTERVENTIONAL STUDY ML21519: AVASTIN FIRST LINE IN METASTATIC RENAL CELL CARCINOMA.

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THE PATIENT WAS CONCOMITANTLY TREATED WITH CORTICOSTEROID NOS, INTERFERON ALFA-2B AND CLEMASTINE. THE PATIENT HAD A LUMBAL TUMORNEPHRECTOMY RIGHT AND A HAND SURGERY DUE TO CUT INJURY IN THE PAST. THE PATIENT DID NOT HAVE THE RISK FACTORS OF PRE-EXISTING HEPATOBILIARY DISORDER, PANCREATIC DISORDER, DRUG ALLERGY, BLOOD TRANSFUSION, ALCOHOL USE, TATTOO, ACUPUNCTURE, INTRAVENOUS DRUG ABUSE, DIABETES MELLITUS, OBESITY, NON-ALCOHOLIC STEATOHEPATITIS, VIRAL HEPATITIS, FAMILY HISTORY OF LIVER DISEASE, RECENT TRAVEL TO ENDEMIC AREAS OF VIRAL HEPATITIS OR CONGENITAL HEPATIC FIBROSIS. THE PATIENT HAD NO HYPERTENSION, THYROID DISORDER, NO THROMBOSIS, NO MYOCARDIAL INFARCTION AND NO BLOOD COAGULATION DISORDER.

THE PATIENT STARTED BEVACIZUMAB AT 780 MG EVERY TWO WEEKS AND INTERFERON ALFA-2A AT 3 MIO UNITS EVERY SECOND DAY ON 6 APR 2011. SINCE 20 APR 2011 DOSAGE OF BEVACIZUMAB WAS REDUCED TO 730 MG DUE TO THE PATIENT'S LOWER WEIGHT. LAST APPLICATION OF BEVACIZUMAB WAS ON 18 MAY 2011, LAST APPLICATION OF INTERFERON ALFA-2A WAS ON 30 MAY 2011. ON 20 MAY 2011 THE PATIENT DEVELOPED RASH GRADE 2 WHICH LASTED UNTIL 5 JUN 2011. APPROXIMATELY ON 28 JUN 2011 THE PATIENT

DEVELOPED NAUSEA, VOMITING AND IKTERUS (CO-MANIFESTATIONS) AND HEPATIC FAILURE (PREVIOUSLY REPORTED) WAS DIAGNOSED ON 30 MAY 2011. THE PATIENT WAS HOSPITALISED ON 31 MAY 2011. THE PATIENT'S ALANINE AMINOTRANSFERASE AND ASPARTATE AMINOTRANSFERASE WERE INCREASED FOR MORE THAN 3 TIMES THE UPPER LIMIT OF NORMAL AND TOTAL BILIRUBIN FOR MORE THAN 2 TIMES THE UPPER LIMIT OF NORMAL FROM 31 MAY 2011 UNTIL 10 JUN 2011 AND AGAIN ON 15 JUN 2011 (SEE LAB VALUES). SONOGRAM ON 31 MAY 2011 AND 1 JUN 2011 SHOWED NO REASON FOR THE HIGH LIVER VALUES. AS CAUSE FOR THE HEPATIC FAILURE TOXIC LIVER DAMAGE WAS DETERMINED. THE EVENTS IMPROVED ON AN UNREPORTED DATE. ON 10 JUN 2011 THE PATIENT WAS DISCHARGED FROM HOSPITAL. MAGNETIC RESONANCE IMAGING ON 23 JUN 2011 SHOWED THAT ONE TUMOUR IN LOWER LOBE HAD SLIGHTLY DECREASED.

THE REPORTER ASSESSED THE EVENTS OF HEPATIC FAILURE AND TOXIC LIVER DAMAGE AS POSSIBLY RELATED TO PARACETAMOL AND INTERFERON ALFA-2A AND DID NOT PROVIDE A DRUG/EVENT RELATIONSHIP FOR BEVACIZUMAB. THE REPORTER DID ALSO NOT PROVIDE ANY SERIOUSNESS CRITERIA OR CAUSALITY ASSESSMENT FOR THE EVENT OF RASH. THE COMPANY ASSESSED THE EVENT OF TOXI

C LIVER DAMAGE AS RELATED TO BEVACIZUMAB AND THE EVENT OF RASH AS NON SERIOUS AND AS RELATED TO BEVACIZUMAB AND INTERFERON ALFA-2A.

ADDITIONAL INFORMATION WAS RECEIVED ON 4 AUG 2011 FROM THE SAME ONCOLOGIST AS BEFORE:
THE EVENT OF HEPATIC FAILURE RESOLVED AND THE PATIENT WAS DISCHARGED FROM HOSPITAL ON 10 JUN 2011.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11187503

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19291127	Age: 81 Year	weight (kg): 63	BMI_int: 28	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110806			13 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Lyrica, 75 mg, Kps, Pfizer pregabalin	Oral	2*75 Mg milligram(s)/ Day 5	20110725-20110808 15 Day	
2 palexia retard tapentadol hydrochloride	Oral	/Day 212	20110725-20110806 13 Day	
3 Perfolgan, 1g, Amp, BMS paracetamol	Intravenous (not otherwis	/Day 293	20110725-20110802 9 Day	
4 Stilnox, 10 mg, Filmtabl, Sanofi-Aventis zolpidem	Oral	10 Mg milligram(s)/Day 82	20110723-20110808 17 Day	
5 Tetrazepam, 50 mg, Filmtabl, Ratiopharm tetrazepam	Oral	/Day 82	20110724-20110806 14 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Foramen ovale patent	192912		Yes	
Hyperlipoproteinemia			Yes	
Hypertension arterial			Yes	
Middle cerebral artery infarct	200706		Unknown	
Iliolumbar syndrome	2004		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20110723	23	U/l	
Alanine aminotransferase increased	20110810	56	U/l	
Alanine aminotransferase normal	20110830	18	U/l	
Aspartate aminotransferase increased	20110723	35	U/l	
Aspartate aminotransferase increased	20110806	151	U/l	
Aspartate aminotransferase increased	20110810	45	U/l	
Aspartate aminotransferase normal	20110830	27	U/l	
Alkaline phosphatase increased	20110806	348	U/l	
Alkaline phosphatase increased	20110810	311	U/l	
Alkaline phosphatase increased	20110830	168	U/l	
Bilirubin normal	20110723	0.2	mg/dl	
Bilirubin normal	20110806	0.5	mg/dl	

Narratives (nur bei elektronischem Eingang)PVZ-FAKOS, Klinische Pharmakologie, 27.09.2011 DE-FAKOS-170407

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report, more info follows.

Reporter's comment:PVZ-FAKOS, Klinische Pharmakologie, 28.10.2011 DE-FAKOS-170407

Sender diagnosis: not reported

Narrative

Die Patientin wurde am 05.08.2011 zur Weiterbehandlung und Mobilisierung bei Verdacht auf ISG (Iliosakralgelenk)-Schmerzsyndrom aus einer neuro-chirurgischen Abteilung in eine geriatrische Klinik verlegt. Am 23.07.2011 waren die Leberwerte im Normbereich (ALAT 23 U/l, Normbereich: <34 U/l; ASAT 35 U/l, Normbereich: <35 U/l und Bilirubin 0,2 mg/dl, Normbereich: bis 1 mg/dl). Nach Verlegung in die Geriatrie wurden am 06.08.2011 erstmalig erhöhte Transaminasen (ASAT 151 U/l) und Cholestaseparameter (Gamma-GT 1018 U/l, Normbereich: <38 U/l; AP (Alkalische Phosphatase) 348 U/l, Normbereich: 35-104 U/l) nachgewiesen. Nach Beendigung der Medikation mit Simvastatin 20 mg Tabl., Stilnox 10 mg Tabl., Lyrica 75 mg Kps., Palexia retard 100 mg Retardtabl., Tetrazepam-ratiopharm 50 mg Tabl. und Perfalgan Injektionslösung waren die Transaminasen und Cholestaseparameter rückläufig (10.08.2011: ALAT 56 U/l, ASAT 45 U/l, Gamma-GT 596 U/l, AP 311 U/l). Am 30.08.2011 lagen die Transaminasen wieder im Normbereich (ALAT 18 U/l, ASAT 27 U/l). Die Gamma-GT und AP befanden sich in weiterer Besserung (Gamma-GT 345 U/l, AP 168 U/l). Am 01.09.2011 wurde die Patientin aus der stationären Behandlung entlassen.

Die Serologie auf Hepatitis A, B und C war negativ, die Leber stellte sich sonographisch unauffällig dar.

Eine medikamentös-bedingte Genese der vermuteten akuten Hepatitis ist wahrscheinlich. Verdächtig werden alle oben genannten Arzneimittel, außer Simvastatin, welches schon seit 2008 eingenommen wurde und als Ursache der vermuteten akuten Hepatitis eher nicht in Frage kommt:

Stilnox 10 mg Filmtabl., Sanofi-Aventis, Einnahmezeitraum: 23.07.-08.08.2011,

Lyrica 75 mg Kps., Pfizer, Einnahmezeitraum: 25.07.-08.08.2011,

Palexia retard 100 mg Retardtabl., Grünenthal, Einnahmezeitraum: 25.07.-06.08.2011,

Tetrazepam 50 mg Filmtabl., Ratiopharm, Einnahmezeitraum: 24.07.-06.08.2011 und

Perfalgan 1 g Amp., BMS, Einnahmezeitraum: 25.07.-02.08.2011.

Da die genannten Arzneimittel etwa zur gleichen Zeit eingenommen wurden, kann keine genauere Zuordnung zu einem bestimmten Arzneimittel erfolgen. Die Arzneimittelkausalität wird deshalb jeweils mit "möglich" bewertet. Die UAW hat sich im Krankenhaus entwickelt.<dbtext>

To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 07.11.2011 DE-SANOFI-AVENTIS-2011SA071407

Sender diagnosis: not reported

Narrative

Initial report: This case had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-11187503) on 31-Oct-2011. The pharmacovigilance (PV) centre is conducting a hospitalisation registry (FAKOS). Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission.

The case involves an 81-year-old female patient with a past medical history of middle cerebral artery infarct in Jun-2007. At time of event, the patient was suffering from iliolumbar syndrome, patent foramen ovale, hyperlipoproteinaemia, and arterial hypertension.

On 23-Jul-2011, liver values were within normal ranges (ALAT 23 U/l, normal ranges < 34 U/l, ASAT 35 U/l, normal ranges < 35 U/l, and bilirubin 0.2 mg/dl, normal ranges up to 1 mg/dl).

The patient started treatment with zolpidem (Stilnox) 1x10 mg/day orally on 23-Jul-2011 (indication not reported). On 24-Jul-2011, treatment with tetrazepam 25 to 50 mg/day orally was started, and on 25-Jul-2011, treatment with pregabalin 2x75 mg/day orally, tapentadol 200 to 600 mg/day orally and paracetamol 1000 to 4000 mg/day intravenously was added. Pregabalin was paused from 26- to 29-Jul-2011, and paracetamol was paused on 26-Jul-2011.

On 05-Aug-2011, the patient was transferred from neurosurgical ward to a geriatric clinic for further treatment and mobilisation in the course of suspicion of iliosacral pain syndrome.

On 06-Aug-2011, increased transaminases (ASAT 151 U/l) and increased cholestasis parameters (gamma GT 1018 U/l, normal ranges < 38 U/l, alkaline phosphatase 348 U/l, normal ranges 35 to 104 U/l) were detected for the first time.

After end of treatment with simvastatin 20 mg (begin of therapy in 2008, last intake not provided), zolpidem (last intake on 08-Aug-2011), pregabalin (last intake on 08-Aug-2011), tapentadol (last intake on 06-Aug-2011), tetrazepam (last intake on 06-Aug-2011), and paracetamol (last application on 02-Aug-2011), transaminases and cholestasis parameters started to regress (values on 10-Aug-2011: ALAT 56 U/l, ASAT 45 U/l, gamma GT 596 U/l, alkaline phosphatase 311 U/l).

On 30-Aug-2011, transaminases had returned to normal ranges (ALAT 18 U/l, ASAT 27 U/l); gamma GT and alkaline phosphatase were decreasing (gamma GT 345 U/l, alkaline phosphatase 168 U/l).

Serology for hepatitis A, B and C was negative; sonography of liver was unremarkable.

On 01-Sep-2011, the patient was discharged from hospital.

The PV center reported acute hepatitis as adverse event.

Outcome of event was reported as "recovering/resolving".

The event led to prolongation of the patient's hospitalisation.

According to the PV centre, drug-toxic aetiology of assumed acute hepatitis was likely; suspected were all drugs with the exception of simvastatin, as this drug had been taken since 2008 and was not considered a likely cause of the acute hepatitis.

A precise correlation with one of the drugs could not be made because all medication was administered at the same time.

Causality determined by PV centre (method: WHO global introspection): "possible" for zolpidem, pregabalin, tapentadol, tetrazepam, and paracetamol.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11200531

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth:	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	201109				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol (acetaminophen)		/	-	
paracetamol			-	
2 REBIF	Subcutaneous	/	-	Multiple sclerosis
interferon beta-1a		120	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Celiac disease			Yes	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 14.10.2011 DE-Merck KGaA-7089067

Sender diagnosis: not reported

Narrative

This case report from a patient in the USA was received at EMD Serono on 04 Oct 2011.

This 28 year old female patient developed hepatitis, which was assessed as medically significant by the company, while being treated with Rebif.

Medical history included celiac disease that was ongoing.

Cosuspect medications included Paracetamol (acetaminophen).

The patient received Rebif (interferon beta-1a) at unspecified dose by subcutaneous injection since an unknown date for multiple sclerosis that was ongoing at the time of event.

In Sep 2011, the patient developed increased liver values (ten fold elevated), except gamma-glutamyl transpsptidase (GGT), and hepatitis which she suspected to be related to Rebif. It was not reported whether the patient continued administrations of Rebif.

At the time of reporting, the patient had not recovered from hepatitis.

suspected to be related to Rebif.

Hepatitis is listed for Rebif as per reference safety information (i.e. CSDS).

Follow-up information (Rebif treatment data, latency, liver function tests values, treatment, outcome, medical history, causality assessment) has been requested.

Reporter's comment:

Merck KGaA, 19.10.2011 DE-Merck KGaA-7089067

Sender diagnosis: not reported

Narrative

This case report from a patient in Germany was received at Merck Serono on 04 Oct 2011.

This 28 year old female patient developed hepatitis, which was assessed as medically significant by the company, while being treated with Rebif. Medical history included celiac disease.

No concomitant medications were reported.

The patient received Rebif (interferon beta-1a) at unspecified dose by subcutaneous injection since an unknown date for multiple sclerosis that was ongoing at the time of event.

Additionally, the patient received the following suspected medications: Paracetamol (acetaminophen).

In Sep 2011, the patient developed increased liver values (ten fold elevated), except gamma-glutamyl transpeptidase (GGT), and hepatitis. The patient suspected the increased liver values to be related to Rebif. It was not reported whether the patient continued administrations of Rebif.

At the time of reporting, the patient had not recovered from hepatitis.

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On 17 Oct 2011, clarification was received that the patient suspected that the increased liver values were related to Rebif therapy and made no comment on the relationship with Rebif and hepatitis.

patient is suspected to be related to Rebif.

Hepatitis is listed for Rebif as per reference safety information (i.e. CSDS).

Follow-up information (Rebif treatment data, latency, liver function tests values, treatment, outcome, medical history, causality assessment) has been requested.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11205098

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 9 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					recovered/resolved
2 Drug overdose accidental					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol	Oral	/	-	
paracetamol			3 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Apostolidou S et al. Cutaneous zygomycosis in a 9 year old patient after liver transplantation. 22nd Annual Medical and Nursing Congress of the European Society of Paediatric and Neonatal Intensive Care (ESPNIC). 2011. Abstr. 512	

Narratives (nur bei elektronischem Eingang)GlaxoSmithKline, 24.10.2011 DE-GlaxoSmithKline-B0757250A

Sender diagnosis: not reported

Narrative

This case was reported in a literature article and described the occurrence of acute hepatic failure in a 9-year-old female patient who received paracetamol (brandname/tradenname unknown) over a period of 3 days for an unknown drug indication.

On an unknown date, the patient accidentally overdosed on paracetamol (6g over 3 days). At an unknown time after the accidental overdose, the patient developed acute liver failure. On an unknown date, the patient underwent an auxiliary hepatic lobe transplantation of the left liver of the patient's mother and immunosuppression with tacrolimus, which resulted in the regression of the encephalopathy and stabilisation of the patient's clinical condition.

The author considered the events were related to treatment with Paracetamol.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11224315

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 00081 Year	weight (kg): 00063	BMI_int: 28	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110806				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 LYRICA pregabalin	Oral	150 Mg milligram(s)/Day	20110725-20110808 15 Day	
2 PERFALGAN 10 mg/ml Infusionslösung paracetamol	Intravenous (not otherwis	00000001 G gram(s)/	20110725-20110802 9 Day	
3 STILNOX zolpidem tartrate	Oral	10 Mg milligram(s)/Day	20110723-20110808 17 Day	
4 tapentadol tapentadol	Oral	00000001 Df dosage form	20110725-20110806 13 Day	
5 TETRAZEPAM tetrazepam	Oral	00000001 Df dosage form	20110724-20110806 14 Day	

Drugs (concomitant)

6 FENTANYL PATCH fentanyl	00000001 Df dosage form	20110724- -
7 FRAXIPARINE nadroparin calcium	00000001 Df dosage form	20110723- -
8 HYDROCHLOROTHIAZIDE hydrochlorothiazide	00000025 Mg milligram(s)	201105- -
9 kalinor potassium chloride	00000001 Df dosage form	20110724- -
10 mcp metoclopramide hydrochloride	00000001 Df dosage form	20110731-20110731 -
11 METAMIZOLE SODIUM metamizole sodium	00000001 Df dosage form	- -
12 METOHEXAL metoprolol tartrate	00000001 Df dosage form	2007- -
13 novalgin metamizole sodium	00000001 Df dosage form	20110723- -
14 PANTOZOL pantoprazole sodium	80 Mg milligram(s)/Day	200911- -
15 SIMVASTATIN simvastatin	00000020 Mg milligram(s)	2008- -
16 THYRONAJOD levothyroxine sodium; potassium iodide	/001 Day	20110723- -
17 VOTUM PLUS hydrochlorothiazide; olmesartan medoxomil	00000001 Df dosage form	20110718- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Foramen ovale patent	192911		k.A.	
Middle cerebral artery infarct	200706		k.A.	
Iliolumbar syndrome	2004		k.A.	
Hyperlipoproteinemia			k.A.	
Drug therapy	2008		k.A.	simvastatin
Hypertension arterial			k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20110723	23	IU/L	
Aspartate aminotransferase	20110723	35	IU/L	
Bilirubin	20110723	0.2	MG/DL	
Aspartate aminotransferase	20110806	151	IU/L	
Gamma-glutamyltransferase	20110806	1018	IU/L	
Investigation NOS	20110806	348	IU/L	
Alanine aminotransferase	20110810	56	IU/L	
Aspartate aminotransferase	20110810	45	IU/L	
Gamma-glutamyltransferase	20110810	596	IU/L	
Alanine aminotransferase	20110830	18	IU/L	
Aspartate aminotransferase	20110830	27	IU/L	
Investigation NOS	20110830	64	IU/L	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 09.11.2011 DE-Bristol-Myers Squibb Company-16205015

Sender diagnosis: not reported

Narrative

This report was received via German Health Authority (Agency number: 11187503/ Regional Pharmacovigilance Center number: 170407) in BMS Germany (Local file number: 2011102801).

A physician reported that an 81-year-old female patient hospitalized due to acute hepatitis while on therapy with acetaminophen, pregabalin, zolpidem tartrate, tapentadol hydrochloride and tetrazepam.

The patient received intravenous therapy with acetaminophen 1 g vial (dosage form: 293, infusion) from 25-Jul-2011 to 02-Aug-2011 with a daily dose of 1000 - 4000 mg and a break on 26-Jul-2011. The cumulative dose of acetaminophen was 24000. The patient had also received oral therapy with pregabalin 75 mg capsules (dosage form: 5), from 25-Jul-2011 to 08-Aug-2011 with a dose of 2 x 75 mg daily, cumulative dose: 1350 mg, break between 26-Jul-2011 to 29-Jul-2011. Oral therapy with zolpidem tartrate 10 mg film-coated tablets (dosage form: 82), from 23-Jul-2011 to 08-Aug-2011, 1x1 film-coated tablet at 10 mg daily, cumulative dose: 150 mg. Oral therapy with tapentadol hydrochloride 100/200 mg prolonged-release tablets (dosage form: 212), from 25-Jul-2011 to 06-Aug-2011, daily dose: 200-600 mg, cumulative dose: 4300. Oral therapy with tetrazepam 50 mg film-coated tablets (dosage form: 82), from 24-Jul-2011 to 06-Aug-2011, daily dose: 25 - 50 mg, cumulative dose: 550 mg.

On 23-Jul-2011 the patient's liver values were in the normal range. Her alanin amino transferase (ALAT) was 23 U/l, normal range < 34; aspartate aminotransferase (ASAT) was 35 U/l, normal range < 35; and bilirubin was 0.2 mg/dl, normal range until 1 mg/dl. The patient was transferred on 05-Aug-2011 for further treatment and mobilization from a neurosurgical department to the geriatric clinic due to suspicion of ISG (sacroiliac joint) pain syndrome. After transfer to the geriatric department on 06-Aug-2011, the patient had increased transaminases (ASAT 151 U/l) and cholestasis parameters - gamma-GT was 1018 U/l, normal range < 38; alkaline phosphatase was 348 U/l, normal range 35 - 104 were detected for the first time. After discontinuation of the medication including simvastatin 20 mg tablets, zolpidem tartrate 10 mg tablets, pregabalin 75 mg capsules, tapentadol hydrochloride 100 mg retard tablets, tetrazepam 50 mg tablets and acetaminophen injection solution the transaminases and cholestasis parameters

were regressive.

On 10-Aug-2011 her ALAT was 56 U/l, ASAT was 45 U/l, gamma-GT was 596 U/l and alkaline phosphatase was 311 U/l. On 30-Aug-2011 the transaminases were in the normal range again. Her ALAT was 18 U/l, ASAT was 27 U/l. Gamma-GT and alkaline phosphatase were further improving. Gamma-GT 345 U/l, alkaline phosphatase was 168 U/l. On 01-Sep-2011, the patient was discharged from inpatient treatment. Serology regarding hepatitis A, B and C were negative, the liver presented sonographically inconspicuous. A drug-induced origin of the suspected acute hepatitis was considered likely. All above mentioned drugs were considered to be suspect drugs despite simvastatin, which was administered already since 2008 and which did more likely not come into question as cause of the suspected acute hepatitis. Therapies with zolpidem tartrate, pregabalin 75 mg capsules, tapentadol hydrochloride, tetrazepam and acetaminophen 1 g vial were administered approximately at the same time and no more exact relationship to a specific drug could be made. Thus the causality was assessed to be possible for each drug. At the time of this report the event acute hepatitis was resolving. According to the Global Introspection (WHO GI) the reporter considered the event acute hepatitis possibly related to therapy with acetaminophen, pregabalin, zolpidem, tapentadol and tetrazepam.

The patient's medical history included iliolumbar syndrome since 2004, which was ongoing at the time of this report; middle cerebral artery infarct in Jun-2007; patent foramen ovale since Nov-1929, which was ongoing at the time of this report; hyperlipoproteinemia, which was ongoing at the time of this report; arterial hypertension, which was ongoing at the time of this report.

Supplemental information received on 03-Nov-2011 from the physician via Regional Pharmacovigilance Center noted that concomitant medications included pantoprazole sodium, levothyroxine sodium + potassium iodide, olmesartan medoxomil + hctz, metamizole sodium, metoprolol tartrate, simvastatin, potassium cit+potassium bicarb, metoclopramide hcl, nadroparin calcium, dipyrone, fentanyl and hydrochlorothiazide.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11231446

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19980217	Age: 13 Year	weight (kg): 60	BMI_int: 23.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased					recovered/resolved
2 SGOT increased					recovered/resolved
3 LDH increased					recovered/resolved
4 Acute hepatic failure	20110527			11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol paracetamol	Unknown	/	20110517-20110519 3 Day	Surgery

Drugs (concomitant)

2 KETAMIN ketamine hydrochloride	Intravenous (not otherwis	70 Mg milligram(s)/	20110517- -	Restlessness
3 midazolam midazolam	Intravenous (not otherwis	1 Mg milligram(s)/	20110517- -	Restlessness
4 PROMETHAZINE promethazine	Oral	30 Gtt drop(s)/ 158	20110519- -	Restlessness
5 TAVOR lorazepam	Oral	1 Mg milligram(s)/	20110517-20110519 3 Day	Restlessness
6 vomacur dimenhydrinate	Oral	50 Mg milligram(s)/	20110517- -	Restlessness

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Torticollis			No	torticollis
Restlessness			Yes	
Atlantoaxial instability			Yes	VENTRAL RE-RELAXATION OF THE LEFT ATLANTO-AXIAL JOINT BY GRISEL-SYNDROME

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20110527	2972	U/l	
SGPT	20110527	3664	U/l	
Blood LDH	20110527	2657	U/L	
SGPT	20110527	3664	U/L	
GPT	20110528	1869	U/L	
GPT	20110530	977	U/L	
LDH	20110527	2657	U/l	

LDH	20110528	189	U/L
LDH	20110530	211	U/L
SGOT	20110527	2972	U/l
SGOT	20110528	348	U/L
SGOT	20110530	61	U/L

Narratives (nur bei elektronischem Eingang)

Fresenius Kabi, 06.12.2011 DE-Fresenius Kabi-FK201102853

Sender diagnosis: not reported

Narrative

Verbatim of the reporter:
Acute hepatic failure

Narrative:

The case was reported by the BfArM (German Health Authority) with Worldwide Reference Number DE-BFARM-11231446.
A 13-year-old male patient received paracetamol for 3 days with an unknown dosage. Nine days after stopping the drug acute hepatic failure occurred.

Medical history: torticollis

Other suspected drugs: -

Concomitant medication: tavor 1mg, 17-19 May, promethazin 30 drops, vomacur 50mg, midazolam 1mg, ketamin 70mg

Drugs to treat AE:

sanofi-aventis, 06.12.2011 DE-SANOFI-AVENTIS-2011SA078886

Sender diagnosis: not reported

Narrative

Initial report:

On 01-Dec-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11231446); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association about a 13-year-old male patient (height 159 cm, weight 60 kg).

The present medical history included torticollis and atlantic oepistrophealis (ventral re-reluxation of the left atlanto-axial joint by Grisel-syndrome). Furthermore restlessness was treated with lorazepam, promethazine hydrochloride, dimenhydrinate, midazolam and ketamine.

In connection with a surgery the patient received paracetamol from 17-May-2011 to 19-May-2011. The exact dosage was not provided.

On 27-MAY-2011 the acute hepatic failure was diagnosed with increased SGOT (2972 U/L), increased GPT (3664 U/L) and increased LDH (2657 U/L). Corrective treatment was not mentioned; however the patient recovered at unspecified time point. The event of acute hepatic failure was considered as medically important by the reporting physician.

Causality by Drug Commission: possible

GlaxoSmithKline, 07.12.2011 DE-GlaxoSmithKline-B0767343A

Sender diagnosis: not reported

Narrative

This case was reported by a physician via the German Regulatory Authority and described the occurrence of acute hepatic failure in a 13-year-old male patient who received Paracetamol over a period of 3 days for an unspecified operation.

Concurrent medical conditions included torticollis and restlessness.
Concurrent medications included lorazepam (Tavor), promethazine (Promethazin), dimenhydrinate (Vomacur), Midazolam and Ketamine.

From 17 May 2011 to 19 May 2011, the patient received Paracetamol.
Approximately 10 days after starting Paracetamol, on 27 May 2011, the patient developed acute hepatic failure. The regulatory authority reported that the event was clinically significant (or requiring intervention). On 27 May 2011, laboratory values showed SGOT:2972 U/l, SGPT: 3664 U/l, LDH: 2657 U/l. Normal ranges were not provided. At the time of reporting, the event was resolved.
The regulatory authority reported that the event was possibly related to treatment with Paracetamol.

sanofi-aventis, 22.02.2012 DE-SANOFI-AVENTIS-2011SA078886

Sender diagnosis: not reported

Narrative

Initial report:

On 01-Dec-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11231446); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association about a 13-year-old male patient (height 159 cm, weight 60 kg).

The present medical history included torticollis and atlanto-occipital synovitis (ventral re-relaxation of the left atlanto-axial joint by Grisel-syndrome).

Furthermore restlessness was treated with lorazepam, promethazine hydrochloride, dimenhydrinate, midazolam and ketamine.

In connection with a surgery the patient received paracetamol from 17-May-2011 to 19-May-2011. The exact dosage was not provided.

On 27-MAY-2011 the acute hepatic failure was diagnosed with increased SGOT (2972 U/L), increased GPT (3664 U/L) and increased LDH (2657 U/L). Correctively, symptomatic therapy was initiated and the patient recovered (GOT 348 U/L, GPT 1869 U/L and LDH 189 U/L on 28-May-2011, and GOT 61 U/L, GPT 977 U/L and LDH 211 U/L on 30-May-2011).

The event of acute hepatic failure was considered as medically important by the reporting physician.

Causality by Drug Commission: possible

Additional information received on 17-Feb-2012:

The original document for the initial data collection was provided by the Drug Commission of the German Medical Association. Therefore patients initials, birth date as well as further laboratory values and corrective therapy could be amended.

Bayer Vital GmbH, 13.03.2012 DE-BAYER-2012-023187

Sender diagnosis: not reported

Narrative

serious; listed; related

This spontaneous case report was received from a physician in Germany via the Drug Commission of the German Medical Association. The present report was identified on the ADR-internet page of the German Health Authority BfArM (Mfr.control number: DE-BfARM-11231446) on 07-Mar-2012, referring to a 13 year old male patient.

Note: This report was marked as 'follow-up report'. However, no initial case could be identified in the local database.

The patient's concurrent condition included torticollis (torticollis atlanto-occipitalis, ventral re-relaxation of the left atlanto-axial joint with Grisel's syndrome).

The patient received concomitant Tavor, Prometazin, Vomacur, Midazolam, Ketamin.

On 17-May-2011 the patient started Paracetamol for surgery (not further specified).

On 27-May-2011 the patient experienced acute hepatic failure reported as medically important event. The patient recovered. Paracetamol was withdrawn.

A reporter causality comment was not reported.

Bayer Vital GmbH, 07.05.2012 DE-BAYER-2012-023187

Sender diagnosis: not reported

Narrative

serious; listed; related

This spontaneous case report was received from a physician in Germany via the Drug Commission of the German Medical Association. The present report was identified on the ADR-internet page of the German Health Authority BfArM (Mfr.control number: DE-BfARM-11231446) on 07-Mar-2012, referring to a 13 year old male patient.

Note: This report was marked as 'follow-up report'. However, no initial case could be identified in the local database.

The patient's concurrent condition included torticollis (torticollis atlantoepistrophealis, ventral re-reluxation of the left atlanto-axial joint with Grisel's syndrome).

The patient received concomitant Tavor, Prometazin, Vomacur, Midazolam, Ketamin.

On 17-May-2011 the patient started Paracetamol for surgery (not further specified).

On 27-May-2011 the patient experienced acute hepatic failure reported as medically important event. The patient recovered. Paracetamol was withdrawn.

A reporter causality comment was not reported.

****FU 23-Apr-2012:**

The patient's birth date and initials were provided.

The patient received Tavor on 17-May-2011 and on 19-May-2011 at a dosage of 1 mg.

The therapy with paracetamol was discontinued due to the event. The event improved after discontinuation of paracetamol therapy.

Summary of hospital report: The patient was hospitalized from 16-May-2011 to 01-Jun-2011. Current therapy: analgetic therapy, reposition of Crutchfield

tongs and spinal fusion. Physical examination: within normal limits.

Lab data on 17-May-2011: differential blood count: increase of monocytes up to 13 %, eosinophils 0.5 %, otherwise normal, GPT within normal limits, fibrinogen increased 372 mg/dl. The intra- and postoperative course was uneventful. During hospitalization the patient was temporary aggressive towards the nurses and his mother. Conversational therapy with the child psychologist was initiated. Vital signs were stable and normal for patient's age of the patient. The patient could be discharged on 01-Jun-2011.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11236823

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 19 Month	weight (kg): 13	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Angioedema	19961211			28 Day	recovering/resolving
2 Ascites	19961204			18 Day	recovering/resolving
3 Airway obstruction NOS	19961207			24 Day	Unknown
4 Stevens Johnson syndrome	19961213			30 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ALDACTONE	/	19961204-	
	spironolactone		-	
2	ambisome	/	19961124-	
	amphotericin b		-	
3	ARTERENOL	/	19961210-	
	norepinephrine hydrochloride		-	
4	BACTRIM	/	19961129-	
	sulfamethoxazole; trimethoprim		-	
5	diflucan	/	19961117-19961125	
	fluconazole		9 Day	
6	DOPAMIN	/	19961210-	
	dopamine hydrochloride		-	
7	DORMICUM	/	19961209-	
	midazolam maleate		-	
8	etoposide	/	19961122-19961122	
	etoposide		1 Day	
9	FENTANYL	/	19961209-	
	fentanyl		-	
10	FORTUM	/	19961122-19961130	
	ceftazidime pentahydrate		9 Day	
11	GAMMA GLOBULIN	/	19961127-19961210	
	immunoglobulin human normal		14 Day	
12	HYDROMEDIN	/	19961212-	
	etacrynic acid		-	
13	LASIX	/	19961124-	
	furosemide		-	
14	luminal	/	19961209-	
	phenobarbital		-	
15	methotrexate	/	19961127-19961129	
	methotrexate sodium		3 Day	
16	micronefrin	/	19961206-	
	epinephrine hydrochloride		-	
17	Morphin	/	19961130-	
	morphine hydrochloride		-	
18	MUCOSOLVAN	/	19961211-	
	ambroxol hydrochloride		-	
19	NEUPOGEN	Intravenous (not otherwise specified)	19961125-	Bone marrow transplant NOS
		120	-	
20	NOVALGIN	/	19961120-19961126	
	metamizole sodium		7 Day	
21	PANCURONIUM BROMIDE	/	19961213-19961213	
	pancuronium bromide		1 Day	
22	paracetamol	/	19961120-19961126	
	paracetamol		7 Day	
23	phenytoin	/	19961114-19961124	
	phenytoin		11 Day	
24	phenytoin	/	19961114-19961124	
	phenytoin		11 Day	
25	prostavaslin	/	19961201-	
	alprostadil		-	
26	sobelin	/	19961203-	
	clindamycin hydrochloride		-	
27	soluvit	/	19961126-	
	vitamins nos		-	
28	tobramycin	/	19961124-	
	tobramycin		-	

29	TRIGLOBE	/	19961116-19961127
	sulfadiazine; trimethoprim		12 Day
30	Urbason	/	19961124-
	methylprednisolone		-
31	VITINTRA	/	19961127-
	ergocalciferol; phytomenadione; retinol; retinol palmitate		-
32	ZANTIC	/	-19961211
	ranitidine hydrochloride		-
33	ZIENAM	/	19961201-
	cilastatin sodium; imipenem		-
34	zovirax	/	19961127-
	aciclovir		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Familial haemophagocytic lymphohistiocytosis			Unknown	
Upper respiratory infection			Unknown	
Autosomal chromosome anomaly			Unknown	
Allogenic bone marrow transplantation therapy	19961126		Unknown	experienced graft versus host disease

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 05.12.2011 DE-PFIZER INC-049-0073-990003

Sender diagnosis: not reported

Narrative

Following the integration of the legacy safety adverse event reporting databases, this case has been migrated from the legacy company database into the current safety database for processing follow-up information. As a consequence of this migration, the follow-up CIOMS I or MedWatch report may indicate in the appropriate field that it is an initial report.

This is a spontaneous report from a contactable physician in Germany via Amgen pharmaceuticals, Mfr. Report #303385. A health care professional reported on 06Apr1999 that a 19-month-old male patient developed angioedema, ascites, and an airway obstruction, events considered medically significant hospitalization, and Stevens Johnson Syndrome, prolonging hospitalization eight days after receiving phenytoin. The patient was also receiving filgrastim (NEUPOGEN) intravenously, acetylcysteine, spironolactone (ALDACTONE), amphotericine B/ liposome (AMBISOME) 40 mg daily, norepinephrine hydrochloride (ARTERENOL), sulfamethoxazole trimethoprim (BACTRIM), fluconazole (DIFLUCAN), dopamine hydrochloride (DOPAMIN), midazolam maleate (DORMICUM), etoposide, fentanyl, ceftazidime pentahydrate (FORTUM), immunoglobulin human normal (GAMMA GLOBULIN), etacrynic acid (HYDROMEDIN), furosemide (LASIX), phenobarbital (LUMINAL), methotrexate, epinephrine hydrochloride (MICRONEFRIN), morphine hydrochloride, ambroxol hydrochloride (MUCOSOLVAN), Novalgin (metamizole sodium), pancuronium bromide, paracetamol, alprostadil (PROSTAVASIN), clindamycin hydrochloride (SOBELIN), vitamins nos (SOLUVIT), tobramycin, sulfadiazine/ trimethoprim (TRIGLOBE), methylprednisolone (URBASON), ergocalciferol/ retinol/ retinol palmitate/ phytomenadione (VITINTRA), ranitidine hydrochloride (ZANTIC), imipenem/ cilastatin sodium (ZIENAM), and aciclovir (ZOVIRAX). He started taking phenytoin (dosage unknown) on 14Nov1996. He started sulfadiazine (TRIGLOBE) on 16Nov1996. He began treatment with fluconazole on 17Nov1996. On 20Nov1996, he started taking metamizole and paracetamol. On 22Nov1996, started ceftazidime pentahydrate and took etoposide for only that day. On 24Nov1996, he started taking amphotericine B/ liposome, furosemide, tobramycin, and methylprednisolone, and phenytoin therapy was discontinued. Fluconazole treatment was discontinued on 25Nov1996 and filgrastim therapy was started for bone marrow transplantation. The patient underwent an allogenic bone marrow transplant on 26Nov1996. Vitamins were started and metamizole sodium and paracetamol therapies were discontinued on 26Nov1996. On 27Nov1996, he started methotrexate, aciclovir, immunoglobulin human normal, and Ergocalciferol/ retinol/ retinol palmitate/ phytomenadione. Sulfadiazine (TRIGLOBE) therapy was discontinued on 27Nov1996. On 29Nov1996, sulfamethoxazole trimethoprim therapy was started and methotrexate therapy was discontinued. He experienced graft versus host disease on an unspecified date. Acetylcysteine and morphine hydrochloride therapies were started on 30Nov1996, and ceftazidime pentahydrate therapy was discontinued. On 01Dec1996, he developed hemorrhagic mucositis and started treatment with alprostadil and imipenem. On 03Dec1996, he developed erosions and began treatment with clindamycin hydrochloride. On 04Dec1996, he experienced erosive oral mucosa and ascites and was started on spironolactone therapy. He developed inspiratory stridor on 06Dec1996, and was treated with epinephrine hydrochloride. The following day he experienced an acute airway obstruction. On 09Dec1996, he started taking midazolam, fentanyl, and phenobarbital. He started taking norepinephrine hydrochloride and dopamine hydrochloride on 10Dec1996, and discontinued immunoglobulin human normal that day. He developed angioedema on 11Dec1996 and was started on ambroxol hydrochloride. Ranitidine hydrochloride was discontinued on 11Dec1996. Etacrynic acid was started on 12Dec1996. On 13Dec1996, he experienced blistering and erosions of his skin and was diagnosed with Stevens Johnson syndrome. He was given pancuronium bromide only on 13Dec1996. By 05Jan1997, the Stevens Johnson syndrome had improved. It was unknown if filgrastim, vitamins, tobramycin, sulfamethoxazole trimethoprim, acetylcysteine, spironolactone, methylprednisolone, alprostadil, clindamycin, imipenem, aciclovir, fentanyl, phenobarbital, amphotericine B/ liposome, norepinephrine hydrochloride,

dopamine, midazolam, ambroxol, ergocalciferol/ retinol/ retinol palmitate/ phytomenadione, etacrynic acid, and furosemide therapies continued. It was unknown if the patient recovered from angioedema, ascites, or the airway obstruction. Per case 0015486501PHAMED, the patient was recovering from angioedema, ascites and Stevens Johnson syndrome. His medical history was remarkable for autosomal anomalies, upper respiratory infection, and familial history of hemophagocytic lymphohistiocytosis with cerebral involvement. He had no known allergies. The reporting physician determined the Stevens Johnson syndrome was possibly related to filgrastim.

Pfizer is the Marketing Authorization Holder of phenytoin and methotrexate sodium in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of phenytoin and methotrexate sodium has submitted the same report to the regulatory authorities.

Follow-up (21Nov2011): This is a follow-up report combining information from duplicate reports case 990408-SK015, case 0015486501PHAMED and case 049-0073-990003. The current and all subsequent follow-up information will be reported under report number case 049-0073-990003. The new information reported from a physician includes.

PFIZERINC, 13.12.2011 DE-PFIZER INC-049-0073-990003

Sender diagnosis: not reported

Narrative

Following the integration of the legacy safety adverse event reporting databases, this case has been migrated from the legacy company database into the current safety database for processing follow-up information. As a consequence of this migration, the follow-up CIOMS I or MedWatch report may indicate in the appropriate field that it is an initial report.

This is a spontaneous report from a contactable physician in Germany via Amgen pharmaceuticals, Mfr. Report #303385. A health care professional reported on 06Apr1999 that a 19-month-old male patient developed angioedema, ascites, and an airway obstruction, events considered medically significant hospitalization, and Stevens Johnson Syndrome, prolonging hospitalization eight days after receiving phenytoin. The patient was also receiving filgrastim (NEUPOGEN) intravenously, acetylcysteine, spironolactone (ALDACTONE), amphotericine B/ liposome (AMBISOME) 40 mg daily, norepinephrine hydrochloride (ARTERENOL), sulfamethoxazole trimethoprim (BACTRIM), fluconazole (DIFLUCAN), dopamine hydrochloride (DOPAMIN), midazolam maleate (DORMICUM), etoposide, fentanyl, ceftazidime pentahydrate (FORTUM), immunoglobulin human normal (GAMMA GLOBULIN), etacrynic acid (HYDROMEDIN), furosemide (LASIX), phenobarbital (LUMINAL), methotrexate, epinephrine hydrochloride (MICRONEFRIN), morphine hydrochloride, ambroxol hydrochloride (MUCOSOLVAN), metamizole sodium (NOVALGIN), pancuronium bromide, paracetamol, alprostadil (PROSTAVASIN), clindamycin hydrochloride (SOBELIN), vitamins nos (SOLUVIT), tobramycin, sulfadiazine/ trimethoprim (TRIGLOBE), methylprednisolone (URBASON), ergocalciferol/ retinol/ retinol palmitate/ phytomenadione (VITINTRA), ranitidine hydrochloride (ZANTIC), imipenem/ cilastatin sodium (ZIENAM), and aciclovir (ZOVIRAX). He started taking phenytoin (dosage unknown) on 14Nov1996. He started sulfadiazine (TRIGLOBE) on 16Nov1996. He began treatment with fluconazole on 17Nov1996. On 20Nov1996, he started taking metamizole and paracetamol. On 22Nov1996, started ceftazidime pentahydrate and took etoposide for only that day. On 24Nov1996, he started taking amphotericine B/ liposome, furosemide, tobramycin, and methylprednisolone, and phenytoin therapy was discontinued. Fluconazole treatment was discontinued on 25Nov1996 and filgrastim therapy was started for bone marrow transplantation. The patient underwent an allogenic bone marrow transplant on 26Nov1996. Vitamins were started and metamizole sodium and paracetamol therapies were discontinued on 26Nov1996. On 27Nov1996, he started methotrexate, aciclovir, immunoglobulin human normal, and Ergocalciferol/ retinol/ retinol palmitate/ phytomenadione. Sulfadiazine (TRIGLOBE) therapy was discontinued on 27Nov1996. On 29Nov1996, sulfamethoxazole trimethoprim therapy was started and methotrexate therapy was discontinued. He experienced graft versus host disease on an unspecified date. Acetylcysteine and morphine hydrochloride therapies were started on 30Nov1996, and ceftazidime pentahydrate therapy was discontinued. On 01Dec1996, he developed hemorrhagic mucositis and started treatment with alprostadil and imipenem. On 03Dec1996, he developed erosions and began treatment with clindamycin hydrochloride. On 04Dec1996, he experienced erosive oral mucosa and ascites and was started on spironolactone therapy. He developed inspiratory stridor on 06Dec1996, and was treated with epinephrine hydrochloride. The following day he experienced an acute airway obstruction. On 09Dec1996, he started taking midazolam, fentanyl, and phenobarbital. He started taking norepinephrine hydrochloride and dopamine hydrochloride on 10Dec1996, and discontinued immunoglobulin human normal that day. He developed angioedema on 11Dec1996 and was started on ambroxol hydrochloride. Ranitidine hydrochloride was discontinued on 11Dec1996. Etacrynic acid was started on 12Dec1996. On 13Dec1996, he experienced blistering and erosions of his skin and was diagnosed with Stevens Johnson syndrome. He was given pancuronium bromide only on 13Dec1996. By 05Jan1997, the Stevens Johnson syndrome had improved. It was unknown if filgrastim, vitamins, tobramycin, sulfamethoxazole trimethoprim, acetylcysteine, spironolactone, methylprednisolone, alprostadil, clindamycin, imipenem, aciclovir, fentanyl, phenobarbital, amphotericine B/ liposome, norepinephrine hydrochloride, dopamine, midazolam, ambroxol, ergocalciferol/ retinol/ retinol palmitate/ phytomenadione, etacrynic acid, and furosemide therapies continued. It was unknown if the patient recovered from angioedema, ascites, or the airway obstruction. Per case 0015486501PHAMED, the patient was recovering from angioedema, ascites and Stevens Johnson syndrome. His medical history was remarkable for autosomal anomalies, upper respiratory infection, and familial history of hemophagocytic lymphohistiocytosis with cerebral involvement. He had no known allergies. The reporting physician determined the Stevens Johnson syndrome was possibly related to filgrastim.

Pfizer is the Marketing Authorization Holder of phenytoin and methotrexate sodium in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of phenytoin and methotrexate sodium has submitted the same report to the regulatory authorities.

Follow-up (21Nov2011): This is a follow-up report combining information from duplicate reports case 990408-SK015, case 0015486501PHAMED and case 049-0073-990003. The current and all subsequent follow-up information will be reported under report number case 049-0073-990003. The new information reported from a physician includes.

Follow-up (30Nov2011): This is a follow-up spontaneous report based on information received by Pfizer from JNJ. The above mentioned case (Local

Reference 049-0073-990003) had been assigned SCEPTRE AER Number 049-0073-990003 and was designated as serious.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11240728

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19590816	Age: 51 Year	weight (kg): 90	BMI_int: 26.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestasis	20110621			0 Day	not recovered/not resolved
2 Icterus	20110621			0 Day	not recovered/not resolved
3 Transaminases increased	20110621			0 Day	not recovered/not resolved
4 Exanthema	20110621			0 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL paracetamol	Oral	1500 Mg milligram(s)/	20110619-20110621 3 Day	Cephalgia
2 tavanic levofloxacin	Oral	500 Mg milligram(s)/ 82	20110621-20110621 1 Day	Body temperature increased
3 TAVANIC levofloxacin	Oral	500 Mg milligram(s)/ 82	20110621-20110621 1 Day	Pharyngitis
4 TAVANIC levofloxacin	Oral	500 Mg milligram(s)/ 82	20110621-20110621 1 Day	Reduced general condition

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol use			Unknown	2 Bier/d
Exanthema			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20110621	2.04	UKAT/L	
Alkaline phosphatase	20110622	4.14	UKAT/L	
Alkaline phosphatase	20110624	3.05	UKAT/L	
Aspartate aminotransferase	20110621	1.92	UKAT/L	
Aspartate aminotransferase	20110627	0.67	UKAT/L	
Aspartate aminotransferase	20110624	2.52	UKAT/L	
Aspartate aminotransferase	20110622	1.90	UKAT/L	
Bilirubin total	20110622	23.4 MCMOL/INA		
Bilirubin total	20110621	56.4 MCMOL/INA		
Bilirubin total	20110624	15.8 MCMOL/INA		
GGT	20110627	5.47 MCMOL/ SIA		
GGT	20110624	7.07 MCMOL/ SIA		
GGT	20110622	5.42 MCMOL/ SIA		
GGT	20110621	5.6 MCMOL/ SNA		

GPT	20110621	2.6 MCMOL/SNA
GPT	20110622	2.53 MCMOL/ SNA
GPT	20110624	2.69 MCMOL/ SNA
GPT	20110627	1.53 MCMOL/ SNA
Hepatitis viral test NOS	20110622	NEGATIVE NA
Histology	20110624	ACUTE PORTAL HEPATITIS AND SIG. EOSINOPHILIA

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 20.12.2011 DE-SANOFI-AVENTIS-2011SA082393

Sender diagnosis: not reported

Narrative

Initial Report received on 16-Dec-2011:

This serious case (hospitalization or prolongation) was extracted from the website of the German Health Authority (authority number DE-BFARM-11240728), initially reported by an anonymous physician via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-11156074).

The case involves a 51-year-old male patient. The male started therapy with paracetamol (brand unknown) 1500 mg/day for cephalgia on 19-Jun-2011. Additionally, therapy with levofloxacin (Tavanic) 500 mg/day for pharyngitis, increased temperature and reduced general condition started on 21-Jun-2011. On the same day, occurrence of cholestasis, icterus, 2-5 fold increase in transaminases (values were not provided) and small spotted exanthema which partly occurred before treatment. No information on corrective treatment. Therapy with levofloxacin as well as with paracetamol was withdrawn immediately. Outcome of the reactions was reported as not recovered.

No details were reported on medical history and concurrent drugs. Alcohol use (2 beer/day) is known in his anamnesis.

Causality assessment from the Drug Commission due to Global Introspection (WHO GI): Possible for levofloxacin as well as for paracetamol.

refer to the international company comment, and thus do not imply a change in the state of scientific knowledge for the suspected product(s).

Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

sanofi-aventis, 07.03.2012 DE-SANOFI-AVENTIS-2011SA082393

Sender diagnosis: not reported

Narrative

Initial Report received on 16-Dec-2011:

This serious case (hospitalization or prolongation) was extracted from the website of the German Health Authority (authority number DE-BFARM-11240728), initially reported by an anonymous physician via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-11156074).

The case involves a 51-year-old male patient. The male started therapy with paracetamol (brand unknown) 1500 mg/day for cephalgia on 19-Jun-2011. Additionally, therapy with levofloxacin (Tavanic) 500 mg/day for pharyngitis, increased temperature and reduced general condition started on 21-Jun-2011. On the same day, occurrence of cholestasis, icterus, 2-5 fold increase in transaminases and 2 fold increase in cholestasis parameters and small spotted exanthema which partly occurred before treatment.

Hepatitis serology was found negative on 22-Jun-2011; histology showed acute portal hepatitis and significant eosinophilia on 24-Jun-2011 which led to suspicion of drug or environmental toxicity.

Therapy with levofloxacin as well as with paracetamol was withdrawn immediately. The patient was hospitalised and received infusion therapy and once prednisolone 100 mg iv. Outcome of the reactions: improved but not recovered at time of report.

No details were reported on medical history and concurrent drugs. Alcohol use (2 beer/day) is known in his anamnesis.

Causality assessment from the Drug Commission due to Global Introspection (WHO GI): Possible for levofloxacin as well as for paracetamol.

Course of lab values:

Day; 21-Jun-2011; 22-Jun-2011; 24-Jun-2011; 27-Jun-2011; Normal range

AP (mckat/l); 2.04; 4.14; 3.05; -; 0.65-2.20

GGT (mcmol/sl); 5.6; 5.42; 7.07; 5.47; 0.00-1.0

GOT (mckat/l); 1.92; 1.90; 2.52; 0.67; 0.00-0.85

GPT (mcmol/sl); 2.60; 2.53; 2.69; 1.53; 0.00-0.85

Bilirubin total (mcmol/l); 56.4; 23.4; 15.8; -; 0.0-20.5

Additional information received on 29-Feb-2012 via the Drug Commission:

Information on examination results, lab values and corrective treatment was amended.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11241625

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 00013 Year	weight (kg): 00060	BMI_int: 23.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute hepatic failure	20110527	2011			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DIMENHYDRINATE dimenhydrinate	Oral	00000050 Mg milligram(s)	20110517- -	Restlessness
2 ketamine ketamine	Intravenous (not otherwis	00000070 Mg milligram(s)	20110517- -	Restlessness
3 midazolam midazolam	Intravenous (not otherwis	00000001 Mg milligram(s)	20110517- -	Restlessness
4 Paracetamol paracetamol		/	20110517-20110519 3 Day	Surgery
5 promethazine promethazine	Oral	00000030 Gtt drop(s)/	20110519- -	Restlessness
6 TAVOR lorazepam	Oral	00000001 Mg milligram(s)	20110517-20110519 3 Day	Restlessness

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Torticollis			Unknown	Atlantoepistrophealis, ventral re-relaxation of the I. atlanto-axial joint, Grisel's syndrome

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 12.12.2011 DE-Bristol-Myers Squibb Company-16261943

Sender diagnosis: not reported

Narrative

Seriousness criterion: Important medical event

This case was received via the German Health Authority (BfArM number 11231446) and the Drug Commission of the German Medical Association (11156005) and BMS Germany (2011113004).

The physician reported that a 13-year-old male patient developed acute hepatic failure while on therapy with acetaminophen, lorazepam, promethazine, dimenhydrinate, midazolam hydrochloride and ketamine hydrochloride.

A patient received acetaminophen for 3 days from 17-May-2011 to 19-May-2011 (time interval between beginning of drug administration and start of event was 11 days; time interval between last dose of drug and start of reaction was 9 days) for surgery. The event occurred on 27-May-2011 and the drug was withdrawn.

As co-medication the patient was treated with oral lorazepam at a dose of 1 mg for 3 days from 17-May-2011 to 19-May-2011 (time interval between beginning of drug administration and start of event was 11 days; time interval between last dose of drug and start of reaction was 9 days), oral promethazine 30 drops since 19-May-2011 (time interval between beginning of drug administration and start of event was 9 days), oral dimenhydrinate 50 mg since 17-May-2011 (time interval between beginning of drug administration and start of event was 11 days), intravenous midazolam hydrochloride 1 mg since 17-May-2011 (time interval between beginning of drug administration and start of event was 11 days), intravenous ketamine hydrochloride 70 mg since 17-May-2011 (time interval between beginning of drug administration and start of event was 11

days), all of which were for the treatment of restlessness.

Results and test procedures relevant to the investigation of the patient from 27-May-2011 were as follows: serum glutamic oxaloacetic transaminase (SGOT) 2972 U/l; serum glutamic pyruvic transaminase (SGPT) 3664 U/l; Lactate dehydrogenase (LDH) 2657 U/l.

At the time of this report the patient had recovered.

The patient's medical history included torticollis (continuing unknown) (comment: torticollis atlantoepistrophealis, ventral re-relaxation of the I. atlanto-axial joint, Grisel's syndrome).

The Drug Commission of the German Medical Association assessed the event possibly related to acetaminophen therapy.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11243521

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19830524 Age: 00028 Year weight (kg): 00068 BMI_int: 28.3 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alanine aminotransferase increase					recovering/resolving
2 Gamma-glutamyltransferase increased					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 parfalgan		/	-	Intervertebral disc protrusion
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Prolapsed disc NOS			Unknown	
Drug therapy			Unknown	Paracetamol, diclofenac, ibuprofen, tetrazepam, cortisone
Hepatitis B immunization			Unknown	on 1999/2000

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase		300	NA	
Glutamic-pyruvate transaminase		1084	NA	
Gamma-glutamyltransferase	20111114	70	IU/L	
Glutamic-pyruvate transaminase	20111114	1085	IU/L	

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 14.12.2011 DE-Bristol-Myers Squibb Company-16244899

Sender diagnosis: not reported

Narrative

This case was received via BMS Germany (cross reference number: 2011112201 and 1-43842128).

A physician reported that a 28-year-old female patient had high levels of glutamate-pyruvate transaminase (GPT) and gamma-glutamyl transpeptidase (GGT) while on therapy with acetaminophen infusion.

The physician stated that the patient had probably received acetaminophen infusion (dose unknown) for one week while she was in hospital. It was reported that he only knew that the patient had a disc prolapse "(C45)" and that acetaminophen was discontinued in the hospital. That was why he suspected that the patient was administered acetaminophen infusion. At the time of this report, the patient had high levels of GPT (1084) and GGT (300). The physician enquired whether it would be feasible to give acetylcysteine (ACC) 3-4 days after the end of the acetaminophen infusion.

Supplemental information received from the physician on 06-Dec-2011 indicated that the patient was hospitalized from **11-Nov-2011** to 18-Nov-2011 due to intervertebral disc protrusion C4/5 and received treatment with acetaminophen infusion. The events of increased GPT and increased GGT were considered as medically significant.

On 12-Nov-2011 increase of transaminases was detected. The physician suspected the occurrence of the event after the first administration of acetaminophen. The patient was not administered with acetaminophen previously. The physician stated that the patient felt somewhat tired. No further measures were performed as cervicobrachialgia had almost faded away.

The physician reported that the patient received pre-treatment with diclofenac sodium, ibuprofen, tetrazepam and local cortisone infiltration. After that the patient was hospitalized and received acetaminophen infusion. De-challenge for acetaminophen was reported to be negative and re-challenge was reported to be not applicable.

Letter from the hospital indicated that during in-patient treatment an increase of transaminase appeared. The sonography showed inconspicuous abdominal findings. Abdomen sonography and Internist council recommend discontinuing acetaminophen and to continue laboratory controls. In the further course an improvement of transaminase values appeared, which should be controlled in the further outpatient course. It was not unambiguously sure that acetaminophen was the only cause of this liver value increase.

Laboratory investigation done on 14-Nov-2011 revealed GPT at 1085 IU/l and GGT at 70 IU/l. At the time of this report the events of increased GPT and GGT were improving.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12006449

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19490609	Age: 62 Year	weight (kg): 70	BMI_int: 27.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alanine aminotransferase increased					recovered/resolved
2 Hepatitis acute	20110801	20110815	15 Day	24 Day	recovered/resolved
3 Aspartate aminotransferase increased					recovered/resolved
4 Alkaline phosphatase increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 75 diclofenac sodium	Oral	175 Mg milligram(s)/Day 212	20110711-20110711	Ache
2 paracetamol paracetamol	Oral	1 Df dosage form/7 Hour 245	20110708-20110710 3 Day	Lumbago

Drugs (concomitant)

3 lidocaine lidocaine	Unknown	1 Df dosage form/Day 120	20110711-20110711 1 Day	Lumbago
4 PREDNISOLONE prednisolone	Unknown	/	20110711-20110711 1 Day	Lumbago

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fatty liver	201108		Yes	
Depressive episode	201108		Yes	
Polymyalgia rheumatica	201108		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20110801	128	U/l	
Alanine aminotransferase increased	20110805	87	U/l	
Alanine aminotransferase increased	20110815	40	U/l	
Aspartate aminotransferase increased	20110801	49	U/l	
Aspartate aminotransferase normal	20110809	20	U/l	
Alkaline phosphatase increased	20110802	116	U/l	
Alkaline phosphatase increased	20110805	124	U/l	
Bilirubin normal	20110802	0.48	mg/dl	
Histology NOS	201108	SINGS OF DRUG-INDUCED HEPATIC DAMAGE		
Ultrasound abdomen	201108	FATTY LIVER		
Hepatitis viral test	201108	NEGATIVE	NA	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 18.01.2012 DE-FAKOS-170435

Sender diagnosis: not reported

Narrative

Die 62-jährige Patientin wurde am 01.08.2011 aufgrund unklarer Ganzkörperschmerzen mit Abgeschlagenheit stationär aufgenommen. Ca. 4 Wochen vor der Einweisung hatte sie einen Hexenschuss erlitten, daraufhin bekam sie Injektionen mit Lidocain und Prednisolon im LWS (Lendenwirbelsäulen)-Bereich sowie Diclofenac und Paracetamol per os. Im Aufnahmelabor (01.08.2011) erhöhter ALAT (Alanin-Aminotransferase)-Wert (128 U/l, Normbereich: bis 35 U/l), die Hepatitis-Serologie war unauffällig. Die Abdomensonographie zeigte eine Steatosis hepatis, in der Histologie Zeichen einer medikamentös-toxischen Leberschädigung, die Kriterien einer NASH (Nichtalkoholische Steatohepatitis) wurden hingegen nicht erfüllt. Im Verlauf kam es zu einer Normalisierung der ALAT (15.08.2011: 40 U/l). Am 18.08.2011 wurde die Patientin in gebessertem Allgemeinzustand aus dem Krankenhaus entlassen.

Die Schmerzen waren am ehesten im Rahmen einer Polymyalgia rheumatica zu diskutieren, nach Kortison-Behandlung kam es zu einem deutlichen Rückgang der Symptomatik.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Diclac 75 ID bzw. Diclac retard 100 mg Retardtbl. Hexal, Einnahmezeitraum: am 11.07.2011. (jeweils 1x1 Tabl.)

Paracetamol Tabl. NN, Einnahmezeitraum: 08.-10.07.2011.

Eine UAW durch Diclac und/ oder Paracetamol ist wahrscheinlich, die restlichen Arzneimittel wurden lokal appliziert. Die Arzneimittelkausalität wird ebenfalls mit "wahrscheinlich" bewertet. Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

PVZ-FAKOS, Klinische Pharmakologie, 18.01.2012 DE-FAKOS-170435

Sender diagnosis: not reported

Narrative

Die 62-jährige Patientin wurde am 01.08.2011 aufgrund unklarer Ganzkörperschmerzen mit Abgeschlagenheit stationär aufgenommen. Ca. 4 Wochen vor der Einweisung hatte sie einen Hexenschuss erlitten, daraufhin bekam sie Injektionen mit Lidocain und Prednisolon im LWS (Lendenwirbelsäulen)-Bereich sowie Diclofenac und Paracetamol per os. Im Aufnahmelabor (01.08.2011) erhöhter ALAT (Alanin-Aminotransferase)-Wert (128 U/l, Normbereich: bis 35 U/l), die Hepatitis-Serologie war unauffällig. Die Abdomensonographie zeigte eine Steatosis hepatis, in der Histologie Zeichen einer medikamentös-toxischen Leberschädigung, die Kriterien einer NASH (Nichtalkoholische Steatohepatitis) wurden hingegen nicht erfüllt. Im Verlauf kam es zu einer Normalisierung der ALAT (15.08.2011: 40 U/l). Am 18.08.2011 wurde die Patientin in gebessertem Allgemeinzustand aus dem Krankenhaus entlassen.

Die Schmerzen waren am ehesten im Rahmen einer Polymyalgia rheumatica zu diskutieren, nach Kortison-Behandlung kam es zu einem deutlichen Rückgang der Symptomatik.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Diclac 75 ID bzw. Diclac retard 100 mg Retardtbl. Hexal, Einnahmezeitraum: am 11.07.2011. (jeweils 1x1 Tabl.)

Paracetamol Tabl. NN, Einnahmezeitraum: 08.-10.07.2011.

Eine UAW durch Diclac und/ oder Paracetamol ist wahrscheinlich, die restlichen Arzneimittel wurden lokal appliziert. Die Arzneimittelkausalität wird ebenfalls mit "wahrscheinlich" bewertet. Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 26.01.2012 DE-SANOFI-AVENTIS-2012SA004179

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre (DE-FAKOS-170435) and was extracted from the website of the German health authority (authority number: DE-BFARM-12006449) on 20-Jan-2011. The pharmacovigilance (PV) centre is conducting a hospitalization registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 62-year-old female patient who was admitted to hospital due to unclear pain at entire body with exhaustion on 01-Aug-2011. About 4 weeks prior to the hospitalization, she had suffered lumbago and been treated with paracetamol (no trade name provided) 1x1 tablet every seven hours orally from 08-Jul-2011 to 10-Jul-2011 and received injections with lidocaine (1x1 syringe local) and prednisolone (no dose provided) into the lumbar spine area as well as oral diclofenac (1x75 mg and 1x100 mg) on 11-Jul-2011.

Laboratory test at the hospital admission (01-Aug-2011) detected increased alanine aminotransferase (ALAT) (128 U/l, normal high range: 35 U/l).

Serology for hepatitis was unremarkable.

Abdominal ultrasound showed fatty liver and histology revealed signs of drug-induced hepatic damage. However, the criteria of nonalcoholic steatohepatitis were not met.

Following laboratory values were additionally reported:

Alanine aminotransferase: 87 U/L (05-Aug-2011)

Aspartate aminotransferase (normal high range: 35 U/L): 49 U/L (01-Aug-2011), 20 U/L (09-Aug-2011)

Alkaline phosphatase (normal range: 35 - 105 U/L): 116 U/L (02-Aug-2011), 124 U/L (05-Aug-2011)

Bilirubin (normal high range: 1.2 mg/dl): 0.48 mg/dl (02-Aug-2011)

In the clinical course, the ALAT returned to be normal (15-Aug-2011: 40 U/l). On 18-Aug-2011 the patient was discharged from hospital in the improved general condition. The pain was considered to be most likely in the context of polymyalgia rheumatica and after the cortisone therapy the symptom significantly abated. In addition to fatty liver and polymyalgia rheumatica, depressive episode was reported as medical history (start: August 2011).

The PV centre reported acute hepatitis from 01-Aug-2011 to 15-Aug-2011 as adverse drug reaction. Diclofenac and paracetamol were suspected.

Causality assessment by the PV centre concerning acute hepatitis:

"probable/likely": diclofenac, paracetamol

Sender's comments:

Initial report, complete information. Under diclofenac (Diclac 75 ID or 100 mg retard tablets of Hexal), "liver cell necrosis" is labelled as "adverse events", as of: Aug-2009 or Apr-2011.

Under paracetamol tablets of Ratiopharm, "increase in the liver transaminases" is labelled as "adverse events", as of: Nov-2008.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12017754

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: 19471207 Age: 64 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quick's value decreased	20120127				recovered/resolved
2 Intentional overdose	20120127				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CITALOPRAM 20 citalopram	Oral	400 Mg milligram(s)/ 245	20120127-20120127	Suicide attempt
2 MARCUMAR phenprocoumon	Oral	60 Mg milligram(s)/ 245	20120127-20120127	Suicide attempt
3 PARACETAMOL 500 paracetamol	Oral	10 G gram(s)/ 245	20120127-20120127	Suicide attempt

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Coagulation test	20120127	10	%	

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 03.02.2012 DE-MEDA-M-EU-2012010594

Sender diagnosis: not reported

Narrative

A report was received by the GIFTINFORMATIONSZENTRALE (German Drug and Poison Information Call Center, reference no. AZ 2012002098) on 27-Jan-2012.

A hospital physician reported on a 64-year-old male patient who attempted suicide by taking an intentional overdose of presumably 20 tablets of Marcumar 3(phenprocoumon, 60 mg in total), 20 tablets of Paracetamol 500 (paracetamol, 10.000 mg in total) and 20 tablets of Citalopram 20 (citalopram, 400 mg in total), all were taken orally on 27-Jan-2012.

At the time of the report the patient had not experienced any adverse events. Control of coagulation was reported as 10 % (Quick's value assumed). The GIFTINFORMATIONSZENTRALE recommended the following diagnostics and therapies: blood level, acetylcysteine therapy, symptomatic therapy, Vitamin K, fresh frozen plasma if necessary as well as intensive care treatment.

A the time of reporting the outcome was unknown.

Reporter's comment:

MEDA Pharma GmbH & Co. KG, 10.04.2012 DE-MEDA-M-EU-2012010594

Sender diagnosis: not reported

Narrative

A report was received by the GIFTINFORMATIONSZENTRALE (German Drug and Poison Information Call Center, reference no. AZ 2012002098) on 18.03.2013

27-Jan-2012.

A hospital physician reported on a 64-year-old male patient who attempted suicide by taking an intentional overdose of presumably 20 tablets of Marcumar 3(phenprocoumon, 60 mg in total), 20 tablets of Paracetamol 500 (paracetamol, 10.000 mg in total) and 20 tablets of Citalopram 20 (citalopram, 400 mg in total), all were taken orally on 27-Jan-2012.

At the time of the report the patient had not experienced any adverse events. Control of coagulation was reported as 10 % (Quick's value assumed). The GIFTINFORMATIONSZENTRALE recommended the following diagnostics and therapies: blood level, acetylcysteine therapy, symptomatic therapy, Vitamin K, fresh frozen plasma if necessary as well as intensive care treatment.

A the time of reporting the outcome was unknown.

A follow-report was created by the GIFTINFORMATIONSZENTRALE (reference no. AZ 2012002098)) on 03-Apr-2012.

A hospital physician reported that the patient was hospitalised and monitored. He was treated with vitamin K and acetylcysteine. The patient recovered completely within an unspecified time.

A causality assessment by the reporter was not provided.

expected. The causality to Marcumar is assessed as probable. Regarding the event term intentional overdose the causality is not assessed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12021531

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Female	Birth: 19320618	Age: 79 Year	weight (kg): 69	BMI_int: 26.95	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver cirrhosis	20120112				not recovered/not resolved
2 Lipids increased					not recovered/not resolved
3 Jaundice	20120112				recovering/resolving
4 Iron increased	20120126				not recovered/not resolved
5 Fatigue	20120112				recovering/resolving
6 Ascites	20120112				recovering/resolving
7 Edema peripheral	20120112				recovering/resolving
8 Liver function tests abnormal	201201				Unknown
9 Inappetence	20120112				recovering/resolving
10 Urine discoloration	20120112				recovering/resolving
11 Pruritus	20120112				recovering/resolving
12 Nausea	20120112				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CHLORMADINONE chlormadinone	Oral	1 Df dosage form/Day 245	1992-20120127 -	Menopausal symptoms
2 estradot estradiol	Transdermal	/ 250	1992-20120127 -	Hormone replacement therapy
3 PARACETAMOL paracetamol	Oral	/ 245	1992- -	Dysmenorrhea

Drugs (concomitant)

4 CETIRIZINE cetirizine	Oral	/ 245	20090326- -	Allergy
5 METOPROLOL SUCCINATE metoprolol succinate	Oral	1 Df dosage form/Day 245	2005- -	Hypertension
6 RAMIPRIL ramipril	Oral	1 Df dosage form/Day 245	2005- -	Hypertension

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ultrasound scan NOS			Unknown	
Hypercholesterolemia			Unknown	
Allergy			Unknown	
Histaminosis			Unknown	
Urticaria			Unknown	
Pollinosis			Unknown	
Hypertension arterial			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20120328	198	U/L	
Alkaline phosphatase	20120126	367	U/L	
Alkaline phosphatase	20120229	286l	U/L	
Alkaline phosphatase	20120214	380	U/L	
Aspartate aminotransferase	20120126	801	U/L	
Aspartate aminotransferase	20120214	391	U/L	
Aspartate aminotransferase	20120229	223	U/L	
Aspartate aminotransferase	20120328	199	U/L	
Aspartate aminotransferase	20120417	151	U/L	
Aspartate aminotransferase	20090720	20	U/L	
Bilirubin	20120126	6.7	mg/dl	
Bilirubin	20120214	5.2	mg/dl	
Bilirubin	20120229	3.8	mg/dl	
Bilirubin	20120328	2	mg/dl	
Bilirubin	20120417	1.8	NA	
GGT	20120126	320	U/L	
GGT	20120214	231	U/L	
GGT	20120229	149	U/L	
GGT	20120328	121	U/L	
GGT	20120417	101	U/L	
GGT	20090720	60	U/L	
GPT	20090720	14	U/L	
GPT	20120417	78	U/L	
GPT	20120328	78	U/L	
GPT	20120229	106	U/L	
GPT	20120214	224	U/L	
GPT	20120126	384	U/L	
Quick's test	20120229	57	%	
Serum ferritin	20120126	2140	ng/ml	
Serum ferritin	20120214	1290	ng/ml	
Serum ferritin	20120229	789	ng/ml	
Serum ferritin	20120328	637	ng/ml	
Serum ferritin	20120417	454	ng/ml	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.02.2012 DE-002147023-PHHY2012DE008655

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE008655, is an initial spontaneous report received from a consumer (patient's relative) on 31 Jan 2012. This report refers to an adult female patient. The patient's medical history included a treatment with Praemulud, a gestagen containing drug for post menopausal hormone replacement therapy. Concomitant medication included an unspecified gestagen containing drug. The patient received Estradot (estradiol) 50mcg/24 hours, transdermal route, for the treatment of post menopausal hormone replacement therapy, for approximately 30 years at a dose of 1 patch every 3 or 4 days. On Jan 2012, the patient developed jaundice and edema of lower legs. The patient was diagnosed with liver cirrhosis by ultrasound. The patient's liver values and iron storage values were increased. Also, lipid values were reported as slightly increased. On Jan 2012, the

patient was admitted to the hospital due to these events. The treatment with Estradot was discontinued on Jan 2012. The patient would have a liver biopsy on an unknown date. The events outcomes and causality were not reported.

Information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

Edema Peripheral [hospitalization], assessed as listed according to the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

MR: Review of available reported data does not allow for a proper causality assessment.

Reporter's comment:

NOVARTIS, 08.05.2012 DE-002147023-PHHY2012DE008655

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE008655, is an initial spontaneous report received from a consumer (patient's relative) on 31 Jan 2012 with follow up physician report received on 02 May 2012. This report refers to a 79-year-old female patient. The patient's medical history included a treatment with Praemulud, a gestagen containing drug for post menopausal hormone replacement therapy. The patient also had arterial hypertension, hypercholesterolaemia, pollinosis, total endoprosthesis of hip joint due to arthrosis, urticaria, and allergies against grasses, early blossoms and suspicion of histaminosis. Concomitant medication included an unspecified gestagen containing drug, metoprolol succinate tablet, ramipril tablet and cetirizine tablet. The patient received Estradot (estradiol) 50mcg/24 hours, transdermally, for the treatment of post menopausal hormone replacement therapy, since 1992 at a dose of 1 patch every 3 or 4 days. The patient also received chlormadinone (manufacturer unknown) 2 mg tablet for menopausal complaint at 2 mg per day since 1992 and paracetamol (manufacturer unknown) 500 mg tablet for dysmenorrhea at 3-4 times per month since 1992. On 20 Jul 2009, gamma-glutamyltransferase (GGT) was at 60 U/l (normal range: 40), aspartate aminotransferase (GOT) was at 20 U/l (normal range: <35), alanine aminotransferase (GPT) was at 14 (normal range: <35). On 12 Jan 2012, the patient developed jaundice, inappetence and edema of feet. The patient was admitted to hospital for etiological evaluation of symptoms. The patient was diagnosed with liver cirrhosis classified as child B by ultrasound of liver. Liver function test, hepatitis testing, autoantibodies, ultrasound of abdomen and liver/ gall were also performed. Patient presented with jaundice, dark urine, pruritus, inappetence/anorexia, ascites, fatigue and nausea. ACE-inhibitor, estrogen, paracetamol and cetirizine were administered within the prior 6 months. The patient's liver values and iron storage values were increased. Also, lipid values were reported as slightly increased. On 26 Jan 2012, Gamma-glutamyltransferase (GGT) was at 320 U/l, aspartate aminotransferase (GOT) was at 801 U/l, alanine aminotransferase (GPT) was at 384, alanine phosphatase was at 367 U/l (normal range <105), bilirubin was at 6.7 mg/dl (normal range <1.1) and ferritin was at 2140 ng/ml (normal range 10-291). Therapy with Estradot and chlormadinon was discontinued on 27 Jan 2012. On 14 Feb 2012, gamma-glutamyltransferase (GGT) was at 231 U/l, aspartate aminotransferase (GOT) was at 391 U/l, alanine aminotransferase (GPT) was at 224, bilirubin was at 5.2 mg/dl and ferritin was at 1290 ng/ml. On 29 Feb 2012, Quick test was at 57 % (70- 100). On 28 Mar 2012, gamma-glutamyltransferase (GGT) was at 121 U/l, aspartate aminotransferase (GOT) was at 199 U/l, alanine aminotransferase (GPT) was at 78, bilirubin was at 2.0 mg/dl (normal range <1.1) and ferritin was at 637 ng/ml (normal range 10-291). On 17 Apr 2012, gamma-glutamyltransferase (GGT) was at 101 U/l, aspartate aminotransferase (GOT) was at 151 U/l, alanine aminotransferase (GPT) was at 78, bilirubin was at 1.8 mg/dl and ferritin was at 454 ng/ml. The event outcome for jaundice, inappetence and edema of feet was reported as condition improving while outcome for other event were condition unchanged. Jaundice, inappetence and edema of feet were assessed as medical significant by physician. Causality was not reported.

Follow up physician report received on 02 May 2012: Added additional co-suspect drug, concomitant medication, medical history, lab. Value, event outcome and seriousness for Jaundice, inappetence and edema of feet.

NOVARTIS, 08.05.2012 DE-SZ09-PHHY2012DE008655

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE008655, is an initial spontaneous report received from a consumer (patient's relative) on 31 Jan 2012 with follow up physician report received on 02 May 2012. This report refers to a 79-year-old female patient. The patient's medical history included a treatment with Praemulud, a gestagen containing drug for post menopausal hormone replacement therapy. The patient also had arterial hypertension, hypercholesterolaemia, pollinosis, total endoprosthesis of hip joint due to arthrosis, urticaria, and allergies against grasses, early blossoms and suspicion of histaminosis. Concomitant medication included an unspecified gestagen containing drug, metoprolol succinate tablet, ramipril tablet and cetirizine tablet. The patient received Estradot (estradiol) 50mcg/24 hours, transdermally, for the treatment of post menopausal hormone replacement therapy, since 1992 at a dose of 1 patch every 3 or 4 days. The patient also received chlormadinone (manufacturer unknown) 2 mg tablet for menopausal complaint at 2 mg per day since 1992 and paracetamol (manufacturer unknown) 500 mg tablet for dysmenorrhea at 3-4 times per month since 1992. On 20 Jul 2009, gamma-glutamyltransferase (GGT) was at 60 U/l (normal range: 40), aspartate aminotransferase (GOT) was at 20 U/l (normal range: <35), alanine aminotransferase (GPT) was at 14 (normal range: <35). On 12 Jan 2012, the patient developed jaundice, inappetence and edema of feet. The patient was admitted to hospital for etiological evaluation of symptoms. The patient was diagnosed with liver cirrhosis

classified as child B by ultrasound of liver. Liver function test, hepatitis testing, autoantibodies, ultrasound of abdomen and liver/ gall were also performed. Patient presented with jaundice, dark urine, pruritus, inappetence/anorexia, ascites, fatigue and nausea. ACE-inhibitor, estrogen, paracetamol and cetirizine were administered within the prior 6 months. The patient's liver values and iron storage values were increased. Also, lipid values were reported as slightly increased. On 26 Jan 2012, Gamma-glutamyltransferase (GGT) was at 320 U/l, aspartate aminotransferase (GOT) was at 801 U/l, alanine aminotransferase (GPT) was at 384, alanine phosphatase was at 367 U/l (normal range <105), bilirubin was at 6.7 mg/dl (normal range <1.1) and ferritin was at 2140 ng/ml (normal range 10-291). Therapy with Estradot and chlormadinon was discontinued on 27 Jan 2012. On 14 Feb 2012, gamma-glutamyltransferase (GGT) was at 231 U/l, aspartate aminotransferase (GOT) was at 391 U/l, alanine aminotransferase (GPT) was at 224, bilirubin was at 5.2 mg/dl and ferritin was at 1290 ng/ml. On 29 Feb 2012, Quick test was at 57 % (70- 100). On 28 Mar 2012, gamma-glutamyltransferase (GGT) was at 121 U/l, aspartate aminotransferase (GOT) was at 199 U/l, alanine aminotransferase (GPT) was at 78, bilirubin was at 2.0 mg/dl (normal range <1.1) and ferritin was at 637 ng/ml (normal range 10-291). On 17 Apr 2012, gamma-glutamyltransferase (GGT) was at 101 U/l, aspartate aminotransferase (GOT) was at 151 U/l, alanine aminotransferase (GPT) was at 78, bilirubin was at 1.8 mg/dl and ferritin was at 454 ng/ml. The event outcome for jaundice, inappetence and edema of feet was reported as condition improving while outcome for other event were condition unchanged. Jaundice, inappetence and edema of feet were assessed as medical significant by physician. Causality was not reported.

Follow up physician report received on 02 May 2012: Added additional co-suspect drug, concomitant medication, medical history, lab. Value, event outcome and seriousness for Jaundice, inappetence and edema of feet.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12037357

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 43 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Hepatitis B
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fulminant hepatic failure					fatal
2 Cytomegalovirus infection					fatal
3 Cholestatic hepatitis					fatal
4 Drug intoxication					fatal
5 Hepatitis B					fatal
6 Graft failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL		/	-	
paracetamol			-	
2 tacrolimus		/	-	
tacrolimus			-	
3 tacrolimus		/	-	
tacrolimus			-	
4 tacrolimus		/	-	
tacrolimus			-	
5 tacrolimus		/	-	
tacrolimus			-	
6 tacrolimus		/	-	Immunosuppressant drug therapy
tacrolimus			-	

Drugs (concomitant)

7 EMTRICITABINE		/	-	HIV infection
emtricitabine			-	
8 HBS-HYPERIMMUNOGLOBULIN	Intramuscular	/Month	-	
immunoglobulin antihepatitis b			-	
9 HBS-HYPERIMMUNOGLOBULIN	Intravenous (not otherwis 10000 lu international unit		-	Hepatitis prophylaxis
immunoglobulin antihepatitis b			-	
10 LAMIVUDINE		300 Mg milligram(s)/Day	-	HIV infection
lamivudine			-	
11 TENOFOVIR		/	-	HIV infection
tenofovir			-	
12 TRIMETHOPRIM - SULPHAMETHOXASAZOL		/	-	Antifungal prophylaxis
sulfamethoxazole; trimethoprim			-	
13 TRIMETHOPRIM - SULPHAMETHOXASAZOL		/	-	Pneumocystis carinii pneumonia
sulfamethoxazole; trimethoprim			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chronic hepatitis B			Unknown	
End stage liver disease			Unknown	
HIV infection			Unknown	
Liver transplantation			Unknown	
Drug resistance			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Vernadakis S,Sotiropoulos G,Brokalaki E,Esser S,Kaiser G,Cicinnati V,et al. Long-term outcomes of liver transplant patients with Human Immunodeficiency Virus infection and end-stage-liver-disease: single center experience. Eur J Med Res 2011;16:8:342-348.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
CD4 lymphocytes		219	counts/mcl	
HIV viral load		1450	copies/ml	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 23.02.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-00933GD

Sender diagnosis: not reported

Narrative

P12-01757

A 43-year-old female patient (ID 1, table 1, 2 and 3) with end stage liver disease (MELD score 27), HIV infection (CDC stage A2; diagnosed at liver transplantation, see below) and HBV coinfection is reported. The patient had fulminant hepatic failure due to paracetamol intoxication secondary to chronic HBV. The patient underwent orthotopic liver transplantation (OLT, whole graft, from deceased donor). Post-transplant immunosuppression consisted of tacrolimus with tapering corticosteroids. Cytomegalovirus infection (CMV) occurred as early postoperative complication (diagnosed within 4-8 weeks after OLT) and was successfully treated.

The patient experienced HBV recurrence resulting in progressive HBV-associated cholestatic hepatitis with secondary graft failure and the patient's death 34 months post-OLT. HBV recurrence as a consequence of lamivudine resistance is considered by the authors.

Graft failure after HBV recurrence was reported as cause of death.

In the patient who had not been on HAART pre-OLT, HAART including lamivudine, tenofovir and emtricitabine was initiated after OLT.

The case is from a retrospective analysis of a group of HIV-infected patients with end-stage liver disease (ESDL) that underwent orthotopic liver transplantation (OLT). The aim was to describe the long-term outcomes and the management strategies of HIV positive patients with ESDL who undergo OLT.

8 Patients with ESDL who underwent OLT at the University Hospital of Essen, from January 1996 to December 2009 were included.

The same standard criteria for listing a non-HIV ESDL-patient were used, in order for a HIV-infected patient to be waitlisted for OLT. Additional required criteria were a CD4 cell count >100 cells/mcl and a serum HIV-viral load in undetectable levels <50 copies/ml.

Immunosuppression consisted of tacrolimus-based dual therapy with tapering corticosteroids over a 3-month period. Tacrolimus doses were adjusted to achieve a target trough level of 12-15 ng/ml during the first week post-OLT, 10-12 ng/ml during the first 3 months posttransplant, 8-10 ng/ml thereafter during months 4-6, 6-8 ng/ml during months 7-12 and approximately 5 ng/ml thereafter.

Cyclosporine A (CSA) based immunosuppression plus prednisone for induction was also used. CSA was dosed according to target levels (150-200 ng/ml for weeks 0-2, 100-150 ng/ml for weeks 3-11, 75-125 ng/ml for week 12 and thereafter).

The HBV-recurrence prevention protocol consisted of HBs-hyperimmunoglobulin administration of 10.000 IU i.v starting during the anhepatic phase of OLT, followed by 10.000 IU i.v once pro day for 1 week and maintenance therapy by monthly intramuscular infusions in order to keep serum levels of anti-HBs >100 IU/ml. All patients transplanted for HBV infection received dual immunoprophylaxis with additional lamivudine administration 300 mg/d (additional lamivudine, tenofovir and emtricitabine were part of the post-OLT HAART therapy; it is mentioned that each patient received individualized regimen based on known response and resistance patterns and preference of the physician).

Patients with preexisting HCV infection were treated with pegylated interferon and ribavirin combination therapy post transplant, as soon as stable immunosuppression after initiation of HAART was established.

All patients received anti-fungal prophylaxis and anti-pneumocystis carinii pneumonia prophylaxis with trimethoprim - sulphamethoxazole after OLT. Anti-cytomegalovirus and herpes simplex virus prophylaxis was also administered with ganciclovir, acyclovir and/or cytomegalovirus immune globulin. HAART therapy was continued posttransplant in all HIV infected patients (in the single patient who had not received HAART before OLT, it was initiated after the OLT).

Authors' statement: 'Five patients died 1, 3, 10, 31 and 34 months posttransplant respectively as a result of severe septic conditions after early postoperative complications such as recurrent thrombosis of the hepatic artery, biliary leakage with abscess formation, situations causing mainly graft failure. In conjunction to our observations, these data suggest that HIV-infected population is particularly prone to infection as a cause of mortality after OLT despite good control of the HIV disease. (...) Recurrent hepatitis C infection remains a serious problem in HIV-negative patients. Because the degree of immunosuppression is a key factor on the activity of recurrent HCV infection, there has been concern that HIV-infected recipients may be at

particular risk of experiencing aggressive HCV reinfection. (...) it is possible that HAART-induced hepatotoxicity may have contributed to graft dysfunction in HCV coinfecting patients. It is possible that inadequate HIV treatment and therefore immunosuppression related to HIV may have led to more aggressive recurrence of HCV infection. However, any effect on outcome is likely to reflect efficacy of the combination and not an effect of an individual drug. (...) Morbidity due to chronic infection with chronic-hepatitis-B (HBV) and HCV viruses has become a leading cause of death in HIV-infected patients with synchronous chronic viral hepatitis. (...) HCV recurrent infection can affect both graft and patient survival in the medium and long-term.'

4 Additional single cases available:

2012-EU-00944GD

2012-EU-00952GD

2012-EU-00956GD

2012-EU-00959GD

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12045675

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19540405	Age: 57 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Attempted suicide	20120219				Unknown
2 Elevated liver enzyme levels	20120219				Unknown
3 Intentional drug misuse	20120219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Oral	/	20120219-20120219	Suicidal ideation
paracetamol		245	1 Day	
2 Vivinox Sleep Schlaftabletten stark	Oral	1000 Mg milligram(s)/	20120219-20120219	Suicidal ideation
diphenhydramine hydrochloride		245	1 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ELEVATED LIVER ENZYME LEVELS	20120219	UNKNOWN	UNKNOWN	
BLOOD ALCOHOL	20120219	2.8	PER MILLE	
BLOOD GASES NORMAL	20120219	7.36	PH	

Narratives (nur bei elektronischem Eingang)

Bausch & Lomb Incorporated, 05.03.2012 DE-BAUSCH-2012BL001161

Sender diagnosis: not reported

Narrative

A report was received from a physician working in a hospital via a Regional Poisons Information Center indicating that a 57-year-old male with a body weight of 60 kg attempted suicide (SUICIDE ATTEMPT) on 19-Feb-2012 by taking a maximum of 20 tablets of VIVINOX SLEEP STARK SCHLAFTABLETTE [DIPHENHYDRAMINE HYDROCHLORIDE] (INTENTIONAL DRUG MISUSE) (= 1000 mg) together with an unspecified amount of PARACETAMOL [manufacturer unknown] and an unknown amount of cognac. The patient showed elevated liver values (HEPATIC ENZYME INCREASED) and a blood alcohol level of 2.8 per mille. A blood gas analyses revealed a pH value of 7.36, blood level of Paracetamol was below the threshold of detection. According to the reporter the symptoms were assessed as moderate and hospitalization was recommended. The event outcome was not reported. No further information was provided.

Of the patient was reported 60 kg. Therefore, the dose was considered as not life-threatening. The overall assessment of this case was assessed as serious, expected and probably related to the product. In the SmPC of Vivinox Sleep Schlaftabletten stark it is clearly stated that the recommended dosage of 50 mg diphenhydramine should not be exceeded and that the product should only be used for a maximum of 2 weeks consecutively. Warnings regarding dependency and tolerance effects are included in the SmPC and the patient information leaflet. Signs and symptoms of overdoses or intoxications are explicitly described therein.

With regards to the drug substance paracetamol it is clearly stated that the maximum daily dosage should not be exceeded. Furthermore it is mentioned in the SmPC that consumption of alcoholic beverages leads to increased toxicity of the drugs. Therefore further measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12055731

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 57 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Acute liver failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ethanol	Oral	/	-	Drug use for unknown indication
ethanol			-	indication
2 Paracetamol	Oral	/	-	Drug use for unknown indication
paracetamol			-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012; doi: 10.1016/j.cgh.2012.02.016	
Qualification:	Other Health Professional	observestudytype:
Literature:	Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012; doi: 10.1016/j.cgh.2012.02.016	
Qualification:	Physician	observestudytype:
Literature:	Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012;doi: 10.1016/j.cgh.2012.02.016	

Narratives (nur bei elektronischem Eingang)Krewel Meuselbach GmbH, 16.03.2012 DE-KREWEL-003438

Sender diagnosis: not reported

Narrative

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF) a 57-year-old male patient (case 24 in article) was identified who developed ALF after ingestion of paracetamol. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 24 in article: in a 57-year old male patient, the suspected ALF aetiology was paracetamol. Significant comorbidity was alcohol abuse. The patient died due to ALF. Days between referral and death were 35. The patient was one of 20 patients who died after ALF. The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 824%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

Dr. R. Pfleger Chemische Fabrik GmbH, 21.03.2012 DE-PFLEGER-001678

Sender diagnosis: not reported

Narrative

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF) a 57-year-old male patient (case 24 in article) was identified who developed ALF after ingestion of paracetamol. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 24 in article: In a 57-year old male patient, the suspected ALF aetiology was paracetamol. Significant comorbidity was alcohol abuse. The patient died due to ALF. Days between referral and death were 35. The patient was one of 20 patients who died after ALF.<dbtext>

To_Char_NARRATIVE_2</dbtext>

excluded. Therefore, the causality related to the drug is evaluated as possible.

In the RSI (chapter overdose) is pointed out that a risk of intoxication exists especially in people with chronic alcohol abuse which can lead to death. This information is considered as sufficient at the moment. Moreover, the company recommends in the instruction for use (package leaflet): If the disorders would last more than 3 days a physician should be contacted.

Reporter's comment:

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate (24%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

MENARINIP, 23.03.2012 DE-MEN-25855

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, was presented within a publication concerning a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF). Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Thirty cases had a fatal outcome. Two of these patients had received ibuprofen and paracetamol, respectively.

This case involved patient 24, a 57-year old male patient, with suspected ALF aetiology of paracetamol. Significant comorbidity was alcohol abuse. The patient died due to ALF. Days between referral and death were 35.

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate (24%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

This case was derived from the same publication as DE-MEN-25854 (ibuprofen).

Heumann Pharma GmbH & Co. Generika KG, 30.03.2012 DE-HEUMANN1-001789

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF) a 57-year-old male patient (case 24 in article) was identified who developed ALF after ingestion of paracetamol. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with

ALF.

Case 24 in article: in a 57-year-old male patient, the suspected ALF aetiology was paracetamol. Significant comorbidity was alcohol abuse. The patient died due to ALF. Days between referral and death were 35. The patient was one of 20 patients who died after ALF.<dbtext>

To_Char_NARRATIVE_2</dbtext>

abuse of the patient. Therefore, this case is assessed as possibly related to paracetamol. This case does not affect the positive benefit/risk ratio of Paracetamol Heumann.

Reporter's comment:

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 824%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

Boehringer Ingelheim GmbH, Germany, 22.06.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-03166GD

Sender diagnosis: not reported

Narrative

P12-07087

A 57-year-old male patient (case 24, suppl. table 1) is reported who developed drug-induced acute liver failure (ALF) with hepatic encephalopathy due to use of acetaminophen. The patient had alcohol abuse as significant comorbidity and died on day 35 after referral and diagnosis of ALF.

In 2010, all university clinics participating in the Acute Liver Failure Study Group Germany (including all major liver transplant centers) were asked to retrospectively identify patients with imminent and overt primary ALF treated at their clinics during a 2-year period (2008-2009).

Eleven university medical centers in Germany were asked to report patients with (primary) severe acute liver injury (sALI) (international normalized ratio (INR) >1.5 but no hepatic encephalopathy) and primary ALF (INR > 1.5 with overt hepatic encephalopathy) treated from 2008 to 2009. Data were analyzed from 46 patients with sALI and 109 patients with ALF.

Imminent ALF or severe acute liver injury (sALI) was defined as acute (< 12 weeks) liver dysfunction leading to an INR >1.5, but without documented level of hepatic encephalopathy (HE). Overt ALF was defined as acute (<12 weeks) liver dysfunction leading to an INR > 1.5 plus any documented level of HE.

Two hundred seventy-two patients were reported by the centers and assumed to have ALF. The remaining 46 patients with primary sALI and 109 patients with primary ALF were included in the analysis.

2 additional cases are available:

2012-EU-03163GD

2012-EU-03167GD

BAUSCH, 11.10.2012 DE-BAUSCH-BL-2012-001190

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a 57-year-old male died of acute liver failure (ACUTE HEPATIC FAILURE) following use of PARACETAMOL (manufacturer, dose and therapy duration unknown) in combination with alcohol abuse 35 days after referral to hospital. No further information is available at the time of this report.

In this article, severe acute liver injury cases treated between 2008 and 2009 and reported retrospectively from eleven university medical centers participating in the Acute Liver Failure Study Group in Germany were analyzed.

SFC. The liver failure is possibly related to the intake of paracetamol.

Measures for risk mitigation are not planned.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12065645

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No	YES	

Patient

Initials:	Sex: Female	Birth: 19870314	Age: 21 Year	weight (kg):	BMI_int:	Age fetus: 1 Month
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20080422			3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Oral	7 G gram(s)/ Day	20080420-20080422	Toothache
paracetamol		245	3 Day	

Drugs (concomitant)

2 Kontrazeptivum	Oral	21* / Month	-200803	Contraception
		245	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine renal clearance	20080424	>1	ml/s	

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 30.03.2012 DE-PVZIBE-2-3615

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
LEBERVERSAGEN

KURZZUSAMMENFASSUNG:

Leberversagen unter hochdosierter Paracetamol-Einnahme

BESCHREIBUNG:

Patientin stellte sich mit Übelkeit, Unwohlsein und schweren therapieresistenten Zahnschmerzen in der (Wismarer) Kliniknotaufnahme vor;
anamnestisch: 20.04.08 Zahnextraktion, innerhalb von 2 Tagen 14 g Paracetamol eingenommen,

Art des medikamentenbedingten Problems: Überdosierung

THERAPIE:

ACC-Schema 9.5 g/5 g/ 7g;

23.04.08 1A Konakion

VERLAUF:

Normalisierung der Leber-/gerinnungswerte;

Verlegung ins Heimatkrankenhaus

ERLÄUTERUNGEN ZUR UAW:

Der zeitliche und symptomatologische Verlauf der Leberschädigung ist mit der Paracetamoleinnahme vereinbar.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12099540

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PARACETAMOL	Unknown	/	-	Drug use for unknown
paracetamol			-	indication

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Bantel H and Schulze-Osthoff K. Can cell death biomarkers predict the outcome of acute liver failure? Liver International 2012; 32(2): 346.	

Narratives (nur bei elektronischem Eingang)

[sanofi-aventis, 11.05.2012 DE-SANOFI-AVENTIS-2012SA031367](#)

Sender diagnosis: not reported

Narrative

This unsolicited literature summary case was identified on 02 May 2012 via an article: Bantel H and Schulze-Osthoff K. Can cell death biomarkers predict the outcome of acute liver failure? Liver International 2012; 32(2): 346.

This summary case concerns group of nine patients who developed paracetamol-induced acute liver failure whilst receiving paracetamol for unknown indication.

On an unspecified date, the patients commenced treatment with paracetamol (route, form, dosage, frequency not provided). After unspecified time, they experienced paracetamol-induced acute liver failure.

The outcome of paracetamol-induced acute liver failure and therapy status of paracetamol was not provided.

No medical history, concomitant or past drugs were reported.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12118054

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting	200809			2 Month	Unknown
2 Nausea	200809			2 Month	Unknown
3 Jaundice	200809			2 Month	not recovered/not resolved
4 Cholestatic hepatitis	200810			3 Month	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone		6 Mg milligram(s)/Day	200807-	Brain edema
dexamethasone			-	
2 dexamethasone		1 Mg milligram(s)/Day	-200809	
dexamethasone			-	
3 PANTOPRAZOLE		40 Mg milligram(s)/Day	200807-	Prophylaxis against gastrointestinal ulcer
pantoprazole			-	
4 PANTOPRAZOLE		20 Mg milligram(s)/Day	-200809	
pantoprazole			-	
5 PARACETAMOL		3000 Mg milligram(s)/Day	200807-200807	Postoperative pain
paracetamol			-	
6 TEMOZOLOMIDE	Oral	/	200808-200810	Glioblastoma multiforme
temozolomide			62 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Glioblastoma multiforme			k.A.	
Gastroduodenal ulcer			No	
Brain oedema			No	
Radiotherapy	200808	200809	Unknown	
Brain tumor operation	200807	200807	Unknown	
Pancreas divisum			Unknown	
Pain			No	Post-operative. Treated with paracetamol.

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Sarganas G, Orzechowski H, Klimpel A, Thomae M, Kauffmann W, Herbst H, et al. Severe sustained cholestatic hepatitis following temozolomide in a patient with glioblastoma multiforme: case study and review of data from the FDA adverse event reporting system. Neuro-oncology 2012 May;14:5:541-546.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	200807	29	U/l	
Alanine aminotransferase	200810	639	U/l	
Aspartate aminotransferase	200810	109	U/l	
Alkaline phosphatase	200810	261	U/l	
Bilirubin total	200810	12.77	mg/dl	
Alanine aminotransferase	200812	166	U/l	

Aspartate aminotransferase	200812	87	U/l
Bilirubin total	200812	18.2	mg/dl
Prothrombin time	200812	within normal range	
Bilirubin		18.2 - 23.6	mg/dl
Bilirubin conjugated	200904	11.2	mg/dl
Bilirubin	200904	14.3	mg/dl
Alanine aminotransferase	200904	286	U/l
Aspartate aminotransferase	200904	75	U/l
Alkaline phosphatase	200904	545	U/l

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 06.06.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-02975GD

Sender diagnosis: not reported

Narrative

P12-06579

Severe sustained cholestatic hepatitis (leading to hospitalization) is reported in a patient with glioblastoma multiforme (GBM), who had received temozolomide (TZM), pantoprazole, paracetamol and dexamethasone. A drug-induced liver injury is discussed.

A 51-year-old male came to the emergency room in mid-July 2008 because of impaired vision to the left side, gait disturbance, and right-sided temporal headache. The patient reported that he had noticed the first symptoms a couple of days ago. Ophthalmologic examination revealed homonymous hemianopsia to the left side. Magnetic resonance tomography (MRT) demonstrated a right occipital lesion suspect of a malignant brain tumor. Serum level of aspartate aminotransferase (AST) was normal (29 U/L). After tumor resection, histological examination of the tumor confirmed the diagnosis of GBM. Immediately after the surgery, therapy was initiated consisting of dexamethasone (initially 2 mg 3 times a day and then 1 mg/day) for prevention of brain edema, pantoprazole (40 mg once a day) for prevention of gastroduodenal ulcer, and paracetamol (1000 mg 3 times a day) for postoperative pain. By the end of July, the patient was discharged from the hospital, and paracetamol was discontinued.

In mid-August 2008, fractionated radiotherapy (total dose 60 Gy, fractionated 5 x 2 Gy per week) with concomitant TZM (75 mg/m² per day p.o.) was initiated. The radiochemotherapy regimen was completed by the end of September 2008. The day after completion, the patient complained of nausea and vomiting. On the following day, he developed generalized jaundice. At the beginning of October 2008, laboratory investigations revealed a markedly increased serum level of alanine aminotransferase (ALT): 639 U/L (10-50 U/L); a moderately increased AST: 109 U/L (10-50 U/L); and increased alkaline phosphatase (AP): 261 U/L (<130 U/L), with an ALT/AP ratio of 1.75, indicating cholestatic hepatitis. Total bilirubin was markedly increased at 12.77 mg/dL (<1 mg/dL).

During follow-up care, serum levels of hepatic enzymes and of bilirubin were determined by the general practitioner. Because of persisting jaundice, the patient was referred to a hepatologist for further diagnosis. Sonographic examination of the abdomen did not reveal pathologies of the liver or of the biliary tract.

By the end of October 2008, the patient went to the neurosurgery outpatient clinic for a postoperative follow-up. Because the liver injury was considered as possibly drug-induced, it was recommended not to proceed with the adjuvant TZM as monotherapy. One week later, the patient returned to the neurosurgery outpatient department and an MRT of the brain did not reveal a recurrence of the tumor. At the beginning of December 2008, the patient was again referred to the neurosurgery department because of progressive visual disturbances. A tumor relapse was excluded by MRT. At this time point, the physical examination revealed generalized jaundice and the ultrasound examination showed a modestly enlarged liver without signs of biliary obstruction. The laboratory investigations showed only modestly elevated transaminases (ALT: 166 U/L, AST: 87 U/L), whereas the total bilirubin remained markedly elevated at 18.2 mg/dL. The prothrombin time was within the normal range. The patient was referred to the gastroenterology department for further diagnostics. Viral etiology of liver disease (hepatitis B, hepatitis C, cytomegalovirus, and Epstein-Barr virus) was excluded by serological testing. Tests were negative for autoimmune markers (antinuclear antibodies, antimitochondrial antibodies, liver-kidney microsomal antibodies, liver cytosol type 1 antibodies, soluble liver antigen/liver-pancreas antigen antibodies, liver membrane antibodies, and smooth muscle antibodies). Sonographic and MRT examination of the abdomen did not reveal any pathological findings, in particular no signs of liver or biliary tract abnormalities. The finding of a partial pancreas divisum was considered unrelated to the patient's liver disease. Liver biopsy was performed and histological examination showed a pattern of cholestatic hepatitis. Morphologic changes were described as canalicular cholestasis, feathery degeneration, and necrosis of some hepatocytes, as well as faint portal infiltration with lymphocytes, plasma cells, and some polymorphonuclear leukocytes. Bile ductules were intact, and there was no evidence for intrahepatic or extrahepatic bile duct obstruction. In the absence of histological and clinical evidence for a viral, autoimmune, alcoholic, or other toxic etiology, the findings were considered consistent with toxic liver injury most likely caused by TZM.

A therapy regimen including silymarin, ursodeoxycholic acid, and prednisolone was initiated but failed to show any therapeutic benefit. During hospitalization, the bilirubin values remained elevated within the range of 18.2 mg/dL to 23.6 mg/dL. By the end of January 2009 neurological symptoms (vertigo, gait disturbances, and lateropulsion) worsened. By the end of February 2009, the patient was referred to the Neurology Department of the University Hospital, where a tumor relapse was diagnosed. The patient was again referred to the Department of Radiotherapy. Laboratory findings from April 2009 showed bilirubin of 14.3 mg/dL (conjugated bilirubin 11.2 mg/dL), ALT: 286 U/L, AST: 75 U/L, and AP: 545 U/L. A few days after hospital discharge in April 2009, the patient died.

The authors state: 'We consider that the liver injury in this case is probably causally related to TZM exposure. First of all, the manifestation of liver disease was in a close temporal relation to preceding drug treatment. Second, previous liver disease was excluded as well as other disease associated with disturbed liver function, alcohol or illicit drug abuse, and previous consumption of prescription-free synthetic drugs or herbal medicines as possible alternative causes of current liver disease. ... Additionally, he received a low dose of dexamethasone (1 mg/d) and pantoprazole (20

mg/d). Incidentally, this drug treatment was completed the day before the onset of clinical symptoms. The cholestatic hepatitis had a sustained course over more than 6 months and the liver pathology was clearly consistent with toxic liver injury, confirming the possibility of drug etiology. The temporal pattern of serum parameters was interpreted as initial hepatocellular damage, which was followed by sustained cholestatic liver disease. ... In our case TZM was coadministered with pantoprazole. Even though pantoprazole is known to induce elevation of liver enzymes, according to a recent review by Thomson et al., pantoprazole has not been associated with hepatitis. Furthermore, before TZM was initiated, our case had received pantoprazole for 4 weeks without signs of liver toxicity. The underlying mechanism of this rare, serious liver toxicity remains unclear.'

TEVA, 18.07.2012 DE-TEVA-348334GER

Sender diagnosis: not reported

Narrative

09-Jul-2012, Serious Literature Report received from Reaction (# 803073313). (SPRINGER HEALTHCARE; NZ-ADIS-3073313-26792)

Case of a Male patient 51 Years of age who experienced Severe sustained cholestatic hepatitis while on TEMOZOLOMIDE (Teva's product cannot be excluded).

At the time of the report the outcome of the AEs were: Severe sustained cholestatic hepatitis : Unknown.

Narrative provided by ADIS:

Sarganas G, Orzechowski HD, Klimpel A, Thomae M, Kauffmann W, et al.

Severe sustained cholestatic hepatitis following temozolomide in a patient with glioblastoma multiforme: Case study and review of data from the FDA adverse event reporting system. Neuro-Oncology 2012; 14(5) 541-546

A 51-year-old man developed sustained and severe cholestatic hepatitis following treatment with temozolomide for glioblastoma multiforme. The man received oral temozolomide 75 mg/m² per day as part of a radio-chemotherapy regimen in mid-August 2008. The regimen was finished by the end of September 2008. The day after, he developed nausea and vomiting. He developed generalised jaundice the following day. At the start of October 2008, laboratory tests revealed markedly increased ALT levels, moderately increased AST levels, increased ALP, and an ALT/ALP ratio of 1.75, which indicated cholestatic hepatitis. His total bilirubin level was also markedly elevated. His liver injury was considered to be possibly drug-induced so temozolomide monotherapy was not recommended. A physical examination in December 2008 showed generalized jaundice and ultrasound revealed that his liver was modestly enlarged. His total bilirubin remained markedly increased and his transaminases were modestly increased. Histological evaluation of a liver biopsy revealed a pattern of cholestatic hepatitis. Findings were considered consistent with toxic liver injury most likely caused by temozolomide.

The man was treated with prednisolone, silymarin, and ursodeoxycholic acid but there was no therapeutic benefit. During his hospitalisation, his bilirubin values remained raised. A tumour relapse was diagnosed by the end of February 2009. From April 2009, his bilirubin and transaminases continued to be elevated. He was discharged in April 2009 and died a few days later (cause of death not stated).

Author comment: "We consider that the liver injury in this case is probably causally related to (temozolomide) exposure."

Concomitant Medication:

dexamethasone; pantoprazole

Medical History:

GLIOBLASTOMA MULTIFORME(Concomitant) ,SURGERY(Past) ,PAIN(Past) ,BRAIN OEDEMA(Concomitant) ,GASTRODUODENAL ULCER (Concomitant)

Action Taken with suspect Drugs: TEMOZOLOMIDE - Not applicable

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to temporal relation, reported "severe sustained cholestatic hepatitis" is possibly related to Temozolamide treatment. Nevertheless, contribution of other reasons and circumstances which might have contributed to the occurrence of the adverse event (e.g. concomitantly taken drugs) cannot be completely excluded.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12133434

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19950411	Age: 17 Year	weight (kg): 55	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme increased	20120618			2.5 Week	recovered/resolved
2 Suicide attempt	201206				recovered/resolved
3 Renal insufficiency	20120618			2.5 Week	recovered/resolved
4 Renal colic	20120618			2.5 Week	recovered/resolved
5 Inflammation NOS	20120618			2.5 Week	recovered/resolved
6 Multiple drug overdose intentional	201206				recovered/resolved
7 Bradycardia	20120618			2.5 Week	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Grippostad C Hartkapseln caffeine; chlorphenamine maleate; paracetamol	Oral	/ 6	201206-201206 -	Product used for unknown indication
2 ibuprofen ibuprofen	Oral	30 Df dosage form/Day 245	201206-201206 -	Product used for unknown indication
3 Omeprazol omeprazole	Oral	/ 245	201206-201206 -	Product used for unknown indication
4 Paracetamol 500 paracetamol	Oral	/ 245	201206-201206 -	Product used for unknown indication

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
C-reactive protein	20120618	100	-	
Bilirubin	20120618	1.7	-	
Aspartate aminotransferase	20120618	normal	-	
GPT	20120618	normal	-	
Creatinine	20120618	3.3	-	

Narratives (nur bei elektronischem Eingang)

STADA Arzneimittel AG, 25.06.2012 DE-STADA-048489

Sender diagnosis: not reported

Narrative

This report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jun-19.

A 17-year-old female patient took 2-3 capsules Grippostad C Hartkapseln (INN: ascorbic acid, caffeine, chlorphenamine, paracetamol), 30 tablets ibuprofen, 10-15 tablets Paracetamol 500 (INN: paracetamol) and 10-15 tablets Omeprazol (INN: omeprazole) two and a half week prior to this report in a suicide attempt.

On 2012-Jun-18 she suffered from renal colic on the right side, renal insufficiency, increased hepatic values, inflammation nos and bradycardia 45-50.

Performed tests showed following values: CRP 100 (norm <5), bilirubin 1.7, GOT and GPT normal, creatinine 3.3.

Hospitalisation was recommended.

No further information was provided. Follow-up requested.<dbtext>To_Char_NARRATIVE_2</dbtext>

GRIPPOSTAD C, PARACETAMOL

Renal failure and hepatic enzymes increased are listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Informations (RSIs).

IBUPROFEN

Renal failure is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI). Impaired liver functions and liver damage are listed, hepatic enzymes increased is not explicitly stated.

OMEPRAZOLE

Hepatic enzymes increased is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI).

This case does not change the overall benefit-risk balance of the medicinal products.

Reporter's comment:

STADA Arzneimittel AG, 18.07.2012 DE-STADA-048489

Sender diagnosis: not reported

Narrative

This report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jun-19.

A 17-year-old female patient took 2-3 capsules Grippostad C Hartkapseln (INN: ascorbic acid, caffeine, chlorphenamine, paracetamol), 30 tablets ibuprofen, 10-15 tablets Paracetamol 500 (INN: paracetamol) and 10-15 tablets Omeprazol (INN: omeprazole) two and a half week prior to this report in a suicide attempt.

On 2012-Jun-18 she suffered from renal colic on the right side, renal insufficiency, increased hepatic values, inflammation nos and bradycardia 45-50.

Performed tests showed following values: CRP 100 (norm <5), bilirubin 1.7, GOT and GPT normal, creatinine 3.3.

Hospitalisation was recommended.

No further information was provided. Follow-up requested.

Follow-up #1 report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jul-18.

The outcome was reported to be completely recovered.<dbtext>To_Char_NARRATIVE_2</dbtext>

IBUPROFEN

Renal failure is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI). Impaired liver functions and liver damage are listed, hepatic enzymes increased is not explicitly stated.

OMEPRAZOLE

Hepatic enzymes increased is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI).

This case does not change the overall benefit-risk balance of the medicinal products.

Follow-up information #1 received on 2012-Jul-18:

The company's assessment of the case remains unchanged.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12185514

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19770526	Age: 34 Year	weight (kg): 55	BMI_int: 20.2	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Impaired liver function	20120113	18.11.2011		2 Day	recovered/resolved
2 Impaired liver function	20120113			1 Day	recovered/resolved
3 Hepatic failure	201111				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BELARA chlormadinone acetate; ethinylestradiol	Oral	/	-	Drug use for unknown indication
2 METAMIZOLE SODIUM metamizole sodium	Oral	20 Gtt drop(s)/ 160	20120112-20120112 1 Day	Allergy test
3 novaminsulfon-Tropfen metamizole sodium	Oral	20 Gtt drop(s)/Day 169	20120112- -	Drug tolerance
4 PARACETAMOL paracetamol	Oral	/	- -	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spinal column injury			Yes	
Hepatic failure	201111		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20120102	53	U/L	
Alkaline phosphatase	20111215	66	U/L	
Alkaline phosphatase	20111221	60	U/L	
Alkaline phosphatase	20111118	126	U/L	
ANA	20111118	1:5120	NA	
Antimitochondrial antibody	20111118	< 1:80	NA	
Bilirubin	20111118	13.15	MG/DL	
Bilirubin total	20111221	3.42	MG/DL	
Bilirubin total	20120102	2.21	MG/DL	
Bilirubin total	20111215	4.28	MG/DL	
Biopsy of liver	20111121	PATHOLOGIC	MA	
C-reactive protein	20111129	NEGATIVE	NA	
C-reactive protein	20111118	NEGATIVE	NA	
Gamma GT	20111215	50	U/L	
Gamma GT	20120102	42	U/L	

Gamma GT	20111221	38	U/L
Gamma GT	20111129	209	U/L
Gamma GT	20111118	183	U/L
LDH	20111215	130	U/L
LDH	20111129	223	U/L
LDH	20111118	578	U/L
LDH	20111221	318	U/L
SGOT	20111118	2443	U/L
SGOT	20120102	58	U/L
SGOT	20111221	382	U/L
SGOT	20111129	235	U/L
SGOT	20111215	48	U/L
SGPT	20111118	2162	U/L
SGPT	20120102	64	U/L
SGPT	20111215	49	U/L
SGPT	20111221	163	U/L
SGPT	20111129	348	U/L
Ammonia	20111118	53	MCMOL/L
Ammonia	20111129	WITHIN NORMAL RANGES	
Double stranded DNA antibody	20111118	0.18	NA
ANCA	20111118	< 1:10	NA
Quick's test	20111129	97	%
Quick's test	20111118	50	%
Sonogram	20111121	PRONOUNCED FATTY LIVER	
Blood beta human chorionic gonadotropin	20111118	NEGATIVE	NA
Hepatitis C virus test	20111118	NEGATIVE	NA
Alkaline phosphatase	20120202	48	U/L
Alkaline phosphatase	20120222	50	U/L
Bilirubin total	20120222	0.72	MG/DL
Bilirubin total	20120202	0.77	MG/DL
Gamma GT	20120202	27	U/L
Gamma GT	20120222	24	U/L
SGOT	20120222	51	U/L
SGOT	20120202	37	U/L
SGOT	20120113	THREEFOLD INCREASE	
SGPT	20120113	THREEFOLD INCREASE	
SGPT	20120222	47	U/L
SGPT	20120202	39	U/L

Narratives (nur bei elektronischem Eingang)

Drug Commission of the German Medical Association, 17.08.2012 DE-DCGMA-12157959

Sender diagnosis: not reported

Narrative

Sender's comment:Reporter's comment:

Patientin erlitt 11/2011 Leberversagen bei Einnahme von Belara sowie gelegentlicher Paracetamol-Einnahme und fraglicher Metamizol-Natrium-Einnahme.

sanofi-aventis, 29.08.2012 DE-SANOFI-AVENTIS-2012SA060927

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Aug-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12185514); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association (DE-DCGMA-12157959) about a 34-year-old female patient.

On 12-Jan-2012 she underwent drug tolerance test with metamizole sodium 20 drops/day (Novalminsulfon-Tropfen). On the next day an impaired liver function was detected. Corrective therapy was not reported; however the patient recovered at unspecified time later.

Causality by Drug Commission: possible

Within the reporter's comment concerning this case it was mentioned that the patient had suffered from hepatic failure in Nov-2011, during the treatment with chlormadinone acetate/ethinylestradiol, occasional use of paracetamol and questionable intake of metamizole sodium. This case was documented with 2012SA059991.

Reporter's comment:

Patientin erlitt 11/2011 Leberversagen bei Einnahme von Belara sowie gelegentlicher Paracetamol-Einnahme und fraglicher Metamizol-Natrium-Einnahme.

sanofi-aventis, 30.08.2012 DE-SANOFI-AVENTIS-2012SA059991

Sender diagnosis: not reported

Narrative

On 21-Aug-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12185514); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association (DE-DCGMA-12157959) about a 34-year-old female patient.

On 12-Jan-2012 she underwent drug tolerance test with metamizole sodium 20 drops/day (Novalminsulfon-Tropfen). On the next day an impaired liver function was detected. This case was documented under the no. 2012SA060927.

Within the reporter's comment concerning this case (2012SA060927) it was mentioned that the patient had suffered from hepatic failure in Nov-2011, during the treatment with chlormadinone acetate/ethinylestradiol, occasional use of paracetamol and questionable intake of metamizole sodium. Further information was not provided.

Reporter's comment:

sanofi-aventis, 09.01.2013 DE-SANOFI-AVENTIS-2012SA059991

Sender diagnosis: not reported

Narrative

On 21-Aug-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12185514); documenting a
18.03.2013

case, in which a physician had reported to the Drug Commission of German Medical Association (DE-DCGMA-12157959) about a 34-year-old female patient.

On 12-Jan-2012 she underwent drug tolerance test with metamizole sodium 20 drops/day (Novaminsulfon-Tropfen). On the next day an impaired liver function was detected. This case was documented under the no. 2012SA060927.

Within the reporter's comment concerning this case (2012SA060927) it was mentioned that the patient had suffered from hepatic failure in Nov-2011, during the treatment with chlormadinone acetate/ethinylestradiol, occasional use of paracetamol and questionable intake of metamizole sodium. Further information was not provided.

The patient was hospitalised on 18-Nov-2011 due to acute liver failure. Physical examination at admission showed a pronounced skin icterus, abdomen was soft, no abdominal guarding, no tenderness, no resistances.

Laboratory on 18-Nov-2011 revealed the following values: GOT 2443 U/l, GPT 2162 U/l, gamma GT 183 U/l, alkaline phosphatase 126 U/l, bilirubin 13.15 mg/dl, LDH 578 U/l (no normal ranges provided for liver values), C-reactive protein negative, Quick 50 % (normal ranges not provided), beta HCG negative, ammonia 53 µmol/l (normal ranges 11 to 150), hepatitis C serology negative, serum plasmin 0.27 g/l (within normal ranges), ANA 1:5120 (titre pattern finely dotted nuclear fluorescence with positive chromosome area), double stranded DNA antibodies 0.18 (within normal ranges), AMA < 1:80 (within normal ranges), SLA < 2.0 U/ml (within normal ranges), C-ANCA < 1:10 (within normal ranges), P-ANCA < 1:10 (within normal ranges).

Upper abdominal sonogram on 21-Nov-2011 revealed a pronounced fatty liver.

Liver puncture was performed on 21-Nov-2011 (sonomorphologically complicated fine-needle puncture of right lower lobe). Histology performed on 24-Nov-2011 showed a liver sample with active hepatitis with pericentral collapse and bridge necroses as well as liver cell necroses in hepatic lobules and mild portal and pericentral fibrosis; findings correlated with severe drug-toxic liver damage.

Hepatitis C and B, as well as cytomegalic inclusion disease and acute infection with Epstein Barr virus were ruled out. No signs of autoimmune hepatitis were detected (antinuclear antibodies were pronouncedly increased but unspecific).

As cause of liver failure, treatment with chlormadinone acetate/ethinylestradiol and/or occasional intake of paracetamol was discussed.

Under symptomatic therapy, liver function improved considerably.

Laboratory values at discharge (29-Nov-2011) showed GOT of 235 U/l, GPT of 348 U/l, gamma GT 209 U/l, LDH 223 U/l, negative C-reactive protein, and Quick of 97%; ammonia was within normal ranges.

Discharge diagnosis read: drug-toxic liver failure; cause: suspicion of toxic effect of chlormadinone acetate/ethinylestradiol as well as occasional paracetamol intake.

Intake of metamizole prior to the event was not mentioned in the hospital report of 10-Dec-2011.

Laboratory on 15-Dec-2011 showed the following values: GOT 48 U/l (normal ranges up to 35), GPT 49 U/l (normal ranges up to 35), alkaline phosphatase 66 U/l (normal ranges 35 to 105), total bilirubin 4.28 mg/dl (normal ranges up to 1.2), gamma GT 50 U/l (normal ranges up to 40), LDH 130 U/l (normal ranges up to 250)

Laboratory on 21-Dec-2011 showed the following values: GOT 382 U/l, GPT 163 U/l, alkaline phosphatase 60 U/l, total bilirubin 3.42 mg/dl, gamma GT 38 U/l, LDH 318 U/l (normal ranges same as of 15-Dec-2011).

Laboratory on 02-Jan-2012 showed the following values: GOT 58 U/l, GPT 64 U/l, alkaline phosphatase 53 U/l, total bilirubin 2.21 mg/dl, gamma GT 42 U/l (normal ranges same as of 15-Dec-2011).

Additional information received from the Drug Commission of German Medical Association by letter on 07-Jan-2012 (hospital reports of 10-Dec-2011 and 26-Mar-2012, physician's report of 15-Feb-2012, and laboratory sheets):

Dates of hospitalisation in Nov. 2011, examinations during hospitalisation, discharge diagnosis, and laboratory values after hospitalisation were added, event was amended, seriousness criterion was amended (hospitalization).

sanofi-aventis, 09.01.2013 DE-SANOFI-AVENTIS-2012SA060927

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Aug-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12185514); documenting a case, in which a physician had reported to the Drug Commission of German Medical Association (DE-DCGMA-12157959) about a 34-year-old female patient.

Within the reporter's comment concerning this case it was mentioned that the patient had suffered from hepatic failure in Nov-2011, during the treatment with chlormadinone acetate/ethinylestradiol, occasional use of paracetamol and questionable intake of metamizole sodium and was hospitalised from 18-Nov-2011 to 29-Nov-2011 (This case was documented with 2012SA059991). Metamizole sodium was not mentioned in the hospital discharge report dated 10-Dec-2011.

Laboratory on 02-Jan-2012 revealed the following liver values: alkaline phosphatase 53 U/l (normal ranges 35-105), total bilirubin 2.21 mg/dl (normal ranges up to 1.2), gamma GT 42 U/l (normal ranges up to 40), GOT 58 U/l (normal ranges up to 35), and GPT 64 U/l (normal ranges up to 35).

On 12-Jan-2012, the patient underwent drug tolerance test with metamizole sodium 20 drops/day (Novaminsulfon-Tropfen). On the following day, impaired liver function was detected (three-fold increase of transaminases; values were not provided); transaminases normalised slowly. Corrective therapy was not reported.

Laboratory on 03-Feb-2012 (sample collected on 02-Feb-2012) revealed the following liver values: alkaline phosphatase 48 U/l, total bilirubin 0.77 mg/dl, gamma GT 27 U/l, GOT 37 U/l, and GPT 39 U/l.

Laboratory on 22-Feb-2012 showed the following values: alkaline phosphatase 50 U/l, total bilirubin 0.72 mg/dl, gamma GT 24 U/l, GOT 51 U/l, GPT 47 U/l.

Causality by Drug Commission: possible

Hospital report from 26-Mar-2012 mentioned, that increase in liver values after intake of metamizole sodium in Jan. 2012 could have also been induced by intake in the course of pre-damaged liver.

Additional information received from the Drug Commission of German Medical Association by letter on 07-Jan-2012 (hospital reports from 10-Dec-2011 and 26-Mar-2012, physician's report of 15-Feb-2012, and laboratory sheets):
Dates of hospitalisation in Nov. 2011, laboratory values, information on transaminases increase on 13-Jan-2012, and comment of hospital report on 26-Mar-2012 were added.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12188410

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute pancreatitis					Unknown
2 Hepatotoxicity					Unknown
3 Cholangitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol	Unknown	1 G gram(s)/	-	Drug use for unknown indication
paracetamol			-	indication
2 pelargonium sidoides	Unknown	/	-20071130	Drug use for unknown indication
pelargonium sidoides			-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	without specified treatment
Multiple allergies			Unknown	
Penicillin allergy			Unknown	
Type 2 diabetes mellitus			Unknown	without specified treatment.

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Rolf Teschke, Christian Frenzel, Albrecht Wolff, Jutta Herzog, Xaver Glass, Johannes Schulze, Axel Eickhoff. Initially purported hepatotoxicity by Pelargonium sidoides: the dilemma of Pharmacovigilance and proposal for improvement. Annals of Hepatology. 2012;Vol 11 No4:500-512	

Qualification:	Physician	observestudytype:
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Literature:

Qualification:	Other Health Professional	observestudytype:
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Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		505	N/A	
Alanine aminotransferase		140	N/A	
Amylase		374	N/A	
Amylase		29	N/A	
Aspartate aminotransferase		471	N/A	
Aspartate aminotransferase		129	N/A	
Aspartate aminotransferase		normal	N/A	
Blood alkaline phosphatase		178	N/A	
Bilirubin		56	N/A	
C-reactive protein		22.3	N/A	
GGT		586	N/A	
Lipase		175	N/A	

Serology test

without pathological changes

Sonogram

The report of the hepatobiliary sonography describes

Narratives (nur bei elektronischem Eingang)RANBAXY, 21.08.2012 DE-RANBAXY-2012RR-59131

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-59131 from GERMANY refers to a 67 year-old female (Patient 3) who experienced hepatotoxicity, acute pancreatitis and cholangitis following treatment with paracetamol and Pelargonium sidoides that required hospitalisation.

This case was reported in the literature, regarding 13 patients (6 patients for which Ranbaxy's drugs use cannot be excluded and 7 patients that used other company's drugs). Case supplied by BfArM.

Patient's current condition:

Penicillin allergy.

Multiple allergies.

Arterial hypertension: without specific treatment.

Type 2 diabetes mellitus: without specific treatment.

The patient received the following suspect products:

Paracetamol: 1 g/day, for unknown indication.

Pelargonium sidoides: 30 drops daily, until 30-NOV-2007, for unknown indication.

Reactio text: "The present study examines whether and to what extent treatment by PS was associated with the risk of liver injury in these spontaneous cases. Study design: Overall, 13 spontaneous cases with primarily suspected PS hepatotoxicity were included in the study. Their data were submitted to a thorough clinical evaluation that included the use of the original and updated scale of CIOMS (Council for International Organizations of Medical Sciences) to assess causality levels. These scales are liver specific, validated for liver toxicity, structured and quantitative. None of the 13 spontaneous cases of liver disease generated a positive signal of safety concern, since causality for PS could not be established on the basis of the applied CIOMS scales in any of the assessed patients. Confounding variables included co-medication with synthetic drugs, major co-morbidities, low data quality, lack of appropriate consideration of differential diagnoses, and multiple alternative diagnoses. Among these were liver injury due to co-medication, acute pancreatitis and cholangitis, acute cholecystitis, hepatic involvement following lung contusion, hepatitis in the course of virus and bacterial infections, ANA positive autoimmune hepatitis, and other preexisting liver diseases. In the course of the case assessments and under pharmacovigilance aspects, data and interpretation deficits became evident. Possible improvements include appropriate data quality of cases in spontaneous reports, case assessment by skilled specialists, use of a validated liver specific causality assessment method, and inclusion only of confirmed cases into the final regulatory case database.

This study shows lack of hepatotoxicity by PS in all 13 spontaneous cases as opposed to initial judgment that suggested a toxic potential of PS. Major shortcomings emerged in the pharmacovigilance section that require urgent improvements.

In the present study was assessed causality for PS in additional spontaneous cases of assumed PS hepatotoxicity in purported connection with PS use partially going back to 2004. These cases were presented in 2011 by European regulatory agencies, the WHO, and the manufacturer. None of these cases now presented has been thoroughly evaluated for causality before.

Report from a hospital physician. PS used for unknown indication with 30 drops daily short-term without further specification until November 30, 2007 when right upper abdominal pains and scleral jaundice led to hospitalization after vomiting, nausea and pyrexia 2 days before. At admission, ALT 505 U/L, AST 471 U/L, amylase 374 U/L. After 5 days, ALT 140 U/L, AST 129 U/L, and amylase 29 U/L. At discharge, ALT value not reported, but AST normalised. During hospitalisation, maximum values for GGT 586 U/L, ALP 178 U/L, bilirubin 5.6 mg%, lipase 175 U/L, and CRP 22.3 mg/L. Hepatitis serology was reported as without pathological changes, but details not communicated. The report of the hepatobiliary sonography describes that there is no passage of a biliary stone, but there is lack of details regarding gall bladder, bile ducts within and outside the liver, and pancreas. Comedication with paracetamol (1,000 mg/d), but no details presented. Known penicillin allergy and multiple other allergies, arterial hypertension without specified treatment, and type II diabetes mellitus without specified treatment. The hospital diagnosis is drug induced jaundice or calculus passage.

Final diagnoses: Acute pancreatitis and cholangitis, possibly related to transient choledocholithiasis; paracetamol hepatotoxicity.

Causality: Possible causality with respect to PS only on formal grounds according to CIOMS criteria, but this particular judgement is actually a false positive signal in face of the established alternative and final diagnosis.

Comment: Considering the clinical course with upper abdominal pains, nausea, vomiting, and increased values of amylase and lipase, there is no question regarding the diagnosis of acute pancreatitis. Fever and increased CRP values are in line with this diagnosis, but also with the diagnosis of cholangitis. The diagnosis of cholangitis is supported by increased liver values. The two diseases are most probably caused by a transient choledocholithiasis, but there is lack of a detailed description of the hepatobiliary sonography results including the gall bladder. Short-term use of PS with only 30 drops daily, hardly to be considered as cause for the presented disease entities."

The outcome of the events was not reported at the time of this report.

The case was considered as serious because the patient was hospitalised.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12202633

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19951013	Age: 16 Year	weight (kg): 68	BMI_int: 22.72	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bradypnea	20120418			1 Day	Unknown
2 Cyanosed	20120419			1 Day	Unknown
3 Vigilance decreased	20120418			1 Day	Unknown
4 SGPT increased	20120419			1 Day	Unknown
5 Quick's value decreased	20120419			1 Day	Unknown
6 Cyanosis	20120418			0 Day	Unknown
7 SGOT increased	20120419			1 Day	Unknown
8 SGPT increased	20120419			1 Day	Unknown
9 Bilirubin increased	20120419			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 paracetamol Kabi paracetamol	Intravenous (not otherwis	1000 Mg milligram(s)/ 120	20120418-20120418	Analgesia
2 sevorane Abbott sevoflurane	Respiratory (inhalation)	/	20120418-20120418	Narcosis

Drugs (concomitant)

3 fentanyl fentanyl	Intravenous (not otherwis	0.3 Mg milligram(s)/ 120	20120418-20120418	Analgesia
4 nimbex cisatracurium besilate	Intravenous (not otherwis	8 Mg milligram(s)/ 120	20120418-20120418	Muscle relaxant
5 piritramide piritramide	Intravenous (not otherwis	7.5 Mg milligram(s)/ 120	20120418-20120418	Analgesia
6 propofol 1 % fresenius propofol	Intravenous (not otherwis	200 Mg milligram(s)/ 120	20120418-20120418	Narcosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Appendectomy	201103		No	
Anterior cruciate ligament tear			Unknown	
Hepatic failure	20120420		Unknown	nach Inhalationsnarkose am 19.04.2012
unbekannt			Unknown	Opiatueberhang
Surgery	20120418		No	
Commotio cerebri			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGOT	20120419	1590	U/l	
SGPT	20120419	1903	U/l	
Quick's test	20120419	40	%	

	DE-BFARM-12202633	
Quick's test	89	%
SGPT	18	U/l
Quick's test	89	%

Narratives (nur bei elektronischem Eingang)

Fresenius Kabi, 13.09.2012 DE-Fresenius Kabi-FK201202610

Sender diagnosis: not reported

Narrative

Verbatim of the reporter:

Increased at the first operation day of SGPT, SGOT, Bilirubin,

4 hours after unremarkable transferring from the recovery room: bradypnea, cyanosed, vigilance decreased, Quick's value decreased

Narrative:

The case was reported by The BfArM (German Health Authority) with Worldwide Reference Number DE-BFARM-12202633.

A 16-year-old male patient received 1000mg Paracetamol Kabi for analgesia, together with Sevoflurane Abbott and other drugs. Four hours after transfer from the recovery room bradypnea, cyanosis, decreased Quick values and Vigilance occurred. At the first postoperative day, elevation of SGPT, SGOT and bilirubin developed.

Medical history: -

Other suspected drugs: Sevoflurane Abbott (sevoflurane)

Concomitant medication: Nimbex (cisatracurium)8mg, Propofol 1% Fresenius 200mg, fentanyl 0.3 mg, piritramide 7.5 mg

Drugs to treat AE:-

001307602, 13.09.2012 DE-ABBOTT-12P-062-0975875-00

Sender diagnosis: not reported

Narrative

Spontaneous report from GERMANY of SGPT INCREASED, SGOT INCREASED, BILIRUBIN INCREASED, BRADYPNEA, CYNOSED, VIGILANCE DECREASED and QUICK'S VALUE DECREASED with SEVORANE (SEVOFLURANE).

On 18 Apr 2012, the patient experienced BRADYPNEA, CYNOSED and VIGILANCE DECREASED. Approximately four hours after normal shift out of postanesthesia care unit, the patient experienced bradypnea, cyanosed, and decreased vigilance. On 19 Apr 2012, the patient experienced SGPT INCREASED, SGOT INCREASED, BILIRUBIN INCREASED and QUICK'S VALUE DECREASED. On first post surgery day, the patient experienced increased SGPT, SGOT, bilirubin, and decreased Quick's value. PARACETAMOL was also considered suspect.

Reporter's comment:

001307602, 25.01.2013 DE-ABBOTT-12P-062-0975875-00

Sender diagnosis: not reported

Narrative

Spontaneous report from GERMANY of VIGILANCE DECREASED, SGPT INCREASED, SGOT INCREASED, BILIRUBIN INCREASED, BRADYPNEA, CYANOSIS and QUICK'S VALUE DECREASED with SEVORANE (SEVOFLURANE).

Case received on 15 Jan 2013 from Health Authority, reference No.: DE-BFARM-12202633

On 18 Apr 2012 male patient underwent surgery and received SEVORANE for narcosis. On 18 Apr 2012, the patient experienced VIGILANCE DECREASED, BRADYPNEA and CYANOSIS. About four hours after inconspicuous transfer out of the recovery room he experienced bradypnea, cyanosis, and decreased vigilance. On 19 Apr 2012, the patient experienced SGPT INCREASED, SGOT INCREASED, BILIRUBIN INCREASED and QUICK'S VALUE DECREASED. On first post surgery day, the patient experienced increased SGPT, SGOT, bilirubin, and decreased Quick's value. PARACETAMOL was also considered suspect.

CHANGE HISTORY

On 15 Jan 2013, received updates to medical history, event information, reporter opinion of causality, suspect drug information, concomitant drug information, laboratory/diagnostic procedures and narrative description.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12248083

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sedation					Unknown
2 Overdose					Unknown
3 Hepatotoxicity					Unknown
4 Cardiovascular disorder					Unknown
5 Liver injury					Unknown
6 Toxicity renal					Unknown
7 Hemarthrosis					Unknown
8 Gastro intestinal bleed					Unknown
9 Peptic ulcer					Unknown
10 Constipation					Unknown
11 Addiction					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 celecoxib		/	-	Pain
celecoxib			-	
2 codeine		/	-	Pain
codeine			-	
3 diclofenac		/	-	Pain
diclofenac			-	
4 ETORICOXIB		/	-	Pain
etoricoxib			-	
5 FENTANYL	Transdermal	/	-	Pain
fentanyl			-	
6 ibuprofen		/	-	Pain
ibuprofen			-	
7 MELOXICAM		/	-	Pain
meloxicam			-	
8 METAMIZOL		/	-	Pain
metamizole magnesium			-	
9 MORPHINE		/	-	Pain
morphine			-	
10 naproxen		/	-	Pain
naproxen			-	
11 PARACETAMOL		/	-	Pain
paracetamol			-	
12 piroxicam		/	-	Pain
piroxicam			-	
13 TRAMADOL	Unknown	/	-	Pain
tramadol hydrochloride			-	
14 TRAMADOL HYDROCHLORIDE		/	-	Pain
tramadol hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Haemophilia			Yes	
Pain			Yes	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia Sep-2012;18 (5):743-752.	

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of

administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

GRUNENTHAL, 01.10.2012 DE-GRUNENTHAL-2012-15367

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2012-15367

This spontaneous literature case from Germany was reported by a physician via license partner and was initially received on 18-Sep-2012. It was extracted from the literature article 'Pain management in patients with haemophilia: a European survey' published by Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. in Haemophilia Sep-2012;18 (5):743-752

concerns a patient of unknown age (height and weight unknown) who experienced cardiovascular disorder, liver injury, nephropathy toxic, haemarthrosis, gastrointestinal haemorrhage, peptic ulcer, constipation, dependence, sedation and overdose under tramadol hydrochloride treatment.

Other suspected medication included fentanyl (Fentanyl, transdermal) for pain, paracetamol (Paracetamol, unknown route of administration) for pain, naproxen (Naproxen, unknown route of administration) for pain, ibuprofen (Ibuprofen, unknown route of administration) for pain, codeine (Codeine, unknown route of administration) for pain, metamizole magnesium (Metamizol, unknown route of administration) for pain, diclofenac (Diclofenac, unknown route of administration) for pain, piroxicam (Piroxicam, unknown route of administration) for pain, meloxicam (Meloxicam, unknown route of administration) for pain, morphine (Morphine, unknown route of administration) for pain, celecoxib (Celecoxib, unknown route of administration) for pain and etoricoxib (Etoricoxib, unknown route of administration) for pain.

No relevant concomitant medication was reported.

The patient's concurrent conditions included haemophilia and pain.

The patient was treated with tramadol hydrochloride (Tramadol hydrochloride,) for pain.

Reporter's notes: Report received from literature.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74

% of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

No treatment of the AEs was reported.

Third party case numbers

e2b company number: de-jnjfoc-20120909195

grt litdb accno: 803382

Literature citation:

holstein k, klamroth r, richards m, carvalho m, perez-garrido r, gringeri a. pain management in patients with haemophilia: a european survey. haemophilia. 2012; 18(5): 743-752

Janssen-Cilag, Germany Local Safety Officers, 18.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 22.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

Janssen-Cilag, Germany Local Safety Officers, 07.11.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12261672

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus: 008 Week
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes					recovered/resolved
2 Pleuritis					Unknown
3 Pregnancy					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 PERFALGAN IV INFUSION		/	-	
paracetamol			-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		8.8		

Narratives (nur bei elektronischem Eingang)

Bristol-Myers Squibb Company, 10.10.2012 DE-Bristol-Myers Squibb Company-17003922

Sender diagnosis: not reported

Narrative

Seriousness criteria: Important medical events.

This prospective pregnancy case was received via BMS Germany (cross reference number: 2012092604 and 1-52479095).

A pharmacist reported that a female pregnant patient (age unspecified) experienced extremely high liver values and pleuritis while on therapy with acetaminophen infusion. The events of high liver values and pleuritis were considered as important medical events.

The patient was administering acetaminophen in her 8th week of pregnancy and developed extremely high liver values (high alanine transaminase at 8.8). The patient was administered acetaminophen in the first-aid-station at 0300 hours in the morning. Then the patient's blood values were taken in the Gynaecology department about 15 minutes after the acetaminophen infusion was administered and the physician identified that the patient had high transaminase values.

The pharmacist stated that the liver values gradually reduced and that the patient's levels stabilized again. The reporter further stated that the patient also received sulbactam/ampicillin for suspected pleuritis and the patient was not taking any further medication.

The pharmacist enquired to know whether the acetaminophen infusion could have caused the high transaminases of the patient.

[^] according to current Summary of Product Characteristics

^{**} DEF=definite, PRB=probable, PSB=possible, NOL=not likely, NOR=not related

Change of current scientific knowledge: no.

Comments: This pregnant patient had increased liver values (alanine transaminase) 15 minutes after acetaminophen infusion and also experienced pleuritis. Hepatic dysfunction including raised liver values are known to be associated with acetaminophen. Limited information regarding pleuritis, precludes medically meaningful assessment.

Reporter's comment:

Bristol-Myers Squibb Company, 08.03.2013 DE-Bristol-Myers Squibb Company-17003922

Sender diagnosis: not reported

Narrative

Seriousness criteria: Important medical events.

This prospective pregnancy case was received via BMS Germany (cross reference number: 2012092604 and 1-52479095).

A pharmacist reported that a female pregnant patient (age unspecified) experienced extremely high liver values and pleuritis while on therapy with acetaminophen infusion. The events of high liver values and pleuritis were considered as important medical events.

The patient was administering acetaminophen in her 8th week of pregnancy and developed extremely high liver values (high alanine transaminase at 8.8). The patient was administered acetaminophen in the first-aid-station at 0300 hours in the morning. Then the patient's blood values were taken in the Gynaecology department about 15 minutes after the acetaminophen infusion was administered and the physician identified that the patient had high transaminase values.

The pharmacist stated that the liver values gradually reduced and that the patient's levels stabilized again. The reporter further stated that the patient also received sulbactam/ampicillin for suspected pleuritis and the patient was not taking any further medication.

The pharmacist enquired to know whether the acetaminophen infusion could have caused the high transaminases of the patient.

Supplemental information received on 27-Feb-2013 from the pharmacist noted that she was not able to provide further information as the patient's physician refused to provide information.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12311299

Serious Yes	Death Yes	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 200904	deathcause: Cholestatic hepatitis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestatic hepatitis	200809				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Paracetamol paracetamol		3000 Mg milligram(s)/Day	200807-200807 2 Week	Postoperative pain
2 Temozolomide temozolomide	Oral	75 mg/m2 milligram(s)/sq.	200808-200810 62 Day	Glioblastoma multiforme

Drugs (concomitant)

3 dexamethasone dexamethasone		6 Mg milligram(s)/Day	200807- -	Brain edema
4 dexamethasone dexamethasone		1 Mg milligram(s)/Day	200807- -	Brain edema
5 pantoprazole pantoprazole		40 Mg milligram(s)/Day	200807- 4 Week	Gastric ulcer prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Brain tumour operation			Unknown	
Craniotomy			Unknown	
Glioblastoma multiforme	200807		Unknown	
Radiotherapy	200808	200809	Unknown	fractionated radiotherapy

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Zamani N, Mohammad Alizadeh A. Drug-induced cholestatic hepatitis: how late can it occur even after the cessation of the culpable drug? Neuro Oncol. 2012 Jul;14(7):830. **Sarganas G, Orzechowski HD, Klimpel A, Thomae M, Kauffmann W, Herbst H, Bronder E, Garbe E. Severe sustained cholestatic hepatitis following temozolomide in a patient with glioblastoma multiforme: case study and review of data from the FDA adverse event reporting system. Neuro Oncol. 2012 May;14(5):541-6.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	200807	29	U/L	
Alanine aminotransferase	200810	639	U/L	
Aspartate aminotransferase	200810	109	U/L	
Alkaline phosphatase	200810	216	U/L	
Bilirubin total	200810	12.77	mg/dL	
Ultrasound abdomen	200810	no pathology	NA	
Ultrasound abdomen	200812	enlarged liver, no	NA	signs of biliary obstruction
Alanine aminotransferase	200812	166	U/L	
Aspartate aminotransferase	200812	87	U/L	

DE-BFARM-12311299			
Bilirubin total	200812	18.2	mg/dL
Pro-thrombin time	200812	normal	NA
Hepatitis B virus test	200812	negative	NA
Hepatitis C virus test	200812	negative	NA
Cytomegalovirus serology	200812	negative	NA
Epstein-Barr virus serology	200812	negative	NA
Antinuclear antibody	200812	negative	NA
Antimitochondrial antibody	200812	negative	NA
Liver-kidney microsomal antibody	200812	negative	NA
Liver-kidney microsomal antibody	200812	negative	NA
Smooth muscle antibody	200812	negative	NA
Nuclear magnetic resonance imaging abc	200812	no signs of liver or biliary tract abnormalities	
Liver biopsy	200812	pattern of cholestatic hepatitis	
Bilirubin total	200812	23.6	mg/dL
Bilirubin	200904	14.3	mg/dL
Bilirubin conjugated	200904	11.2	mg/dL
Alanine aminotransferase	200904	286	U/L
Aspartate aminotransferase	200904	75	U/L
Alkaline phosphatase	200904	545	U/L

Narratives (nur bei elektronischem Eingang)

ACTAVISPR, 26.11.2012 DE-ACTAVISPR-ACTA037619

Sender diagnosis: not reported

Narrative

This literature report was received from Iran.

A 51-year-old male patient experienced cholestatic hepatitis (PT: Hepatitis cholestatic) following treatment with paracetamol at a dose of 1000 mg three times daily for postoperative pain and whilst receiving chemotherapy with temozolomide at unknown dose for glioblastoma multiforme.

Concomitant medication was not reported.

The patient had a medical history of glioblastoma multiforme and craniotomy.

He was hospitalized for about 2 weeks after craniotomy and then discharged. The patient had been on paracetamol for postoperative pain within the 2 weeks between the craniotomy and discharge from the hospital (1000 mg 3 times per day).

Therapy with paracetamol was stopped at discharge. About one month after the craniotomy, he underwent chemotherapy with temozolomide. About 1 and a half months later, about 2 months after therapy with paracetamol was discontinued, the patient began to complain of nausea and vomiting, and after evaluations were performed, the diagnosis of cholestatic hepatitis due to temozolomide was established. However, the authors commented that "Drugs that produce predictable liver injury, such as paracetamol, usually do so within a few days and are generally a result of direct liver toxicity of the parent drug or its metabolites. However, unpredictable events manifest as overt or symptomatic disease and can occur with intermediate (1-8-week) or long (1-year) periods of latency even after cessation of the culpable medication". Action taken with temozolomide and the outcome of reaction have not been reported.

12-NOV-2012 Follow up was received as literature publication with the reference "Sarganas G, Orzechowski HD, Klimpel A, Thomae M, Kauffmann W, Herbst H, Bronder E, Garbe E. Severe sustained cholestatic hepatitis following temozolomide in a patient with glioblastoma multiforme: case study and review of data from the FDA adverse event reporting system. Neuro Oncol. 2012 May;14(5):541-6.". The occurrence country was changed to Germany. The case has been upgraded to fatal. Dexamethasone and pantoprazole were added as concomitant medication. Laboratory investigations have been added.

"A 51-year-old male came to the emergency room in mid-July 2008 because of impaired vision to the left side, gait disturbance, and right-sided temporal headache. The patient reported that he had noticed the first symptoms a couple of days ago. Ophthalmologic examination revealed homonymous hemianopsia to the left side. Magnetic resonance tomography (MRT) demonstrated a right occipital lesion suspect of a malignant brain tumor. Serum level of aspartate aminotransferase (AST) was normal (29 U/L). After tumor resection, histological examination of the tumor confirmed the diagnosis of glioblastoma multiforme. Immediately after the surgery, therapy was initiated consisting of dexamethasone (initially 2 mg 3 times a day and then 1 mg/ day) for prevention of brain edema, pantoprazole (40 mg once a day) for prevention of gastroduodenal ulcer, and paracetamol (1000 mg 3 times a day) for postoperative pain. By the end of July, the patient was discharged from the hospital, and paracetamol was discontinued. In mid-August 2008, fractionated radiotherapy (total dose 60 Gy, fractionated 5 x 2 Gy per week) with concomitant temozolomide (75 mg/m2 per day p.o.) was initiated. The radiochemotherapy regimen was completed by the end of September 2008. The day after completion, the patient complained of nausea and vomiting. On the following day, he developed generalized jaundice. At the beginning of October 2008, laboratory investigations

revealed a markedly increased serum level of alanine aminotransferase (ALT): 639 U/L (10-50 U/L); a moderately increased AST: 109 U/L (10-50 U/L); and increased alkaline phosphatase (AP): 261 U/L (<130U/L), with an ALT/AP ratio of 1.75, indicating cholestatic hepatitis. Total bilirubin was markedly increased at 12.77 mg/dL (<1 mg/dL).

During follow-up care, serum levels of hepatic enzymes and of bilirubin were determined by the general practitioner. Because of persisting jaundice, the patient was referred to a hepatologist for further diagnosis. Sonographic examination of the abdomen did not reveal pathologies of the liver or of the biliary tract.

By the end of October 2008, the patient went to the neurosurgery outpatient clinic for a postoperative follow-up. Because the liver injury was considered as possibly drug-induced, it was recommended not to proceed with the adjuvant temozolomide as monotherapy. One week later, the patient returned to the neurosurgery outpatient department and an MRT of the brain did not reveal a recurrence of the tumor. At the beginning of December 2008, the patient was again referred to the neurosurgery department because of progressive visual disturbances. A tumor relapse was excluded by MRT. At this time point, the physical examination revealed generalized jaundice and the ultrasound examination showed a modestly enlarged liver without signs of biliary obstruction. The laboratory investigations showed only modestly elevated transaminases (ALT: 166 U/L, AST: 87 U/L), whereas the total bilirubin remained markedly elevated at 18.2 mg/dL. The prothrombin time was within the normal range. The patient was referred to the gastroenterology department for further diagnostics. Viral etiology of liver disease (hepatitis B, hepatitis C, cytomegalovirus, and Epstein-Barr virus) was excluded by serological testing. Tests were negative for autoimmune markers (antinuclear antibodies, antimitochondrial antibodies, liver-kidney microsomal antibodies, liver cytosol type 1 antibodies, soluble liver antigen /liver-pancreas antigen antibodies, liver membrane antibodies, and smooth muscle antibodies). Sonographic and MRT examination of the abdomen did not reveal any pathological findings, in particular no signs of liver or biliary tract abnormalities. The finding of a partial pancreas divisum was considered unrelated to the patient's liver disease. Liver biopsy was performed and histological examination showed a pattern of cholestatic hepatitis. Morphologic changes were described as canalicular cholestasis, feathery degeneration, and necrosis of some hepatocytes, as well as faint portal infiltration with lymphocytes, plasma cells, and some polymorphonuclear leukocytes. Bile ductules were intact, and there was no evidence for intrahepatic or extrahepatic bile duct obstruction. In the absence of histological and clinical evidence for a viral, autoimmune, alcoholic, or other toxic etiology, the findings were considered consistent with toxic liver injury most likely caused by temozolomide. A therapy regimen including silymarin, ursodeoxycholic acid, and prednisolone was initiated but failed to show any therapeutic benefit. During hospitalization, the bilirubin values remained elevated within the range of 18.2 mg/dL to 23.6 mg/dL. By the end of January 2009 neurological symptoms (vertigo, gait disturbances, and lateropulsion) worsened. By the end of February 2009, the patient was referred to the Neurology Department of the University Hospital, where a tumor relapse was diagnosed. The patient was again referred to the Department of Radiotherapy. Laboratory findings from April 2009 showed bilirubin of 14.3 mg/dL (conjugated bilirubin 11.2 mg/dL), ALT: 286 U/L, AST: 75 U/L, and AP: 545 U/L. A few days after hospital discharge in April 2009, the patient died".

FIRST LOOK

Country: Deutschland

Type: Not available to sender (unknown)

case: FR-BFARM-12325258

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19620808	Age: 50 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Obstructive jaundice	20121017			176 Day	not recovered/not resolved
2 Anemia					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BACLOFEN	Oral	6 Df dosage form/Day	-	
baclofen			-	
2 BACTRIM	Oral	3*1 Df dosage form/Week	-	
sulfamethoxazole; trimethoprim			-	
3 coltramyl	Oral	3*4 Mg milligram(s)/Week	-	
thiocolchicoside			-	
4 DAFALGAN	Oral	4 G gram(s)/	-	
paracetamol			-	
5 HALDOL /00027401/	Oral	0.5 Mg milligram(s)/Day	-	
haloperidol			-	
6 INEXIUM /01479302/	Oral	40 Mg milligram(s)/Day	-	
esomeprazole magnesium			-	
7 LAROXYL	Oral	/	-	Neuropathic pain
amitriptyline hydrochloride			-	
8 LASILIX /00032601/	Oral	20 Mg milligram(s)/Day	-	
furosemide			-	
9 Levact	Intravenous (not otherwise specified)	120 mg/m2 milligram(s)/sc	20120425-	Myeloma
bendamustine hydrochloride			-	
10 LYRICA	Oral	450 Mg milligram(s)/Day	-	
pregabalin			-	
11 TEMESTA /00273201/	Unknown	1 Df dosage form/Day	-	
lorazepam			-	
12 TOPALGIC /00599202/	Oral	2 Df dosage form/Day	-	
tramadol hydrochloride			-	
13 ZOPICLONE	Oral	/	-	
zopiclone			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Femoral neck fracture	20120929		Unknown	Leading to osteosynthesis on 03OCT2012
IGG myeloma			k.A.	stage 3
Multiple myeloma			k.A.	
Pyelonephritis			k.A.	P. aruginosa pyelonephritis related to septicemia. Hospitalization from 21AUG2012 to 30AUG2012
Sepsis	20120821	20120830	No	Related to pyelonephritis

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Conjugated bilirubin level increased	20121017	123	umol/L	
Alkaline phosphatase	20121017	427	IU/l	
Bilirubin total	20121017	140	umol/L	
GGT	20121017	923	IU/l	
Hemoglobin	20121017	7	g/dl	
Aspartate aminotransferase	20121017			
Alanine aminotransferase	20121017			

number of reports: 301